

PassReady Prep — NCE Question Bank · Full bank

Generated: 2026-10-04 · Showing: In review · Total items: 1000

How to review

✓ **KEY** marks the correct option. Every option shows why it is right or wrong; check both the key and each distractor.

Mark each item **Approve**, **Revise** (note what to change) or **Retire**. Approved items are published from Admin → NCE Questions.

▮ **Check** notes mark specific facts or citations the writer was less than certain of — verify those first.

Check: one clearly best answer · factual accuracy (theorists, dates, test properties, code sections) · distractors plausible but defensibly wrong · no answer given away by length or wording · references support the key.

NCE domains (NBCC 2023 content outline)

Professional Practice and Ethics 19 · Intake, Assessment, and Diagnosis 19 · Areas of Clinical Focus 47 · Treatment Planning 14 · Counseling Skills and Interventions 48 · Core Counseling Attributes 13 — of 160 scored items

In this copy

Professional Practice and Ethics **120** · Intake, Assessment, and Diagnosis **120** · Areas of Clinical Focus **295** · Treatment Planning **85** · Counseling Skills and Interventions **300** · Core Counseling Attributes **80** · **132** flagged ▮

Professional Practice and Ethics

120 items · NCE weight 12% (19 of 160 scored items)

Q1 of 1000 · nce-s-eth-001 · Professional Counseling Orientation and Ethical Practice · informed consent and its required elements · easy · In review

A newly licensed counselor is drafting an informed consent document for a private practice. Which element is a required part of informed consent under the ACA Code of Ethics?

A. A guarantee that the presenting problem will improve within a set number of sessions

Counselors explain potential benefits and risks; promising a specific outcome is misleading.

B. An explanation of the limits of confidentiality and how fees and billing are handled ✓ **KEY**

A.2.b lists confidentiality limits and fees and billing arrangements among required disclosures.

C. A summary of the counselor's personal religious and political values for the client

Counselors avoid imposing personal values (A.4.b); disclosing them is not a consent element.

D. A signed waiver releasing the counselor from liability for any outcome of treatment

Liability waivers are not part of informed consent and do not serve client welfare.

Rationale: Informed consent gives clients the information needed to choose whether to enter or continue counseling. ACA A.2.b names elements such as the purposes, goals, techniques, risks and benefits of services, the counselor's credentials, fees and billing, the limits of confidentiality, and the client's right to refuse services. Consent is an ongoing process, not a single signature.

References: ACA Code of Ethics — A.2.b Types of Information Needed

☐ Approve ☐ Revise ☐ Retire Notes:

A 12-year-old is brought to counseling by his custodial grandmother, who holds legal guardianship. The boy says he does not want to be there. How should the counselor BEST approach consent?

- A. Treat the grandmother's signature as sufficient and skip any discussion with the boy
Legal consent from a guardian does not remove the duty to seek the minor's assent.
- B. Decline to begin services until the boy is old enough to provide legal consent alone
Minors routinely receive counseling with guardian consent; refusing service is not required.
- C. Let the boy decide alone whether to continue, since his preference overrides consent
A minor generally cannot give legal consent; the guardian's role cannot simply be set aside.
- D. Obtain the guardian's consent and seek the boy's assent in language he can understand ✓ KEY
A.2.d directs counselors to seek the assent of clients who cannot give consent and to include guardians.

Rationale: When a client cannot legally consent, counselors obtain consent from the parent or guardian and still seek the client's assent, explaining services in developmentally appropriate terms. This balances the guardian's legal rights with the minor's autonomy and welfare. Age-of-consent rules for minors vary by state, so counselors also check state law.

References: ACA Code of Ethics — A.2.d Inability to Give Consent

☐ Approve ☐ Revise ☐ Retire Notes:

A counselor is starting a grief group for adults. During the first session, what should the counselor tell members about confidentiality?

- A. That members are asked to keep what is shared private, but this cannot be guaranteed ✓ KEY
B.4.a asks counselors to explain the importance and parameters of confidentiality in group work.
- B. That group confidentiality is protected by the same law that binds the counselor
Group members are generally not bound by the counselor's legal duties; this overstates protection.
- C. That confidentiality applies only to the counselor, so members may share freely
Members should be asked to respect each other's privacy; this undermines group safety.
- D. That any member who discloses outside the group will face legal action by the counselor
This is an inaccurate threat; the counselor cannot enforce group confidentiality this way.

Rationale: In group work the counselor can promise to protect confidentiality but cannot guarantee that other members will. ACA B.4.a requires counselors to clearly explain the importance and parameters of confidentiality for the specific group. Setting this expectation at the outset supports informed consent and group trust.

References: ACA Code of Ethics — B.4.a Group Work

☐ Approve ☐ Revise ☐ Retire Notes:

A client discloses a new diagnosis of a contagious, life-threatening disease and says she has not told her long-term partner, with whom she continues to have unprotected sex. State law permits disclosure in this situation. What should the counselor do FIRST?

A. Contact the partner right away and inform him of the diagnosis without telling the client

B.2.c calls for assessing the client's intent to inform the partner before any disclosure.

B. Keep the information confidential, since health conditions are outside the limits

B.2.c recognizes that disclosure to an identifiable third party at high risk may be justified.

C. Assess whether the client intends to inform her partner or to change the risky behavior ✓ KEY

B.2.c directs counselors to assess client intent before disclosing to third parties at risk.

D. Refer the client to a medical provider and end counseling until she discloses to him

Ending services as leverage is coercive and may amount to abandonment (A.12).

Rationale: ACA B.2.c states that when clients disclose a contagious, life-threatening disease, counselors may be justified in disclosing to identifiable third parties at serious and foreseeable risk if laws permit. Before doing so, counselors assess the client's intent to inform those parties or to stop harmful behavior. State law varies, so counselors confirm what their jurisdiction permits or requires.

References: ACA Code of Ethics — B.2.c Contagious, Life-Threatening Diseases

☐ Approve ☐ Revise ☐ Retire Notes:

During a session, a client names a former coworker, states he has bought a handgun, and describes a plan to shoot her at her home this weekend. Which action is MOST consistent with the counselor's ethical duties?

A. Keep the threat confidential and explore the client's anger at the next session

A specific, imminent threat to an identifiable person is a recognized limit of confidentiality.

B. Ask the client to sign a no-harm contract and continue with the planned session work

No-harm contracts do not reduce risk and do not meet the duty to protect an identified victim.

C. Take protective steps such as warning the victim or police, as state law directs ✓ KEY

B.2.a permits disclosure to protect identified others from serious and foreseeable harm.

D. End the session early and refer the client to a psychiatrist for medication review

Referral alone does nothing to protect the identified person from imminent harm.

Rationale: Following Tarasoff v. Regents of the University of California (1976), counselors may have a duty to protect identifiable victims of serious threats. ACA B.2.a states that confidentiality does not apply when disclosure is required to protect identified others from serious and foreseeable harm. Whether the duty is mandatory or permissive, and what steps satisfy it, varies by state law.

References: ACA Code of Ethics — B.2.a Serious and Foreseeable Harm and Legal Requirements · State law — Duty-to-warn/protect statutes (mandatory vs permissive, Tarasoff)

☐ Approve ☐ Revise ☐ Retire Notes:

A client frustrated about a custody dispute says, "Sometimes I could just kill my ex." He names no plan, has no weapons, has no history of violence, and laughs it off. What is the counselor's BEST response?

A. Explore the statement further to assess intent, plan, means, and history of violence ✓ KEY

An ambiguous statement calls for violence risk assessment before any decision to disclose.

B. Notify the ex-spouse and local police immediately that a threat has been made

Breaching confidentiality without assessing seriousness or foreseeability is premature.

C. Ignore the remark, since venting about an ex-spouse is common in custody conflicts

Dismissing a statement of violence without assessment risks missing real danger.

D. Terminate services because the client has made a threat against another person

Termination does not address risk and may leave the client and others less safe.

Rationale: The duty to protect is triggered by serious and foreseeable harm, not by every angry remark. The counselor should assess intent, plan, means, target, and history, consult as needed, and document the reasoning. If assessment reveals a serious threat, protective action under B.2.a and state law follows.

References: ACA Code of Ethics — B.2.a Serious and Foreseeable Harm and Legal Requirements

☐ Approve ☐ Revise ☐ Retire Notes:

Q7 of 1000 · nce-s-eth-007 · Professional Counseling Orientation and Ethical Practice · mandated reporting of abuse and neglect · medium · In review

A school-based counselor notices bruises in the shape of a hand on a 7-year-old's arm. The child says, "Dad gets mad," and then stops talking. The counselor is unsure whether abuse occurred. What should the counselor do?

A. Gather more evidence by questioning the parents before deciding whether to report

Investigating is the role of child protective services and may place the child at risk.

B. Wait to see whether new injuries appear before making a report to the authorities

Delay is not permitted once reasonable suspicion exists; certainty is not required.

C. Ask the child's teacher to make the report so the counseling relationship is protected

In most states the duty to report is personal and cannot be handed off to a colleague.

D. Make a report to child protective services based on reasonable suspicion of abuse ✓ KEY

Mandated reporters report reasonable suspicion; proof or certainty is not required.

Rationale: All U.S. states have mandated reporting laws for suspected child abuse and neglect that cover counselors, though definitions, thresholds, and procedures vary by state. The threshold is reasonable suspicion or cause to believe, not proof, and the counselor should not investigate. Specific timelines, reporting agencies, and procedures vary by state law, and many school policies add internal notification steps.

References: State law — Child abuse reporting statutes: reasonable suspicion standard · ACA Code of Ethics — B.2.a Serious and Foreseeable Harm and Legal Requirements

☐ Approve ☐ Revise ☐ Retire Notes:

A counselor in a HIPAA-covered agency keeps process notes about session dynamics and her own impressions in a file separate from the medical record. Under the HIPAA Privacy Rule, how are these notes treated?

A. They are part of the designated record set and may be released for any payment purpose

Separately kept psychotherapy notes are excluded from routine payment disclosures.

B. They are psychotherapy notes, and most disclosures require a specific authorization ✓ KEY

HIPAA defines psychotherapy notes and requires specific authorization for most uses.

C. They lose all protection once a client signs a general release of medical information

A general release does not suffice; psychotherapy notes need their own authorization.

D. They must be given to the client on request under the HIPAA right of access to records

Psychotherapy notes are excluded from the HIPAA right of access, though state law may differ.

Rationale: HIPAA gives psychotherapy notes, meaning a counselor's notes analyzing session content that are kept separate from the medical record, special protection. Most uses and disclosures require a specific authorization, and they are excluded from the client's general right of access. Items such as diagnosis, medications, session times, and treatment plans are not psychotherapy notes.

References: HIPAA — Privacy Rule: psychotherapy notes definition and authorization (45 CFR 164.501, 164.508)

☐ Approve ☐ Revise ☐ Retire Notes:

A competent adult client asks to see a copy of her counseling records. What does the ACA Code of Ethics direct the counselor to do?

A. Provide reasonable access, limiting it only with compelling evidence of harm to her ✓ KEY

B.6.e directs reasonable access, limited only when access would cause the client harm.

B. Refuse the request, since counseling records belong only to the counselor and agency

Clients have a right of reasonable access to their own records under B.6.e.

C. Release the records only after she obtains a court order or a lawyer makes a request

No court order is needed for a competent client to access her own records.

D. Give her a summary only, because clients may not see the counselor's written records

The Code does not limit clients to summaries; it supports access and help interpreting records.

Rationale: ACA B.6.e states that counselors provide reasonable access to records and copies when requested by competent clients. Access is limited only when there is compelling evidence that it would cause harm to the client, and counselors document the request and their reasoning. Counselors also offer assistance in interpreting records (B.6.f).

References: ACA Code of Ethics — B.6.e Client Access

☐ Approve ☐ Revise ☐ Retire Notes:

Two years after ending counseling with a client, a counselor runs into the former client at a community event, and the former client asks the counselor out on a date. Under the ACA Code of Ethics, what applies?

A. A romantic relationship is allowed now, since more than one year has passed since the end

The ACA standard is five years, not one year.

B. A romantic relationship is allowed if a colleague agrees there is no risk of exploitation

Consultation does not override the five-year prohibition.

C. A romantic relationship is prohibited for five years after the last professional contact ✓ KEY

A.5.c prohibits such relationships for five years following the last professional contact.

D. A romantic relationship is permitted as long as the former client initiated the request

Who initiates does not change the prohibition; responsibility rests with the counselor.

Rationale: ACA A.5.c prohibits sexual or romantic relationships with former clients, their romantic partners, or family members for five years after the last professional contact, in person or electronic. Even after five years, counselors must show forethought and document that the relationship is not exploitative. The responsibility rests with the counselor regardless of who initiates.

References: ACA Code of Ethics — A.5.c Sexual and/or Romantic Relationships With Former Clients

☐ Approve ☐ Revise ☐ Retire Notes:

Q11 of 1000 · nce-s-eth-011 · Counseling and Helping Relationships · multiple relationships and boundary crossings · medium · In review

A counselor in a small rural town is invited by a long-term client to attend the client's high school graduation, which the client has worked toward despite major barriers. The counselor believes attending could benefit the client. What should the counselor do?

A. Decline, because any contact outside the office is an ethical violation for counselors

The Code permits boundary extensions that benefit clients when handled with safeguards.

B. Weigh benefits and risks, discuss it with the client, and document the rationale ✓ KEY

A.6.b asks counselors to document rationale, benefit, and anticipated consequences.

C. Attend without discussion, since a graduation is a public event open to everyone

A boundary extension should be discussed and documented, not done casually.

D. Attend only if the client agrees to end counseling before the graduation ceremony

Terminating services to permit contact is unnecessary and harms the client.

Rationale: ACA A.6.b recognizes that extending boundaries, such as attending a formal ceremony, can benefit clients. Before doing so, counselors document the rationale, potential benefit, and anticipated consequences, and take precautions such as informed consent, consultation, and supervision. Rural settings make such decisions more common, which increases the need for careful reasoning.

References: ACA Code of Ethics — A.6.b Extending Counseling Boundaries

☐ Approve ☐ Revise ☐ Retire Notes:

A licensed counselor completes a one-day introductory workshop on a specialized trauma therapy and wants to start using it with several clients who have complex trauma. Which course of action BEST meets the counselor's ethical obligations?

A. Use the method now, since the counselor's license covers all trauma treatments

A license defines legal scope, not competence in every specialized method.

B. Use the method now but tell clients it is experimental and get written consent

Consent does not replace the training needed to use a new specialty competently.

C. Use the method now, since a workshop certificate shows enough competence to practice

A single introductory workshop rarely provides the training and supervision C.2.b requires.

D. Pursue further training and supervised practice before using it, and refer if needed ✓ KEY

C.2.b requires education, training, and supervised experience before practicing a new specialty.

Rationale: ACA C.2.a limits practice to the boundaries of competence based on education, training, supervised experience, and credentials. When entering a new specialty area, C.2.b requires appropriate education, training, and supervised experience, and steps to protect clients from harm while skills develop. Referral is appropriate when the counselor cannot yet provide competent care.

References: ACA Code of Ethics — C.2.b New Specialty Areas of Practice · ACA Code of Ethics — C.2.a Boundaries of Competence

☐ Approve ☐ Revise ☐ Retire Notes:

A counselor wants guidance on keeping separate professional and personal social media profiles. Which section of the 2014 ACA Code of Ethics MOST directly addresses this issue?

A. Section B, Confidentiality and Privacy

Section B covers privacy broadly but social media is addressed in Section H.

B. Section D, Relationships With Others

Section D addresses relationships with colleagues and other professionals.

C. Section C, Professional Responsibility

Section C covers competence, advertising, and credentials, not social media profiles.

D. Section H, Distance Counseling ✓ KEY

Section H covers distance counseling, technology, and social media, including H.6.

Rationale: The 2014 ACA Code has nine sections, A through I. Section H, Distance Counseling, Technology, and Social Media, was new in 2014; H.6.a directs counselors who keep both a professional and a personal presence on social media to create separate profiles. Knowing the section layout helps counselors find guidance quickly.

References: ACA Code of Ethics — Section H; H.6.a Virtual Professional Presence

☐ Approve ☐ Revise ☐ Retire Notes:

A licensed counselor who also holds the National Certified Counselor (NCC) credential is accused of an ethics violation. Which statement about the NBCC Code of Ethics is accurate?

A. It replaces the state licensing board, so state law no longer applies to the counselor

State licensure law applies independently of national certification.

B. It binds NCCs, and NBCC can act on the certification apart from any state board action ✓ **KEY**

NBCC enforces its Code for its certificants; boards enforce licensure separately.

C. It applies only to counselor educators who teach in CACREP-accredited graduate programs

The NBCC Code applies to NCCs and applicants, not only to educators.

D. It is advisory only, so NBCC cannot suspend or revoke a credential for a violation

NBCC can impose sanctions on certification, including revocation.

Rationale: The NBCC Code of Ethics applies to National Certified Counselors and applicants for NBCC credentials. NBCC can investigate complaints and sanction or revoke certification, which is separate from any action a state licensing board takes on a license. A counselor may therefore answer to more than one body for the same conduct.

References: NBCC Code of Ethics — Scope and application to NCCs and applicants; enforcement of certification

☐ Approve ☐ Revise ☐ Retire Notes:

Q15 of 1000 · nce-s-eth-015 · Professional Counseling Orientation and Ethical Practice · ethical decision-making models · medium · In review

While evaluating a chosen course of action, a counselor asks, "Would I be comfortable if this decision were reported in the local newspaper?" Which test from Stadler's model is the counselor applying?

A. The test of justice

Justice asks whether the counselor would treat others in the same situation the same way.

B. The test of universality

Universality asks whether the counselor would recommend the action to others in the same situation.

C. The test of publicity ✓ **KEY**

Publicity asks whether the counselor would be comfortable having the action made public.

D. The test of fidelity

Fidelity is a moral principle about keeping commitments, not one of Stadler's tests.

Rationale: Many ethical decision-making models, including the Forester-Miller and Davis practitioner's guide, include Stadler's three tests for evaluating a selected action. Justice asks whether others would be treated the same, publicity asks whether the counselor would accept public reporting, and universality asks whether the counselor would recommend it to colleagues. Passing all three supports the soundness of the decision.

References: Forester-Miller & Davis (Practitioner's Guide to Ethical Decision Making) — Step 6, evaluating the selected course of action: Stadler's tests of justice, publicity, and universality

☐ Approve ☐ Revise ☐ Retire Notes:

A clinical supervisor learns that a post-master's supervisee has not told clients that she is under supervision or that their cases are discussed with the supervisor. What is the supervisor's MOST appropriate response?

A. Require the supervisee to inform clients of her status and how supervision affects privacy ✓ **KEY**

F.1.c requires that clients know supervisee qualifications and supervision's effect on confidentiality.

B. Allow it, since supervision is internal and does not change confidentiality for clients

Case discussion with a supervisor is a disclosure clients must be told about.

C. Report the supervisee to the licensing board before discussing the problem with her

Corrective supervision is the first step; a board report is premature here.

D. End the supervision contract at once and tell the supervisee to find a new supervisor

Abrupt termination leaves clients unprotected and skips remediation.

Rationale: A supervisor's primary obligation is to monitor the welfare of the clients served by the supervisee (F.1.a). Supervisors ensure that clients are aware of the supervisee's qualifications and of how the supervision process affects the limits of confidentiality (F.1.c). Supervisors share responsibility for client care and should correct the problem promptly through supervision.

References: ACA Code of Ethics — F.1.c Informed Consent and Client Rights · ACA Code of Ethics — F.1.a Client Welfare

☐ Approve ☐ Revise ☐ Retire Notes:

Q17 of 1000 · nce-s-eth-017 · Professional Counseling Orientation and Ethical Practice · licensure, credentialing and professional identity · easy · In review

Which statement BEST describes the difference between state licensure and national certification for counselors?

A. Licensure is legal permission to practice in a state; certification is a voluntary credential ✓ **KEY**

Licensure is state regulation; certifications such as the NCC are voluntary and national.

B. Certification is required by law in every state; licensure is optional for private practice

This reverses the two; licensure is the legal requirement for independent practice.

C. Licensure and certification are the same credential, issued by different agencies by region

They are distinct credentials with different legal weight and issuing bodies.

D. Certification allows practice in all states; licensure applies only to school counselors

National certification does not authorize practice in any state.

Rationale: Licensure is a state legal credential that regulates the practice and title of professional counseling within that jurisdiction. National certification, such as the NBCC's National Certified Counselor credential, is voluntary and shows that a counselor met national standards but does not by itself permit practice. Interstate practice is addressed through state law and mechanisms such as the Counseling Compact.

References: Gladding (Counseling: A Comprehensive Profession) — Credentialing: licensure, certification, and accreditation

☐ Approve ☐ Revise ☐ Retire Notes:

A counselor learns that a client who uses a wheelchair is being denied a reasonable accommodation by her landlord. The counselor wants to contact the housing authority about this client's case. What must the counselor do FIRST?

A. Contact the housing authority right away, since advocacy is a core professional duty

Advocating for an identifiable client without consent breaches confidentiality.

B. Avoid advocacy entirely, because it falls outside the counselor's professional role

A.7.a affirms advocacy to address barriers to client growth and development.

C. Write a public letter about the landlord to raise awareness of housing discrimination

Publicizing the case exposes the client and still lacks her consent.

D. Obtain the client's consent before advocating on her behalf as an identifiable client ✓ **KEY**

A.7.b requires client consent before advocating on behalf of an identifiable client.

Rationale: ACA A.7.a supports advocacy at individual, group, institutional, and societal levels to address barriers that inhibit client growth. A.7.b requires counselors to obtain client consent before engaging in advocacy on behalf of an identifiable client. Seeking consent also respects the client's autonomy and may support her own self-advocacy.

References: ACA Code of Ethics — A.7.b Confidentiality and Advocacy

☐ Approve ☐ Revise ☐ Retire Notes:

Q19 of 1000 · nce-s-eth-019 · Professional Counseling Orientation and Ethical Practice · telehealth and technology ethics · medium · In review

A counselor licensed in one state is asked to provide video counseling to a client who lives in another state. Which consideration is MOST important before beginning?

A. Only the laws of the counselor's home state apply, since that is where the license is

H.1.b notes that the client's state laws may also apply.

B. The counselor may be subject to the laws of both the counselor's and client's locations ✓ **KEY**

H.1.b directs counselors to know the laws of both locations in distance counseling.

C. Licensure rules do not apply to video sessions, since no in-person contact takes place

Distance counseling is still the practice of counseling and is regulated.

D. Any license is valid nationwide for telehealth as long as the platform is encrypted

Encryption addresses security, not legal authority to practice across states.

Rationale: ACA H.1.b states that counselors providing distance services understand they may be subject to the laws and regulations of both their practicing location and the client's place of residence. In practice, many states regulate based on where the client is located, and rules vary by state. Interstate options such as the Counseling Compact apply only among member states.

References: ACA Code of Ethics — H.1.b Laws and Statutes · Counseling Compact — Privilege to practice in member states

☐ Approve ☐ Revise ☐ Retire Notes:

A counselor is starting video sessions with a new client who has a history of suicidal ideation. Which step is MOST important to complete during the first telehealth session?

A. Ask the client to keep the camera off so that the session feels less intrusive

Turning off video reduces the counselor's ability to assess and does not address safety.

B. Record the session so it can be reviewed later if any emergency happens

Recording requires consent and does nothing to prepare for an emergency.

C. Confirm the client's location and a local emergency plan and contact person ✓ KEY

Knowing the client's location and local resources is essential for responding to a crisis.

D. Use any free video app the client prefers so that the client feels comfortable

Platforms must protect confidentiality; preference alone is not the standard (H.2.d).

Rationale: Distance counseling requires planning for emergencies the counselor cannot handle in person. Counselors confirm the client's physical location, identify local emergency services, and agree on a plan as part of telehealth informed consent (H.2.a). This is especially important for clients with elevated risk.

References: ACA Code of Ethics — H.2.a Informed Consent and Disclosure (includes emergency procedures when the counselor is unavailable)

☐ Approve ☐ Revise ☐ Retire Notes:

Q21 of 1000 · nce-s-eth-021 · Social and Cultural Diversity · termination and referral · medium · In review

A counselor with strong religious beliefs is assigned a client who wants help with stress in her same-sex marriage. The counselor feels uncomfortable and wants to refer the client elsewhere. What does the ACA Code of Ethics state?

A. Referral is appropriate whenever a client's issue conflicts with the counselor's faith

A.11.b prohibits referral based solely on the counselor's personally held values.

B. Referral is required, since counselors must avoid any client whose values differ

The Code calls for respecting diversity and avoiding value imposition, not avoiding clients.

C. Referral based only on the counselor's personal values is not permitted by the Code ✓ KEY

A.11.b prohibits values-based referral; counselors seek training to work with diverse clients.

D. Referral is acceptable if the counselor explains the religious reason to the client

Explaining the reason does not make a values-based referral ethical and may harm the client.

Rationale: ACA A.11.b states that counselors refrain from referring prospective and current clients based solely on the counselor's personally held values, attitudes, beliefs, and behaviors. A.4.b directs counselors to avoid imposing their own values. Counselors seek training and supervision when values affect their work, and referral is reserved for genuine gaps in competence.

References: ACA Code of Ethics — A.11.b Values Within Termination and Referral · ACA Code of Ethics — A.4.b Personal Values

☐ Approve ☐ Revise ☐ Retire Notes:

A counselor notices that a colleague has been arriving late, smelling of alcohol, and falling asleep in team meetings. The colleague carries a full caseload. What should the counselor do FIRST, if it can be done safely?

A. Speak privately with the colleague about the concerns and encourage seeking help ✓ KEY

C.2.g and I.2.a support helping colleagues recognize impairment and informal resolution first.

B. File a complaint with the state licensing board without speaking to the colleague

Formal reports follow when informal steps fail or clients face imminent harm.

C. Tell the colleague's clients about the concerns so that they can make choices

This breaches the colleague's privacy and is not the counselor's role.

D. Say nothing, since a colleague's personal life is outside the counselor's concern

The Code expects counselors to act when colleague impairment may harm clients.

Rationale: ACA C.2.g asks counselors to monitor themselves for impairment and to assist colleagues or supervisors in recognizing their own impairment, providing consultation and help when warranted. I.2.a directs counselors to first attempt informal resolution with a colleague when feasible. If the problem persists or clients are at substantial risk, counselors take further action, such as reporting to supervisors or the licensing board.

References: ACA Code of Ethics — C.2.g Impairment · ACA Code of Ethics — I.2.a Informal Resolution

☐ Approve ☐ Revise ☐ Retire Notes:

Q23 of 1000 · nce-s-eth-023 · Professional Counseling Orientation and Ethical Practice · professional organizations and their roles · easy · In review

A student comparing master's programs in clinical mental health counseling asks which organization accredits counseling graduate programs. Which organization is it?

A. NBCC

The National Board for Certified Counselors (NBCC) grants national certification (such as the NCC) and administers the NCE.

B. CACREP ✓ KEY

The Council for Accreditation of Counseling and Related Educational Programs (CACREP) accredits master's and doctoral counseling programs.

C. ACA

The American Counseling Association (ACA) is a professional membership association that publishes the Code of Ethics.

D. AMHCA

The American Mental Health Counselors Association (AMHCA) is a professional association for clinical mental health counselors.

Rationale: CACREP, the Council for Accreditation of Counseling and Related Educational Programs, accredits counseling graduate programs. NBCC certifies individual counselors and administers the NCE, while ACA and its divisions and affiliates, such as AMHCA and ASCA, are membership associations. State licensing boards regulate practice within each state.

References: CACREP 2024 Standards — Accreditation of counseling master's and doctoral programs · Gladding (Counseling: A Comprehensive Profession) — Professional organizations and accreditation

☐ Approve ☐ Revise ☐ Retire Notes:

A counselor researcher wants to study help-seeking and plans to briefly mislead participants about the study's true aim, because full disclosure would bias their answers. Under the ACA Code of Ethics, when is this acceptable?

- A. Whenever participants have signed a general consent form for the overall study
General consent does not by itself justify deception.
- B. Whenever the researcher believes the study results will be published widely
Publication plans do not justify deceiving participants.
- C. Whenever participants are students, since course credit offsets any harm done
Students need added protection, and credit does not offset deception.
- D. Only if no alternative is feasible, the value justifies it, and debriefing follows ✓ KEY
G.2 permits deception only when alternatives are not feasible, value justifies it, and debriefing occurs.

Rationale: The ACA Code allows deception in research only when alternative procedures are not feasible and the prospective value of the research justifies it. Deception that is likely to cause physical or emotional harm is not permitted, and researchers explain the deception during debriefing as soon as feasible. Institutional review boards also review such designs.

References: ACA Code of Ethics — G.2 Rights of Research Participants (deception and debriefing)

► **Check:** Cited as ACA G.2 generally; confirm subsection letters for deception and debriefing.

☐ Approve ☐ Revise ☐ Retire Notes:

Q25 of 1000 · nce-s-eth-025 · Professional Counseling Orientation and Ethical Practice · informed consent and its required elements · medium · In review

A client in private practice stopped attending sessions and has an unpaid balance of several hundred dollars. The counselor is considering turning the account over to a collection agency. What should the counselor do FIRST?

- A. Inform the client of the intended collection step and give a chance to pay the balance ✓ KEY
A.10.d requires counselors to inform clients of intended collection actions and offer an opportunity to pay first.
- B. Send the account to the agency right away, since the client has already ended counseling
Ending services does not remove the duty to notify the client before using collection measures.
- C. Write off the debt, because counselors may not pursue payment from any current or past client
The Code permits collection agencies or legal measures when clients are informed first.
- D. Give the agency the client's diagnosis and session notes so it can justify the charges
Only the minimum information needed for collection is disclosed; clinical details are not shared.

Rationale: Under A.10.d, counselors who intend to use collection agencies or legal measures for nonpayment first tell the client what they intend and give the client an opportunity to pay. Fee and billing arrangements, including how nonpayment is handled, also belong in the initial informed consent (A.2.b). Any disclosure to a collector is limited to what is needed.

References: ACA Code of Ethics — A.10.d Nonpayment of Fees

☐ Approve ☐ Revise ☐ Retire Notes:

In a rural farming community where trading services is common, a client who recently lost his job asks whether he could repair the counselor's office roof in exchange for sessions. Under the ACA Code of Ethics, when is bartering acceptable?

A. Whenever the counselor sets the value of the work, since the counselor bears responsibility

One-sided valuation by the counselor increases the risk of exploitation.

B. Only when the counselor proposes it first, so that the client does not feel pressured to offer

The Code says the client should request the arrangement, not the counselor.

C. If the client requests it, it is not exploitive, it is accepted locally, and it is in writing ✓ **KEY**

A.10.e lists client request, no exploitation or harm, and community acceptance, with a written contract.

D. Only for clients with no insurance, as long as a supervisor signs off on the trade in advance

Insurance status and supervisor approval are not the conditions the Code sets for bartering.

Rationale: A.10.e allows bartering only when the client requests it, the arrangement does not result in exploitation or harm, and such arrangements are an accepted practice among professionals in the community. Counselors consider cultural implications, discuss concerns with the client, and document the agreement in a clear written contract.

References: ACA Code of Ethics — A.10.e Bartering

☐ Approve ☐ Revise ☐ Retire Notes:

A counselor accepts a job in another state and will close her practice in two months. Several of her clients are still working on active treatment goals. Which action BEST avoids client abandonment?

A. Stop scheduling sessions now and mail each client a letter after the office has closed

Ending contact abruptly without preparation or referral is a form of abandonment.

B. Keep seeing clients until the last day and leave referrals to the clients' own judgment

Counselors help arrange continued care rather than leaving clients to find it alone.

C. Ask clients to keep texting her after the move so that their counseling can continue informally

Informal contact across state lines raises licensure and boundary problems, not continuity.

D. Tell clients early, provide pretermination counseling, and help arrange suitable referrals ✓ **KEY**

A.11.c and A.12 call for timely notice, pretermination counseling, and referral to other providers.

Rationale: A.12 prohibits abandoning or neglecting clients and requires arrangements for continuation of treatment during interruptions such as relocation. A.11.c and A.11.d direct counselors to provide pretermination counseling, recommend other providers, and coordinate an appropriate transfer of services.

References: ACA Code of Ethics — A.12 Abandonment and Client Neglect · ACA Code of Ethics — A.11.c Appropriate Termination

☐ Approve ☐ Revise ☐ Retire Notes:

A counselor receives a subpoena from the attorney for a client's former spouse, demanding the client's full counseling file for a custody hearing. The client has not signed a release. What is the counselor's BEST course of action?

A. Send the complete file at once, since a subpoena carries the same force as a court order

An attorney-issued subpoena generally does not by itself authorize release of privileged records.

B. Contact the client, seek legal advice, and release nothing without consent or a court order ✓ KEY

B.2.d and state privilege law favor client consent or a judicial order, with disclosure limited.

C. Ignore the subpoena, because counseling records are privileged and cannot be required

Ignoring a subpoena can bring legal penalties; privilege must be asserted, not assumed.

D. Call the former spouse's attorney to discuss the client's progress and resolve it informally

Discussing the client with the opposing attorney is itself a breach of confidentiality.

Rationale: A subpoena is a demand that must be answered, but it is not the same as a judge's order to disclose. Counselors notify the client, consult an attorney, and assert privilege when appropriate. Under B.2.d, even when a court orders disclosure, counselors seek the client's written consent or ask that disclosure be prohibited or limited as narrowly as possible. State law varies.

References: ACA Code of Ethics — B.2.d Court-Ordered Disclosure · State law — Privileged communication and response to subpoenas vs court orders

► **Check:** Verify that the subpoena-vs-court-order distinction is stated accurately enough for a general (non-state-specific) NCE item.

☐ Approve ☐ Revise ☐ Retire Notes:

Q29 of 1000 · nce-s-eth-029 · Professional Counseling Orientation and Ethical Practice · confidentiality and its limits · medium · In review

Three months after a long-term client dies of natural causes, her adult son calls the counselor and asks for copies of his mother's session notes so he can "understand her better." What should guide the counselor's response?

A. Release the notes, since confidentiality obligations end once a client has died

B.3.f states that counselors protect the confidentiality of deceased clients.

B. Release the notes, since any adult child is the legal heir to a parent's own records

Family relationship alone does not confer legal authority over a decedent's records.

C. Protect her confidentiality, following state law on who may act for the deceased ✓ KEY

B.3.f protects deceased clients' confidentiality consistent with law and agency policy.

D. Share a verbal summary instead, since a summary does not breach confidentiality

A summary is still a disclosure of confidential information without valid authorization.

Rationale: B.3.f directs counselors to protect the confidentiality of deceased clients, consistent with legal requirements and agency or setting policies. Typically only a legally authorized personal representative, such as an executor, may authorize release, and who qualifies is set by state law, which varies.

References: ACA Code of Ethics — B.3.f Deceased Clients · State law — Personal representative authority over a deceased person's records

☐ Approve ☐ Revise ☐ Retire Notes:

A licensed counselor wants advice from a peer consultation group about a difficult case involving a well-known local business owner. The client has not given consent to discuss the case. How should the counselor proceed?

A. Present the case without details that could reasonably identify the client to the group ✓ KEY

B.7 bars identifying disclosures in consultation without consent, sharing only what is needed.

B. Share the client's name, since members of a consultation group are bound by confidentiality

Peer consultants do not share the counselor's duty in a way that permits identifying the client.

C. Avoid consultation altogether, since any discussion of a case breaches client confidentiality

Consultation is encouraged; it is done with de-identified information or with consent.

D. Share full details but ask the group to sign a confidentiality agreement before the meeting

A group agreement does not replace client consent for identifying disclosures.

Rationale: In case consultation, counselors do not disclose information that could reasonably lead to the client's identification unless the client has consented or disclosure cannot be avoided, and they share only what is needed for the consultation. With a well-known client, extra care is needed to remove identifying details.

References: ACA Code of Ethics — B.7 Case Consultation

☐ Approve ☐ Revise ☐ Retire Notes:

A counselor is about to begin couples counseling with two partners in their fifties. To prevent problems later, what should the counselor do regarding confidentiality at the outset?

A. Promise each partner that anything said in an individual call will stay secret from the other

Blanket secret-keeping promises can undermine the couple work and should not be made by default.

B. Tell both partners that the counselor will decide case by case what to reveal between them

An unannounced, case-by-case approach leaves clients unable to give informed consent.

C. Skip the topic, since confidentiality rules do not apply when both partners are present

B.4.b requires discussing confidentiality expectations and limits in couples work.

D. Define who the client is and get a written agreement on how confidences will be handled ✓ KEY

B.4.b asks counselors to define the client and document agreement on confidentiality.

Rationale: In couples and family counseling, B.4.b directs counselors to clearly define who is considered the client and to discuss expectations and limits of confidentiality. Counselors seek agreement among all parties, document it in writing, and A.8 adds that the nature of each relationship is clarified at the outset.

References: ACA Code of Ethics — B.4.b Couples and Family Counseling · ACA Code of Ethics — A.8 Multiple Clients

☐ Approve ☐ Revise ☐ Retire Notes:

A counselor is forming a 10-week psychoeducational group on managing workplace stress. According to the ACA Code of Ethics, what is the main purpose of screening prospective members?

A. To make sure the group is full before the first meeting so the program stays funded

Enrollment targets are not the ethical purpose of screening.

B. To select members whose goals fit the group and whose well-being will not be at risk ✓ **KEY**

A.9.a calls for selecting compatible members who will not impede the group or be harmed.

C. To collect enough personal history that members need not share anything in sessions

Screening is about fit and safety, not replacing disclosure during the group.

D. To exclude any applicant who has a mental health diagnosis from a psychoeducation group

A diagnosis alone is not a reason to exclude; fit with group goals is the standard.

Rationale: A.9.a directs counselors to screen prospective group members to the extent possible, selecting members whose needs and goals are compatible with the group's goals, who will not impede the group process, and whose well-being will not be jeopardized by the group experience.

References: ACA Code of Ethics — A.9.a Screening · Yalom & Leszcz (Group Psychotherapy) — Selection of clients for group therapy

☐ Approve ☐ Revise ☐ Retire Notes:

During the fourth session of an adolescent anger-management group, several members begin mocking a quieter member's accent, and she starts to cry. What is the leader's MOST appropriate response?

A. Intervene at once to stop the mocking and protect the member, then process what happened ✓ **KEY**

A.9.b requires reasonable precautions to protect group members from emotional harm.

B. Let the group handle it alone, since peer feedback is how adolescent groups mature over time

Allowing ridicule to continue fails the leader's duty to protect members.

C. Ask the upset member to leave until she is calm enough to rejoin the group discussion

Removing the targeted member punishes her rather than addressing the harmful behavior.

D. Note the incident and raise it with each member's parents after the session has ended

Delaying action leaves the member unprotected during the session.

Rationale: In a group setting, A.9.b directs counselors to take reasonable precautions to protect clients from physical, emotional, or psychological trauma. Blocking harmful interactions is a core leader skill; once safety is restored, the incident can be processed, including the bias it reflects.

References: ACA Code of Ethics — A.9.b Protecting Clients · Yalom & Leszcz (Group Psychotherapy) — Leader role in establishing group norms and safety

☐ Approve ☐ Revise ☐ Retire Notes:

An agency director asks a newly licensed counselor to administer and interpret an individually administered intelligence test. The counselor has never been trained on this instrument. What should the counselor do?

A. Administer it after reading the manual, since the license covers all forms of assessment

Reading a manual does not establish competence with a complex individual test.

B. Administer it but have the computer scoring report serve as the clinical interpretation

Counselors remain responsible for interpretation even when scoring is automated (E.9).

C. Decline until trained and supervised in its use, and refer the testing to a qualified peer ✓ KEY

E.2.a limits counselors to assessments they are trained and competent to use.

D. Administer it, since the director has the authority to approve staff for any test that is needed

An employer cannot confer competence; the ethical duty rests with the counselor.

Rationale: E.2.a states that counselors use only those testing and assessment services for which they have been trained and are competent. Counselors who rely on technology-assisted interpretation are trained in the construct and the instrument before using it. Until competent, the counselor refers the assessment.

References: ACA Code of Ethics — E.2.a Limits of Competence · Hays (Assessment) — Test user qualifications

☐ Approve ☐ Revise ☐ Retire Notes:

A client involved in a disability claim signs a release asking the counselor to send her raw personality test responses and scores directly to her attorney, who has no training in psychological testing. How should the counselor respond under the ACA Code of Ethics?

A. Send the raw data, since a signed release obligates the counselor to send anything asked for

Consent is necessary but E.4 also limits release to qualified recipients.

B. Refuse any release, since test results may not leave the counselor's office in any form

Results can be released with consent; the issue is who receives raw data.

C. Send the raw data only after the attorney promises not to share it with the opposing side

A promise of nondisclosure does not make an untrained recipient qualified to interpret.

D. Offer a written interpretive report, or send raw data to a professional qualified to use it ✓ KEY

E.4 releases identified data with consent only to persons qualified to interpret them.

Rationale: E.4 states that counselors release assessment data in which the client is identified only with consent of the client or a legal representative, and only to persons the counselor recognizes as qualified to interpret the data. A report written for lay readers, or release to a qualified professional, meets both the client's need and the standard.

References: ACA Code of Ethics — E.4 Release of Data to Qualified Personnel · HIPAA — Individual right of access to the designated record set

► **Check:** Verify how E.4 interacts with HIPAA and state client-access rights to raw test data, which may require release to the client.

☐ Approve ☐ Revise ☐ Retire Notes:

A counselor plans to use a depression inventory with a client who immigrated from Vietnam a year ago and is learning English. The inventory was normed mainly on U.S.-born English speakers. What is the BEST ethical approach?

A. Use the inventory as usual, since depression symptoms appear the same across all cultures

Symptom expression and item meaning can differ across cultures and languages.

B. Use it with caution, note its norm limits, and weigh results with interviews and context ✓ KEY

E.8 calls for cautious use of measures normed on other groups and cultural context.

C. Translate the items on the spot and score the client's answers with the published norms

An informal translation changes the test, so the published norms no longer apply.

D. Skip all formal assessment, since no standardized test can be fair to a recent immigrant

Assessment can be used with appropriate caution; avoiding it entirely is not required.

Rationale: E.8 directs counselors to select and use with caution assessment techniques normed on populations other than the client's, and to recognize the effects of culture, language, and other factors on administration and interpretation. Results are placed in the context of other information, and limits are noted in the report.

References: ACA Code of Ethics — E.8 Multicultural Issues/Diversity in Assessment · Hays (Assessment) — Culturally fair assessment and norm-group relevance

☐ Approve ☐ Revise ☐ Retire Notes:

Q37 of 1000 · nce-s-eth-037 · Professional Counseling Orientation and Ethical Practice · supervision and consultation responsibilities · hard · In review

A site supervisor has given a post-master's supervisee written feedback for six months about missed risk assessments and late notes. Despite a written remediation plan, the problems continue. What is the supervisor's MOST appropriate next step?

A. Document the deficits, consult, and decline to endorse licensure while following due process ✓ KEY

F.6.b supports remediation, consultation, and withholding endorsement with due process.

B. Endorse the supervisee for licensure anyway, since all of the required hours have been completed

Hours alone do not justify endorsing a supervisee who is not competent.

C. Quietly end supervision and let the supervisee find another supervisor without a record

Ending without documentation or due process sidesteps the gatekeeping role.

D. Report the supervisee to the licensing board for misconduct without telling the supervisee

Performance deficits call for documented gatekeeping steps, not a secret misconduct report.

Rationale: Supervisors act as gatekeepers. Under F.6.b, they identify limitations that could impede performance, help secure remediation, and when needed recommend dismissal or decline to endorse supervisees who are unable to provide competent services. They consult, document their decisions, and ensure the supervisee knows the options for addressing the decision.

References: ACA Code of Ethics — F.6.b Gatekeeping and Remediation

☐ Approve ☐ Revise ☐ Retire Notes:

A counselor educator teaching a group counseling course requires students to take part in an experiential group as members. How should the educator handle evaluation of students in this experience?

A. Grade each student on how much personal material the student discloses in the group

Basing grades on depth of self-disclosure is what the Code prohibits.

B. Share notable disclosures with faculty so they can assess students' fitness for practice

Students should know the risks of self-disclosure; casual sharing of it misuses the group.

C. Keep grades apart from the level of self-disclosure and explain the risks of disclosing ✓ KEY

Section F requires that evaluation not depend on self-disclosure and that risks be explained.

D. Require that students disclose a past trauma, since depth is needed for group learning

Requiring specific personal disclosures exploits students and is not required to learn.

Rationale: When educational programs include self-growth or self-disclosure experiences, counselor educators inform students of the ethical ramifications of self-disclosure, and evaluation is not dependent on the student's level of self-disclosure. Many programs also use outside facilitators so the instructor does not evaluate what students reveal.

References: ACA Code of Ethics — Section F, self-growth experiences in counselor education · CACREP 2024 Standards — Experiential group learning in the group counseling core area

► **Check:** Confirm the exact ACA 2014 subsection number for self-growth experiences in Section F (believed to be F.8.c) before citing it.

☐ Approve ☐ Revise ☐ Retire Notes:

After a session in which a new client mentions a popular online video channel she runs, the counselor is tempted to look up the channel and the client's public social media posts. What does the ACA Code of Ethics advise?

A. Viewing is fine, since anything posted publicly by a client is open to the counselor

Public availability does not override the Code's respect for client privacy online.

B. Viewing is fine if the counselor does not comment, like, or follow the client's posts

Passive viewing without consent still intrudes on the client's virtual presence.

C. Viewing is required, since it gives collateral information useful for diagnosis

There is no duty to search clients online, and doing so needs consent.

D. Respect the client's online privacy and view it only if the client gives consent ✓ KEY

H.6.c directs counselors to respect clients' social media privacy unless given consent.

Rationale: H.6.c directs counselors to respect the privacy of clients' presence on social media unless given consent to view such information. Counselors also explain their own social media policies as part of informed consent (H.6.b).

References: ACA Code of Ethics — H.6.c Client Virtual Presence

☐ Approve ☐ Revise ☐ Retire Notes:

A counselor in solo practice is retiring and has paper files for clients seen over the past 15 years. How should the counselor handle retention and disposal of these records?

A. Discard all files in the office recycling bin once the practice has officially closed

Recycling bins expose confidential records and ignore retention requirements.

B. Keep files for the period state law requires, then destroy them in a secure manner ✓ KEY

B.6.h links retention to law and requires disposal that protects confidentiality.

C. Give each file to a family member to store at home until the clients ask for them

Untrained family members lack authority and safeguards for client records.

D. Shred all files at once, since a closed practice has no further duty for its records

Retention duties continue after closing; immediate destruction may violate law.

Rationale: B.6.h directs counselors to store records after termination to ensure reasonable future access, to maintain them in accordance with federal and state law and licensure rules, and to dispose of them in a way that protects confidentiality, such as shredding. Retention periods are set by state law and vary. Counselors also plan for records in case of retirement or incapacity.

References: ACA Code of Ethics — B.6.h Storage and Disposal After Termination · State law — Record retention periods for licensed counselors

☐ Approve ☐ Revise ☐ Retire Notes:

A counselor at a state agency is told that a new agency rule requires sharing full session notes with a client's employer whenever the employer pays for services. The counselor believes this conflicts with the ACA Code of Ethics. What should the counselor do FIRST?

A. Follow the rule without comment, since a legal requirement replaces any ethical code

I.1.c first calls for stating the ethical commitment and trying to resolve the conflict.

B. Make known the commitment to the Code and take steps to resolve the conflict ✓ KEY

I.1.c directs counselors to state their commitment and work to resolve the conflict.

C. Refuse to follow the rule, since ethical codes take legal priority over agency rules

Ethical codes do not override law; outright refusal skips the steps the Code sets out.

D. Stop taking notes so that there is no record that could be given to employers

Failing to document violates record-keeping duties and does not resolve the conflict.

Rationale: Under I.1.c, when ethical responsibilities conflict with laws, regulations, or other legal authority, counselors make known their commitment to the ACA Code and take steps to resolve the conflict. If it cannot be resolved that way, counselors, acting in the client's best interest, may adhere to the legal requirement.

References: ACA Code of Ethics — I.1.c Conflicts Between Ethics and Laws

☐ Approve ☐ Revise ☐ Retire Notes:

A counselor learns that a colleague has been billing insurers for family sessions that never took place. When she raised it privately, the colleague dismissed her concern and the billing continued. What should the counselor do NEXT?

A. Report it to the licensing board or another proper authority, since informal efforts failed ✓ **KEY**

I.2.b calls for formal reporting when informal resolution fails or harm is likely.

B. Raise it with the colleague again every month, since informal resolution must still be continued

The Code does not require repeating failed informal efforts while harm continues.

C. Warn the colleague's clients about the billing so that they can decide to change counselors

Contacting the colleague's clients is not an appropriate reporting channel.

D. Drop the matter, since billing disputes are between insurers and providers, not counselors

Fraud is an ethical violation counselors are expected to address.

Rationale: I.2.a supports informal resolution first when appropriate. When an apparent violation has substantially harmed or is likely to harm others and is not appropriate for, or not resolved by, informal resolution, I.2.b directs counselors to take further action such as referral to state licensure boards, ethics committees, certification bodies, or institutional authorities.

References: ACA Code of Ethics — I.2.b Reporting Ethical Violations · ACA Code of Ethics — I.2.a Informal Resolution

☐ Approve ☐ Revise ☐ Retire Notes:

A licensed professional counselor holds a master's in counseling and a doctorate in music history. On her practice website she calls herself "Dr." and lists her credentials. What does the ACA Code of Ethics say about this?

A. It is acceptable, since any earned doctorate allows the title "Dr." in every context

C.4.d restricts using "Dr." in a counseling context when the doctorate is unrelated.

B. It is acceptable if the website also lists her current state counseling license number too

A license number does not correct a misleading implication of doctoral-level competence.

C. It is acceptable as long as no client has ever complained that the title was misleading to them

Misrepresentation is unethical whether or not anyone complains.

D. It is misleading, because the doctorate is not in counseling or a closely related field ✓ **KEY**

C.4.d bars implying doctoral-level competence with an unrelated doctorate.

Rationale: C.4.a requires accurate representation of credentials, and C.4.d states that counselors do not imply doctoral-level competence by using "Dr." in a counseling context when their doctorate is not in counseling or a related field. C.3.a likewise requires advertising to be accurate and not misleading.

References: ACA Code of Ethics — C.4.d Implying Doctoral-Level Competence · ACA Code of Ethics — C.3.a Accurate Advertising

☐ Approve ☐ Revise ☐ Retire Notes:

A 78-year-old client with a terminal illness tells his counselor he is considering ending his life through his state's legal medical aid-in-dying process. He has no other risk factors. Under the ACA Code of Ethics, how should the counselor approach confidentiality?

A. Disclose the plan to his family at once, since any talk of dying requires a breach of trust

B.2.b gives counselors discretion; automatic disclosure is not required.

B. Arrange involuntary hospitalization, since his choice must be treated as suicidal

A lawful end-of-life choice is not automatically treated as a psychiatric emergency.

C. Use discretion on breaching, guided by consultation, law, and the client's situation ✓ **KEY**

B.2.b gives counselors discretion, informed by consultation and applicable law.

D. Refer him to another counselor, since end-of-life choices fall outside counseling

Supporting end-of-life decision making is within the counselor's role.

Rationale: B.2.b gives counselors who work with terminally ill clients considering hastening death the option to maintain confidentiality, depending on applicable laws and the specific circumstances, after seeking consultation or supervision from appropriate professional and legal parties. Counselors also help clients explore options, values, and supports. State law on medical aid in dying varies.

References: ACA Code of Ethics — B.2.b Confidentiality Regarding End-of-Life Decisions · State law — Medical aid-in-dying statutes (legal in some states only)

► **Check:** Confirm the 2014 code location B.2.b for end-of-life confidentiality (the 2005 code placed end-of-life care at A.9).

☐ Approve ☐ Revise ☐ Retire Notes:

A 24-year-old client from a close-knit Mexican American family lives at home and gives part of his pay to his parents. The counselor, who values independence, begins urging him to move out to "become his own person." The client has not named this as a goal. What is the ethical concern?

A. The counselor is right to push independence, since adulthood requires leaving home

This is a culturally bound assumption, not a universal developmental requirement.

B. The counselor is imposing personal values instead of exploring the client's own ✓ **KEY**

A.4.b bars imposing the counselor's values; goals should reflect the client's worldview.

C. The counselor should refer him, since cultural differences make counseling unworkable

Cultural difference is not grounds for referral; the counselor builds competence instead.

D. The concern is billing, since family issues are not covered as a reason for counseling

Billing is unrelated to the ethical issue of value imposition.

Rationale: A.4.b directs counselors to be aware of, and avoid imposing, their own values, attitudes, and beliefs. Many collectivist families see interdependence and financial support of parents as healthy. Goals should come from the client, explored within the client's cultural context (A.2.c, A.1.c).

References: ACA Code of Ethics — A.4.b Personal Values · Sue & Sue (Counseling the Culturally Diverse) — Individualism vs collectivism and culture-bound counseling values

☐ Approve ☐ Revise ☐ Retire Notes:

A counselor researcher is preparing a consent form for a study of coping in caregivers of people with dementia. Which statement is a required element of informed consent for research participants?

A. Participants may decline or withdraw at any time without penalty ✓ KEY

G.2.a lists the freedom to withdraw without penalty as a required element.

B. Participants must finish the study once they have signed the form

Participation is voluntary throughout; withdrawal must be allowed.

C. Participants give up the right to see results in exchange for pay

Payment does not remove participant rights, and results may be shared.

D. Participants are told the hypotheses only after data have been used

Purposes and procedures are explained up front, except in justified deception.

Rationale: G.2.a requires researchers to explain the purpose and procedures, risks, benefits, alternatives, limits of confidentiality, the format and audience for results, and that participants are free to withdraw at any time without penalty. Consent is an ongoing process, not a one-time signature.

References: ACA Code of Ethics — G.2.a Informed Consent in Research · Erford (Research and Evaluation in Counseling) — Informed consent and voluntary participation

☐ Approve ☐ Revise ☐ Retire Notes:

A counselor in a community agency plans to survey clients about service satisfaction and later publish the findings. The agency has access to a university institutional review board (IRB). Which step is MOST appropriate before collecting data?

A. Begin the survey now, since satisfaction surveys of clients carry no risk to them

Research with clients involves risks, including pressure to participate and privacy.

B. Ask clients to put their names on surveys so their answers can be checked later

Collecting names without need raises risk; data should be de-identified when possible.

C. Collect the data first and seek a review only if the results turn out to be publishable

Review occurs before data collection, not after results are known.

D. Seek IRB review and plan how consent is obtained and how data will be kept private ✓ KEY

Section G calls for review of research and for protecting participants' consent and privacy.

Rationale: Section G directs counselors to follow applicable law and institutional review requirements, to obtain informed consent, and to protect the confidentiality of participant information. Planned publication makes this research rather than routine program evaluation, so IRB review is sought before data collection, and data are de-identified and stored securely.

References: ACA Code of Ethics — Section G, Research and Publication (institutional review and confidentiality of information) · Erford (Research and Evaluation in Counseling) — Institutional review boards and protection of participants

► **Check:** Check whether a specific G subsection (e.g., G.1.b or G.2.d) should be cited for IRB review and data confidentiality.

☐ Approve ☐ Revise ☐ Retire Notes:

A counselor's outcome study finds that a group program her agency promotes did not reduce symptoms more than a waitlist. The agency director suggests leaving these results out of the annual report. What should the counselor do?

A. Omit the findings, since agency staff must support the programs their employer runs

Loyalty to an employer does not permit withholding results of professional value.

B. Adjust the analysis until the program shows a positive effect worth reporting on

Manipulating analyses to produce a result is a form of data distortion (G.4.a).

C. Report the results accurately, since unfavorable findings may not be withheld ✓ **KEY**

G.4.b states that results reflecting unfavorably on programs are not withheld.

D. Report only the subgroups in which scores improved and describe them as overall gains

Selective reporting misrepresents results and violates G.4.a.

Rationale: G.4.a requires counselors to plan, conduct, and report research accurately without distorting data or biasing results. G.4.b states that results of professional value are reported, and results that reflect unfavorably on institutions, programs, services, or vested interests are not withheld.

References: ACA Code of Ethics — G.4.b Obligation to Report Unfavorable Results · ACA Code of Ethics — G.4.a Accurate Results

☐ Approve ☐ Revise ☐ Retire Notes:

Q49 of 1000 · nce-s-eth-049 · Professional Counseling Orientation and Ethical Practice · mandated reporting of abuse and neglect · medium · In review

A 68-year-old client with an intellectual disability lives in a licensed group home. She tells her counselor that night staff often lock the pantry and send her to bed without dinner as punishment. She has lost weight since her last visit. State law names counselors as mandated reporters of abuse and neglect of vulnerable adults. What should the counselor do?

A. Tell the group home administrator, who is responsible for investigating staff

Notifying the facility does not satisfy a statutory duty to report to the designated agency, and the facility has a conflict of interest.

B. Wait until the client shows clear physical injury before making any report

Reporting laws require reasonable suspicion, not proof of injury; withholding food as punishment is itself neglect or abuse.

C. Report the suspected neglect to adult protective services or the named agency ✓ **KEY**

A mandated reporter with reasonable suspicion reports to the agency the statute designates, such as adult protective services.

D. Keep the disclosure confidential unless the client signs a release to share it

Confidentiality yields when law requires disclosure (ACA B.2.a); a release is not needed for a mandated report.

Rationale: Most states require counselors to report suspected abuse, neglect or exploitation of older or vulnerable adults to adult protective services or another designated agency, and the threshold is reasonable suspicion rather than proof. ACA B.2.a recognizes that confidentiality does not apply when legal requirements demand disclosure. Telling the facility alone does not meet the duty.

References: State law — Vulnerable adult / adult protective services mandatory reporting statutes (vary by state) · ACA Code of Ethics — B.2.a Serious and Foreseeable Harm and Legal Requirements

☐ Approve ☐ Revise ☐ Retire Notes:

A 36-year-old client discloses that an uncle sexually abused her from ages 8 to 11. She does not want to file a report. In passing, she mentions that the uncle now coaches a youth soccer team. How should the counselor approach the question of reporting?

- A. Make no report, since child abuse laws cover only victims who are minors now
Some states require or permit reports when the alleged abuser has current access to children, so this is too absolute.
- B. Report at once, since abuse of a child must be reported regardless of when it occurred
Many states do not require reports of past abuse disclosed by an adult; the counselor must check the statute.
- C. Ask the client to file her own police report before discussing the abuse further
Making the report a condition of counseling ignores client autonomy and does not resolve the counselor's own duty.
- D. Check the state statute and consult, weighing that children may be at present risk ✓ KEY
The duty depends on state law, and current access to children is the key fact; consultation and the client's involvement guide next steps.

Rationale: Mandated reporting laws are written mainly to protect children who are currently at risk, and states differ on whether a report is required when an adult discloses abuse from childhood. Some states require or allow a report when the alleged abuser still has access to children. The counselor reviews the specific statute, consults, documents the reasoning, and involves the client in the process where possible.

References: State law — Child abuse reporting statutes: adult disclosure of past abuse and current risk to minors · ACA Code of Ethics — B.2.a Serious and Foreseeable Harm and Legal Requirements

► **Check:** Reporting of past abuse disclosed by an adult varies widely by state; confirm the framing that the duty turns on state statute and current risk to children.

☐ Approve ☐ Revise ☐ Retire Notes:

A counselor has just phoned in a report of suspected child neglect to the state child protection hotline. Which documentation practice is MOST appropriate?

- A. Note the date, time, facts reported, agency worker's name and report number ✓ KEY
A factual record of what was reported, when and to whom shows that the legal duty was met.
- B. Keep no note of the call, so that the family cannot later learn who reported
Failing to document leaves no record that the duty was fulfilled; reporter identity is protected by law.
- C. Record the counselor's conclusion about which parent is guilty of the neglect
Records should contain observed facts and disclosures, not conclusions of guilt, which are for investigators.
- D. Store the report details only in a personal file kept at the counselor's home
Records belong in the secure clinical record, not in a personal file outside the agency or practice.

Rationale: After a mandated report, the counselor documents objective facts: the date and time of the report, the agency contacted, the name of the person who took it, any reference number, and the information conveyed, including the client's own words where relevant. This record shows the legal duty was met and supports continuity of care. Speculation about guilt does not belong in the note.

References: Wiger (Documentation) — Documenting mandated reports and critical incidents · State law — Child abuse and neglect reporting procedures

☐ Approve ☐ Revise ☐ Retire Notes:

Under the Family Educational Rights and Privacy Act (FERPA), when do the rights to inspect and control a student's education records transfer from the parents to the student?

A. When the student turns 16 and may leave school

FERPA does not tie the transfer of rights to compulsory-attendance age.

B. At age 18, or on entering college at any age ✓ KEY

The student becomes an "eligible student" at 18 or on attending a postsecondary institution.

C. On graduating from high school, whatever the age

Graduation alone does not transfer FERPA rights; age 18 or postsecondary attendance does.

D. At age 21, the age set for all federal privacy laws

FERPA sets 18, and there is no single age for all federal privacy laws.

Rationale: FERPA gives parents rights over their child's education records until the student turns 18 or attends a postsecondary institution, at which point the student becomes an "eligible student" and the rights transfer. Schools may still disclose to parents in limited situations, such as when the student is a tax dependent or in a health or safety emergency.

References: FERPA — 34 CFR 99.3 and 99.5: eligible student and transfer of parental rights

☐ Approve ☐ Revise ☐ Retire Notes:

A middle school counselor jots brief notes after student meetings to jog her memory. A parent asks to see "everything the school has" on his son. Under FERPA, which condition would keep the counselor's notes outside the definition of education records?

A. They are typed on a school computer and saved in the student's file

Placing notes in the student file makes them part of the education record.

B. They are shared with the student support team to plan interventions

Once notes are revealed to others, the sole-possession exclusion is lost.

C. They are labeled "confidential" and locked in the main office cabinet

A label and a lock do not change status if others in the office can access them.

D. They are used only as her memory aid and are not shown to anyone else ✓ KEY

Sole-possession records kept as a personal memory aid and not shared are excluded from education records.

Rationale: FERPA excludes from "education records" notes that are kept in the sole possession of the maker, used only as a personal memory aid, and not accessible or revealed to any other person except a temporary substitute. Once such notes are shared, placed in the student file, or used to make decisions, they become education records that parents may inspect.

References: FERPA — 34 CFR 99.3 definition of education records: sole-possession exclusion

☐ Approve ☐ Revise ☐ Retire Notes:

A client at a HIPAA-covered clinic reads her records and asks the counselor to change an intake note that lists past alcohol misuse, which she says is wrong. The counselor reviewed the intake carefully and believes the note is accurate and complete. What does the HIPAA Privacy Rule require?

A. Delete the note, because clients control the content of their own records

Clients may request amendment, but the provider is not required to accept a change it finds inaccurate.

B. Refuse to discuss it, because clients may view but not seek changes to records

HIPAA gives individuals an explicit right to request amendment of their records.

C. Answer in writing; if denied, explain why and let her file a disagreement ✓ KEY

A denial must be written, give the basis, and tell the client she may submit a statement of disagreement.

D. Amend it only if a supervisor agrees, then send the change to all insurers

Supervisor approval is not the HIPAA standard, and blanket disclosure to insurers violates minimum necessary.

Rationale: Under 45 CFR 164.526, individuals may ask a covered entity to amend their protected health information. The entity may deny the request if the information is accurate and complete, but it must respond in writing within set time limits, state the basis for denial, and explain the right to file a statement of disagreement that is then linked to the record. Disclosures of the amendment follow the minimum necessary standard.

References: HIPAA — 45 CFR 164.526 Amendment of protected health information

☐ Approve ☐ Revise ☐ Retire Notes:

A counselor is licensed in her state, holds the National Certified Counselor (NCC) credential, and is a member of the American Counseling Association. Which ethical standards apply to her professional conduct?

A. The ACA Code, the NBCC Code and her licensing board's rules all apply ✓ KEY

Each body binds her through a separate tie: membership, certification and licensure.

B. Only the state board rules, since licensure overrides voluntary codes

Licensure does not cancel obligations she accepted by joining ACA and holding the NCC.

C. Only the NBCC Code, since national certification outranks state law

National certification does not outrank state law; licensure regulates legal practice.

D. Only whichever code is strictest, as the others then fall away entirely

All applicable codes remain in force; she is accountable to each body separately.

Rationale: Counselors are often bound by several codes at once. ACA members agree to follow the ACA Code of Ethics, NCCs and applicants are bound by the NBCC Code of Ethics, and licensees must follow the rules and code adopted by their state board. Each body can act on a violation within its own authority, so the counselor attends to all of them.

References: NBCC Code of Ethics — Application to NCCs and applicants · ACA Code of Ethics — Preamble and Purpose: application to ACA members

☐ Approve ☐ Revise ☐ Retire Notes:

A National Certified Counselor receives a public reprimand from her state licensing board for a documentation lapse. She wonders whether she must tell NBCC, since the matter was handled by the state. What does the NBCC Code of Ethics expect?

A. No notice is needed, as NBCC reviews only complaints that clients file

NBCC can act on licensure discipline as well as on client complaints.

B. She reports the board action to NBCC within the time the Code sets ✓ **KEY**

The NBCC Code requires NCCs to report disciplinary actions by licensing boards to NBCC.

C. She tells NBCC only if the board fully revokes or suspends her license

The reporting duty is not limited to revocation or suspension.

D. She waits for the renewal form and mentions it at her next recertification

The Code sets a reporting window rather than deferring disclosure to renewal.

Rationale: The NBCC Code of Ethics requires NCCs to report to NBCC disciplinary actions taken by licensing or credentialing bodies, along with certain legal matters, within a set period (60 days in recent versions). This lets NBCC decide independently whether action on the certification is warranted.

References: NBCC Code of Ethics — NCC duty to report licensure discipline and legal actions to NBCC

► **Check:** Confirm the current NBCC Code wording on self-reporting duties and the 60-day window cited in the rationale.

☐ Approve ☐ Revise ☐ Retire Notes:

Q57 of 1000 · nce-s-eth-057 · Professional Counseling Orientation and Ethical Practice · counselor impairment and self-care · medium · In review

Two months after her father's death, a counselor finds herself tearful and distracted in sessions with several bereaved clients, and she lost track of one client's risk disclosure last week. What is her MOST appropriate course of action?

A. Keep her full caseload, since staying busy is the best way to work through grief

Continuing unchanged after a missed risk disclosure puts clients at risk of harm.

B. Quietly close her practice to new and current clients without notice to them

Abrupt closure without notice or transfer amounts to client abandonment.

C. Seek help and consultation, and limit or transfer cases until she can serve safely ✓ **KEY**

C.2.g directs impaired counselors to seek assistance and limit, suspend or end services as needed.

D. Disclose her loss to grieving clients so that they can support her in sessions

Using clients for support reverses roles and serves the counselor's needs, not the clients'.

Rationale: ACA C.2.g directs counselors to monitor themselves for signs of impairment, seek assistance for problems that reach the level of impairment, and limit, suspend or terminate professional responsibilities until it is safe to resume. Consultation, personal counseling and a temporary transfer of affected cases protect clients without abandoning them.

References: ACA Code of Ethics — C.2.g Impairment

☐ Approve ☐ Revise ☐ Retire Notes:

After three years of working mainly with survivors of violent crime, a counselor notices that she now sees the world as unsafe, distrusts strangers, and has intrusive images from clients' stories. Which term BEST describes this?

A. Vicarious trauma ✓ KEY

Vicarious trauma is a lasting change in the helper's beliefs about safety and trust from empathic exposure to trauma.

B. Countertransference

Countertransference is the counselor's reaction to a particular client, often tied to the counselor's own history.

C. Simple job burnout

Burnout is exhaustion and cynicism from workload and conditions, not a shift in trauma-related schemas.

D. Moral distress

Moral distress arises when one cannot act on what one believes is ethically right.

Rationale: Vicarious traumatization, described by McCann and Pearlman, is a cumulative change in the counselor's inner experience and core beliefs, such as safety, trust and control, that results from empathic engagement with clients' trauma. It differs from burnout, which stems from work conditions, and calls for supervision, caseload balance and self-care as part of ethical practice (ACA C.2.g).

References: Gladding (Counseling: A Comprehensive Profession) — Counselor burnout, vicarious trauma and self-care · ACA Code of Ethics — C.2.g Impairment

► **Check:** Confirm the Gladding chapter locator for vicarious trauma vs burnout; the McCann & Pearlman attribution is standard.

☐ Approve ☐ Revise ☐ Retire Notes:

A counselor wants to join the American Counseling Association division that focuses on multicultural counseling and that endorsed the Multicultural and Social Justice Counseling Competencies. Which division should she join?

A. ASGW

The Association for Specialists in Group Work focuses on group work and its best practices.

B. ACES

The Association for Counselor Education and Supervision focuses on training and supervision.

C. IAMFC

The International Association of Marriage and Family Counselors focuses on couple and family work.

D. AMCD ✓ KEY

The Association for Multicultural Counseling and Development focuses on multicultural counseling.

Rationale: The Association for Multicultural Counseling and Development (AMCD) is the ACA division devoted to multicultural and diversity issues, and it developed the Multicultural and Social Justice Counseling Competencies with ACA endorsement. ACA divisions each serve a specialty area, such as group work (ASGW), counselor education (ACES) and family counseling (IAMFC).

References: MSJCC (Ratts et al., 2016) — AMCD development and ACA endorsement of the competencies · Gladding (Counseling: A Comprehensive Profession) — Professional organizations and ACA divisions

☐ Approve ☐ Revise ☐ Retire Notes:

A counselor educator is preparing a slide that matches counseling bodies with their roles. Which pairing is accurate?

A. CACREP: issues licenses that let counselors practice in each state

CACREP accredits graduate programs; it does not license anyone.

B. Chi Sigma Iota: the international honor society of the counseling field ✓ **KEY**

Chi Sigma Iota is the counseling academic and professional honor society.

C. NBCC: accredits master's and doctoral counseling graduate programs

NBCC certifies individual counselors and administers the NCE; CACREP accredits programs.

D. State licensing board: awards the National Certified Counselor title

The NCC is awarded by NBCC; state boards grant licenses to practice.

Rationale: Chi Sigma Iota is the international honor society for counseling students, professional counselors and counselor educators, promoting scholarship, leadership and excellence. CACREP accredits programs, NBCC grants national certification such as the NCC, and state boards license counselors and enforce state practice law.

References: Gladding (Counseling: A Comprehensive Profession) — Professional organizations: Chi Sigma Iota, NBCC, CACREP and state boards

☐ Approve ☐ Revise ☐ Retire Notes:

The California Supreme Court decided Tarasoff v. Regents of the University of California twice, in 1974 and again on rehearing in 1976. How did the 1976 decision change the clinician's obligation?

A. It framed it as a duty to protect, met by warning or other reasonable steps ✓ **KEY**

Tarasoff II held a duty to use reasonable care to protect the victim, which warning may or may not satisfy.

B. It removed any duty, ruling that therapy privilege outweighs third parties

The court reaffirmed liability, stating that the protective privilege ends where public peril begins.

C. It limited the duty to cases in which the client has already hurt someone

The duty concerns foreseeable future violence against an identifiable victim, not past harm.

D. It required clinicians to call the police after any client voices anger

The duty arises from a serious threat of violence, not ordinary expressions of anger.

Rationale: Tarasoff I (1974) held that a therapist had a duty to warn a foreseeable victim. On rehearing, Tarasoff II (1976) restated this as a broader duty to protect: when a therapist determines, or should determine, that a client poses a serious danger of violence to another, the therapist must use reasonable care to protect the intended victim, which may include warning, notifying police or other steps.

References: State law — Tarasoff v. Regents of the University of California (1974, 1976) and state duty-to-protect law · ACA Code of Ethics — B.2.a Serious and Foreseeable Harm and Legal Requirements

☐ Approve ☐ Revise ☐ Retire Notes:

A counselor relocates her practice to a new state. A colleague tells her that "duty to warn works the same everywhere because of Tarasoff." What is the MOST accurate response?

A. Tarasoff is a federal ruling, so every state follows the same rule

Tarasoff was a California Supreme Court case, not a federal ruling binding all states.

B. Duty to warn applies only to psychiatrists, not licensed counselors

State duty-to-protect laws commonly cover licensed mental health professionals, including counselors.

C. States differ, so she must learn her new state's duty-to-protect law ✓ **KEY**

Some states mandate action, some permit disclosure, and some have no statute, so she checks local law.

D. The ACA Code sets one duty-to-warn rule that replaces state statutes

The ACA Code defers to legal requirements and does not replace state statutes.

Rationale: Tarasoff was a California decision. States have since adopted different approaches: some impose a mandatory duty to warn or protect, some permit but do not require disclosure, and some have no statute or case law. Counselors must know the law in each state where they practice, including what triggers the duty and what actions discharge it.

References: State law — Duty-to-warn/protect statutes: mandatory, permissive or none · ACA Code of Ethics — B.2.a Serious and Foreseeable Harm and Legal Requirements

☐ Approve ☐ Revise ☐ Retire Notes:

Q63 of 1000 · nce-s-eth-063 · Professional Counseling Orientation and Ethical Practice · ACA Code of Ethics structure · medium · In review

The Preamble of the 2014 ACA Code of Ethics lists core professional values of the counseling profession. Which of the following is one of those core values?

A. Protecting the field's public image

Reputation of the profession is not listed among the Preamble's core values.

B. Maximizing cost-effectiveness

Cost-effectiveness is not a core professional value in the Preamble.

C. Deferring to client families

The Code emphasizes client autonomy, not deference to family on treatment.

D. Promoting social justice ✓ **KEY**

Promoting social justice is one of the core professional values listed in the Preamble.

Rationale: The Preamble names the profession's core values: enhancing human development throughout the life span; honoring diversity and embracing a multicultural approach; promoting social justice; safeguarding the integrity of the counselor-client relationship; and practicing in a competent and ethical manner. It also lists the ethical principles of autonomy, nonmaleficence, beneficence, justice, fidelity and veracity.

References: ACA Code of Ethics — Preamble: core professional values and fundamental principles

☐ Approve ☐ Revise ☐ Retire Notes:

A client's partner has twice confronted the counselor in the parking lot after sessions and left threatening voicemails. The counselor no longer feels safe continuing the relationship. Under the ACA Code of Ethics, what may the counselor do?

A. End services by letter right away, giving no referrals, since the threat is not her fault

Even when termination is justified, the counselor offers pretermination counseling and referrals as appropriate.

B. Terminate, since threats from someone close to the client are an accepted reason ✓ **KEY**

A.11.c permits termination when the counselor is in jeopardy of harm by someone in a relationship with the client.

C. Continue indefinitely, because ending services would punish the client for the partner

The Code does not require counselors to keep working while in jeopardy of harm.

D. Invite the partner into joint sessions to resolve the conflict before any decision

Bringing a threatening person into sessions raises risk and is not required before termination.

Rationale: A.11.c states that counselors may terminate when they are in jeopardy of harm by the client or by another person with whom the client has a relationship. Counselors still provide pretermination counseling and recommend other service providers when necessary, and they document the reasons and safety steps taken.

References: ACA Code of Ethics — A.11.c Appropriate Termination

☐ Approve ☐ Revise ☐ Retire Notes:

A counselor realizes that a client's severe eating disorder requires specialized care beyond her competence. She offers three referrals to qualified providers, but the client declines all of them and asks to keep seeing her. What does the ACA Code of Ethics direct?

A. Continue, since the client's wish to stay outweighs the competence concern

Client preference does not permit practice outside competence when serious risk is present.

B. Keep working with her but drop eating-related goals from the treatment plan

Ignoring the main clinical problem does not address the risk and is not what the Code directs.

C. Discontinue the counseling relationship once the referrals have been offered ✓ **KEY**

A.11.a states that if clients decline the suggested referrals, counselors discontinue the relationship.

D. Ask a friend with eating-disorder training to coach her by phone each week

Informal coaching is not supervision or consultation and does not establish competence.

Rationale: A.11.a provides that when counselors lack the competence to help a client, they avoid entering or continuing the relationship, know culturally and clinically appropriate referral resources, and suggest them. If the client declines the suggested referrals, the counselor discontinues the relationship, handling the transition with care to avoid abandonment.

References: ACA Code of Ethics — A.11.a Competence Within Termination and Referral

☐ Approve ☐ Revise ☐ Retire Notes:

At the final session, a client who immigrated from the Philippines brings her counselor a small box of homemade pastries to thank her, explaining that in her family such gifts express respect. How should the counselor approach this gift?

A. Weigh its value, the client's motive, the relationship and cultural meaning ✓ KEY

A.10.f directs counselors to consider the relationship, monetary value, and the client's and counselor's motives.

B. Decline it politely, since the Code bars all gifts from current clients

The Code recognizes that small tokens of appreciation may be acceptable and does not bar all gifts.

C. Accept it, then offer a gift of equal value back so that the two are even

Reciprocal gift-giving is not required and can blur the professional relationship.

D. Accept it only if the client agrees to put the gift in writing beforehand

The Code sets factors to weigh, not a written agreement, for small gifts.

Rationale: A.10.f recognizes that in some cultures small gifts are a token of respect and gratitude. When deciding whether to accept, counselors consider the therapeutic relationship, the monetary value of the gift, the client's motivation for giving it, and the counselor's motivation for accepting or declining. Refusing a modest, culturally meaningful gift can itself harm the relationship.

References: ACA Code of Ethics — A.10.f Receiving Gifts

☐ Approve ☐ Revise ☐ Retire Notes:

A counselor has a guardian's consent to give a career interest inventory to a 17-year-old student. Under the ACA Code of Ethics, what must the counselor explain to the student before the assessment?

A. The raw scoring keys, so she can check the scoring on her own later on

Sharing scoring keys would compromise test security and is not part of informed consent.

B. Nothing until results are in, so that her answers stay honest and unbiased

E.3.a requires the explanation before the assessment, not after.

C. Only the time the inventory will take, since interest tests are low risk

Low risk does not remove the duty to explain the nature, purpose and use of results.

D. Its nature and purpose and who will use the results, in words she grasps ✓ KEY

E.3.a requires explaining nature, purposes and specific use of results in understandable language.

Rationale: E.3.a requires counselors, before an assessment, to explain its nature and purposes and the specific use of results by potential recipients, in terms and language the client can understand. This applies to low-stakes instruments too, and with minors the counselor also involves parents or guardians as appropriate.

References: ACA Code of Ethics — E.3.a Explanation to Clients

☐ Approve ☐ Revise ☐ Retire Notes:

A counselor running a workshop on test anxiety wants to photocopy several items from a published, copyrighted achievement test to use as practice handouts. What is the ethical issue?

A. None, as long as the counselor removes the publisher's name from each page

Removing attribution makes the problem worse; it does not grant permission.

B. Copying items without permission breaks test security and publisher rights ✓ **KEY**

E.10 requires maintaining test security and not reproducing published tests without permission.

C. It is acceptable only if the handouts are collected and shredded at the end

Collecting handouts does not cure unauthorized reproduction or exposure of items.

D. It is allowed for any educational purpose under the fair use of all tests

Exposing secure items can invalidate the test for future takers; fair use does not cover this.

Rationale: E.10 directs counselors to maintain the integrity and security of tests and assessment techniques consistent with legal and contractual obligations, and not to appropriate, reproduce or modify published assessments without acknowledgment and permission from the publisher. Exposing items also threatens the validity of the test for future examinees.

References: ACA Code of Ethics — E.10 Assessment Security

☐ Approve ☐ Revise ☐ Retire Notes:

Q69 of 1000 · nce-s-eth-069 · Professional Counseling Orientation and Ethical Practice · supervision and consultation responsibilities · medium · In review

A licensed counselor agrees to provide clinical supervision for a new graduate working toward licensure. Under the ACA Code of Ethics, what should the supervisor do at the start of the relationship?

A. Start case review at once and discuss evaluation only when problems arise

Delaying the discussion of evaluation denies the supervisee informed consent.

B. Ask the supervisee to waive any right to appeal a negative evaluation

Supervisees must be told of due process and appeal mechanisms, not asked to waive them.

C. Review policies, evaluation methods and due process for appeals with her ✓ **KEY**

F.4.a builds informed consent into supervision, including policies and due process appeal.

D. Rely on the graduate program's handbook, since it covers all expectations

Post-degree supervision needs its own informed consent; a school handbook does not cover it.

Rationale: F.4.a requires supervisors to incorporate the principles of informed consent and participation into supervision, informing supervisees of the policies and procedures to which supervisors are to adhere and the mechanisms for due process appeal of individual supervisor actions. Many supervisors put this in a written supervision agreement that also covers evaluation, emergencies and fees.

References: ACA Code of Ethics — F.4.a Informed Consent for Supervision

☐ Approve ☐ Revise ☐ Retire Notes:

A counselor's close friend asks her to serve as the licensure supervisor for the friend's adult son, who just finished a counseling degree. The counselor is confident she could be fair. What does the ACA Code of Ethics direct?

A. Accept, provided she documents the friendship in the supervision file

Documentation does not cure the loss of objectivity the Code warns about.

B. Accept, but have a second supervisor sign his final evaluation as well

A co-signer does not remove the conflict in the ongoing supervisory relationship.

C. Accept if the son agrees in writing that their personal ties will not matter

Supervisee consent does not resolve the supervisor's inability to stay objective.

D. Decline, since she may be unable to judge him with full objectivity ✓ **KEY**

F.3.d bars supervisory relationships with friends or family members where objectivity is at risk.

Rationale: F.3.d states that supervisors are prohibited from engaging in supervisory relationships with individuals with whom they have an inability to remain objective, such as friends or family members. The counselor's own confidence does not settle the matter; the safer course is to decline and help the son find another qualified supervisor.

References: ACA Code of Ethics — F.3.d Friends or Family Members

☐ Approve ☐ Revise ☐ Retire Notes:

A group practice is moving from paper charts to a cloud-based electronic health record. Under the ACA Code of Ethics, what should counselors tell clients about this change?

A. Nothing, because record storage is an internal business decision only

H.5.a requires informing clients how records are kept electronically.

B. How records are stored, including encryption and archive retention ✓ **KEY**

H.5.a lists the type of encryption and security and archival storage as things clients are told.

C. That electronic records are fully secure and cannot ever be breached

Counselors acknowledge the limits of technology security rather than promising it is perfect.

D. That clients must sign a waiver of all liability for any data breach

Liability waivers are not part of the Code and do not replace security duties.

Rationale: H.5.a directs counselors to maintain electronic records in accordance with relevant laws and to inform clients how records are maintained electronically, including the type of encryption and security assigned to the records and whether, and for how long, archival storage is kept. HIPAA's Security Rule adds administrative, physical and technical safeguards for electronic records.

References: ACA Code of Ethics — H.5.a Records · HIPAA — Security Rule safeguards for electronic PHI (45 CFR Part 164, Subpart C)

☐ Approve ☐ Revise ☐ Retire Notes:

The mother of a 14-year-old client calls after the third session and asks the counselor to tell her "everything he says," noting that she has legal custody and pays for services. The teen has asked that his talks stay private. What is the counselor's BEST approach?

A. Respect her rights, clarify what will be shared, and keep safety limits clear ✓ KEY

B.5.b asks counselors to respect parental rights while explaining the confidential nature of counseling.

B. Give her full session details, since a parent who pays controls the record

Payment does not entitle a parent to all session content, and full disclosure can damage trust.

C. Refuse to speak with her at all, since the teen asked for complete privacy

Ignoring a custodial parent disregards the parent's legal rights and responsibilities.

D. End counseling, since the parent and teen want different things from it

Differing wishes are common and are worked through, not a reason to terminate.

Rationale: B.5.b directs counselors to inform parents and guardians about the role of counselors and the confidential nature of the counseling relationship, consistent with legal and custodial arrangements, and to respect the inherent rights and responsibilities of parents. Ideally the counselor sets these terms with parent and minor at the outset: general progress may be shared, specific content stays private, and safety concerns are disclosed.

References: ACA Code of Ethics — B.5.b Responsibility to Parents and Legal Guardians

☐ Approve ☐ Revise ☐ Retire Notes:

A counselor at a community clinic is reviewing her progress notes before an internal audit. Which entry BEST reflects what belongs in a progress note?

A. A near-verbatim transcript of everything the client said during the hour

Transcripts are excessive and expose more private material than the record needs.

B. Client presentation, interventions used, response, risk status and plan ✓ KEY

A progress note documents observable presentation, the service provided, the client's response, risk and the next steps.

C. The counselor's guesses about the private lives of the client's coworkers

Speculation about third parties is not clinically necessary and should not be recorded.

D. A list of the counselor's own feelings and frustrations about the client

Personal reactions belong in supervision or consultation, not in the clinical record.

Rationale: Progress notes should be timely, accurate and objective, recording what services were provided and why, how the client responded, any risk findings, and the plan. ACA B.6.a asks counselors to create and maintain records needed to provide professional services. Excess detail, speculation about others and counselor feelings add risk without clinical value.

References: Wiger (Documentation) — Progress note content: presentation, interventions, response, risk and plan · ACA Code of Ethics — B.6.a Creating and Maintaining Records and Documentation

☐ Approve ☐ Revise ☐ Retire Notes:

A week after writing a progress note in the clinic's electronic record, a counselor realizes she entered the wrong medication dose that the client reported. How should she correct the error?

A. Delete the original note and write a new one dated to the session day

Deleting and back-dating destroys the original entry and can look like falsification.

B. Edit the dose in the original note quietly so the record reads correctly

An unmarked change hides that the record was altered and when.

C. Leave the note as written, since signed entries may not be changed later

Known errors should be corrected; leaving false information harms care.

D. Add a dated, signed addendum noting the error and the correct dose ✓ **KEY**

An addendum corrects the record while keeping the original visible and showing who changed it and when.

Rationale: Records are corrected in a way that keeps the original entry visible and marks the correction with the date and the counselor's signature: a single line through the error on paper, or a dated addendum or amendment in an electronic record. Erasing, overwriting or back-dating can be viewed as falsifying records. Accurate correction protects both care and the record's integrity.

References: Wiger (Documentation) — Correcting errors: late entries and addenda that preserve the original entry · ACA Code of Ethics — B.6.a Creating and Maintaining Records and Documentation

☐ Approve ☐ Revise ☐ Retire Notes:

Q75 of 1000 · nce-s-eth-075 · Professional Counseling Orientation and Ethical Practice · record keeping and documentation · hard · In review

A counselor saw a couple in joint sessions and also met once with each partner alone. After the couple separates, one partner, a 42-year-old man, asks for a copy of "the whole file." The other partner has not given consent. What does the ACA Code of Ethics direct?

A. Release only the parts of the record that relate directly to him ✓ **KEY**

B.6.e limits access in multiple-client records to the parts that relate to the requesting client, without other clients' confidential material.

B. Release the entire file, since he took part in most of the sessions

The file includes his partner's confidential material, including her individual session.

C. Refuse all access, since couple records belong to the counselor alone

Clients have a right to reasonable access to their records; a blanket refusal is not supported.

D. Release the file only after his former partner reviews and edits it

Letting one client edit the record is improper and does not protect either client.

Rationale: ACA B.6.e states that in situations involving multiple clients, counselors provide individual clients with only those parts of records that relate directly to them and do not include confidential information related to any other client. Setting rules for couple records at the start, including how individual sessions are documented, prevents disputes later.

References: ACA Code of Ethics — B.6.e Client Access

☐ Approve ☐ Revise ☐ Retire Notes:

A counselor promised a client she would call him back with a referral by Friday. On Friday, her schedule is overloaded, but she makes the call before leaving work. Which of Kitchener's moral principles does this action MOST directly reflect?

A. Justice

Justice concerns fairness and equal treatment, not keeping a specific promise.

B. Veracity

Veracity is truthfulness; the act here is honoring a commitment.

C. Fidelity ✓ KEY

Fidelity means being faithful to commitments and promises made to clients.

D. Autonomy

Autonomy is respect for the client's right to make his own choices.

Rationale: Kitchener's principles include autonomy, beneficence, nonmaleficence, justice and fidelity, with veracity often added in later work. Fidelity refers to honoring commitments and being trustworthy, which builds the trust the counseling relationship depends on.

References: Forester-Miller & Davis (Practitioner's Guide to Ethical Decision Making) — Moral principles: autonomy, justice, beneficence, nonmaleficence, fidelity (and veracity)

☐ Approve ☐ Revise ☐ Retire Notes:

A nonprofit agency fills open counseling slots by giving priority to clients with private insurance. A counselor argues that slots should go first to clients with the most severe needs, regardless of payer. Which moral principle is the counselor MOST directly invoking?

A. Justice, the fair allocation of services and resources ✓ KEY

Justice concerns fairness and equitable distribution of services, which is the issue raised.

B. Fidelity, keeping promises made to current clients

No promise to a particular client is at stake in the policy dispute.

C. Veracity, being truthful with clients about services

The concern is how services are allocated, not honesty in disclosure.

D. Autonomy, respecting each client's freedom of choice

Clients' self-determination is not the issue in how slots are assigned.

Rationale: Justice means treating people fairly and distributing services equitably; treating clients differently requires a rationale that is relevant to the difference. Prioritizing by ability to pay rather than clinical need raises a justice concern, and counselors may advocate within the agency for fairer policies.

References: Forester-Miller & Davis (Practitioner's Guide to Ethical Decision Making) — Moral principles: justice as fairness and equitable treatment

☐ Approve ☐ Revise ☐ Retire Notes:

A competent 55-year-old client decides to leave a stable job to open a risky small business. She is not in danger, but her counselor believes the move will cause her great stress and financial harm and is tempted to keep steering sessions toward talking her out of it. Which two principles are MOST in tension?

A. Justice and fidelity, since the client may lose her income

Neither fair distribution nor a broken promise is at issue.

B. Veracity and justice, since the plan affects her family too

Truthfulness and fairness are not the competing values in this choice.

C. Fidelity and veracity, since she has set her goals for therapy

No commitment or truthfulness conflict drives the counselor's dilemma.

D. Autonomy and beneficence, since her choice and welfare differ ✓ KEY

Her right to self-determination conflicts with the counselor's wish to do good for her.

Rationale: Autonomy is respect for a competent client's right to make her own decisions; beneficence is the duty to promote her welfare. When a client's free choice differs from what the counselor believes is good for her, and no serious harm is foreseeable, autonomy generally prevails. The counselor can explore consequences openly without imposing her own values (ACA A.4.b).

References: Forester-Miller & Davis (Practitioner's Guide to Ethical Decision Making) — Moral principles: autonomy versus beneficence · ACA Code of Ethics — A.4.b Personal Values

☐ Approve ☐ Revise ☐ Retire Notes:

A counselor holds a full, unencumbered license in her home state, which belongs to the Counseling Compact. She obtains a privilege to practice in another member state to see a client there by video. Whose practice laws govern her work with that client?

A. Only her home state's laws, since the privilege rests on that license

The privilege is based on the home license, but practice follows the remote state's laws.

B. The laws of the member state where the client is located during care ✓ KEY

Under the Compact, a privilege holder must follow the laws and rules of the remote state where the client is located.

C. Only federal rules, since telehealth across states is federally run

Counseling practice is regulated by states; no federal license governs it.

D. The Compact Commission's own national practice act for counselors

The Commission administers the Compact; it does not replace state practice acts.

Rationale: The Counseling Compact lets a counselor with an unencumbered license in a home member state practice in other member states through a privilege to practice. While using that privilege, the counselor is subject to the laws and regulations of the remote state where the client is located, and that state may act against the privilege. ACA H.1.b likewise notes that distance counselors may be subject to the laws of the client's location.

References: Counseling Compact — Privilege to practice: adherence to the laws and rules of the remote state · ACA Code of Ethics — H.1.b Laws and Statutes

► **Check:** Compact privileges were not yet being issued in every member state when written; confirm current operational status and the "law of the client's state" framing against the Compact text.

☐ Approve ☐ Revise ☐ Retire Notes:

A counselor holds an associate-level counseling license that, under her state's law, requires practice under an approved supervisor. She also holds the NCC credential. A friend urges her to open a solo private practice, saying "the NCC shows you're qualified." What determines whether she may practice independently?

A. Her NCC, since national certification overrides tiered state licenses

National certification is voluntary and does not authorize practice or override state law.

B. Her degree, since a CACREP master's permits independent counseling

A degree is a requirement for licensure, not a legal authorization to practice.

C. Her state's law for her license tier, which limits independent work ✓ KEY

The scope and conditions of each license tier are set by state law; an associate license requires supervision.

D. Her own judgment of her skills, since competence is self-assessed

Competence matters ethically, but self-assessment cannot expand a legal scope of practice.

Rationale: State licensure law defines who may practice counseling and under what conditions, including tiered licenses for counselors still completing supervised experience. National certification such as the NCC is a voluntary professional credential and does not grant the legal right to practice. ACA C.2.a also directs counselors to practice only within the boundaries of their credentials.

References: State law — Counselor licensure tiers: associate licenses and supervision requirements · ACA Code of Ethics — C.2.a Boundaries of Competence

► **Check:** Associate-level license titles and supervision rules vary by state; confirm the general framing.

☐ Approve ☐ Revise ☐ Retire Notes:

A state's counselor law is described as a "title act" rather than a "practice act." What does a title act do?

A. It limits who may use a protected title such as "licensed counselor" ✓ KEY

A title act restricts use of the protected professional title to people who hold the credential.

B. It limits who may perform counseling services for a fee in the state

Restricting the activity itself is what a practice act does.

C. It sets the standards that graduate programs must meet to accredit

Program standards are set by accreditors such as CACREP, not by state title laws.

D. It requires every counselor in the state to hold national certification

National certification is voluntary and is not created by a state title act.

Rationale: A title act protects the use of a professional title, so only credentialed people may call themselves, for example, a licensed professional counselor. A practice act goes further and defines the scope of practice, restricting who may perform the regulated activities. Most states now regulate both the title and the practice of counseling.

References: Gladding (Counseling: A Comprehensive Profession) — Credentialing: licensure, certification, title and practice acts

☐ Approve ☐ Revise ☐ Retire Notes:

A counselor who works with many clients facing eviction testifies before a state legislative committee in support of a bill to expand tenant protections. In the ACA Advocacy Competencies, which domain does this work fall under?

A. Client/student domain, through direct client empowerment

Client-level work helps an individual build self-advocacy skills or acts for one client.

B. School/community domain, through community collaboration

Community collaboration joins local groups; legislative testimony targets public policy.

C. Clinical domain, through individual treatment planning

There is no clinical domain in the competencies; treatment planning is not advocacy.

D. Public arena domain, through social/political advocacy ✓ **KEY**

Influencing legislation and public policy is social/political advocacy in the public arena.

Rationale: The ACA Advocacy Competencies describe three domains: client/student, school/community and public arena, each with an "acting with" and an "acting on behalf" level. Testifying to lawmakers to change policy that affects many clients is social/political advocacy in the public arena. ACA A.7.a supports advocacy at individual, group, institutional and societal levels.

References: ACA Advocacy Competencies — ACA Advocacy Competencies: client/student, school/community and public arena domains · ACA Code of Ethics — A.7.a Advocacy

☐ Approve ☐ Revise ☐ Retire Notes:

A counselor notices that several clients from the same apartment complex report asthma flare-ups from mold, and the landlord ignores repair requests. Which action BEST represents systemic rather than client-level advocacy?

A. Help one client draft a letter to the landlord about her own unit

Supporting one client's self-advocacy is valuable but stays at the client level.

B. Teach each affected client coping skills for the stress of the mold

Coping skills address individual distress, not the conditions causing it.

C. Work with a tenants' group and housing officials on building repairs ✓ **KEY**

Collaborating with community groups to change the conditions affecting many tenants is systems advocacy.

D. Refer each client separately to a legal aid office for their claims

Individual referrals address each case one at a time rather than the system.

Rationale: Client-level advocacy helps a particular client access resources or self-advocate. Systemic advocacy targets the policies, institutions or conditions that create barriers for many people, often by collaborating with community groups. When counselors see a pattern across clients, the MSJCC and ACA A.7.a support intervening at the institutional and community levels, with client consent for any case-specific action.

References: MSJCC (Ratts et al., 2016) — Counseling and advocacy interventions: institutional and community levels · ACA Code of Ethics — A.7.a Advocacy

☐ Approve ☐ Revise ☐ Retire Notes:

A 29-year-old client who is Deaf tells her counselor that her employer keeps refusing to provide an interpreter for staff meetings. She is articulate, knows her rights, and wants to handle it herself, but feels discouraged. Which advocacy approach BEST fits this situation?

A. Call the employer's human resources office on the client's behalf

Acting for a client who wants to self-advocate undercuts her autonomy and needs consent.

B. Help her identify strengths and plan her own self-advocacy steps ✓ **KEY**

Empowerment, acting with the client, fits a capable client who wants to advocate for herself.

C. Focus sessions on accepting the workplace as it currently operates

Encouraging adjustment to discrimination ignores the systemic barrier she faces.

D. File a formal discrimination complaint to a federal agency for her

Filing for her overrides her stated wish to act on her own behalf.

Rationale: At the client level, the ACA Advocacy Competencies distinguish empowerment (acting with clients to build self-advocacy) from advocacy on their behalf. When a client is able and wants to act for herself, the counselor helps her recognize strengths, identify resources and plan steps. ACA A.7.b requires client consent before counselors advocate on a client's behalf.

References: ACA Advocacy Competencies — Client/student empowerment: acting with clients before acting on their behalf · ACA Code of Ethics — A.7.b Confidentiality and Advocacy · Gladding (Counseling: A Comprehensive Profession) — ACA Advocacy Competencies: client/student empowerment

☐ Approve ☐ Revise ☐ Retire Notes:

Q85 of 1000 · nce-s-eth-085 · Professional Counseling Orientation and Ethical Practice · counselor impairment and self-care · easy · In review

A counseling intern says self-care is "a personal lifestyle choice that has nothing to do with ethics." How does the 2014 ACA Code of Ethics treat counselor self-care?

A. As a private matter outside the scope of professional standards

The Code directly links self-care to meeting professional responsibilities.

B. As required only for counselors who are already showing impairment

Self-care is framed as ongoing and preventive, not only a response to impairment.

C. As a duty that applies only to supervisors overseeing trainees

The expectation applies to all counselors, not only supervisors.

D. As part of the professional responsibility to serve clients well ✓ **KEY**

The Code expects counselors to engage in self-care to best meet their professional responsibilities.

Rationale: The introduction to Section C of the ACA Code states that counselors engage in self-care activities to maintain and promote their emotional, physical, mental and spiritual well-being to best meet their professional responsibilities. Standard C.2.g adds that counselors monitor themselves for impairment and refrain from offering services when impaired.

References: ACA Code of Ethics — Section C Introduction (self-care); C.2.g Impairment

☐ Approve ☐ Revise ☐ Retire Notes:

A counselor in solo practice is diagnosed with a progressive neurological illness. She is fully able to work now but knows sudden decline is possible. Under the ACA Code of Ethics, what should she do to protect her clients?

A. Prepare a plan naming a colleague to handle clients and records ✓ KEY

C.2.h calls for a plan for transfer of clients and files if the counselor becomes incapacitated.

B. Close her practice right away, since any illness is an impairment

A diagnosis alone is not impairment; she may practice while able, with planning and monitoring.

C. Tell each client her full diagnosis so they can decide whether to stay

Full medical disclosure is not required and may shift focus from clients' needs.

D. Rely on her family to contact clients if she becomes unable to work

Family members are not qualified to manage confidential records or clinical transitions.

Rationale: ACA C.2.h directs counselors to prepare a plan for the transfer of clients and the distribution of records to an identified colleague or records custodian in case of incapacitation, death or termination of practice, often called a professional will. Under C.2.g, she continues to monitor for signs of impairment and limits practice if it appears.

References: ACA Code of Ethics — C.2.h Counselor Incapacitation, Death, Retirement, or Termination of Practice · ACA Code of Ethics — C.2.g Impairment

☐ Approve ☐ Revise ☐ Retire Notes:

Q87 of 1000 · nce-s-eth-087 · Professional Counseling Orientation and Ethical Practice · ACA Code of Ethics structure · easy · In review

A new clinical supervisor wants the section of the 2014 ACA Code of Ethics that covers supervisor gatekeeping, evaluation of supervisees and counselor educator duties. Which section should she read?

A. Section D

Section D, Relationships With Other Professionals, covers colleagues, employers and consultation, not supervision duties.

B. Section E

Section E, Evaluation, Assessment, and Interpretation, covers testing and assessment of clients.

C. Section F ✓ KEY

Section F, Supervision, Training, and Teaching, addresses supervision, evaluation and gatekeeping, and counselor educator duties.

D. Section I

Section I, Resolving Ethical Issues, covers ethics complaints, conflicts with law and cooperation with committees.

Rationale: The 2014 ACA Code has nine sections: A The Counseling Relationship; B Confidentiality and Privacy; C Professional Responsibility; D Relationships With Other Professionals; E Evaluation, Assessment, and Interpretation; F Supervision, Training, and Teaching; G Research and Publication; H Distance Counseling, Technology, and Social Media; and I Resolving Ethical Issues. Gatekeeping and supervisee evaluation appear in Section F.

References: ACA Code of Ethics — Section F Supervision, Training, and Teaching (F.6.b Gatekeeping and Remediation)

☐ Approve ☐ Revise ☐ Retire Notes:

A counselor asks whether the 2014 ACA Code of Ethics tells counselors exactly which decision-making model to use when facing a dilemma. Which statement is accurate?

A. It requires the Forester-Miller and Davis model for all ethics cases

The Code names no single required model.

B. It expects a credible model able to bear public scrutiny, not a set one ✓ KEY

The Code asks counselors to use a credible model whose application can withstand scrutiny.

C. It ranks its standards so higher sections settle conflicts automatically

The Code does not rank its standards to resolve dilemmas mechanically.

D. It leaves decision making entirely to each employer's written policy

Employer policy does not replace the counselor's ethical decision-making duty.

Rationale: The Purpose section and standard I.1.b note that no single decision-making model is best for every situation, so counselors are expected to use a credible model that can bear public scrutiny of its application. Counselors also consult, document their reasoning, and draw on ethical principles and the Code's standards.

References: ACA Code of Ethics — Purpose; I.1.b Ethical Decision Making

☐ Approve ☐ Revise ☐ Retire Notes:

Q89 of 1000 · nce-s-eth-089 · Professional Counseling Orientation and Ethical Practice · NBCC Code of Ethics · medium · In review

A counselor's National Certified Counselor (NCC) credential lapsed eight months ago after she did not recertify. Her website, business cards and email signature still list "NCC." What is the ethical concern?

A. She is misrepresenting a credential she no longer holds ✓ KEY

Listing a lapsed credential misleads the public about her qualifications.

B. There is none, since she met the standard when she earned it

A credential may be claimed only while it is current and in good standing.

C. It is a concern only if a client is harmed by her services

Misrepresentation is a violation whether or not harm results.

D. It is a concern only for her state license, not certification

The issue is her claim of national certification, which NBCC governs.

Rationale: Both the NBCC Code of Ethics and ACA C.4.a require counselors to represent their credentials accurately. Continuing to list a lapsed certification misleads clients and the public. She should remove the designation at once or complete the steps to reinstate it.

References: NBCC Code of Ethics — Accurate representation of credentials and qualifications · ACA Code of Ethics — C.4.a Accurate Representation

► **Check:** Confirm that the current NBCC Code of Ethics addresses misrepresenting a lapsed credential; the exact directive number is not cited.

☐ Approve ☐ Revise ☐ Retire Notes:

A counselor who just passed the NCE posts a list of questions she remembers from the exam in an online study group to help friends who test next month. How is this viewed under NBCC standards?

A. As acceptable, since the items came from her own memory
Recalled items are still secure exam content; the source does not change that.

B. As acceptable, as long as she posts the items anonymously
Anonymity does not make sharing secure content ethical.

C. As a concern only if her friends end up passing the exam
The breach occurs when content is shared, whatever the result.

D. As a breach of exam security that can put her NCC at risk ✓ **KEY**
Sharing secure exam content violates NBCC standards and may lead to sanctions.

Rationale: NBCC requires applicants and certificants to protect the security of its examinations, and disclosing recalled exam content is a violation that can lead to sanctions such as denial or revocation of certification. Exam security protects the validity of the credential for every counselor who holds it.

References: NBCC Code of Ethics — Examination security and integrity of the certification process

► **Check:** Confirm that the NBCC Code of Ethics (rather than only the candidate agreement) addresses examination security.

☐ Approve ☐ Revise ☐ Retire Notes:

Q91 of 1000 · nce-s-eth-091 · Professional Counseling Orientation and Ethical Practice · professional organizations and their roles · easy · In review

A newly hired high school counselor wants to join the national professional association for school counselors that publishes a national model for school counseling programs. Which organization is it?

A. AMHCA
The American Mental Health Counselors Association serves clinical mental health counselors.

B. ASCA ✓ **KEY**
The American School Counselor Association publishes the ASCA National Model.

C. NBCC
NBCC is a certification board, not a membership association for school counselors.

D. CACREP
CACREP accredits graduate programs; it is not a membership association.

Rationale: The American School Counselor Association (ASCA) is the national association for school counselors and publishes the ASCA National Model, a framework for comprehensive school counseling programs, along with its own ethical standards for school counselors.

References: Gladding (Counseling: A Comprehensive Profession) — Professional organizations: ASCA and the ASCA National Model

☐ Approve ☐ Revise ☐ Retire Notes:

A state counseling board wants to join a national organization made up of state counselor licensing boards that works on issues such as license portability and consistent regulatory standards. Which organization fits this description?

A. CSI

Chi Sigma Iota (CSI) is the counseling honor society for students and professionals.

B. ACA

The American Counseling Association (ACA) is a membership association of individual counselors, not of boards.

C. AASCB ✓ KEY

The American Association of State Counseling Boards (AASCB) brings together state counselor licensing boards on regulatory issues such as portability.

D. NBCC

The National Board for Certified Counselors (NBCC) certifies individuals and develops exams; its members are not state boards.

Rationale: The American Association of State Counseling Boards (AASCB) is the organization of state counselor licensing boards. It works on regulatory consistency, licensure portability and public protection. ACA serves individual members, NBCC grants national certification and develops exams, and CSI is the profession's honor society.

References: Gladding (Counseling: A Comprehensive Profession) — Credentialing bodies: AASCB, NBCC and state licensure boards

☐ Approve ☐ Revise ☐ Retire Notes:

A 15-year-old client reveals that she has been cutting her thighs and has had thoughts of "not waking up." Her father emailed the counselor that morning demanding to know what his daughter talks about. She begs the counselor not to tell her parents anything. What should the counselor do FIRST?

A. Complete a risk assessment, then plan with her what parents must learn ✓ KEY

Risk must be assessed first; safety information is then shared with parents with the teen involved.

B. Reply to the father's email with a full summary of what she disclosed

Disclosure before assessing risk and without the teen's involvement shares more than needed.

C. Keep the disclosure private, since she asked that nothing be shared

A minor's request cannot override the duty to protect when safety is at stake.

D. Tell her sessions will stop unless she tells her parents herself today

Threatening to end services is coercive and risks abandonment of an at-risk teen.

Rationale: Self-injury and passive suicidal ideation call first for a structured risk assessment. If the assessment shows risk, ACA B.2.a permits disclosure to protect the client, and with a minor the counselor involves the teen in deciding how parents are told, sharing only what is needed for safety (B.2.e). The father's general demand for content is handled separately, balancing parental rights with the teen's privacy (B.5.b).

References: ACA Code of Ethics — B.2.a Serious and Foreseeable Harm; B.2.e Minimal Disclosure · ACA Code of Ethics — B.5.b Responsibility to Parents and Legal Guardians

☐ Approve ☐ Revise ☐ Retire Notes:

A father in individual counseling for depression asks his counselor to testify as an expert in his upcoming custody trial and to recommend that he receive primary custody. He says, "You know me better than anyone." What should the counselor do?

A. Agree, since a long alliance gives her the best basis for an opinion

Familiarity from treatment is not a basis for a forensic opinion and creates a dual role.

B. Agree if the other parent and both attorneys consent to her testimony

Consent of others does not cure the conflict between therapeutic and forensic roles.

C. Write a letter supporting custody instead of appearing at the hearing

A custody recommendation in writing is the same forensic opinion in a different form.

D. Explain she cannot give a custody opinion and refer him to an evaluator ✓ **KEY**

E.13.c bars evaluating current clients for forensic purposes; a neutral evaluator is the proper source.

Rationale: ACA E.13.c states that counselors do not evaluate current or former clients, clients' romantic partners or clients' family members for forensic purposes. A treating counselor lacks the neutrality and full information a custody evaluation requires. If legally compelled, she may testify as a fact witness with appropriate consent, describing treatment rather than recommending custody.

References: ACA Code of Ethics — E.13.c Client Evaluation Prohibited · ACA Code of Ethics — A.6.e Nonprofessional Interactions or Relationships (Other Than Sexual or Romantic)

☐ Approve ☐ Revise ☐ Retire Notes:

Q95 of 1000 · nce-s-eth-095 · Professional Counseling Orientation and Ethical Practice · record keeping and documentation · hard · In review

A client with adjustment disorder learns his insurer will pay for more sessions only for "a serious diagnosis." He asks his counselor to record major depressive disorder, and the practice manager says, "Everyone does it." What should the counselor do?

A. Record major depression, since the client has consented to the code

Client consent does not make a false diagnosis to an insurer honest or legal.

B. Keep the accurate diagnosis and discuss other payment or care options ✓ **KEY**

Diagnoses must be accurate; the counselor can explore sliding fees, self-pay or referral.

C. Record major depression only until the client's finances stabilize

A time-limited false diagnosis is still fraudulent billing.

D. Follow the manager's direction, since the practice bears the billing risk

Employer pressure does not excuse misrepresentation; the counselor is personally accountable.

Rationale: Counselors provide proper diagnoses (ACA E.5.a) and are accurate, honest and objective in reports to third parties, including insurers (C.6.b). Upcoding a diagnosis to obtain payment is insurance fraud and can also harm the client through an inaccurate permanent record. The counselor addresses the financial barrier through legitimate options instead.

References: ACA Code of Ethics — E.5.a Proper Diagnosis; C.6.b Reports to Third Parties

☐ Approve ☐ Revise ☐ Retire Notes:

A client signed a release last month letting her counselor share records in her divorce case. On the same day, the counselor receives a subpoena for the records from the spouse's attorney and a signed letter from the client revoking the release. What should the counselor do?

- A.** Send the records, since the release was valid when the subpoena was issued
Revocation applies to future disclosures; nothing has yet been released in reliance on it.
- B.** Ignore the subpoena, since the client has now withdrawn her permission
A subpoena is a legal document that must receive a timely response.
- C.** Honor the revocation, respond to the subpoena, and seek a court order ✓ **KEY**
The revocation stops voluntary release; the counselor responds without disclosing and asserts privilege.
- D.** Send a summary only, since that limits what the spouse's side will see
Any disclosure now lacks valid consent or a court order.

Rationale: A client may revoke an authorization in writing, and the revocation applies to any disclosure not yet made. A subpoena alone is not a court order, so the counselor responds in a timely way, notifies the client, asserts privilege on her behalf, consults an attorney, and discloses only what a court orders, limiting it to what is necessary (ACA B.2.c and B.2.e).

References: ACA Code of Ethics — B.2.c Court-Ordered Disclosure · HIPAA — 45 CFR 164.508(b)(5) Revocation of authorizations

► **Check:** Subpoena response and revocation handling vary by state and court rules; confirm the general guidance.

☐ Approve ☐ Revise ☐ Retire Notes:

After a careful risk assessment, a counselor concludes that she must notify a man whom her client, a 31-year-old warehouse worker, has credibly threatened to stab. What information should she share when she makes the call?

- A.** The client's full diagnosis, treatment history and recent session notes for context
Diagnosis, history and notes go well beyond what is needed to protect the man.
- B.** Nothing that names the client, so the client's privacy is kept fully intact
A warning that does not identify the source of danger may not let the man protect himself.
- C.** Only the essential facts: who poses the threat and the nature of the danger ✓ **KEY**
B.2.e directs that only essential information is revealed when disclosure is required.
- D.** Whatever the man asks about the client, since the duty overrides all privacy
The duty to protect permits a limited disclosure, not open access to the client's information.

Rationale: When serious and foreseeable harm requires a disclosure, ACA B.2.e limits it to essential information. In a warning, that means enough for the person to protect themselves, such as the source and nature of the threat, without diagnosis or treatment details. Where possible, the client is told about the disclosure beforehand and involved in the decision.

References: ACA Code of Ethics — B.2.e Minimal Disclosure · ACA Code of Ethics — B.2.a Serious and Foreseeable Harm and Legal Requirements

☐ Approve ☐ Revise ☐ Retire Notes:

The father of an adult client calls a counselor and says his son has been talking about "getting even" with his ex-partner's new boyfriend and has been driving past the boyfriend's home. The client has said nothing like this in session. How should the counselor BEST view the father's report?

A. As risk information to assess promptly, since some state laws treat it as able to trigger a duty ✓ KEY

Courts in some states (e.g., Ewing v. Goldstein) have held that a family member's report to the therapist can trigger the duty to protect.

B. As hearsay that cannot create any duty, because only the client's own words count in law

This is not true in every jurisdiction; some courts have held otherwise, and clinically the report must be assessed.

C. As grounds to warn the boyfriend at once, before speaking with the client about any of it

A warning without assessment skips the step that tells the counselor whether the risk is serious and foreseeable.

D. As a confidentiality breach by the father, which the counselor should report to the client

The father did not breach any duty, and treating the call this way ignores a possible risk to an identifiable person.

Rationale: Information about a client's dangerousness does not have to come from the client to be clinically and legally significant. In Ewing v. Goldstein (2004), a California appellate court held that a family member's report to the therapist could trigger the statutory duty to protect. Counselors assess such reports promptly, consult, and follow the law of their own state.

References: State law — Ewing v. Goldstein (Cal. Ct. App. 2004): family member communications and the duty to protect · ACA Code of Ethics — B.2.a Serious and Foreseeable Harm and Legal Requirements

☐ Approve ☐ Revise ☐ Retire Notes:

A counseling student asks why many textbooks now use the phrase "duty to protect" rather than "duty to warn." Which answer is MOST accurate?

A. Duty to protect applies only to suicide risk, while duty to warn covers harm to others

Both terms concern danger to others; suicide risk is handled under separate standards.

B. Duty to protect requires that police be called first, before any other step is taken

No single required first step applies; options depend on the case and on state law.

C. Duty to protect means the counselor shares liability for any harm the client commits

The term describes the clinician's obligation to act reasonably, not automatic liability.

D. Protecting may involve steps besides a warning, such as hospitalization or police contact ✓ KEY

The 1976 Tarasoff ruling framed the duty as reasonable care to protect, which a warning alone may or may not meet.

Rationale: The 1976 Tarasoff decision held that when a therapist determines a patient presents a serious danger of violence to another, the therapist must use reasonable care to protect the intended victim. Warning the victim is one option; others include notifying police, arranging hospitalization, or intensifying treatment. State laws define which actions discharge the duty.

References: State law — Tarasoff v. Regents of the University of California (1976): duty to use reasonable care to protect

☐ Approve ☐ Revise ☐ Retire Notes:

An agency counselor tells her supervisor that a 9-year-old client described being burned with a cigarette by a parent's partner. The supervisor says, "I'll take care of the report." Two days later, the counselor cannot confirm that any report was made. What should she do?

A. Wait for the supervisor to follow up, since the agency chain of command now owns it

In most states the mandated reporter's duty is personal and is not met by telling a supervisor.

B. Make the report herself now, since telling a supervisor does not discharge her duty ✓ **KEY**

Mandated reporting duties generally rest on the individual; she must make sure a report is made promptly.

C. Ask the child about the burn in more detail first, so the report will be complete

Investigating is the role of child protective services; more questioning can harm the child and the case.

D. Call the parent's partner to hear his account before deciding whether to report

Contacting the alleged abuser can put the child at further risk and is not the counselor's role.

Rationale: In most jurisdictions a mandated reporter's obligation is individual: informing a supervisor or administrator does not relieve the counselor of the duty to make sure a report reaches child protective services or law enforcement within the required time. Agency procedures can support reporting but do not replace it. The counselor should report and document the date, time and agency contacted.

References: State law — Child abuse reporting statutes: individual duty of mandated reporters and reporting timelines

☐ Approve ☐ Revise ☐ Retire Notes:

A 40-year-old father in counseling for anger tells his counselor that last week he struck his 6-year-old with a belt, leaving welts. He says it will not happen again and that counseling is how he is "fixing it." He asks the counselor to keep it private. What should the counselor do?

A. Keep it confidential, since the client is already in treatment for the very problem

Being in treatment does not remove the legal duty to report suspected child abuse.

B. File the required report and discuss it with the client, as reviewed in informed consent ✓ **KEY**

Mandated reporting applies when the client is the alleged abuser; the counselor can stay engaged and transparent.

C. Report only if the client strikes the child again, since this was a single incident

Reporting laws do not wait for repeated abuse; one incident with injury can trigger the duty.

D. End counseling and refer him elsewhere, so a new provider can decide about reporting

Referring out evades the duty and abandons a client who is seeking help.

Rationale: Child abuse reporting laws apply regardless of whether the person disclosing is the victim, a witness, or the alleged perpetrator, and they do not exempt clients who are already in treatment. Because reporting is a limit of confidentiality, it should be covered in informed consent at the start. Counselors can report while staying engaged and supporting the client's change efforts.

References: State law — Child abuse reporting statutes: duty applies to disclosures by alleged perpetrators · ACA Code of Ethics — B.2.a Serious and Foreseeable Harm and Legal Requirements

☐ Approve ☐ Revise ☐ Retire Notes:

A new school counselor hesitates to report a suspicion of neglect because she fears the family will sue her if child protective services finds nothing. What should her supervisor tell her?

A. She should wait for proof, since a report that turns out unfounded can lead to liability

The standard is reasonable suspicion, not proof, and good-faith reports are protected.

B. She may report anonymously, which is the only way to avoid being named in a lawsuit

Mandated reporters usually must identify themselves; protection comes from good-faith immunity.

C. State laws give immunity for reports made in good faith, even if abuse is not found ✓ KEY

All states grant immunity from liability for good-faith reports, and failing to report can carry penalties.

D. She should get the parents' consent before reporting, which protects her from a suit

Parental consent is not required to report and seeking it could put the child at risk.

Rationale: Mandated reporters must report when they have reasonable cause to suspect abuse or neglect. State laws, consistent with federal CAPTA requirements, protect people who report in good faith from civil and criminal liability even when the report is not substantiated. By contrast, knowingly failing to report can bring fines or other penalties.

References: State law — Good-faith immunity for child abuse and neglect reporters; penalties for failure to report

☐ Approve ☐ Revise ☐ Retire Notes:

A billing specialist at a HIPAA-covered group practice asks a counselor to forward complete session notes for every client so claims can be submitted faster. Which HIPAA principle MOST directly applies?

A. Minimum necessary: share only the information that is needed to process the claims ✓ KEY

For payment and operations uses, the minimum necessary standard limits what staff may access.

B. Right of access: staff have the same rights to full records that clients themselves do

The right of access belongs to the individual client, not to workforce members.

C. Treatment exception: anyone in the practice may see full notes without any limits

Billing is a payment function, not treatment, and is subject to minimum necessary limits.

D. Breach notification: clients must be told each time a billing staff member views notes

Breach notification applies to impermissible disclosures, not routine permitted access.

Rationale: The HIPAA Privacy Rule requires covered entities to limit uses and disclosures of protected health information to the minimum necessary to accomplish the purpose. Claims usually need dates, codes and diagnoses, not full session content. The minimum necessary standard does not apply to disclosures to other providers for treatment, but billing is a payment function.

References: HIPAA — Privacy Rule minimum necessary standard (45 CFR 164.502(b), 164.514(d))

☐ Approve ☐ Revise ☐ Retire Notes:

A counselor opening a private practice that bills health insurance electronically is setting up her intake paperwork. Under the HIPAA Privacy Rule, what must she give new clients?

A. A full copy of her case notes from every past client so they can judge her methods

Sharing other clients' records would itself be a serious privacy violation.

B. A signed waiver giving up their right to see their own records for the treatment term

HIPAA gives clients a right of access; providers may not require waiving it.

C. A written guarantee that no information will be shared without their permission ever

HIPAA and law permit some disclosures without authorization, so this promise is inaccurate.

D. A Notice of Privacy Practices, with a good-faith effort to get written acknowledgment ✓ **KEY**

Direct treatment providers give the notice at first service and try to obtain acknowledgment of receipt.

Rationale: A covered health care provider with a direct treatment relationship must give clients a Notice of Privacy Practices no later than the first service and make a good-faith effort to obtain written acknowledgment of receipt. The notice explains how information may be used and disclosed and describes clients' rights, such as access and amendment.

References: HIPAA — Notice of Privacy Practices and acknowledgment of receipt (45 CFR 164.520)

☐ Approve ☐ Revise ☐ Retire Notes:

The mother of a 20-year-old university student calls the campus counseling center and demands her daughter's counseling records, saying FERPA gives her a right to them because she pays tuition. The records are used only for the student's treatment. What is the BEST response?

A. Release the records, since FERPA gives parents of college students full access

At the postsecondary level FERPA rights belong to the student, not the parent.

B. Release a summary only, since paying tuition gives the parent a partial FERPA right

Paying tuition does not create a FERPA access right to counseling records.

C. Decline; these are treatment records, and the student's consent would be needed ✓ **KEY**

Records made and used only for treatment are excluded from FERPA education records.

D. Release them after a 45-day wait, the timeline FERPA sets for record requests

The 45-day timeline applies to eligible students' own access requests, not to this parent.

Rationale: Under FERPA, records on a student who is 18 or older or in postsecondary education that are made by a mental health professional, used only for treatment, and shared only with treating providers are treatment records, excluded from education records. FERPA rights at a university belong to the student. Counselors protect these records and release them with the student's consent or as law allows.

References: FERPA — Definition of education records; exclusion of treatment records (34 CFR 99.3) · ACA Code of Ethics — B.6.g Disclosure or Transfer

☐ Approve ☐ Revise ☐ Retire Notes:

A counselor provides text-based counseling through a secure platform. A new client prefers to use only a screen name and has not turned on video. Under the ACA Code of Ethics, what should the counselor do?

A. Take steps to verify the client's identity at the outset and during the counseling ✓ KEY

H.3 calls for verifying client identity at the beginning and throughout distance counseling.

B. Accept the screen name alone, since anonymity is a core benefit of online counseling

Anonymity does not remove the need to know who the client is, for example in emergencies.

C. Require the client to mail in a notarized identity form before any text exchange

This is an unnecessary barrier; the Code suggests simpler methods such as code words.

D. Decline to work by text, since the Code permits distance counseling by video only

The Code addresses many technologies, including text, and does not limit services to video.

Rationale: ACA H.3 directs counselors who use distance counseling to verify a client's identity at the beginning and throughout the process, for example with code words, numbers or other identifiers. Verification matters for consent, for protecting confidentiality, and for responding if the client is in crisis.

References: ACA Code of Ethics — H.3 Client Verification

☐ Approve ☐ Revise ☐ Retire Notes:

Q107 of 1000 · nce-s-eth-107 · Professional Counseling Orientation and Ethical Practice · telehealth and technology ethics · hard · In review

A counselor has seen a 66-year-old client by video for two months. The client's hearing loss and unreliable internet now make sessions hard to follow, and both agree little progress is being made. The counselor's office is a short drive from the client. What does the ACA Code of Ethics direct?

A. Keep using video, since the client chose telehealth at the start and consent is set

Informed consent is ongoing, and the Code addresses what to do when distance services are not working.

B. Consider delivering services face-to-face, since distance services are not effective ✓ KEY

H.4.d directs counselors to consider face-to-face services when distance counseling is ineffective.

C. End counseling, because poor technology means the client cannot benefit from services

Ending services ignores an available in-person option that could meet the client's needs.

D. Switch to email only, since written messages avoid both hearing and connection issues

Email-only work is a major change that may not be effective and should not be imposed this way.

Rationale: ACA H.4.d states that when distance counseling services are deemed ineffective by the counselor or client, counselors consider delivering services face-to-face; if that is not possible, they help the client find appropriate services. H.4.e also asks counselors to consider access issues. Here an in-person option is close at hand.

References: ACA Code of Ethics — H.4.d Effectiveness of Services

☐ Approve ☐ Revise ☐ Retire Notes:

A counselor in private practice wants to use a free consumer video-chat app for telehealth sessions because clients already have it on their phones. What is the MAIN ethical concern?

A. Free apps cost too little to count as a legitimate professional business expense

Cost is not the ethical issue; security of client information is.

B. Clients may enjoy video sessions more than in-person ones and stop coming to the office

Client preference for telehealth is not an ethical problem.

C. Video sessions cannot be billed, so offering them would mislead clients about fees

Many payers reimburse telehealth; billing rules are not the main concern here.

D. The platform may not use encryption and safeguards that meet legal privacy standards ✓ KEY

H.2.d requires current encryption standards that meet applicable legal requirements.

Rationale: ACA H.2.d directs counselors to use current encryption standards in technology-based communications that meet applicable legal requirements. Under HIPAA, a covered counselor also needs appropriate safeguards and, typically, a business associate agreement with the platform vendor. Convenience does not outweigh protecting client confidentiality.

References: ACA Code of Ethics — H.2.d Security · HIPAA — Security Rule safeguards for electronic PHI and business associate agreements

☐ Approve ☐ Revise ☐ Retire Notes:

A 52-year-old client has met all of his goals for managing panic attacks and has had no symptoms for three months. He enjoys sessions and asks to keep coming every week "just to talk." What is the counselor's MOST appropriate response?

A. Discuss ending counseling, since he no longer appears to need the counselor's help ✓ KEY

A.11.c says to terminate when it is reasonably apparent the client no longer needs assistance.

B. Continue weekly sessions indefinitely, since the client wants them and pays for them

Continuing services that are no longer needed does not serve the client's welfare.

C. End services at the next session without discussion, since his goals have been met

Termination should be planned and discussed, not abrupt.

D. Set new goals for him so that continued weekly sessions can be justified to the payer

Creating goals to justify services puts the counselor's or payer's needs ahead of the client.

Rationale: ACA A.11.c directs counselors to terminate when it becomes reasonably apparent that clients no longer need assistance, are not likely to benefit, or are being harmed by continued counseling. Termination is a planned process that reviews gains, prepares for setbacks, and leaves the door open to return if new concerns arise.

References: ACA Code of Ethics — A.11.c Appropriate Termination

☐ Approve ☐ Revise ☐ Retire Notes:

In week five of a 12-week closed process group for young adults, a member privately tells the leader he plans to quit and will not come back. He says he feels criticized by other members. What should the leader do FIRST?

A. Insist that he finish all 12 weeks, as he agreed to that commitment when he joined

Members have the right to leave; the leader cannot require them to stay.

B. Tell the group next week that he quit because the others had criticized him too much

Disclosing his private reasons without consent would breach his trust.

C. Explore his reasons and encourage him to raise the decision in the group before leaving ✓ KEY

Discussing the decision in group can resolve the conflict and lets members process the ending.

D. Accept his choice at once and remove him from the group without any more discussion

Quick acceptance misses a chance to address the conflict and can harm the member and the group.

Rationale: A member's abrupt departure affects both the member and the group. Leaders respect a member's right to leave but explore the reasons and encourage the member to talk about the decision in the group, where interpersonal tension can often be worked through. Policies on leaving the group are best explained during pre-group orientation.

References: Yalom & Leszcz (Group Psychotherapy) — Premature termination: dropouts and discussing departure in the group · ASGW Best Practice Guidelines — Planning and performing: informing members of group policies and procedures

☐ Approve ☐ Revise ☐ Retire Notes:

A client in private practice has missed three payments despite reminders and a reduced fee. The counselor's consent form explains what happens when fees go unpaid. The client is not in crisis. Which course BEST fits the ACA Code of Ethics?

A. Keep seeing the client for free, since ending services over money is forbidden

A.11.c permits termination for nonpayment of agreed fees.

B. Stop scheduling sessions and send a letter, with no need for further discussion

Ending without pretermination counseling risks abandonment.

C. Hold the client's records until the balance is paid, then plan a referral after

Withholding records over a debt harms the client and is not an appropriate step.

D. End services for nonpayment, with pretermination counseling and referral options ✓ KEY

The Code allows termination for unpaid fees when the counselor provides pretermination counseling and referrals.

Rationale: ACA A.11.c permits counselors to terminate when clients do not pay fees as agreed, but they provide pretermination counseling and recommend other service providers when necessary. Explaining nonpayment procedures in informed consent at the start makes this process fair and predictable.

References: ACA Code of Ethics — A.11.c Appropriate Termination

☐ Approve ☐ Revise ☐ Retire Notes:

Four months into talk therapy, a counselor believes a structured exposure-based approach would help a client with social anxiety. The client signed a general consent form at intake. What should the counselor do before starting the new approach?

A. Start it next session, since the intake consent already covered the counseling services

General consent at intake does not cover a major change in modality.

B. Explain the approach, its risks and benefits, and the client's right to decline it ✓ KEY

A.2.a and A.2.b describe consent as ongoing and give clients the right to refuse modality changes.

C. Get a supervisor's approval, since the client cannot judge the merits of the technique

Consultation may help, but it does not replace the client's own informed choice.

D. Introduce the exercises gradually without labeling them so the client does not worry

Hiding the nature of an intervention undermines informed consent and trust.

Rationale: ACA A.2.a treats informed consent as an ongoing part of counseling that is documented throughout the relationship. A.2.b includes the client's right to be informed about techniques, risks and benefits, and to refuse any services or modality changes and be told the consequences. A new approach calls for a fresh consent discussion.

References: ACA Code of Ethics — A.2.a Informed Consent; A.2.b Types of Information Needed

☐ Approve ☐ Revise ☐ Retire Notes:

A counselor will lead a 16-week group for adults ordered by a court to attend after domestic violence convictions. The probation office requires attendance and progress reports. What must the counselor explain to members before the group begins?

A. What information will be shared with the probation office and with whom it is shared ✓ KEY

A.2.e requires explaining to mandated clients what information is shared and with whom.

B. That everything said in group stays confidential, the same as in voluntary counseling

This misstates the limits; reports to probation are required in a mandated group.

C. That members have no right to refuse services because the court ordered the group

Mandated clients may refuse; counselors discuss the possible consequences of refusing.

D. That progress reports will be shared only if a member breaks one of the group's rules

Reports are routine in this setting, and members need to know that up front.

Rationale: ACA A.2.e directs counselors working with mandated clients to discuss the required limits of confidentiality and to explain what type of information is shared and with whom before counseling begins. Clients may still choose to refuse services, and counselors discuss the potential consequences. In groups, this pre-group orientation is part of informed consent.

References: ACA Code of Ethics — A.2.e Mandated Clients · ASGW Best Practice Guidelines — Planning: informed consent and orientation of prospective members

☐ Approve ☐ Revise ☐ Retire Notes:

A counselor educator wants students in her practicum course to complete surveys for her study on counselor self-efficacy. What must she make clear to the students under the ACA Code of Ethics?

A. Taking part is a course requirement, since research is part of professional training

Making participation mandatory removes the voluntary choice the Code protects.

B. Students who take part will get extra credit that others cannot earn any other way

Credit available only through participation pressures students; an equal alternative is needed.

C. Students may decline, but declining will be noted in the course evaluation file

Recording refusals in evaluations links participation to academic standing.

D. Choosing not to take part will not affect their grades, and an alternative is offered ✓ **KEY**

The Code requires that participation not affect academic standing and that an alternative be offered.

Rationale: Under ACA Section G, researchers who involve students or supervisees make clear that the decision to participate does not affect academic standing or the supervisory relationship. Students who choose not to participate are given an appropriate alternative way to meet any requirement. This protects voluntary consent where a power imbalance exists.

References: ACA Code of Ethics — G.2 Rights of Research Participants (Student/Supervisee Participation) · Erford (Research and Evaluation in Counseling) — Informed consent and voluntary participation

☐ Approve ☐ Revise ☐ Retire Notes:

An IRB approved a study in which participants were told it measured "attention" when it really measured reactions to staged peer feedback. Data collection for each participant has just ended. What is the researcher's main obligation at this point?

A. Keep the true purpose hidden so that future participants will not learn of it from them

Protecting the design does not override participants' right to a full explanation.

B. Wait to explain the study until results are published so the findings stay unbiased

Clarification is owed after data collection, not delayed until publication.

C. Fully explain the study's nature and correct any misconceptions the deception caused ✓ **KEY**

The Code requires full clarification after data collection to remove misconceptions.

D. Send a brief thank-you note, since the IRB approval already covers the deception used

IRB approval does not replace the duty to debrief participants.

Rationale: ACA Section G directs that after data are collected, counselors give participants full clarification of the nature of the study to remove any misconceptions. When deception was used, this debriefing explains what was concealed and why. Where withholding information is justified, researchers take reasonable steps to avoid harm.

References: ACA Code of Ethics — G.2 Rights of Research Participants (Explanations After Data Collection) · Erford (Research and Evaluation in Counseling) — Deception and debriefing in research with human participants

☐ Approve ☐ Revise ☐ Retire Notes:

Midway through a timed achievement test given to a 14-year-old student, a fire alarm forces a 20-minute evacuation. The counselor resumes the test afterward. How should the counselor handle the results?

A. Score and report the results as usual, since the student finished all of the test

An interruption is an irregularity that may affect validity and must not be ignored.

B. Note the irregularity in the interpretation, and treat results as questionable ✓ **KEY**

The Code requires noting nonstandard conditions; results may be invalid or of questionable validity.

C. Add 20 minutes of credit to the student's raw score to make up for the lost time

Adjusting scores by hand has no basis in the norms and distorts the results.

D. Destroy the answer sheet and record that the student did not take the test at all

Destroying data and misreporting events is dishonest and is not required.

Rationale: Under ACA Section E, counselors give assessments under the conditions set during standardization. When irregularities occur during administration, those conditions are noted in interpretation, and results may be designated as invalid or of questionable validity. Retesting with an alternate form may be considered if appropriate.

References: ACA Code of Ethics — E.7 Conditions of Assessment Administration (administration conditions and irregularities) · Hays (Assessment) — Standardized administration and threats to validity

☐ Approve ☐ Revise ☐ Retire Notes:

A school team wants to decide a 12-year-old's placement using intelligence and achievement scores from testing done when the student was 7, on an edition of the test that has since been revised. Under the ACA Code of Ethics, what should the counselor advise?

A. Do not rely on those outdated results for this decision; seek current assessment data ✓ **KEY**

E.11 bars using obsolete or outdated assessment results for the current purpose.

B. Use the old scores, since intelligence is stable and the earlier testing was valid

Results from five years earlier on an older edition are outdated for today's decision.

C. Use the old scores but raise each by a few points to adjust for the student's growth

Ad hoc score adjustment has no psychometric basis.

D. Let the team decide, since placement is an education matter outside assessment ethics

Counselors have an ethical duty to prevent misuse of outdated assessment data by others.

Rationale: ACA E.11 states that counselors do not use data or results from assessments that are obsolete or outdated for the current purpose, such as noncurrent versions of instruments, and they make every effort to prevent others from misusing obsolete measures and data. High-stakes decisions call for current, appropriate assessment.

References: ACA Code of Ethics — E.11 Obsolete Assessment and Outdated Results

☐ Approve ☐ Revise ☐ Retire Notes:

While screening for a new depression support group, a counselor learns that one applicant is her next-door neighbor, whose children play with hers. The applicant is a good clinical fit. What is the BEST course of action?

A. Admit the neighbor without comment, since screening is about clinical fit and not the leader

Screening also weighs relationships that could harm the member or the group.

B. Admit the neighbor but avoid any contact at home for the full length of the group

Avoiding the neighbor is unrealistic and does not address the dual relationship itself.

C. Ask other members to vote on whether the neighbor may join at the first session

This exposes the applicant's private information and shifts the leader's duty to members.

D. Discuss the dual-relationship risks with the applicant and help find another group ✓ KEY

A pre-existing personal relationship is a reason to explore risks and refer to another group.

Rationale: ACA A.9.a asks group leaders to screen prospective members and select those whose well-being will not be jeopardized by the group experience. A close personal relationship with the leader creates a dual relationship that can compromise objectivity and the member's comfort in group. When an alternative exists, referral is usually best.

References: ACA Code of Ethics — A.9.a Screening · ASGW Best Practice Guidelines — Screening and selection of members; avoiding dual relationships

☐ Approve ☐ Revise ☐ Retire Notes:

A counselor using Forester-Miller and Davis's Practitioner's Guide has clearly identified the problem: a client's employer wants session updates. According to the model, what should the counselor do NEXT?

A. Brainstorm as many possible courses of action as she can with a trusted colleague

Generating options is a later step in the model.

B. Pick the action she favors and test it against justice, publicity and universality

Evaluating a chosen action comes after options are generated and weighed.

C. Apply the ACA Code of Ethics to see whether it addresses or resolves the issue ✓ KEY

Applying the ACA Code is the second step, right after identifying the problem.

D. Carry out the most cautious response at once and record her reasons in the file

Implementing a course of action is the final step of the model.

Rationale: Forester-Miller and Davis outline seven steps: identify the problem, apply the ACA Code of Ethics, determine the nature and dimensions of the dilemma, generate potential courses of action, consider consequences and choose, evaluate the selected course, and implement it. If the Code clearly resolves the issue, the counselor follows it; if not, the process continues.

References: Forester-Miller & Davis (Practitioner's Guide to Ethical Decision Making) — Steps 1 and 2: identify the problem, then apply the ACA Code of Ethics

☐ Approve ☐ Revise ☐ Retire Notes:

A counselor who leads psychoeducational and therapy groups wants to join the ACA division that publishes best practice guidelines and training standards for group workers. Which organization should she join?

A. American Group Psychotherapy Association

AGPA is an independent interdisciplinary group, not an ACA division.

B. Association for Specialists in Group Work ✓ KEY

ASGW is the ACA division for group work and publishes best practice and training standards.

C. American Mental Health Counselors Association

AMHCA represents clinical mental health counselors, not group work specifically.

D. National Board for Certified Counselors

NBCC is a credentialing body, not an ACA division.

Rationale: The Association for Specialists in Group Work (ASGW) is the ACA division devoted to group work. It publishes the Best Practice Guidelines, the Professional Standards for the Training of Group Workers and the Multicultural and Social Justice Competence Principles for Group Workers. Leaders often use these documents in planning, performing and processing groups.

References: ASGW Best Practice Guidelines — ASGW as the ACA division for group work and its guidelines

☐ Approve ☐ Revise ☐ Retire Notes:

Intake, Assessment, and Diagnosis

120 items · NCE weight 12% (19 of 160 scored items)

Q121 of 1000 · nce-s-ass-001 · Counseling and Helping Relationships · intake interview structure · easy · In review

A counselor at a community agency begins a first session with a 34-year-old client who has been referred for "stress." Before asking about presenting concerns or history, what should the counselor address?

A. A full family genogram covering at least three generations

A genogram can be useful later, but it is not the opening task and does not secure informed consent.

B. Informed consent, including the limits of confidentiality ✓ **KEY**

Clients need to know their rights, the nature of services, and the limits of confidentiality before disclosing sensitive information.

C. A provisional DSM-5-TR diagnosis for the referral form

A diagnosis requires assessment data that have not yet been gathered; it cannot come first.

D. Long-term treatment goals the client hopes to reach

Goal setting follows assessment of the presenting problem; it is premature at the very start.

Rationale: Intake interviews typically open with informed consent and an explanation of confidentiality and its limits, then move to presenting problem, history, mental status, risk, and goals. Discussing limits first lets the client make an informed choice about what to disclose.

References: ACA Code of Ethics — A.2.a Informed Consent; B.1.d Explanation of Limitations

☐ Approve ☐ Revise ☐ Retire Notes:

Q122 of 1000 · nce-s-ass-002 · Counseling and Helping Relationships · mental status examination · hard · In review

When asked why he came in, a 61-year-old client describes his commute, the parking garage, a conversation with a neighbor, and the weather in great detail before finally explaining that he has been sleeping poorly. How should the counselor record this in the mental status examination?

A. Tangentiality, noted under thought process

Tangential speech drifts away and does not return to the original question; this client does return to it.

B. Flight of ideas, noted under thought content

Flight of ideas involves rapid, accelerated shifts, usually with pressured speech, and is a thought process finding, not content.

C. Circumstantiality, noted under thought process ✓ **KEY**

Excessive, irrelevant detail that eventually reaches the point is circumstantiality, a disturbance of thought process.

D. Perseveration, noted under perceptual disturbance

Perseveration is repetition of the same response, and it is not a perceptual finding.

Rationale: The MSE separates thought process (how ideas connect) from thought content (what the person thinks about). Circumstantial speech is roundabout but goal-directed in the end; tangential speech never gets back to the goal. Both are recorded under thought process.

References: Hays (Assessment) — Mental status examination: thought process

☐ Approve ☐ Revise ☐ Retire Notes:

During intake, a 19-year-old college student says, "Sometimes I wish I could just go to sleep and not wake up." What should the counselor do FIRST?

A. Ask directly about thoughts of suicide, plans, intent, and means ✓ KEY

Passive death wishes call for direct, specific questions about ideation, plan, intent, means, and past behavior to estimate risk.

B. Avoid the subject so as not to put the idea of suicide in her mind

Asking about suicide does not increase risk; avoiding it leaves risk unassessed.

C. Arrange an involuntary hospitalization before the session ends

Level of care is decided after assessment; a passive wish alone does not establish need for involuntary admission.

D. Refer her to a psychiatrist and end the intake for the day

Referral without assessment abandons the risk question and may leave the client without support.

Rationale: A wish to be dead is the lowest rung of suicidal ideation on structured tools such as the C-SSRS, but it requires follow-up. The counselor asks plainly about active thoughts, method, intent, plan, preparatory behavior, and prior attempts, then decides on next steps based on that picture.

References: C-SSRS — Ideation severity: wish to be dead through active ideation with plan and intent

☐ Approve ☐ Revise ☐ Retire Notes:

A 47-year-old veteran reports recurring suicidal thoughts without a current plan or intent. He mentions that he keeps a loaded handgun in his nightstand. Which step is MOST important to include in his safety plan?

A. A written promise that he will not harm himself before next week

No-harm contracts lack evidence of protective value and do not address lethal means.

B. A list of reasons for living to read when he feels hopeless

Reasons for living can help, but they do not reduce access to a highly lethal method.

C. A plan to increase his exercise and sleep routine this month

Wellness activities are reasonable adjuncts but leave the most urgent risk factor untouched.

D. Lethal means counseling to store the gun outside the home ✓ KEY

Firearm access sharply raises lethality; collaborative means restriction is a core safety-plan step.

Rationale: Firearms are the most lethal common method of suicide, and reducing access during high-risk periods saves lives. The Stanley-Brown Safety Planning Intervention includes making the environment safe, such as having a trusted person hold a firearm or storing it locked away from home.

References: Stanley-Brown SPI — Step 6 (final step): Making the environment safe, including lethal means

☐ Approve ☐ Revise ☐ Retire Notes:

A counselor trained years ago asks a colleague where to record the Axis V Global Assessment of Functioning (GAF) score when using DSM-5-TR. What is the BEST answer?

A. On Axis V, as in earlier versions of the manual
DSM-5-TR has no Axis V; the multiaxial system is gone.

B. On Axis IV, together with psychosocial stressors
There are no axes in DSM-5-TR, so Axis IV does not exist either.

C. Nowhere; DSM-5-TR uses nonaxial documentation ✓ **KEY**
DSM-5 dropped the multiaxial system and the GAF; diagnoses are listed together without axes.

D. In a required appendix scored from zero to 100
No GAF appendix exists; the GAF was removed from the manual.

Rationale: DSM-5 and DSM-5-TR replaced the five-axis system with nonaxial documentation. Psychosocial and contextual factors are noted with codes from "Other Conditions That May Be a Focus of Clinical Attention," and the GAF was dropped; the WHODAS 2.0 is offered as a measure of disability.

References: DSM-5-TR — Section I: Use of the Manual (nonaxial documentation)

☐ Approve ☐ Revise ☐ Retire Notes:

A 28-year-old client currently meets criteria for a major depressive episode. Two years ago she had a 5-day period of elevated mood, decreased need for sleep, and rapid speech that friends noticed, with no marked impairment, hospitalization, or psychotic features. She has had no other mood episodes. Which diagnosis BEST fits?

A. Major depressive disorder, recurrent
A past hypomanic episode rules out major depressive disorder.

B. Bipolar II disorder ✓ **KEY**
At least one hypomanic episode plus at least one major depressive episode, with no manic episode, defines bipolar II.

C. Bipolar I disorder
Bipolar I requires a manic episode, which lasts at least one week (or any duration if hospitalized) and causes marked impairment, hospitalization, or psychosis.

D. Cyclothymic disorder
Cyclothymia requires two years of subthreshold symptoms and is excluded once a major depressive episode occurs.

Rationale: Hypomania lasts at least four consecutive days, is observable by others, and does not cause marked impairment or require hospitalization. Mania lasts at least a week (or any duration if hospitalized) and is severe enough to cause marked impairment, require hospitalization, or include psychotic features. A hypomanic episode plus a major depressive episode equals bipolar II.

References: DSM-5-TR — Bipolar II Disorder criteria; Hypomanic Episode Criterion A

☐ Approve ☐ Revise ☐ Retire Notes:

A test developer gives the same anxiety inventory to the same group of adults twice, two weeks apart, and correlates the two sets of scores. Which type of reliability is being estimated?

A. Test-retest reliability ✓ **KEY**

Correlating scores from the same test given twice to the same people estimates stability over time.

B. Alternate-form reliability

Alternate-form reliability uses two different but equivalent versions of the test.

C. Split-half reliability

Split-half reliability comes from one administration divided into two halves.

D. Interrater reliability

Interrater reliability is agreement between two or more scorers or observers.

Rationale: Test-retest reliability (the coefficient of stability) reflects how consistent scores are across time. The interval matters: too short invites practice and memory effects, too long lets real change lower the coefficient.

References: Hays (Assessment) — Reliability: test-retest (coefficient of stability)

☐ Approve ☐ Revise ☐ Retire Notes:

A school counselor has a single administration of a 40-item vocabulary test in which every item is scored simply right or wrong. Which statistic is BEST suited to estimate the test's internal consistency?

A. A test-retest correlation

Test-retest requires two administrations and measures stability, not internal consistency.

B. Cohen's kappa coefficient

Kappa indexes agreement between raters, corrected for chance.

C. An alternate-form coefficient

Alternate-form reliability needs a second, equivalent version of the test.

D. Kuder-Richardson Formula 20 ✓ **KEY**

KR-20 estimates internal consistency from one administration of dichotomously scored items.

Rationale: Internal consistency estimates come from one administration. KR-20 is used with dichotomous (right/wrong) items; coefficient (Cronbach's) alpha generalizes it to items with more than two score points. Split-half reliability, corrected with the Spearman-Brown formula, is another single-administration option.

References: Erford (Research and Evaluation in Counseling) — Internal consistency: KR-20 and coefficient alpha

☐ Approve ☐ Revise ☐ Retire Notes:

Developers of a new mechanical aptitude test give it to newly hired technicians and correlate those scores with supervisor performance ratings collected 18 months later. What type of validity evidence does this provide?

A. Concurrent validity

Concurrent validity uses a criterion measured at about the same time as the test.

B. Predictive validity ✓ KEY

The criterion is measured well after testing, so this is criterion-related predictive validity.

C. Content validity

Content validity concerns how well items sample the domain, judged by experts, not a criterion correlation.

D. Discriminant validity

Discriminant validity is construct evidence showing low correlation with unrelated constructs.

Rationale: Criterion-related validity has two forms that differ by timing: concurrent (criterion gathered at the same time) and predictive (criterion gathered later). Correlating test scores with later job ratings shows how well the test forecasts future performance.

References: Hays (Assessment) — Validity: criterion-related (concurrent and predictive)

☐ Approve ☐ Revise ☐ Retire Notes:

A new depression scale correlates .78 with an established depression inventory and .06 with a measure of musical aptitude. The low correlation with musical aptitude is BEST described as evidence of which type of validity?

A. Convergent validity

Convergent evidence is the high correlation with the established depression inventory, not the low one.

B. Predictive validity

Predictive validity concerns forecasting a later criterion, which is not described here.

C. Discriminant validity ✓ KEY

A low correlation with a theoretically unrelated construct supports discriminant (divergent) validity.

D. Face validity

Face validity is whether a test looks like it measures its target; it is not shown by correlations.

Rationale: Construct validity is supported by convergent evidence (strong relations with measures of the same construct) and discriminant evidence (weak relations with measures of different constructs). The multitrait-multimethod matrix examines both together.

References: Erford (Research and Evaluation in Counseling) — Construct validity: convergent and discriminant evidence

☐ Approve ☐ Revise ☐ Retire Notes:

A district reading test reports, for each fourth grader, whether the student has mastered at least 80% of the grade-level reading objectives, regardless of how classmates performed. What kind of test is this?

A. A norm-referenced test

Norm-referenced tests compare a score with a norm group, which this report does not do.

B. An ipsative measure

Ipsative measures compare traits within one person, not mastery of a standard.

C. A projective technique

Projective techniques use ambiguous stimuli to explore personality.

D. A criterion-referenced test ✓ KEY

Comparing performance with a fixed standard of mastery is criterion-referenced.

Rationale: Criterion-referenced tests interpret a score against a set standard or body of content (what the person can do). Norm-referenced tests interpret a score against the performance of a comparison group (how the person ranks).

References: Hays (Assessment) — Norm-referenced vs criterion-referenced interpretation

☐ Approve ☐ Revise ☐ Retire Notes:

Scores on a test are normally distributed. Approximately what percentage of test takers score between one standard deviation below the mean and two standard deviations above the mean?

A. About 82% ✓ KEY

About 34% fall between -1 SD and the mean, and about 47.5% between the mean and +2 SD, for roughly 82%.

B. About 68%

68% is the share within plus or minus one SD of the mean.

C. About 95%

95% is the share within plus or minus two SD of the mean.

D. About 48%

About 47.5% lies between the mean and +2 SD alone; the band below the mean is left out.

Rationale: On the normal curve about 68% of scores fall within 1 SD of the mean, about 95% within 2 SD, and about 99.7% within 3 SD. Halving those figures gives the share on one side: about 34% from the mean to 1 SD and about 47.5% from the mean to 2 SD.

References: Erford (Research and Evaluation in Counseling) — The normal curve: 68-95-99.7 rule

☐ Approve ☐ Revise ☐ Retire Notes:

A counselor compares four results from different normally distributed tests. Which result reflects the HIGHEST standing relative to its norm group?

A. A z score of +1.2

A z of +1.2 is below the T score of 65, which equals $z = +1.5$.

B. A deviation IQ of 118

With mean 100 and SD 15, an IQ of 118 equals $z = +1.2$.

C. A T score of 65 ✓ KEY

With mean 50 and SD 10, a T of 65 is 1.5 SD above the mean, the highest of the four.

D. A percentile rank of 88

The 88th percentile corresponds to a z of about +1.17.

Rationale: Converting each score to z allows comparison: $z = (\text{score} - \text{mean}) / \text{SD}$. T scores have a mean of 50 and SD of 10; deviation IQs have a mean of 100 and SD of 15; the 84th percentile is about $z = +1.0$ and the 88th about $z = +1.17$.

References: Hays (Assessment) — Standard scores: z, T and deviation IQ conversions

☐ Approve ☐ Revise ☐ Retire Notes:

On a normally distributed test, one student's percentile rank rose from 50 to 60 and another student's rose from 89 to 99. Which statement about their raw-score gains is accurate?

A. Both students gained about the same number of raw-score points

Percentile ranks are not equal-interval units, so equal percentile gains do not mean equal raw gains.

B. The second student needed a much larger raw-score gain ✓ KEY

Scores thin out in the upper tail, so moving from the 89th to 99th percentile takes far more raw-score distance.

C. The first student needed a much larger raw-score gain

Scores cluster near the mean, so a 10-rank gain there takes a small raw-score change.

D. The gains cannot be compared without first converting to stanines

Stanines are not needed; converting percentiles to z scores shows the difference directly.

Rationale: Percentile ranks are ordinal. Because most scores pile up near the mean, a small raw change in the middle moves the percentile a lot, while the same percentile change in the tail requires a large raw change. Here 50 to 60 is about 0.25 SD; 89 to 99 is about 1.1 SD.

References: Hays (Assessment) — Percentile ranks: ordinal scale and interpretation limits

☐ Approve ☐ Revise ☐ Retire Notes:

A client obtains a score of 92 on a test with a standard error of measurement (SEM) of 4. Which range is the approximate 95% confidence band for the client's true score?

A. 88 to 96

Plus or minus 1 SEM gives a band of about 68% confidence.

B. 90 to 94

Plus or minus half an SEM is far narrower than any standard band.

C. 80 to 104

Plus or minus 3 SEM gives about 99.7% confidence, wider than asked.

D. 84 to 100 ✓ KEY

Plus or minus about 2 SEM (1.96×4 , roughly 8 points) gives the 95% band.

Rationale: The SEM is the standard deviation of a person's hypothetical scores around the true score. Bands of plus or minus 1 SEM, about 1.96 SEM (often rounded to 2), and 3 SEM around the obtained score give roughly 68%, 95%, and 99.7% confidence that the true score lies inside. Here $1.96 \times 4 = 7.84$, so the 95% band is about 84 to 100.

References: Hays (Assessment) — Standard error of measurement and confidence intervals

☐ Approve ☐ Revise ☐ Retire Notes:

A test has a standard deviation of 15 and a reliability coefficient of .91. What is its standard error of measurement?

A. 4.5 points ✓ KEY

$SEM = 15 \times \text{the square root of } (1 - .91) = 15 \times 0.3 = 4.5$.

B. 1.35 points

This multiplies the SD by .09 and omits the square root.

C. 13.65 points

This multiplies the SD by the reliability coefficient itself.

D. 0.3 points

This is only the square root of $(1 - .91)$, not multiplied by the SD.

Rationale: $SEM = SD \times \text{the square root of } (1 - r)$. Higher reliability means a smaller SEM and narrower confidence bands; a perfectly reliable test would have an SEM of zero.

References: Erford (Research and Evaluation in Counseling) — Standard error of measurement formula

☐ Approve ☐ Revise ☐ Retire Notes:

A school-based counselor is part of a team evaluating the intellectual functioning of a 10-year-old. Which instrument is designed for this child's age?

A. WAIS-IV

The WAIS-IV is normed for adults and older adolescents (16 and up).

B. WPPSI-IV

The WPPSI-IV is for preschool and early primary children, below age 8.

C. WISC-V ✓ KEY

The WISC-V covers ages 6 through 16, including a 10-year-old.

D. MMPI-A

The MMPI-A is an adolescent personality inventory, not an intelligence test.

Rationale: The Wechsler scales are split by age: WPPSI-IV for young children (about 2½ to 7½), WISC-V for ages 6 to 16, and WAIS-IV for ages 16 to 90. Each yields a Full Scale IQ with a mean of 100 and SD of 15.

References: Hays (Assessment) — Intelligence testing: Wechsler scales by age range

► **Check:** Check the Wechsler age ranges used in the rationale (WPPSI-IV, WISC-V, WAIS-IV).

☐ Approve ☐ Revise ☐ Retire Notes:

A counselor wants a personality inventory built on the five-factor model (neuroticism, extraversion, openness, agreeableness, and conscientiousness). Which instrument fits this purpose?

A. Rorschach Inkblot Test

The Rorschach is a projective technique, not a five-factor inventory.

B. NEO Personality Inventory-3 ✓ KEY

The NEO-PI-3 measures the five broad domains of the five-factor model and their facets.

C. Myers-Briggs Type Indicator

The MBTI sorts people into Jungian types rather than measuring the five factors.

D. MMPI-3

The MMPI-3 focuses on psychopathology and validity scales, not the five-factor model.

Rationale: The NEO-PI-3 (Costa and McCrae) assesses neuroticism, extraversion, openness, agreeableness, and conscientiousness, each with six facets. The MMPI family is used mainly to assess psychopathology and includes validity scales for over- and underreporting.

References: Hays (Assessment) — Personality inventories: NEO-PI-3 and the five-factor model

☐ Approve ☐ Revise ☐ Retire Notes:

A counselor shows an adolescent a series of ambiguous drawings of people in everyday situations and asks him to tell a story about each one, with a beginning, middle, and end. Which instrument is being used?

A. Rorschach Inkblot Test

The Rorschach uses inkblots, not pictures of people, and asks what the person sees.

B. House-Tree-Person

In House-Tree-Person the client makes the drawings rather than responding to them.

C. Rotter Incomplete Sentences

The Rotter uses sentence stems for the client to finish, not pictures.

D. Thematic Apperception Test ✓ KEY

The TAT presents ambiguous picture cards and asks for a story about each.

Rationale: The Thematic Apperception Test, developed by Morgan and Murray, is a projective technique in which stories told about ambiguous scenes are thought to reveal needs, motives, and conflicts. Projective tests rely on ambiguous stimuli and generally have weaker psychometric support than objective inventories.

References: Hays (Assessment) — Projective techniques: Thematic Apperception Test

☐ Approve ☐ Revise ☐ Retire Notes:

On the PHQ-9, a client's total score is 6, in the mild range, but she marks "several days" on the last item, which asks about thoughts that she would be better off dead or of hurting herself. What should the counselor do next?

A. Follow up at once with a direct suicide risk assessment ✓ KEY

Any endorsement of the self-harm item calls for direct risk assessment whatever the total score.

B. Note mild depression and recheck the PHQ-9 next month

Waiting ignores a positive response on the item that flags possible suicidal thinking.

C. Give the GAD-7 to see whether anxiety explains the item

An anxiety screen does not assess or rule out suicide risk.

D. Treat the item as invalid because the total score is low

A low total does not cancel a positive item 9; risk can exist with few other symptoms.

Rationale: The PHQ-9 has nine items scored 0 to 3 (total 0 to 27). Item 9 screens for passive or active thoughts of death or self-harm; any positive answer should be followed by a clinician's direct suicide risk assessment, since a screening total alone does not measure risk.

References: Hays (Assessment) — Screening instruments: Patient Health Questionnaire-9 (PHQ-9)

☐ Approve ☐ Revise ☐ Retire Notes:

A 52-year-old client says that when he goes a day without drinking he gets shaky, sweaty, and cannot sleep. Using the ASAM Criteria (3rd edition), under which dimension should the counselor rate these symptoms?

A. Readiness to change

This dimension rates motivation and engagement, not physical withdrawal.

B. Recovery environment

This dimension rates the client's living situation and supports.

C. Intoxication and withdrawal ✓ KEY

Tremor, sweating, and insomnia on stopping alcohol are withdrawal signs, rated in Dimension 1.

D. Relapse potential

This dimension rates risk of continued use, not current withdrawal.

Rationale: The ASAM Criteria rate six dimensions to match level of care. Dimension 1 covers intoxication and withdrawal; in the widely taught third edition the others address biomedical conditions, emotional/behavioral/cognitive conditions, readiness to change, relapse or continued-use potential, and the recovery environment. The 2023 fourth edition renamed and reorganized several later dimensions, but intoxication and withdrawal remains part of Dimension 1. Alcohol withdrawal can be dangerous and may call for medically managed care.

References: ASAM Criteria — 3rd edition, Dimension 1: acute intoxication and/or withdrawal potential

☐ Approve ☐ Revise ☐ Retire Notes:

A counselor is asked to assess the cognitive abilities of a 13-year-old who arrived from Guatemala six months ago and is still learning English. Which approach is MOST appropriate?

A. Give a verbal English IQ test and report the score as usual

The score would mostly reflect English proficiency, so reporting it without caveats is invalid.

B. Use a nonverbal or native-language measure, interpreted with care ✓ KEY

Reducing language load and noting norm limits gives a fairer, more valid picture of ability.

C. Delay all testing until he has lived in the country for five years

A rigid wait is arbitrary and may delay needed services; fairer methods exist now.

D. Add 15 points to his score to adjust for the language barrier

Arbitrary score adjustment has no psychometric basis and distorts results.

Rationale: Counselors select instruments whose norms and language suit the client and interpret results in light of culture, language, and acculturation. When a test was not normed on the client's population, the report should say so. Nonverbal measures or measures in the client's dominant language reduce bias from limited English.

References: ACA Code of Ethics — E.8 Multicultural Issues/Diversity in Assessment · Sue & Sue (Counseling the Culturally Diverse) — Culturally biased assessment and test interpretation

☐ Approve ☐ Revise ☐ Retire Notes:

During a biopsychosocial intake, a 55-year-old client with no prior mental health history describes new anxiety, a racing heart, weight loss despite a good appetite, and feeling hot all the time. What is the counselor's BEST next step?

A. Diagnose generalized anxiety disorder and begin relaxation training

GAD requires that symptoms not be due to a medical condition, which has not been ruled out.

B. Explore childhood experiences that may underlie the new anxiety

This skips the possibility that a medical problem is driving the symptoms.

C. Reassure the client that midlife anxiety is common and expected

Reassurance without evaluation may miss a treatable physical cause.

D. Recommend a medical evaluation to rule out a physical cause ✓ **KEY**

Late onset with weight loss and heat intolerance suggests a possible medical cause, such as thyroid disease.

Rationale: The biological part of a biopsychosocial history covers health, medications, and physical symptoms. New-onset anxiety in midlife with physical signs such as weight loss and heat intolerance can reflect conditions like hyperthyroidism, so a medical evaluation comes before a mental disorder diagnosis.

References: DSM-5-TR — Anxiety Disorder Due to Another Medical Condition; GAD Criterion E

☐ Approve ☐ Revise ☐ Retire Notes:

A client being assessed for alcohol use says he "only has a couple of beers on weekends." With his written consent, the counselor also interviews his spouse. What is the PRIMARY value of this collateral information?

A. It can confirm or add to self-report that may be minimized ✓ **KEY**

Collateral sources help check self-report, which can be limited by minimization or poor insight.

B. It removes the need for a structured diagnostic interview

Collateral data supplement, not replace, a thorough clinical interview.

C. It lets the spouse set the goals of the client's treatment

Treatment goals are set collaboratively with the client, who directs his own care.

D. It supplies the evidence that state law requires to diagnose

Diagnosis does not legally require collateral reports; they are clinically useful, not mandatory.

Rationale: Collateral information from family, prior records, or other providers broadens the assessment, especially when the client may underreport, lack insight, or have memory problems. It is gathered with the client's consent and weighed alongside the client's own account.

References: Hays (Assessment) — Collateral sources of assessment information

☐ Approve ☐ Revise ☐ Retire Notes:

A 23-year-old graduate student wants to know how her likes and dislikes compare with those of people who are employed in, and satisfied with, a wide range of occupations. Which instrument is designed to provide this comparison?

A. Strong Interest Inventory ✓ KEY

Its Occupational Scales compare the client's responses with those of satisfied workers in many occupations.

B. Myers-Briggs Type Indicator

The MBTI sorts people into psychological types; it does not compare interests with occupational groups.

C. Differential Aptitude Tests

The DAT measures aptitudes (abilities), not interests or occupational fit.

D. Minnesota Importance Questionnaire

The MIQ measures work needs and values, not interests compared with workers in occupations.

Rationale: The Strong Interest Inventory reports General Occupational Themes (Holland's RIASEC types), Basic Interest Scales, Occupational Scales, and Personal Style Scales. The Occupational Scales are built empirically from people who are employed in an occupation and report liking it, so a high score means the client's interests resemble theirs. Interest similarity predicts satisfaction, not ability or success.

References: Sharf (Career Development Theory) — Interest inventories: Strong Interest Inventory scales · Hays (Assessment) — Career assessment: interest inventories

☐ Approve ☐ Revise ☐ Retire Notes:

A rural high school with limited counseling staff wants a career interest tool that students can take, score, and begin interpreting on their own, producing a three-letter summary code they can use to look up matching occupations. Which instrument BEST fits this need?

A. Strong Interest Inventory

The Strong is scored by the publisher and is usually interpreted with a trained counselor, not self-scored.

B. ASVAB career aptitude battery

The ASVAB measures aptitudes and is not a self-scored interest inventory.

C. Self-Directed Search ✓ KEY

Holland designed the SDS to be self-administered and self-scored, yielding a summary code linked to an occupations finder.

D. Thematic Apperception Test

The TAT is a projective personality technique, not a career interest measure.

Rationale: John Holland's Self-Directed Search asks about activities, competencies, occupations, and self-estimates, and the person totals the scores into a three-letter RIASEC summary code. The code is matched to occupations with the same code. Its self-guided design makes it practical where counselor time is limited, although counselor follow-up still improves interpretation.

References: Sharf (Career Development Theory) — Holland's theory: the Self-Directed Search

☐ Approve ☐ Revise ☐ Retire Notes:

A counselor working with a 46-year-old who lost a long-held factory job asks him to sort cards naming occupations into "would choose," "would not choose," and "undecided" piles and then talk through why he placed each card where he did. What is the MAIN advantage of this method over a standardized interest inventory?

A. It yields norm-referenced scores that compare clients fairly

Card sorts are qualitative and generally unnormed; they do not produce norm-referenced scores.

B. It brings out the client's own reasons and personal meanings ✓ KEY

The discussion of each placement reveals the client's constructs, values, and reasoning in his own words.

C. It offers strong predictive validity for later job success

Card sorts are not designed or validated to predict job success.

D. It removes the need for counselor interpretation of results

Card sorts rely heavily on counselor-client dialogue; interpretation is the heart of the method.

Rationale: Card sorts (occupational, values, or skills sorts) are qualitative, interactive assessments. Their strength is flexibility and insight into how the client thinks about work, which is useful for adults in transition. The tradeoff is the lack of norms and standardized psychometric evidence.

References: Sharf (Career Development Theory) — Qualitative career assessment: card sorts

☐ Approve ☐ Revise ☐ Retire Notes:

A 31-year-old client's interest profile fits her field well, yet she has left three jobs, each time saying the work offered little recognition or independence. Her counselor gives her the Minnesota Importance Questionnaire. What does this instrument primarily measure?

A. Her verbal, numerical, and spatial aptitudes for job training

Aptitudes are measured by ability batteries, not the MIQ.

B. Her personality type according to the five-factor model

Five-factor traits are measured by instruments such as the NEO-PI-3, not the MIQ.

C. How closely her likes resemble those of satisfied workers

That describes the Occupational Scales of an interest inventory such as the Strong.

D. Her work needs and values, compared with what jobs reward ✓ KEY

The MIQ assesses vocational needs and values so they can be matched with the reinforcers that occupations provide.

Rationale: The Minnesota Importance Questionnaire comes from Dawis and Lofquist's Theory of Work Adjustment, which holds that satisfaction depends on the match between a worker's needs and values and the reinforcers a job supplies. Interests alone may fit, but a values mismatch (such as low recognition or autonomy) can still lead to dissatisfaction and turnover.

References: Sharf (Career Development Theory) — Theory of Work Adjustment: Minnesota Importance Questionnaire

► **Check:** Confirm the MIQ is still described in Sharf as the TWA values/needs measure (it is now largely available through O*NET-related tools).

☐ Approve ☐ Revise ☐ Retire Notes:

A high school junior is exploring careers. Her counselor wants an estimate of her potential to learn in several separate areas, such as mechanical comprehension, arithmetic reasoning, and word knowledge, rather than a record of what she has already mastered in class. Which type of instrument BEST fits?

A. A multiaptitude battery such as the ASVAB ✓ **KEY**

Multiaptitude batteries estimate potential in several distinct ability areas and are widely used in career exploration.

B. An achievement battery covering school subjects

Achievement tests measure what has already been learned, which the counselor explicitly does not want.

C. A general intelligence test with one overall IQ

A single global score does not profile separate occupational ability areas.

D. An interest inventory such as the Strong

Interest inventories measure likes and dislikes, not ability or potential.

Rationale: Aptitude tests predict future learning or performance; achievement tests measure past learning from instruction; intelligence tests estimate general cognitive ability. A multiaptitude battery such as the ASVAB or the Differential Aptitude Tests yields a profile of specific abilities, which is useful alongside interests and values in career planning.

References: Hays (Assessment) — Aptitude, achievement, and intelligence testing

☐ Approve ☐ Revise ☐ Retire Notes:

A counselor educator runs an item analysis on a classroom test. Item 7: $p = .52$, $D = .41$. Item 12: $p = .48$, $D = -.22$. Item 19: $p = .90$, $D = .10$. Item 23: $p = .30$, $D = .35$. Which item should be revised or removed FIRST?

A. Item 7, because its difficulty sits too near the midpoint

A p near .50 with $D = .41$ is close to ideal; it is the strongest item here.

B. Item 23, because fewer than a third of students got it right

A hard item is acceptable when it discriminates well, and $D = .35$ is good.

C. Item 12, because low scorers outperform the high scorers ✓ **KEY**

A negative discrimination index means the item works backward and is likely flawed (miskeyed or misleading).

D. Item 19, because nearly every student answered it correctly

An easy item with weak but positive discrimination is a lower priority than one that discriminates negatively.

Rationale: Item difficulty (p) is the proportion of examinees answering correctly, so a higher p means an easier item. The discrimination index (D) is the proportion correct in the high-scoring group minus the proportion correct in the low-scoring group. Positive D is desired; a negative D signals a defective item and is the top priority for review.

References: Hays (Assessment) — Item analysis: item difficulty and item discrimination

☐ Approve ☐ Revise ☐ Retire Notes:

A test developer splits a 60-item anxiety scale into odd- and even-numbered items and finds that the two halves correlate .60. Using the Spearman-Brown formula, what is the estimated reliability of the full 60-item scale?

A. .36

Squaring .60 gives shared variance, not a reliability estimate for the lengthened test.

B. .60

This is the half-test correlation, which underestimates the reliability of the full-length test.

C. .80

This overstates the correction; $2(.60)/(1 + .60)$ is .75.

D. .75 ✓ KEY

Spearman-Brown: $2r/(1 + r) = 1.20/1.60 = .75$.

Rationale: Split-half reliability correlates two halves of one administration, but each half is only half as long as the real test, and shorter tests are less reliable. The Spearman-Brown prophecy formula corrects for this: $r(\text{full}) = 2r(\text{half}) / (1 + r(\text{half}))$. The formula can also project reliability for other changes in test length.

References: Hays (Assessment) — Split-half reliability and the Spearman-Brown formula

☐ Approve ☐ Revise ☐ Retire Notes:

Two counselors independently review 50 recorded intakes and judge whether each client meets criteria for a substance use disorder. They agree on 90% of cases, but most clients fall into the "does not meet" category. Why would a researcher report Cohen's kappa instead of simple percent agreement?

A. Kappa shows how stable each rater's judgments are over time

Stability over time is test-retest (or intra-rater) reliability, not what kappa between two raters addresses.

B. Kappa adjusts for the agreement expected by chance alone ✓ KEY

When one category dominates, raters agree often by chance; kappa removes that chance agreement.

C. Kappa proves the raters' diagnoses are accurate and valid

Agreement is reliability; two raters can agree and both be wrong, so kappa does not establish validity.

D. Kappa estimates internal consistency of the rating form items

Internal consistency is estimated with coefficient alpha or KR-20, not kappa.

Rationale: Inter-rater reliability reflects how consistently different raters score the same material. Percent agreement is inflated when base rates are lopsided, because raters will often agree by chance. Cohen's kappa compares observed agreement with chance-expected agreement, giving a more honest estimate for categorical judgments.

References: Erford (Research and Evaluation in Counseling) — Inter-rater reliability: Cohen's kappa

☐ Approve ☐ Revise ☐ Retire Notes:

Clients who try out a new workplace stress inventory say, "It's obviously asking about job stress." The developer cites these comments in the manual as the main evidence that the inventory is valid. What do the comments actually reflect?

A. Face validity, which is not real psychometric evidence ✓ KEY

Face validity is how a test looks to test takers; it can aid cooperation but is not evidence of what the test measures.

B. Content validity, shown through expert review of items

Content validity requires systematic expert judgment of how well items sample the domain, not test takers' impressions.

C. Construct validity, shown by the internal factor structure

Construct validity requires accumulated empirical evidence such as factor analysis or convergent correlations.

D. Concurrent validity, shown by agreement with a criterion

Concurrent validity requires correlating scores with a criterion measured at the same time.

Rationale: Face validity is the superficial appearance that a test measures what it claims. Content validity, by contrast, is established when experts judge items against a defined content domain or table of specifications. Only content, criterion-related, and construct evidence support score interpretation; face validity does not.

References: Hays (Assessment) — Validity: content validity versus face validity

☐ Approve ☐ Revise ☐ Retire Notes:

A developer theorizes that counselor burnout has three distinct dimensions and writes items for each. A factor analysis of responses from 900 counselors shows the items clustering on three factors that match the theorized dimensions. This result is BEST described as evidence of which kind of validity?

A. Predictive validity tied to a future outcome criterion

Predictive validity requires correlating scores with a criterion measured later.

B. Test-retest reliability across repeated administrations

Factor structure from one administration says nothing about stability over time, and it is reliability, not validity.

C. Construct validity based on the test's internal structure ✓ KEY

Items grouping into the theorized dimensions supports the claim that the test measures the intended construct.

D. Content validity confirmed by the judgments of experts

Content validity rests on expert review of item coverage, not on statistical clustering of responses.

Rationale: Construct validity asks whether a test measures the theoretical trait it is meant to measure. Factor analysis provides internal-structure evidence: if items load on factors that match the theory, the score structure fits the construct. Convergent and discriminant correlations are other common sources of construct evidence.

References: Hays (Assessment) — Construct validity: factor analysis and internal structure

☐ Approve ☐ Revise ☐ Retire Notes:

A counselor plans to give a self-esteem inventory to a 72-year-old Filipino American retiree. The manual reports norms from about 2,000 undergraduates at four Midwestern universities. What is the counselor's PRIMARY concern in interpreting her percentile score?

A. A 2,000-person sample is too small to yield stable norms

Two thousand is a substantial norm sample; size is not the problem here.

B. Self-report formats are invalid for clients over age 65

Older adults can complete self-report measures; age alone does not invalidate the format.

C. Percentile ranks first need a criterion-referenced cutoff

Percentiles are norm-referenced by definition and do not depend on a criterion cutoff.

D. The norm group does not represent clients like this one ✓ KEY

Her score would be compared with young college students, a group that differs from her in age, culture, and life stage.

Rationale: Norm-referenced scores mean something only relative to the norm group. Counselors check that the norm sample is representative of the client (age, culture, language, setting) and reasonably current. When the norm group does not match, percentile and standard scores can mislead, so the counselor should seek better-matched norms or interpret with explicit caution.

References: Hays (Assessment) — Norm groups: representativeness, size, and recency

☐ Approve ☐ Revise ☐ Retire Notes:

A third grader earns a grade-equivalent score of 6.2 on a mathematics achievement test given to her class. Her parent asks whether she should be moved into sixth-grade math. Which interpretation should the counselor give?

A. She has mastered most of the sixth-grade math curriculum

The test sampled third-grade content; it did not test sixth-grade skills.

B. She did very well on third-grade content, not sixth-grade ✓ KEY

A grade equivalent of 6.2 means her score matches what an average early sixth grader is estimated to earn on this third-grade test.

C. Her score is three standard deviations above her grade mean

Grade equivalents are not equal-interval units and cannot be converted into standard deviations this way.

D. She would score at the 62nd percentile among sixth graders

The decimal in a grade equivalent refers to the month of the school year, not a percentile.

Rationale: Grade and age equivalents are developmental scores that are easy to misread. They are often extrapolated rather than based on students actually tested at that grade, they are not equal-interval, and they do not indicate mastery of higher-grade material. Percentile ranks or standard scores within the student's own grade are better for interpretation.

References: Hays (Assessment) — Developmental scores: limits of grade and age equivalents

☐ Approve ☐ Revise ☐ Retire Notes:

A school district reports its students' achievement results as normal curve equivalents (NCEs) and as stanines. A school board member asks the counselor how to read these scores. Which statement is accurate?

A. NCEs are equal-interval scores (mean 50) that can be averaged ✓ KEY

NCEs are normalized standard scores (mean 50, SD about 21.06) on an equal-interval scale, which is why programs average them.

B. Stanines divide students into nine bands holding equal percentages

Stanine bands are equal in width (half an SD) but hold unequal percentages, about 4% at each end and 20% in the middle.

C. An NCE of 50 and a stanine of 9 describe the same relative standing

An NCE of 50 is average, matching stanine 5; stanine 9 is the top band.

D. NCEs equal percentile ranks at every point along the whole score scale

NCEs match percentile ranks only at 1, 50, and 99; elsewhere they differ because percentiles are not equal-interval.

Rationale: Stanines ("standard nines") run from 1 to 9 with a mean of 5 and an SD of about 2. Normal curve equivalents run from 1 to 99 with a mean of 50 and an SD of about 21.06, chosen so they line up with percentiles at 1, 50, and 99. Unlike percentile ranks, NCEs are equal-interval and can be averaged and compared across groups.

References: Hays (Assessment) — Standard scores: stanines and normal curve equivalents

☐ Approve ☐ Revise ☐ Retire Notes:

Q158 of 1000 · nce-s-ass-038 · Research and Program Evaluation · standard scores, percentiles and the normal curve · medium · In review

On a very difficult practice exam, most examinees earn low scores and a handful earn very high scores. How are the mean, median, and mode MOST likely to be ordered in this distribution?

A. Mean lowest, median next, and mode highest of the three

This ordering describes a negatively skewed distribution, where a few very low scores pull the mean down.

B. Mean, median, and mode fall at the same point in the curve

The three measures coincide only in a symmetrical distribution such as the normal curve.

C. Mode lowest, median next, and mean highest of the three ✓ KEY

A few very high scores create a positive skew and pull the mean toward the right tail.

D. Median lowest, with mean and mode equal and above it

The median lies between the mode and mean in a skewed distribution; it is not the lowest.

Rationale: Skew is named for the tail. When scores bunch at the low end with a tail to the right, the distribution is positively skewed, and the mean (most sensitive to extreme scores) is pulled toward the tail: mode < median < mean. In a negative skew the order reverses. The median is often preferred for describing skewed data.

References: Erford (Research and Evaluation in Counseling) — Descriptive statistics: skewness and measures of central tendency

☐ Approve ☐ Revise ☐ Retire Notes:

In a clinic sample, weekly exercise hours correlate $-.70$ with depression scores, and nightly hours of sleep correlate $+.40$ with depression scores. Which conclusion is accurate?

A. The sleep link is stronger because its coefficient is positive

The sign shows direction only; strength depends on the absolute value, and $.70$ exceeds $.40$.

B. Exercise accounts for 70% of the variance in depression scores

Shared variance is r squared, so $-.70$ corresponds to 49%, not 70%.

C. The negative coefficient shows exercise reduces depression

Correlation does not establish causation; a third variable or reverse direction could explain it.

D. The exercise link is stronger and shares 49% of the variance ✓ **KEY**

The absolute value $.70$ is larger than $.40$, and r squared ($.49$) gives the proportion of shared variance.

Rationale: A correlation coefficient's sign gives direction and its absolute value gives strength. Squaring r gives the coefficient of determination, the proportion of variance the two variables share. No correlation, however strong, demonstrates cause and effect.

References: Erford (Research and Evaluation in Counseling) — Correlation: direction, strength, and coefficient of determination

☐ Approve ☐ Revise ☐ Retire Notes: _____

An elementary school principal proposes using scores from a 10-minute behavior screening checklist, completed once by each teacher, to decide which students are placed in special education services. What is the school counselor's BEST advice?

A. Accept the plan if the checklist has internal consistency above $.80$

Good reliability does not make a brief screener valid for a high-stakes placement decision.

B. Use the screener to flag students for a full multi-measure evaluation ✓ **KEY**

Screeners identify who needs further assessment; high-stakes decisions require comprehensive evaluation from multiple sources.

C. Raise the cutoff score so that fewer students are wrongly identified

Changing the cutoff trades false positives for false negatives and still leaves a single screener driving placement.

D. Keep the screener but have a second teacher rescore every checklist

Double scoring reduces clerical error but does not address the misuse of a screener for a high-stakes decision.

Rationale: Screening instruments are brief and built to cast a wide net, accepting some false positives. High-stakes decisions such as placement, diagnosis, or eligibility should rest on multiple measures, multiple informants, and qualified professional judgment rather than a single score.

References: Hays (Assessment) — Screening versus comprehensive assessment; high-stakes testing

☐ Approve ☐ Revise ☐ Retire Notes: _____

A counselor at a university counseling center plans to give the Beck Depression Inventory-II (BDI-II) to new clients. Which description of the BDI-II is accurate?

A. A 21-item self-report of depressive symptoms over two weeks ✓ KEY

The BDI-II is a 21-item self-report of depressive symptom severity covering the past two weeks, including today.

B. A clinician-administered interview that yields DSM-5-TR diagnoses

The BDI-II is a self-report severity measure, not a diagnostic interview.

C. A projective technique that uncovers depressive themes in children

It is an objective self-report inventory, and it is intended for adolescents and adults.

D. A nine-item primary care screen keyed to DSM depression criteria

That describes the PHQ-9; the BDI-II has 21 items and was developed separately from primary care screening.

Rationale: The BDI-II measures the severity of depressive symptoms in adolescents and adults, with each item rated 0 to 3. It is useful for screening and tracking change but does not by itself establish a diagnosis; a clinical interview is still needed.

References: Hays (Assessment) — Assessment of depression: Beck Depression Inventory-II

☐ Approve ☐ Revise ☐ Retire Notes:

On an MMPI-3 completed during a disability evaluation, the inconsistent-responding scales are within normal limits, but the infrequent-responding (overreporting) scales are markedly elevated, as are many clinical scales. What is the MOST appropriate interpretation?

A. Random responding, so the client probably could not read the items

Normal inconsistency scales argue against random or content-unrelated responding.

B. Defensive underreporting, so true problems are likely even more severe

Underreporting is shown by the underreporting scales, not by elevated infrequent-responding scales.

C. A valid profile, so the elevations confirm severe psychopathology

Marked elevations on overreporting scales call the validity of substantive scale scores into question.

D. Possible overreporting, so the clinical scales need cautious reading ✓ KEY

Elevated infrequent-responding scales with consistent responding suggest symptom overreporting, which limits face-value interpretation.

Rationale: MMPI-3 validity scales fall into three groups: inconsistent responding (CRIN, VRIN, TRIN), overreporting (F, Fp, Fs, FBS, RBS), and underreporting (L, K). The validity scales are reviewed before any substantive scale. When overreporting indicators are markedly elevated, substantive elevations may reflect exaggeration rather than true symptoms, particularly in settings with external incentives.

References: Hays (Assessment) — MMPI-3 validity scales: inconsistency, overreporting, underreporting

► **Check:** Verify the MMPI-3 validity scale names and groupings listed in the rationale against the current manual.

☐ Approve ☐ Revise ☐ Retire Notes:

A counselor uses the SCID-5 at intake. It follows a set order of questions keyed to DSM-5 criteria but lets the counselor ask follow-up questions and use clinical judgment in rating each criterion. How is this interview format BEST classified?

A. Fully structured, read word for word with no clinical judgment

Fully structured interviews (often designed for lay interviewers) leave no room for follow-up probes or clinician judgment.

B. Unstructured, with topics guided mainly by the client's story

Unstructured interviews have no set question sequence, unlike the SCID-5.

C. Semi-structured, a set guide with room for clinician probes ✓ **KEY**

The SCID-5 is a semi-structured interview: standard questions plus clinician follow-up and judgment.

D. A mental status exam, since it records observed presentation

An MSE describes current presentation; the SCID-5 is a diagnostic interview organized by DSM criteria.

Rationale: Structured interviews maximize reliability and coverage but limit flexibility; unstructured interviews maximize flexibility and rapport but are less reliable and can miss areas. Semi-structured interviews such as the SCID-5 combine a standard framework with clinician probes, balancing reliability and clinical depth.

References: Hays (Assessment) — Clinical interviews: structured, semi-structured, and unstructured

☐ Approve ☐ Revise ☐ Retire Notes:

Asked how she has been feeling, a 40-year-old client says, "Honestly, I'm good, really good." Throughout the interview she is tearful, her voice trembles, and her facial expression changes little. How should the counselor document this in the mental status examination?

A. Mood as the observed tears; affect as her statement of "good"

This reverses the terms: mood is self-reported, affect is observed.

B. Mood reported as "good"; affect tearful and mood-incongruent ✓ **KEY**

Mood is the client's stated emotional state; affect is the counselor's observation, here incongruent with the stated mood.

C. Thought content as denial; thought process as tearful and flat

Tearfulness is an affect finding, and thought process describes how ideas connect, not emotional expression.

D. Insight as intact, because she can describe how she is feeling

Her report conflicts with her presentation, so this does not show intact insight into her condition.

Rationale: In the MSE, mood is the sustained emotion the client reports (often quoted), and affect is the observed expression of emotion, described by range, intensity, stability, and congruence with mood and content. Thought process is how thoughts connect; thought content is what the client thinks about; insight and judgment are rated separately.

References: Hays (Assessment) — Mental status examination: mood and affect

☐ Approve ☐ Revise ☐ Retire Notes:

A 9-year-old is referred for inattention and angry outbursts. On a behavior rating scale, the parent's ratings are highly elevated, the teacher's ratings are in the average range, and the child's self-report is low. How should the counselor interpret these results?

A. Informant differences are common and show where problems appear ✓ **KEY**

Discrepancies often reflect real differences across settings and relationships, and they guide further assessment.

B. The teacher's ratings are most objective, so set the parent's aside

No single informant is automatically correct; discarding the parent data loses information about home behavior.

C. Self-reports from children this age are invalid and add nothing

Children can provide useful information, especially about internal states, when measures fit their age.

D. The gap shows the parent is exaggerating, so assessment can end

The discrepancy is a reason to explore further, not evidence of exaggeration or a reason to stop.

Rationale: Assessment of children and adolescents relies on multiple informants (parents, teachers, the youth) and multiple methods, as in the Achenbach System of Empirically Based Assessment. Low cross-informant agreement is typical, because behavior varies by setting and each rater sees a different slice. Counselors integrate the reports with observation and history rather than choosing one.

References: Hays (Assessment) — Assessing children: multiple informants and settings

☐ Approve ☐ Revise ☐ Retire Notes:

For the past four weeks, a 33-year-old client has had depressed mood and two other depressive symptoms, causing clear impairment at work. There is no identifiable stressor, and no other mental disorder or medical cause explains the symptoms. The counselor wants the record to state why full criteria are not met. Which diagnosis BEST fits?

A. Unspecified depressive disorder, because no full criteria are met

Unspecified is used when the clinician chooses not to give the reason or lacks the information; here the counselor wants to state it.

B. Major depressive disorder, single episode, mild in severity

A major depressive episode requires at least five symptoms; this client has three.

C. Adjustment disorder with depressed mood, given the impairment

Adjustment disorder requires an identifiable stressor, which is absent.

D. Other specified depressive disorder, noting the specific reason ✓ **KEY**

Other specified is used when the clinician records the reason the presentation falls short, such as insufficient symptoms.

Rationale: DSM-5-TR offers two options for presentations that cause distress or impairment but do not meet full criteria for a disorder in a class. "Other specified" is used when the clinician chooses to communicate the specific reason (for example, a depressive episode with insufficient symptoms). "Unspecified" is used when the clinician chooses not to specify the reason, including when there is not enough information, as in an emergency setting.

References: DSM-5-TR — Use of other specified and unspecified disorders; Other Specified Depressive Disorder

☐ Approve ☐ Revise ☐ Retire Notes:

A counselor is seeing a 38-year-old client who recently immigrated and describes her distress mainly in terms of family duty and spiritual concerns. The counselor wants a DSM-5-TR tool that explores the client's own explanation of the problem, cultural identity, sources of support, and expectations of help. Which tool should the counselor use?

A. Level 1 Cross-Cutting Symptom Measure

This measure screens symptom domains; it does not explore cultural explanations or help-seeking.

B. WHO Disability Assessment Schedule 2.0

The WHODAS 2.0 assesses functioning and disability, not cultural understanding of the problem.

C. Cultural Formulation Interview ✓ KEY

The CFI asks about the cultural definition of the problem, perceived causes, cultural identity, stressors and supports, and help-seeking.

D. Structured Clinical Interview (SCID-5)

The SCID-5 is a diagnostic interview organized by criteria, not a cultural assessment.

Rationale: The DSM-5-TR Cultural Formulation Interview is a person-centered, semi-structured set of questions in Section III. It covers the cultural definition of the problem, cultural perceptions of cause, context, and support, cultural factors affecting coping and help-seeking, and current help-seeking expectations. Informant and supplementary modules extend it.

References: DSM-5-TR — Section III: Cultural Formulation Interview · Sue & Sue (Counseling the Culturally Diverse) — Culturally sensitive assessment

☐ Approve ☐ Revise ☐ Retire Notes:

A 76-year-old client's daughter reports that he has trouble finding words and has missed paying several bills. On a brief cognitive screen such as the Montreal Cognitive Assessment (MoCA), he scores below the screening cutoff. What should the counselor do NEXT?

A. Diagnose a major neurocognitive disorder based on the score

A screening score cannot establish a neurocognitive diagnosis or its cause.

B. Refer for a full medical and neuropsychological evaluation ✓ KEY

A positive screen calls for comprehensive evaluation to clarify the cause, including reversible conditions.

C. Readminister the same screen next week to confirm the result

Repeating the screen delays evaluation and adds practice effects without clarifying the cause.

D. Reassure the family that these changes reflect normal aging

Word-finding problems and new difficulty with finances go beyond expected age-related change and need evaluation.

Rationale: Cognitive screens such as the MoCA or MMSE flag possible impairment but are influenced by education, language, hearing, vision, and mood. A positive screen in an older adult should prompt medical evaluation (to rule out causes such as medication effects, thyroid problems, delirium, or depression) and fuller neuropsychological testing before any diagnosis is made.

References: Hays (Assessment) — Assessment of older adults: cognitive screening · DSM-5-TR — Neurocognitive disorders: major and mild NCD

☐ Approve ☐ Revise ☐ Retire Notes:

An 8-year-old is referred for poor concentration, restlessness, and unfinished classwork. His teacher from last year describes him as attentive and settled. The problems began about five months ago, shortly after he witnessed a violent break-in at his home, and he now has nightmares and startles easily. Which finding MOST argues against ADHD as the primary explanation?

A. His restlessness is seen both at home and at school

Symptoms in two or more settings are required for ADHD, so this finding is consistent with ADHD rather than against it.

B. He is a boy referred in the early elementary years

Age and gender describe common referral patterns; they do not help separate ADHD from a trauma response.

C. The symptoms began suddenly after a traumatic event ✓ **KEY**

An abrupt onset after trauma, with no earlier history and with intrusion and arousal symptoms, points toward a trauma-related disorder.

D. His classwork is left unfinished, not done poorly

Failing to finish schoolwork is itself a listed ADHD inattention symptom, so it does not argue against ADHD.

Rationale: ADHD is a neurodevelopmental disorder with a persistent pattern that typically shows up early and across settings. When attention problems and restlessness appear abruptly after a traumatic event in a child who was previously attentive, and they come with nightmares and an exaggerated startle response, a trauma- and stressor-related disorder better explains the picture. Hyperarousal and intrusive memories can look like inattention and hyperactivity, so onset timing and trauma history are key differentiators.

References: DSM-5-TR — Attention-deficit/hyperactivity disorder: Differential Diagnosis · SAMHSA TIP 57 — Trauma-related symptoms that resemble other disorders

☐ Approve ☐ Revise ☐ Retire Notes:

A 35-year-old client reports three weeks of depressed mood, loss of interest, guilt, and poor concentration. His partner says he has been "snappy and irritable" with everyone. He sleeps about four hours a night but wakes exhausted and drags through the day. He reports no past periods of elevated mood or unusual energy. Which diagnosis is BEST supported?

A. Major depressive disorder, single episode ✓ **KEY**

He has at least five depressive symptoms for two weeks or more; irritability and insomnia with fatigue fit a depressive episode.

B. Bipolar II disorder, current episode depressed

Bipolar II requires at least one past hypomanic episode, and he reports no periods of elevated mood or increased energy.

C. Cyclothymic disorder with depressive features

Cyclothymic disorder requires at least two years of fluctuating subthreshold symptoms, not a three-week episode.

D. Disruptive mood dysregulation disorder

DMDD is diagnosed only in children and adolescents (first diagnosis between ages 6 and 18), with onset before age 10.

Rationale: Irritability is common in depression and does not by itself suggest bipolarity. Hypomania requires a distinct period of abnormally elevated, expansive, or irritable mood together with increased activity or energy. This client sleeps little but feels exhausted, which is insomnia, not the decreased need for sleep seen in hypomania. With no history of hypomanic episodes, the depressive episode is best diagnosed as major depressive disorder.

References: DSM-5-TR — Major depressive disorder: Differential Diagnosis; hypomanic episode Criteria A-B

☐ Approve ☐ Revise ☐ Retire Notes:

A 42-year-old client in a severe depressive episode believes she is being punished for terrible sins and that her organs are rotting. Her family reports that these beliefs appeared only after the depression began, and she has never had psychotic symptoms while her mood was stable. Which diagnosis BEST fits?

A. Schizophrenia, first episode, acute

Psychosis that occurs only during mood episodes points to a mood disorder; schizophrenia is excluded when mood episodes explain it.

B. Schizoaffective disorder, depressive type

Schizoaffective disorder requires delusions or hallucinations for two weeks or more without a major mood episode.

C. Delusional disorder, somatic type

Delusional disorder is not diagnosed when delusions occur only during major depressive or manic episodes.

D. MDD, severe, with psychotic features ✓ KEY

Delusions confined to a major depressive episode are coded with the psychotic features specifier.

Rationale: The deciding question is when the psychotic symptoms occur. If delusions or hallucinations happen only during a major depressive episode, the diagnosis is major depressive disorder with psychotic features; here the content (guilt, punishment, bodily decay) is mood-congruent. Schizoaffective disorder requires a period of at least two weeks of psychosis without a major mood episode, and schizophrenia is ruled out when mood episodes account for the psychosis.

References: DSM-5-TR — Depressive disorders: with psychotic features specifier; schizoaffective disorder Criterion B

☐ Approve ☐ Revise ☐ Retire Notes:

A 24-year-old is brought in after four days of paranoid beliefs and hearing voices. He reports using methamphetamine daily for the past month and stopping two days ago. He has no prior psychiatric history. Which approach to diagnosis is MOST appropriate?

A. Diagnose schizophrenia, as he has delusions and hallucinations

Schizophrenia requires continuous signs for six months and is excluded when a substance explains the symptoms.

B. Consider a substance-induced psychosis and see if it clears ✓ KEY

Onset during heavy stimulant use suggests a substance-induced disorder; persistence well after stopping would point elsewhere.

C. Diagnose brief psychotic disorder, as it has lasted under a month

Brief psychotic disorder is not diagnosed when the disturbance is attributable to a substance.

D. Postpone any psychosis assessment until a 90-day program ends

Psychosis needs prompt assessment for safety and care; waiting months is not a diagnostic strategy.

Rationale: Stimulants such as methamphetamine can produce paranoia and hallucinations. DSM-5-TR asks whether the symptoms began during or soon after substance use and whether they persist for a substantial period (for example, about a month) after intoxication or withdrawal ends. Symptoms that clear with abstinence support a substance/medication-induced psychotic disorder; symptoms that persist, or a history of psychosis without use, suggest an independent psychotic disorder.

References: DSM-5-TR — Substance/medication-induced psychotic disorder: Criterion C and Differential Diagnosis

☐ Approve ☐ Revise ☐ Retire Notes:

A counselor gives a client a form with stems such as "I regret..." and "My father..." and asks her to finish each one in her own words. Which type of instrument is this?

A. A sentence completion test such as the Rotter ✓ KEY

Incomplete sentence stems that the client finishes freely define the sentence completion method, such as the Rotter Incomplete Sentences Blank.

B. An objective personality inventory like the MMPI-3

The MMPI-3 uses fixed true/false items scored against scales, not open-ended responses.

C. A word association technique in the style of Jung

Word association asks for the first word that comes to mind after a single stimulus word, not completing sentences.

D. A symptom checklist such as the SCL-90-R measure

The SCL-90-R asks clients to rate how much listed symptoms have bothered them, a structured self-report format.

Rationale: Sentence completion tests are often called semi-structured projective techniques: the stem gives some structure, but the client supplies the content, which may reveal attitudes, conflicts, and concerns. The Rotter Incomplete Sentences Blank is the best-known example. Like other projectives, responses are interpreted with caution and alongside other data.

References: Hays (Assessment) — Projective personality assessment: sentence completion techniques

☐ Approve ☐ Revise ☐ Retire Notes:

As part of an evaluation for possible depression, a school counselor asks a 9-year-old to draw a house, a tree, and a person. Afterward, a colleague proposes diagnosing depression because the tree in the drawing has no leaves. Which response BEST reflects sound practice with this technique?

A. Accept the sign, as drawing tests are well validated for mood

Individual drawing signs have weak and inconsistent research support for diagnosing specific disorders.

B. Rescore the drawing with a longer and more detailed checklist

A longer checklist does not fix the underlying validity problem of reading diagnoses from single drawing features.

C. Treat the drawing as a hypothesis to check against other data ✓ KEY

Projective drawings are best used for rapport and hypothesis generation, confirmed by interviews, ratings, and other measures.

D. Have the child draw again and diagnose if the sign is repeated

Repeating the task does not make the sign a valid indicator of depression; the diagnosis still lacks evidence.

Rationale: The House-Tree-Person technique, developed by Buck, asks the client to draw and then talk about the drawings. Interpreting single features (such as a bare tree) as signs of a disorder has limited reliability and validity support. Projectives are best used to build rapport and generate hypotheses that are then checked against interview data, behavior ratings, collateral reports, and standardized measures.

References: Hays (Assessment) — Projective drawings (House-Tree-Person): uses and psychometric limitations

☐ Approve ☐ Revise ☐ Retire Notes:

Over the past year, a client's drinking has met these DSM-5-TR criteria: drinking more than intended, unsuccessful efforts to cut down, craving, continued use despite arguments with his spouse about it, and tolerance. He reports no withdrawal symptoms. Which severity specifier applies?

A. Mild, as no withdrawal symptoms are reported

Withdrawal is not required for any severity level; severity is set by how many criteria are met.

B. Moderate, based on the number of criteria met ✓ KEY

Five criteria fall in the moderate range (4-5 criteria); mild is 2-3 and severe is 6 or more.

C. Severe, as tolerance shows physical dependence

Tolerance is one criterion among eleven; it does not by itself raise the severity level.

D. Unspecified, until withdrawal is also confirmed

An alcohol use disorder can be diagnosed and rated without withdrawal; at least two criteria in 12 months are needed.

Rationale: DSM-5-TR substance use disorders have 11 criteria; at least two within a 12-month period are needed for the diagnosis. Severity is rated by count: mild (2-3 symptoms), moderate (4-5), and severe (6 or more). Physiological features such as tolerance and withdrawal are not required, and neither one alone determines severity.

References: DSM-5-TR — Alcohol use disorder: Criteria A and severity specifiers

☐ Approve ☐ Revise ☐ Retire Notes:

In an integrated primary care clinic, a 44-year-old patient screens positive on the AUDIT-C. What does this result BEST support as the next step?

A. Diagnosing alcohol use disorder, mild severity

A screening tool flags possible risk; it cannot establish whether DSM-5-TR criteria are met.

B. Referring the patient to inpatient detoxification

Level of care depends on a full assessment, such as of withdrawal risk, not on a positive screen alone.

C. Placing the patient in a relapse prevention group

Treatment placement before any diagnostic assessment skips the step that tells what care is needed.

D. Fuller assessment to see if a disorder exists ✓ KEY

A positive screen calls for further evaluation, such as the full AUDIT and a diagnostic interview.

Rationale: The AUDIT-C is the three consumption items of the 10-item AUDIT. Like all screeners, it is built to identify people who may have a problem, trading some precision for speed. A positive result is followed by a fuller assessment (for example, the complete AUDIT, a clinical interview against DSM-5-TR criteria, and review of consequences) before diagnosis or placement.

References: Hays (Assessment) — Substance use screening (AUDIT, AUDIT-C) versus diagnostic assessment

☐ Approve ☐ Revise ☐ Retire Notes:

A counselor in a high school health center wants a brief substance use screening tool that was developed and validated specifically for adolescents. Which instrument BEST fits this need?

A. CRAFFT ✓ KEY

The CRAFFT was developed for adolescents and asks about situations such as riding in a car with an impaired driver.

B. CAGE questionnaire

The CAGE was developed for adults and focuses on lifetime alcohol problems, not adolescent use.

C. Michigan Alcoholism Screening Test

The MAST was developed with adults and screens for alcohol problems only.

D. Addiction Severity Index

The ASI is a longer adult interview measuring problem severity, not a brief adolescent screen.

Rationale: The CRAFFT is a brief screen for adolescent substance use risk. Its name comes from its key words: Car, Relax, Alone, Forget, Friends, Trouble. It covers alcohol and other drugs and fits settings such as schools and pediatric care. Like any screen, a positive result leads to further assessment rather than diagnosis.

References: Hays (Assessment) — Substance use screening with adolescents: CRAFFT

☐ Approve ☐ Revise ☐ Retire Notes:

A parent brings a 6-year-old for an intake because of delayed speech and difficulty playing with peers. Which part of the biopsychosocial history is MOST important to gather in detail at this point?

A. The parent's own job history and career plans

Parent employment can matter for context, but it does not address the child's developmental concerns.

B. The child's scores on recent reading tests

At age 6, formal reading data are limited and do not explain early speech and social delays.

C. Developmental milestones and early health ✓ KEY

Timing of speech, motor, and social milestones, plus prenatal and early medical history, is central to these concerns.

D. The family's faith practices and affiliation

Spiritual context can be relevant to a full history, but it is not the priority for speech and social delays.

Rationale: For young children, the developmental history is a core section of the biopsychosocial assessment. When to first words, phrases, walking, and social play emerged, and any prenatal, birth, hearing, or medical issues, help distinguish typical variation from conditions such as language disorder, hearing loss, or autism spectrum disorder, and guide referrals.

References: Hays (Assessment) — Clinical interview: developmental and biopsychosocial history · Berk (Development Through the Lifespan) — Language and social development in early childhood

☐ Approve ☐ Revise ☐ Retire Notes:

During a biopsychosocial intake for her first episode of depression, a 22-year-old mentions that her mother was hospitalized for bipolar disorder. Why is this family history clinically important?

A. It confirms that the client has bipolar disorder herself

Family history raises risk but cannot establish a diagnosis; the client's own history must meet criteria.

B. It raises concern for bipolarity, so screening is key ✓ **KEY**

A first-degree relative with bipolar disorder increases risk, so past hypomania and future mood shifts need careful screening.

C. It is not relevant once the client is an adult living alone

Family psychiatric history keeps its relevance to risk regardless of the client's age or living situation.

D. It shows that the client's depression is mostly learned

Family history reflects genetic and environmental influences; it does not show that symptoms were learned.

Rationale: Family psychiatric history is a standard part of the biopsychosocial assessment. Bipolar disorder is strongly heritable, so a depressed client with a first-degree relative who has bipolar disorder warrants careful questioning about past hypomanic or manic symptoms and monitoring over time. The information also matters for any referral for medication evaluation, because antidepressants alone may carry risk in bipolar disorder.

References: DSM-5-TR — Bipolar I disorder: Risk and Prognostic Factors (genetic and physiological) · Hays (Assessment) — Clinical interview: family history

☐ Approve ☐ Revise ☐ Retire Notes:

Near the end of a biopsychosocial interview, a counselor asks a 58-year-old client about his friendships, his faith community, times he coped well in the past, and things he does well. What is the main purpose of gathering this information?

A. To estimate whether he qualifies for disability benefits

Benefit eligibility depends on functional limits and outside rules, not on a strengths inventory.

B. To screen him for grandiosity and inflated self-esteem

Asking about strengths is not a mania screen; grandiosity is assessed through mood history and the MSE.

C. To set the DSM-5-TR severity specifier for his diagnosis

Severity specifiers depend on symptom criteria, not on the client's supports or strengths.

D. To find resources that the treatment plan can build on ✓ **KEY**

Strengths, supports, and past coping successes are resources that inform goals and interventions.

Rationale: A complete biopsychosocial history covers not only problems and risks but also strengths, social supports, spiritual resources, and past coping. These protective factors shape a realistic, collaborative treatment plan, can support safety planning, and convey to the client that the counselor sees the whole person rather than a list of deficits.

References: Wiger (Documentation) — Biopsychosocial assessment: strengths and resources

☐ Approve ☐ Revise ☐ Retire Notes:

When structuring an initial intake interview, which sequence is generally recommended for exploring the client's concerns?

A. Begin with closed questions on the most sensitive topics

Starting with sensitive, closed questions before rapport tends to limit disclosure and raise anxiety.

B. Ask about trauma history before the presenting problem

The client's reason for coming usually comes first; detailed trauma questions follow once rapport is built.

C. Start with open questions, then narrow to specifics ✓ **KEY**

This funnel approach lets the client tell the story in their own words, then fills gaps with targeted questions.

D. Read a full symptom checklist before any open question

Leading with a checklist structures the interview around the form rather than the client's concerns.

Rationale: Most intake models use a funnel: after informed consent, the counselor invites the client to describe the presenting concern in their own words with open-ended questions, then narrows to specific questions about onset, duration, severity, and history. This builds rapport, captures the client's view of the problem, and still yields the detail needed for diagnosis and planning.

References: Hays (Assessment) — Clinical interviewing: structure and sequencing of the intake · Gladding (Counseling: A Comprehensive Profession) — The initial interview

☐ Approve ☐ Revise ☐ Retire Notes:

An agency is deciding between a fully structured diagnostic interview and an unstructured clinical interview for its intakes. Which statement accurately compares the two approaches?

A. Structured interviews yield higher interrater reliability ✓ **KEY**

Fixed wording and order reduce variation between interviewers, which raises agreement on diagnoses.

B. Unstructured interviews are less open to clinician bias

Unstructured interviews depend more on clinician judgment, which makes them more open to bias.

C. Structured interviews let clinicians freely vary questions

Freedom to vary questions describes unstructured or semi-structured interviews, not fully structured ones.

D. Unstructured interviews are too unreliable to be of use

Unstructured interviews offer flexibility and rapport and remain widely used, despite lower reliability.

Rationale: Fully structured interviews ask set questions in a set order with set scoring, so different interviewers reach more similar conclusions (higher reliability). The tradeoff is less flexibility to follow the client's lead. Unstructured interviews offer flexibility and rapport but rely heavily on the clinician, which lowers reliability and leaves more room for bias. Semi-structured interviews fall between the two.

References: Hays (Assessment) — Clinical interviews: structured, semi-structured and unstructured formats

☐ Approve ☐ Revise ☐ Retire Notes:

During a mental status examination, a client correctly gives her name and says she is in a hospital, but she cannot state the month, the year, or the day of the week. How should orientation be recorded?

A. Fully oriented in all three spheres

She cannot give the date, so she is not oriented to time.

B. Oriented to person and place, not time ✓ **KEY**

She knows who she is and where she is but not the date, so she is oriented to person and place only.

C. Oriented to time and place, not person

She correctly gave her own name, so orientation to person is intact.

D. Disoriented in all three spheres

She is oriented to both person and place, so she is not fully disoriented.

Rationale: Orientation in the MSE is usually checked for person, place, and time (sometimes also situation) and recorded as, for example, "oriented x2." Orientation to time is typically the first to be lost in delirium and dementia, while orientation to person is usually the last. Any disorientation warrants attention to possible medical or neurocognitive causes.

References: Hays (Assessment) — Mental status examination: sensorium and orientation

☐ Approve ☐ Revise ☐ Retire Notes:

A 39-year-old client says, "I know my drinking is out of control, and it's hurting my kids." Later in the session she mentions that she drove her children to school that morning after drinking. How are insight and judgment BEST described in the mental status examination?

A. Insight is poor, but judgment is good

She recognizes the problem and its effects, so insight is not poor; driving after drinking is not good judgment.

B. Insight and judgment are both intact

Driving her children after drinking shows impaired judgment, despite her awareness.

C. Insight is absent, and judgment is poor

She names the problem and its harm to her children, so insight is present, not absent.

D. Insight fair, but judgment impaired ✓ **KEY**

She understands the problem (insight) but made a dangerous decision (judgment), so the two are rated differently.

Rationale: In the MSE, insight is the client's awareness that a problem exists and understanding of its causes and effects. Judgment is the ability to make sound decisions and anticipate consequences, best inferred from real recent behavior. The two can diverge: a client may describe a problem accurately yet still act in risky ways, as here.

References: Hays (Assessment) — Mental status examination: insight and judgment

☐ Approve ☐ Revise ☐ Retire Notes:

A 36-year-old client reports a plan to overdose this weekend on medication she has been saving and says she intends to act on it. She adds that her two children "are the reason I'm still here." How should the counselor weigh her children as a protective factor?

A. Note it, but treat risk as high and act on safety now ✓ KEY

Protective factors matter, but a plan with intent and means signals high acute risk that needs immediate action.

B. View it as reducing her risk to low; plan to meet next week

A single protective factor does not offset a specific plan, stated intent, and access to means.

C. Have her sign a no-harm contract, then end the session

No-suicide contracts are not supported as protection and do not address the plan or her access to means.

D. Set it aside, since protective factors do not change risk

Protective factors are part of a sound formulation and can be used in safety planning; they are not ignored.

Rationale: Suicide risk formulation weighs risk factors, warning signs, and protective factors together, but protective factors do not cancel out acute warning signs. Ideation with a specific plan, stated intent, and available means indicates high acute risk and calls for immediate safety steps, such as restricting means, involving supports, and arranging a higher level of care as needed. Her reasons for living can be built into the safety plan.

References: C-SSRS — Suicidal ideation with specific plan and intent · Stanley-Brown SPI — Reasons for living and means restriction

☐ Approve ☐ Revise ☐ Retire Notes:

A counselor is assessing an adult client's risk of harming others. Which factor is generally considered the strongest single predictor of future violent behavior?

A. Having any diagnosed mental disorder

Most people with mental disorders are not violent; diagnosis alone is a weak predictor.

B. A score below average on an IQ test

Intelligence test scores are not a strong or direct predictor of violence.

C. A history of past violent behavior ✓ KEY

Past violence is the most consistently supported predictor of future violence.

D. Living alone, away from family members

Living alone is not an established predictor of violence toward others.

Rationale: In violence risk assessment, past behavior is the best predictor of future behavior: a history of violence, especially recent, frequent, or severe violence, carries the most weight. Other factors that raise risk include substance use, stated threats or intent, access to weapons, and certain active symptoms, such as command hallucinations or paranoid beliefs about a specific person.

References: Hays (Assessment) — Risk assessment: violence and harm to others

☐ Approve ☐ Revise ☐ Retire Notes:

A 20-year-old has had delusions, hallucinations, and disorganized speech for two months. The symptoms continue, and it is not yet known whether he will recover within six months. Which diagnosis is MOST appropriate now?

A. Schizophrenia, first episode

Schizophrenia requires continuous signs of disturbance for at least six months.

B. Brief psychotic disorder

Brief psychotic disorder lasts at least one day but less than one month.

C. Schizoaffective disorder

No major mood episode is described, which schizoaffective disorder requires.

D. Schizophreniform disorder (provisional) ✓ KEY

Symptoms of one to six months with recovery not yet known call for a provisional schizophreniform diagnosis.

Rationale: DSM-5-TR uses "provisional" when there is a strong presumption that full criteria will be met but not enough information is yet available. Schizophreniform disorder lasts at least one month but less than six. When the diagnosis must be made before recovery is known, it is labeled provisional; if symptoms reach six months, the diagnosis changes to schizophrenia.

References: DSM-5-TR — Schizophreniform disorder: Diagnostic Criteria B; provisional diagnosis (Use of the Manual)

☐ Approve ☐ Revise ☐ Retire Notes:

A counselor on a school evaluation team is documenting autism spectrum disorder for a 7-year-old using DSM-5-TR. How are the severity levels for this diagnosis assigned?

A. As a single overall level based on the child's IQ score

Intellectual impairment is recorded with its own specifier; it does not set autism severity.

B. Separately for social communication and restricted behavior ✓ KEY

Each of the two core domains receives its own level, from requiring support to requiring very substantial support.

C. By the total count of criteria the child meets across domains

Autism severity reflects the support needed, not a criteria count as in substance use disorders.

D. By how early in life the symptoms were first noticed by parents

Age of recognition is part of the history, but it does not determine the severity level.

Rationale: For autism spectrum disorder, DSM-5-TR rates current severity separately for social communication and for restricted, repetitive behaviors, using Level 1 ("requiring support"), Level 2 ("requiring substantial support"), and Level 3 ("requiring very substantial support"). The levels can differ between domains and can change over time. Accompanying intellectual or language impairment is recorded with separate specifiers.

References: DSM-5-TR — Autism spectrum disorder: severity levels and specifiers

☐ Approve ☐ Revise ☐ Retire Notes:

A clinic wants a single individually administered intelligence test it can use with clients ranging from a 4-year-old to a 70-year-old. Which instrument covers this full age range?

A. Wechsler Intelligence Scale for Children-V

The WISC-V is normed for school-age children (about 6 to 16), not preschoolers or older adults.

B. Wechsler Preschool and Primary Scale-IV

The WPPSI-IV covers young children (about 2½ to 7), not adults.

C. Stanford-Binet Intelligence Scales (SB5) ✓ KEY

The SB5 is normed from age 2 through 85 and older, covering both clients.

D. Wechsler Adult Intelligence Scale-IV

The WAIS-IV starts at age 16, so it cannot be used with a 4-year-old.

Rationale: The Stanford-Binet Intelligence Scales, Fifth Edition (SB5), spans roughly ages 2 to 85+ and measures five factors (fluid reasoning, knowledge, quantitative reasoning, visual-spatial processing, and working memory) in both verbal and nonverbal domains. The Wechsler scales instead use separate tests by age band: WPPSI for preschoolers, WISC for school-age children, and WAIS for older adolescents and adults.

References: Hays (Assessment) — Intelligence testing: Stanford-Binet Fifth Edition and the Wechsler scales

► **Check:** Age ranges stated approximately (SB5 2-85+, WISC-V 6-16, WPPSI-IV 2:6-7:7, WAIS-IV 16-90); a WAIS-5 has since been published with a similar adult range. SME to confirm editions.

☐ Approve ☐ Revise ☐ Retire Notes:

A 10-year-old's WISC-V Full Scale IQ is 112, while her reading standard score on the Woodcock-Johnson IV Tests of Achievement is 78. Both scales have a mean of 100 and a standard deviation of 15. What has this pattern traditionally been taken to suggest?

A. A possible specific learning disability in reading ✓ KEY

A large gap between average-or-better ability and low achievement is the classic ability-achievement discrepancy.

B. Mild intellectual disability that affects reading

An IQ of 112 is above average, which rules out intellectual disability.

C. Invalid testing, as the two scores should match

Ability and achievement measure different things; a gap is meaningful, not proof of invalid testing.

D. Above-average reading skill for a child of her age

A standard score of 78 is about 1.5 SD below the mean, well below average.

Rationale: The ability-achievement discrepancy model compares an intelligence score (such as WISC-V FSIQ) with an achievement score (such as on the WIAT or Woodcock-Johnson). A gap of more than two standard deviations, as here (34 points), was long used to identify a specific learning disability. IDEA 2004 no longer requires schools to use a discrepancy and permits response-to-intervention approaches, and DSM-5-TR does not require a discrepancy.

References: Hays (Assessment) — Achievement testing: ability-achievement discrepancy and learning disabilities

☐ Approve ☐ Revise ☐ Retire Notes:

A counselor wants to use an English-language anxiety inventory with Vietnamese-speaking clients and asks a bilingual staff member to translate it word for word. What is the MOST important limitation of this plan?

A. Any translated test loses its reliability completely

Translation does not erase reliability by definition; it must be re-examined in the new version.

B. Translation alone does not ensure equivalence or norms ✓ KEY

Items may change meaning across languages, and English-speaking norms may not apply to the new group.

C. Clients must take it in English to compare to the norms

Testing in a client's weaker language threatens validity; it is not the solution.

D. Anxiety cannot be assessed across different cultures

Anxiety can be assessed cross-culturally with suitably adapted and validated measures.

Rationale: Culturally fair assessment requires more than literal translation. Adapted tests need linguistic, conceptual, and metric equivalence, usually established through processes such as back-translation and review by bilingual experts, followed by evidence of reliability and validity with the new population. Scores should also be interpreted against appropriate norms; ACA Code E.8 calls for caution when using instruments normed on other populations.

References: ACA Code of Ethics — E.8 Multicultural Issues/Diversity in Assessment · Hays (Assessment) — Culturally fair assessment: test translation and equivalence

☐ Approve ☐ Revise ☐ Retire Notes:

A 30-year-old client of Puerto Rican heritage describes an episode at her father's funeral in which she screamed uncontrollably, trembled, felt heat rising in her chest, and briefly felt detached, then felt like herself again. Which approach BEST reflects culturally fair assessment?

A. Diagnose panic disorder based on the physical symptoms she reports

One episode tied to a stressful event does not meet panic disorder criteria, and cultural meaning is not explored.

B. Record it as a dissociative disorder due to the brief detachment

Brief detachment within a culturally recognized distress episode does not establish a dissociative disorder.

C. Note it as a psychotic episode given the loss of control described

Nothing in the episode suggests delusions or hallucinations; loss of control is not psychosis.

D. Explore it as a possible cultural idiom of distress before labeling ✓ KEY

The pattern matches ataque de nervios, a cultural concept of distress, so its meaning should be explored first.

Rationale: DSM-5-TR describes ataque de nervios as a cultural concept of distress common among people of Latin American, and especially Caribbean, heritage. It often follows a stressful family event and can include uncontrollable shouting, crying, trembling, heat rising to the head or chest, and brief dissociation, with a quick return to usual functioning. Exploring the client's own explanation, for example with the Cultural Formulation Interview, guards against misdiagnosing culturally shaped distress.

References: DSM-5-TR — Culture and Psychiatric Diagnosis: cultural concepts of distress (ataque de nervios) · Sue & Sue (Counseling the Culturally Diverse) — Culturally appropriate assessment and diagnosis

☐ Approve ☐ Revise ☐ Retire Notes:

A parent is reading her son's score report from a statewide mathematics test. Which statement on the report is a norm-referenced interpretation?

A. He answered 34 of the 40 test items correctly

A raw score of items correct makes no comparison with other test takers, so it is not norm-referenced.

B. He scored higher than 70% of same-grade students ✓ **KEY**

Comparing his score with a norm group of same-grade students is the defining feature of a norm-referenced interpretation.

C. He met the state proficiency standard in math

Meeting a fixed proficiency standard compares him with a criterion, not with other students.

D. He mastered 9 of the 10 fraction objectives tested

Mastery of stated objectives is a criterion-referenced interpretation of what he can do.

Rationale: A norm-referenced interpretation describes a score by its position relative to a norm group, as with percentile ranks and standard scores. A criterion-referenced interpretation describes performance against a fixed standard or set of objectives, regardless of how others performed. The same test can support both kinds of statement, so the question is what the score is being compared with.

References: Hays (Assessment) — Norm-referenced and criterion-referenced score interpretation

☐ Approve ☐ Revise ☐ Retire Notes:

A counselor educator is building a criterion-referenced mastery test for a crisis-intervention training module. After instruction, 96% of trainees answer one item correctly. The item covers an essential safety skill named in the module objectives. What should the developer do with this item?

A. Drop it, because an item this easy fails to spread scores apart

Spreading scores apart is a norm-referenced goal; a mastery test is not built to rank trainees.

B. Rewrite it so that about half of all the trainees will miss it

Targeting a difficulty near .50 maximizes variance for norm-referenced tests, not for mastery testing.

C. Replace it with an item that better separates the top trainees

Separating high from low scorers serves ranking; it would drop coverage of a required objective.

D. Keep it, since it measures a required objective most mastered ✓ **KEY**

A criterion-referenced item is judged by its match to the objective; high success after instruction is the desired result.

Rationale: Norm-referenced tests favor items of moderate difficulty that discriminate among examinees, because their purpose is to rank people. Criterion-referenced tests sample the content domain defined by the objectives, so an item tied to an essential skill stays in the test even when nearly everyone passes it after good instruction. Low score variance is expected, and even welcome, on a mastery test.

References: Hays (Assessment) — Criterion-referenced test construction; item difficulty and discrimination

☐ Approve ☐ Revise ☐ Retire Notes:

A counselor plans to use a nonverbal ability test whose manual reports norms that were gathered about 30 years ago. When comparing today's clients with that norm group, what is the MAIN interpretive concern?

A. Scores may run high, since average performance has risen over time ✓ **KEY**

Population gains in ability-test performance mean old norms set a lower bar, inflating current standard scores.

B. Old norms pose no problem when internal consistency is still high

Reliability says nothing about whether the norm group still represents the people being tested.

C. Scores may run low, since older norm groups were more test-savvy

This reverses the documented direction; outdated ability norms tend to overstate current standing.

D. Outdated norms alter raw scores but leave percentile ranks intact

Percentile ranks are computed from the norm group, so they are exactly what outdated norms distort.

Rationale: A norm-referenced score is only as meaningful as the norm group behind it. On ability tests, average performance in many populations rose over the 20th century (the Flynn effect), so clients compared with an old norm group tend to receive inflated standard scores and percentiles. Counselors should check the age, size, and makeup of the norm sample and prefer recently normed editions.

References: Hays (Assessment) — Evaluating norm groups: recency, representativeness, and size

► **Check:** Flynn effect framing: gains have slowed or reversed in some countries in recent decades. SME to confirm the "may run high" wording is appropriately hedged.

☐ Approve ☐ Revise ☐ Retire Notes:

A college counseling center uses a regression equation to predict first-year GPA from an admissions test score. A counselor wants a statistic that shows how far students' actual GPAs typically fall from their predicted GPAs. Which statistic should she use?

A. The standard error of measurement of the admissions test

The SEM describes how obtained test scores vary around a person's true score, not prediction error for a criterion.

B. The standard error of the mean for the sample of GPAs

The standard error of the mean describes how precisely a sample mean estimates a population mean.

C. The standard error of estimate for the regression line ✓ **KEY**

The standard error of estimate is the typical spread of actual criterion scores around predicted scores.

D. The standard deviation of the admissions test scores

This describes how spread out the predictor scores are, not how accurate the GPA predictions are.

Rationale: Three standard errors are easy to confuse. The standard error of measurement concerns a single test and the error around an individual's true score; the standard error of estimate concerns prediction and the error around a predicted criterion score; the standard error of the mean concerns sampling and the error in a sample mean as an estimate of the population mean. Predicting GPA from a test calls for the standard error of estimate.

References: Hays (Assessment) — Standard error of measurement vs standard error of estimate · Erford (Research and Evaluation in Counseling) — Standard error of the mean; regression and prediction error

☐ Approve ☐ Revise ☐ Retire Notes:

Two anxiety scales use the same score metric and have the same standard deviation. Scale X has a reliability coefficient of .95, and Scale Y has one of .70. Why will Scale X have the smaller standard error of measurement?

A. Higher reliability increases the spread of the obtained scale scores

The two scales have the same standard deviation; reliability does not enlarge score spread.

B. Less of its score variance is error, so scores sit nearer true scores ✓ **KEY**

SEM = SD times the square root of (1 - r); as r rises, the error portion and the SEM both shrink.

C. Reliable scales have more items, and the SEM is divided by item count

The SEM formula uses the SD and reliability coefficient; there is no division by the number of items.

D. Higher reliability means a stronger correlation with a criterion

Correlation with a criterion is validity evidence, and it is not what determines the SEM.

Rationale: The standard error of measurement is computed as SD multiplied by the square root of (1 minus the reliability coefficient). Reliability is the share of observed-score variance that is true-score variance, so the remainder is error. With equal standard deviations, the scale with .95 reliability leaves only 5% error variance versus 30%, giving a smaller SEM and narrower confidence bands around an individual's score.

References: Hays (Assessment) — Standard error of measurement and its relation to reliability

☐ Approve ☐ Revise ☐ Retire Notes:

On a test battery where each subtest has a standard error of measurement of 4, a 12-year-old earns 103 on Verbal Reasoning and 109 on Spatial Reasoning. His parent asks whether spatial reasoning is a real strength. Using bands of plus or minus 1 SEM, what should the counselor say?

A. The bands overlap, so the gap may reflect measurement error ✓ **KEY**

The bands run 99-107 and 105-113; overlap means the true scores may not differ.

B. The 6-point gap exceeds one SEM, so it is a genuine strength

Both scores carry error, so a gap slightly larger than one SEM is not enough to call the difference real.

C. The gap can be judged only after converting both to percentiles

Changing the score metric does not remove measurement error from either score.

D. Error bands apply to composite scores, not to subtest scores

Every score, including a subtest score, has measurement error and can be reported with a band.

Rationale: Reporting scores as bands rather than points reminds everyone that each obtained score contains error. A common rule of thumb is that when two scores' bands overlap, the difference should not be treated as meaningful; when they do not overlap, a real difference is more likely. A more precise approach uses the standard error of the difference, which is larger than either SEM because both scores contribute error.

References: Hays (Assessment) — Confidence bands; comparing two scores with the standard error of measurement

☐ Approve ☐ Revise ☐ Retire Notes:

A 27-year-old graduate student completes the NEO-PI-3 as part of career counseling. Her Extraversion score is very low, and her Neuroticism score is average. She asks whether this means something is wrong with her. What is the BEST interpretation to offer?

A. It suggests social anxiety that should be assessed further

Low Extraversion is not a measure of anxiety, and her Neuroticism score, which taps distress, is average.

B. It shows emotional instability and a tendency toward distress

Emotional instability and proneness to distress describe high Neuroticism, not low Extraversion.

C. It points to difficulty cooperating with and trusting others

Trust and cooperativeness belong to the Agreeableness domain, not Extraversion.

D. It reflects a reserved style and a liking for quieter settings ✓ KEY

Low Extraversion describes a reserved, independent person who prefers less stimulation; it is a normal trait.

Rationale: The NEO-PI-3 measures normal-range personality on the five-factor model. Low Extraversion (introversion) means a person tends to be reserved, prefers smaller groups or solitude, and seeks less stimulation; it does not mean unhappiness or anxiety. Distress proneness is captured by Neuroticism. Interpreting traits as normal variation, and relating them to the client's goals, is the appropriate use of the profile.

References: Hays (Assessment) — NEO Personality Inventory-3: five-factor domains and interpretation

☐ Approve ☐ Revise ☐ Retire Notes:

A 34-year-old client took the Myers-Briggs Type Indicator (MBTI) at work two years ago and was typed ENFP. Taking it again in counseling, she is typed INFP. Her Extraversion-Introversion preference score was close to the midpoint both times. What BEST explains the change?

A. Her basic personality has shifted a great deal over the past two years

A small move across the midpoint does not show major personality change.

B. The second result is invalid, so the first type is her true type

Nothing suggests an invalid administration; neither type is more "true" when the preference is unclear.

C. Splitting a continuous score into two types can flip near the middle ✓ KEY

The MBTI sorts scores into either/or categories, so small shifts near the cut point change the reported type.

D. A change in type like this shows that she is in a period of crisis

The MBTI is not a measure of distress, and a type change near the midpoint has no clinical meaning.

Rationale: The MBTI reports four dichotomous preferences that combine into 16 types, but the underlying preference scores are continuous. People whose scores fall near the dividing point can be assigned a different type on retest, which is a well-known limit of type measures. The MBTI is used for self-understanding and career exploration; it is not designed for diagnosis or for selection decisions.

References: Hays (Assessment) — Myers-Briggs Type Indicator: type scoring and limitations · Sharf (Career Development Theory) — Use of the MBTI in career counseling

☐ Approve ☐ Revise ☐ Retire Notes:

A counselor uses the Sixteen Personality Factor Questionnaire (16PF) in premarital counseling. Which description of this instrument is accurate?

A. A normal-range personality measure built through factor analysis ✓ KEY

Raymond Cattell derived the 16PF's primary factors through factor analysis of trait descriptors.

B. An inventory empirically keyed to identify psychiatric patient groups

Empirical keying against clinical groups describes the MMPI tradition, not the 16PF.

C. A projective task in which clients describe ambiguous inkblots

Describing inkblots is the Rorschach, a projective test; the 16PF is a self-report questionnaire.

D. A type indicator that sorts people into Jungian categories

Sorting people into Jungian types describes the MBTI, not the 16PF's trait dimensions.

Rationale: The 16PF, developed by Raymond Cattell, is an objective self-report inventory of normal adult personality. Its 16 primary factors were identified through factor analysis and are summarized in broader global factors that resemble the Big Five. Because it describes normal traits rather than psychopathology, it is often used in career, couples, and personal growth counseling.

References: Hays (Assessment) — Sixteen Personality Factor Questionnaire (16PF): primary and global factors

☐ Approve ☐ Revise ☐ Retire Notes:

During intake, a 45-year-old client mentions that she saw a counselor at another agency for depression three years ago. The counselor would like to review those records. What is the BEST way to obtain them?

A. Phone the former counselor and ask for a verbal case summary

An informal call without the client's knowledge or authorization bypasses her control over her information.

B. Have her sign a release naming the agency and records sought ✓ KEY

A written, informed release specifying the source and the information requested respects her rights.

C. Ask her to describe that treatment and skip the records entirely

Her recollection is useful, but prior records add diagnoses, dates, and responses she may not recall.

D. Request the full file, including all notes, as routine practice

Requesting everything by default ignores the client's consent and the principle of seeking only what is needed.

Rationale: Prior treatment records can clarify past diagnoses, treatments tried, and response to care. The counselor should explain why the records would help and obtain the client's written authorization that names the releasing agency, the recipient, the information sought, and its purpose. Limiting the request to what is relevant respects privacy, and some material, such as psychotherapy notes, carries extra protection.

References: ACA Code of Ethics — B.6.g Disclosure or Transfer · HIPAA — 45 CFR 164.508 Uses and disclosures requiring an authorization

► **Check:** HIPAA permits provider-to-provider disclosure for treatment without authorization (psychotherapy notes and Part 2 records excepted); the key is framed as best practice. SME to confirm the HIPAA citation fits.

☐ Approve ☐ Revise ☐ Retire Notes:

A client on probation asks her counselor to send attendance and progress information to her probation officer. The release she signs and dates names the counselor, the probation officer, the information to be sent, the purpose, and the required notices on revocation and redisclosure. Under HIPAA, which required element is still missing?

A. A notary's seal that verifies the client's signature

HIPAA does not require notarization for an authorization to be valid.

B. The diagnosis that supports her referral to treatment

A diagnosis is not a required element; the form must describe the information, not justify the treatment.

C. A second signature from a witness at the counseling agency

A witness signature is not one of the required core elements of a valid authorization.

D. An expiration date or event that ends the authorization ✓ **KEY**

A valid authorization must state when it expires, either as a date or as a defined event.

Rationale: Under the HIPAA Privacy Rule, a valid authorization includes a specific description of the information, who may disclose it, who may receive it, the purpose, an expiration date or event, the client's signature and date, and statements about the right to revoke and the potential for redisclosure. A release missing its expiration is defective, so the counselor should have the client complete it before sending anything.

References: HIPAA — 45 CFR 164.508(c) Core elements and required statements of a valid authorization

☐ Approve ☐ Revise ☐ Retire Notes:

A counselor is assessing a 74-year-old widower for possible cognitive decline. He says he manages fine. His daughter, who visits three times a week, describes missed medications and getting lost while driving. His son, who lives overseas and speaks with him by phone monthly, says he "sounds like his old self." How should the counselor weigh these reports?

A. Average the two accounts, since each child knows him well

Averaging ignores how differently placed the two informants are to observe his daily functioning.

B. Defer to his own report, since he is a competent adult client

People with cognitive decline often lack awareness of their deficits, so self-report alone is unreliable here.

C. Weigh the daughter's direct observations and seek testing ✓ **KEY**

Her frequent, specific, firsthand observations carry more weight, and objective testing can corroborate them.

D. Discount the daughter's report, as caregiver strain biases it

Possible bias is worth noting, but it does not erase concrete, firsthand observations of safety problems.

Rationale: When collateral reports conflict, counselors weigh each informant's opportunity to observe, how recent and specific the observations are, and any reasons for bias. Brief phone calls can mask deficits that show up in daily tasks such as managing medications or driving. Rather than picking a side, the counselor uses the conflict to guide further assessment, such as cognitive screening and a medical evaluation.

References: Hays (Assessment) — Collateral sources: evaluating informant reports · DSM-5-TR — Major and mild neurocognitive disorders: Criterion A (concern of the individual, a knowledgeable informant, or the clinician)

☐ Approve ☐ Revise ☐ Retire Notes:

A counselor reviewing a psychological report notes that the Rorschach was formally scored, while the Thematic Apperception Test (TAT) was interpreted. Which statement BEST compares how these two projective tests are typically scored?

A. The Rorschach has normed coding systems such as Exner's and R-PAS; TAT stories are usually read clinically ✓ **KEY**

The Rorschach is commonly coded with normed systems, while the TAT is most often interpreted without a single standard system.

B. The TAT is coded with Exner's Comprehensive System; the Rorschach is read only through clinical intuition

This reverses the tests: Exner's Comprehensive System was developed for the Rorschach, not the TAT.

C. Both are scored by computer against national norms, so examiner training matters relatively little

Projective tests require extensive examiner training, and the TAT lacks a single computerized normed scoring.

D. Neither test has any formal scoring system, so both rest entirely on the examiner's personal judgment

The Rorschach has detailed, research-based coding systems, so it does not rest on judgment alone.

Rationale: Exner's Comprehensive System brought standardized administration, coding, and norms to the Rorschach, and the Rorschach Performance Assessment System (R-PAS) later built on that work. The TAT has several research scoring approaches, but in practice it is usually interpreted qualitatively by examining themes, needs, and relationships in the stories. This difference affects how much weight each result can bear in an assessment.

References: Hays (Assessment) — Projective personality assessment: Rorschach scoring systems and the TAT

► **Check:** Formal TAT scoring systems exist (e.g., SCORS, need-achievement coding) but are not uniformly used; SME to confirm the "usually read clinically" framing.

☐ Approve ☐ Revise ☐ Retire Notes:

A middle school principal asks the school counselor to "give some tests" to a seventh grader who is "having problems." What should the counselor do FIRST?

A. Give a broad battery so that no possible problem is overlooked

Testing without a focus wastes time, burdens the student, and may produce results no one can use.

B. Choose the instruments the school already owns and knows well

Convenience should not drive test choice; the instrument must fit the question being asked.

C. Clarify the specific question the assessment needs to answer ✓ **KEY**

A clear referral question determines what to assess and which instruments are appropriate.

D. Begin with a personality inventory to screen for any disorder

Picking an instrument before knowing the concern puts the tool ahead of the purpose.

Rationale: Assessment begins by defining its purpose. A vague request such as "having problems" must be turned into a specific referral question, for example about attention, mood, learning, or behavior, before any instrument is chosen. The question then guides test selection, the information sources needed (including parent consent and input), and how results will be used.

References: Hays (Assessment) — The assessment process: clarifying the referral question; selecting instruments

☐ Approve ☐ Revise ☐ Retire Notes:

A vocational rehabilitation agency asks a counselor whether a 30-year-old client meets criteria for intellectual developmental disorder. Which assessment plan BEST answers this referral question?

A. An individual intelligence test alone, since IQ defines the disorder

DSM-5-TR requires deficits in adaptive functioning as well; an IQ score alone cannot establish the diagnosis.

B. An individual intelligence test plus a standardized adaptive measure ✓ **KEY**

The diagnosis requires both intellectual and adaptive deficits, so both must be measured directly.

C. A group achievement test plus a brief cognitive screening instrument

Group achievement tests and brief screens are not adequate for a diagnostic decision about intellect.

D. A personality inventory plus an interest inventory for job matching

These may help with vocational planning, but they do not address intellectual or adaptive functioning.

Rationale: Intellectual developmental disorder requires deficits in intellectual functions, confirmed by clinical assessment and individualized standardized testing, together with deficits in adaptive functioning, with onset during the developmental period. Severity levels are defined by adaptive functioning. A sound plan therefore pairs an individually administered intelligence test with a standardized adaptive behavior measure and a developmental history.

References: DSM-5-TR — Intellectual developmental disorder: Criteria A-C and severity levels · Hays (Assessment) — Selecting instruments to fit the referral question

☐ Approve ☐ Revise ☐ Retire Notes:

A test manual reports a reliability coefficient of .80 for a self-esteem scale. Which interpretation of this value is correct?

A. About 64% of the variance is true variance, found by squaring .80

Squaring applies to a correlation between two different measures; a reliability coefficient is read directly.

B. About 80% of the people who take the scale receive accurate scores

Reliability describes score variance in a group, not the share of individuals scored accurately.

C. The scale correlates .80 with an outside self-esteem criterion

A correlation with an outside criterion is a validity coefficient, not a reliability coefficient.

D. About 80% of observed score variance reflects true-score differences ✓ **KEY**

A reliability coefficient estimates the proportion of observed-score variance that is true-score variance.

Rationale: In classical test theory, a reliability coefficient is interpreted directly as the proportion of observed-score variance attributable to true-score differences. A coefficient of .80 means about 80% true variance and 20% error variance. Unlike a validity coefficient, it is not squared, because it already represents a ratio of variances.

References: Hays (Assessment) — Interpreting reliability coefficients: true and error variance · Erford (Research and Evaluation in Counseling) — Classical test theory and reliability

☐ Approve ☐ Revise ☐ Retire Notes:

An aptitude test correlates .35 with supervisors' later job performance ratings. A colleague tells the counselor the test is worthless because this figure is so far below the test's .90 reliability. Which response is MOST accurate?

A. Modest validity coefficients are common and can still aid prediction ✓ KEY

Validity coefficients are typically much lower than reliability coefficients and can still improve decisions.

B. The test explains about 35% of the variance in job performance

Shared variance is the coefficient squared, so .35 explains roughly 12% of criterion variance.

C. A validity coefficient must equal the reliability to be of any use

Reliability sets an upper limit on validity; validity is not expected to reach it.

D. The coefficient shows that the true reliability of the test is .35

A test-criterion correlation is validity evidence; it does not re-estimate reliability.

Rationale: A validity coefficient is the correlation between test scores and a criterion. Because criteria such as supervisor ratings are themselves imperfect and many factors affect performance, validity coefficients are usually far lower than reliability coefficients. Squaring .35 gives about .12, so the test accounts for roughly 12% of criterion variance, which can still add useful information when combined with other data.

References: Hays (Assessment) — Validity coefficients: interpretation and typical size · Erford (Research and Evaluation in Counseling) — Coefficient of determination (r squared)

☐ Approve ☐ Revise ☐ Retire Notes:

Two cultural groups differ in average scores on a clerical aptitude test. Research shows the test predicts later job performance equally well for both groups, using the same regression line. A hiring manager concludes that the test is biased. What is the BEST response?

A. Any gap in group means is itself proof that a test is biased

Group mean differences can reflect real differences in opportunity or preparation, not only test bias.

B. A mean gap alone does not show bias; equal prediction argues against it ✓ KEY

Bias is a technical question; equal prediction across groups is evidence against predictive bias.

C. Equal prediction proves the test is fair in every possible use and setting

Fairness is broader than bias and includes access, consequences, and how scores are used.

D. Bias can be judged only by a panel reviewing items for offense

Content review helps, but statistical evidence such as differential prediction is central to judging bias.

Rationale: Test bias is a psychometric property: systematic error that makes a test measure or predict differently for different groups, studied through item functioning and differential prediction. Test fairness is a broader judgment about equitable treatment, access, and the consequences of test use. A mean difference alone does not establish bias, yet a technically unbiased test can still be used unfairly, so both questions deserve attention.

References: Hays (Assessment) — Test bias and test fairness; differential prediction · Sue & Sue (Counseling the Culturally Diverse) — Cultural bias in assessment

☐ Approve ☐ Revise ☐ Retire Notes:

A client with low vision takes a timed, standardized reading-comprehension test in large print with extended time as an accommodation for her disability. How should the counselor handle the results?

A. Score it with the usual norms and make no note of the changes

Leaving out the accommodations would hide a departure from standard conditions that affects interpretation.

B. Discard the results, since accommodated scores are unusable

Accommodated results can still be informative; the code calls for noting conditions, not discarding results.

C. Note the changed conditions in the report and interpret with care ✓ **KEY**

Documenting nonstandard conditions and qualifying interpretation is the ethically required approach.

D. Lower her score to offset the advantage given to her by the extra time

Ad hoc score adjustments have no psychometric basis and would distort the results further.

Rationale: Accommodations for disability are often necessary so that a test measures the intended skill rather than the disability. Because norms were built under standard conditions, the counselor notes the accommodations in the interpretation and treats the comparison with norms cautiously; results may be designated as of questionable validity when conditions differ substantially. Some publishers provide guidance on which accommodations preserve comparability.

References: ACA Code of Ethics — E.7.a Administration Conditions · Hays (Assessment) — Testing accommodations for individuals with disabilities

☐ Approve ☐ Revise ☐ Retire Notes:

To save session time, a counselor wants clients to complete a symptom inventory online at home before intake. The publisher validated the inventory only for proctored administration in an office. Under the ACA Code of Ethics, what should the counselor do?

A. Allow home use, since online forms are equal to paper forms

Equivalence across formats and settings must be shown; it cannot be assumed.

B. Allow home use once the client signs an informed consent form

Consent does not make an instrument valid for a use it was not designed or validated for.

C. Allow home use but double the time limit to offset household distractions

Changing the time limit adds another departure from standard conditions rather than solving one.

D. Have clients take it under supervision, as it was validated that way ✓ **KEY**

Instruments not designed and validated for self-administration should not be used unsupervised.

Rationale: The ACA Code of Ethics directs counselors not to permit unsupervised use of an assessment unless it was designed, intended, and validated for self-administration or scoring. Counselors using technology to administer tests must also ensure the platform functions properly and gives accurate results. Convenience does not justify a use that the validity evidence does not support.

References: ACA Code of Ethics — E.7.d Unsupervised Assessments; E.7.c Technological Administration

► **Check:** Confirm ACA 2014 section numbers E.7.c (Technological Administration) and E.7.d (Unsupervised Assessments).

☐ Approve ☐ Revise ☐ Retire Notes:

A counselor is reviewing a personality inventory profile with a 50-year-old client, who looks worried when she sees that one scale is elevated. Which approach to communicating the results is BEST?

A. Explain the scale in plain terms and ask how it fits her life ✓ KEY

Clear language and inviting her reactions make the results understandable and collaborative.

B. Read the computer narrative aloud so she hears the full report

Automated narratives are generic and technical; the counselor is responsible for interpretation.

C. Skip the elevated scale so that she will not feel more upset

Withholding results disregards her right to know and leaves her worry unaddressed.

D. Hand her the profile to study at home and talk next session

Leaving her alone with an elevated score and no explanation invites misunderstanding.

Rationale: Clients have the right to know their results and the basis for the counselor's conclusions. Good interpretation uses everyday language, presents scores as estimates and one source of information, links findings to the client's goals, and invites the client to compare the results with her own experience. This turns the profile into a shared working tool rather than a verdict.

References: ACA Code of Ethics — E.1.b Client Welfare · Hays (Assessment) — Communicating assessment results to clients

☐ Approve ☐ Revise ☐ Retire Notes:

A counselor at a veterans' clinic gives a client the PTSD Checklist for DSM-5 (PCL-5) at intake and again every few weeks during treatment. Which description of the PCL-5 is accurate?

A. A structured clinician interview that is the standard for diagnosis

This describes the Clinician-Administered PTSD Scale for DSM-5 (CAPS-5), not the PCL-5.

B. A self-report symptom measure used to screen and to track change ✓ KEY

The PCL-5 is a brief self-report of DSM-5 PTSD symptoms used for screening and monitoring.

C. A checklist of the traumatic events a client has been exposed to

Listing exposure to events is the job of a trauma-exposure checklist, not this symptom measure.

D. A projective measure that uncovers unconscious trauma conflicts

The PCL-5 is an objective self-report scale with rated items, not a projective technique.

Rationale: The PCL-5 is a 20-item self-report measure in which clients rate how much each DSM-5 PTSD symptom has bothered them in the past month. It is widely used to screen, to support a provisional diagnosis, and to monitor change across treatment as part of measurement-based care. A positive result should be followed by a full clinical assessment rather than treated as a diagnosis on its own.

References: VA/DoD CPG — PTSD guideline: screening and measurement-based care with the PCL-5 · DSM-5-TR — Posttraumatic stress disorder: Criteria B-E

☐ Approve ☐ Revise ☐ Retire Notes:

In early sessions with a couple, a counselor draws a diagram of three generations of both partners' families, marking marriages, divorces, deaths, illnesses, substance use, and close or cut-off relationships. What is the main assessment purpose of this tool?

A. To produce a standardized score for each partner's satisfaction

A genogram is a qualitative map, not a normed instrument that yields scores.

B. To decide which partner is mainly to blame for the couple's ongoing conflict

Systemic assessment looks at patterns and context rather than assigning individual blame.

C. To screen each partner for a personality disorder before treatment

A genogram maps family structure and patterns; it is not a diagnostic screen.

D. To reveal family patterns across generations that shape current issues ✓ **KEY**

Genograms show repeating patterns, alliances, and cutoffs that may bear on present problems.

Rationale: The genogram, rooted in Bowen family systems theory, maps at least three generations of a family's structure, events, and relationships. As an assessment tool, it helps counselor and clients see recurring patterns, such as conflict, cutoffs, illness, or substance use, that may be carried into the current relationship. It is also a collaborative way to gather family history.

References: Goldenberg (Family Therapy) — Bowen family systems: the genogram as an assessment tool

☐ Approve ☐ Revise ☐ Retire Notes:

On the Strong Interest Inventory, a 26-year-old client's highest General Occupational Theme is Artistic. Within that theme, her Basic Interest Scale for Culinary Arts is high, while those for Writing and Mass Communication and for Visual Arts and Design are low. How should the counselor interpret this?

A. The low scales cancel the theme, so artistic work does not fit her

One high Basic Interest Scale shows a real artistic interest; the low scales only narrow where it lies.

B. The Basic Interest Scales reflect skills, and themes reflect interests

Both scales measure interests; self-rated skill confidence is assessed separately.

C. The theme is broad, and the scales show which artistic areas she favors ✓ **KEY**

Basic Interest Scales break each broad theme into specific areas of interest.

D. The Basic Interest Scales compare her with workers in each occupation

Comparison with satisfied workers in specific occupations is the role of the Occupational Scales.

Rationale: The Strong Interest Inventory reports results at several levels. The six General Occupational Themes follow Holland's RIASEC model and describe broad interest patterns; the Basic Interest Scales break those themes into more specific interest areas; the Occupational Scales compare the client's likes and dislikes with those of satisfied workers in particular occupations. Reading the Basic Interest Scales within a theme helps the counselor pinpoint which kinds of work fit best.

References: Sharf (Career Development Theory) — Strong Interest Inventory: General Occupational Themes, Basic Interest Scales, Occupational Scales · Hays (Assessment) — Career assessment: interest inventories

► **Check:** Confirm that Culinary Arts, Writing and Mass Communication, and Visual Arts and Design are Basic Interest Scales under the Artistic theme in the current Strong edition.

☐ Approve ☐ Revise ☐ Retire Notes:

A counselor is starting a first telehealth intake with a 29-year-old client who has joined the video call from a parked car. Before moving into the presenting concern, which step BEST belongs at the opening of this intake?

A. Ask the client to describe the main reason they are seeking counseling now

Exploring the presenting concern matters, but it follows the setup steps that make remote work safe.

B. Give a brief symptom checklist so the session can begin with numbers

A screening measure can be useful later; it does not address consent or safety for remote sessions.

C. Confirm where the client is right now and an emergency plan for the session ✓ KEY

Knowing the client's location and how to get help locally is part of informed consent and emergency planning for distance counseling.

D. Suggest the client wait to talk until they can return home to a private room

Privacy can be discussed collaboratively, but sending the client away skips the needed consent and safety steps.

Rationale: An intake opens with orienting tasks before clinical content: introductions, informed consent, limits of confidentiality, and, for distance counseling, verification of the client's identity and current location plus emergency procedures. Location matters because the counselor needs to know which local resources to contact if a crisis arises during the session. Only after this foundation does the interview move to the presenting problem and history.

References: ACA Code of Ethics — H.2.a Informed Consent and Disclosure (distance counseling, including emergency procedures); H.3 Client Verification

☐ Approve ☐ Revise ☐ Retire Notes:

Many intake forms ask the counselor to record the presenting problem, or chief complaint, as a short quotation in the client's own words. What is the main reason for this practice?

A. It preserves how the client sees the problem before any clinical framing ✓ KEY

A direct quote keeps the client's perspective and language on record, separate from the counselor's interpretation.

B. It lets the counselor skip the later questions about history and context

The quote opens the interview; history and context are still gathered afterward.

C. It is required so that a diagnosis can be billed to the client's insurer

Billing depends on a diagnosis and documentation of necessity, not on quoting the client.

D. It shows the counselor has accepted the client's view as the diagnosis

Recording the client's words does not mean adopting them as the clinical formulation.

Rationale: The presenting problem is usually recorded first and, where possible, in the client's own words. This documents what brought the client in from their point of view, which later helps the counselor check whether the treatment plan addresses the client's actual concern. The counselor's clinical impressions are recorded separately.

References: Wiger (Documentation) — Intake documentation: presenting problem in the client's own words

☐ Approve ☐ Revise ☐ Retire Notes:

During a mental status examination, a 31-year-old client with insomnia mentions that a few times, just as she was drifting off to sleep, she heard someone call her name and then realized no one was there. She reports no other unusual perceptions, and her thinking is organized. How should the counselor record this?

- A. As auditory hallucinations that meet the first criterion for schizophrenia
Perceptions that occur only while falling asleep are not counted as psychotic hallucinations.
- B. As a disorder of thought process, since the voice reflects loosened associations
Hearing a voice is a perceptual experience; thought process concerns how ideas are connected.
- C. As a delusion, since she believed for a moment that someone was in the room
A delusion is a fixed false belief; she corrected her impression immediately.
- D. As a hypnagogic experience, which is common and not in itself a sign of psychosis ✓ KEY
Brief perceptions at sleep onset are hypnagogic and fall within the range of normal experience.

Rationale: The perception section of the mental status examination notes hallucinations and illusions, but context matters. DSM-5-TR notes that hallucinations occurring while falling asleep (hypnagogic) or waking (hypnopompic) are within the range of normal experience. With organized thinking and no other perceptual disturbance, the counselor records the experience accurately without labeling it psychotic.

References: DSM-5-TR — Schizophrenia spectrum: key features — hallucinations; hypnagogic and hypnopompic experiences

☐ Approve ☐ Revise ☐ Retire Notes:

A 45-year-old client calmly tells the counselor that the evening news anchor has been sending him personal messages about a secret mission. In which part of the mental status examination does this belong?

- A. Perception
Perception covers hallucinations and illusions; he describes a belief about real broadcasts.
- B. Thought content ✓ KEY
Delusions and ideas of reference are recorded under thought content.
- C. Thought process
Thought process describes how ideas connect, such as tangential or logical.
- D. Orientation
Orientation covers awareness of person, place, and time.

Rationale: The mental status examination separates what a person thinks (thought content) from how the thoughts are organized (thought process). Beliefs that television or other media carry personal messages are delusions of reference and are recorded under thought content, along with obsessions, preoccupations, and suicidal or homicidal ideation.

References: Hays (Assessment) — Mental status examination: thought content vs thought process

☐ Approve ☐ Revise ☐ Retire Notes:

A counselor reviews intake notes on a 52-year-old client. Which piece of information is a WARNING SIGN of possible near-term suicide risk rather than a longer-standing risk factor?

A. A brother died by suicide when the client was in their early twenties

Family history of suicide is an enduring risk factor, not a sign of imminent risk.

B. The client recently began giving away prized items and saying goodbye ✓ **KEY**

Giving away possessions and saying goodbye are recognized warning signs that call for immediate assessment.

C. The client has lived with a painful chronic back condition for ten years

Chronic pain raises risk over time but is a background factor, not an acute warning sign.

D. The client made a suicide attempt by overdose about fifteen years ago

A past attempt is a strong long-term risk factor, but it is historical rather than a current warning sign.

Rationale: Risk factors such as prior attempts, family history, and chronic illness raise risk over a lifetime; warning signs signal that risk may be rising now. Warning signs include talking about wanting to die or being a burden, seeking means, withdrawing, giving away possessions, and saying goodbye. A warning sign calls for direct questioning about ideation, plan, intent, and access to means in the current session.

References: SAMHSA 988 Lifeline — Suicide warning signs · C-SSRS — Screening for current ideation, intent, plan, and recent behavior

☐ Approve ☐ Revise ☐ Retire Notes:

A 27-year-old warehouse worker, furious about being passed over for promotion, says of his supervisor, "One of these days he's going to get what's coming to him." He then changes the subject. What should the counselor do FIRST?

A. Ask directly what he means, including any plan, means, and past violence ✓ **KEY**

An ambiguous remark calls for direct assessment of intent, plan, access to weapons, and history before any decision.

B. Call the supervisor to warn him, since a target has now been identified

Warning is considered only after assessment shows serious and foreseeable danger under the governing law.

C. Follow the change of subject so the client does not feel accused or judged

Letting a possible threat pass unexamined leaves the counselor unable to judge the level of risk.

D. Schedule a personality inventory to measure his general level of hostility

A trait measure cannot answer the immediate question of whether he intends to harm a specific person.

Rationale: When a client makes an ambiguous statement about harming someone, the counselor's first job is to clarify it: whether there is intent, a plan, a means such as a weapon, an identified victim, and a history of violence. That assessment determines whether the situation reaches the threshold of serious and foreseeable harm. Duties to warn or protect, and how they are carried out, depend on that finding and on state law.

References: ACA Code of Ethics — B.2.a Serious and Foreseeable Harm and Legal Requirements · State law — Duty-to-warn and duty-to-protect statutes vary by jurisdiction

☐ Approve ☐ Revise ☐ Retire Notes:

A 41-year-old client in Minnesota has had major depressive episodes that began in late October and fully remitted by April in each of the last three years. There have been no depressive episodes at other times of year, and no clear yearly stressor explains the timing. Which specifier BEST fits?

A. With melancholic features

Melancholic features describe symptom quality, such as loss of pleasure and early waking, not timing.

B. With atypical features

Atypical features describe mood reactivity and related symptoms, not a link to time of year.

C. With peripartum onset

Peripartum onset applies to episodes during pregnancy or within four weeks after delivery.

D. With seasonal pattern ✓ KEY

Regular onset and remission at set times of year, with no nonseasonal episodes, fit this specifier.

Rationale: The seasonal pattern specifier applies to recurrent major depressive disorder (and to bipolar episodes) when onset and full remission occur at characteristic times of year. In the last two years there must be at least two episodes showing the seasonal relationship and no nonseasonal episodes, and seasonal episodes must substantially outnumber nonseasonal ones over the lifetime. Timing explained by an obvious seasonal stressor, such as yearly unemployment, does not count.

References: DSM-5-TR — Depressive disorders: specifier "with seasonal pattern"

☐ Approve ☐ Revise ☐ Retire Notes:

For 14 months, a 26-year-old has had depressed mood plus four other depressive symptoms lasting about three days, roughly once a month, unrelated to the menstrual cycle. She has never met criteria for another depressive or bipolar disorder. The counselor wants the diagnosis to state why full criteria are not met. Which diagnosis fits?

A. Unspecified depressive disorder, with the reason left unstated

The unspecified category is used when the clinician chooses not to state the reason or lacks the information.

B. Major depressive disorder, recurrent, with the provisional label

Provisional is for a strong expectation that full criteria will be met; three-day episodes cannot meet the two-week rule.

C. Other specified depressive disorder (recurrent brief depression) ✓ KEY

The other specified category lets the clinician name the specific reason, here recurrent brief depression.

D. Adjustment disorder with depressed mood, recurrent episodes type

Adjustment disorder requires an identifiable stressor and has no recurrent subtype.

Rationale: DSM-5-TR offers two residual categories in each chapter. "Other specified" is used when the clinician chooses to communicate the specific reason criteria are not met, by recording the name of the category followed by the reason; "unspecified" is used when the clinician chooses not to give a reason or lacks enough information. Recurrent brief depression, with full-symptom episodes of 2 to 13 days at least monthly for 12 months, is a listed example of other specified depressive disorder.

References: DSM-5-TR — Other specified depressive disorder (recurrent brief depression); use of other specified vs unspecified categories

☐ Approve ☐ Revise ☐ Retire Notes:

An early intervention team wants an individually administered measure of cognitive, language, and motor development for an 18-month-old born prematurely. Which instrument is designed for a child this age?

A. Bayley Infant and Toddler Scales ✓ KEY

The Bayley scales assess cognitive, language, and motor development from early infancy to about 42 months.

B. Wechsler Intelligence Scale (WISC-V)

The WISC-V is for children aged 6 through 16 and is far above this child's age.

C. Wechsler Preschool Scale (WPPSI-IV)

The WPPSI-IV begins at 2 years 6 months, so it does not reach an 18-month-old.

D. Peabody Picture Vocabulary Test-5

The PPVT measures receptive vocabulary only, not cognitive and motor development.

Rationale: Infant and toddler assessment uses developmental scales rather than conventional intelligence tests. The Bayley Scales of Infant and Toddler Development cover roughly the first three and a half years and assess cognitive, language, and motor domains, with social-emotional and adaptive behavior gathered by caregiver report. Scores in infancy are less predictive of later IQ than scores in school age, so results guide early intervention rather than fixed labels.

References: Hays (Assessment) — Assessment of infants and young children: Bayley Scales · Berk (Development Through the Lifespan) — Infant intelligence tests and their predictive value

☐ Approve ☐ Revise ☐ Retire Notes:

A counselor at an adult education program wants a brief, individually administered screen of basic academic skills, such as word reading, spelling, and math computation, for a 38-year-old returning learner. Which instrument fits this purpose?

A. Wechsler Adult Intelligence Scale-IV

The WAIS-IV measures intellectual ability, not acquired academic skills.

B. Raven's Progressive Matrices

Raven's measures nonverbal reasoning with visual patterns, not academic skills.

C. Vineland Adaptive Behavior Scales-3

The Vineland measures adaptive behavior in daily life, not reading or math skills.

D. Wide Range Achievement Test-5 ✓ KEY

The WRAT-5 is a brief achievement measure of word reading, comprehension, spelling, and math computation.

Rationale: Achievement tests measure what a person has learned, while intelligence and aptitude tests estimate ability or potential to learn. The Wide Range Achievement Test is a short, individually administered achievement screen covering reading, spelling, and math computation across a wide age range. A full intelligence test or an adaptive behavior scale answers a different referral question.

References: Hays (Assessment) — Achievement testing: individually administered screening batteries

☐ Approve ☐ Revise ☐ Retire Notes:

A counselor asks an 8-year-old to "draw a picture of everyone in your family, including you, doing something." Which projective technique is this?

A. House-Tree-Person

The House-Tree-Person asks for separate drawings of a house, a tree, and a person.

B. Kinetic Family Drawing ✓ KEY

The Kinetic Family Drawing asks for the whole family shown doing something, to reveal relationships and roles.

C. Bender-Gestalt Test

The Bender-Gestalt asks the person to copy geometric designs to assess visual-motor skill.

D. Draw-A-Person Test

The Draw-A-Person asks for a single human figure, not the family in action.

Rationale: In the Kinetic Family Drawing, the "kinetic" element is the instruction to show family members doing something. Clinicians look at who is included, relative size and placement, interaction, and activity as hypotheses about how the child experiences the family. Like other drawing techniques, it is best used to open conversation rather than to diagnose.

References: Hays (Assessment) — Projective drawing techniques: Kinetic Family Drawing

☐ Approve ☐ Revise ☐ Retire Notes:

A psychological report states that the Rorschach was administered and coded using the Rorschach Performance Assessment System (R-PAS), with the results compared against international reference norms. What does this description tell the counselor reading the report?

A. The client told stories about pictures that were scored for needs and press

Storytelling scored for needs and press describes Murray's Thematic Apperception Test.

B. The examiner read the inkblot responses freely with no set coding rules

R-PAS exists precisely to replace free interpretation with standardized coding.

C. The inkblots were given and coded with a standardized, normed system ✓ KEY

R-PAS specifies administration and coding rules and compares scores with reference norms.

D. A true-false self-report inventory was given in place of the inkblots

The report says the Rorschach itself was administered; R-PAS is a way of scoring it.

Rationale: The Rorschach has been used with several scoring approaches. Exner's Comprehensive System standardized administration and coding for decades, and R-PAS, developed later by researchers including Meyer and Mihura, updated the coding with standardized procedures and norm-referenced scores. These systems improve reliability compared with free interpretation, though the Rorschach remains a performance-based measure whose results are integrated with other data.

References: Hays (Assessment) — Projective and performance-based personality assessment: Rorschach scoring systems

☐ Approve ☐ Revise ☐ Retire Notes:

A 60-year-old client has taken a prescribed opioid exactly as directed for chronic back pain for two years. She needs a higher dose than at first for the same relief and feels ill if she misses a dose. She reports no other problems with the medication. How should these two findings be weighed toward opioid use disorder?

A. They are not counted, because she takes the drug as prescribed ✓ **KEY**

DSM-5-TR excludes tolerance and withdrawal from the count when an opioid is taken solely under medical supervision.

B. They count as two criteria, enough for a mild opioid use disorder

Two criteria would mean mild severity, but these two are not counted when use is as prescribed.

C. They count, but only once she has used the drug for over a year

Duration of use does not change the exclusion for appropriately prescribed medication.

D. They count as one criterion, since both reflect body adaptation

Tolerance and withdrawal are separate criteria; the issue is whether they count at all.

Rationale: Tolerance and withdrawal are expected physiological effects of long-term opioid treatment. DSM-5-TR therefore notes that these two criteria are not considered met for people taking opioids solely under appropriate medical supervision. A diagnosis would require other criteria, such as using more than prescribed, craving, or continued use despite harm.

References: DSM-5-TR — Opioid use disorder, Criteria 10 and 11 notes (tolerance and withdrawal under medical supervision)

☐ Approve ☐ Revise ☐ Retire Notes:

To get a detailed picture of a client's drinking, a counselor sits with him and a calendar of the past 90 days. Using paydays, holidays, and family events as memory anchors, they estimate how many drinks he had on each day. Which method is the counselor using?

A. The CAGE questionnaire

The CAGE is four yes-or-no screening questions, not a day-by-day record.

B. The AUDIT screening test

The AUDIT is a 10-item screen, not a calendar-based recall method.

C. Addiction Severity Index

The ASI is a structured interview rating problem severity across life areas.

D. The Timeline Followback ✓ **KEY**

The Timeline Followback uses a calendar and memory anchors to reconstruct daily use.

Rationale: The Timeline Followback is a calendar-based interview that reconstructs day-by-day substance use over a set window, often 30 to 90 days, using personal events to aid recall. It yields detailed data on frequency, quantity, and patterns such as weekend binges, and it can be repeated to track change. Screening tools such as the CAGE and AUDIT only flag possible problems.

References: Sobell & Sobell (Timeline Followback) — Calendar method with memory anchors for daily use estimates · Hays (Assessment) — Substance use assessment: structured and calendar-based methods

☐ Approve ☐ Revise ☐ Retire Notes:

A 17-year-old who arrived in the United States as a refugee eight months ago is referred for sleep problems and irritability. When taking the biopsychosocial history, which approach to the migration history is MOST complete?

A. Focus on current school stress, since past events are better left for later

Leaving out the migration history can miss trauma and losses that explain current symptoms.

B. Ask mainly about the war and danger that led the family to leave home

Pre-migration events matter, but stressors during flight and after arrival are also important.

C. Explore life before leaving, the journey itself, and life since arriving ✓ **KEY**

Covering pre-migration, migration, and post-migration phases captures the full range of stressors and supports.

D. Ask about legal status first, since it decides which services are offered

Opening with legal status can frighten the client and is not the core of a clinical history.

Rationale: Refugee and immigrant clients may face stressors in three phases: before leaving (violence, persecution, loss), during migration (danger, camps, separation from family), and after arrival (acculturation, discrimination, language barriers, poverty). A biopsychosocial history that covers all three, along with strengths and supports, gives a fuller picture than one focused on a single phase. Questions should be paced with care to avoid retraumatizing the client.

References: Sue & Sue (Counseling the Culturally Diverse) — Counseling refugees and immigrants: pre-migration, migration, and post-migration stressors

☐ Approve ☐ Revise ☐ Retire Notes:

A biopsychosocial intake with a 20-year-old college student shows that her mother has an anxiety disorder, panic attacks began after a car accident last month, she now skips classes held in the building where her first attack happened, and she is close to her older sister. Which finding is a PERPETUATING factor?

A. Her mother's long history of being treated for an anxiety disorder

Family history is a predisposing factor that raised vulnerability before the problem began.

B. Skipping classes held in the building where her first attack happened ✓ **KEY**

Avoidance keeps the fear from being disconfirmed, so it maintains the problem over time.

C. The car accident last month that came just before the first attack

An event that triggered onset is a precipitating factor, not one that maintains the problem.

D. Her close and supportive relationship with her older sister at home

A supportive relationship is a protective factor that can be drawn on in treatment.

Rationale: Biopsychosocial data are often organized into predisposing factors (vulnerabilities such as family history), precipitating factors (events that triggered onset), perpetuating factors (what keeps the problem going), and protective factors (strengths and supports). Perpetuating factors are especially useful for treatment planning because many can be changed; here, avoidance maintains panic and would be a target for exposure-based work.

References: Hays (Assessment) — Integrating intake and history data into a case conceptualization

☐ Approve ☐ Revise ☐ Retire Notes:

A client's score on a self-report scale converts to a z-score of -1.5. What is the equivalent T-score?

A. 15

This multiplies by 10 without adding the mean of 50.

B. 45

This treats -1.5 as half a standard deviation below the mean.

C. 65

This places the score 1.5 standard deviations above the mean.

D. 35 ✓ KEY

$T = 50 + 10(-1.5) = 50 - 15 = 35$.

Rationale: T-scores have a mean of 50 and a standard deviation of 10, so $T = 50 + 10z$. A z-score of -1.5 lies one and a half standard deviations below the mean, which is 15 T-score points below 50, giving a T-score of 35.

References: Hays (Assessment) — Standard scores: z-scores and T-scores

☐ Approve ☐ Revise ☐ Retire Notes:

A normally distributed test has a mean of 50 and a standard deviation of 15. A client earns a raw score of 65. Approximately what is the client's percentile rank?

A. About the 84th percentile ✓ KEY

The z-score is $(65 - 50) / 15 = +1.0$; about $50\% + 34\% = 84\%$ of scores fall below it.

B. About the 65th percentile

This confuses the raw score with the percentile rank.

C. About the 98th percentile

The 98th percentile corresponds to about $z = +2.0$, two standard deviations above the mean.

D. About the 68th percentile

About 68% of scores fall between -1 and +1 SD; that is not the percentile for $z = +1$.

Rationale: First convert to a z-score: $(65 - 50) / 15 = +1.0$. In a normal distribution, 50% of scores fall below the mean and about 34% fall between the mean and +1 standard deviation, so a z-score of +1.0 corresponds to roughly the 84th percentile. Confusing this with the 68% that falls within one standard deviation on both sides is a common error.

References: Hays (Assessment) — The normal curve: converting z-scores to percentile ranks

☐ Approve ☐ Revise ☐ Retire Notes:

A test has a standard deviation of 10 and a reliability coefficient of .91. A client obtains a score of 60. Within which range does the client's true score fall with about 68% confidence?

A. 54 to 66

This is roughly the 95% band, using about two SEMs on each side.

B. 51 to 69

This uses three SEMs on each side, far wider than a 68% band.

C. 57 to 63 ✓ KEY

SEM = 10 x the square root of .09 = 3, so 60 plus or minus 3 gives 57 to 63.

D. 59 to 61

This uses $10 \times .09 = 0.9$ and forgets the square root in the formula.

Rationale: The standard error of measurement is SD multiplied by the square root of $(1 - r)$: $10 \times \text{the square root of } .09 = 10 \times .3 = 3$. About 68% of the time the true score lies within one SEM of the obtained score (60 plus or minus 3, or 57 to 63), and about 95% of the time within roughly two SEMs (about 54 to 66). Reporting a band rather than a single score reminds clients that every obtained score contains error.

References: Hays (Assessment) — Standard error of measurement and confidence intervals

☐ Approve ☐ Revise ☐ Retire Notes:

In career counseling, a 44-year-old client completes a work values exercise in which she must rank 12 values from most to least important. Security ranks first and Prestige ranks last. Which interpretation is accurate?

A. She values security more strongly than most adults in a comparison norm group

A forced ranking has no norm group comparison, so it cannot show standing relative to others.

B. Security outranks her other values; it says nothing about other people ✓ KEY

Ranked (ipsative) results show the relative order of values within one person only.

C. She has met a set cutoff that marks mastery of career values clarity

A values ranking has no mastery criterion; it is not a criterion-referenced test.

D. Prestige is of no importance to her, as it was ranked in last place

Last place means least important of the 12, not unimportant; all values may matter to some degree.

Rationale: Ipsative measures, such as forced-choice or ranking formats, compare a person's preferences with one another, so the results show what matters most and least to that individual. Because every person's rankings sum to the same total, ipsative results cannot be used to compare one person's strength of a value with other people's, which requires normative scoring. They are useful for clarifying priorities in career decisions.

References: Hays (Assessment) — Ipsative vs normative score interpretation · Sharf (Career Development Theory) — Assessing work values in career counseling

☐ Approve ☐ Revise ☐ Retire Notes:

Seven months after her husband's death, a 66-year-old client reports intense yearning for him nearly every day, trouble accepting the death, and feeling that part of her has died. These reactions impair her daily functioning. Why would prolonged grief disorder not be diagnosed at this point?

A. For adults, at least 12 months must have passed since the death ✓ **KEY**

DSM-5-TR requires that at least 12 months have elapsed since the death for adults (6 months for children).

B. Yearning for the person who died is not one of its core symptoms

Intense yearning or preoccupation with the deceased is a core required symptom.

C. The diagnosis applies only when the death was sudden or violent

Prolonged grief disorder does not require a particular manner of death.

D. It is listed only in Section III as a condition for further study

In DSM-5-TR, prolonged grief disorder is an official diagnosis in Section II.

Rationale: DSM-5-TR added prolonged grief disorder to the trauma- and stressor-related disorders. For adults, at least 12 months must have passed since the death (at least 6 months for children and adolescents), with intense yearning or preoccupation plus additional symptoms such as identity disruption or marked emotional pain. Before that point, intense grief is usually understood as an expected response, though the counselor should still assess for a co-occurring major depressive episode.

References: DSM-5-TR — Prolonged grief disorder, Criterion A (time since death)

☐ Approve ☐ Revise ☐ Retire Notes:

In a primary care behavioral health program, a 35-year-old patient scores 12 on the GAD-7. How should the counselor interpret and use this result?

A. As severe anxiety that confirms a diagnosis of generalized anxiety disorder

A score of 12 falls in the moderate range, and no screening score confirms a diagnosis.

B. As minimal anxiety, since only scores above 20 call for any further review

The maximum score is 21; scores of 10 or more are a common cutoff for further evaluation.

C. As a diagnosis of panic disorder, since the GAD-7 measures panic frequency

The GAD-7 rates generalized anxiety symptoms; it does not count panic attacks.

D. As moderate anxiety that calls for a fuller diagnostic assessment to follow ✓ **KEY**

Scores of 10 to 14 are moderate, and a positive screen leads to a clinical diagnostic evaluation.

Rationale: The GAD-7 is a seven-item screen scored 0 to 21, with cutoffs of 5, 10, and 15 marking mild, moderate, and severe anxiety. A score of 10 or higher is commonly used to flag possible clinically significant anxiety. Like all screening instruments, it identifies who needs further assessment; a diagnosis rests on a clinical interview against DSM-5-TR criteria.

References: Hays (Assessment) — Brief screening instruments: GAD-7 scoring and interpretation

☐ Approve ☐ Revise ☐ Retire Notes:

A counselor in an adolescent treatment program wants an MMPI-family inventory for a 16-year-old client. Which version is normed for this age group?

A. MMPI-3

The MMPI-3 is normed for adults aged 18 and older.

B. MMPI-2

The MMPI-2 is an adult instrument for ages 18 and older.

C. MMPI-A-RF ✓ KEY

The MMPI-A-RF is the restructured form normed for adolescents aged 14 to 18.

D. MMPI-2-RF

The MMPI-2-RF is the restructured form for adults, not adolescents.

Rationale: The MMPI family includes separate adult and adolescent versions. The MMPI-2, MMPI-2-RF, and MMPI-3 are normed on adults, while the MMPI-A and its restructured form, the MMPI-A-RF, are normed for adolescents aged 14 to 18. Using an adult form with a 16-year-old would compare the client with the wrong norm group.

References: Hays (Assessment) — Minnesota Multiphasic Personality Inventory: adult and adolescent versions

☐ Approve ☐ Revise ☐ Retire Notes:

A counselor needs to reassess a client's reading comprehension three weeks after a first testing and worries that the client will remember specific passages and answers. The test publisher offers two versions built to the same specifications. Which kind of reliability evidence matters MOST for this plan?

A. Interrater reliability between two independent scorers

Interrater reliability concerns agreement between scorers, not equivalence of two versions.

B. Alternate-forms reliability between the two test versions ✓ KEY

Switching versions is sound only if the two forms are shown to yield equivalent scores.

C. Internal consistency of the items within a single version

Internal consistency shows items hang together but says nothing about whether forms match.

D. Test-retest reliability of the same version over 3 weeks

Reusing the same version invites the memory effects the counselor wants to avoid.

Rationale: Alternate-forms (parallel-forms) reliability is estimated by giving two equivalent versions of a test to the same people and correlating the scores. It is the relevant evidence when a counselor plans to retest with a different form to reduce practice and memory effects. Test-retest reliability uses the same form twice and is vulnerable to exactly those effects.

References: Hays (Assessment) — Reliability: alternate-forms (parallel-forms) reliability

☐ Approve ☐ Revise ☐ Retire Notes:

Areas of Clinical Focus

295 items · NCE weight 29% (47 of 160 scored items)

Q241 of 1000 · nce-s-clf-001 · Assessment and Testing · depressive disorders · easy · In review

Under DSM-5-TR, what is the minimum duration for the symptoms of a major depressive episode?

A. One week

One week is the minimum duration for a manic episode, not a major depressive episode.

B. Two weeks ✓ KEY

A major depressive episode requires five or more symptoms present during the same 2-week period.

C. One month

One month is not the major depressive episode threshold; it is used for other diagnoses, such as the active phase of schizophrenia.

D. Six months

Six months is the duration for generalized anxiety disorder, not a major depressive episode.

Rationale: DSM-5-TR requires at least five of nine symptoms, including depressed mood or loss of interest or pleasure, present nearly every day during the same 2-week period and representing a change from prior functioning. Knowing duration thresholds helps counselors tell a depressive episode apart from shorter, situational low mood.

References: DSM-5-TR — Major Depressive Disorder, Criterion A

☐ Approve ☐ Revise ☐ Retire Notes:

Q242 of 1000 · nce-s-clf-002 · Assessment and Testing · depressive disorders · medium · In review

A 41-year-old client reports feeling down more days than not for the past 3 years, with low energy, poor appetite, and hopelessness. She cannot recall a stretch of more than a few weeks without these symptoms and has never had a manic or hypomanic episode. Full criteria for a major depressive episode have not been met. Which diagnosis BEST fits?

A. Cyclothymic disorder

Cyclothymic disorder requires numerous periods of hypomanic symptoms alongside depressive symptoms; she reports no elevated mood.

B. Adjustment disorder with depressed mood

Adjustment disorder follows an identifiable stressor and does not persist more than 6 months after the stressor ends; this is a chronic 3-year pattern.

C. Disruptive mood dysregulation disorder

DMDD is a childhood diagnosis (first given between ages 6 and 18, onset before 10) marked by severe temper outbursts.

D. Persistent depressive disorder ✓ KEY

Depressed mood more days than not for at least 2 years in an adult, with two or more associated symptoms and no symptom-free period longer than 2 months, fits persistent depressive disorder.

Rationale: Persistent depressive disorder requires depressed mood for most of the day, more days than not, for at least 2 years in adults (1 year in children and adolescents), plus at least two associated symptoms such as appetite change, low energy, or hopelessness. During that period the person has not been without symptoms for more than 2 months at a time, and there has never been a manic or hypomanic episode.

References: DSM-5-TR — Persistent Depressive Disorder, Criteria A–C

☐ Approve ☐ Revise ☐ Retire Notes:

A 15-year-old has shown irritable mood on most days for the past 14 months, along with low self-esteem and poor concentration. He has not gone more than a few weeks without these symptoms, has no history of severe temper outbursts, and has never met criteria for a manic or hypomanic episode. Which statement is MOST accurate?

A. Criteria for persistent depressive disorder can be met at this point ✓ KEY

For children and adolescents, mood may be irritable and the duration requirement is 1 year, so 14 months is sufficient.

B. Persistent depressive disorder requires 2 full years in any age group

The 2-year requirement applies to adults; DSM-5-TR shortens it to 1 year for children and adolescents.

C. Irritable rather than sad mood rules out a depressive disorder diagnosis

DSM-5-TR allows irritable mood to satisfy the mood criterion for depressive disorders in children and adolescents.

D. Disruptive mood dysregulation disorder is the required diagnosis here

DMDD requires severe, recurrent temper outbursts with onset before age 10, which this teen does not show.

Rationale: DSM-5-TR modifies persistent depressive disorder for youth in two ways: mood can be irritable rather than depressed, and the minimum duration is 1 year instead of 2. Counselors working with adolescents should recognize that chronic irritability can be a depressive presentation rather than simply a behavior problem.

References: DSM-5-TR — Persistent Depressive Disorder, Criterion A (children and adolescents)

☐ Approve ☐ Revise ☐ Retire Notes:

Unless hospitalization is required, what is the minimum duration of a manic episode in DSM-5-TR?

A. Four consecutive days

Four consecutive days is the minimum for a hypomanic episode, not a manic episode.

B. Two weeks

Two weeks is the duration requirement for a major depressive episode.

C. One week ✓ KEY

A manic episode lasts at least 1 week and is present most of the day, nearly every day, or any duration if hospitalization is needed.

D. One month

One month is not a mood episode threshold; it is too long for mania.

Rationale: A manic episode is a distinct period of abnormally and persistently elevated, expansive, or irritable mood plus increased activity or energy lasting at least 1 week (any duration if hospitalization is necessary). A hypomanic episode needs only 4 consecutive days and does not cause marked impairment or require hospitalization.

References: DSM-5-TR — Bipolar I Disorder, Manic Episode Criterion A

☐ Approve ☐ Revise ☐ Retire Notes:

A 30-year-old client has had two major depressive episodes. Last spring she had 5 days of unusually elevated mood, less need for sleep, and increased productivity that friends noticed but that did not impair her work or require hospitalization. She has never had psychotic symptoms or a longer elevated period. Which diagnosis is MOST appropriate?

A. Bipolar II disorder ✓ KEY

At least one hypomanic episode (4+ days, no marked impairment) plus at least one major depressive episode, with no history of mania, defines bipolar II disorder.

B. Bipolar I disorder

Bipolar I requires at least one full manic episode; her elevated period was hypomanic.

C. Cyclothymic disorder

Cyclothymic disorder requires symptoms that never meet full criteria for a major depressive episode; she has had two.

D. Major depressive disorder

A history of a hypomanic episode excludes a diagnosis of major depressive disorder.

Rationale: Bipolar II disorder requires at least one hypomanic episode and at least one major depressive episode, with no lifetime manic episode. Hypomania lasts at least 4 consecutive days, is observable by others, and is not severe enough to cause marked impairment or hospitalization. Screening for past hypomania is essential before treating depression as unipolar.

References: DSM-5-TR — Bipolar II Disorder, Criteria A–B; Hypomanic Episode

☐ Approve ☐ Revise ☐ Retire Notes:

A 27-year-old with no prior psychiatric history develops grandiosity, pressured speech, almost no sleep, and reckless spending. On the third day he is psychiatrically hospitalized because his behavior has become dangerous. Substances and medical causes are ruled out. Which statement is MOST accurate?

A. The episode is hypomanic because it lasted fewer than 4 days

Hypomania itself requires at least 4 days and, by definition, does not require hospitalization.

B. No bipolar diagnosis applies until symptoms reach 1 full week

The 1-week requirement is waived when hospitalization is necessary.

C. Bipolar I also requires a prior major depressive episode on record

A single manic episode is sufficient for bipolar I; a depressive episode is common but not required.

D. The episode is manic since hospitalization waives the duration ✓ KEY

DSM-5-TR counts a manic episode of any duration if hospitalization is necessary, so bipolar I disorder can be diagnosed.

Rationale: The 1-week duration for a manic episode applies unless hospitalization is necessary, in which case any duration qualifies. Bipolar I disorder requires only one lifetime manic episode; major depressive episodes are common but are not required for the diagnosis.

References: DSM-5-TR — Bipolar I Disorder, Manic Episode Criterion A and Criterion for Bipolar I

☐ Approve ☐ Revise ☐ Retire Notes:

For a diagnosis of generalized anxiety disorder, excessive anxiety and worry must occur more days than not for at least how long?

A. 1 month

One month is the post-attack worry period for panic disorder, not the GAD duration.

B. 6 months ✓ KEY

GAD requires excessive anxiety and worry more days than not for at least 6 months.

C. 3 months

Three months is not the GAD threshold.

D. 2 years

Two years is the adult duration for persistent depressive disorder.

Rationale: Generalized anxiety disorder involves excessive, hard-to-control worry about a number of events or activities, occurring more days than not for at least 6 months, with three or more associated symptoms in adults (only one in children), such as restlessness, fatigue, or sleep disturbance.

References: DSM-5-TR — Generalized Anxiety Disorder, Criterion A

☐ Approve ☐ Revise ☐ Retire Notes:

A 36-year-old client has had several sudden, unexpected surges of intense fear with a pounding heart, shortness of breath, and dizziness. For the past 2 months he has worried constantly about having another attack and has stopped exercising because a racing heart might trigger one. Medical causes have been ruled out. Which diagnosis BEST fits?

A. Panic disorder ✓ KEY

Recurrent unexpected panic attacks followed by at least 1 month of persistent worry or maladaptive behavior change define panic disorder.

B. Agoraphobia

Agoraphobia requires fear or avoidance in at least two situations such as crowds or public transit; his avoidance centers on bodily sensations.

C. Illness anxiety disorder

Illness anxiety centers on a preoccupation with having a serious illness, not on recurrent unexpected panic attacks.

D. Specific phobia, situational

Specific phobia involves fear cued by a particular object or situation, whereas these attacks are unexpected.

Rationale: Panic disorder requires recurrent unexpected panic attacks, with at least one attack followed by 1 month or more of persistent concern about further attacks or their consequences, or a significant maladaptive change in behavior such as avoiding exercise. Interoceptive avoidance like this is a common target of cognitive behavioral treatment.

References: DSM-5-TR — Panic Disorder, Criteria A–B · Barlow PCT — Interoceptive avoidance and exposure

☐ Approve ☐ Revise ☐ Retire Notes:

Ten days after a serious highway collision, a 24-year-old reports intrusive memories, nightmares, feeling as if the world is unreal, avoiding driving, and trouble sleeping and concentrating. The symptoms began right after the crash and are impairing her work. Which diagnosis is MOST appropriate at this time?

A. Posttraumatic stress disorder

PTSD requires symptoms lasting more than 1 month; only 10 days have passed.

B. Adjustment disorder with anxiety

When symptoms meet criteria for a trauma-specific disorder, adjustment disorder is not the best fit.

C. Acute stress disorder ✓ KEY

Acute stress disorder applies when trauma-related symptoms last from 3 days to 1 month after exposure.

D. Generalized anxiety disorder

GAD involves diffuse worry for at least 6 months and is not tied to a traumatic event.

Rationale: Acute stress disorder is diagnosed when nine or more symptoms across intrusion, negative mood, dissociation, avoidance, and arousal last from 3 days to 1 month after trauma exposure. If symptoms persist beyond 1 month and meet PTSD criteria, the diagnosis is changed to PTSD.

References: DSM-5-TR — Acute Stress Disorder, Criteria B–C

☐ Approve ☐ Revise ☐ Retire Notes:

A firefighter developed some sleep problems and irritability soon after a fatal fire. Eight months later, after more symptoms emerged, she meets full criteria for PTSD for the first time. She does not report depersonalization or derealization. Which specifier applies?

A. With dissociative symptoms

The dissociative specifier requires persistent depersonalization or derealization, which she denies.

B. With panic attacks

The with-panic-attacks specifier requires panic attacks alongside the disorder, and none are described here.

C. With acute onset

DSM-5-TR PTSD has no acute-onset specifier; the DSM-IV acute and chronic specifiers were removed.

D. With delayed expression ✓ KEY

This specifier is used when full criteria are not met until at least 6 months after the event, even if some symptoms began earlier.

Rationale: In DSM-5-TR, PTSD "with delayed expression" is specified when full diagnostic criteria are not met until at least 6 months after the event, although the onset and expression of some symptoms may be immediate. The other PTSD specifier is "with dissociative symptoms" (depersonalization or derealization).

References: DSM-5-TR — Posttraumatic Stress Disorder, specifiers

☐ Approve ☐ Revise ☐ Retire Notes:

A client with contamination obsessions washes his hands for hours daily. He says his family will probably become seriously ill if he stops washing, although he admits he could be wrong. Which DSM-5-TR insight specifier is MOST accurate?

A. With good or fair insight

Good or fair insight means the person recognizes the beliefs are definitely or probably not true.

B. With poor insight ✓ KEY

Poor insight applies when the person thinks the OCD beliefs are probably true.

C. With absent insight

Absent insight/delusional beliefs applies when the person is completely convinced the beliefs are true; he admits doubt.

D. With tic-related features

The tic-related specifier concerns a current or past tic disorder, not the degree of insight.

Rationale: DSM-5-TR rates insight in OCD on three levels: good or fair (beliefs definitely or probably not true), poor (beliefs probably true), and absent insight/delusional beliefs (completely convinced the beliefs are true). A separate "tic-related" specifier notes a current or past tic disorder.

References: DSM-5-TR — Obsessive-Compulsive Disorder, insight specifiers

☐ Approve ☐ Revise ☐ Retire Notes:

Which psychological intervention has the strongest evidence as a first-line treatment for obsessive-compulsive disorder?

A. Insight-oriented psychodynamic therapy

Psychodynamic therapy lacks the evidence base that exposure-based CBT has for OCD.

B. Thought stopping with reassurance

Reassurance acts as a compulsion and maintains OCD; thought stopping tends to backfire.

C. Exposure and response prevention ✓ KEY

ERP, a form of CBT, exposes clients to feared cues while they refrain from compulsions and is the first-line psychological treatment.

D. Nondirective supportive counseling

Supportive counseling alone does not target the obsession–compulsion cycle.

Rationale: Exposure and response prevention has clients face feared thoughts, images, or situations while resisting compulsions, allowing anxiety to decline without the ritual and weakening the negative reinforcement that maintains OCD. Reassurance-giving by the counselor or family is itself a compulsion to be reduced.

References: NICE guidelines — CG31 Obsessive-compulsive disorder and body dysmorphic disorder: CBT including ERP

☐ Approve ☐ Revise ☐ Retire Notes:

Over the past 12 months, a client has met five of the eleven DSM-5-TR criteria for alcohol use disorder. Which severity specifier applies?

A. Mild

Mild applies when two or three criteria are met.

B. Severe

Severe applies when six or more criteria are met.

C. Moderate ✓ KEY

Moderate applies when four or five criteria are met.

D. Subthreshold

Two criteria within 12 months are enough for a diagnosis, so five is well above threshold.

Rationale: DSM-5-TR substance use disorders require at least two of eleven criteria within a 12-month period. Severity is specified as mild (2–3 criteria), moderate (4–5), or severe (6 or more). Severity guides level-of-care decisions alongside a dimensional assessment such as the ASAM Criteria.

References: DSM-5-TR — Alcohol Use Disorder, severity specifiers

☐ Approve ☐ Revise ☐ Retire Notes:

A 52-year-old takes a prescribed opioid exactly as directed for chronic back pain under a physician's care. She reports needing a higher dose than at first and feeling withdrawal symptoms if a dose is late. No other use disorder criteria are present. How should the counselor understand these symptoms?

A. They are not counted toward an opioid use disorder ✓ KEY

Tolerance and withdrawal are not counted when the opioid is taken solely under appropriate medical supervision.

B. They meet criteria for mild opioid use disorder

Two counted criteria would be mild, but these two are excluded in supervised medical use.

C. They confirm a severe opioid use disorder is present

Physiological dependence alone does not establish any use disorder, let alone a severe one.

D. They point to opioid use disorder in remission

Remission requires a prior full use disorder, which has not been established.

Rationale: DSM-5-TR notes that tolerance and withdrawal are expected physiological adaptations when medications such as opioids, sedatives, or stimulants are taken as prescribed under medical supervision, and they are not counted toward a use disorder in that situation. A diagnosis requires other evidence of problematic use, such as craving or loss of control.

References: DSM-5-TR — Opioid Use Disorder, Criteria 10–11 note on medical supervision

► **Check:** The cited 'Criteria 10-11 note' detail is the writer's wording of where the DSM-5-TR note sits.

☐ Approve ☐ Revise ☐ Retire Notes:

Under DSM-5-TR, several inattentive or hyperactive-impulsive symptoms of ADHD must have been present before what age?

A. Age 7

Onset before age 7 was the DSM-IV standard; DSM-5 raised it to 12.

B. Age 12 ✓ KEY

DSM-5-TR requires several symptoms to have been present before age 12.

C. Age 16

Age 16 is not a criterion; age 17 marks where the adult symptom count of five applies.

D. Age 18

Age 18 is not the ADHD onset threshold.

Rationale: ADHD requires several inattentive or hyperactive-impulsive symptoms before age 12, present in two or more settings, with clear interference in functioning. Children need six symptoms in a domain; adolescents 17 and older and adults need five. Adult assessment therefore includes a developmental history.

References: DSM-5-TR — Attention-Deficit/Hyperactivity Disorder, Criteria A and C

☐ Approve ☐ Revise ☐ Retire Notes:

A 7-year-old struggles to take turns in conversation, misses jokes and hints, and speaks to teachers the same way he speaks to peers. A counselor is weighing autism spectrum disorder against social (pragmatic) communication disorder. Which finding would MOST support autism spectrum disorder?

A. Late onset of first spoken words

Language delay can occur in many conditions and is not a defining criterion that separates the two.

B. Trouble using language socially

Difficulty with social use of language is central to both disorders, so it does not distinguish them.

C. Below-average intellectual ability

Intellectual impairment is a possible specifier for ASD but is neither required nor distinguishing.

D. Restricted, repetitive behaviors ✓ KEY

ASD requires restricted, repetitive patterns of behavior, interests, or activities; their presence rules out SCD.

Rationale: Both ASD and social (pragmatic) communication disorder involve deficits in social communication. ASD also requires at least two kinds of restricted, repetitive patterns of behavior, interests, or activities (such as rigid routines, intense interests, or sensory differences). SCD is diagnosed only when such patterns are absent, past or present.

References: DSM-5-TR — Social (Pragmatic) Communication Disorder, Criterion D; Autism Spectrum Disorder, Criterion B

☐ Approve ☐ Revise ☐ Retire Notes:

A 22-year-old college student has had auditory hallucinations, persecutory delusions, and disorganized speech for about 3 months, including any early warning signs. There have been no mood episodes, and substance and medical causes are ruled out. Which diagnosis is MOST appropriate?

A. Schizophreniform disorder ✓ **KEY**

Schizophreniform disorder has schizophrenia-like symptoms lasting at least 1 month but less than 6 months.

B. Schizophrenia

Schizophrenia requires continuous signs of disturbance for at least 6 months.

C. Brief psychotic disorder

Brief psychotic disorder lasts at least 1 day but less than 1 month, with full return to functioning.

D. Delusional disorder

Delusional disorder involves delusions without prominent hallucinations or disorganized speech.

Rationale: The schizophrenia spectrum is partly sorted by duration: brief psychotic disorder lasts 1 day to less than 1 month; schizophreniform disorder lasts 1 month to less than 6 months; schizophrenia requires at least 6 months of disturbance, including at least 1 month of active-phase symptoms.

References: DSM-5-TR — Schizophreniform Disorder, Criteria A–B

☐ Approve ☐ Revise ☐ Retire Notes:

A 26-year-old has had 5 weeks of prominent hallucinations and delusions. His family reports that for about 6 months before that, he withdrew from friends, showed flat affect, and voiced odd beliefs. No mood episodes have occurred, and substance and medical causes are excluded. Which diagnosis BEST fits?

A. Schizophreniform disorder

The prodromal period counts toward duration, so the total exceeds the less-than-6-months limit for schizophreniform disorder.

B. Schizophrenia ✓ **KEY**

More than 1 month of active symptoms plus a prodromal period gives over 6 months of continuous disturbance.

C. Brief psychotic disorder

Brief psychotic disorder lasts less than 1 month in total; the active phase alone is longer.

D. Schizoaffective disorder

Schizoaffective disorder requires a major mood episode concurrent with psychotic symptoms; none occurred.

Rationale: Schizophrenia requires continuous signs of disturbance for at least 6 months, which must include at least 1 month of active-phase symptoms and may include prodromal or residual periods with negative symptoms or attenuated positive symptoms. Collateral history from family is often essential for establishing the prodromal phase.

References: DSM-5-TR — Schizophrenia, Criterion C

☐ Approve ☐ Revise ☐ Retire Notes:

Which personality disorder belongs to DSM-5-TR Cluster C (the anxious or fearful cluster)?

A. Schizotypal personality disorder

Schizotypal is in Cluster A, the odd or eccentric cluster.

B. Histrionic personality disorder

Histrionic is in Cluster B, the dramatic, emotional, or erratic cluster.

C. Avoidant personality disorder ✓ KEY

Avoidant personality disorder is in Cluster C with dependent and obsessive-compulsive personality disorders.

D. Paranoid personality disorder

Paranoid is in Cluster A with schizoid and schizotypal.

Rationale: DSM-5-TR groups personality disorders into three clusters: A (paranoid, schizoid, schizotypal) for odd or eccentric patterns; B (antisocial, borderline, histrionic, narcissistic) for dramatic or erratic patterns; and C (avoidant, dependent, obsessive-compulsive) for anxious or fearful patterns.

References: DSM-5-TR — Personality Disorders, cluster groupings

☐ Approve ☐ Revise ☐ Retire Notes:

A 20-year-old shows a pervasive pattern of deceitfulness, impulsivity, repeated fights, and lack of remorse since age 15. To diagnose antisocial personality disorder, which additional finding does DSM-5-TR require?

A. A documented history of arrest or jail

Legal involvement is common but not required; failure to conform to social norms can be shown in other ways.

B. No co-occurring substance use disorder

Substance use disorders frequently co-occur and do not rule out antisocial personality disorder.

C. Onset of the pattern after age 18 only

The pattern must be evident since age 15; onset is not restricted to adulthood.

D. Evidence of conduct disorder before 15 ✓ KEY

DSM-5-TR requires evidence of conduct disorder with onset before age 15, and the person must be at least 18.

Rationale: Antisocial personality disorder requires a pervasive pattern of disregard for and violation of the rights of others occurring since age 15, a current age of at least 18, and evidence of conduct disorder with onset before age 15. The behavior must not occur only during schizophrenia or bipolar disorder.

References: DSM-5-TR — Antisocial Personality Disorder, Criteria B–C

☐ Approve ☐ Revise ☐ Retire Notes:

A 33-year-old describes episodes, about twice a week for 6 months, of eating far more than most people would in a short period while feeling unable to stop. She eats rapidly, eats alone out of embarrassment, and feels disgusted afterward. She does not vomit, fast, misuse laxatives, or exercise to compensate. Which diagnosis BEST fits?

A. Bulimia nervosa

Bulimia nervosa requires recurrent inappropriate compensatory behaviors, which she does not use.

B. Binge-eating disorder ✓ KEY

Recurrent binges with loss of control and marked distress, at least weekly for 3 months, without regular compensatory behavior, fit binge-eating disorder.

C. ARFID

ARFID involves restricted intake, not binge episodes.

D. Anorexia nervosa, binge/purge type

Anorexia nervosa requires restriction leading to significantly low body weight and fear of weight gain, none of which is described, and she does not purge.

Rationale: Binge-eating disorder requires recurrent binge episodes with a sense of loss of control, three or more associated features (such as eating rapidly, eating alone from embarrassment, or feeling disgusted afterward), marked distress, and a frequency of at least once a week for 3 months. Unlike bulimia nervosa, there are no recurrent compensatory behaviors.

References: DSM-5-TR — Binge-Eating Disorder, Criteria A–E

☐ Approve ☐ Revise ☐ Retire Notes:

Since choking on a piece of food 4 months ago, a 10-year-old refuses most solid foods, eats only liquids and soft foods, and has lost weight. She expresses no concern about body shape or size and wants to be "back to normal." Which diagnosis is MOST likely?

A. Anorexia nervosa

Anorexia nervosa involves fear of weight gain or disturbed body image, which she does not show.

B. Rumination disorder

Rumination disorder involves repeated regurgitation of food, not avoidance after choking.

C. ARFID ✓ KEY

ARFID covers restriction driven by fear of aversive consequences such as choking, without body image disturbance.

D. Pica

Pica involves persistent eating of nonnutritive, nonfood substances.

Rationale: Avoidant/restrictive food intake disorder involves restricted eating that leads to weight loss, nutritional deficiency, or impaired functioning, motivated by lack of interest in food, sensory aversions, or fear of aversive consequences such as choking or vomiting. It is distinguished from anorexia nervosa by the absence of fear of weight gain or disturbed body image.

References: DSM-5-TR — Avoidant/Restrictive Food Intake Disorder, Criteria A and C

☐ Approve ☐ Revise ☐ Retire Notes:

A 74-year-old retired teacher and her daughter both report a gradual decline in her memory over the past year. Testing shows a modest decline from her prior level. She still manages her own bills, medications, and shopping, although these tasks now take more effort and reminder lists. Which diagnosis BEST fits?

A. Major neurocognitive disorder

Major NCD requires substantial decline that interferes with independence in everyday activities.

B. Delirium

Delirium has an acute onset with fluctuating attention and awareness, not a gradual year-long decline.

C. Mild neurocognitive disorder ✓ KEY

Modest documented decline that does not interfere with independence, even if compensation is needed, fits mild NCD.

D. Dissociative amnesia

Dissociative amnesia involves inability to recall autobiographical information, usually traumatic, not a gradual cognitive decline.

Rationale: Mild and major neurocognitive disorder differ chiefly in functional impact. Mild NCD shows modest decline from a previous level, supported by concern and testing, without loss of independence in everyday activities, though greater effort or compensatory strategies may be needed. Major NCD involves significant decline that interferes with independence.

References: DSM-5-TR — Mild Neurocognitive Disorder, Criteria A–B

☐ Approve ☐ Revise ☐ Retire Notes:

An 81-year-old hospitalized for a urinary tract infection became confused over 2 days. Staff note she is alert one hour and disoriented the next. A counselor on the unit is asked to help distinguish delirium from major neurocognitive disorder. Which feature MOST points to delirium?

A. Acute onset with fluctuating attention ✓ KEY

Delirium develops over hours to days and fluctuates in severity, with disturbed attention and awareness.

B. Memory impairment found on screening

Memory deficits occur in both delirium and major NCD, so they do not distinguish them.

C. Onset of symptoms after the age of 65

Both conditions are more common in older adults; age does not separate them.

D. Presence of vivid visual hallucinations

Visual hallucinations can occur in delirium and in major NCD, notably with Lewy bodies.

Rationale: Delirium is a disturbance in attention and awareness that develops over a short period (hours to a few days), represents a change from baseline, and tends to fluctuate during the day. It often has a medical cause, such as infection or medication effects, and requires prompt medical evaluation.

References: DSM-5-TR — Delirium, Criteria A–B

☐ Approve ☐ Revise ☐ Retire Notes:

A 58-year-old whose wife died 7 months ago reports intense yearning for her nearly every day, feeling that part of himself has died, and difficulty re-engaging with friends. His grief is far beyond what is expected in his community and is impairing his work. Which statement about prolonged grief disorder is MOST accurate?

A. It can be diagnosed now because symptoms have lasted over 6 months

Six months is the threshold for children and adolescents, not adults.

B. It cannot yet be diagnosed because 12 months have not yet passed ✓ **KEY**

For adults, DSM-5-TR requires at least 12 months since the death.

C. It is not in DSM-5-TR, so his grief is coded only as a Z code

Prolonged grief disorder was added to the trauma- and stressor-related disorders in DSM-5-TR; uncomplicated bereavement is the Z code.

D. It is ruled out, and major depression must be diagnosed instead

Nothing here establishes a major depressive episode; yearning and identity disruption are grief-specific.

Rationale: DSM-5-TR added prolonged grief disorder to the trauma- and stressor-related disorders. It requires that at least 12 months have passed since the death for adults (6 months for children and adolescents), with intense yearning or preoccupation nearly every day for at least the last month, plus at least three additional symptoms such as identity disruption or emotional numbness.

References: DSM-5-TR — Prolonged Grief Disorder, Criterion A

☐ Approve ☐ Revise ☐ Retire Notes:

Six weeks after his mother died, a client reports sadness, poor sleep, and crying spells. Which feature is MORE typical of grief than of a major depressive episode?

A. Pervasive worthlessness and self-loathing

Pervasive worthlessness and self-loathing are more typical of a major depressive episode.

B. Persistent unhappiness unrelated to reminders

Mood that stays low regardless of reminders of the loss is more typical of depression.

C. Self-esteem that is generally preserved ✓ **KEY**

In grief, self-esteem is usually preserved, while depression often brings worthlessness.

D. Inability to anticipate any future pleasure

Pervasive anhedonia and pessimism about the future are characteristic of a depressive episode.

Rationale: DSM-5-TR notes that in grief the predominant feelings are emptiness and loss, dysphoria tends to come in waves ("pangs") linked to reminders of the deceased, humor can coexist with pain, and self-esteem is usually preserved. In a major depressive episode, mood is persistently low and is often accompanied by worthlessness and self-loathing.

References: DSM-5-TR — Major Depressive Disorder, note distinguishing grief from a major depressive episode

☐ Approve ☐ Revise ☐ Retire Notes:

A counselor volunteers at a shelter the day after a tornado destroyed many homes. Survivors are shaken, tired, and worried about relatives. Which approach is MOST appropriate in this early phase?

A. Lead a required group debriefing on each survivor's experience

Mandatory single-session debriefing is not recommended and may increase distress for some survivors.

B. Give each survivor a structured PTSD diagnostic interview

Most survivors recover naturally; diagnostic interviews this early are premature and intrusive.

C. Start trauma-focused exposure work with the survivors who are upset

Trauma-focused therapy is for persistent symptoms after the acute phase, not as an immediate shelter intervention.

D. Offer psychological first aid for safety, comfort, and needs ✓ **KEY**

Psychological first aid promotes safety, calm, connection, and practical help, and is the recommended early response.

Rationale: In the immediate aftermath of a disaster, the recommended response is psychological first aid: ensuring safety, providing comfort, meeting practical needs, connecting people with loved ones and supports, and screening for those needing more help. Routine single-session psychological debriefing is not recommended for preventing PTSD.

References: Psychological First Aid Field Operations Guide (NCTSN/NCPTSD) — PFA core actions: safety and comfort, practical assistance, connection with social supports · NICE guidelines — NG116 Post-traumatic stress disorder: psychologically focused debriefing not offered

☐ Approve ☐ Revise ☐ Retire Notes:

A client calls the counselor sobbing after receiving an eviction notice. She says, "I just don't see the point of anything anymore." What should the counselor do FIRST?

A. Ask her directly about suicidal thoughts and intent ✓ **KEY**

A statement of hopelessness in crisis calls for direct assessment of suicidal ideation, intent, and plan before other steps.

B. Help her brainstorm options for finding new housing now

Problem solving is useful later, but safety must be assessed first.

C. Reflect feelings and save risk questions for later

Delaying risk assessment after a possible warning sign could miss imminent danger.

D. Give her the number of a local legal aid clinic

A referral addresses the stressor but skips assessment of her immediate safety.

Rationale: In crisis intervention, ensuring safety comes before problem solving. Hopeless statements such as "I don't see the point" are possible warning signs, and asking directly about suicidal thoughts does not increase risk. Structured tools such as the C-SSRS guide questions about ideation, intent, plan, and behavior.

References: C-SSRS — Screener: suicidal ideation and intent questions

☐ Approve ☐ Revise ☐ Retire Notes:

During the individual portion of a couples intake, one partner discloses that the other checks her phone, controls all the money, and shoved her last month. She says she is afraid of what will happen if he learns she told. What is the counselor's BEST next step?

A. Begin joint sessions on communication skills

Conjoint work is contraindicated while coercive control and fear are present, as it can increase danger.

B. Raise her disclosure in the next joint session

Revealing the disclosure to the partner could trigger retaliation.

C. Refer both partners to one anger management group

Anger management does not address coercive control, and joint referral may compromise her safety.

D. Assess danger with her alone and plan for safety ✓ KEY

Individual risk assessment and safety planning come first when violence and fear are disclosed.

Rationale: When intimate partner violence involving fear and coercive control is disclosed, conjoint counseling is generally contraindicated because sessions can provoke retaliation. The counselor should meet with the person alone, assess lethality and danger, develop a safety plan, and connect her with advocacy resources while protecting the confidentiality of the disclosure.

References: ACA Code of Ethics — A.4.a Avoiding Harm

► **Check:** IPV item cites general sources (ACA, Sue & Sue); no IPV-specific source on the approved list.

☐ Approve ☐ Revise ☐ Retire Notes:

A recently immigrated client discloses that her husband hits her and threatens that she will be deported if she leaves him, since her visa depends on his sponsorship. She speaks limited English and has no family nearby. Which counselor response is MOST helpful?

A. Advise her to stay until her immigration status is secure

This accepts the abuser's threat at face value and leaves her in danger without exploring legal protections.

B. Connect her with an immigration-informed IPV advocate ✓ KEY

Specialized advocates can explain protections available to abused immigrants and support culturally responsive safety planning.

C. Report the husband's conduct to immigration authorities

This is not the counselor's role, breaches confidentiality, and may put the client at greater risk.

D. Suggest couples counseling to ease conflict at home now

Couples counseling is contraindicated while there is ongoing violence and threats.

Rationale: Abusers of immigrant partners often use immigration status as a tool of control. Counselors should recognize this cultural and structural vulnerability, prioritize safety, provide interpreter and language access, and link clients with advocates who understand immigration-related protections rather than offering legal advice themselves.

References: Sue & Sue (Counseling the Culturally Diverse) — Immigrant and refugee clients: structural barriers and advocacy

► **Check:** IPV item cites general sources; the key deliberately avoids specific immigration-law (VAWA) claims.

☐ Approve ☐ Revise ☐ Retire Notes:

During a session at an elementary school, a 7-year-old tells the school counselor that her stepfather hit her with a belt last night, and the counselor sees welts on her arm. What should the counselor do FIRST?

A. Make a report to child protective services as the law requires of mandated reporters ✓ KEY

Reasonable suspicion of abuse triggers a mandated report; the counselor reports promptly and lets child protective services investigate.

B. Call the stepfather to hear his account of what happened before deciding to file any report

Contacting the alleged offender can put the child at risk and is not required before reporting; investigation belongs to CPS.

C. Examine and photograph all of the child's injuries so the evidence is complete before reporting

Counselors do not conduct forensic examinations; doing so can retraumatize the child and is the investigator's role.

D. Schedule a follow-up meeting for next week to see whether new marks appear before reporting

Waiting delays protection; the reporting duty arises at reasonable suspicion, which is already present.

Rationale: Counselors are mandated reporters in every U.S. state. The threshold is reasonable suspicion, not proof, and the report is made promptly to child protective services or law enforcement. The counselor's job is to report and support the child, not to investigate or confront the alleged perpetrator.

References: State law — Mandated reporting statutes for suspected child abuse and neglect · ACA Code of Ethics — B.2.a Serious and Foreseeable Harm and Legal Requirements

☐ Approve ☐ Revise ☐ Retire Notes:

A 10-year-old client mentions that he is often home alone for whole weekends with little food in the house. He gives few details and changes the subject. A colleague advises the counselor not to report neglect until the boy gives a clear account. Which statement BEST reflects the reporting standard?

A. A report is required only after the child gives a clear and consistent disclosure

Children often disclose partially; the law does not require a complete or consistent disclosure before reporting.

B. A report is required only after a parent has a chance to explain the circumstances

Mandated reporters do not need to hear the caregiver's explanation first; that is part of the CPS investigation.

C. A report is required when there is reasonable suspicion; proof of neglect is not needed ✓ KEY

The legal threshold is reasonable cause to suspect abuse or neglect, which repeated lack of supervision and food can meet.

D. A report is required only once a supervisor independently confirms that neglect occurred

Consulting a supervisor is good practice, but the individual duty to report does not wait for anyone to confirm the facts.

Rationale: Mandated reporting laws use a reasonable-suspicion standard. Counselors report what they have observed or been told and leave fact-finding to child protective services. Neglect, including inadequate supervision and failure to provide food, is reportable in the same way as physical abuse.

References: State law — Child abuse and neglect reporting statutes: reasonable cause to suspect standard

☐ Approve ☐ Revise ☐ Retire Notes:

In Holland's theory, consistency describes how close the first two letters of a person's code sit on the RIASEC hexagon. Which Holland code shows the LOWEST consistency?

A. RIC (Realistic-Investigative)

Realistic and Investigative are adjacent on the hexagon, so this code is highly consistent.

B. SEA (Social-Enterprising)

Social and Enterprising are adjacent on the hexagon, so this code is highly consistent.

C. ECR (Enterprising-Conventional)

Enterprising and Conventional are adjacent on the hexagon, so this code is highly consistent.

D. RSI (Realistic-Social) ✓ KEY

Realistic and Social sit opposite each other on the hexagon, the least consistent pairing.

Rationale: Holland arranged the six types in the order R-I-A-S-E-C around a hexagon. Adjacent types share the most in common and opposite types the least, so a code whose first two letters are opposite (R-S, I-E, A-C) has low consistency. Consistency is distinct from differentiation (how sharply one type stands out) and congruence (person-environment fit).

References: Sharf (Career Development Theory) — Holland's theory of types: consistency, differentiation, congruence

☐ Approve ☐ Revise ☐ Retire Notes:

A 52-year-old machinist who expected to finish his career at one plant is laid off when it closes. He is now researching training programs, trying out new interests, and reconsidering what work means to him. Which concept from Super's life-span, life-space theory BEST describes this process?

A. Recycling through exploration tasks during a later life stage ✓ KEY

Super held that adults can recycle through earlier stage tasks, such as exploration, when a transition disrupts their career.

B. Circumscription of options that conflict with his social class

Circumscription belongs to Gottfredson's theory and describes early elimination of options by sex type and prestige.

C. Planned happenstance that turns the chance layoff into learning

Planned happenstance is Krumboltz's concept, not Super's, and describes using unplanned events, not a stage process.

D. Correspondence between his abilities and the reinforcers of work

Correspondence is the core idea of the theory of work adjustment (Dawis and Lofquist), not of Super's stage model.

Rationale: Super's stages (growth, exploration, establishment, maintenance, disengagement) form a maxicycle across the life span, but people can recycle through minicycles of earlier stages at transitions such as job loss. A midlife worker re-exploring options is the classic example of recycling.

References: Sharf (Career Development Theory) — Super's life-span, life-space theory: maxicycle, minicycles and recycling

☐ Approve ☐ Revise ☐ Retire Notes:

A 7-year-old girl who loves building things tells her counselor that she cannot be an engineer because "that is a job for boys." In Gottfredson's theory, what is this child doing?

A. Compromising by settling for a job that is more accessible to her

Compromise comes later, when people give up preferred options because of real-world barriers to access.

B. Circumscribing her options by ruling out jobs seen as wrong for her sex ✓ **KEY**

Ages 6 to 8 are Gottfredson's orientation to sex roles, when children eliminate jobs that seem wrong for their gender.

C. Showing low differentiation because her interests are not yet clear

Differentiation is a Holland concept about how distinct a profile is; her interest in building is actually clear.

D. Crystallizing a vocational preference as the exploration stage begins

Crystallization is a task in Super's exploration stage during adolescence, not a process of ruling options out in childhood.

Rationale: Gottfredson's theory of circumscription and compromise describes children progressively eliminating occupations: by size and power (ages 3-5), by sex type (ages 6-8), by social valuing or prestige (ages 9-13), and then by internal, unique self (14 and older). Options eliminated early are often never reconsidered, so counselors can help children revisit them.

References: Sharf (Career Development Theory) — Gottfredson's theory of circumscription and compromise: orientation to sex roles

☐ Approve ☐ Revise ☐ Retire Notes:

In couples sessions, one partner regularly rolls her eyes, mocks her spouse's tone, and calls him "pathetic." According to Gottman's research, which of the Four Horsemen is this, and the single strongest predictor of divorce?

A. Criticism

Criticism attacks the partner's character, but it lacks the superiority and disgust that mark contempt.

B. Stonewalling

Stonewalling is emotional withdrawal from the interaction, not mockery or eye-rolling.

C. Defensiveness

Defensiveness is self-protection through excuses or counterattack, not a display of disgust.

D. Contempt ✓ **KEY**

Eye-rolling, mockery and name-calling express contempt, which Gottman found to be the strongest predictor of divorce.

Rationale: Gottman's Four Horsemen are criticism, contempt, defensiveness and stonewalling. Contempt communicates superiority and disgust through sarcasm, mockery, eye-rolling and insults, and it was the single best predictor of divorce in his studies. The antidote is building a culture of appreciation and respect.

References: Gladding (Counseling: A Comprehensive Profession) — Marriage, couple and family counseling: Gottman's research on the Four Horsemen

► **Check:** Confirm the Gladding section that covers Gottman's Four Horsemen.

☐ Approve ☐ Revise ☐ Retire Notes:

A newly formed stepfamily seeks counseling because the 13-year-old daughter openly defies her new stepfather's rules, and he responds by giving stricter punishments. Which recommendation is MOST consistent with research on stepfamily adjustment?

A. Have the stepfather take over discipline so the teen accepts him as the new head of the home

Early stepparent discipline before a bond forms tends to increase conflict and resentment.

B. Have the biological parent lead discipline while the stepparent builds a warm relationship first ✓ **KEY**

Stepfamily research supports the biological parent handling discipline early while the stepparent focuses on connection.

C. Have both adults avoid setting any rules until the teen says she is ready to accept the stepfather

Removing structure leaves the teen without limits and puts her in charge of the family's functioning.

D. Have the teen spend more time with her other parent until she is willing to follow the new rules

Using contact with the other parent as leverage triangulates the teen and can deepen loyalty conflicts.

Rationale: Stepfamilies form after losses and bring loyalty binds. Stepparents gain authority gradually, after a relationship exists, so the biological parent usually remains the primary disciplinarian at first while the stepparent acts more like a supportive adult. The couple still agrees on house rules together.

References: Gladding (Counseling: A Comprehensive Profession) — Family counseling: blended families and stepparent roles

► **Check:** Confirm the Gladding section that covers stepfamily discipline roles.

☐ Approve ☐ Revise ☐ Retire Notes:

A counselor is about to begin conjoint counseling with a married couple. Each partner may also call or meet with the counselor alone from time to time. What should the counselor establish at the outset?

A. A promise that any individual disclosure will stay sealed from the other partner

Promising to hold secrets can trap the counselor in a hidden alliance and undermine the couple work.

B. An agreement that each partner will be assigned their own separate counselor

Separate counselors are an option but do not answer how this counselor will handle the couple case.

C. A clear policy, agreed by both, on how information shared individually is handled ✓ **KEY**

Clarifying who the client is and how individual information will be handled is required at the outset of multiple-client work.

D. A rule that the counselor will refuse to speak to either partner outside sessions

Some contact outside joint sessions can be appropriate; the issue is a clear policy, not a ban.

Rationale: When counseling more than one person who share a relationship, counselors clarify at the outset who the client is and the limits of confidentiality, including whether a no-secrets or limited-secrets policy applies. Both partners agree to that policy before individual contacts happen.

References: ACA Code of Ethics — A.8 Multiple Clients; B.4.b Couples and Family Counseling

☐ Approve ☐ Revise ☐ Retire Notes:

A counselor uses Schlossberg's transition model with a 58-year-old woman who has just retired early to care for her spouse. Which set of factors does the model use to assess her resources for coping?

A. Situation, self, support and strategies ✓ KEY

Schlossberg's 4 S's are situation, self, support and strategies, used to weigh a person's coping resources.

B. Denial, anger, bargaining and acceptance

These are stages from Kübler-Ross's grief model, not Schlossberg's factors for coping with a transition.

C. Growth, exploration, establishment and decline

These are life stages in Super's career theory, not resources for coping with a transition.

D. Shock, withdrawal, adjustment and integration

This is a generic phase sequence, not the four coping-resource factors of Schlossberg's model.

Rationale: Schlossberg defines a transition as any event or non-event that changes roles, relationships, routines and assumptions. Coping is assessed through the 4 S's: the situation (timing, control, concurrent stress), the self (personal and psychological resources), support (relationships and institutions) and strategies (ways of coping).

References: Sharf (Career Development Theory) — Adult career transitions: Schlossberg's transition model (4 S's)

☐ Approve ☐ Revise ☐ Retire Notes:

A 24-year-old client lives with his parents, has changed jobs and majors several times, and says he feels "not really a kid, but not quite an adult." He is still exploring identity, work and relationships. Which concept BEST fits this period?

A. Identity foreclosure

Foreclosure is commitment without exploration; this client is exploring and has not committed.

B. Midlife transition

Levinson's midlife transition occurs around age 40, not in the early twenties.

C. Generativity versus stagnation

Erikson's generativity stage concerns middle adulthood and guiding the next generation.

D. Emerging adulthood ✓ KEY

Arnett described emerging adulthood (about ages 18 to 25) as a time of identity exploration and feeling in-between.

Rationale: Arnett proposed emerging adulthood as a distinct period, roughly ages 18 to 25 in industrialized societies, marked by identity exploration, instability, self-focus, feeling in-between and a sense of possibilities. Counselors can frame this exploration as developmentally typical rather than as failure to launch.

References: Berk (Development Through the Lifespan) — Early adulthood: Arnett's emerging adulthood

☐ Approve ☐ Revise ☐ Retire Notes:

A 79-year-old widower in assisted living spends sessions reviewing his life, some proud moments and some regrets, and asks whether his life "added up to anything." Which of Erikson's psychosocial stages is he working through?

A. Generativity versus stagnation

Generativity is the middle-adulthood task of guiding the next generation and being productive.

B. Intimacy versus isolation

Intimacy is the young-adult task of forming close, committed relationships.

C. Integrity versus despair ✓ KEY

Late adulthood centers on accepting one's life as meaningful (integrity) rather than feeling regret (despair).

D. Identity versus role confusion

Identity versus role confusion is the adolescent task of forming a coherent sense of self.

Rationale: Erikson's final stage, integrity versus despair, involves looking back and accepting one's life as it was. Life review, described by Butler, is a structured way to help older adults integrate successes and regrets and move toward integrity.

References: Berk (Development Through the Lifespan) — Late adulthood: Erikson's ego integrity versus despair; life review

☐ Approve ☐ Revise ☐ Retire Notes:

An adult daughter brings her 74-year-old mother to counseling, worried about "early dementia." Over two months the mother has lost interest in her garden, sleeps poorly, and often says "I just can't remember anything anymore." She seems distressed by her memory lapses. What should the counselor do FIRST?

A. Screen for depression and refer for a medical evaluation of the memory complaints ✓ KEY

Depression in older adults can mimic cognitive decline, and a medical workup rules out treatable causes.

B. Tell the family that memory loss of this kind is a normal part of growing older

Noticeable decline with mood change is not normal aging and should not be dismissed.

C. Begin memory-training exercises in sessions to slow the progression of dementia

This assumes a dementia diagnosis that has not been made and skips assessment of depression.

D. Recommend that the daughter begin looking into memory-care residential placement

Placement planning is premature without assessment and could cause harm if the cause is depression.

Rationale: Late-life depression often presents with memory and concentration complaints, sometimes called pseudodementia. Clues include recent onset with mood symptoms and a person who is distressed by and emphasizes their lapses. The counselor screens for depression (for example with the Geriatric Depression Scale) and coordinates a medical evaluation.

References: DSM-5-TR — Major depressive disorder: diminished ability to think or concentrate; differential with major neurocognitive disorder

☐ Approve ☐ Revise ☐ Retire Notes:

A client who uses a wheelchair after a spinal cord injury says, "My body is not the problem. The problem is buildings without ramps and managers who assume I can't do the job." Which model of disability does her statement reflect?

A. Medical model

The medical model locates disability in the individual's impairment, to be diagnosed and treated.

B. Moral model

The moral model treats disability as a sign of sin or a test of character.

C. Rehabilitation model

This model focuses on restoring individual function, still locating the problem in the person.

D. Social model ✓ KEY

The social model locates disability in environmental and attitudinal barriers, not in the person.

Rationale: The social model distinguishes impairment (a bodily difference) from disability (exclusion created by inaccessible environments and attitudes). Counselors working from it attend to barriers, advocacy and accommodation as well as the client's adjustment.

References: Sue & Sue (Counseling the Culturally Diverse) — Counseling individuals with disabilities: medical versus social models of disability

► **Check:** Confirm Sue & Sue's disability coverage; 'rehabilitation model' distractor is a looser term than medical/moral/social.

☐ Approve ☐ Revise ☐ Retire Notes:

A 34-year-old client with type 1 diabetes often skips glucose checks and insulin because, she says, "thinking about it makes it too real." Her blood sugar control has worsened and she has had two ER visits. She reports no persistent low mood, worry about having other illnesses, or other psychiatric symptoms. Which DSM-5-TR diagnosis BEST fits?

A. Illness anxiety disorder of the care-avoidant type

Illness anxiety involves preoccupation with having or getting a serious illness, which she does not report.

B. Psychological factors affecting other medical conditions ✓ KEY

Her avoidance is a psychological factor that worsens a diagnosed medical condition, with no other mental disorder.

C. Somatic symptom disorder with a persistent course

Somatic symptom disorder requires distressing physical symptoms with excessive related thoughts, not avoidance of care.

D. Factitious disorder imposed on self, with recurrent episodes

Factitious disorder involves deliberately falsifying or inducing illness, which is not described here.

Rationale: Psychological factors affecting other medical conditions applies when psychological or behavioral factors, such as denial or poor adherence, adversely affect a medical condition and are not better explained by another mental disorder. It is listed with somatic symptom and related disorders but does not require distressing somatic symptoms.

References: DSM-5-TR — Psychological Factors Affecting Other Medical Conditions: criteria A-B

☐ Approve ☐ Revise ☐ Retire Notes:

A bisexual client has never experienced overt harassment, yet she hides her identity at work, constantly scans for signs of rejection, and says part of her believes something is wrong with her. In Meyer's minority stress model, these experiences are BEST described as which kind of stressors?

A. Distal stressors from discrimination

Distal stressors are external events such as harassment or discrimination, which she has not experienced.

B. General life stressors shared by all

These stressors are tied specifically to her stigmatized identity, not common to everyone.

C. Proximal stressors tied to stigma ✓ KEY

Concealment, expected rejection and internalized stigma are Meyer's proximal (internal) minority stressors.

D. Coping resources within the community

Community connection is a protective factor in the model, not a source of stress.

Rationale: Meyer's model separates distal stressors (external prejudice events, discrimination, violence) from proximal stressors (expectations of rejection, concealment and internalized homophobia or biphobia). Both add to the general stress everyone faces, and community connection and coping can buffer their effects on mental health.

References: Sue & Sue (Counseling the Culturally Diverse) — Counseling LGBTQ individuals: minority stress

☐ Approve ☐ Revise ☐ Retire Notes:

The parents of a 16-year-old who recently came out as gay ask a counselor to "help him become straight." The teen does not want to change his orientation. How should the counselor respond?

A. Decline to attempt orientation change and offer support for the teen and family ✓ KEY

Orientation change efforts lack scientific support and carry risk of harm; family support work is appropriate.

B. Agree to the goal because parents hold legal authority over a minor's treatment

Parental consent does not make a harmful, unsupported technique ethical to use.

C. Refer the family to a provider who specializes in sexual orientation change work

Referring for a harmful practice still facilitates it and does not protect the client.

D. Explore with the teen whether his orientation might be a phase that will soon pass

Framing his identity as a phase invalidates it and reflects the parents' goal, not his own.

Rationale: The ACA Code bars techniques with substantial evidence of harm even when requested, and sexual orientation change efforts fall in that category. The counselor can decline that goal while still helping the family: providing education, supporting the parents' reactions and reducing family rejection, which is a strong risk factor for LGBTQ+ youth.

References: ACA Code of Ethics — C.7.a Scientific Basis for Treatment; C.7.c Harmful Practices

☐ Approve ☐ Revise ☐ Retire Notes:

A man who immigrated from Vietnam 15 years ago has stopped speaking Vietnamese, no longer observes family traditions, and says he identifies only with mainstream American culture. Which of Berry's acculturation strategies does this describe?

A. Integration

Integration keeps the heritage culture while also taking part in the host culture.

B. Assimilation ✓ KEY

Assimilation means giving up heritage culture and adopting the host culture.

C. Separation

Separation keeps the heritage culture while avoiding the host culture.

D. Marginalization

Marginalization means little connection to either the heritage or host culture.

Rationale: Berry's model crosses two questions: is the heritage culture maintained, and is contact with the host society sought? Yes and yes is integration; no and yes is assimilation; yes and no is separation; no and no is marginalization.

References: Sue & Sue (Counseling the Culturally Diverse) — Acculturation: Berry's four acculturation strategies

☐ Approve ☐ Revise ☐ Retire Notes:

A refugee teen has drifted away from her parents' language and customs but also feels unwelcome and isolated among peers at her new school. In Berry's framework, which strategy does this reflect, and how does it generally compare with the others in acculturative stress?

A. Separation, generally tied to the lowest acculturative stress

Separation keeps heritage culture; she has moved away from hers, and integration, not separation, is lowest in stress.

B. Integration, generally tied to the highest acculturative stress

She does not hold both cultures, and integration is usually associated with the least stress.

C. Assimilation, generally tied to the lowest acculturative stress

She has not joined the host culture, and assimilation is not the lowest-stress strategy.

D. Marginalization, generally tied to the highest acculturative stress ✓ KEY

Losing ties to both cultures is marginalization, which Berry found carries the greatest acculturative stress.

Rationale: In Berry's research, integration is generally associated with the best adaptation and least acculturative stress, while marginalization, low engagement with both cultures, is associated with the most. Assimilation and separation fall in between. Counselors can help marginalized clients rebuild connection to one or both cultures.

References: Sue & Sue (Counseling the Culturally Diverse) — Acculturation and acculturative stress: Berry's model

☐ Approve ☐ Revise ☐ Retire Notes:

A devout client feels intense guilt about her divorce because of her faith community's teachings. The counselor is not religious and privately views the teachings as harmful. Which approach is MOST appropriate?

A. Encourage her to question the teachings that are leading her to feel guilty

Steering her away from her beliefs imposes the counselor's values on the client.

B. Explore her guilt within her own faith framework and the resources it offers ✓ **KEY**

Working within the client's belief system respects her values and can draw on her faith as a resource.

C. Refer her to clergy because religious concerns fall outside counseling scope

Spiritual concerns are within counseling scope; referral based on the topic alone is not warranted.

D. Disclose the counselor's own views so she can weigh both sides of the issue

Sharing personal views on her religion serves the counselor and risks pressuring the client.

Rationale: Counselors avoid imposing their own values, attitudes and beliefs and respect the diversity of clients' spiritual lives. Exploring guilt within the client's framework, including forgiveness or grace in her tradition, honors her worldview and can turn faith into a source of coping.

References: ACA Code of Ethics — A.4.b Personal Values

☐ Approve ☐ Revise ☐ Retire Notes:

A 15-year-old discloses repeated nonsuicidal self-injury and says, "When everything builds up, it is the only thing that makes the feeling stop." Which function of self-injury does her statement MOST clearly reflect?

A. A wish to end her life that has not yet been spoken aloud

NSSI by definition lacks suicidal intent; her words describe emotional relief, not a wish to die.

B. A bid to get attention from peers and adults around her

NSSI is often hidden; attributing it to attention-seeking is a common, inaccurate assumption here.

C. Relief from intense negative emotion (affect regulation) ✓ **KEY**

Affect regulation, gaining relief from overwhelming feelings, is the most common reported function of NSSI.

D. Self-punishment for having broken a rule in her family

Self-punishment is a possible function, but she describes stopping a feeling, not punishing herself.

Rationale: Research consistently finds that the most common function of nonsuicidal self-injury is affect regulation: obtaining relief from a negative feeling or cognitive state. DSM-5-TR lists this expectation in its proposed criteria for nonsuicidal self-injury disorder. Treatment builds alternative regulation skills, as in DBT.

References: DSM-5-TR — Conditions for Further Study: Nonsuicidal Self-Injury Disorder, criterion B

☐ Approve ☐ Revise ☐ Retire Notes:

A college student shows the counselor healed marks on her forearm and says she has hurt herself during stressful weeks for two years. She denies wanting to die. Which counselor response is BEST?

A. Ask her to sign a no-harm contract before agreeing to continue in counseling

No-harm contracts lack evidence of benefit and can shut down honest disclosure.

B. Arrange an inpatient admission because self-injury signals imminent danger

NSSI without suicidal intent or other acute risk does not by itself call for hospitalization.

C. Respond calmly, explore what the behavior does for her, and assess suicide risk ✓ **KEY**

A calm, curious stance keeps her engaged, and NSSI still raises suicide risk, so risk is assessed.

D. Avoid discussing the marks so that attention does not reinforce the behavior

Ignoring the disclosure misses assessment and can feel dismissive, discouraging further honesty.

Rationale: A respectful, nonjudgmental response to self-injury builds trust and allows functional assessment. NSSI is distinct from a suicide attempt but is a strong risk factor for later suicidal behavior, so counselors also assess suicidal ideation and behavior directly, for example with the C-SSRS.

References: C-SSRS — Suicidal behavior section: distinguishing nonsuicidal self-injurious behavior from suicide attempts

☐ Approve ☐ Revise ☐ Retire Notes:

A counselor is using cognitive behavioral therapy for insomnia (CBT-I) with a client who lies awake in bed for hours most nights. Which instruction is part of the stimulus control component?

A. Avoid caffeine after noon and keep the bedroom cool, dark and quiet

These are sleep hygiene recommendations, a separate and weaker component of insomnia care.

B. Limit time in bed to the hours actually slept, then extend it slowly

This describes sleep restriction, a different CBT-I component that consolidates sleep.

C. Practice progressive muscle relaxation for twenty minutes at bedtime

Relaxation training is a separate CBT-I component aimed at lowering arousal.

D. Get out of bed if unable to sleep and return only once feeling sleepy ✓ **KEY**

Stimulus control re-links the bed with sleep by leaving it when awake and returning when sleepy.

Rationale: Stimulus control therapy aims to restore the bed and bedroom as cues for sleep: go to bed only when sleepy, use the bed only for sleep and sex, leave the bed when unable to sleep, keep a fixed wake time and avoid daytime naps. It is combined with sleep restriction, cognitive work and education in CBT-I, the recommended first-line treatment for chronic insomnia.

References: AASM Clinical Practice Guideline (Insomnia) — Behavioral and psychological treatments for chronic insomnia (2021): CBT-I recommended; stimulus control as a component · NICE guidelines — Clinical Knowledge Summary: Insomnia, CBT-I as first-line management of long-term insomnia

☐ Approve ☐ Revise ☐ Retire Notes:

A client reports difficulty staying asleep, with daytime fatigue that affects her work. The problem is not explained by a medical condition, substance or other sleep disorder. According to DSM-5-TR, how often and for how long must the sleep difficulty occur to meet criteria for insomnia disorder?

A. At least 3 nights a week for at least 3 months ✓ KEY

DSM-5-TR requires the difficulty at least 3 nights per week, present for at least 3 months.

B. At least 5 nights a week for at least 2 weeks

This is far shorter than required; two weeks fits a brief, situational sleep problem.

C. At least 1 night a week for at least 6 months

One night a week is below the frequency threshold for insomnia disorder.

D. At least 4 nights a week for at least 1 month

One month falls short of the 3-month duration; brief insomnia is coded as other specified insomnia disorder.

Rationale: DSM-5-TR insomnia disorder requires dissatisfaction with sleep quantity or quality (trouble initiating sleep, maintaining sleep or early waking) occurring at least 3 nights per week for at least 3 months, despite adequate opportunity for sleep, with clinically significant distress or impairment.

References: DSM-5-TR — Insomnia Disorder: criteria C and D (frequency and duration)

☐ Approve ☐ Revise ☐ Retire Notes:

A client with bipolar I disorder who takes lithium had a stomach virus with vomiting for two days. In session she has a coarse hand tremor, seems confused, and walks unsteadily. What should the counselor do?

A. Reassure her that tremor is a common side effect and continue the planned session

A mild tremor can occur on lithium, but confusion and unsteady gait after dehydration suggest toxicity.

B. Suggest she skip her next lithium dose until she feels better, then call her prescriber

Advising medication changes is outside counselor scope, and the symptoms need urgent evaluation now.

C. Arrange immediate medical evaluation because the symptoms may signal lithium toxicity ✓ KEY

Dehydration can raise lithium levels; confusion, coarse tremor and ataxia warrant urgent medical care.

D. Explore whether anxiety about her illness is causing her shaking and trouble focusing

Attributing these signs to anxiety risks missing a medical emergency.

Rationale: Lithium has a narrow therapeutic range and requires regular blood-level monitoring. Vomiting, diarrhea, dehydration and some medications can raise levels into the toxic range, producing coarse tremor, confusion, ataxia and gastrointestinal symptoms. Counselors do not adjust medication but recognize warning signs and arrange urgent medical evaluation.

References: NICE guidelines — Bipolar disorder (CG185): lithium monitoring and toxicity

☐ Approve ☐ Revise ☐ Retire Notes:

A client with major depressive disorder started an SSRI prescribed by her physician 10 days ago. She says, "It is not doing anything, so I am going to stop taking it." What is the counselor's BEST response?

A. Agree that stopping makes sense since there has been no improvement

Ten days is too early to judge, and stopping without the prescriber is not the counselor's call to endorse.

B. Explain SSRIs often take weeks to work; urge a talk with the prescriber ✓ **KEY**

SSRI benefit usually builds over several weeks, and medication decisions belong with the prescriber.

C. Suggest she ask her physician for a benzodiazepine to work more quickly

Recommending a specific drug is outside counselor scope, and benzodiazepines carry dependence risk.

D. Advise her to take half the dose for a week to see if she feels different

Counselors do not advise dose changes; that is a prescriber decision.

Rationale: SSRIs typically take several weeks to produce a full antidepressant effect, although side effects can appear sooner. Counselors provide basic education, support adherence and coordinate with the prescriber, but they do not recommend starting, stopping or changing specific medications.

References: APA CPG — Depression across the lifespan: second-generation antidepressants · ACA Code of Ethics — C.2.a Boundaries of Competence

☐ Approve ☐ Revise ☐ Retire Notes:

A 9-year-old with ADHD performs at grade level when given extended test time and a seat near the teacher. He does not need specially designed instruction. Which school plan BEST fits his needs?

A. An Individualized Education Program under IDEA

An IEP is for students who need specially designed instruction, which he does not.

B. A gifted education plan for advanced placement

Nothing indicates a need for gifted programming.

C. A behavior intervention plan for disruptive acts

A behavior plan targets problem behaviors; the need described is for accommodations.

D. A Section 504 plan for classroom accommodations ✓ **KEY**

Section 504 provides accommodations for a disability that limits learning without special education.

Rationale: Section 504 of the Rehabilitation Act covers students with a disability that substantially limits a major life activity such as learning, and provides accommodations such as extended time or preferential seating. IDEA provides an IEP only for students in eligible categories who need specially designed instruction.

References: Gladding (Counseling: A Comprehensive Profession) — School counseling: Section 504 plans and IEPs under IDEA

☐ Approve ☐ Revise ☐ Retire Notes:

An 8-year-old has missed most of three weeks of school, crying and complaining of stomachaches each morning that ease once his mother lets him stay home. He worries something bad will happen to her while he is away. Pediatric causes have been ruled out. Which plan is MOST appropriate?

A. Arrange home-based instruction until his worry about his mother eases on its own

Staying home relieves anxiety in the short term but reinforces avoidance and lets the fear grow.

B. A gradual return to school with exposure, coping skills and school collaboration ✓ **KEY**

Graduated exposure with parent and school coordination is the evidence-based approach to anxiety-based school refusal.

C. Tell his mother to keep him home on any morning that he has physical complaints

Making symptoms the ticket to stay home rewards them and maintains the cycle.

D. Delay any school plan until weekly play therapy has resolved the underlying fear

Each week out of school makes return harder; return should begin alongside treatment.

Rationale: School refusal driven by separation anxiety is maintained by avoidance, because staying home brings immediate relief. Treatment uses CBT with graduated exposure to attending school, coping skills, parent training to reduce accommodation, and close collaboration with school staff to support a prompt, stepwise return.

References: DSM-5-TR — Separation Anxiety Disorder: reluctance or refusal to go to school; physical complaints on anticipated separation

☐ Approve ☐ Revise ☐ Retire Notes:

A 31-year-old Army veteran meets criteria for PTSD after combat deployments. He is willing to engage in therapy and asks what treatment has the strongest support. According to the VA/DoD clinical practice guideline, which approach is recommended as first-line?

A. Individual, manualized trauma-focused therapy such as PE or CPT ✓ **KEY**

The VA/DoD guideline recommends manualized trauma-focused psychotherapies such as PE, CPT and EMDR first.

B. A benzodiazepine to reduce his hyperarousal and improve sleep

The guideline recommends against benzodiazepines for PTSD because of harm and lack of benefit.

C. Single-session psychological debriefing about his deployments

Psychological debriefing is not supported for PTSD and may worsen outcomes.

D. Nondirective supportive counseling without trauma processing

Supportive counseling is less effective than trauma-focused therapy and is not first-line.

Rationale: The VA/DoD PTSD guideline recommends individual, manualized trauma-focused psychotherapies, chiefly prolonged exposure (PE), cognitive processing therapy (CPT) and EMDR, over pharmacotherapy. It recommends against benzodiazepines for PTSD. Counselors working with veterans should know these options and refer when they are not trained to deliver them.

References: VA/DoD CPG — Management of PTSD and Acute Stress Disorder: trauma-focused psychotherapy recommendations

☐ Approve ☐ Revise ☐ Retire Notes:

A Marine veteran says his worst memory is not being in danger himself but following an order that led to civilian deaths. He is consumed by guilt and shame, believes he is "a bad person," and has pulled away from his faith community. Which concept BEST describes his distress?

A. Moral injury from acts that violated his own deeply held moral beliefs ✓ KEY

Moral injury describes guilt, shame and loss of trust after perpetrating or witnessing acts that violate one's moral code.

B. Compassion fatigue from caring for wounded civilians and fellow troops

Compassion fatigue affects helpers worn down by caregiving; his distress centers on his own actions.

C. Survivor guilt from having lived through events in which comrades died

Survivor guilt centers on surviving when others did not; his guilt is about what he did.

D. Adjustment disorder triggered by the stress of returning to civilian life

His distress is tied to a specific morally violating combat event, not to the transition home.

Rationale: Moral injury, described by Litz and colleagues, is lasting distress after perpetrating, failing to prevent or witnessing acts that transgress deeply held moral beliefs. It shows up mainly as guilt, shame, self-condemnation and spiritual or relational withdrawal rather than fear. It overlaps with PTSD criterion D (negative beliefs about oneself, guilt, shame) but is not a DSM diagnosis.

References: DSM-5-TR — PTSD criterion D: persistent negative beliefs about oneself and persistent guilt or shame

☐ Approve ☐ Revise ☐ Retire Notes:

A 44-year-old with major depressive disorder has had a depressive episode each of the last three winters, with full remission every spring. A counselor is considering the "with seasonal pattern" specifier. Which finding would argue MOST strongly AGAINST using it?

A. She sleeps more and craves carbohydrates in her episodes

Hypersomnia and carbohydrate craving are common in winter-pattern episodes and are consistent with the specifier.

B. She lives far north, where winter daylight is quite short

Higher latitude is associated with winter-pattern depression and does not argue against the specifier.

C. Her construction job lays her off each winter, without pay ✓ KEY

DSM-5-TR excludes cases in which an obvious seasonal psychosocial stressor, such as regular seasonal unemployment, explains the timing.

D. Each episode has ended completely by the following April

Full remission at a characteristic time of year is one of the requirements for the specifier.

Rationale: The seasonal pattern specifier requires a regular temporal relationship between episode onset and a particular time of year, full remission (or a switch to mania or hypomania) at a characteristic time, two seasonal episodes in the last 2 years with no nonseasonal episodes in that period, and seasonal episodes substantially outnumbering nonseasonal ones over the lifetime. It does not apply when an obvious season-linked psychosocial stressor, such as being unemployed every winter, accounts for the pattern.

References: DSM-5-TR — Specifiers for Depressive Disorders: with seasonal pattern

☐ Approve ☐ Revise ☐ Retire Notes:

A 29-year-old developed a major depressive episode during the second trimester of her first pregnancy. Her midwife assumed the "with peripartum onset" specifier could only be used after delivery. Which statement is accurate under DSM-5-TR?

A. It applies to onset in pregnancy or within 4 weeks after delivery ✓ KEY

DSM-5-TR applies the peripartum onset specifier when mood symptoms begin during pregnancy or in the 4 weeks following delivery.

B. It applies only to episodes that begin after the baby is delivered

DSM-5 replaced the older "postpartum onset" specifier with "peripartum onset" to include onset during pregnancy.

C. It applies to any onset up to one year following the delivery date

One year is used in some research and screening, but the DSM-5-TR specifier window is 4 weeks after delivery.

D. It can be used only when the episode includes psychotic features

Psychotic features are not required; peripartum episodes may occur with or without psychosis.

Rationale: DSM-5-TR uses "with peripartum onset" when the onset of mood symptoms occurs during pregnancy or in the 4 weeks following delivery. A substantial share of postpartum major depressive episodes actually begin during pregnancy, which is why the earlier postpartum-only specifier was broadened. Peripartum episodes should be distinguished from the brief, milder "baby blues."

References: DSM-5-TR — Specifiers for Depressive Disorders: with peripartum onset

☐ Approve ☐ Revise ☐ Retire Notes:

A client in a current major depressive episode also says she feels keyed up and tense most days and has a nagging fear that something awful is about to happen. She does not meet criteria for an anxiety disorder. Which specifier BEST captures these features?

A. With mixed features

"With mixed features" refers to manic or hypomanic symptoms during a depressive episode, not anxiety.

B. With anxious distress ✓ KEY

Feeling keyed up or tense and fearing that something awful may happen are two of the five anxious distress symptoms; two are required.

C. With melancholic features

Melancholic features center on near-total loss of pleasure, early-morning awakening, and marked psychomotor change.

D. With atypical features

Atypical features involve mood reactivity plus symptoms such as hypersomnia, leaden paralysis, and rejection sensitivity.

Rationale: The anxious distress specifier requires at least two of five symptoms during most days of the episode: feeling keyed up or tense, feeling unusually restless, difficulty concentrating because of worry, fear that something awful may happen, and feeling one might lose control. Anxious distress is linked to higher suicide risk, longer illness, and poorer treatment response, so it is worth assessing even when no separate anxiety disorder is present.

References: DSM-5-TR — Specifiers for Depressive Disorders: with anxious distress

☐ Approve ☐ Revise ☐ Retire Notes:

For about 3 years, a 34-year-old graphic designer has swung between stretches of high energy, talkativeness, and big plans and stretches of low mood and fatigue. Neither the highs nor the lows have ever met full criteria for a hypomanic, manic, or major depressive episode, and she is rarely symptom-free for more than a month. Which diagnosis BEST fits?

A. Bipolar II disorder

Bipolar II requires at least one full hypomanic episode and at least one major depressive episode.

B. Persistent depressive disorder

Persistent depressive disorder involves chronic depressed mood without the recurring hypomanic symptoms she reports.

C. Bipolar I disorder, mild

Bipolar I requires at least one full manic episode, which has never occurred.

D. Cyclothymic disorder ✓ KEY

Two or more years of numerous subthreshold hypomanic and depressive periods, with no gap over 2 months and no full episodes, fits cyclothymic disorder.

Rationale: Cyclothymic disorder requires at least 2 years (1 year in children and adolescents) of numerous periods with hypomanic symptoms and numerous periods with depressive symptoms that never meet full episode criteria. Symptoms are present at least half the time, the person is never symptom-free for more than 2 months, and criteria for a major depressive, manic, or hypomanic episode have never been met.

References: DSM-5-TR — Cyclothymic Disorder, Criteria A–C

☐ Approve ☐ Revise ☐ Retire Notes:

Three weeks after starting an antidepressant for depression, a 38-year-old develops elevated mood, grandiosity, little need for sleep, and reckless spending. His prescriber stops the medication, but the full manic syndrome continues well beyond the drug's expected physiological effect. Which statement is MOST accurate?

A. Mania that starts on an antidepressant is coded only as medication-induced

Onset during antidepressant treatment does not by itself settle the question; persistence beyond the drug effect matters.

B. Persisting at full level past the drug effect supports bipolar I disorder ✓ KEY

DSM-5-TR states that a full manic episode persisting beyond the physiological effect of antidepressant treatment is sufficient evidence for bipolar I.

C. Bipolar II should be diagnosed because a medication triggered the mood

Bipolar II requires hypomanic, not manic, episodes; the trigger does not change a full manic syndrome into hypomania.

D. No bipolar diagnosis can be made until a medication-free year has passed

DSM-5-TR sets no medication-free waiting period; the criterion is persistence beyond the physiological effect.

Rationale: DSM-5-TR notes that a full manic episode emerging during antidepressant treatment (medication or electroconvulsive therapy) that persists at a fully syndromal level beyond the physiological effect of that treatment is sufficient evidence for a manic episode and therefore a bipolar I diagnosis. If the symptoms resolve as the drug effect wears off, a substance/medication-induced bipolar and related disorder is more appropriate.

References: DSM-5-TR — Bipolar I Disorder, Manic Episode Criterion D note on antidepressant treatment

☐ Approve ☐ Revise ☐ Retire Notes:

For 8 months, a 47-year-old has avoided buses, crowded grocery stores, and long checkout lines. She fears that if she suddenly felt faint or lost bladder control, escape would be hard and no one would help. She has never had an unexpected panic attack and is not worried about being judged by others. Which diagnosis BEST fits?

A. Specific phobia, situational type

Specific phobia centers on one circumscribed situation; fear across several situations for this escape-related reason points to agoraphobia.

B. Social anxiety disorder

Social anxiety centers on fear of scrutiny or negative evaluation, which she denies.

C. Agoraphobia ✓ KEY

Fear in two or more agoraphobic situations because escape may be difficult or help unavailable if incapacitating symptoms occur fits agoraphobia.

D. Panic disorder

Panic disorder requires recurrent unexpected panic attacks, which she has never had.

Rationale: Agoraphobia requires marked fear or anxiety about at least two of five situations: public transportation, open spaces, enclosed spaces, standing in line or being in a crowd, and being outside the home alone. The fear is driven by thoughts that escape might be difficult or help unavailable if panic-like or other incapacitating or embarrassing symptoms occur. Since DSM-5, agoraphobia can be diagnosed without panic disorder, and it typically lasts 6 months or more.

References: DSM-5-TR — Agoraphobia, Criteria A–B

☐ Approve ☐ Revise ☐ Retire Notes:

A 6-year-old has not spoken at school for 4 months, although she talks freely with her parents at home. Her teacher refers her for evaluation of selective mutism. Which additional finding would make selective mutism LESS likely as the explanation?

A. Her family moved recently and she is still learning the school language ✓ KEY

DSM-5-TR excludes failure to speak that is due to lack of knowledge of, or comfort with, the language required in that setting.

B. She whispers to one close friend at recess but to no one else there

Limited speech with one trusted peer is common in selective mutism and does not argue against it.

C. Her silence has lasted well beyond the first month of the school year

Duration of at least 1 month, not limited to the first month of school, is a requirement and supports the diagnosis.

D. Her parents describe her as very shy and anxious in new situations

Shyness and social anxiety are commonly associated with selective mutism.

Rationale: Selective mutism is a consistent failure to speak in specific social situations where speech is expected, such as school, despite speaking in other situations. It must last at least 1 month (not limited to the first month of school), interfere with achievement or communication, and not be due to lack of knowledge of or comfort with the spoken language. Counselors working with newly arrived immigrant children should consider normal second-language "silent periods" before diagnosing.

References: DSM-5-TR — Selective Mutism, Criteria A–D

☐ Approve ☐ Revise ☐ Retire Notes:

Five weeks after being unexpectedly laid off from a job of 20 years, a 56-year-old reports sadness, frequent replaying of the termination meeting, poor sleep, and withdrawal from friends. His distress is out of proportion to what is typical and is affecting family life. Criteria for a major depressive episode are not met. Which diagnosis is MOST appropriate?

A. Posttraumatic stress disorder

PTSD requires exposure to actual or threatened death, serious injury, or sexual violence; a layoff does not meet Criterion A.

B. Acute stress disorder

Acute stress disorder has the same Criterion A trauma requirement as PTSD, which a layoff does not satisfy.

C. Major depressive disorder

The stem states that criteria for a major depressive episode are not met.

D. Adjustment disorder ✓ KEY

Distressing symptoms beginning within 3 months of an identifiable, non-traumatic stressor, out of proportion and impairing, fit adjustment disorder.

Rationale: Adjustment disorder involves emotional or behavioral symptoms that begin within 3 months of an identifiable stressor, are out of proportion to the stressor or impair functioning, and do not meet criteria for another mental disorder. Unlike PTSD and acute stress disorder, the stressor need not involve death, serious injury, or sexual violence. Symptoms should resolve within 6 months after the stressor or its consequences end.

References: DSM-5-TR — Adjustment Disorders, Criteria A–E

☐ Approve ☐ Revise ☐ Retire Notes:

A 4-year-old was adopted at age 2 after living in an understaffed orphanage. Her adoptive parents say she happily wanders off with strangers at the park, climbs onto unfamiliar adults' laps, and does not look back to check on them. Which diagnosis BEST fits this pattern?

A. Reactive attachment disorder, persistent

Reactive attachment disorder involves an inhibited, emotionally withdrawn child who rarely seeks or responds to comfort.

B. Disinhibited social engagement disorder ✓ KEY

Overly familiar behavior with strangers, wandering off without checking back, and a history of insufficient care define this disorder.

C. Attention-deficit/hyperactivity disorder

ADHD impulsivity is not specific to indiscriminate social approach and is not tied to a history of social neglect.

D. Autism spectrum disorder, level 1

Autism involves deficits in social communication and restricted interests, not eager, indiscriminate approach to strangers.

Rationale: Both reactive attachment disorder and disinhibited social engagement disorder follow a pattern of extremely insufficient care, such as neglect or institutional rearing. Reactive attachment disorder is an inhibited, withdrawn pattern with minimal comfort seeking, while disinhibited social engagement disorder is a pattern of culturally inappropriate, overly familiar behavior with unfamiliar adults and diminished checking back with caregivers. The child must have a developmental age of at least 9 months.

References: DSM-5-TR — Disinhibited Social Engagement Disorder, Criteria A–C

☐ Approve ☐ Revise ☐ Retire Notes:

A 22-year-old man who lifts weights daily is convinced his body looks small and weak, although friends describe him as very muscular. He checks mirrors for hours, wears baggy layers to hide his build, and skips social events. He is not focused on body fat or thinness. Which diagnosis BEST fits?

A. Anorexia nervosa, restricting type

Anorexia nervosa involves significantly low body weight and fear of weight gain, not a belief of being too small.

B. Obsessive-compulsive disorder

When obsessions and rituals center only on perceived appearance flaws, body dysmorphic disorder is the more specific diagnosis.

C. Body dysmorphic disorder ✓ KEY

Preoccupation with a perceived appearance flaw plus repetitive checking fits BDD; the belief that his build is too small calls for the muscle dysmorphia specifier.

D. Illness anxiety disorder

Illness anxiety disorder centers on fear of having a serious illness, not on perceived appearance flaws.

Rationale: Body dysmorphic disorder is preoccupation with perceived defects in appearance that are not observable or appear slight to others, along with repetitive behaviors such as mirror checking or comparing, causing distress or impairment. The "with muscle dysmorphia" specifier applies when the person believes his or her build is too small or insufficiently muscular. DSM-5-TR also specifies the degree of insight.

References: DSM-5-TR — Body Dysmorphic Disorder, Criteria A–D and muscle dysmorphia specifier

☐ Approve ☐ Revise ☐ Retire Notes:

A 66-year-old describes himself as "a collector." Which finding MOST clearly points to hoarding disorder rather than ordinary collecting?

A. Clutter fills living areas, and parting with items causes distress ✓ KEY

Persistent difficulty discarding, distress at parting with items, and clutter that blocks the use of living spaces are the core of hoarding disorder.

B. He owns several hundred items of one specific type in his home

A large number of items is common in collecting; what matters is whether clutter compromises living areas.

C. He spends a good share of his spare money on additions each year

Spending on a hobby is not diagnostic; collectors often budget substantial amounts.

D. He feels warm sentimental attachment to items that were gifts

Sentimental attachment to meaningful objects is normal and not in itself a sign of hoarding disorder.

Rationale: Hoarding disorder is persistent difficulty discarding or parting with possessions, regardless of their actual value, due to a perceived need to save them and distress associated with discarding. The result is an accumulation that congests and clutters active living areas and substantially compromises their intended use. Collecting, by contrast, tends to be organized and intentional and does not block the use of living space. A specifier notes whether excessive acquisition is present.

References: DSM-5-TR — Hoarding Disorder, Criteria A–C

☐ Approve ☐ Revise ☐ Retire Notes:

Two days after stopping several weeks of heavy methamphetamine use, a client reports feeling deeply down and exhausted. He is sleeping for long stretches, has vivid unpleasant dreams, and is eating much more than usual. Which withdrawal syndrome do these symptoms MOST closely match?

A. Opioid withdrawal

Opioid withdrawal features flu-like signs such as tearing, runny nose, yawning, dilated pupils, muscle aches, and diarrhea.

B. Cannabis withdrawal

Cannabis withdrawal typically brings irritability, anxiety, restlessness, trouble sleeping, and decreased appetite.

C. Alcohol withdrawal

Alcohol withdrawal involves autonomic hyperactivity, tremor, insomnia, and possible seizures, not hypersomnia with increased appetite.

D. Stimulant withdrawal ✓ KEY

Dysphoric mood with fatigue, vivid unpleasant dreams, sleep change, and increased appetite matches stimulant withdrawal.

Rationale: Stimulant withdrawal requires dysphoric mood plus at least two of: fatigue; vivid, unpleasant dreams; insomnia or hypersomnia; increased appetite; and psychomotor retardation or agitation, developing within hours to days after stopping. The "crash" can bring marked depression, so counselors should screen for suicidal thinking during early stimulant withdrawal.

References: DSM-5-TR — Stimulant Withdrawal, Criteria A–B

☐ Approve ☐ Revise ☐ Retire Notes:

After losing a large sum at an online casino, a client logs back in the next morning, certain that a few more bets will win back what he lost. Which DSM-5-TR gambling disorder criterion does this behavior illustrate?

A. Tolerance

Tolerance in gambling disorder is needing to gamble with increasing amounts of money to achieve the desired excitement.

B. Chasing losses ✓ KEY

Returning another day to get even after losing money is the "chasing" criterion of gambling disorder.

C. Relying on bailouts

This criterion refers to relying on others for money to relieve a desperate financial situation caused by gambling.

D. Withdrawal

Withdrawal refers to restlessness or irritability when attempting to cut down or stop gambling.

Rationale: Gambling disorder is the only behavioral (non-substance) addiction in the DSM-5-TR substance-related and addictive disorders chapter. It requires at least four of nine criteria within 12 months, including chasing losses, needing larger amounts to get the desired excitement, restlessness when cutting down, lying to conceal gambling, and relying on others for bailouts. Chasing is considered one of the hallmark features of problem gambling.

References: DSM-5-TR — Gambling Disorder, Criterion A6 (chasing losses)

☐ Approve ☐ Revise ☐ Retire Notes:

A 30-year-old with opioid use disorder says he is not ready to stop using but wants to keep seeing the counselor. Which counselor action BEST reflects a harm reduction approach?

A. Require a period of abstinence before further sessions are offered

Making abstinence a condition of care pushes away clients who are not ready and conflicts with harm reduction.

B. Confront his denial each session until he admits he needs to quit

Confrontational approaches tend to raise resistance and are not part of harm reduction or motivational work.

C. Help him get naloxone and plan not to use alone, while exploring change ✓ KEY

Reducing overdose risk now, while keeping him engaged and exploring change, is the essence of harm reduction.

D. Refer him to a mutual-help group and end individual counseling for now

Mutual-help groups can help, but ending counseling removes support when he is open to engagement.

Rationale: Harm reduction meets clients where they are, aiming to reduce the health and social harms of substance use without requiring abstinence first. For opioid use, this includes naloxone access, not using alone, and sterile supplies, alongside offering medications for opioid use disorder. Staying engaged also leaves room for motivational work that may later lead to reduced use or recovery.

References: SAMHSA TIP 63 — Medications for Opioid Use Disorder: overdose prevention and naloxone

► **Check:** Confirm that SAMHSA TIP 63 is the best approved citation for naloxone distribution as a harm reduction practice.

☐ Approve ☐ Revise ☐ Retire Notes:

Under DSM-5-TR, the severity level of intellectual developmental disorder (mild, moderate, severe, profound) is determined mainly by which factor?

A. Adaptive functioning in conceptual, social and practical areas ✓ KEY

DSM-5-TR defines severity by adaptive functioning, because that determines the level of support needed.

B. The full-scale IQ score range obtained on standardized testing

IQ is part of assessing intellectual deficits, but DSM-5 moved severity away from IQ ranges.

C. The age at which the intellectual deficits were first observed

Onset during the developmental period is required, but age of onset does not set severity.

D. Whether a specific genetic or medical cause has been identified

Etiology may be noted, but it does not determine the severity level.

Rationale: Intellectual developmental disorder requires deficits in intellectual functions and in adaptive functioning, with onset during the developmental period. DSM-5-TR bases the severity specifiers on adaptive functioning across conceptual, social, and practical domains rather than IQ scores, because adaptive functioning determines the level of supports required. IQ scores remain part of the assessment but are less valid at the lower end of the range.

References: DSM-5-TR — Intellectual Developmental Disorder, severity specifiers

☐ Approve ☐ Revise ☐ Retire Notes:

An 11-year-old has had eye blinking and shoulder shrugging since age 9, and over the past year he has also developed frequent throat clearing and sniffing. The tics wax and wane but have never stopped for long. He has never used obscene words. Which diagnosis BEST fits?

A. Persistent (chronic) motor tic disorder

Persistent tic disorder involves motor or vocal tics, but not both; he has both.

B. Provisional tic disorder

Provisional tic disorder applies when tics have been present for less than 1 year.

C. Stereotypic movement disorder

Stereotypies are rhythmic, seemingly purposeless movements that begin earlier and lack the vocal tics described.

D. Tourette's disorder ✓ KEY

Multiple motor tics and at least one vocal tic, present for more than 1 year with onset before 18, meet criteria for Tourette's disorder.

Rationale: Tourette's disorder requires both multiple motor tics and one or more vocal tics at some time during the illness, although not necessarily at the same time. Tics may wax and wane but must have persisted for more than 1 year since the first tic, with onset before age 18. Coprolalia (uttering obscenities) occurs in only a minority of cases and is not required.

References: DSM-5-TR — Tourette's Disorder, Criteria A–D

☐ Approve ☐ Revise ☐ Retire Notes:

For 8 months, a 58-year-old accountant has been certain that a neighbor is poisoning his garden and reading his mail. He has filed police reports and installed cameras. He reports no hallucinations, his speech is organized, and he continues to work effectively. No mood episodes or substance causes are present. Which diagnosis is MOST appropriate?

A. Schizophrenia

Schizophrenia requires two or more Criterion A symptoms and usually marked functional decline; he has a single fixed delusion and functions well.

B. Delusional disorder ✓ KEY

One or more delusions lasting 1 month or longer, without other Criterion A symptoms and with functioning not markedly impaired, fits delusional disorder (persecutory type).

C. Paranoid personality disorder

Paranoid personality disorder involves pervasive distrust and suspiciousness, not a fixed false belief held with delusional conviction.

D. Brief psychotic disorder

Brief psychotic disorder lasts at least 1 day but less than 1 month, and he has had symptoms for 8 months.

Rationale: Delusional disorder requires one or more delusions lasting at least 1 month, Criterion A for schizophrenia never having been met, and functioning that is not markedly impaired apart from the impact of the delusion. Behavior is not obviously bizarre. Subtypes include persecutory, jealous, erotomanic, grandiose, and somatic, and DSM-5-TR adds a "with bizarre content" specifier.

References: DSM-5-TR — Delusional Disorder, Criteria A–E and persecutory type

☐ Approve ☐ Revise ☐ Retire Notes:

Over 2 years of illness, a 33-year-old has had major depressive episodes for most of the time, often with voices and persecutory beliefs. Between episodes there was a 3-week stretch when the voices and delusions continued while her mood was stable. She has never had mania, and substances are ruled out. Which diagnosis is MOST appropriate?

A. Major depressive disorder with psychotic features

In MDD with psychotic features, psychosis occurs only during mood episodes; here it persisted for weeks without one.

B. Schizophrenia with comorbid depressive episodes

Schizophrenia fits when mood episodes are present for only a minority of the illness; hers spanned most of it.

C. Schizoaffective disorder, depressive type ✓ KEY

Psychosis for 2 or more weeks without a mood episode, with mood episodes present for most of the illness, fits schizoaffective disorder.

D. Bipolar I disorder with psychotic features

Bipolar I requires a manic episode, which has never occurred.

Rationale: Schizoaffective disorder requires an uninterrupted period of illness in which a major mood episode co-occurs with Criterion A of schizophrenia, plus delusions or hallucinations for 2 or more weeks in the absence of a major mood episode at some point in the lifetime illness. Mood episodes must be present for the majority of the total duration of the active and residual illness. The depressive type applies when only major depressive episodes have occurred.

References: DSM-5-TR — Schizoaffective Disorder, Criteria A–C

☐ Approve ☐ Revise ☐ Retire Notes:

A 26-year-old seeks counseling for "bipolar mood swings." She describes intense anger, chronic emptiness, and fear of being left by her partner. A counselor is weighing borderline personality disorder against a bipolar disorder. Which finding MOST supports borderline personality disorder?

A. Impulsive spending sprees during periods of high distress

Impulsive spending occurs in both borderline personality disorder and manic or hypomanic episodes, so it does not discriminate.

B. Mood shifts lasting hours, set off by perceived rejection ✓ KEY

Reactive mood shifts lasting hours and triggered by interpersonal events are typical of BPD, whereas bipolar episodes last days or longer.

C. Several past periods of low mood with fatigue and poor focus

Depressive periods occur in both conditions and do not distinguish them.

D. A biological parent with a documented bipolar I diagnosis

A family history of bipolar disorder raises the likelihood of a bipolar diagnosis rather than supporting BPD.

Rationale: Affective instability in borderline personality disorder involves marked reactivity of mood, with episodes usually lasting a few hours and rarely more than a few days, often triggered by interpersonal stress or perceived abandonment. Bipolar mood episodes are sustained, distinct periods (at least 4 days for hypomania, 1 week for mania) with changes such as decreased need for sleep that are not simply reactions to relationships. The two can co-occur, so careful history taking matters.

References: DSM-5-TR — Borderline Personality Disorder, Differential Diagnosis (bipolar disorders)

☐ Approve ☐ Revise ☐ Retire Notes:

A 35-year-old has few close friends, dresses in an unusual way, and speaks in vague, odd phrases. She believes she can sense what strangers are thinking and that song lyrics on the radio carry special messages for her. Her social anxiety does not ease as she gets to know people. Which personality disorder BEST fits?

A. Schizoid personality disorder

Schizoid personality disorder involves detachment and restricted emotion without the odd beliefs and perceptual distortions described.

B. Avoidant personality disorder

Avoidant personality disorder (Cluster C) involves fear of rejection without magical thinking or ideas of reference.

C. Paranoid personality disorder

Paranoid personality disorder centers on distrust and suspicion of others' motives rather than eccentric beliefs and speech.

D. Schizotypal personality disorder ✓ KEY

Ideas of reference, magical thinking, odd speech and appearance, and persistent social anxiety define schizotypal personality disorder.

Rationale: Schizotypal personality disorder, in Cluster A, combines acute discomfort with close relationships and cognitive or perceptual distortions and eccentric behavior. Features include ideas of reference, odd beliefs or magical thinking, unusual perceptual experiences, odd speech, eccentric appearance, and excessive social anxiety that does not diminish with familiarity. Schizoid personality disorder shares the social detachment but lacks the cognitive and perceptual oddities.

References: DSM-5-TR — Schizotypal Personality Disorder, Criterion A

☐ Approve ☐ Revise ☐ Retire Notes:

A 19-year-old reports recurrent binge-eating episodes followed by purging, along with an intense fear of gaining weight. Her physician notes that her body weight is significantly low for her age, sex, and developmental trajectory. Which diagnosis is MOST appropriate?

A. Anorexia nervosa ✓ KEY

Significantly low body weight with fear of weight gain and recurrent binge-purge behavior fits anorexia nervosa, binge-eating/purging type.

B. Bulimia nervosa

Bulimia nervosa cannot be diagnosed when the disturbance occurs exclusively during episodes of anorexia nervosa.

C. Binge-eating disorder

Binge-eating disorder excludes regular compensatory behaviors such as purging.

D. Purging disorder

Purging disorder (an other specified presentation) involves purging without binge eating, and she does binge; she also meets full criteria for anorexia nervosa.

Rationale: The key feature separating anorexia nervosa from bulimia nervosa is significantly low body weight. Binge eating and purging can occur in both, but when they happen in a person with significantly low weight and fear of weight gain, the diagnosis is anorexia nervosa, binge-eating/purging type. Bulimia nervosa Criterion E states the disturbance does not occur exclusively during episodes of anorexia nervosa.

References: DSM-5-TR — Anorexia Nervosa subtypes; Bulimia Nervosa, Criterion E

☐ Approve ☐ Revise ☐ Retire Notes:

A client in outpatient counseling for bulimia nervosa says her purging has increased over the past month. Today she mentions feeling dizzy and noticing her heart "fluttering" at times. What should the counselor do?

A. Teach slow breathing, since palpitations often reflect anxiety

Attributing cardiac symptoms to anxiety before medical evaluation risks missing a dangerous physical complication.

B. Keep the session focused on the binge triggers and revisit later

Delaying attention to new physical warning signs is unsafe in an eating disorder with frequent purging.

C. Arrange prompt medical evaluation of her physical warning signs ✓ **KEY**

Frequent purging can cause electrolyte disturbances that affect heart rhythm, so dizziness and palpitations warrant prompt medical assessment.

D. Suggest she adjust her own fluid intake and recheck how she feels

Self-management advice is outside the counselor's scope and does not address possible electrolyte or cardiac problems.

Rationale: Bulimia nervosa carries significant medical risk; repeated purging can lead to fluid and electrolyte disturbances that may cause dangerous heart rhythm problems. Counselors should coordinate with medical providers and refer for prompt evaluation when clients report physical warning signs such as dizziness, fainting, or palpitations. Eating disorder care is best delivered by a team that includes medical monitoring.

References: NICE guidelines — NG69 Eating disorders: physical health assessment and monitoring

☐ Approve ☐ Revise ☐ Retire Notes:

A 72-year-old has had a gradual decline in thinking over the past year. His wife reports that he is clear one day and very confused the next, often sees detailed images of children in the house who are not there, and shouts and kicks while dreaming. Mild stiffness and a shuffling gait appeared months after the cognitive changes. Which diagnosis BEST fits?

A. Major NCD due to Alzheimer's disease

Alzheimer's disease usually begins with memory and learning decline; fluctuations, early visual hallucinations, and REM sleep behavior are not core features.

B. Major frontotemporal NCD

Frontotemporal disorder presents with early behavioral disinhibition, apathy, or language decline rather than these features.

C. Major vascular NCD

Vascular disorder is tied to cerebrovascular events and often shows stepwise decline in attention and executive function.

D. Major NCD with Lewy bodies ✓ **KEY**

Fluctuating cognition, recurrent detailed visual hallucinations, and later parkinsonism are core Lewy body features; REM sleep behavior disorder is a suggestive feature.

Rationale: In DSM-5-TR, the core features of neurocognitive disorder with Lewy bodies are fluctuating cognition with marked variations in attention and alertness, recurrent well-formed and detailed visual hallucinations, and spontaneous parkinsonism beginning after cognitive decline. Suggestive features include REM sleep behavior disorder and severe sensitivity to antipsychotic (neuroleptic) medications, which is why accurate identification matters for care coordination.

References: DSM-5-TR — Major or Mild Neurocognitive Disorder With Lewy Bodies, Criteria C–D

☐ Approve ☐ Revise ☐ Retire Notes:

Which presentation MOST suggests vascular neurocognitive disorder rather than neurocognitive disorder due to Alzheimer's disease?

A. Stepwise drops in attention and planning after small strokes ✓ KEY

Onset tied to cerebrovascular events and prominent decline in complex attention and executive function point to vascular NCD.

B. Slow, steady loss of recent memory that began without warning

Insidious onset with gradual, early memory and learning decline is typical of Alzheimer's disease.

C. Early disinhibition and loss of empathy with intact memory

Early behavioral disinhibition and loss of empathy suggest the behavioral variant of frontotemporal NCD.

D. Recurrent visual hallucinations with alertness that fluctuates

Recurrent visual hallucinations with fluctuating alertness are core features of NCD with Lewy bodies.

Rationale: Vascular neurocognitive disorder is suggested when onset is temporally related to one or more cerebrovascular events, or when decline is prominent in complex attention (including processing speed) and frontal-executive function. Its course is often stepwise. Alzheimer's disease, by contrast, has an insidious onset and gradual progression, with memory and learning typically affected early.

References: DSM-5-TR — Major or Mild Vascular Neurocognitive Disorder, Criterion B

☐ Approve ☐ Revise ☐ Retire Notes:

Fourteen months after her husband died, a 67-year-old client accepts the death and has let herself feel its pain. Now she is struggling to manage the finances he always handled, to drive on the highway alone, and to think of herself as "single." Which of Worden's tasks of mourning is she MOST working on?

A. Accepting the reality of the loss

The stem states she already accepts that the death has happened.

B. Processing the pain of the grief

She has allowed herself to feel the pain; her current struggle is with new roles and identity.

C. Adjusting to a world without him ✓ KEY

Taking on roles the deceased filled and redefining her sense of self are the external and internal adjustments of this task.

D. Finding an enduring connection

This task involves keeping a bond with the deceased while moving forward, not mastering new skills and roles.

Rationale: Worden described four tasks of mourning: accepting the reality of the loss, processing the pain of grief, adjusting to a world without the deceased, and finding an enduring connection with the deceased while embarking on a new life. Adjustment includes external changes (new roles and skills), internal changes (sense of self), and spiritual changes (beliefs about the world). Unlike stage models, tasks are active and can be revisited.

References: Berk (Development Through the Lifespan) — Ch. 19: Bereavement, the experience of grieving

► **Check:** Verify that the cited Berk chapter covers Worden's four tasks; Worden's own text is not on the approved source list.

☐ Approve ☐ Revise ☐ Retire Notes:

Six months after his teenage son died, a father has some days filled with looking at photos and crying, and other days focused on returning to work and learning to cook for his family. He worries that this "back and forth" means he is grieving the wrong way. Which framework BEST normalizes his experience?

A. Kübler-Ross's stages of dying, read as a fixed sequence

Kübler-Ross described reactions of dying patients; read as a fixed sequence, it would frame his back-and-forth as a problem.

B. Stroebe and Schut's dual process model of coping with loss ✓ KEY

The dual process model describes healthy oscillation between loss-oriented and restoration-oriented coping.

C. Bowlby's phases of mourning, ending in reorganization

Bowlby's phases describe a general progression and do not center on alternating between two kinds of coping.

D. Caplan's crisis theory, with its time-limited disequilibrium

Crisis theory addresses short-term disequilibrium after an event, not the ongoing rhythm of bereavement coping.

Rationale: Stroebe and Schut's dual process model proposes that bereaved people oscillate between loss-oriented coping (grief work, yearning, reviewing the relationship) and restoration-oriented coping (attending to life changes, new roles, distraction). Moving back and forth, including taking breaks from grief, is considered adaptive. Kübler-Ross's stages (denial, anger, bargaining, depression, acceptance) came from work with dying patients and are not a required sequence for the bereaved.

References: Berk (Development Through the Lifespan) — Ch. 19: Bereavement, the dual-process model of coping with loss

► **Check:** Berk is cited as "Ch. 19"; confirm the chapter number for the edition in use.

☐ Approve ☐ Revise ☐ Retire Notes:

According to Caplan's crisis theory, which statement BEST describes a crisis?

A. A time-limited upset when usual coping fails, which may lead to growth ✓ KEY

Caplan described crisis as a temporary disequilibrium when customary problem solving fails, with potential for growth or deterioration.

B. A lasting pattern of poor coping rooted in long-standing personality traits

Crisis is an acute, time-limited state, not an enduring personality pattern.

C. A mental disorder that calls for formal diagnosis and long-term therapy

A crisis is a normal human reaction, not a mental disorder, and calls for brief, focused intervention.

D. A reaction set by how severe the event is, apart from how it is perceived

Crisis theory stresses the person's perception of the event and coping resources, not the event alone.

Rationale: Gerald Caplan described a crisis as a state of psychological disequilibrium that occurs when a person faces an obstacle to important life goals that cannot be overcome with customary problem-solving methods. Crises are time-limited, typically resolving within weeks, and the outcome can be either growth and improved coping or deterioration. This is why early, brief intervention is emphasized.

References: Gladding (Counseling: A Comprehensive Profession) — Crisis counseling: the nature of crises

► **Check:** Confirm Gladding's crisis counseling coverage of Caplan; no crisis-theory text (e.g., James & Gilliland) is on the approved list.

☐ Approve ☐ Revise ☐ Retire Notes:

Four months after a flood, the volunteers and news crews have left a small town. Residents who once worked side by side now voice anger at slow insurance payouts and agency red tape, and many feel forgotten. In the commonly used phases of community response to disaster, which phase is the town MOST likely in?

A. Heroic phase

The heroic phase occurs during and right after impact, when people act to save lives and property.

B. Honeymoon phase

The honeymoon phase brings community bonding and optimism as aid arrives, which has faded here.

C. Reconstruction phase

Reconstruction comes later, as residents take responsibility for rebuilding and find new meaning.

D. Disillusionment phase ✓ KEY

As outside support withdraws and bureaucratic delays mount, frustration, anger, and feeling abandoned mark disillusionment.

Rationale: Disaster mental health models describe community phases: pre-disaster, impact, heroic, honeymoon, disillusionment, and reconstruction. The disillusionment phase can last months or longer, as optimism gives way to fatigue and anger when aid slows and the reality of losses sets in. Counselors can anticipate rising stress during this phase and increase outreach.

References: Gladding (Counseling: A Comprehensive Profession) — Crisis and disaster counseling: phases of disaster response

► **Check:** Phase names follow the SAMHSA/Zunin and Myers disaster model; confirm the Gladding citation or substitute a SAMHSA disaster source if one is added.

☐ Approve ☐ Revise ☐ Retire Notes:

A week after her partner assaulted her, a client tells the counselor that he cried, brought flowers, promised it would never happen again, and has been "the man I fell in love with." She feels hopeful. In Walker's cycle of violence, which phase is this?

A. Tension-building phase

Tension building involves rising strain, minor incidents, and the victim trying to keep the peace.

B. Acute battering phase

The acute battering phase is the explosion of violence itself, which has already passed.

C. Honeymoon phase ✓ KEY

Apologies, gifts, promises, and loving behavior after an assault mark the honeymoon (loving contrition) phase.

D. Resolution phase

Walker's cycle has no resolution phase; without intervention, the cycle tends to repeat.

Rationale: Lenore Walker's cycle of violence has three phases: tension building, an acute battering incident, and loving contrition (the honeymoon phase). The honeymoon phase reinforces hope that the abuse will end and helps explain why many survivors stay. Over time the honeymoon phase often shortens or disappears, so counselors should use this period to support safety planning.

References: Gladding (Counseling: A Comprehensive Profession) — Intimate partner violence: the cycle of violence

► **Check:** Walker's cycle is cited via Gladding; confirm coverage, since no IPV-specific text is on the approved list.

☐ Approve ☐ Revise ☐ Retire Notes:

A client says her husband controls all their money, reads her messages, has cut her off from friends, and threatens to take the children if she leaves. She adds, "It isn't really abuse, because he has never hit me." Which framework would BEST help her see these behaviors as connected forms of abuse?

A. Walker's cycle of violence

The cycle of violence describes phases around violent episodes, and she reports no physical violence.

B. Duluth Power and Control Wheel ✓ KEY

The wheel shows tactics such as economic abuse, isolation, and using children as parts of one pattern of coercive control.

C. Gottman's Four Horsemen

The Four Horsemen describe destructive communication in conflict, not coercive control or abuse.

D. Bowen's triangulation concept

Triangulation describes drawing a third party into a dyad's anxiety, not a pattern of abusive control.

Rationale: The Power and Control Wheel, developed by the Domestic Abuse Intervention Project in Duluth, Minnesota, shows abuse as a pattern of coercive control. Its spokes are tactics such as intimidation, isolation, economic abuse, using children, and threats, held together by a rim of physical and sexual violence. Coercive control can be dangerous even without physical assault, and conjoint couples counseling is generally contraindicated while it is ongoing.

References: Sue & Sue (Counseling the Culturally Diverse) — Counseling women: intimate partner violence and coercive control

► **Check:** The Duluth model is cited via Sue & Sue; confirm coverage or substitute a better approved source.

☐ Approve ☐ Revise ☐ Retire Notes:

A 46-year-old warehouse supervisor lost his job four months ago. A generous severance package covers his bills for the next year, yet he reports feeling aimless, sleeping late, missing his coworkers, and no longer knowing "who I am." Which framework BEST explains why his distress persists despite financial security?

A. Holland's congruence between personality type and setting

Congruence concerns fit between a person's type and a work environment; it does not explain distress from losing work altogether.

B. Krumboltz's planned happenstance and openness to chance

Planned happenstance is a stance for using unplanned events, not an account of why job loss is distressing.

C. Jahoda's latent deprivation model of unemployment ✓ KEY

Jahoda argued that work supplies latent benefits such as time structure, social contact, purpose, status and activity; losing them causes distress even when income continues.

D. Gottfredson's compromise among sex type, prestige and field

Compromise describes how people give up preferred options when choosing; it does not address the psychological functions of employment.

Rationale: Jahoda's latent deprivation model holds that employment has a manifest function (income) and latent functions: time structure, regular social contact, shared purpose, status and identity, and regular activity. Unemployed people lose these latent benefits even when money is not a problem, which helps explain aimlessness and identity loss. Counselors can help clients rebuild structure, contact and purpose while job searching.

References: Sharf (Career Development Theory) — Career counseling for unemployment and job loss; Jahoda's latent deprivation model

► **Check:** Verify that the cited Sharf edition discusses Jahoda's latent deprivation model; the model itself (Jahoda, 1981-1982) is well established.

☐ Approve ☐ Revise ☐ Retire Notes:

A 34-year-old client has completed several interest inventories, researched dozens of occupations, and shadowed workers in two fields, yet still cannot choose. She describes the same agonizing paralysis when picking an apartment, a car, or even a restaurant, and says any choice feels like it could ruin her life. Which approach BEST fits her concern?

A. Address the anxiety behind her pervasive indecisiveness ✓ KEY

Indecision that spans life domains and persists despite ample information reflects chronic indecisiveness, which calls for work on anxiety and personal concerns, not more data.

B. Give more occupational information and a new inventory

More information helps a developmentally undecided client who lacks data; this client already has extensive information and remains stuck.

C. Treat her concern as normal, developmental undecidedness

Developmental undecidedness is situational and resolves with exploration; her paralysis is pervasive and long-standing.

D. Have her pick the highest-rated field and commit to it now

Pushing a fast commitment ignores the anxiety driving her indecisiveness and is likely to increase her distress.

Rationale: Career literature distinguishes career indecision (undecidedness), a common developmental state that usually responds to information and exploration, from indecisiveness, a chronic trait-like difficulty making decisions across many areas of life, often tied to anxiety, perfectionism or fear of commitment. Indecisive clients benefit from attention to these personal factors alongside career work.

References: Sharf (Career Development Theory) — Career indecision versus chronic indecisiveness in career counseling

☐ Approve ☐ Revise ☐ Retire Notes:

An emergency room nurse works predictable shifts and is home every evening. She says the fatigue, tension and irritability she carries home from chaotic shifts leave her snapping at her children and too drained to enjoy family time. Which form of work-family conflict does this BEST illustrate?

A. Time-based conflict over hours that work takes from home

Time-based conflict occurs when hours spent in one role make it hard to meet another role's demands; her schedule leaves her home every evening.

B. Behavior-based conflict from work habits that misfit home

Behavior-based conflict involves role behaviors, such as a brisk clinical style, that are inappropriate at home; her problem is spillover of strain.

C. Family-to-work conflict in which home duties disrupt her job

This reverses the direction; here work is interfering with family life, not the other way around.

D. Strain-based conflict as work stress spills into home life ✓ KEY

Strain-based conflict occurs when stress, fatigue or irritability from one role impairs functioning in another, as her work strain does at home.

Rationale: Greenhaus and Beutell described three forms of work-family conflict: time-based, strain-based and behavior-based, each of which can run work-to-family or family-to-work. Identifying the form guides intervention; strain-based conflict points toward recovery routines, stress management and boundary rituals between roles rather than schedule changes.

References: Sharf (Career Development Theory) — Work-family conflict and life roles: time-based, strain-based and behavior-based conflict

► **Check:** Confirm the cited Sharf edition covers the Greenhaus and Beutell (1985) three-form typology of work-family conflict.

☐ Approve ☐ Revise ☐ Retire Notes:

A 27-year-old client with well-controlled epilepsy has a job interview next week. She asks her counselor whether she is required to tell the interviewer about her seizure disorder. Under the Americans with Disabilities Act, which statement is MOST accurate?

A. She must disclose it at the interview so the employer can judge risk

The ADA does not require applicants to disclose a disability at an interview; employers may ask only about the ability to do job functions.

B. Disclosure is her choice; it is needed if she seeks accommodation ✓ **KEY**

Disclosure under the ADA is generally voluntary; an employer needs to know about the disability only when an accommodation is requested.

C. Disclosure is only required if the job involves operating machinery

Job tasks do not create a duty to disclose at the interview; safety concerns are handled through job-related standards after an offer.

D. The employer may ask about any medical condition before an offer

Before a conditional job offer, the ADA bars disability-related questions and medical exams.

Rationale: The ADA prohibits disability-related inquiries and medical examinations before a conditional job offer and does not require applicants to disclose. Disclosure becomes necessary mainly when a person requests a reasonable accommodation. Career counselors help clients with disabilities weigh whether, when and how to disclose, and how to describe needed accommodations in terms of job performance.

References: Sharf (Career Development Theory) — Career development of people with disabilities; ADA disclosure and reasonable accommodation

► **Check:** Confirm the ADA pre-offer inquiry rules as stated and that the cited source covers disability disclosure; Sharf may not discuss ADA specifics.

☐ Approve ☐ Revise ☐ Retire Notes:

A 19-year-old first-generation college student from a Filipino American family says her parents expect her to become a nurse, though she is drawn to graphic design. She values her family's sacrifices and wants their blessing. Her counselor is trained in a decision model that stresses individual choice. What is the MOST culturally responsive approach?

A. Encourage her to choose design and assert independence from family

This imposes an individualistic value and ignores that family harmony and approval matter to her.

B. Advise her to follow her parents' wishes to keep peace in the family

This dismisses her own interests and is just as directive as pushing independence.

C. Explore her values and include family input in the decision process ✓ **KEY**

Honoring collectivist values while exploring her interests, and inviting family into the process when she wishes, respects her worldview.

D. Use the standard model as written, since good models fit all clients

Career models built on Western individualism may not fit clients from collectivist backgrounds and need cultural adaptation.

Rationale: Many career theories assume individualistic decision making. With clients from collectivist backgrounds, career choice may be a family decision tied to obligation and gratitude. Culturally responsive counselors explore the client's own values, acknowledge family expectations, and may involve family in the process rather than framing autonomy as the goal.

References: Sue & Sue (Counseling the Culturally Diverse) — Collectivism, family roles and culturally responsive career counseling

☐ Approve ☐ Revise ☐ Retire Notes:

Parents of a 15-year-old argue constantly with their son about curfew, friends and privacy, while also arranging care for the father's ailing mother. In Carter and McGoldrick's family life cycle, which key emotional task does the family face in this stage?

A. Increasing the flexibility of family boundaries for independence ✓ KEY

The families-with-adolescents stage calls for more flexible boundaries to allow teen independence and to absorb grandparents' frailties.

B. Accepting new members into the system as the couple has children

Accepting new members is the task of families with young children.

C. Accepting many exits from and entries into the family system

Accepting exits and entries is the task of launching children and moving on.

D. Accepting the shifting of roles from one generation to the next

Accepting shifting generational roles is the task of families in later life.

Rationale: Carter and McGoldrick describe stages of the family life cycle, each with a key emotional process. In families with adolescents, boundaries must become more flexible so teens can move in and out of the system, while parents often begin caring for aging grandparents. Stress tends to rise when a family remains rigid at a transition point.

References: Gladding (Counseling: A Comprehensive Profession) — Family life cycle stages and key emotional tasks (Carter & McGoldrick)

☐ Approve ☐ Revise ☐ Retire Notes:

A divorcing couple with children aged 6 and 10 asks a counselor what will MOST affect how well their children adjust over the coming years. Based on research on divorce and child outcomes, which factor should the counselor highlight?

A. Whether the children are told about the divorce at the same time

How the news is shared matters, but a single conversation is not a major long-term predictor of adjustment.

B. Whether a sole or a joint legal custody arrangement is chosen

Custody type alone predicts outcomes less than the quality of parenting and the level of conflict children face.

C. Whether either parent remarries within the first year after

Remarriage brings its own adjustments, but it is not the strongest predictor of children's post-divorce adjustment.

D. How much ongoing conflict between parents the children see ✓ KEY

Exposure to persistent, hostile interparental conflict is among the strongest predictors of poorer child adjustment after divorce.

Rationale: Research by Hetherington, Amato and others shows that most children of divorce adjust well within a few years. Poorer outcomes are most strongly linked to ongoing interparental conflict, especially when children are caught in the middle, along with declines in parenting quality and economic hardship. Counselors help parents shield children from conflict and keep warm, consistent parenting.

References: Berk (Development Through the Lifespan) — Divorce: factors in children's adjustment, interparental conflict and parenting quality

☐ Approve ☐ Revise ☐ Retire Notes:

During a heated discussion in couples counseling, a husband pauses, smiles, and says, "Okay, I'm getting defensive. Can I try saying that again?" His wife's tone softens. In Gottman's research, what is the husband's statement called?

A. Stonewalling

Stonewalling is withdrawing from the interaction, one of the Four Horsemen; he is re-engaging, not shutting down.

B. A repair attempt ✓ **KEY**

A repair attempt is any statement or action that de-escalates conflict and keeps negativity from spiraling.

C. A harsh startup

A harsh startup is opening a discussion with criticism or contempt; his comment de-escalates.

D. Flooding

Flooding is physiological overwhelm during conflict; his calm self-correction shows the opposite.

Rationale: Gottman found that stable couples argue too, but they make and accept repair attempts, such as humor, apologies or requests to start over, that stop negativity from escalating. Whether repairs succeed depends largely on the strength of the couple's friendship. Counselors teach partners to offer and recognize repair attempts.

References: Gladding (Counseling: A Comprehensive Profession) — Couples counseling: Gottman's research on repair attempts and the Four Horsemen

► **Check:** Gottman's own texts are not on the approved source list; confirm the cited Gladding edition covers repair attempts.

☐ Approve ☐ Revise ☐ Retire Notes:

A 20-year-old college student raised in a close-knit congregation tells her counselor she has begun questioning teachings she once accepted without thought. She is reading widely, testing beliefs against her own reasoning, and wants a faith she has chosen for herself. Which of Fowler's stages of faith does this BEST describe?

A. Synthetic-conventional

Synthetic-conventional faith conforms to the beliefs of one's group without critical examination, which she is now moving beyond.

B. Mythic-literal

Mythic-literal faith, typical of school-age children, takes stories and symbols literally.

C. Individuative-reflective ✓ **KEY**

This stage involves critically examining inherited beliefs and taking personal responsibility for one's own commitments.

D. Conjunctive

Conjunctive faith, usually in midlife or later, embraces paradox and other traditions after the reflective stage.

Rationale: Fowler proposed stages of faith development: primal (undifferentiated), intuitive-projective, mythic-literal, synthetic-conventional, individuative-reflective, conjunctive and universalizing. The shift from synthetic-conventional to individuative-reflective often occurs in late adolescence or young adulthood, when people move from group-held belief to a self-examined faith. Counselors can normalize this questioning rather than treat it as a crisis.

References: Gladding (Counseling: A Comprehensive Profession) — Spirituality in counseling: Fowler's stages of faith development

☐ Approve ☐ Revise ☐ Retire Notes:

After a long illness in his family, a 63-year-old deacon says he has lost his sense that God listens and feels unmoored from his faith. He is sad at times but sleeps, eats and works normally, and his distress is in proportion to his experience. He meets criteria for no mental disorder. How should the counselor document the focus of treatment?

A. Adjustment disorder with depressed mood after the stressor

Adjustment disorder requires distress out of proportion to the stressor or significant impairment; neither is present.

B. Persistent depressive disorder, given his low periods

He has occasional sadness with normal functioning, not depressed mood most days for two years.

C. No code, since spiritual concerns fall outside counseling

Spiritual concerns can be a legitimate focus of counseling and have a code for documentation.

D. Religious or spiritual problem as a focus of attention ✓ KEY

DSM-5-TR lists religious or spiritual problem (Z65.8) for distress such as loss or questioning of faith that is a focus of care.

Rationale: DSM-5-TR includes Religious or Spiritual Problem (Z65.8) among Other Conditions That May Be a Focus of Clinical Attention. It covers distressing experiences such as loss or questioning of faith, conversion, or conflicts with a religious institution. It lets counselors document a legitimate focus of care without pathologizing a spiritual struggle that does not meet criteria for a mental disorder.

References: DSM-5-TR — Other Conditions That May Be a Focus of Clinical Attention: Religious or Spiritual Problem (Z65.8)

☐ Approve ☐ Revise ☐ Retire Notes:

A mother tells her counselor that her 11-year-old's new volunteer tutor is "amazing." He buys her daughter small gifts, offers rides so the mother can rest, texts the girl privately at night, and recently told her some of their talks are "just between us." Which feature MOST strongly suggests possible grooming?

A. Private contact paired with a request to keep secrets ✓ KEY

Grooming typically moves toward private access and secrecy, which isolate the child and lower the chance of disclosure.

B. His popularity with the parents in the neighborhood

Groomers may win over parents, but being well liked is common among safe adults and is not a specific warning sign.

C. His strong interest in the girl's grades and reading

Academic interest is expected of a tutor and is not by itself a warning sign.

D. His volunteering without pay for a nonprofit program

Unpaid volunteering is ordinary and does not distinguish grooming from safe mentoring.

Rationale: Grooming is a gradual process in which an offender builds trust with a child and often the family, gives special attention or gifts, tests boundaries, increases private access and introduces secrecy. Secret-keeping and isolated contact are the most telling signs. Counselors educate parents about these patterns and follow mandated reporting law when abuse is reasonably suspected.

References: Gladding (Counseling: A Comprehensive Profession) — Child sexual abuse: grooming patterns and warning signs

► **Check:** Confirm the cited source covers grooming; the concept itself is standard in child-protection literature.

☐ Approve ☐ Revise ☐ Retire Notes:

A school counselor notices bruises on an active 6-year-old who plays rough sports at recess. Which bruise location should raise the GREATEST concern about possible physical abuse?

A. Scattered bruises over both shins and knees

Shins and knees are bony areas that are commonly bruised in normal childhood play.

B. A bruise on the forehead after a playground fall

The forehead is a common site of accidental injury in falls, especially with a matching account.

C. Bruises on the ears, cheeks and the upper back ✓ **KEY**

Ears, cheeks and the back are protected, soft areas rarely bruised by accident, so injuries there suggest inflicted harm.

D. Bruises on the elbows of both arms from falling

Elbows are bony prominences that often take the impact of accidental falls.

Rationale: Accidental bruises in children usually appear over bony prominences such as the shins, knees, elbows and forehead. Bruises on soft or protected areas such as the ears, cheeks, neck, torso, back, buttocks or upper arms, or bruises in the shape of an object or hand, are more concerning for abuse. Reasonable suspicion triggers a mandated report.

References: Gladding (Counseling: A Comprehensive Profession) — Recognizing physical indicators of child abuse · State law — Child abuse mandated reporting statutes: reasonable-suspicion standard

► **Check:** Confirm the cited Gladding edition lists physical indicators of abuse; the injury-location pattern is standard pediatric guidance.

☐ Approve ☐ Revise ☐ Retire Notes:

A middle school counselor is preparing a parent workshop on cyberbullying. Which feature MOST clearly sets cyberbullying apart from traditional in-person bullying?

A. It involves a real or perceived imbalance of power

A power imbalance is part of the definition of all bullying, not a distinguishing feature of cyberbullying.

B. It can reach the child at home, at any hour, by many ✓ **KEY**

Online bullying follows children home around the clock and can be seen and shared by large audiences, often anonymously.

C. It is intentional and is repeated over a period of time

Intent and repetition define bullying in general, both online and in person.

D. It harms the target's mood and school performance

Both forms of bullying are linked to depression, anxiety and academic problems.

Rationale: Bullying is unwanted aggressive behavior that is intentional, repeated or likely to be repeated, and involves a power imbalance. Cyberbullying shares these features but differs in that it can occur at any time and place, reach a wide audience, persist as a permanent record, and allow anonymity, so home is no longer a refuge.

References: Gladding (Counseling: A Comprehensive Profession) — School counseling: bullying and cyberbullying

☐ Approve ☐ Revise ☐ Retire Notes:

For several weeks, two larger eighth graders have mocked a smaller classmate, taken his lunch, and posted edited photos of him. The principal asks the school counselor to hold a peer mediation session so the three boys can "work it out together." What is the counselor's BEST response?

A. Agree, and set clear ground rules so each boy can share equally

Ground rules do not remove the power imbalance that makes mediation unsafe and unfair for a bullied student.

B. Agree, but ask the target to suggest ways he could avoid the boys

This places responsibility on the target and implies he shares blame for being bullied.

C. Decline, and leave the matter to discipline by the administration

Discipline may be needed, but the counselor still has a role in supporting the target and working with the students who bully.

D. Explain why mediation fits poorly and meet with the boys separately ✓ KEY

Because bullying involves a power imbalance, separate meetings that support the target and hold the others accountable are preferred.

Rationale: Peer mediation assumes two parties with roughly equal power and shared responsibility for a conflict. Bullying is repeated, intentional aggression with a power imbalance, so mediation can retraumatize the target and send the message that both are to blame. Recommended practice is to support the target, address the behavior of those who bully separately, and involve families and school-wide prevention.

References: Gladding (Counseling: A Comprehensive Profession) — School counseling: responding to bullying; limits of peer mediation

► **Check:** Confirm the cited source states that peer mediation is not recommended for bullying (a position also taken by federal StopBullying guidance).

☐ Approve ☐ Revise ☐ Retire Notes:

A counselor reviewing a new referral for a 77-year-old with low mood, poor sleep and withdrawal thinks, "Anyone her age would feel down. Therapy probably won't change much at this point." What does the counselor's reaction BEST reflect?

A. Ageist bias that risks undertreating treatable depression ✓ KEY

Treating depression as a normal part of aging, and assuming older adults cannot change, is ageism and leads to undertreatment.

B. Sound clinical judgment about late-life treatment outcomes

Older adults respond well to psychotherapy for depression; the assumption is not supported.

C. Appropriate humility about his limits with older clients

Humility would lead to consultation or training, not a belief that therapy is futile.

D. A correct view of depression as part of normal aging

Depression is common in later life but is not a normal part of aging.

Rationale: Ageism includes stereotypes that older adults are rigid, unable to change, or naturally depressed. In clinicians it can show up as therapeutic nihilism and missed diagnoses. Depression is not a normal part of aging, and evidence-based therapies such as CBT and problem-solving therapy work well for older adults.

References: Berk (Development Through the Lifespan) — Late adulthood: ageism, stereotypes of aging and late-life depression

☐ Approve ☐ Revise ☐ Retire Notes:

An 84-year-old client mentions that her nephew, who now manages her finances, has her sign blank checks, and that her utilities were shut off last month. She seems anxious when he is mentioned and asks the counselor not to "make trouble." What should the counselor do?

A. Wait to act until she gives proof that funds were taken

Reporting standards rest on reasonable suspicion; counselors do not need proof before acting.

B. Call the nephew to ask him to explain how he uses the money

Confronting a suspected abuser can raise the risk to the client and breaches confidentiality.

C. Follow state law and report suspected exploitation to APS ✓ **KEY**

Financial exploitation of an older adult is reportable abuse in many states; counselors follow their state's law and report to Adult Protective Services.

D. Respect her wish and only document what she has disclosed

Her wishes matter, but many states mandate reports of suspected elder abuse regardless of consent.

Rationale: Elder abuse includes physical, emotional, sexual and financial abuse and neglect; perpetrators are often family members or trusted caregivers. Signs of financial exploitation include unpaid bills despite adequate income and unusual control of money. Reporting laws vary, so counselors know their state's requirements, report suspected abuse to Adult Protective Services when required, and involve the client as much as possible.

References: State law — Elder and vulnerable adult abuse reporting statutes (requirements vary by state) · ACA Code of Ethics — B.2.a Serious and Foreseeable Harm and Legal Requirements

☐ Approve ☐ Revise ☐ Retire Notes:

A 29-year-old nonbinary client, assigned male at birth, has lived in line with their gender identity for years and feels settled and content with it. They seek counseling for work stress after a promotion. Which statement about a gender dysphoria diagnosis is MOST accurate?

A. It applies, since gender identity differs from assigned sex

Incongruence alone is not a disorder; DSM-5-TR also requires clinically significant distress or impairment.

B. It does not apply without related distress or impairment ✓ **KEY**

Gender dysphoria requires clinically significant distress or impairment linked to the incongruence, which this client does not report.

C. It applies, since the incongruence has lasted over 6 months

Duration is only part of the criteria; distress or impairment is also required.

D. It does not apply, since nonbinary identities are excluded

DSM-5-TR criteria include a desire to be or be treated as an alternative gender, which covers nonbinary identities.

Rationale: In DSM-5-TR, gender dysphoria requires a marked incongruence between experienced or expressed gender and assigned gender lasting at least 6 months, plus clinically significant distress or impairment. The diagnosis centers on distress, not identity; being transgender or nonbinary is not itself a mental disorder. Counselors avoid diagnosing identity and focus on the client's presenting concerns.

References: DSM-5-TR — Gender Dysphoria in adolescents and adults: Criterion A duration and Criterion B distress or impairment

☐ Approve ☐ Revise ☐ Retire Notes:

In the third session, a 17-year-old client tells the counselor that he is transgender, goes by Eli, and uses he/him pronouns. He has not told his parents. Which counselor response is MOST affirming?

A. Ask him to wait a few months before changing his name

Delaying use of his name invalidates his identity and can weaken trust in the relationship.

B. Use his legal name in session until his parents know

Withholding his name in session is not affirming; safety concerns apply to records and outside contacts, not to how the counselor addresses him.

C. Explore whether this is a phase before using his name

Treating his identity as a phase to be tested is not affirming practice.

D. Use Eli and he/him, and ask where it is safe to do so ✓ **KEY**

Affirming care uses the client's name and pronouns and plans together where they are used, such as in records or with family, to avoid outing him.

Rationale: Affirming counseling with transgender clients means using their chosen name and pronouns, treating gender identity as valid, and collaborating on safety, including when names appear in records or communications that parents or others may see. This respects the client's autonomy and protects against unwanted disclosure.

References: ACA Code of Ethics — A.1.a Primary Responsibility; C.5 Nondiscrimination · Sue & Sue (Counseling the Culturally Diverse) — Counseling LGBTQ+ individuals: affirming practice

☐ Approve ☐ Revise ☐ Retire Notes:

A 45-year-old Somali refugee who resettled two years ago works steadily and cares for his children. He grieves the loss of his land, his community's rituals, and his parents' graves, feels guilty for leaving, and sometimes dreams that his ancestors are calling him. Which concept BEST describes his experience?

A. Cultural bereavement after losing homeland and community ✓ **KEY**

Cultural bereavement is grief over the loss of one's homeland, social structures and cultural identity, a normal response to uprooting.

B. Culture shock from early contact with an unfamiliar setting

Culture shock is the early disorientation of contact with a new culture; his grief centers on what was lost.

C. Prolonged grief disorder after the death of a close person

Prolonged grief disorder requires the death of someone close and impairing grief; his loss is of culture and place, and he functions well.

D. Posttraumatic stress disorder with prominent survivor guilt

He reports no intrusion, avoidance or arousal symptoms; guilt and longing alone do not indicate PTSD.

Rationale: Eisenbruch described cultural bereavement as the grief of uprooted people for lost land, culture and social bonds, which can include guilt over leaving, longing, and dreams or visits from ancestors. He cautioned that such experiences may be misread as pathology. Counselors validate the loss and draw on cultural and community resources for mourning.

References: Sue & Sue (Counseling the Culturally Diverse) — Refugees and immigrants: cultural bereavement and migration-related loss

► **Check:** Verify the Eisenbruch (1991) attribution and that the cited Sue & Sue edition discusses cultural bereavement.

☐ Approve ☐ Revise ☐ Retire Notes:

A 12-year-old regularly interprets for her Spanish-speaking parents at medical visits and the bank. At the start of a family counseling session about the father's job loss, the parents ask her to interpret for them. What should the counselor do?

A. Let her interpret, since the family trusts her the most

Trust does not offset the role reversal and exposure to adult concerns that interpreting places on a child.

B. Hold the session in English and simplify the language

Simplifying English does not give the parents full access to the session.

C. Arrange a qualified interpreter so she can be a child ✓ **KEY**

A qualified interpreter gives the parents full access and frees the child from a parental role in adult matters.

D. Ask her to sum up each part instead of translating it

Summaries still make the child the gatekeeper of adult content and risk distortion.

Rationale: Language brokering, in which children of immigrants interpret for parents, can build skills but also brings role reversal, stress and exposure to adult worries. In counseling, using a child as interpreter compromises accuracy and confidentiality and burdens the child. The ACA Code directs counselors to arrange qualified interpreters when clients have difficulty understanding the language used.

References: ACA Code of Ethics — A.2.c Developmental and Cultural Sensitivity (qualified interpreters) · Sue & Sue (Counseling the Culturally Diverse) — Language barriers, language brokering and the use of interpreters

☐ Approve ☐ Revise ☐ Retire Notes:

A 19-year-old with a three-year history of superficial cutting to "calm down" tells her counselor that last weekend she swallowed a bottle of pills because she "wanted to never wake up." She slept for 12 hours and woke without treatment. Which statement is MOST accurate?

A. The overdose is part of her NSSI pattern, given her history

NSSI is defined by the absence of suicidal intent; she clearly stated she wanted to die.

B. The overdose was a suicide attempt, and NSSI raises her risk ✓ **KEY**

Intent to die makes the overdose a suicide attempt, and a history of NSSI is a known risk factor for suicidal behavior.

C. Her low-lethality outcome shows the act was mainly a gesture

Outcome does not define intent; calling it a gesture minimizes a real suicide attempt.

D. Her NSSI lowers suicide risk because it works as coping for her

NSSI may regulate emotion short term, but it is associated with higher, not lower, suicide risk.

Rationale: NSSI and suicidal behavior are distinguished mainly by intent: NSSI is self-inflicted harm without intent to die, usually to relieve distress, whereas a suicide attempt involves at least some intent to die. The two often co-occur, and NSSI history is a robust predictor of later suicide attempts. Each act should be assessed for intent, and safety planning updated.

References: DSM-5-TR — Conditions for Further Study: Nonsuicidal Self-Injury Disorder and Suicidal Behavior Disorder · C-SSRS — Distinguishing suicidal from non-suicidal self-injurious behavior by intent

☐ Approve ☐ Revise ☐ Retire Notes:

The DSM-5-TR proposed criteria for nonsuicidal self-injury disorder, a condition for further study, require intentional self-inflicted injury without suicidal intent on how many days in the past year?

A. At least 2 days

Two days is below the proposed threshold.

B. At least 12 days

Twelve days exceeds the proposed threshold.

C. At least 30 days

Thirty days is far above the proposed threshold.

D. At least 5 days ✓ KEY

The proposed criteria require self-injury on 5 or more days in the past year.

Rationale: In DSM-5-TR Section III, the proposed nonsuicidal self-injury disorder requires intentional self-inflicted damage to the body on 5 or more days in the past year, with the expectation of only minor or moderate harm. DSM-5-TR also added codes for current or past nonsuicidal self-injury among conditions that may be a focus of clinical attention.

References: DSM-5-TR — Conditions for Further Study: Nonsuicidal Self-Injury Disorder, Criterion A

☐ Approve ☐ Revise ☐ Retire Notes:

A 16-year-old cannot fall asleep before 2 a.m. on school nights and is exhausted in morning classes. On weekends he sleeps soundly from 2 a.m. to 11 a.m. and wakes refreshed. Months of sleep hygiene tips such as no caffeine and a cool, dark room have not helped. Which explanation BEST fits?

A. A circadian rhythm disorder of the delayed sleep phase type ✓ KEY

Sleep that is normal in quality but shifted later, causing conflict with required schedules, fits delayed sleep phase type.

B. An insomnia disorder needing more strict sleep hygiene rules

He sleeps well when allowed his own schedule, so the problem is timing, and sleep hygiene alone has not helped.

C. A hypersomnolence disorder marked by excessive sleep need

His total sleep is normal and he wakes refreshed on weekends, which argues against hypersomnolence.

D. Narcolepsy with sleep attacks in his early morning classes

Narcolepsy involves recurrent irresistible sleep episodes regardless of schedule, not a simple shift in timing.

Rationale: Circadian rhythm sleep-wake disorders arise from a mismatch between the internal clock and required schedules. Delayed sleep phase type, common in adolescents, causes trouble falling asleep and waking at conventional times, while sleep at the preferred time is normal. Sleep hygiene alone rarely fixes a timing problem; approaches such as a consistent wake time and morning light, coordinated with a medical provider, target the clock itself.

References: DSM-5-TR — Circadian Rhythm Sleep-Wake Disorders: delayed sleep phase type

☐ Approve ☐ Revise ☐ Retire Notes:

A 58-year-old client with schizophrenia has taken a first-generation antipsychotic for many years. The counselor notices repetitive lip smacking, tongue thrusting and facial grimacing that the client does not seem aware of. Which side effect do these signs MOST likely reflect?

A. Akathisia, an inner restlessness and urge to keep moving

Akathisia is subjective restlessness with pacing or fidgeting, not involuntary oral and facial movements.

B. Acute dystonia, with sudden and sustained muscle spasms

Acute dystonia appears early in treatment as painful sustained contractions, not repetitive movements after years.

C. Tardive dyskinesia from long-term antipsychotic use ✓ KEY

Involuntary repetitive movements of the mouth, tongue and face after long-term antipsychotic use are classic tardive dyskinesia.

D. Neuroleptic malignant syndrome, a fever-and-rigidity crisis

NMS is an acute emergency with high fever, severe rigidity and confusion, not chronic oral-facial movements.

Rationale: Tardive dyskinesia is a late-emerging, sometimes irreversible movement disorder linked to long-term dopamine-blocking medications, especially first-generation antipsychotics. Counselors who notice possible signs should promptly inform the prescriber. Second-generation antipsychotics carry lower TD risk but greater metabolic risks, such as weight gain, diabetes and lipid changes, which also warrant medical monitoring.

References: DSM-5-TR — Medication-Induced Movement Disorders: Tardive Dyskinesia

☐ Approve ☐ Revise ☐ Retire Notes:

A client who takes phenelzine, a monoamine oxidase inhibitor, mentions he is planning a celebration dinner of aged cheeses, cured salami and draft beer. The counselor encourages him to check with his prescriber first. Which reaction is the main concern?

A. Agranulocytosis

Agranulocytosis is a blood disorder linked mainly to clozapine, not to diet with MAOIs.

B. Hypertensive crisis ✓ KEY

Tyramine in aged, cured and fermented foods can trigger a dangerous blood pressure spike in people taking MAOIs.

C. Lithium toxicity

Lithium toxicity relates to lithium levels, not to tyramine or MAOIs.

D. Tardive dyskinesia

Tardive dyskinesia is a movement disorder from long-term antipsychotics, not a dietary reaction.

Rationale: MAOIs block the breakdown of tyramine, found in aged cheeses, cured meats, some tap or draft beers and other fermented foods. Tyramine buildup can cause a hypertensive crisis with severe headache, stiff neck and rapid blood pressure rise. MAOIs also interact dangerously with many drugs, including other antidepressants, raising the risk of serotonin syndrome. Counselors reinforce prescriber guidance rather than giving medical advice.

References: Gladding (Counseling: A Comprehensive Profession) — Psychopharmacology for counselors: MAOIs and tyramine dietary restrictions

► **Check:** Confirm the cited source covers MAOI dietary restrictions; the pharmacology is standard.

☐ Approve ☐ Revise ☐ Retire Notes:

A 20-year-old college student without an ADHD diagnosis tells her counselor she takes a friend's prescription stimulant before exams to "focus better." She says it is safe "because doctors prescribe it to kids." What is the counselor's BEST response?

A. Tell her she must stop now or the counselor will alert the school

Threats damage the alliance, and confidential disclosure of past use does not by itself permit a report.

B. Agree it is low risk if used only during the exam period

Nonmedical stimulant use carries real risks even when occasional, so this response gives false reassurance.

C. Advise her to seek her own prescription so she can use it safely

Steering her to obtain a stimulant without a clinical need endorses misuse and is outside the counselor's role.

D. Explore her use nonjudgmentally and discuss its health risks ✓ **KEY**

A nonjudgmental, MI-consistent discussion of risks such as dependence and cardiac strain respects autonomy and invites change.

Rationale: Nonmedical use of prescription stimulants is common among college students and carries risks such as elevated heart rate and blood pressure, anxiety, insomnia and stimulant use disorder. Counselors respond with motivational interviewing: exploring the student's reasons, offering accurate information with permission, and addressing underlying academic stress, rather than lecturing or threatening.

References: Miller & Rollnick (MI) — Exchanging information with permission (elicit-provide-elicite) and avoiding the righting reflex · DSM-5-TR — Stimulant Use Disorder

☐ Approve ☐ Revise ☐ Retire Notes:

Two months after her husband returned from a 9-month deployment, a military spouse says he keeps overriding household routines she ran on her own, the children test his rules, and they feel like "strangers sharing a house." In the emotional cycle of deployment, which phase is the family in?

A. Post-deployment, as the family renegotiates its roles ✓ **KEY**

Post-deployment (reintegration) involves renegotiating roles, routines and intimacy after the service member returns.

B. Sustainment, as the family settles into new routines

Sustainment occurs during deployment, when the family at home builds new routines and support.

C. Redeployment, as the family anticipates his return

Redeployment is the period just before return, marked by excitement and apprehension.

D. Pre-deployment, as the family prepares for the leave

Pre-deployment covers preparation and anticipatory loss before departure.

Rationale: The emotional cycle of deployment is often described in phases: pre-deployment, deployment, sustainment, redeployment and post-deployment. Post-deployment reintegration can take months as families renegotiate roles the at-home partner took on, rebuild intimacy and adjust parenting. Counselors normalize this friction and help families communicate about changed roles.

References: Gladding (Counseling: A Comprehensive Profession) — Counseling military families: emotional cycle of deployment and reintegration

► **Check:** Verify the five-phase deployment cycle (Pincus et al.) and that the cited Gladding edition covers it.

☐ Approve ☐ Revise ☐ Retire Notes:

A 38-year-old Air Force veteran discloses that she was sexually assaulted by a fellow service member during her enlistment. She never reported it, has no VA disability rating, and assumes VA will not help her without proof. Which statement BEST guides the counselor's response?

A. She must first file a report so the assault can be verified

VA does not require that the assault was reported or documented to provide MST-related care.

B. She can get care only after a service-connection rating

MST-related care does not require a service-connected disability rating.

C. VA offers free MST care without a report or a rating ✓ **KEY**

VA provides free care for mental and physical conditions related to military sexual trauma without requiring a prior report or a rating.

D. MST must first be diagnosed as a disorder to get services

MST is an experience, not a diagnosis; care addresses conditions related to it.

Rationale: Military sexual trauma (MST) is the VA term for sexual assault or threatening sexual harassment during military service. It is an experience, not a diagnosis. VA screens veterans for MST and provides free MST-related treatment even when the veteran never reported the event and has no service-connected rating. Knowing this helps counselors connect survivors to care and reduce barriers.

References: VA/DoD CPG — Management of PTSD and acute stress disorder: military sexual trauma and access to care

► **Check:** Verify current VA MST eligibility rules (free MST-related care without documentation or rating) and whether the VA/DoD CPG is the best citation; a VA program source may fit better.

☐ Approve ☐ Revise ☐ Retire Notes:

The mother of a 9-year-old with cerebral palsy says she accepted her son's diagnosis long ago and loves their life, yet waves of sadness return each time his classmates reach milestones he cannot, such as riding a bike. She asks whether something is wrong with her. Which concept BEST frames her experience?

A. Prolonged grief disorder from an unresolved earlier loss

Prolonged grief disorder follows a death and involves persistent, impairing grief; her sadness is episodic and she functions well.

B. Chronic sorrow, a normal and recurring grief response ✓ **KEY**

Chronic sorrow describes periodic, recurring grief triggered by milestones in parents of children with lasting disabilities; it is not pathological.

C. Denial, the first stage that she has not yet moved beyond

She openly acknowledges the disability; renewed sadness is not denial, and stage models are not strictly linear.

D. A major depressive episode with a seasonal recurring pattern

Brief, triggered waves of sadness with good functioning do not meet criteria for a depressive episode.

Rationale: Olshansky introduced chronic sorrow to describe the recurring sadness of parents of children with lifelong disabilities. Grief resurfaces at milestones, transitions and reminders of the gap between expected and actual life, while day-to-day functioning and love for the child remain intact. Counselors normalize this pattern instead of framing it as failed acceptance.

References: Gladding (Counseling: A Comprehensive Profession) — Counseling families of people with disabilities: chronic sorrow

► **Check:** Confirm the cited Gladding edition discusses chronic sorrow (Olshansky, 1962).

☐ Approve ☐ Revise ☐ Retire Notes:

A 31-year-old says that in the week before her period she becomes intensely irritable, tearful, and anxious and argues with her partner. The symptoms ease within a few days after menses begins and are gone the following week. She recalls this pattern for about a year. Under DSM-5-TR, what is needed to CONFIRM a diagnosis of premenstrual dysphoric disorder?

A. Prospective daily symptom ratings over at least two symptomatic cycles ✓ KEY

DSM-5-TR requires confirmation by prospective daily ratings during at least two symptomatic cycles; the diagnosis may be made provisionally before then.

B. Blood tests showing hormone levels outside the expected range for her age

No laboratory test confirms PMDD; hormone levels are typically within normal limits.

C. Her partner's report confirming the same pattern over the previous year

Collateral reports can help, but the criterion calls for the client's own prospective daily ratings.

D. Evidence that she has no co-occurring depressive or anxiety disorders

PMDD may co-occur with other disorders; it must simply not be merely an exacerbation of another disorder.

Rationale: Premenstrual dysphoric disorder requires at least five symptoms, including at least one marked affective symptom, in the final week before menses in most cycles of the past year, improving within a few days after onset and becoming minimal or absent in the week after. Because retrospective recall overstates cyclic patterns, DSM-5-TR requires confirmation by prospective daily ratings during at least two symptomatic cycles, although a provisional diagnosis may be given first.

References: DSM-5-TR — Premenstrual Dysphoric Disorder, Criterion G (prospective daily ratings)

☐ Approve ☐ Revise ☐ Retire Notes:

A 76-year-old is referred for "memory problems" after several months of low mood and withdrawal. During a cognitive screen, which observation MOST suggests that depression, rather than a neurodegenerative neurocognitive disorder, is driving her poor performance?

A. She works hard at each task and covers missed items with plausible guesses

Effortful attempts with confabulated or near-miss answers are more typical of a neurodegenerative disorder.

B. She quickly answers "I don't know" and stresses how poorly she is doing ✓ KEY

Low effort, quick "don't know" answers and highlighting of her own failures are classic signs of depression-related cognitive impairment.

C. Her family dates a slow, steady decline back over the past several years

An insidious, gradual course over years points toward a neurodegenerative process such as Alzheimer's disease.

D. She seems unconcerned, and her daughter notices more lapses than she does

Reduced awareness of deficits, with family noticing more than the client, is more typical of neurocognitive disorder.

Rationale: Depression in later life can produce cognitive complaints (sometimes called pseudodementia). Clients with depression tend to report and emphasize their deficits, give up quickly with "I don't know" answers, and show a more recent onset tied to mood change, whereas clients with neurodegenerative disorders often try hard, minimize or are unaware of deficits, and have a gradual course. The distinction guides referral for both mood treatment and a medical workup.

References: DSM-5-TR — Major or Mild Neurocognitive Disorder: differential diagnosis (major depressive disorder) · Berk (Development Through the Lifespan) — Late adulthood: depression and cognitive decline

► **Check:** The effort/awareness contrasts are classic clinical teaching ("pseudodementia"), not DSM-5-TR criteria; SME to confirm the Berk chapter locator.

☐ Approve ☐ Revise ☐ Retire Notes:

Over the past 12 months, a client with bipolar I disorder has had one manic episode, two major depressive episodes, and one hypomanic episode, each separated by a period of remission. Which DSM-5-TR specifier applies?

A. With mixed features

Mixed features describes opposite-pole symptoms occurring within a single episode, not the number of episodes.

B. With seasonal pattern

Seasonal pattern requires a regular link between episodes and a time of year, which is not described.

C. With rapid cycling ✓ KEY

Rapid cycling is specified when at least four mood episodes occur within 12 months.

D. With psychotic features

No delusions or hallucinations are described.

Rationale: The rapid cycling specifier applies to bipolar I or II disorder when at least four mood episodes (manic, hypomanic, or major depressive) occur in the previous 12 months. Episodes must be demarcated by partial or full remission of at least 2 months or by a switch to an episode of the opposite polarity. Rapid cycling is linked to a poorer prognosis and has treatment implications.

References: DSM-5-TR — Specifiers for Bipolar and Related Disorders: with rapid cycling

☐ Approve ☐ Revise ☐ Retire Notes:

For 8 days, a 25-year-old has had elevated mood, little need for sleep, and racing thoughts. She still works her shifts, but she now believes a television news anchor is sending her coded messages. A colleague suggests hypomania because her work has not suffered. Which statement is MOST accurate?

A. Hypomania fits, since her job performance has stayed largely intact

Preserved work functioning does not override the rule that psychotic features make an episode manic.

B. No diagnosis is possible until symptoms have lasted at least 2 weeks

Two weeks is the duration for a major depressive episode; mania requires 1 week, which is met.

C. Her beliefs point to schizophrenia rather than to any mood episode

Psychotic symptoms limited to a mood episode are part of the mood disorder, not evidence of schizophrenia.

D. Psychotic features mean the episode cannot be classed as hypomanic ✓ KEY

DSM-5-TR states that if psychotic features are present, the episode is, by definition, manic.

Rationale: A hypomanic episode is not severe enough to cause marked impairment or require hospitalization, and it has no psychotic features. DSM-5-TR states that if psychotic features are present, the episode is by definition manic. Here the 1-week duration for mania is met and the delusion makes the episode a manic episode with psychotic features, supporting bipolar I disorder.

References: DSM-5-TR — Bipolar I Disorder: Manic Episode Criterion C; Hypomanic Episode Criterion E

☐ Approve ☐ Revise ☐ Retire Notes:

For 9 months, a 50-year-old has been convinced he has undiagnosed heart disease despite normal cardiac workups. He has few physical symptoms but checks his pulse many times a day and reads medical websites for hours. He reports little worry about other areas of life. Which diagnosis BEST fits?

A. Illness anxiety disorder ✓ KEY

Preoccupation with having a serious illness for at least 6 months, with absent or mild somatic symptoms and excessive checking, fits illness anxiety disorder.

B. Generalized anxiety disorder

GAD involves worry about many areas of life, whereas his worry is focused on having one serious disease.

C. Somatic symptom disorder

Somatic symptom disorder centers on distressing somatic symptoms; his symptoms are minimal.

D. Panic disorder

No recurrent unexpected panic attacks are described.

Rationale: Illness anxiety disorder involves preoccupation with having or acquiring a serious illness for at least 6 months, with somatic symptoms absent or only mild, high health anxiety, and excessive health-related behaviors (care-seeking type) or maladaptive avoidance (care-avoidant type). GAD is distinguished by worry spanning multiple events or activities, of which health may be only one.

References: DSM-5-TR — Illness Anxiety Disorder: criteria and differential diagnosis (GAD)

☐ Approve ☐ Revise ☐ Retire Notes:

A client with PTSD has sudden surges of intense fear with a racing heart, sweating, and trembling whenever she encounters reminders of the assault. She does not worry about having attacks out of the blue and has not changed her routine to prevent them. How should these attacks be documented?

A. Add a separate diagnosis of panic disorder

Panic disorder requires recurrent unexpected attacks plus a month of worry or behavior change, neither of which is present.

B. Replace PTSD with panic disorder with agoraphobia

The attacks are cued by trauma reminders and agoraphobia is not described; DSM-5-TR also diagnoses panic disorder and agoraphobia separately.

C. Record PTSD with the "with panic attacks" specifier ✓ KEY

DSM-5-TR allows "with panic attacks" as a specifier for any mental disorder in which panic attacks occur.

D. Leave them out, as expected attacks are not recorded

Expected panic attacks can be recorded through the specifier; they are clinically meaningful.

Rationale: In DSM-5-TR, panic attacks can be expected or unexpected and are not themselves a mental disorder. When they occur in the context of another disorder, the "with panic attacks" specifier may be added (for example, PTSD with panic attacks). Panic disorder is reserved for recurrent unexpected attacks followed by at least 1 month of persistent concern or maladaptive behavior change.

References: DSM-5-TR — Panic Attack Specifier

☐ Approve ☐ Revise ☐ Retire Notes:

For 5 weeks, a 9-year-old has refused sleepovers, had nightmares about her mother being hurt, and sobbed whenever her mother leaves for work. The symptoms are causing problems at home and school. Her father asks whether a diagnosis of separation anxiety disorder requires symptoms to last 6 months. What is the minimum duration for a child?

A. Six months, the same as for adults

Six months is the typical duration for adults; children need a shorter duration.

B. At least 4 weeks ✓ KEY

DSM-5-TR requires at least 4 weeks of symptoms in children and adolescents.

C. At least 3 months

Three months is not the DSM-5-TR duration for separation anxiety disorder.

D. At least 2 weeks

Two weeks is the major depressive episode duration, not the separation anxiety duration.

Rationale: Separation anxiety disorder requires developmentally inappropriate and excessive fear about separation from attachment figures, shown by at least three of the listed symptoms, lasting at least 4 weeks in children and adolescents and typically 6 months or more in adults. DSM-5 removed the requirement that onset occur before age 18, so the disorder can also be diagnosed in adults.

References: DSM-5-TR — Separation Anxiety Disorder, Criterion B (duration)

☐ Approve ☐ Revise ☐ Retire Notes:

A 34-year-old who endured years of childhood abuse reports intrusive memories and avoidance, along with pervasive shame, a sense of being worthless, great difficulty calming strong emotions, and pulling away from close relationships. A colleague wants to give a DSM-5-TR diagnosis of "complex PTSD." Which statement is MOST accurate?

A. Complex PTSD is a DSM-5-TR subtype of PTSD that is recorded with a specifier

DSM-5-TR has no complex PTSD subtype or specifier; its PTSD specifiers are dissociative symptoms and delayed expression.

B. Shame and emotion dysregulation rule out PTSD, so another diagnosis is needed

Negative self-beliefs, persistent negative emotions, and detachment are part of PTSD Criterion D.

C. Borderline personality disorder must be diagnosed when these features appear

These features overlap with borderline personality disorder but do not require that diagnosis; a full assessment is needed.

D. Complex PTSD is an ICD-11 diagnosis; in DSM-5-TR, PTSD covers these features ✓ KEY

ICD-11 includes complex PTSD; DSM-5-TR captures these features within PTSD, chiefly Criterion D.

Rationale: ICD-11 lists complex PTSD, which adds disturbances in self-organization (affect dysregulation, negative self-concept, relational difficulties) to core PTSD symptoms, usually after prolonged or repeated trauma. DSM-5-TR has no separate complex PTSD diagnosis; its broadened PTSD criteria, especially negative alterations in cognitions and mood, capture many of these features. Counselors should know both systems when reading records and research.

References: ICD-11 — 6B41 Complex post-traumatic stress disorder · DSM-5-TR — Posttraumatic Stress Disorder, Criterion D and specifiers

☐ Approve ☐ Revise ☐ Retire Notes:

A 40-year-old with PTSD after an armed robbery at her store is ready for trauma-focused treatment. She keeps telling herself, "It was my fault for staying open late," and says she would rather not retell the event in detail session after session. Which evidence-based approach BEST matches her needs and preferences?

A. Prolonged exposure, which centers on repeated imaginal retelling

PE is effective, but repeated imaginal exposure to the memory is its core, which she prefers to avoid.

B. Critical incident stress debriefing, held in a single group session

Single-session debriefing is not an evidence-based treatment for PTSD.

C. Cognitive processing therapy, which targets stuck points like self-blame ✓ **KEY**

CPT focuses on identifying and challenging stuck points such as self-blame, and a written account is optional.

D. Psychological first aid, focused on early safety, comfort, and practical help

PFA is an early post-event support approach, not a treatment for established PTSD.

Rationale: Both prolonged exposure and cognitive processing therapy are strongly recommended for PTSD. PE uses repeated imaginal exposure to the trauma memory and in vivo exposure to avoided situations. CPT helps clients identify and challenge "stuck points," such as self-blame, and can be delivered without a written trauma account. Client preference is a legitimate factor in choosing among effective treatments.

References: APA CPG — Clinical Practice Guideline for the Treatment of PTSD (2017): CPT and PE recommendations

☐ Approve ☐ Revise ☐ Retire Notes:

A paramedic reports intrusive images, avoidance, and constant hypervigilance after years of responding to fatal crashes, including the deaths of children. He was never in danger himself. A peer tells him he cannot have PTSD because his own life was never threatened. Which statement is MOST accurate?

A. Repeated or extreme exposure to aversive details on the job can meet Criterion A ✓ **KEY**

Criterion A4 includes repeated or extreme exposure to aversive details, as with first responders.

B. Only a direct threat to his own life or body can satisfy the stressor criterion

Criterion A also includes witnessing, learning about a close other's trauma, and repeated work exposure.

C. Secondary traumatic stress is the DSM-5-TR diagnosis that applies in its place

Secondary traumatic stress is a useful concept but is not a DSM-5-TR diagnosis.

D. His symptoms are better classed as adjustment disorder because of his job role

Occupational exposure does not shift the diagnosis to adjustment disorder when PTSD criteria are met.

Rationale: PTSD Criterion A can be met by direct exposure, witnessing, learning that a traumatic event happened to a close family member or friend, or experiencing repeated or extreme exposure to aversive details of traumatic events, as with first responders or police. Exposure through electronic media counts only when it is work related. First responders face cumulative exposure and should be assessed accordingly.

References: DSM-5-TR — Posttraumatic Stress Disorder, Criterion A4

☐ Approve ☐ Revise ☐ Retire Notes:

A 23-year-old spends hours each day picking at small bumps on her face and arms, causing open sores. She has tried many times to stop, feels ashamed, and wears long sleeves to hide the marks. A physician found no skin condition, and she uses no stimulants. Which diagnosis is MOST likely?

A. Trichotillomania

Trichotillomania involves recurrent hair pulling, not skin picking.

B. Excoriation disorder ✓ KEY

Recurrent skin picking causing lesions, with repeated attempts to stop and distress, fits excoriation disorder.

C. Body dysmorphic disorder

BDD picking aims to fix a perceived appearance flaw; no such preoccupation is described.

D. Nonsuicidal self-injury

NSSI involves intentional self-injury for relief or other functions, not a body-focused repetitive habit.

Rationale: Excoriation (skin-picking) disorder, in the obsessive-compulsive and related disorders chapter, requires recurrent skin picking resulting in skin lesions, repeated attempts to decrease or stop, and clinically significant distress or impairment. The picking must not be due to a substance, such as stimulants, or a dermatologic condition, and must not be better explained by another disorder, such as BDD.

References: DSM-5-TR — Excoriation (Skin-Picking) Disorder: criteria and differential diagnosis

☐ Approve ☐ Revise ☐ Retire Notes:

Q370 of 1000 · nce-s-clf-130 · Counseling and Helping Relationships · obsessive-compulsive and related disorders · medium · In review

A counselor and a client with contamination-focused OCD are building an exposure and response prevention (ERP) hierarchy. Which practice is MOST consistent with ERP?

A. Start with the most feared item so that change comes quickly

Beginning at the top of the hierarchy is flooding and can reduce willingness to continue; ERP usually starts lower.

B. Allow one quick wash after each exposure to keep distress low

Washing after exposure is a compulsion that undercuts response prevention.

C. End each exposure as soon as the client's distress starts to rise

Ending exposures when distress rises rewards escape and prevents new learning.

D. Begin with tasks rated as moderately distressing and work upward ✓ KEY

Graded ERP typically starts with moderately distressing items and moves up the hierarchy.

Rationale: In ERP, the client rates feared situations (often with subjective units of distress) to build a graded hierarchy, then repeatedly confronts them while refraining from compulsions, including subtle or mental rituals. Exposures usually begin with moderately distressing items and progress upward. Remaining in the situation without ritualizing lets the client learn that feared outcomes do not occur and that distress can be tolerated.

References: NICE guidelines — CG31 Obsessive-compulsive disorder and body dysmorphic disorder: CBT including ERP

► **Check:** NICE CG31 supports ERP; the graded-hierarchy details are standard ERP practice rather than guideline text. No OCD-specific treatment manual is on the approved list.

☐ Approve ☐ Revise ☐ Retire Notes:

A 48-year-old who has drunk heavily every day for years plans to quit "cold turkey" this weekend while home alone. He mentions morning shakes and sweats that go away once he has a drink. What should the counselor do FIRST?

A. Affirm his plan and schedule a check-in call for Monday

Affirming an unsupervised stop ignores the risk of dangerous withdrawal.

B. Start a relapse prevention plan focused on his triggers

Relapse prevention matters later; immediate medical safety comes first.

C. Refer him for medical evaluation before he stops drinking ✓ **KEY**

Signs of physical dependence call for medical assessment and supervised withdrawal management.

D. Suggest he taper on his own by cutting back day by day

An unsupervised self-taper still carries withdrawal risk; the decision belongs with a medical provider.

Rationale: Alcohol withdrawal can progress to seizures and withdrawal delirium, which can be fatal. Morning tremor and sweating relieved by drinking suggest physical dependence, so the counselor should arrange medical evaluation to determine the appropriate withdrawal management setting before the client stops. ASAM Dimension 1 (acute intoxication and withdrawal potential) guides this decision.

References: ASAM Criteria — Dimension 1: acute intoxication and/or withdrawal potential · DSM-5-TR — Alcohol Withdrawal: features and complications

☐ Approve ☐ Revise ☐ Retire Notes:

A 16-year-old is referred after cannabis is found in his backpack. He uses most days and his grades are slipping. He says, "Weed is natural. It doesn't hurt anybody." Which counselor response BEST reflects a motivational rather than a confrontational approach?

A. "Research shows cannabis harms the teen brain, so you need to stop now."

Leading with warnings and demands is the righting reflex and tends to increase resistance.

B. "Until you admit you have a problem, counseling will not help you much."

Requiring the client to accept a label is a confrontational stance MI avoids.

C. "It feels pretty harmless to you. What, if anything, concerns you about it?" ✓ **KEY**

A reflection followed by an open question rolls with discord and invites the teen's own reasons for change.

D. "Your parents and school are worried, so let's agree that you quit today."

Pressing for a commitment the teen has not voiced bypasses his ambivalence and autonomy.

Rationale: Motivational interviewing avoids arguing, labeling, and the righting reflex, which tend to provoke sustain talk, especially in adolescents sensitive to autonomy. Instead, the counselor reflects the client's view and asks open questions that evoke the client's own concerns and reasons for change. Research does not support confrontational approaches for substance use.

References: Miller & Rollnick (MI) — The righting reflex; responding to sustain talk and discord · SAMHSA TIP 35 — Motivational approaches with adolescents

☐ Approve ☐ Revise ☐ Retire Notes:

A 32-year-old seeks an ADHD evaluation for lifelong disorganization, forgetfulness, and trouble finishing tasks at work and home. Which statement about diagnosing ADHD in adults under DSM-5-TR is accurate?

A. Five symptoms in a domain are needed for those aged 17 and older ✓ **KEY**

DSM-5-TR lowers the threshold to five symptoms in a domain for older adolescents and adults.

B. Six symptoms in a domain are needed at every age, adults included

Six symptoms are required for children up to age 16; five for those 17 and older.

C. Impairment from symptoms must be documented from before age 7

DSM-5 replaced age 7 with several symptoms present before age 12; impairment need not be documented then.

D. Symptoms need to appear in only one setting, such as the workplace

Several symptoms must be present in two or more settings.

Rationale: For ADHD, children up to age 16 need six or more symptoms of inattention and/or hyperactivity-impulsivity, while adolescents aged 17 and older and adults need five or more. Several symptoms must have been present before age 12 and in two or more settings. Adult presentations often show more inattention and inner restlessness than overt hyperactivity.

References: DSM-5-TR — Attention-Deficit/Hyperactivity Disorder, Criterion A (symptom thresholds by age)

☐ Approve ☐ Revise ☐ Retire Notes:

An autistic 8-year-old speaks in full sentences but struggles with back-and-forth conversation and making friends. His insistence on sameness and great distress at small changes markedly interfere with functioning across settings. How should severity be recorded under DSM-5-TR?

A. Assign one overall level, based mainly on his measured IQ

Severity is not based on IQ; intellectual impairment is recorded with a separate specifier.

B. Rate social communication and restricted behaviors separately ✓ **KEY**

DSM-5-TR rates the level of support needed separately for each of the two domains.

C. Assign a support level only if he has intellectual disability

Severity levels are recorded for all autistic individuals, with or without intellectual disability.

D. Use only the more impaired domain to set one combined level

The domains are rated separately, so a child may need different levels of support in each.

Rationale: DSM-5-TR asks clinicians to specify current severity of autism spectrum disorder separately for social communication and for restricted, repetitive behaviors, using levels 1 (requiring support), 2 (requiring substantial support), and 3 (requiring very substantial support). Severity can change across contexts and over time, and it is recorded alongside specifiers for intellectual and language impairment.

References: DSM-5-TR — Autism Spectrum Disorder: severity levels for each domain

☐ Approve ☐ Revise ☐ Retire Notes:

Two weeks after giving birth, a 28-year-old suddenly develops delusions and disorganized speech. She has no manic or depressive symptoms. With treatment, she returns fully to her prior level of functioning after 10 days. Which diagnosis BEST fits?

A. Schizophreniform disorder, with good prognostic features

Schizophreniform disorder requires symptoms lasting at least 1 month.

B. Major depressive disorder, with peripartum onset

No depressive episode is described.

C. Schizophrenia, first episode, currently in acute episode

Schizophrenia requires continuous signs for at least 6 months.

D. Brief psychotic disorder, with peripartum onset ✓ **KEY**

Psychotic symptoms lasting at least 1 day but less than 1 month, beginning within 4 weeks postpartum, fit this diagnosis and specifier.

Rationale: Brief psychotic disorder requires at least one of delusions, hallucinations, or disorganized speech (plus possibly grossly disorganized behavior), lasting at least 1 day but less than 1 month, with full return to premorbid functioning. The "with peripartum onset" specifier applies when onset is during pregnancy or within 4 weeks postpartum. Postpartum psychosis is a psychiatric emergency, and clinicians should also consider an underlying bipolar disorder.

References: DSM-5-TR — Brief Psychotic Disorder: criteria and specifiers

☐ Approve ☐ Revise ☐ Retire Notes:

Parents of a 17-year-old report that over 8 months he has pulled away from friends, his grades have dropped, he neglects hygiene, and he has become suspicious and voices odd ideas. He has no clear hallucinations or fixed delusions and does not use substances. What is the MOST appropriate action?

A. Refer for a full evaluation, as these may be early signs of psychosis ✓ **KEY**

A decline in functioning with emerging odd beliefs can signal a prodrome; early evaluation and intervention improve outcomes.

B. Reassure the parents that these are typical changes during adolescence

A marked, sustained decline with suspiciousness goes beyond typical adolescent change.

C. Diagnose schizophrenia, since negative signs have lasted over 6 months

Schizophrenia requires an active phase of at least 1 month of Criterion A symptoms, which is not present.

D. Wait to act until hallucinations or delusions have clearly appeared

Waiting for frank psychosis delays care; a long duration of untreated psychosis worsens outcomes.

Rationale: Prodromal signs of schizophrenia often include social withdrawal, declining school or work performance, poor self-care, and attenuated positive symptoms such as unusual beliefs or suspiciousness. These signs do not establish a diagnosis, but they warrant prompt referral for comprehensive evaluation, because shorter duration of untreated psychosis is associated with better outcomes. DSM-5-TR lists attenuated psychosis syndrome as a condition for further study.

References: DSM-5-TR — Schizophrenia: development and course (prodromal phase) · DSM-5-TR — Conditions for Further Study: Attenuated Psychosis Syndrome

☐ Approve ☐ Revise ☐ Retire Notes:

A 30-year-old cannot decide what to wear or what to order at a restaurant without reassurance from her mother. She agrees with others even when she disagrees, for fear of losing their support, feels helpless when alone, and quickly began a new relationship after a breakup. Which personality disorder BEST fits?

A. Avoidant personality disorder

Avoidant PD centers on fear of criticism and rejection leading to social avoidance, not a need to be taken care of.

B. Histrionic personality disorder

Histrionic PD involves attention seeking and shallow, shifting emotion.

C. Dependent personality disorder ✓ KEY

Difficulty making everyday decisions, fear of disagreeing, and urgently seeking a new relationship fit dependent PD.

D. Borderline personality disorder

Borderline PD involves unstable identity, affect, and relationships, not submissive reliance for everyday decisions.

Rationale: Dependent personality disorder, in Cluster C, is a pervasive and excessive need to be taken care of that leads to submissive, clinging behavior and fears of separation. Criteria include difficulty making everyday decisions without excessive advice, difficulty expressing disagreement, feeling helpless when alone, and urgently seeking another relationship when one ends.

References: DSM-5-TR — Dependent Personality Disorder: criteria and differential diagnosis

☐ Approve ☐ Revise ☐ Retire Notes:

A 45-year-old manager follows rules rigidly, misses deadlines because work is never perfect enough, will not delegate, is reluctant to spend money, and has let friendships lapse because of work. He sees his ways as simply "the right way." He reports no intrusive thoughts or rituals. Which statement is MOST accurate?

A. OCD fits best, since his perfectionism counts as an obsession

Perfectionism that is experienced as correct is a trait, not an intrusive, unwanted obsession.

B. OCPD fits; his traits form a pattern without true obsessions ✓ KEY

His pervasive, ego-syntonic perfectionism, rigidity, and miserliness, without obsessions or compulsions, fit OCPD.

C. OCPD can be diagnosed only when OCD is also present with it

OCPD does not require OCD; the two may co-occur but are distinct.

D. Narcissistic PD fits, since he sees his own way as superior

His focus is on order and control, not grandiosity or a need for admiration.

Rationale: Obsessive-compulsive personality disorder is a pervasive pattern of preoccupation with orderliness, perfectionism, and control at the expense of flexibility and efficiency, including excessive devotion to work, reluctance to delegate, and miserly spending. Unlike OCD, it involves no true obsessions or compulsions, and the traits are usually ego-syntonic. The two can co-occur, but neither requires the other.

References: DSM-5-TR — Obsessive-Compulsive Personality Disorder: differential diagnosis (OCD)

☐ Approve ☐ Revise ☐ Retire Notes:

Parents of a 7-year-old say he eats only a handful of familiar foods and refuses new textures. A pediatrician has ruled out a medical cause. Which finding would MOST support avoidant/restrictive food intake disorder (ARFID) rather than ordinary picky eating?

A. He refuses most vegetables and prefers plain pasta at dinner

Food preferences like this are common in young children and do not show clinical significance.

B. He eats more readily when meals are served in a favorite bowl

This is a typical childhood preference with no sign of nutritional or functional impact.

C. He complains that many new foods look strange or smell bad to him

Sensory dislikes occur in both picky eating and ARFID, so they do not distinguish them.

D. He now relies on nutritional supplements and growth has faltered ✓ **KEY**

Faltering growth and dependence on supplements are among the consequences that define ARFID.

Rationale: ARFID requires an eating disturbance that leads to at least one of: significant weight loss or faltering growth in children, significant nutritional deficiency, dependence on enteral feeding or oral nutritional supplements, or marked interference with psychosocial functioning. Picky eating without these consequences is developmentally common. ARFID also differs from anorexia nervosa in lacking body image disturbance.

References: DSM-5-TR — Avoidant/Restrictive Food Intake Disorder, Criterion A

☐ Approve ☐ Revise ☐ Retire Notes:

A 20-year-old in outpatient counseling for anorexia nervosa says her recent blood work was "totally normal," so she no longer needs to see her physician. Which counselor response is MOST accurate?

A. Normal labs do not rule out serious risk, so medical monitoring should continue ✓ **KEY**

Laboratory results can be normal despite serious medical risk, so ongoing medical monitoring remains part of care.

B. Normal labs show she is medically stable and can pause physician visits for now

Normal labs do not establish medical stability in anorexia nervosa.

C. Medical monitoring is needed only if she begins to report purging behaviors

Restriction alone carries significant medical risk; monitoring is not limited to purging.

D. The counselor can track her physical health during sessions in place of a doctor

Medical monitoring is outside counseling scope and requires a qualified medical provider.

Rationale: Anorexia nervosa has one of the highest mortality rates among mental disorders, and laboratory values can remain normal even when risk is serious. Treatment is multidisciplinary: the counselor coordinates with a medical provider who monitors physical health throughout care, and helps the client see monitoring as part of recovery rather than a sign of failure.

References: NICE guidelines — NG69 Eating disorders: recognition and treatment, physical health monitoring · DSM-5-TR — Anorexia Nervosa: functional consequences and laboratory findings

☐ Approve ☐ Revise ☐ Retire Notes:

A nursing home resident with moderate Alzheimer's disease has, over the past 2 days, become much more confused, drowsy at times and agitated at others, and unable to keep her attention on a conversation. Staff say, "Her dementia is just getting worse." What should the counselor do?

A. Agree that this is expected decline and update her care plan

Alzheimer's disease progresses gradually; an abrupt change over days is not expected progression.

B. Begin reminiscence sessions to help reorient her to the past

Reminiscence does not address a possible acute medical cause of the change.

C. Alert medical staff promptly, as this may signal a delirium ✓ **KEY**

Acute onset, fluctuating alertness, and inattention suggest delirium, which needs urgent medical evaluation.

D. Repeat a cognitive screen in a month to compare the scores

Waiting a month delays care for a potentially serious, treatable medical cause.

Rationale: Delirium is a disturbance in attention and awareness that develops over hours to days, fluctuates, and is caused by a medical condition, substance, or medication. People with major neurocognitive disorder are at higher risk, and delirium superimposed on dementia is often missed because staff attribute the change to the dementia. Any abrupt change in cognition calls for prompt medical evaluation.

References: DSM-5-TR — Delirium: criteria, risk factors, and differential diagnosis (major neurocognitive disorder)

☐ Approve ☐ Revise ☐ Retire Notes:

A month after her grandfather died, a 4-year-old keeps asking when he will be finished being dead and come home. Her parents worry that something is wrong. What BEST explains her questions?

A. Her questions show a denial that calls for treatment

This reflects normal cognitive development, not pathological denial.

B. Young children often do not yet see death as permanent ✓ **KEY**

Preschoolers commonly view death as temporary or reversible; irreversibility is understood later.

C. She is showing early signs of prolonged grief disorder

Her questions are developmentally expected, and too little time has passed for that diagnosis.

D. She is too young to grieve, so the topic should wait

Young children do grieve and benefit from simple, honest explanations.

Rationale: Children gradually develop an understanding of death's components: irreversibility (permanence), nonfunctionality (the body stops working), universality (all living things die), and causality. Preschoolers often think of death as temporary, like sleep or a trip. Adults can help by using clear, concrete words such as "died" rather than euphemisms, answering repeated questions patiently, and including children in mourning rituals as appropriate.

References: Berk (Development Through the Lifespan) — Death, dying, and bereavement: children's understanding of death · Worden (Grief Counseling) — Grieving children: developmental understanding of death

☐ Approve ☐ Revise ☐ Retire Notes:

A 52-year-old's former husband, with whom she stayed close after their divorce, died suddenly. His family did not invite her to the funeral, coworkers seem puzzled that she is upset, and she says she has "no right" to grieve. Which concept BEST describes her experience?

A. Anticipatory grief

Anticipatory grief occurs before an expected death; this death was sudden.

B. Prolonged grief disorder

Nothing indicates the duration or impairment required for prolonged grief disorder.

C. Chronic grief

Chronic grief refers to excessively long mourning, not grief that lacks social recognition.

D. Disenfranchised grief ✓ KEY

Grief that is not openly acknowledged, socially supported, or publicly mourned is disenfranchised grief.

Rationale: Disenfranchised grief, a term introduced by Doka, describes grief that is not openly acknowledged or socially supported, such as the death of a former spouse, an unrecognized partner, a pet, or a pregnancy loss. Lack of social recognition removes supports and rituals and can complicate mourning. Counselors can validate the loss and help clients create their own ways to mourn.

References: Worden (Grief Counseling) — Mediators of mourning: social mediators and socially unrecognized losses

► **Check:** The term is Doka's; Worden discusses socially unspeakable/negated losses among social mediators. SME to confirm the Worden locator wording.

☐ Approve ☐ Revise ☐ Retire Notes:

The husband of a woman with advancing ALS says he already grieves the losses of her voice and their plans for retirement. He feels guilty "mourning someone who is still here." Which counselor response is MOST helpful?

A. Normalize his anticipatory grief and help him face both loss and time left ✓ KEY

Anticipatory grief is a normal response to expected loss; counseling can attend to it while supporting time together.

B. Advise him to set the grief aside until after she has died, to stay strong

Suppressing grief is not recommended and adds to his guilt and isolation.

C. Explain that grieving now shows a detachment that could harm her care

Anticipatory grief does not indicate harmful detachment and should not be pathologized.

D. Assure him that grieving now means he will grieve far less after she dies

Anticipatory grief does not reliably reduce grief after the death, so this promise is misleading.

Rationale: Anticipatory grief is mourning that begins before an expected death, often in response to a series of losses during a progressive illness. It is normal and may include guilt, sadness, and fatigue. Worden notes that it does not necessarily shorten or lessen grief after the death; counselors can normalize it, address the losses already occurring, and help the family make meaningful use of the time remaining.

References: Worden (Grief Counseling) — Anticipatory grief

☐ Approve ☐ Revise ☐ Retire Notes:

A counselor returns from a meeting to four voicemails from clients. Which client should the counselor contact FIRST?

A. A client who reports her panic attacks are back and she cannot sleep

This is distressing and needs follow-up, but it shows no imminent danger.

B. A client who is furious about a custody ruling and wants to vent today

Anger about a court ruling warrants a call but shows no stated risk of harm.

C. A client who has written goodbye letters and given away her pet cats ✓ **KEY**

Goodbye letters and giving away belongings are preparatory behaviors that signal acute suicide risk.

D. A client who missed his appointment and would like to reschedule it

A routine scheduling request has the lowest priority.

Rationale: Crisis triage puts imminent risk to life first. Preparatory behaviors, such as writing goodbye notes or giving away valued possessions, are among the strongest warning signs of suicide and call for immediate contact and risk assessment, for example with the C-SSRS. Other clients' needs are real but can be addressed afterward.

References: C-SSRS — Suicidal behavior: preparatory acts or behavior

☐ Approve ☐ Revise ☐ Retire Notes:

After a high school student dies by suicide, the principal asks the school counselor to lead the response. Which action BEST reduces the risk of suicide contagion among students?

A. Set up a lasting memorial on campus where students can gather and grieve

Permanent or prominent memorials can glamorize the death; postvention guidance favors time-limited, low-key options.

B. Share facts without method details and screen students most at risk ✓ **KEY**

Safe messaging plus identifying and reaching out to vulnerable students are core postvention steps.

C. Avoid any mention of the death so that students do not become upset

Silence fuels rumors and leaves grieving students without support.

D. Explain that stress caused the death, so students have a simple answer

Attributing suicide to a single cause oversimplifies it and can lead others to identify with it.

Rationale: Postvention aims to support grief and prevent contagion. Recommended steps include sharing accurate information in small groups or classrooms without describing the method, avoiding glamorizing or simplifying the death, offering time-limited and low-key memorial options, and actively identifying and reaching out to students at elevated risk, such as close friends, those with prior suicidal behavior, and those exposed to the death.

References: Gladding (Counseling: A Comprehensive Profession) — Crisis counseling in schools: responding after a suicide

► **Check:** Content follows AFSP/SPRC "After a Suicide: A Toolkit for Schools," which is not on the approved source list; the Gladding locator is a general crisis-counseling citation.

☐ Approve ☐ Revise ☐ Retire Notes:

A client who recently left an abusive partner says he keeps appearing wherever she goes and texts her details about her day. She plans to change all her passwords and delete her accounts tonight. Which counselor response is MOST appropriate?

A. Plan with her first, since sudden changes may alert him and raise risk ✓ KEY

Abrupt loss of access can signal to an abuser that the victim is acting and may escalate danger, so changes are planned as part of safety planning.

B. Urge her to block him and delete every account tonight to cut off his access

This may feel protective but can provoke escalation and destroy evidence of stalking.

C. Reassure her that tracking her is unlikely unless he holds her phone

Shared accounts, spyware, location sharing, and tracking devices allow monitoring without physical access.

D. Suggest she meet with him in person to demand that he stop following her

Confronting a stalking ex-partner can increase risk; contact decisions belong in safety planning.

Rationale: Technology-facilitated abuse and stalking are common after separation, which is itself a high-risk period. Safety planning considers how the abuser may be monitoring the client (shared accounts, location sharing, spyware, trackers), uses a safer device for sensitive contacts, documents incidents for possible legal action, and times changes so they do not alert the abuser. Connecting the client with a domestic violence advocate adds specialized expertise.

References: SAMHSA TIP 57 — Trauma-informed principles: safety and client choice · Gladding (Counseling: A Comprehensive Profession) — Counseling survivors of intimate partner violence

► **Check:** Technology-safety guidance comes from NNEDV Safety Net materials, which are not on the approved source list; citations are general.

☐ Approve ☐ Revise ☐ Retire Notes:

A mother says her 6-year-old was "asleep upstairs" during her partner's assaults on her and was never hit, so the girl has not been affected. Since the mother left, the child has had nightmares, startles easily, and clings to her at bedtime. Which statement is MOST accurate?

A. Children who are not hit themselves seldom show lasting effects

Children exposed to IPV show emotional and behavioral effects comparable to those of directly abused children.

B. Her symptoms most likely reflect a normal phase at her age

Nightmares, startle, and clinging after exposure to violence are trauma-related signs, not a typical phase.

C. Young children forget such events, so focus on the mother

Young children are affected by violence they overhear, and they need their own assessment and support.

D. Hearing or seeing IPV can cause trauma reactions in children ✓ KEY

Witnessing or overhearing violence against a caregiver is a traumatic exposure that can produce trauma symptoms.

Rationale: Children often see or hear IPV even when parents believe they are asleep or unaware. DSM-5-TR PTSD criteria for children 6 years and younger specifically include witnessing traumatic events occurring to others, especially primary caregivers. Exposed children may show sleep problems, hyperarousal, regression, and clinginess, and benefit from trauma-informed assessment and a strengthened caregiver relationship.

References: DSM-5-TR — Posttraumatic Stress Disorder for Children 6 Years and Younger, Criterion A2 · SAMHSA TIP 57 — Trauma across the life span: children exposed to violence

☐ Approve ☐ Revise ☐ Retire Notes:

A 53-year-old client says that since her youngest child left for college, she wanders the house feeling useless. Her husband, by contrast, is enjoying the extra time together. Based on life-span research on the empty nest, which factor MOST increases the risk that launching children will be a painful transition?

A. Having a spouse who adjusts to the change faster

A spouse adapting faster may highlight the contrast, but research does not identify it as the main risk factor.

B. An identity built mainly around the parenting role ✓ **KEY**

Empty-nest distress is most likely when a parent has invested identity chiefly in parenting and has few other roles to turn to.

C. Launching the last child after the age of fifty

Timing that fits social expectations (on-time launching) is usually easier; age over fifty is not itself the risk factor.

D. Children who move to a college far from the home

Distance may add sadness, but most parents adjust well regardless; the key risk lies in how central parenting is to the self.

Rationale: Most parents adjust well to the empty nest, and many report relief and improved marital satisfaction. Distress is more likely when parenting has been the core of a person's identity, when other roles are thin, or when children leave off-time. Counseling can help the client build roles and meaning beyond active parenting.

References: Berk (Development Through the Lifespan) — Middle adulthood: launching children and the empty nest

☐ Approve ☐ Revise ☐ Retire Notes:

Parents worry that their 22-year-old son has "failed to launch." In two years he has moved apartments three times, switched jobs twice, and ended two relationships. He is curious and hopeful about his future. Within Arnett's theory of emerging adulthood, how is this pattern BEST understood?

A. Instability, a typical feature of emerging adulthood ✓ **KEY**

Arnett names instability in love, work and residence as one of the defining features of emerging adulthood.

B. Identity diffusion that shows his development stalled

Diffusion (Marcia) means neither exploring nor committing; he is actively exploring, so the label does not fit.

C. Role confusion left over from an unresolved adolescence

Arnett frames these moves as normative for the age period, not as a failure of Erikson's adolescent stage.

D. Delayed launching caused by overinvolved, enmeshed parents

Nothing in the stem points to enmeshment; frequent change is expected in this period.

Rationale: Arnett describes emerging adulthood (roughly ages 18–25) by five features: identity explorations, instability, self-focus, feeling in-between, and possibilities. Frequent changes of residence, work and partners are expected. Counselors can reframe parents' worry while still supporting the young adult's goal setting.

References: Berk (Development Through the Lifespan) — Emerging adulthood (Arnett): features of the period

☐ Approve ☐ Revise ☐ Retire Notes:

A 47-year-old client works full time, is raising two teenagers, and now helps her mother, who has early dementia, with meals, bills and appointments. She is exhausted, snaps at her children, and feels guilty that she is "failing everyone." What should the counselor do FIRST?

A. Urge her to move her mother into a memory care facility

Placement may become an option, but directing it first ignores her values and skips assessment.

B. Explore early conflicts with her mother that fuel the guilt

Family history may matter later, but her current overload and well-being take priority.

C. Assess her caregiving load and link her to respite support ✓ **KEY**

For a "sandwich generation" caregiver, assessing strain and connecting her with respite, services and shared care is the practical first step.

D. Advise her to cut back her hours at work to free up time

Reducing work may add financial stress and is a decision for her after options are explored.

Rationale: Midlife adults who care for both children and aging parents (the "sandwich generation") face role overload that raises the risk of depression and burnout. Counselors first assess the demands and resources, then help the client mobilize respite, family sharing of tasks and community services, and address guilt and self-care.

References: Berk (Development Through the Lifespan) — Middle adulthood: caring for aging parents

☐ Approve ☐ Revise ☐ Retire Notes:

Four months after the birth of their first child, a couple seeks counseling. They argue about night feedings and chores, and both say they feel less close than before. Which statement BEST reflects research on the transition to parenthood?

A. Their conflict signals a poor match that a baby has exposed

A dip in satisfaction after a first birth is common and does not by itself show the couple is mismatched.

B. Satisfaction usually rises soon after a first child arrives

On average, relationship satisfaction declines modestly rather than rises after the first birth.

C. Strain mostly reflects postpartum mood in the birth parent

Postpartum mood should be screened, but strain on the couple is common even without a mood disorder.

D. Satisfaction often dips, less so when care is shared fairly ✓ **KEY**

Research shows a common decline in satisfaction that is smaller when partners share caregiving and chores in a way both see as fair.

Rationale: Studies of the transition to parenthood find that relationship satisfaction often declines after a first child, driven by sleep loss, role change and disputes over the division of labor. Couples who share child care and household tasks fairly, and who hold realistic expectations, show less decline. Counseling can target these modifiable factors.

References: Berk (Development Through the Lifespan) — Early adulthood: transition to parenthood and marital satisfaction

☐ Approve ☐ Revise ☐ Retire Notes:

A 39-year-old client with relapsing-remitting multiple sclerosis had seemed at peace with her diagnosis. After a new relapse, she is again angry, tearful and asking "why me?" Her counselor wonders if earlier progress was lost. Which understanding of adjustment to chronic illness BEST fits?

A. Adjustment is often cyclical, and reactions return as the illness changes ✓ **KEY**

Contemporary adjustment models describe nonlinear, recurring reactions, especially when the illness course shifts.

B. Her anger shows she skipped a stage and must now go back and finish it

Rigid stage sequences are not supported; people do not need to complete stages in a fixed order.

C. Once a client reaches acceptance, a return of grief signals a new disorder

Renewed grief after a relapse is expected, not evidence of a new mental disorder.

D. Her distress points to a failed treatment that calls for a higher level of care

An expected emotional response to a relapse does not by itself warrant a higher level of care.

Rationale: Models of psychosocial adaptation to chronic illness and disability describe reactions such as shock, anxiety, depression, anger and acknowledgment as overlapping and recurring rather than a fixed sequence. Changes in the illness, such as a relapse, can bring earlier reactions back. Counselors normalize this cycling and help the client re-engage coping resources.

References: Gladding (Counseling: A Comprehensive Profession) — Rehabilitation counseling: psychosocial adaptation to chronic illness and disability

► **Check:** Confirm Gladding (8th ed.) covers nonlinear psychosocial adaptation (e.g., Livneh & Antonak); a rehabilitation counseling text may be a better citation.

☐ Approve ☐ Revise ☐ Retire Notes:

A 50-year-old with chronic low back pain for three years also meets criteria for major depressive disorder. He tells the counselor, "Once the doctors fix my back, my mood will fix itself, so counseling can wait." Which response BEST reflects current understanding of chronic pain and depression?

A. Agree to pause counseling until medical treatment of the pain ends

Waiting leaves the depression untreated, and the two conditions reinforce each other.

B. Explain that his pain is likely psychological and needs no medical care

Chronic pain is real and needs medical care; implying it is "all in his head" damages trust.

C. Explain that pain and low mood feed each other and both can be treated ✓ **KEY**

Pain and depression are linked in both directions; psychological care can lift mood and lessen pain's grip on daily life.

D. Refer him to a pain clinic and close his counseling case for now

Coordinating with a pain clinic can help, but closing the case abandons treatment of his depression.

Rationale: Chronic pain and depression are closely linked, each worsening sleep, activity, and outlook in a cycle. Guidelines for chronic pain recommend psychological approaches such as CBT or ACT alongside medical care to reduce distress and disability. Counselors treat the depression now and coordinate with the medical team rather than waiting for the pain to resolve.

References: NICE guidelines — NG193 Chronic pain (primary and secondary) in over 16s: psychological therapy (CBT, ACT)

☐ Approve ☐ Revise ☐ Retire Notes:

A counselor works with a 68-year-old man who has cared for his wife with Parkinson's disease for six years. He reports feeling trapped, worn out and resentful, and the counselor wants a standard measure of how much strain caregiving places on him. Which instrument is designed for this purpose?

A. Mini-Mental State Examination

The MMSE screens cognitive impairment, not the strain caregiving places on a caregiver.

B. Zarit Burden Interview ✓ KEY

The Zarit Burden Interview measures perceived caregiver burden across health, finances, social life and the relationship.

C. Geriatric Depression Scale

The GDS screens for depression in older adults; it does not measure caregiving strain.

D. Beck Hopelessness Scale

The BHS measures negative expectations about the future, not caregiver burden.

Rationale: Caregiver burden is the physical, emotional, social and financial strain of caring for an ill or disabled person. The Zarit Burden Interview is a widely used self-report measure of it. High burden predicts caregiver depression and poorer health, so counselors screen for it and link caregivers to respite, support groups and services.

References: Hays (Assessment) — Assessment with older adults and caregivers: caregiver burden measures

► **Check:** Confirm Hays (Assessment) names the Zarit Burden Interview; the fact is standard but the citation locator is uncertain.

☐ Approve ☐ Revise ☐ Retire Notes:

A counselor uses the FICA tool to take a brief spiritual history at intake. The letters stand for Faith or belief, Importance or influence, a third element, and Address in care. What does the "C" ask about?

A. Conflict between faith and treatment

Conflicts may surface, but "C" in FICA does not stand for conflict.

B. Coping skills learned from faith

Faith-based coping may arise in the interview, but it is not what "C" names.

C. Commitment to religious practice

Level of practice may come up under importance, but "C" is not commitment.

D. Community the person belongs to ✓ KEY

The "C" in FICA asks whether the client is part of a spiritual or religious community and whether it is a support.

Rationale: Puchalski's FICA is a brief spiritual history: Faith or belief (what gives meaning), Importance or influence (how beliefs shape life and health choices), Community (membership in and support from a spiritual community), and Address in care (how the client wants these addressed in treatment). It lets counselors assess spirituality respectfully without imposing values.

References: Gladding (Counseling: A Comprehensive Profession) — Spirituality in counseling: spiritual assessment

► **Check:** FICA (Puchalski) components are standard; confirm Gladding mentions FICA or substitute a spirituality-in-counseling source.

☐ Approve ☐ Revise ☐ Retire Notes:

After a cancer diagnosis, a 61-year-old devout client says, "God is punishing me for my sins. Maybe this is what I deserve." She has stopped attending services and feels abandoned. In Pargament's research on religious coping, how is her response BEST classified, and what is it linked to?

A. Negative religious coping, tied to poorer adjustment ✓ KEY

Punishing-God reappraisals and feeling abandoned by God are negative religious coping, which predicts more distress.

B. Positive religious coping, tied to better adjustment

Positive coping includes benevolent reappraisals and seeking spiritual support, not a punishing-God view.

C. A deferring coping style, tied to passive acceptance

Deferring means leaving problems to God; her statement is about punishment and abandonment.

D. Spiritual bypassing, tied to avoidance of emotion

Spiritual bypassing uses spiritual ideas to avoid feelings; she is expressing distress, not avoiding it.

Rationale: Pargament distinguished positive religious coping (benevolent reappraisal, seeking spiritual support, collaborative problem solving) from negative religious coping (punishing-God reappraisal, feeling abandoned by God, questioning God's power). Negative religious coping is associated with greater depression and poorer health outcomes. Counselors assess for it and may collaborate with clergy when the client wishes.

References: Gladding (Counseling: A Comprehensive Profession) — Spirituality in counseling: religious coping

► **Check:** Pargament's positive/negative religious coping (RCOPE) is well established; confirm the Gladding locator or cite a spirituality-specific source.

☐ Approve ☐ Revise ☐ Retire Notes:

Eight months after her infant died, a client who has always been devout says she is furious at God and ashamed of feeling that way. She fears her anger means she has lost her faith. The counselor does not share her religion. Which response is MOST appropriate?

A. Suggest she set aside spiritual topics until her grief eases

Avoiding her spiritual struggle leaves a central part of her grief unaddressed.

B. Reassure her that a loving God would not want her to be angry

This imposes a theological view and may deepen her shame.

C. Normalize anger at God as a common struggle and explore it ✓ KEY

Spiritual struggles such as anger at God are common after loss; exploring them openly reduces shame and supports meaning making.

D. Refer her to clergy since faith is outside the counselor's role

Clergy consultation may help with consent, but spiritual concerns are within a counselor's scope when they affect well-being.

Rationale: Spiritual struggles, such as anger at God, doubt, or conflict with a faith community, are common after trauma and loss and are linked to distress when unaddressed. Counselors respond within the client's own framework, normalize the struggle, and explore it without imposing beliefs. Collaboration with clergy is an option the client can choose.

References: Gladding (Counseling: A Comprehensive Profession) — Spirituality in counseling: addressing spiritual concerns · ACA Code of Ethics — A.4.b Personal Values

☐ Approve ☐ Revise ☐ Retire Notes:

A veteran with PTSD has the same trauma-related nightmare several nights a week. His counselor has him write down the dream, change the storyline in any way he chooses, and rehearse the new version in imagery for a few minutes each day. Which intervention is this?

A. Cognitive processing therapy

CPT targets stuck points in beliefs about the trauma; it does not rescript dreams.

B. Imagery rehearsal therapy ✓ KEY

Imagery rehearsal therapy has clients change a recurring nightmare and rehearse the new version while awake.

C. Stimulus control therapy

Stimulus control re-links the bed with sleep; it does not change dream content.

D. Systematic desensitization

Systematic desensitization pairs relaxation with a fear hierarchy, not dream rescripting.

Rationale: Imagery rehearsal therapy (IRT) treats recurring nightmares, including PTSD-related ones. The client rewrites the nightmare in any way desired and rehearses the new script while awake, which often reduces nightmare frequency and distress. It can be used alongside trauma-focused therapy.

References: VA/DoD CPG — Management of PTSD: treatment of trauma-related nightmares (imagery rehearsal therapy)

► **Check:** Confirm the strength of the current VA/DoD PTSD CPG recommendation on IRT for nightmares; the item only asks for the definition.

☐ Approve ☐ Revise ☐ Retire Notes:

A 56-year-old client being treated for depression says he sleeps eight hours yet nods off while driving. His wife says he snores loudly and sometimes stops breathing and gasps at night. He also wakes with headaches and has high blood pressure. What should the counselor do?

A. Begin CBT-I with sleep restriction to consolidate sleep

Sleep restriction can worsen sleepiness and is not suited to a likely breathing disorder.

B. Teach sleep hygiene and review it again in two weeks

Sleep hygiene does not treat a possible breathing disorder, and the delay is unsafe given drowsy driving.

C. Treat the sleepiness as a symptom of his depression

Snoring, apneas and morning headaches point to a medical sleep problem, not only depression.

D. Refer him to his physician for a sleep apnea evaluation ✓ KEY

Loud snoring, witnessed breathing pauses, sleepiness and hypertension suggest obstructive sleep apnea, which needs medical evaluation.

Rationale: Obstructive sleep apnea hypopnea is a breathing-related sleep disorder marked by snoring, gasping or breathing pauses during sleep, and daytime sleepiness despite adequate sleep time. It is confirmed by polysomnography and treated medically. Untreated apnea can worsen mood and raises accident risk, so counselors refer promptly and continue mental health care.

References: DSM-5-TR — Breathing-related sleep disorders: obstructive sleep apnea hypopnea

☐ Approve ☐ Revise ☐ Retire Notes:

A 66-year-old retired engineer spent his first months of retirement traveling and finishing house projects. Now, about a year in, he feels let down, bored and unsure what his days are for. In Atchley's phases of retirement, which phase is he MOST likely in?

A. Honeymoon

The honeymoon phase is the early period of freedom and activity he has already moved past.

B. Reorientation

Reorientation follows disenchantment, as the retiree builds a realistic new routine.

C. Disenchantment ✓ KEY

Disenchantment is the letdown that can follow the honeymoon, when retirement does not meet expectations.

D. Termination

Termination is the end of the retirement role, often through illness or a return to work.

Rationale: Atchley described retirement phases: preretirement, honeymoon, disenchantment, reorientation, stability (routine), and termination. Not every retiree passes through each phase. Counselors can normalize disenchantment and help the client find meaningful roles such as volunteering, part-time work or learning.

References: Sharf (Career Development Theory) — Late career and retirement: adjustment to retirement

► **Check:** Atchley's phases are standard NCE content; confirm Sharf (6th ed.) presents them or cite Berk (late adulthood: retirement).

☐ Approve ☐ Revise ☐ Retire Notes:

A 63-year-old accountant plans to leave her full-time firm job next year. She does not want to stop working completely, so she is lining up part-time tax preparation work to ease the move into full retirement. What is this type of work called?

A. Bridge employment ✓ KEY

Bridge employment is paid work after leaving a career job and before full retirement.

B. Career plateauing

A plateau is a point where advancement stalls within a career, not a step toward retirement.

C. Role disengagement

Disengagement theory describes withdrawal from roles; she is staying engaged in work.

D. Maintenance stage

Super's maintenance stage involves holding one's place in a career, not shifting to new part-time work.

Rationale: Bridge employment is part-time, temporary or self-employed work that links a career job and full retirement. It can provide income, structure and social contact, and it is linked to better adjustment for many older workers. Career counselors help older clients weigh bridge options alongside finances, health and values.

References: Sharf (Career Development Theory) — Late career: older workers and bridge employment

► **Check:** Confirm Sharf uses the term bridge employment; Berk (late adulthood, retirement) is an alternative locator.

☐ Approve ☐ Revise ☐ Retire Notes:

A 9-year-old tells her school counselor, "My uncle does things to me at night that I don't like, but you can't tell anyone." She looks scared and stops talking. Which response BEST fits recommended practice for handling a sexual abuse disclosure?

A. Promise to keep her secret so she will feel safe telling more

The counselor is a mandated reporter and cannot promise secrecy; a broken promise harms trust.

B. Stay calm, thank her for telling, avoid leading questions, report ✓ **KEY**

A calm, supportive response with minimal open questions protects the child and later investigation; reporting is required.

C. Ask if her uncle touched her private parts so the facts are clear

This is a leading question that can contaminate her account for any later forensic interview.

D. Gather a full, detailed account now so she will not need to retell

Detailed questioning is the role of trained forensic interviewers, not the school counselor.

Rationale: When a child discloses sexual abuse, counselors stay calm, believe and thank the child, use the child's own words, ask only minimal open-ended questions needed to establish reasonable suspicion, explain honestly that they must tell people who can help, and report promptly. Detailed or leading questioning can distress the child and compromise the investigation.

References: ACA Code of Ethics — B.2.a Serious and Foreseeable Harm and Legal Requirements · State law — Mandated reporting of suspected child abuse

☐ Approve ☐ Revise ☐ Retire Notes:

A counselor has seen a 7-year-old for six months for anxiety. After a report of possible abuse, the CPS caseworker asks the counselor to conduct the investigative interview, since "she already trusts you." The counselor has no forensic interview training. What should the counselor do?

A. Agree, since an existing alliance will help her disclose more

Trust does not make a treating counselor a proper forensic interviewer; mixing roles harms both.

B. Agree if the caseworker watches and records the session live

Observation does not solve the dual role or the lack of forensic training.

C. Interview her alone first, then pass the notes on to the caseworker

Investigative questioning by the therapist risks contaminating evidence and the therapy relationship.

D. Decline the forensic role and support a trained interviewer's work ✓ **KEY**

Treating counselors avoid forensic roles with their own clients; a trained interviewer, often at a child advocacy center, conducts it.

Rationale: Forensic interviews are fact-finding interviews conducted by trained interviewers using structured, nonleading protocols, often at a child advocacy center. A treating counselor's role is therapeutic, and the ACA Code bars counselors from evaluating their own current or former clients for forensic purposes. The counselor can support the child and cooperate within the law while keeping roles separate.

References: ACA Code of Ethics — E.13.c Client Evaluation Prohibited

☐ Approve ☐ Revise ☐ Retire Notes:

A counselor in a small rural town sees a 34-year-old Army National Guard member just home from deployment. Compared with many active-duty service members, which stressor is MORE characteristic of Guard and Reserve members and their families?

A. Frequent moves between distant bases that uproot the children

Frequent permanent change of station moves are more typical of active-duty families.

B. Living in base housing within a closely knit military unit

Base housing and on-post community are features of active-duty life, not Guard life.

C. Distance from military supports and strain on civilian jobs ✓ **KEY**

Guard and Reserve families often live far from installations and must leave and return to civilian jobs.

D. Fewer total deployments over the course of an enlistment

Fewer deployments would lessen stress; the distinct burden lies in the civilian-military shift and limited support.

Rationale: National Guard and Reserve members move rapidly between civilian and military life. Their families often live far from installations and military support services, may be surrounded by civilians who do not understand deployment, and face disruption to civilian employment and income. Counselors in community settings help connect them to resources and address these transition stressors.

References: Gladding (Counseling: A Comprehensive Profession) — Counseling military populations: Guard and Reserve families

▶ **Check:** Content is standard; confirm Gladding addresses Guard/Reserve distinctions or cite a military counseling source.

☐ Approve ☐ Revise ☐ Retire Notes:

An 11-year-old lives with her father and stepmother on weekends. She enjoys baking with her stepmother but then feels guilty and turns cold toward her, saying, "Mom would be hurt if she knew I liked her." Which intervention BEST addresses this loyalty conflict?

A. Help her mother give clear permission to like her stepmother ✓ **KEY**

Explicit permission from the biological parent eases the child's loyalty bind most directly.

B. Have her stepmother step in as her primary mother on weekends

Asking the stepmother to replace her mother intensifies the loyalty conflict.

C. Advise the father to limit contact between the two households

Cutting contact does not resolve her guilt and may harm her relationship with her mother.

D. Tell her the guilt is a sign the stepfamily is not yet working

Loyalty conflicts are common in stepfamilies and do not mean the family is failing.

Rationale: Children in stepfamilies often feel that caring for a stepparent betrays the absent biological parent. These loyalty conflicts ease when both biological parents explicitly permit the child to have a good relationship with the stepparent, and when the stepparent is presented as an additional adult rather than a replacement.

References: Goldenberg (Family Therapy) — Remarried families: loyalty conflicts and stepfamily development

☐ Approve ☐ Revise ☐ Retire Notes:

A 64-year-old woman has taken custody of her two grandchildren because her daughter has a severe opioid use disorder. She loves them but says, "This was supposed to be my time." She is tired, short on money, and lies awake worrying about her daughter. Which concern is MOST characteristic of custodial grandparents?

A. A loss of interest in the grandchildren caused by aging

Nothing suggests lost interest; she loves the children and is stressed by the load.

B. An early sign of cognitive decline from added demands

Fatigue and worry do not indicate cognitive decline.

C. Rivalry with the grandchildren's other set of grandparents

Rivalry is not a characteristic theme for custodial grandparents.

D. Off-time role strain together with grief over her daughter ✓ **KEY**

Raising grandchildren is an off-time role, often with financial strain and grief or anger about the adult child.

Rationale: Grandparents raising grandchildren in skipped-generation households take on an unexpected, off-time parenting role. They often face financial strain, health demands, legal and school hurdles, social isolation from peers, and grief, anger or worry about the adult child whose problems led to the arrangement. Counselors address both the practical load and the losses.

References: Berk (Development Through the Lifespan) — Late adulthood: grandparents rearing grandchildren (skipped-generation families)

☐ Approve ☐ Revise ☐ Retire Notes:

A 20-year-old client says, "I'm probably gay. I've started going to an LGBTQ+ student group so I'm not so alone, but I'm not ready to say I like it about myself, and I haven't told my family." In Cass's model of homosexual identity formation, which stage BEST fits?

A. Identity comparison

In comparison the person first considers "I might be gay" and weighs the alienation; she has moved past this to seeking out community.

B. Identity tolerance ✓ **KEY**

In tolerance the person accepts "I probably am gay" and seeks contact with gay communities, tolerating rather than fully accepting the identity.

C. Identity acceptance

Acceptance brings a more positive view of the identity and more contact; she still only tolerates it.

D. Identity pride

Pride involves strong immersion and often anger at heterosexist society, which she does not show.

Rationale: Cass described six stages: identity confusion, comparison, tolerance, acceptance, pride, and synthesis. In tolerance, the person acknowledges a probable gay identity and seeks out others to reduce isolation, while still merely tolerating the identity. Stage models are useful lenses but not every client follows them in order.

References: Sue & Sue (Counseling the Culturally Diverse) — Counseling LGBTQ+ individuals: sexual identity development (Cass model)

☐ Approve ☐ Revise ☐ Retire Notes:

Religious parents of a 15-year-old who has come out as lesbian tell the counselor they cannot approve of her identity, but they love her and want to keep her safe. Which statement BEST reflects family acceptance research on LGBTQ+ youth?

A. Only her parents' full agreement with her identity lowers risk

Research shows benefit from reducing rejecting behaviors even before full acceptance.

B. Parental views matter little once teens find support from peers

Family acceptance and rejection have strong effects on youth health even with peer support.

C. Reducing rejecting behaviors lowers risk, even without agreement ✓ **KEY**

Family rejecting behaviors predict higher suicide, depression and substance risk; reducing them helps even without full agreement.

D. Teens facing rejection mainly benefit from moving out of the home early

Leaving home early raises risks such as homelessness; strengthening family support is preferred.

Rationale: Research from the Family Acceptance Project found that LGBTQ+ youth from highly rejecting families had far higher rates of suicide attempts, depression and substance use, while family acceptance was protective. Because even modest reductions in rejecting behaviors help, counselors can work with ambivalent families on specific behaviors, such as stopping slurs and supporting the teen at school, while respecting their values.

References: Sue & Sue (Counseling the Culturally Diverse) — Counseling LGBTQ+ individuals: family acceptance and rejection

► **Check:** Finding is from Ryan et al. (Family Acceptance Project); confirm Sue & Sue (9th ed.) discusses it or add a source with the original study.

☐ Approve ☐ Revise ☐ Retire Notes:

A 30-year-old woman resettled as a refugee three months ago after fleeing armed conflict. Through an interpreter she reports nightmares and poor sleep, and says she is worried about losing her apartment and about relatives still in a refugee camp. What should the counselor focus on FIRST?

A. Address safety and resettlement needs while building trust ✓ **KEY**

Trauma-informed, phase-oriented care starts with safety, stability and trust, including practical resettlement needs.

B. Begin detailed trauma narration to process her war memories

Detailed trauma processing comes after stabilization, not at the start amid housing insecurity.

C. Encourage her to adopt local customs to speed her adjustment

Pushing assimilation ignores her culture and can raise acculturative stress.

D. Teach her to set aside thoughts of her relatives in the camp

Her worry is realistic; suppressing it is neither feasible nor helpful.

Rationale: Refugees carry pre-migration trauma, migration losses and ongoing post-migration stressors such as housing, finances and separated family. Trauma-informed care begins with safety, stabilization and trust, uses qualified interpreters, and attends to practical needs before any trauma-focused work. Current stressors often drive distress as much as past events.

References: SAMHSA TIP 57 — Trauma-informed principles: safety and trustworthiness; refugee and immigrant populations

☐ Approve ☐ Revise ☐ Retire Notes:

A Korean immigrant family has lived in the United States for six years. The 15-year-old son speaks mostly English, wants to date, and argues that his parents' rules are "old-fashioned." His parents, who still speak mainly Korean, feel they are losing him. Which concept BEST describes this family conflict?

A. Typical adolescent rebellion unrelated to culture

Developmental strivings play a role, but the cultural gap is central to this conflict.

B. Marginalization of the son from both of his cultures

He is engaging with the host culture, not rejected by both cultures.

C. Cultural encapsulation of the parents' worldview

Cultural encapsulation (Wrenn) describes counselors who view others only through their own culture.

D. An intergenerational acculturation gap in the family ✓ KEY

Children often acculturate faster than parents, creating gaps in values and language that fuel conflict.

Rationale: An intergenerational acculturation gap arises when children adopt the host culture's language and values faster than their parents. The gap can strain communication, shift family roles, and heighten parent-child conflict. Counselors help family members understand each other's positions and find bicultural ground.

References: Sue & Sue (Counseling the Culturally Diverse) — Immigrant and refugee families: acculturation conflicts between generations

☐ Approve ☐ Revise ☐ Retire Notes:

Within two weeks, four ninth graders at a high school are found to be cutting after one student posted photos of her wounds online. The principal asks the school counselor how the school should respond. Which plan is MOST consistent with best practice for self-injury in schools?

A. Hold an assembly showing the warning signs and methods of cutting

Describing methods to large groups can spread self-injury through social contagion.

B. Meet each student alone, assess risk, inform parents, limit spread ✓ KEY

Individual assessment, parent notification and steps to curb contagion are core to a school protocol.

C. Start a support group where the four students share their cutting

Groups focused on sharing self-injury can reinforce it and spread it among members.

D. Keep the cutting from parents so students keep trusting staff

Parents of minors at risk should be informed; secrecy leaves students without home support.

Rationale: School responses to self-injury emphasize individual risk assessment (including suicide risk), parent notification, referral for treatment, and limiting social contagion. Counselors avoid large-group presentations about methods and process groups centered on self-injury, and they may address related online sharing. Students are offered support in a calm, nonjudgmental way.

References: Gladding (Counseling: A Comprehensive Profession) — School counseling: responding to self-injury and contagion · ACA Code of Ethics — B.5.b Responsibility to Parents and Legal Guardians

► **Check:** Best-practice content is standard (e.g., Walsh; ASCA); confirm the Gladding locator or cite a school self-injury protocol source.

☐ Approve ☐ Revise ☐ Retire Notes:

A counselor coordinating care for a client with bipolar I disorder learns that the psychiatrist is switching her from lithium to valproate. To support her in keeping medical appointments, the counselor wants to understand the lab monitoring involved. Which monitoring is MOST closely tied to valproate rather than lithium?

A. Kidney function and thyroid hormone levels

Kidney and thyroid monitoring is a hallmark of long-term lithium treatment.

B. Hydration status and daily salt intake

Dehydration and salt changes affect lithium levels, not valproate.

C. Liver function tests and platelet counts ✓ KEY

Valproate carries risks to the liver and blood platelets, so these are monitored.

D. Regular white blood cell counts for safety

Routine white cell monitoring is linked to clozapine, not valproate.

Rationale: Lithium has a narrow therapeutic range; monitoring includes blood levels, kidney and thyroid function, and attention to hydration and salt. Valproate monitoring focuses on liver function and blood counts, including platelets, and it carries serious risks in pregnancy. Counselors do not interpret labs but can encourage adherence to monitoring and communicate concerns to the prescriber.

References: NICE guidelines — CG185 Bipolar disorder: monitoring physical health with lithium and valproate

☐ Approve ☐ Revise ☐ Retire Notes:

Q414 of 1000 · nce-s-clf-174 · Counseling and Helping Relationships · psychopharmacology basics for counselors · medium · In review

A client who takes sertraline started St. John's wort on her own last week. In session she is restless and sweating, with shivering, muscle twitching, a racing heart and diarrhea. She says the symptoms began yesterday and are getting worse. What should the counselor recognize and do?

A. Possible serotonin syndrome needing urgent medical care ✓ KEY

Combining serotonergic agents can cause serotonin syndrome, which can become life-threatening and needs urgent care.

B. A panic attack to treat with breathing and grounding

Muscle twitching, diarrhea and sweating after adding a serotonergic herb point to a medical reaction, not panic.

C. SSRI discontinuation symptoms that will pass on their own

She has not stopped the SSRI; she added another serotonergic agent.

D. Neuroleptic malignant syndrome from her antidepressant

NMS is linked mainly to antipsychotics and features severe rigidity, not twitching after adding an herb.

Rationale: Serotonin syndrome results from excess serotonergic activity, often when an SSRI is combined with another serotonergic agent such as St. John's wort, tramadol, triptans or an MAOI. Signs include agitation, sweating, tremor or twitching, rapid heart rate, diarrhea and fever. Counselors do not manage medications but should recognize warning signs and direct the client to urgent medical care.

References: Gladding (Counseling: A Comprehensive Profession) — Psychopharmacology for counselors: antidepressant adverse effects

► **Check:** Clinical facts are standard; confirm Gladding covers serotonin syndrome or cite a psychopharmacology reference.

☐ Approve ☐ Revise ☐ Retire Notes:

A client who has taken a prescribed benzodiazepine daily for six years tells her counselor she is "done with pills" and plans to stop it all at once this weekend. Why should the counselor urge her to talk with her prescriber before stopping?

A. Stopping will cause relapse of her anxiety in all cases

Anxiety may return, but that is not certain and is not the main safety concern.

B. Stopping suddenly will lower her tolerance to alcohol use

This is not the key risk of abrupt benzodiazepine discontinuation.

C. Stopping is unsafe only if she took more than prescribed

Dependence develops with long-term use at prescribed doses too.

D. Abrupt withdrawal can bring severe effects like seizures ✓ **KEY**

Sedative withdrawal after long-term use can include seizures, so a supervised taper is needed.

Rationale: Long-term benzodiazepine use produces physiological dependence even at prescribed doses. DSM-5-TR sedative, hypnotic, or anxiolytic withdrawal can include autonomic hyperactivity, tremor, insomnia, anxiety, hallucinations and grand mal seizures. Counselors support the client's goal and urge a medically supervised taper.

References: DSM-5-TR — Sedative, hypnotic, or anxiolytic withdrawal: criteria

☐ Approve ☐ Revise ☐ Retire Notes:

Two months into her husband's deployment, a mother says their 4-year-old has started wetting the bed again, clings to her at daycare drop-off, and asks every night when Daddy is coming home. What guidance should the counselor give?

A. These signs point to a likely trauma disorder needing intensive care

Regression and clinging are common reactions to a parent's absence and do not by themselves suggest a trauma disorder.

B. Brief regression is common; keep routines and give honest reassurance ✓ **KEY**

Young children often regress during deployment; stable routines, simple honest answers and connection help.

C. Avoid mentioning her father so she is not reminded of his absence

Avoiding talk of the parent increases confusion; keeping him present through photos and messages helps.

D. Tell her Daddy will be home soon to calm her fears about his return

False reassurance can backfire; age-appropriate, honest information is better.

Rationale: Children's reactions to deployment vary by age. Preschoolers often show regression, clinginess and sleep problems; school-age children may show irritability or school problems; adolescents may take on adult roles or act out. Counselors help the at-home parent keep routines, give honest age-appropriate explanations, maintain connection with the deployed parent, and watch for persistent difficulties.

References: Berk (Development Through the Lifespan) — Early childhood: stress, separation and regression in young children

► **Check:** Content is standard deployment-cycle guidance; Berk does not address deployment directly, so a military family source may be preferable.

☐ Approve ☐ Revise ☐ Retire Notes:

A 14-year-old has missed 15 school days this term. Each morning he leaves home on time, then spends the day at a friend's house playing video games. He forged notes so his parents would not find out, and he shows no worry about school itself. Which pattern does this BEST fit?

A. School refusal due to anxiety about being away from home

School refusal involves emotional distress, often staying home with parents' knowledge.

B. School refusal driven by fear of a specific school setting

He shows no fear of school; he is avoiding it to do something more appealing.

C. Truancy, hidden from parents and without school anxiety ✓ **KEY**

Truancy is typically concealed from parents, lacks marked anxiety, and may involve conduct problems.

D. Avoidance linked to depression and loss of interest at school

Nothing points to depressed mood; he is actively seeking a rewarding activity elsewhere.

Rationale: School refusal involves emotional distress about attending, and the child usually stays home with the parents' knowledge. Truancy involves absence concealed from parents, little anxiety about school, and often conduct or peer-related problems. The distinction guides intervention: anxiety-focused exposure for refusal, versus behavioral contracts, parental monitoring and school engagement for truancy.

References: Gladding (Counseling: A Comprehensive Profession) — School counseling: attendance problems, school refusal and truancy

► **Check:** Distinction follows Berg/Kearney; confirm the Gladding locator or cite a school-attendance source.

☐ Approve ☐ Revise ☐ Retire Notes:

Over 2 years, a 57-year-old engineer has made crude remarks to strangers, lost interest in his grandchildren, shown little concern when his wife cries, and begun eating sweets compulsively. He still recalls recent events and finds his way around town without trouble. Which diagnosis BEST fits?

A. Major neurocognitive disorder due to Alzheimer's disease

Alzheimer's disease typically begins with prominent decline in learning and memory, which is relatively spared here.

B. Major neurocognitive disorder with Lewy bodies, early stage

Lewy body NCD features fluctuating cognition, recurrent visual hallucinations, and parkinsonism, none of which are described.

C. Frontotemporal neurocognitive disorder, behavioral variant ✓ **KEY**

Disinhibition, apathy, loss of empathy, and hyperorality with relatively spared memory and spatial skills fit the behavioral variant.

D. Bipolar I disorder, current episode manic, late onset

There is no elevated mood, decreased need for sleep, or episodic course; the change is gradual and progressive.

Rationale: The behavioral variant of frontotemporal neurocognitive disorder requires three or more of: behavioral disinhibition; apathy or inertia; loss of sympathy or empathy; perseverative, stereotyped, or compulsive behavior; and hyperorality or dietary change, along with prominent decline in social cognition or executive abilities. Learning, memory, and perceptual-motor function are relatively spared. Onset is often in midlife, so the presentation is easily mistaken for a psychiatric or "midlife" problem.

References: DSM-5-TR — Major or Mild Frontotemporal Neurocognitive Disorder, Criterion C1 (behavioral variant)

☐ Approve ☐ Revise ☐ Retire Notes:

A 79-year-old with a known neurocognitive disorder now relies on her son to pay her bills and set out her daily medications because she makes repeated errors on her own. Under DSM-5-TR, what does this change MOST directly indicate?

A. Deficits now interfere with independence, so major NCD fits ✓ KEY

Needing help with complex instrumental activities such as bills and medications marks the functional threshold for major NCD.

B. A delirium is now present on top of her existing disorder

Delirium is an acute, fluctuating disturbance in attention and awareness, which is not described.

C. Her cognitive test scores must now fall in the severe range

The mild/major distinction rests mainly on functional independence, not on a single test cutoff.

D. The cause of her disorder must have shifted to vascular

A change in functional level says nothing about a change in etiological subtype.

Rationale: DSM-5-TR separates mild from major neurocognitive disorder chiefly by function. In mild NCD the deficits do not interfere with independence in everyday activities, although greater effort or compensatory strategies may be needed. In major NCD they do, at minimum requiring assistance with complex instrumental activities such as paying bills or managing medications.

References: DSM-5-TR — Major Neurocognitive Disorder, Criterion B; Mild Neurocognitive Disorder, Criterion B

☐ Approve ☐ Revise ☐ Retire Notes:

A daughter caring for her father with moderate Alzheimer's disease says he asks several times each evening where his late wife is. Each time she reminds him that her mother died, he sobs as if hearing it for the first time. Which guidance should the counselor offer?

A. Keep reminding him of the death so that the fact is gradually relearned

Repeated reorientation to a painful fact he cannot retain causes fresh grief each time without lasting learning.

B. Ask his physician to raise his sedative dose for the evening hours

Medication is not the first response to distress that can be eased by changing how caregivers respond.

C. Avoid the subject entirely and leave the room whenever he raises it

Withdrawing leaves him alone with distress and does not meet the emotional need behind the question.

D. Respond to the feeling behind the question, then gently redirect him ✓ KEY

Acknowledging the emotion (for example, that he misses her) and redirecting reduces distress without repeated re-traumatizing news.

Rationale: In moderate dementia, a person often cannot retain new information, so repeated correction brings repeated fresh grief. Caregivers are usually taught to respond to the emotion behind a question, offer comfort, and redirect to a soothing activity. Counselors also support caregivers with psychoeducation, skills training, and attention to their own strain, as dementia guidelines recommend.

References: NICE guidelines — NG97 Dementia: supporting carers (psychoeducation and skills training) · Berk (Development Through the Lifespan) — Late adulthood: Alzheimer's disease and caregiving

► **Check:** Validation-and-redirect is standard caregiver teaching rather than a quoted NICE recommendation; SME to confirm locators.

☐ Approve ☐ Revise ☐ Retire Notes:

A 63-year-old with hoarding disorder faces eviction because her apartment is blocked by stacked items. Her landlord has offered to send a crew to clear it out over one weekend. Which approach is MOST consistent with effective treatment of hoarding?

A. Support the weekend cleanout, since clearing the space ends the disorder

Forced cleanouts may solve an immediate crisis but are distressing and often followed by rapid reaccumulation.

B. CBT building sorting and decision skills with practice in discarding ✓ **KEY**

CBT for hoarding targets motivation, organizing and decision-making skills, graded discarding, and reduced acquiring.

C. Insight-oriented talk about her childhood losses before any sorting

Exploring history alone does not change saving and acquiring behavior and delays needed work.

D. Have the counselor decide which items to discard to speed progress

Decisions made for the client undercut her own decision-making practice, which is central to treatment.

Rationale: Hoarding disorder involves persistent difficulty discarding possessions because of a perceived need to save them and distress at discarding, leading to clutter that compromises living areas. Cognitive-behavioral treatment for hoarding uses motivational work, skills training in organizing and decision-making, graded exposure to sorting and discarding, and practice resisting acquiring. Externally imposed cleanouts tend to be traumatic and rarely produce lasting change.

References: DSM-5-TR — Hoarding Disorder: diagnostic features and functional consequences · Corey (Theory & Practice) — Cognitive behavior therapy: skills training and exposure

► **Check:** Hoarding-specific CBT (Steketee & Frost) is not in the approved source library; treatment content cited to general sources. SME to confirm.

☐ Approve ☐ Revise ☐ Retire Notes:

Q422 of 1000 · nce-s-clf-182 · Assessment and Testing · obsessive-compulsive and related disorders · hard · In review

A 27-year-old is completely certain that her nose is grotesquely misshapen, although others see nothing unusual. She checks mirrors for hours and has had two rhinoplasties, which left her more distressed. She is now seeking a third surgeon. Which conclusion is MOST accurate?

A. BDD with absent insight; more surgery is unlikely to ease her distress ✓ **KEY**

Total conviction is coded as BDD with absent insight/delusional beliefs, and cosmetic procedures rarely relieve BDD.

B. Delusional disorder, somatic type, as her conviction is fixed and total

DSM-5-TR codes delusional appearance beliefs as BDD with absent insight rather than as delusional disorder.

C. BDD with good insight; a skilled surgeon will likely resolve her concern

Complete conviction is not good insight, and surgery typically fails to resolve or worsens BDD symptoms.

D. No disorder applies, because she has sought medical rather than mental care

Many people with BDD seek cosmetic rather than mental health treatment; the pattern fully meets BDD criteria.

Rationale: Body dysmorphic disorder involves preoccupation with perceived flaws not observable or slight to others, with repetitive behaviors such as mirror checking. Insight is specified as good or fair, poor, or absent insight/delusional beliefs; a person completely convinced of the flaw still receives BDD, not a psychotic diagnosis. Many people with BDD seek cosmetic procedures, which seldom improve and can worsen symptoms, so referral for CBT and medication evaluation is indicated.

References: DSM-5-TR — Body Dysmorphic Disorder: insight specifiers; associated features (cosmetic treatment)

☐ Approve ☐ Revise ☐ Retire Notes:

A 14-year-old with OCD spends hours arranging objects and touching doorframes until things feel "just right." His parents mention that between ages 7 and 10 he had repeated eye blinking and grunting that have since stopped. Which DSM-5-TR specifier should be added to his OCD diagnosis?

A. With poor insight

Insight specifiers depend on his beliefs about the obsessions, which are not described.

B. With panic attacks

No abrupt surges of intense fear are described.

C. With absent insight

Nothing suggests he is fully convinced his obsessional beliefs are true.

D. Tic-related ✓ KEY

The tic-related specifier applies to a current or past history of a tic disorder.

Rationale: OCD is specified as tic-related when the person has a current or past history of a tic disorder. Tic-related OCD is more common in males with childhood onset and often involves symmetry, ordering, and "just right" concerns rather than harm or contamination themes. Past tics count, so a careful developmental history matters.

References: DSM-5-TR — Obsessive-Compulsive Disorder: specify if tic-related

☐ Approve ☐ Revise ☐ Retire Notes:

A 9-year-old with average intellectual ability reads slowly and makes many decoding errors. A school team is considering specific learning disorder with impairment in reading. Under DSM-5-TR, which requirement must be met?

A. A large gap between full-scale IQ and reading achievement scores

DSM-5 dropped the IQ-achievement discrepancy requirement; skills must instead be well below age expectations.

B. Difficulty persisting 6 months despite help aimed at that skill ✓ KEY

Criterion A requires difficulties persisting at least 6 months despite interventions that target them.

C. A formal diagnosis made only after the child reaches age 12

Specific learning disorder is typically identified in the early school years; there is no age 12 threshold.

D. Proof that the difficulty also appears in mathematics and writing

Impairment may involve one domain only; reading, written expression, and mathematics are specified separately.

Rationale: DSM-5-TR requires difficulties learning and using academic skills that have persisted for at least 6 months despite targeted intervention, with skills substantially and quantifiably below those expected for age (confirmed by standardized achievement measures and comprehensive clinical assessment). Onset is in the school-age years, and the difficulties are not better explained by intellectual disability, sensory problems, or inadequate instruction. The older IQ-achievement discrepancy model is no longer required.

References: DSM-5-TR — Specific Learning Disorder, Criteria A and B

☐ Approve ☐ Revise ☐ Retire Notes:

In DSM-5-TR, deficits in adaptive functioning for intellectual developmental disorder are assessed in which three domains?

A. Verbal, perceptual, and processing speed

These resemble index scores from intelligence tests, not adaptive functioning domains.

B. Sensory, motor, and expressive language

These are developmental skill areas, not the three DSM-5-TR adaptive domains.

C. Conceptual, social, and practical ✓ KEY

DSM-5-TR assesses adaptive functioning in the conceptual, social, and practical domains.

D. Cognitive, emotional, and behavioral

This is a general psychological triad, not the DSM-5-TR adaptive framework.

Rationale: Intellectual developmental disorder requires deficits in intellectual functions and in adaptive functioning, with onset during the developmental period. Adaptive functioning covers the conceptual (academic), social, and practical (self-management) domains. Severity is set by adaptive functioning in these domains rather than by IQ alone, because adaptive functioning determines the level of support needed.

References: DSM-5-TR — Intellectual Developmental Disorder, Criterion B and severity table

☐ Approve ☐ Revise ☐ Retire Notes:

For the past year, a 10-year-old girl has shown seven inattentive symptoms of ADHD, such as losing materials and drifting off during lessons, but only two hyperactive-impulsive symptoms. Symptoms began before age 12 and appear at home and school. Which presentation should be specified?

A. Predominantly inattentive presentation ✓ KEY

Inattention criteria are met while hyperactivity-impulsivity criteria are not, which defines this presentation.

B. Combined presentation

Combined presentation requires both symptom criteria to be met for the past 6 months.

C. Predominantly hyperactive presentation

She has only two hyperactive-impulsive symptoms, below the threshold.

D. Other specified ADHD

Full criteria are met, so a specified presentation of ADHD applies.

Rationale: ADHD presentations reflect the past 6 months: combined (both inattention and hyperactivity-impulsivity criteria met), predominantly inattentive (inattention met, hyperactivity-impulsivity not), or predominantly hyperactive/impulsive (the reverse). Children need six or more symptoms in a domain; those 17 and older need five. Girls are more often seen with the inattentive presentation, which can delay recognition.

References: DSM-5-TR — Attention-Deficit/Hyperactivity Disorder: specify current presentation

☐ Approve ☐ Revise ☐ Retire Notes:

Which of the following is a negative symptom of schizophrenia?

A. Persecutory delusions

Delusions are positive symptoms, an addition to normal experience.

B. Disorganized speech

Disorganized speech is a separate symptom domain, not a negative symptom.

C. Auditory hallucinations

Hallucinations are positive symptoms.

D. Avolition ✓ KEY

Avolition, a decrease in self-initiated purposeful activity, is a core negative symptom.

Rationale: Negative symptoms reflect a loss or reduction of normal functions. DSM-5-TR highlights diminished emotional expression and avolition as especially prominent in schizophrenia; others include alogia, anhedonia, and asociality. Negative symptoms account for much of the long-term disability in schizophrenia and respond less well to antipsychotic medication than positive symptoms.

References: DSM-5-TR — Schizophrenia Spectrum and Other Psychotic Disorders: key features (negative symptoms)

☐ Approve ☐ Revise ☐ Retire Notes:

A 45-year-old in treatment for severe depression arrives with his wife, who says he has barely spoken or eaten for 3 days. In session he stays mute, holds a stiff, odd posture for long periods, and resists when the counselor gently offers him a cup of water. What should the counselor do FIRST?

A. Begin behavioral activation to restart his routine and appetite

Behavioral activation cannot address a possible medical emergency that needs urgent evaluation first.

B. Use silence and patience until he is ready to talk in session

Waiting delays care for catatonia, which can lead to dehydration and other serious complications.

C. Arrange urgent medical evaluation for possible catatonia today ✓ KEY

Mutism, posturing, and negativism suggest catatonia, which needs urgent medical care and is often highly treatable.

D. Ask his wife to keep a food log and report back next week

A week of monitoring is far too slow when he is not eating or drinking and may be catatonic.

Rationale: Catatonia is diagnosed when three or more of twelve psychomotor features are present, such as stupor, mutism, posturing, catalepsy, and negativism. It can occur with depressive, bipolar, psychotic, or medical conditions and may become life-threatening through dehydration, malnutrition, or autonomic instability. It typically responds to medical treatment, so the counselor's first job is to get the client to urgent medical and psychiatric evaluation.

References: DSM-5-TR — Catatonia Associated With Another Mental Disorder (Catatonia Specifier)

► **Check:** Placed under schizophrenia spectrum because catatonia sits in that DSM-5-TR chapter; the client here has depression.

☐ Approve ☐ Revise ☐ Retire Notes:

A 24-year-old has just been discharged after a first episode of schizophrenia and is moving back home. His parents are frightened and often criticize him for "lying around," and his mother has given up her job to watch him constantly. Which intervention is MOST likely to reduce his risk of relapse?

A. Family psychoeducation that lowers criticism and overinvolvement ✓ KEY

High expressed emotion predicts relapse, and family interventions that reduce it lower relapse rates.

B. Advising the family to limit contact with him until he stabilizes

Reducing contact removes support and does not teach the family more helpful ways to respond.

C. Individual insight therapy exploring his early family relationships

Insight-oriented therapy is not an evidence-based relapse-prevention approach for schizophrenia.

D. Encouraging his parents to confront his low motivation more firmly

Firmer confrontation increases criticism, a component of high expressed emotion linked to relapse.

Rationale: Expressed emotion describes family attitudes of criticism, hostility, and emotional overinvolvement toward a member with a mental illness. High expressed emotion reliably predicts relapse in schizophrenia. Family psychoeducation and family intervention teach relatives about the illness, problem-solving, and calmer communication, and they reduce relapse and rehospitalization.

References: NICE guidelines — CG178 Psychosis and schizophrenia in adults: family intervention · Goldenberg (Family Therapy) — Psychoeducational family therapy and expressed emotion

☐ Approve ☐ Revise ☐ Retire Notes:

A 34-year-old arrives in a flamboyant outfit and calls the counselor "my new best friend" at their first meeting. She is upset when attention shifts away from her, speaks dramatically but vaguely, and her emotions change quickly. This pattern has lasted since her late teens. Which personality disorder BEST fits?

A. Narcissistic personality disorder

Narcissistic personality centers on grandiosity, entitlement, and need for admiration rather than shallow, shifting emotion.

B. Histrionic personality disorder ✓ KEY

Attention-seeking, rapidly shifting shallow emotion, impressionistic speech, and overestimating intimacy fit histrionic personality.

C. Borderline personality disorder

Borderline personality centers on abandonment fears, unstable identity, and self-harm, which are not described.

D. Dependent personality disorder

Dependent personality centers on needing others to make decisions and fear of being alone.

Rationale: Histrionic personality disorder is a pervasive pattern of excessive emotionality and attention-seeking beginning by early adulthood. Features include discomfort when not the center of attention, rapidly shifting and shallow emotions, use of appearance to draw attention, impressionistic speech lacking detail, theatrical expression, suggestibility, and viewing relationships as more intimate than they are. At least five of eight criteria are required.

References: DSM-5-TR — Histrionic Personality Disorder, diagnostic criteria

☐ Approve ☐ Revise ☐ Retire Notes:

Since adolescence, a 29-year-old has avoided almost all new relationships and job promotions, believing she is inferior and certain to be rejected. She also has marked fear of being judged when eating or speaking in front of others. How does DSM-5-TR describe avoidant personality disorder in relation to social anxiety disorder?

A. Social anxiety disorder must be ruled out before avoidant PD is given

DSM-5-TR does not make social anxiety disorder an exclusion for avoidant personality disorder.

B. Avoidant PD is diagnosed only when fear is limited to one situation

Avoidant personality disorder is pervasive across contexts, not limited to a single situation.

C. The two are unrelated, as one is a mood state and the other a trait

DSM-5-TR describes heavy overlap between them; neither is a mood disorder.

D. They overlap heavily and both may be diagnosed if each is fully met ✓ KEY

DSM-5-TR notes great overlap, possibly alternative conceptualizations, and both can be diagnosed when criteria are met.

Rationale: Avoidant personality disorder is a pervasive pattern of social inhibition, feelings of inadequacy, and hypersensitivity to negative evaluation beginning by early adulthood. DSM-5-TR notes so much overlap with social anxiety disorder that they may be alternative conceptualizations of the same or similar conditions. When criteria for both are met, both may be diagnosed; avoidant PD tends to reflect a broader, identity-level sense of inadequacy across most interpersonal settings.

References: DSM-5-TR — Avoidant Personality Disorder: differential diagnosis (social anxiety disorder)

☐ Approve ☐ Revise ☐ Retire Notes:

Early in counseling, a client with borderline personality disorder calls the counselor several times a week between sessions, often late at night, and feels rejected when calls are not returned at once. The counselor is growing frustrated. Which response BEST reflects an effective treatment frame?

A. Return every call at once so she does not feel abandoned in a crisis

Unlimited availability is unsustainable, can reinforce crisis behavior, and risks counselor burnout.

B. Tell her calls between sessions are not allowed under any condition

A blanket ban leaves her without coaching support and may feel like the rejection she fears.

C. Set clear, agreed limits on contact, with brief skills-coaching calls ✓ KEY

A collaborative, consistent frame with planned coaching contact supports skill use while protecting the work.

D. Refer her elsewhere, since frequent calls show she is not yet ready

Frequent contact is a treatment target, not a reason to abandon the client.

Rationale: Evidence-based treatments for borderline personality disorder share a clear, consistent treatment frame agreed with the client. In dialectical behavior therapy, between-session phone coaching is brief and focused on applying skills, with limits the therapist observes and explains openly. A predictable frame reduces fears of abandonment, discourages reinforcement of crisis behavior, and is supported by consultation for the counselor.

References: Linehan (DBT Skills Training Manual) — DBT treatment modes: between-session phone coaching and observing limits

☐ Approve ☐ Revise ☐ Retire Notes:

An adult client meets criteria for binge-eating disorder and asks which psychological treatment has the strongest evidence. Which approach should the counselor describe as first-line?

A. Eating-disorder-focused cognitive behavioral therapy ✓ KEY

CBT focused on eating, including guided self-help, is the first-line psychological treatment for binge-eating disorder.

B. A strict diet plan aimed mainly at quick weight loss

Dieting is not a treatment for binge eating and rigid restriction can intensify the binge cycle.

C. Long-term psychodynamic therapy as the sole treatment

Psychodynamic therapy lacks the evidence base that CBT has for binge-eating disorder.

D. Hypnosis to suppress cravings for specific trigger foods

Hypnosis is not an evidence-based first-line treatment for binge-eating disorder.

Rationale: Clinical guidelines recommend eating-disorder-focused CBT for binge-eating disorder, often beginning with a guided self-help program and moving to group or individual CBT if needed. Treatment targets regular eating, the binge cycle, triggers, and the over-evaluation of shape and weight. Weight loss is not the primary aim, and restrictive dieting can maintain binge eating.

References: NICE guidelines — NG69 Eating disorders: treating binge eating disorder (guided self-help; group CBT-ED)

☐ Approve ☐ Revise ☐ Retire Notes:

A 17-year-old has lost a significant amount of weight over a year through restrictive eating. She intensely fears weight gain and judges herself almost entirely by her shape. Her physician notes that, despite the loss, her weight remains within the normal range for her age and height. Which diagnosis BEST fits?

A. Anorexia nervosa, restricting type, with mild severity

Anorexia nervosa requires significantly low body weight, which is not present here.

B. Other specified feeding or eating disorder (atypical AN) ✓ KEY

Atypical anorexia nervosa is listed under OSFED when all AN criteria are met except low weight.

C. Avoidant/restrictive food intake disorder, adolescent onset

ARFID excludes restriction driven by concerns about body shape or weight.

D. Bulimia nervosa, low frequency and limited duration

No binge eating or compensatory behaviors are described.

Rationale: DSM-5-TR lists atypical anorexia nervosa as an example of other specified feeding or eating disorder: all criteria for anorexia nervosa are met except that, despite significant weight loss, weight is within or above the normal range. Medical risks can be serious despite a normal-appearing weight, so these clients need the same medical monitoring and specialized care as those with anorexia nervosa.

References: DSM-5-TR — Other Specified Feeding or Eating Disorder: atypical anorexia nervosa

☐ Approve ☐ Revise ☐ Retire Notes:

Parents of a 14-year-old recently diagnosed with anorexia nervosa ask what treatment the eating disorder team will recommend. She is medically stable for outpatient care. Which description BEST fits family-based treatment (FBT) for adolescent anorexia?

A. The teen meets alone with a therapist to explore the roots of her illness

FBT is family based and does not focus on uncovering causes; individual insight work is a different model.

B. Parents step back from meals so that the teen regains a sense of control

FBT does the opposite in its first phase, placing parents in charge of restoring healthy eating.

C. The family first explores how parenting errors caused the eating disorder

FBT takes an agnostic stance on cause and explicitly avoids blaming parents.

D. Parents take charge of renourishment at first, then gradually hand it back ✓ **KEY**

FBT empowers parents to lead renourishment first, then returns control to the teen as she recovers.

Rationale: Family-based treatment (the Maudsley approach) is a leading evidence-based treatment for adolescent anorexia nervosa and is reflected in guideline recommendations for eating-disorder-focused family therapy. It proceeds in phases: parents take charge of restoring healthy eating, control over eating is gradually returned to the adolescent, and then broader adolescent development is addressed. The model does not blame parents and separates the illness from the young person.

References: NICE guidelines — NG69 Eating disorders: anorexia nervosa in children and young people (eating-disorder-focused family therapy)

☐ Approve ☐ Revise ☐ Retire Notes:

A 26-year-old in a major depressive episode brightens noticeably when friends invite her out, then sinks again. She sleeps far more than usual, eats much more, describes her arms and legs as feeling "like lead," and has long been crushed by small hints of criticism. Which specifier BEST fits?

A. With melancholic features

Melancholic features involve loss of reactivity to pleasant events, early waking, and decreased appetite, the reverse of this picture.

B. With anxious distress

Anxious distress involves tension, restlessness, worry, and fear, which are not described.

C. With atypical features ✓ **KEY**

Mood reactivity plus hypersomnia, increased appetite, leaden paralysis, and rejection sensitivity define atypical features.

D. With mixed features

Mixed features require manic or hypomanic symptoms during the episode, which are absent.

Rationale: The "with atypical features" specifier requires mood reactivity (mood brightens in response to positive events) plus at least two of: significant weight gain or increased appetite, hypersomnia, leaden paralysis, and a long-standing pattern of sensitivity to interpersonal rejection. Criteria for melancholic features or catatonia must not be met in the same episode. Despite its name, this pattern is common.

References: DSM-5-TR — Specifiers for Depressive Disorders: with atypical features

☐ Approve ☐ Revise ☐ Retire Notes:

A 32-year-old nurse with bipolar I disorder, stable on medication, is about to switch to rotating night shifts. Her past manic episodes followed weeks of disrupted sleep. Which counseling focus is MOST consistent with interpersonal and social rhythm therapy (IPSRT)?

A. Keep daily routines and sleep-wake times as steady as she can ✓ KEY

IPSRT stabilizes daily rhythms, especially sleep, because disruptions can trigger mood episodes.

B. Explore early memories to find the roots of her mood swings

Uncovering childhood roots is not the IPSRT focus on present rhythms and relationships.

C. Dispute the irrational beliefs that drive her elevated moods

Disputing irrational beliefs comes from REBT, not IPSRT.

D. Build a fear hierarchy for in-vivo exposure to her workplace

Exposure hierarchies treat anxiety disorders; they do not address rhythm disruption.

Rationale: Interpersonal and social rhythm therapy, developed by Ellen Frank, holds that disruptions to daily routines and sleep-wake cycles can trigger mood episodes in people with bipolar disorder. Clients track daily rhythms (for example, with a social rhythm metric), work to keep them regular, and address interpersonal problems and role transitions. A shift to night work is a classic rhythm disruption to plan for.

References: IPSRT — Social rhythm stabilization and the social rhythm metric

☐ Approve ☐ Revise ☐ Retire Notes:

A 22-year-old nursing student has fainted twice while watching blood draws and now avoids clinical rotations. She has specific phobia, blood-injection-injury type. When planning exposure, which added technique is MOST suited to this phobia type?

A. Progressive muscle relaxation before and during each exposure

Relaxation can deepen the drop in blood pressure behind fainting, so it is generally avoided for this type.

B. Applied tension, tensing large muscles to keep blood pressure up ✓ KEY

Applied tension counters the vasovagal blood pressure drop that causes fainting in this phobia type.

C. Interoceptive exposure to dizziness by spinning in a chair first

Interoceptive exposure targets fear of bodily sensations in panic disorder, not vasovagal fainting.

D. Slow diaphragmatic breathing to lower her heart rate and arousal

Lowering arousal does not address, and may worsen, the vasovagal response.

Rationale: Unlike other specific phobias, the blood-injection-injury type often involves a vasovagal response: a brief rise in heart rate and blood pressure followed by a sharp drop that can cause fainting. Applied tension teaches the client to tense large muscle groups to raise blood pressure while being exposed to feared stimuli. Relaxation-based methods are generally avoided because they can make fainting more likely.

References: DSM-5-TR — Specific Phobia: blood-injection-injury type (vasovagal fainting response)

► **Check:** Applied tension (Öst) is not described in DSM-5-TR; no approved source names it directly. SME to confirm or add a citation.

☐ Approve ☐ Revise ☐ Retire Notes:

A 38-year-old meets full criteria for PTSD after a violent home invasion. She also reports that she often feels as though she is watching herself from outside her body and that the world seems dreamlike and unreal. These experiences are not caused by substances or a medical condition. How should this be diagnosed?

A. PTSD plus a separate diagnosis of depersonalization/derealization disorder

When these symptoms occur as part of PTSD, they are captured with the specifier, not a separate disorder.

B. Acute stress disorder, as dissociative symptoms are central to that diagnosis

Full PTSD criteria are already met, and acute stress disorder is limited to the first month after trauma.

C. PTSD with delayed expression, given the presence of dissociative symptoms

Delayed expression refers to full criteria not being met until at least 6 months after the event.

D. PTSD with dissociative symptoms, noting depersonalization and derealization ✓ KEY

The dissociative specifier applies to persistent or recurrent depersonalization or derealization in PTSD.

Rationale: DSM-5-TR allows PTSD to be specified "with dissociative symptoms" when the person also has persistent or recurrent depersonalization (feeling detached from oneself, as an outside observer) or derealization (the world feels unreal, dreamlike, or distant), not attributable to substances or a medical condition. Recognizing this subtype matters because dissociation can affect pacing and stabilization in trauma-focused treatment.

References: DSM-5-TR — Posttraumatic Stress Disorder: specify with dissociative symptoms

☐ Approve ☐ Revise ☐ Retire Notes:

A 36-year-old with opioid use disorder has taken prescribed buprenorphine for 8 months and has not used illicit opioids. In a recovery group, peers tell him he is "not really clean" and urge him to stop the medication. He asks the counselor what to think. Which response is MOST consistent with best practice?

A. Agree that full recovery requires tapering off all opioid medicines soon

No fixed endpoint is required; stopping medication early raises the risk of relapse and overdose.

B. Suggest he follow the group's view, since peer support predicts recovery

Peer support is valuable, but the group's advice here conflicts with the evidence on medication for OUD.

C. Affirm that MOUD is effective treatment, and support talking to his prescriber ✓ KEY

Medication for OUD is evidence-based treatment, and medication decisions belong with the client and prescriber.

D. Advise him to keep his medication secret from others in the recovery group

Encouraging secrecy does not address stigma or help him find supportive recovery settings.

Rationale: Medications for opioid use disorder, such as buprenorphine, methadone, and extended-release naltrexone, reduce illicit use, overdose deaths, and other harms. Taking prescribed medication as directed is treatment, not "substituting one addiction for another." Counselors should challenge stigma, support informed choices, coordinate with prescribers, and help clients find MOUD-friendly recovery supports; duration of medication is individualized, and discontinuing it raises relapse and overdose risk.

References: SAMHSA TIP 63 — Part 1: MOUD effectiveness; Part 4: counseling and peer support for clients on MOUD

► **Check:** SME to confirm the TIP 63 part numbers in the locator.

☐ Approve ☐ Revise ☐ Retire Notes:

Two years after her husband died, a 70-year-old has returned to her volunteer work and made new friends. She still talks to him at his grave on their anniversary and keeps his watch on her nightstand. Her adult children ask the counselor to help her "finally let go." Which response BEST reflects current grief theory?

A. Agree that she must give up the attachment before she can heal

Older "letting go" models have given way to the view that healthy bonds with the deceased can continue.

B. Explain that a lasting bond with him can coexist with a full life ✓ KEY

Continuing bonds, and Worden's fourth task of finding an enduring connection, describe adaptive grief.

C. Assess her for prolonged grief disorder due to the grave visits

She is functioning well; occasional rituals of connection are not signs of prolonged grief disorder.

D. Suggest she put his watch away to signal the grief has ended

Keeping meaningful objects can be part of a healthy continuing bond; removing them is not required.

Rationale: Contemporary grief theory views ongoing connection with the deceased as compatible with healthy adaptation. Worden's fourth task of mourning is to find an enduring connection with the deceased while embarking on a new life, and the continuing bonds perspective holds that memories, rituals, and inner conversations can support coping. Concern arises when connection prevents re-engagement in life, which is not the case here.

References: Worden (Grief Counseling) — Task IV: find an enduring connection with the deceased while embarking on a new life

☐ Approve ☐ Revise ☐ Retire Notes:

Using James and Gilliland's six-step crisis model, a counselor has defined the problem with a client whose house burned down, ensured his safety, and offered support. The client is now calmer. Which step comes NEXT?

A. Obtaining his commitment to a plan

Commitment is the sixth and final step, after alternatives are explored and a plan is made.

B. Making a concrete plan with him

Planning is the fifth step and follows examining alternatives.

C. Redefining the core problem again

Defining the problem is the first step, which has already been completed.

D. Examining alternatives with him ✓ KEY

After the three listening steps, the fourth step is examining alternatives.

Rationale: James and Gilliland's six-step model has three listening steps (defining the problem, ensuring client safety, providing support) followed by three acting steps (examining alternatives, making plans, obtaining commitment). Assessment runs throughout all six steps, and safety is monitored continuously, but the sequence guides the counselor from understanding to action.

References: James & Gilliland (Crisis Intervention Strategies) — Six-step model of crisis intervention

☐ Approve ☐ Revise ☐ Retire Notes:

During a risk assessment, a client experiencing partner abuse discloses several things about her partner. Which disclosure MOST strongly signals elevated risk of lethal violence and calls for urgent safety planning?

A. He has choked her until she could barely breathe in the past ✓ KEY

Prior non-fatal strangulation is one of the strongest known predictors of later intimate partner homicide.

B. He often yells and calls her names when he gets home late

Verbal abuse is harmful but is a weaker predictor of lethality than strangulation.

C. He grew up in a home where his father hit his mother often

Childhood exposure to violence is a risk factor for perpetration but a weaker lethality marker.

D. He sulks and won't speak to her for days after arguments

Withdrawal after conflict does not signal elevated lethality.

Rationale: Lethality assessment in intimate partner violence focuses on factors linked to homicide, such as prior strangulation, access to firearms, threats to kill, escalating frequency or severity, extreme jealousy, and a recent or planned separation. Non-fatal strangulation markedly raises the risk of later homicide and may leave few visible injuries, so counselors should ask about it directly and respond with urgent safety planning and advocacy referrals.

References: James & Gilliland (Crisis Intervention Strategies) — Intimate partner violence: assessing lethality

► **Check:** The strangulation-lethality link comes from Campbell's Danger Assessment research, which is not in the approved library; SME to confirm the James & Gilliland locator.

☐ Approve ☐ Revise ☐ Retire Notes:

A 3-year-old has delays in speech, motor skills, and self-care, but cannot yet complete standardized intellectual testing in a valid way. The evaluating team plans to reassess in a year. Which DSM-5-TR diagnosis BEST fits for now?

A. Intellectual disability, mild

Severity cannot yet be reliably assessed, so a specified level of intellectual disability is premature.

B. Global developmental delay ✓ KEY

This diagnosis is reserved for children under 5 who miss milestones in several areas and cannot yet be reliably assessed.

C. Autism spectrum disorder, level 1

No social communication deficits or restricted, repetitive behaviors are described.

D. Language disorder, expressive type

Delays extend well beyond language to motor and self-care areas.

Rationale: Global developmental delay is diagnosed in children under age 5 when they fail to meet expected milestones in several areas of intellectual functioning and the clinical severity cannot be reliably assessed, often because the child is too young for standardized testing. It requires reassessment after a period of time.

References: DSM-5-TR — Global Developmental Delay

☐ Approve ☐ Revise ☐ Retire Notes:

A 44-year-old previously met full criteria for alcohol use disorder. For the past 5 months he has met no criteria except strong cravings, which come mainly when he passes his old bar. He lives at home, not in a controlled setting. Which specifier applies?

A. In sustained remission, since he has gone months without drinking

Sustained remission requires 12 months or longer without meeting criteria other than craving.

B. No remission specifier, because his ongoing craving is a criterion

Craving is the one criterion explicitly allowed during remission, so it does not block the specifier.

C. In early remission, as only craving has been present for 5 months ✓ **KEY**

Early remission applies after 3 months but less than 12 months with no criteria met except craving.

D. In early remission, in a controlled environment, given the time

He lives at home; the controlled-environment specifier is for settings where access is restricted.

Rationale: For substance use disorders, "in early remission" applies when, after full criteria were met, none have been met for at least 3 months but less than 12 months, with the exception of craving. "In sustained remission" requires 12 months or longer. "In a controlled environment" is added only when the person is in a setting where access to substances is restricted.

References: DSM-5-TR — Alcohol Use Disorder: remission specifiers

☐ Approve ☐ Revise ☐ Retire Notes:

A 30-year-old violinist is comfortable at parties, in meetings, and on dates, but for a year has felt intense dread before solo recitals, fearing the audience will judge her shaking hands. She has started declining solo work. If criteria for social anxiety disorder are met, which specifier applies?

A. With panic attacks

No abrupt surges of intense fear are described, and this specifier does not capture the restricted context.

B. Generalized type

DSM-5 removed the generalized specifier for social anxiety disorder.

C. With poor insight

Social anxiety disorder has no insight specifier in DSM-5-TR.

D. Performance only ✓ **KEY**

This specifier applies when the fear is restricted to speaking or performing in public.

Rationale: Social anxiety disorder may be specified "performance only" when the fear is restricted to speaking or performing in public. People with this pattern are often impaired mainly in professional or academic roles that require performing, while ordinary social interactions are comfortable. DSM-5 replaced the older "generalized" specifier with this narrower one.

References: DSM-5-TR — Social Anxiety Disorder: specify if performance only

☐ Approve ☐ Revise ☐ Retire Notes:

A 4-year-old was in a serious car crash 3 months ago. Since then she repeatedly crashes toy cars together while making siren noises, cries when buckled into her car seat, and clings to her parents at bedtime. She shows no visible distress during the crash play. How should the counselor understand the repetitive play?

A. Play reenactment can express intrusion symptoms in young children ✓ KEY

For children 6 and younger, intrusive memories may appear as play reenactment and need not look distressing.

B. The play is ordinary for her age and unrelated to the crash itself

Repetitive play that mirrors the trauma, alongside avoidance and clinging, points to a trauma response.

C. Because she seems calm during the play, PTSD can be ruled out here

DSM-5-TR notes that intrusive memories in young children may not appear distressing.

D. The play shows the trauma is resolved and needs no further attention

Ongoing reenactment with avoidance and distress at reminders suggests the trauma is not resolved.

Rationale: DSM-5-TR includes separate PTSD criteria for children 6 years and younger. Intrusion symptoms may take the form of play reenactment, and spontaneous intrusive memories may not appear distressing. The avoidance and negative-cognition clusters are combined, so only one symptom from that combined group is needed. Counselors should assess such play in context and involve caregivers in trauma-focused treatment.

References: DSM-5-TR — Posttraumatic Stress Disorder for Children 6 Years and Younger, Criterion B

☐ Approve ☐ Revise ☐ Retire Notes:

A 73-year-old retired teacher with no history of anxiety reports four months of constant worry, a racing heart, restlessness and poor sleep. She recently began two new medications and says, "I just feel keyed up all the time." What should the counselor do FIRST?

A. Begin weekly CBT for generalized anxiety and give a worry log

CBT may help later, but new-onset anxiety in late life needs a medical cause ruled out before a psychological plan is set.

B. Teach slow breathing and suggest she cut back on her caffeine

Coping tips are reasonable but skip the key step of checking for medical and medication causes of her symptoms.

C. Encourage a medical review of her health and new medications ✓ KEY

Late-onset anxiety often has medical or medication contributors (e.g., thyroid, cardiac, drug effects), so medical review comes first.

D. Reassure her that worry is a normal response to growing older

Persistent anxiety is not a normal part of aging; dismissing it reflects ageist assumptions and delays care.

Rationale: Anxiety that first appears in later life is more often linked to medical illness, medication side effects or interactions, or early cognitive change than anxiety that began earlier. Older adults also tend to describe anxiety in somatic terms. Counselors coordinate a medical evaluation first, then provide evidence-based psychological treatment such as CBT, which is effective for older adults.

References: Berk (Development Through the Lifespan) — Late adulthood: physical health, medication use and mental health

☐ Approve ☐ Revise ☐ Retire Notes:

A home-health aide refers an 83-year-old man who lives alone. His home has spoiled food, piled-up mail and no working heat, and he has missed his heart medication for weeks. No one else is involved in his care. He tells the counselor, "I'm fine. Leave me be." Which response BEST fits the counselor's obligations?

A. Assess his safety and capacity and follow state rules on reporting to APS ✓ KEY

Self-neglect that threatens health is reportable to Adult Protective Services in most states, and capacity shapes the response; both must be assessed.

B. Honor his stated wishes and close the case, since he is a competent adult

His capacity has not been assessed, and serious self-neglect may trigger reporting duties regardless of his preference.

C. Arrange for him to move to a nursing home before the cold weather comes

Counselors do not unilaterally relocate adults; least-restrictive options and his rights must be weighed through proper channels.

D. Ask the aide to keep watching and decide again at the next monthly visit

No heat and missed cardiac medication are current dangers; waiting a month risks serious harm.

Rationale: Elder self-neglect, an older adult's failure to meet his own basic needs in ways that threaten health or safety, is the most common type of case reported to Adult Protective Services, and most states treat it as reportable. Counselors balance respect for autonomy against safety by assessing decision-making capacity and immediate risk, following state reporting law, and favoring the least-restrictive supports.

References: State law — Adult protective services statutes: self-neglect and vulnerable-adult reporting · ACA Code of Ethics — B.2.a Serious and Foreseeable Harm and Legal Requirements

► **Check:** Whether self-neglect is reportable, and who is a mandated reporter of vulnerable-adult maltreatment, varies by state; SME to confirm the "most states" framing.

☐ Approve ☐ Revise ☐ Retire Notes:

During a community talk on aging, an attendee says, "Getting old means your mind goes and you end up lonely and miserable." Which statement BEST reflects life-span research on typical aging?

A. Memory and reasoning decline steeply for most people after age sixty

Some abilities slow, but steep, global decline is not typical of normal aging; it suggests a neurocognitive disorder.

B. Most older adults become more depressed as their social circles shrink

Smaller networks reflect chosen focus on close ties; depression is not a normal outcome of aging.

C. Crystallized knowledge and processing speed both fall sharply with age

Processing speed slows, but crystallized knowledge such as vocabulary is stable or grows into late life.

D. Emotional well-being is often stable or better, though processing slows ✓ KEY

Older adults often report steady or improved emotional well-being, while fluid abilities like speed show gradual slowing.

Rationale: Research finds that emotional well-being tends to hold steady or improve in late adulthood. Carstensen's socioemotional selectivity theory explains that older adults focus on fewer, more meaningful relationships and positive emotional experience. Cognitively, processing speed and some fluid abilities slow, but crystallized knowledge stays stable, and severe decline is not a normal part of aging.

References: Berk (Development Through the Lifespan) — Late adulthood: socioemotional selectivity theory; fluid vs crystallized intelligence

☐ Approve ☐ Revise ☐ Retire Notes:

A high school teacher tells the school counselor that a 15-year-old in her class has cuts on her arm. The teacher says, "She is just doing it for attention, so I plan to ignore it so I don't reward it." How should the counselor BEST respond to the teacher?

A. Agree that ignoring it is wise, since attention can reinforce cutting

Ignoring self-injury leaves the student without assessment or help and rests on a common myth about its purpose.

B. Explain cutting usually eases distress, and arrange to see the student ✓ KEY

Self-injury most often regulates painful emotion, and the student needs a prompt, calm assessment that includes suicide risk.

C. Ask the teacher to tell the student to stop and to check her arms daily

Demands and body checks by staff tend to increase shame and secrecy and are not a substitute for assessment.

D. Suggest the teacher discuss self-injury with the whole class that day

Class discussion of self-injury can spread it through social contagion and exposes the student.

Rationale: Nonsuicidal self-injury is most often used privately to relieve intense negative emotion, not to gain attention. Even when it is not suicidal, it is a risk factor for later suicide attempts, so any student who self-injures needs an assessment that includes suicide risk. Counselors consult with staff to replace the "attention" myth with a calm, supportive referral pathway.

References: DSM-5-TR — Conditions for Further Study: Nonsuicidal self-injury disorder (functions and course) · Linehan (DBT Skills Training Manual) — Self-harm as an emotion-regulation behavior

☐ Approve ☐ Revise ☐ Retire Notes:

A 16-year-old has cut herself repeatedly for a year and has made one suicide attempt. She has intense mood swings and frequent conflict with her parents. Which treatment approach has the STRONGEST research support for reducing self-harm in adolescents like her?

A. Dialectical behavior therapy adapted for teens, with family skills work ✓ KEY

DBT for adolescents, which includes caregivers in skills training, has randomized trial support for reducing self-harm.

B. Client-centered therapy offered weekly in a supportive, open format

Supportive therapy builds rapport but has weaker evidence than DBT-A for reducing repeated self-harm.

C. Psychoeducation for her parents alone, with no individual sessions

Parent education helps but cannot replace skills training and individual treatment for a high-risk teen.

D. A peer support group where members discuss their self-harm stories

Groups that focus on sharing self-harm can increase contagion and are not an evidence-based first choice.

Rationale: Dialectical behavior therapy for adolescents (DBT-A) combines individual therapy, multifamily skills groups (mindfulness, distress tolerance, emotion regulation, interpersonal effectiveness, and walking the middle path), phone coaching and team consultation. Randomized trials show it reduces self-harm and suicide attempts in adolescents with repeated self-harm.

References: Linehan (DBT Skills Training Manual) — DBT skills modules; adaptation for adolescents and families

► **Check:** DBT-A trial evidence (Mehlum et al., 2014; McCauley et al., 2018) is not in the approved source list; SME to confirm the Linehan locator covers the adolescent adaptation.

☐ Approve ☐ Revise ☐ Retire Notes:

A 76-year-old widow goes to bed at 8 p.m., lies awake for hours, naps twice a day, and gets up at 7 a.m. feeling unrested. She asks the counselor whether she should ask her doctor for a sleeping pill. Which recommendation is MOST appropriate?

A. A sleeping pill, since medication works best for insomnia in later life

Sedative-hypnotics raise the risk of falls, confusion and dependence in older adults and are not first-line.

B. More time in bed and longer naps so that she can catch up on lost sleep

Extra time in bed and naps weaken sleep drive and fragment night sleep further.

C. CBT for insomnia, including less time in bed and fewer daytime naps ✓ **KEY**

CBT-I is first-line for chronic insomnia at any age; limiting time in bed and naps builds sleep drive and consolidates sleep.

D. Acceptance that broken sleep is normal aging and needs no treatment

Sleep becomes lighter with age, but distressing insomnia with daytime effects is treatable and should not be dismissed.

Rationale: Cognitive behavioral therapy for insomnia is the recommended first-line treatment for chronic insomnia, including in older adults. Sleep restriction (or the gentler sleep compression) matches time in bed to actual sleep, and limiting naps strengthens sleep drive. Hypnotic medications carry added risks of falls and cognitive effects in later life.

References: AASM Clinical Practice Guideline (Insomnia) — CBT-I and sleep restriction therapy for chronic insomnia (strong recommendation)

☐ Approve ☐ Revise ☐ Retire Notes:

A factory worker was moved to a permanent overnight shift six months ago. He cannot stay asleep when he goes to bed at 9 a.m. and fights to stay awake at work around 4 a.m. His sleep was normal on his old day schedule. Which DSM-5-TR diagnosis BEST fits?

A. Hypersomnolence disorder

Hypersomnolence involves excess sleepiness despite adequate main sleep, not a mismatch with a work schedule.

B. Insomnia disorder

His sleep loss is explained by a circadian mismatch with his shift, which a circadian diagnosis better captures.

C. Delayed sleep phase type

Delayed sleep phase is a late internal clock, not a problem caused by night-shift work.

D. Circadian disorder, shift work type ✓ **KEY**

Insomnia during the sleep period and sleepiness during work, tied to a night-shift schedule, defines shift work type.

Rationale: Circadian rhythm sleep-wake disorder, shift work type, involves insomnia during the major sleep period and/or excessive sleepiness during the major awake period, associated with a shift work schedule such as night work. Counseling supports sleep scheduling, light management and protecting daytime sleep.

References: DSM-5-TR — Circadian Rhythm Sleep-Wake Disorders: shift work type

☐ Approve ☐ Revise ☐ Retire Notes:

A new principal asks the high school counselor to serve as the full-time state testing coordinator and to handle discipline for tardy students. The counselor is concerned that these duties will leave little time for students. Which response BEST reflects the ASCA National Model?

A. Accept both duties, since the principal sets the counselor's role

The ASCA model expects counselors to advocate for an appropriate role rather than simply absorb noncounseling duties.

B. Use data on student needs to propose shifting noncounseling duties ✓ **KEY**

The model calls for using data and an annual agreement with the principal to keep most time on direct and indirect student services.

C. Refuse both duties and file a complaint with the district office

Refusal and escalation damage collaboration; the model favors data-based advocacy with the principal first.

D. Take on discipline only, since it gives contact with at-risk students

ASCA lists handling discipline as inappropriate for counselors because it conflicts with the helping role.

Rationale: The ASCA National Model recommends that school counselors spend about 80% of their time in direct and indirect services to students. Coordinating schoolwide testing and administering discipline are listed as inappropriate duties. Counselors use data and an annual administrative conference to advocate for a role focused on academic, career and social/emotional development.

References: ASCA National Model — Appropriate vs inappropriate activities for school counselors · Gladding (Counseling: A Comprehensive Profession) — School counseling: the ASCA National Model

☐ Approve ☐ Revise ☐ Retire Notes:

An 11th grader studies hard and knows the material, but during exams her heart pounds, her mind goes blank, and she thinks, "If I fail this, my future is over." Her grades are dropping. Which intervention should the school counselor offer FIRST?

A. Relaxation training plus work on her catastrophic test thoughts ✓ **KEY**

Test anxiety responds well to combined arousal-reduction and cognitive restructuring, which target both parts of her reaction.

B. A referral for psychoeducational testing for a learning disability

She knows the material; the pattern points to anxiety during tests, not a learning disability.

C. More hours of studying each night so she feels better prepared

She is already prepared; more study does not address the anxiety that blocks retrieval.

D. A request to excuse her from exams for the rest of the semester

Avoidance maintains anxiety and does not build the skills she needs.

Rationale: Test anxiety has physiological (arousal), cognitive (worry and catastrophic thinking) and behavioral parts. Cognitive behavioral approaches that combine relaxation or systematic desensitization with cognitive restructuring, plus test-taking skills when needed, are well supported. Avoidance tends to strengthen the anxiety.

References: Gladding (Counseling: A Comprehensive Profession) — School counseling: test anxiety interventions · Corey (Theory & Practice) — Cognitive behavior therapy: relaxation training and cognitive restructuring

☐ Approve ☐ Revise ☐ Retire Notes:

A 9-year-old in a gifted program reads at a high school level and debates ethics with adults, yet melts down when he loses a board game and struggles to make friends his age. His parents ask how he can be "so mature and so immature." Which concept BEST explains this?

A. A twice-exceptional profile that signals a hidden learning disability

Twice-exceptional means giftedness with a disability; nothing here indicates a disability.

B. Emotional immaturity caused by too much praise for his intellect

This blames parenting without evidence; uneven development is common in gifted children.

C. Asynchronous development across his intellectual and social growth ✓ **KEY**

Gifted children often develop unevenly, with intellect far ahead of social and emotional skills that match their age.

D. Early signs of a disruptive mood dysregulation disorder pattern

DMDD requires severe, frequent outbursts and persistent irritability across settings; this is not described.

Rationale: Asynchronous development means a gifted child's intellectual abilities run well ahead of social, emotional or physical development. This uneven profile can produce frustration, perfectionism and difficulty finding peers. Counselors normalize the pattern for families, support social-emotional skills, and help the child find intellectual peers.

References: Berk (Development Through the Lifespan) — Middle childhood: giftedness, creativity and talent

► **Check:** The term "asynchronous development" comes from gifted-education literature (Columbus Group; Silverman); SME to confirm Berk covers it or swap the reference.

☐ Approve ☐ Revise ☐ Retire Notes:

A 29-year-old Army veteran led a 12-person logistics team for six years. Applying for civilian jobs, his resume lists military job codes and unit titles, and he has had no interviews. He says, "Nobody out here values what I did." What should the counselor do FIRST?

A. Steer him toward entry-level roles until he gains civilian work history

This undervalues his experience and could lead to underemployment.

B. Give an interest inventory to find a field unrelated to his service

His issue is translating proven skills, not a lack of known interests.

C. Explore his anger at civilians as a possible sign of combat trauma

His frustration is an understandable response to rejection; nothing suggests a trauma reaction.

D. Help him translate his military duties into civilian skills and terms ✓ **KEY**

*Translating military roles into transferable civilian skills, using tools like the O*NET military crosswalk, is a key first step.*

Rationale: Many veterans have strong leadership, logistics and technical skills that civilian employers do not recognize when described in military language. Career counselors help veterans identify transferable skills, translate job codes and titles into civilian terms (for example, with the O*NET military crosswalk), and connect with veteran employment resources.

References: Sharf (Career Development Theory) — Career counseling with special populations: transferable skills and occupational information

► **Check:** Veteran-specific content may not be covered directly in Sharf; SME to confirm the locator.

☐ Approve ☐ Revise ☐ Retire Notes:

A career counselor is screening clients for a program that serves people who are underemployed. Which client BEST fits the definition of underemployment?

A. A nurse who left work by choice to care for her newborn twins

Leaving the labor force by choice is not underemployment.

B. An engineer stocking shelves part time who wants full-time work ✓ KEY

Working part time involuntarily in a job well below his training fits underemployment.

C. A teacher on summer break who will return to her job in the fall

A planned seasonal break from a full-time job is not underemployment.

D. A laid-off welder who has been looking for a job for six months

Someone without a job who is seeking work is unemployed, not underemployed.

Rationale: Underemployment describes working fewer hours than desired (involuntary part-time work) or in a job that does not use one's education, skills or experience. It is associated with lower well-being and job satisfaction, much as unemployment is, so counselors address both the practical job search and its emotional toll.

References: Sharf (Career Development Theory) — Adult career crises and transitions: unemployment and underemployment

☐ Approve ☐ Revise ☐ Retire Notes:

A 44-year-old woman with top performance reviews has been passed over three times for vice president. Each time, a man with a similar record was chosen. She tells her counselor, "I must not be leadership material." Which counselor response is BEST?

A. Focus on assertiveness training to correct her leadership weaknesses

This assumes a personal deficit and ignores the strong evidence of her performance.

B. Suggest she lower her goals to match what the company is likely to offer

Lowering goals accepts a barrier as fixed and undermines her career self-efficacy.

C. Help her weigh structural barriers to advancement, not just self-blame ✓ KEY

The pattern fits a glass ceiling; exploring systemic barriers counters self-blame and opens options for action.

D. Advise her to file a discrimination lawsuit against the firm at once

Legal action is her decision to make after exploring options, not the counselor's directive.

Rationale: The glass ceiling is an invisible barrier that limits women's advancement to top leadership despite qualifications. Women may internalize such barriers as personal failings. Counselors help clients recognize contextual and systemic barriers, preserve career self-efficacy, and consider strategies such as mentoring, sponsorship, advocacy or a move to another organization.

References: Sharf (Career Development Theory) — Career development of women: barriers to advancement · MSJCC (Ratts et al., 2016) — Counseling and advocacy interventions: addressing systemic barriers

☐ Approve ☐ Revise ☐ Retire Notes:

A college sophomore has clear interests in health and science but has not declared a major. She says, "I'm bad at making big choices; I don't trust myself to pick right." A counselor using social cognitive career theory wants to build her career decision self-efficacy. Which strategy would be MOST powerful?

A. Hand her more printed details on each major so she can study them

She already knows her interests; more information does not build confidence in deciding.

B. Tell her firmly that she has what it takes to make a strong decision

Verbal persuasion is a weaker source of self-efficacy than direct success experiences.

C. Have her watch peers describe how they each chose their own majors

Vicarious learning helps, but it is less powerful than her own successes.

D. Set graded decision tasks she can complete and succeed at herself ✓ **KEY**

Mastery experiences, personal success at the task, are the strongest source of self-efficacy in Bandura's and SCCT's view.

Rationale: Social cognitive career theory (Lent, Brown and Hackett) draws on Bandura's four sources of self-efficacy: performance accomplishments (mastery), vicarious learning, verbal persuasion, and physiological and emotional states. Mastery experiences are the most powerful. For indecision rooted in low decision-making confidence, counselors structure small, achievable decision steps.

References: Sharf (Career Development Theory) — Social cognitive career theory: sources of self-efficacy; career decision self-efficacy

☐ Approve ☐ Revise ☐ Retire Notes:

A counselor trained in divorce mediation is helping a couple reach a parenting and property agreement. In the third session, the wife privately asks the counselor to also see her for individual therapy for depression after the divorce is final. What should the counselor do?

A. Stay in the neutral mediator role and refer her to another therapist ✓ **KEY**

Taking one party as a therapy client would compromise mediator neutrality; a referral meets her need without a role conflict.

B. Agree, since therapy would begin only after the mediation has ended

Even later, the role change could appear to favor her and undermine the husband's trust in the agreement.

C. Begin individual sessions now so her depression does not slow progress

Concurrent therapy with one party creates a clear conflict of interest in mediation.

D. Ask the husband's permission and then see her alone if he agrees to it

Consent does not remove the conflict created when the mediator becomes one party's therapist.

Rationale: Divorce mediation is a neutral, task-focused process for reaching agreements; it is not therapy. When one party seeks a therapeutic relationship, the counselor clarifies roles and refers to avoid a conflict of interest that would compromise impartiality. The ACA Code addresses role changes and serving multiple clients whose interests may conflict.

References: ACA Code of Ethics — A.6.d Role Changes in the Professional Relationship; A.8 Multiple Clients

☐ Approve ☐ Revise ☐ Retire Notes:

Parents of a 13-year-old are alarmed that she now argues about clothes, chores and bedtime and rolls her eyes at their rules. She has good grades, close friends, and still talks with them about school. What should the counselor tell the parents?

A. The bickering may signal early oppositional defiant disorder

ODD involves a persistent, pervasive pattern of hostility; mild, everyday conflict with good functioning is not ODD.

B. Mild conflict over daily issues is common in early adolescence ✓ **KEY**

Parent-teen conflict over everyday matters rises in early adolescence and usually reflects normal striving for autonomy.

C. Firm limits with no discussion will end the conflict quickest

An authoritarian style tends to escalate conflict; warmth with reasoned limits works better.

D. Letting her set her own rules will reduce tension at home

A permissive approach removes needed structure; teens still benefit from monitoring and clear limits.

Rationale: Parent-adolescent conflict over everyday issues typically increases in early adolescence as teens seek autonomy, and it usually declines later. Severe, constant conflict is not normal and warrants attention. Authoritative parenting, which pairs warmth with clear, explained limits and gradually expanded autonomy, is linked to the best adolescent outcomes.

References: Berk (Development Through the Lifespan) — Adolescence: parent-child relationships, autonomy and conflict

☐ Approve ☐ Revise ☐ Retire Notes:

A family lost their home and business in a wildfire. In counseling, they argue about what went wrong but also describe neighbors who helped and a shared faith that "we will rebuild." A counselor using Froma Walsh's family resilience framework would MOST likely focus on which task?

A. Finding the family member whose symptoms carry the family stress

Locating an identified patient fits other systems models, not Walsh's strengths-based resilience focus.

B. Tracing the family's patterns of fusion back over three generations

Multigenerational tracing is a Bowenian task, not the core of the resilience framework.

C. Strengthening shared meaning, connection and joint problem-solving ✓ **KEY**

Walsh's key processes are belief systems, organizational patterns and communication/problem-solving, all built on family strengths.

D. Rearranging the family hierarchy so the parents hold firmer power

Restructuring hierarchy is a structural family therapy goal, not the main resilience focus.

Rationale: Walsh's family resilience framework views families facing adversity as challenged, not damaged. It strengthens three domains of key processes: belief systems (making meaning of adversity, a positive outlook, transcendence and spirituality), organizational patterns (flexibility, connectedness, social and economic resources) and communication/problem-solving (clarity, open emotional sharing, collaborative problem-solving).

References: Goldenberg (Family Therapy) — Family resilience (Walsh): key processes in family functioning

► **Check:** Walsh's framework is primary in Walsh (2016), Strengthening Family Resilience; SME to confirm the Goldenberg locator.

☐ Approve ☐ Revise ☐ Retire Notes:

A 35-year-old client mentions that during worship at her Pentecostal church she speaks in tongues and sometimes feels the Holy Spirit move through her. She works, has close relationships, and shows no other unusual experiences. How should the counselor BEST understand these reports?

A. As possible psychotic symptoms that call for a psychiatric referral

Experiences accepted within the client's faith community, with intact functioning, are not evidence of psychosis.

B. As accepted religious practices within her own faith community ✓ **KEY**

DSM-5-TR cautions that experiences sanctioned by a person's culture or religion should not be judged as symptoms.

C. As dissociative episodes that need a structured diagnostic interview

DSM-5-TR excludes trance states that are a normal part of broadly accepted religious practice.

D. As avoidance of her problems that should become a focus of treatment

Nothing suggests her faith practice is causing problems; treating it as avoidance imposes the counselor's values.

Rationale: Clinicians must judge unusual experiences in the context of the client's cultural and religious norms. Practices such as glossolalia are normative in some traditions and are not symptoms when they occur within accepted religious contexts and do not cause distress or impairment. DSM-5-TR notes that dissociative disorder diagnoses do not apply to states that are a normal part of broadly accepted cultural or religious practice.

References: DSM-5-TR — Cultural concepts and distinguishing culturally sanctioned experiences from psychopathology; dissociative disorders exclusion for religious practice · ACA Code of Ethics — E.5.b Cultural Sensitivity

☐ Approve ☐ Revise ☐ Retire Notes:

A 27-year-old is referred eight months after a moderate traumatic brain injury from a car crash. He forgets what was discussed, tires quickly, and becomes irritable when sessions run long. Which adjustment to counseling is MOST appropriate?

A. Long open-ended sessions so he can explore at his own pace

Long, unstructured sessions tax attention and fatigue after brain injury.

B. Insight-focused talk therapy without memory aids or notes

Without memory supports, he is unlikely to retain or use session content.

C. Fewer sessions spaced far apart to reduce his frustration

Wide spacing weakens retention; short, frequent, structured sessions fit better.

D. Short, structured sessions with written summaries and aids ✓ **KEY**

Shorter sessions, concrete structure, repetition and written notes compensate for memory and fatigue problems after TBI.

Rationale: Clients with traumatic brain injury often have deficits in memory, attention, processing speed and emotional regulation, along with fatigue. Counselors adapt by using shorter, structured sessions, repetition, written summaries and external aids, involving family when appropriate, and coordinating with rehabilitation and neuropsychology providers.

References: Gladding (Counseling: A Comprehensive Profession) — Counseling clients with disabilities: adapting counseling to cognitive impairment

☐ Approve ☐ Revise ☐ Retire Notes:

A 30-year-old client who was born deaf comes to counseling, with a sign language interpreter, for stress at work. She mentions proudly that she is active in the Deaf community. The counselor plans to begin by helping her grieve the loss of hearing. Why is this plan MOST problematic?

A. It imposes a loss frame on an identity she holds as cultural ✓ KEY

Many culturally Deaf people view deafness as a linguistic and cultural identity, not a loss to grieve.

B. Grief work is not suitable when an interpreter is in the room

Interpreters do not rule out any type of counseling work; the problem is the counselor's assumption.

C. Stage models say grief over hearing ends by early adulthood

No such rule exists; the issue is that she does not experience her deafness as a loss.

D. Counselors should refer deaf clients to a deaf counselor only

Culturally competent hearing counselors can serve Deaf clients; automatic referral is not required.

Rationale: Disability identity varies. Many people who are culturally Deaf (capital D) see themselves as members of a linguistic and cultural minority with its own language, such as ASL, rather than as people with a deficit. Applying a medical or loss model without asking the client imposes the counselor's worldview. Counselors explore how clients define their own identity and focus on the presenting concern.

References: Sue & Sue (Counseling the Culturally Diverse) — Counseling individuals with disabilities: disability identity and Deaf culture

☐ Approve ☐ Revise ☐ Retire Notes:

A 72-year-old transgender woman who transitioned in her forties must move into assisted living after a stroke. She says, "I fought my whole life to be myself. I'm terrified the staff will misgender me or that I'll have to hide again." Which counselor response is BEST?

A. Reassure her that staff are trained, so her worries are probably unfounded

Her fears reflect real risks documented for LGBTQ+ older adults; dismissing them invalidates her.

B. Suggest she keep her history private until she knows the staff better

Advising concealment adds to minority stress and puts the burden of others' bias on her.

C. Explore her fears and help her find and advocate for affirming facilities ✓ KEY

Validating her fears and helping her identify affirming care and supports addresses both her distress and real barriers.

D. Focus on her stroke recovery, since gender identity is a settled matter

Her identity is settled, but her safety and dignity in care are urgent present concerns.

Rationale: Transgender older adults often carry histories of discrimination and may lack family support. Many fear mistreatment, misgendering or pressure to go back into the closet in long-term care. Counselors validate these fears, help clients locate LGBTQ+-affirming providers and aging resources, and advocate for respectful, competent care.

References: MSJCC (Ratts et al., 2016) — Counseling and advocacy interventions at the institutional level · Sue & Sue (Counseling the Culturally Diverse) — Counseling LGBTQ+ individuals; counseling older adults

☐ Approve ☐ Revise ☐ Retire Notes:

A 26-year-old bisexual man in a relationship with a woman says his family calls his bisexuality "a phase," while gay friends tell him he is "really straight now." He feels he belongs nowhere. Which concept BEST describes this source of minority stress?

A. Internalized homonegativity that he holds about his own identity

Internalized stigma is a self-directed belief; here the invalidation comes from others.

B. Bisexual erasure from both straight and gay communities ✓ KEY

Bisexual people often face invalidation from both heterosexual and gay or lesbian communities, sometimes called double discrimination.

C. Identity confusion, the first stage of Cass's identity model

Cass describes stages of gay and lesbian identity; it does not capture rejection from two communities.

D. Heterosexual privilege that protects him from any minority stress

Being in a different-gender relationship does not remove the stigma and invisibility bisexual people face.

Rationale: Bisexual people experience unique minority stressors, including bisexual erasure (denial that bisexuality is real or lasting) and binegativity from both heterosexual and gay or lesbian communities. This double marginalization can limit community support and is linked to higher rates of depression and anxiety than among gay and lesbian peers.

References: Sue & Sue (Counseling the Culturally Diverse) — Counseling LGBTQ+ individuals: bisexual invisibility and double discrimination

► **Check:** Confirm the Sue & Sue chapter discusses bisexual erasure/binegativity and double discrimination.

☐ Approve ☐ Revise ☐ Retire Notes:

A 38-year-old woman who entered the country without documentation seeks counseling for panic, poor sleep and constant fear that she will be separated from her children. At intake she hesitates and asks, "Will you tell anyone about my status?" What should the counselor do FIRST?

A. Explain confidentiality and its limits, which do not include her status ✓ KEY

Immigration status is not a mandated reporting category; explaining confidentiality and its true limits builds the safety she needs.

B. Record her status in detail so that the clinical file is fully complete

Counselors record what is clinically needed; detailed status notes add risk without clinical benefit.

C. Refer her to an immigration attorney before beginning counseling work

Legal referral may help later, but her immediate question is about confidentiality and her panic needs care.

D. Tell her you cannot discuss her legal status because of agency policy

Deflecting her question deepens fear and mistrust; she needs a clear answer about confidentiality.

Rationale: Undocumented clients live with chronic fear of detention and family separation, which can cause anxiety, depression and trauma symptoms. Fear of disclosure is a major barrier to care. Counselors clearly explain confidentiality and its actual limits (such as risk of harm or child abuse), note that immigration status is not a reportable matter, and document only what is clinically necessary.

References: ACA Code of Ethics — B.1.d Explanation of Limitations; A.2.b Types of Information Needed · Sue & Sue (Counseling the Culturally Diverse) — Counseling immigrants and refugees: undocumented status and fear of disclosure

► **Check:** No general mandate requires counselors to report immigration status, but some funded programs or states may have specific rules; SME to confirm the general framing.

☐ Approve ☐ Revise ☐ Retire Notes:

A 58-year-old woman who immigrated from rural China two years ago is brought in by her daughter. She reports headaches, fatigue and poor appetite, says her doctor found nothing wrong, and insists, "I am not crazy. I do not need to talk about feelings." Which approach is BEST?

A. Explain that her physical symptoms are really depression in disguise

Relabeling her experience early challenges her worldview and is likely to end engagement.

B. Ask her daughter to describe her mother's emotions on her behalf

This sidelines the client and may breach her preferences about what is shared within the family.

C. Tell her counseling can begin only once she accepts a diagnosis

Requiring acceptance of a diagnosis is a barrier and does not fit her cultural view of help-seeking.

D. Respect her view of the symptoms and offer practical, active help ✓ **KEY**

Starting with her own framing and offering concrete, problem-focused help builds credibility with less acculturated clients.

Rationale: Less acculturated clients from many cultures underuse counseling, may express distress through somatic complaints, and may see emotional talk as stigmatizing. Sue and Sue describe culturally responsive engagement: accept the client's explanation of the problem, coordinate with medical care, and offer active, structured, practical help. An early, felt benefit (what Sue and Sue call gift giving) builds the counselor's credibility.

References: Sue & Sue (Counseling the Culturally Diverse) — Asian Americans: somatic expression of distress; credibility and gift giving

☐ Approve ☐ Revise ☐ Retire Notes:

A client with schizophrenia has gained 25 pounds in the six months since starting a second-generation antipsychotic. He says he feels constantly hungry and is thinking of stopping the medication. What should the counselor do?

A. Support his plan to stop, since the weight gain is a serious problem

Stopping an antipsychotic abruptly risks relapse; medication changes belong to the prescriber.

B. Reassure him that the weight gain is cosmetic and will soon level off

Weight gain on these drugs signals metabolic risk and is not merely cosmetic.

C. Urge him to talk to his prescriber about the gain and metabolic checks ✓ **KEY**

Second-generation antipsychotics carry metabolic risks, and prescribers monitor weight, glucose and lipids and can adjust treatment.

D. Recommend a specific medication change that has fewer side effects

Recommending specific drugs is outside the counselor's scope of practice.

Rationale: Several second-generation antipsychotics, notably olanzapine and clozapine, are associated with weight gain, insulin resistance, diabetes and lipid changes. Guidelines call for monitoring weight, waist size, blood pressure, glucose and lipids. Counselors recognize these concerns, encourage clients to raise them with the prescriber rather than stop medication, and support healthy lifestyle changes.

References: APA CPG — Treatment of patients with schizophrenia (2020): monitoring for metabolic side effects of antipsychotics

☐ Approve ☐ Revise ☐ Retire Notes:

A father tells the counselor, "We gave our son a dose of his cousin's ADHD stimulant before a test, and he focused much better. So he must have ADHD, right?" Which response reflects an accurate understanding of stimulant medication?

A. Stimulants can sharpen focus in most people, so a response is not a diagnosis ✓ **KEY**

Stimulants improve attention in people with and without ADHD, so a good response cannot confirm the diagnosis.

B. A calming effect from a stimulant is a reliable sign that ADHD is present

The idea of a "paradoxical" response unique to ADHD is a myth; stimulants act similarly in those without ADHD.

C. Because stimulants are addictive, a good response is a sign of misuse risk

One positive response does not indicate misuse risk; the issue is that response is not diagnostic.

D. A good response confirms ADHD only if the effect lasts more than a week

Duration of response does not make it diagnostic; ADHD is diagnosed by DSM-5-TR criteria and history.

Rationale: A common misconception holds that stimulants calm or focus only people with ADHD. In fact, stimulants improve attention and concentration in most people, so response to medication cannot establish a diagnosis. ADHD is diagnosed through a careful history and DSM-5-TR criteria across settings. The counselor should also address the safety concern of giving a child someone else's prescription.

References: DSM-5-TR — Attention-deficit/hyperactivity disorder: diagnostic criteria and differential diagnosis · NBCC Study Supplement — Psychopharmacology: stimulant medications

► **Check:** The fact (non-specific stimulant response; Rapoport et al., 1978) is well established, but neither cited source states it directly; SME to confirm or substitute a pharmacology reference.

☐ Approve ☐ Revise ☐ Retire Notes:

A veteran completed prolonged exposure for PTSD. His fear when recalling a roadside ambush has dropped sharply, yet he still feels intense shame that he froze and a friend died, says he "doesn't deserve" good things, and avoids his family. What should the counselor focus on NEXT?

A. Repeat a full course of exposure, since fear must still be driving him

His fear has already habituated; repeating exposure does not target the guilt and shame that remain.

B. Address guilt and shame through meaning-making and self-forgiveness ✓ **KEY**

Moral injury centers on guilt, shame and broken meaning, which call for work on appraisals, meaning and self-forgiveness.

C. Reassure him that freezing was not his fault and move toward ending

Quick reassurance rarely shifts deep guilt and leaves the moral-injury symptoms untreated.

D. Refer him for a medication to reduce his remaining startle response

His remaining problems are guilt, shame and withdrawal, not hyperarousal.

Rationale: Moral injury describes lasting guilt, shame and loss of meaning after acts that violate one's moral code, including perceived failures to act. Fear-based exposure can reduce fear without resolving these reactions. Treatment adds work on guilt appraisals (as in cognitive processing therapy), meaning-making, self-forgiveness and repair (as in approaches such as adaptive disclosure), often with spiritual or community resources.

References: VA/DoD CPG — Management of PTSD and acute stress disorder: trauma-focused psychotherapies (PE, CPT)

► **Check:** Moral injury is not a formal diagnosis and the VA/DoD CPG does not specifically address it; SME to confirm the citation or add a moral-injury source (e.g., Litz et al., Adaptive Disclosure).

☐ Approve ☐ Revise ☐ Retire Notes:

A 42-year-old retired Navy chief landed a well-paid civilian job after 20 years of service. Six months in, he says, "The work is fine, but I feel pointless. Nobody has each other's back here, and nothing feels urgent." What is the counselor's BEST initial focus?

A. Help him find a higher-paying job with greater responsibility

Pay and rank are not his concern; he describes loss of purpose and belonging.

B. Screen him for PTSD, since his distress began after service

Nothing he describes suggests trauma symptoms; his concern is transition-related.

C. Teach him to adopt a civilian mindset and leave the Navy behind

Dismissing his military identity ignores a valued part of who he is.

D. Explore his loss of military identity, mission and camaraderie ✓ **KEY**

Leaving the military often means losing identity, purpose and close bonds, which are central transition tasks.

Rationale: Military-to-civilian transition involves more than finding a job. Veterans often lose a strong shared identity, a clear mission, structure and unit cohesion. Counselors explore these losses, help the veteran carry valued military strengths into civilian life, and support sources of purpose and belonging such as veteran organizations and service roles.

References: Sharf (Career Development Theory) — Adult career transitions; Schlossberg's model applied to job and role change

► **Check:** Veteran transition content may not appear in Sharf specifically; SME to confirm the locator.

☐ Approve ☐ Revise ☐ Retire Notes:

An 8-year-old from a low-income family has had a painful, untreated dental abscess for months and cannot see the board without glasses. The school nurse has arranged free dental care and free glasses three times, and the parent has not followed through. Which detail MOST supports a concern of neglect rather than poverty alone?

A. The family's low income and lack of dental insurance coverage

Lack of resources alone is not neglect; poverty must be separated from failure to provide care.

B. The child's trouble seeing the board in her daily classroom work

This shows an unmet need but does not show why the need is unmet.

C. Repeated failure to use free care that was arranged for the child ✓ **KEY**

When accessible care is offered and still not used for a serious need, the failure points to neglect rather than lack of means.

D. The length of time the abscess has gone without any treatment

Duration shows severity, but on its own could still reflect lack of access.

Rationale: Neglect is a failure to meet a child's basic needs, including medical and dental care, when the caregiver is able or has been offered reasonable means to do so. Many state definitions exclude conditions caused solely by poverty. Indicators include untreated medical or dental problems, chronic hunger, poor hygiene and frequent absence. Repeated refusal of accessible care for a serious need supports reasonable suspicion and a report.

References: State law — Child abuse and neglect statutes: definitions of neglect and poverty exclusions · ACA Code of Ethics — B.2.a Serious and Foreseeable Harm and Legal Requirements

☐ Approve ☐ Revise ☐ Retire Notes:

A 46-year-old who drank heavily every day for two years developed low mood, poor sleep and hopelessness during that period. He stopped drinking and completed withdrawal six weeks ago. His depressive symptoms have not eased since then. Which conclusion is BEST supported?

A. Alcohol-induced depressive disorder fits, since the mood began while he drank

Onset during use raises the question, but symptoms that persist well past withdrawal argue against a purely induced disorder.

B. No depressive diagnosis fits until he stays abstinent for a full twelve months

DSM-5-TR does not require a year of abstinence; about one month after withdrawal is the usual benchmark.

C. An independent depressive disorder is likely, as it has lasted past one month ✓ KEY

Persistence for a substantial period (about one month) after withdrawal ends is evidence of an independent depressive disorder.

D. His symptoms show a protracted alcohol withdrawal that will soon clear alone

Acute withdrawal resolves within days; six weeks of unchanged depression should not be assumed to be withdrawal.

Rationale: Substance/medication-induced depressive disorder is diagnosed when depressive symptoms develop during or soon after intoxication or withdrawal and are not better explained by an independent depressive disorder. DSM-5-TR names persistence of symptoms for a substantial period, about one month, after acute withdrawal or severe intoxication ends as evidence of an independent disorder. Ongoing reassessment after abstinence is key to the differential.

References: DSM-5-TR — Substance/Medication-Induced Depressive Disorder: Criterion C (evidence of an independent depressive disorder)

☐ Approve ☐ Revise ☐ Retire Notes:

Parents of a 9-year-old say that since age 7 he has had severe temper outbreaks about four times a week at home and school, and is angry and irritable most of the day between outbursts. He has had no distinct periods of elevated mood or decreased need for sleep. Which diagnosis BEST fits?

A. Disruptive mood dysregulation disorder, given the chronic irritability ✓ KEY

Frequent severe outbursts plus persistently irritable mood for 12+ months in two settings, onset before 10, fits DMDD.

B. Bipolar I disorder, given his frequent rages and his very irritable mood

Pediatric bipolar disorder is episodic with distinct manic periods; he shows chronic, nonepisodic irritability.

C. Oppositional defiant disorder, as the outbursts occur in two settings

When DMDD criteria are met, ODD is not diagnosed separately; the severe persistent mood component points to DMDD.

D. Intermittent explosive disorder, due to his repeated angry outbursts

IED lacks the persistently irritable mood between outbursts, and it is not diagnosed when DMDD criteria are met.

Rationale: DMDD requires severe recurrent temper outbursts averaging three or more times per week, with persistently irritable or angry mood most of the day nearly every day between them, for 12 months or more in at least two settings. Onset is before age 10, and the diagnosis is not first made before age 6 or after age 18. It was added to the depressive disorders partly to reduce overdiagnosis of bipolar disorder in children with chronic, nonepisodic irritability.

References: DSM-5-TR — Disruptive Mood Dysregulation Disorder: Criteria A-K

☐ Approve ☐ Revise ☐ Retire Notes:

A 23-year-old had one manic episode last year that led to a brief hospital stay. She has never had a major depressive episode. A classmate in her counseling program says she "cannot have bipolar disorder without depression." Under DSM-5-TR, what is required for bipolar I disorder?

A. At least one manic episode plus at least one major depressive episode

This describes a common course, but a major depressive episode is not required for bipolar I disorder.

B. At least one hypomanic episode plus at least one major depressive episode

These are the requirements for bipolar II disorder, which has no lifetime manic episode.

C. Two or more manic episodes separated by at least two months of remission

A single manic episode suffices; recurrence and remission intervals are not required for the diagnosis.

D. At least one manic episode, with or without any past depressive episodes ✓ KEY

Bipolar I requires criteria for at least one manic episode; hypomanic or depressive episodes may occur but are not required.

Rationale: Bipolar I disorder is diagnosed when criteria have been met for at least one manic episode that is not better explained by a schizophrenia spectrum disorder. The manic episode may be preceded or followed by hypomanic or major depressive episodes, but these are not required. Bipolar II, in contrast, requires at least one hypomanic and at least one major depressive episode and no manic episode.

References: DSM-5-TR — Bipolar I Disorder: Criterion A and text on manic, hypomanic and depressive episodes

☐ Approve ☐ Revise ☐ Retire Notes:

A 37-year-old meets full criteria for a current manic episode. During most days of the episode she also feels intensely sad, has lost interest in her usual hobbies, feels worthless, and has recurrent thoughts of death. Which specifier is MOST appropriate?

A. With anxious distress, because she shows marked emotional turmoil daily

Anxious distress requires symptoms like feeling keyed up, tense, restless or fearful, not depressive symptoms.

B. With mixed features, because of her concurrent depressive symptoms ✓ KEY

Three or more depressive symptoms on most days of a manic episode meet the mixed features specifier.

C. With rapid cycling, because her mood moves between high and low

Rapid cycling requires four or more mood episodes in 12 months, not simultaneous symptoms in one episode.

D. With melancholic features, as she has lost interest in her hobbies

Melancholic features is a specifier for major depressive episodes, not for a current manic episode.

Rationale: The "with mixed features" specifier applies to a manic or hypomanic episode when at least three depressive symptoms (such as dysphoria, diminished interest, worthlessness or guilt, fatigue, psychomotor retardation, or thoughts of death) are present nearly every day during the episode. Mixed states carry elevated suicide risk, so counselors should assess safety closely. The specifier can also apply to major depressive episodes with concurrent manic or hypomanic symptoms.

References: DSM-5-TR — Bipolar and Related Disorders specifiers: with mixed features (manic or hypomanic episode)

☐ Approve ☐ Revise ☐ Retire Notes:

A 29-year-old has, for the past 7 months, felt intense dread whenever his spouse travels, called her many times a day to check that she is safe, and refused overnight work trips. He worries mainly that an accident will take her from him, not about many topics. Which diagnosis BEST fits?

A. Generalized anxiety disorder, since his worry has lasted over six months

GAD involves worry about many events or activities; his fear centers on separation from one attachment figure.

B. Separation anxiety disorder, since adult duration is typically 6 months ✓ **KEY**

Excessive fear of harm befalling an attachment figure, with reluctance to be apart, lasting typically 6+ months in adults fits.

C. Agoraphobia, since he avoids travel and being away from home overnight

Agoraphobic avoidance stems from fear that escape or help will be unavailable, not fear of losing a loved one.

D. No anxiety disorder, as separation anxiety is diagnosed only in childhood

DSM-5-TR permits separation anxiety disorder at any age; onset in adulthood is recognized.

Rationale: Separation anxiety disorder is developmentally inappropriate, excessive fear or anxiety about separation from attachment figures. The duration is at least 4 weeks in children and adolescents and typically 6 months or more in adults. DSM-5-TR removed the earlier requirement of onset before age 18, so adults whose fears center on harm to a partner or child may meet criteria.

References: DSM-5-TR — Separation Anxiety Disorder: Criteria A and B (duration in children vs adults)

☐ Approve ☐ Revise ☐ Retire Notes:

For 7 months, a 10-year-old has worried more days than not about grades, storms, her family's money and being late. She finds the worry hard to control and has trouble sleeping, but no other physical or associated symptoms. How many associated symptoms does DSM-5-TR require for a child to meet GAD criteria?

A. Three of the six associated symptoms, the same number used for adults

Three symptoms are required for adults; DSM-5-TR lowers the threshold for children.

B. Two of the six associated symptoms, one physical and one cognitive

DSM-5-TR sets no two-symptom rule and does not require a physical and cognitive mix.

C. Only one of the six associated symptoms, such as disturbed sleep ✓ **KEY**

For children, only one of the six associated symptoms is required, so her sleep problem can satisfy the criterion.

D. Four of the six associated symptoms, as worry in youth is so common

No criterion raises the threshold for youth; the requirement is lower for children, not higher.

Rationale: Generalized anxiety disorder requires excessive, hard-to-control worry about a number of events or activities on more days than not for at least 6 months. Adults need three or more of six associated symptoms (restlessness, fatigue, poor concentration, irritability, muscle tension, sleep disturbance), but children need only one. Children with GAD often worry about school performance, catastrophes and punctuality and may seek frequent reassurance.

References: DSM-5-TR — Generalized Anxiety Disorder: Criterion C (note: only one item required in children)

☐ Approve ☐ Revise ☐ Retire Notes:

A 3-year-old spent his first 2 years in a series of neglectful placements. Now with stable foster parents, he rarely seeks comfort when hurt and barely responds when they offer it. He shows little positive emotion and has unexplained bouts of fearfulness. He uses language and play typical for his age. Which diagnosis is MOST likely?

A. Reactive attachment disorder linked to his early social neglect ✓ KEY

Rarely seeking or responding to comfort, limited positive affect and unexplained fearfulness after neglect fit RAD.

B. Disinhibited social engagement disorder from his early care setting

DSED involves overly familiar approach to unfamiliar adults, not withdrawal and failure to seek comfort.

C. Autism spectrum disorder based on his limited social response

His age-typical language and play and the clear history of neglect point away from autism and toward RAD.

D. Posttraumatic stress disorder for children six years and under

No intrusion or avoidance tied to a specific traumatic event is described; the core issue is absent attachment behavior.

Rationale: Reactive attachment disorder is a consistent pattern of inhibited, emotionally withdrawn behavior toward caregivers: the child rarely seeks or responds to comfort when distressed, with limited positive affect and unexplained irritability, sadness or fear. It follows extremes of insufficient care, appears before age 5, and requires a developmental age of at least 9 months. Unlike autism spectrum disorder, RAD does not involve restricted, repetitive behaviors, and language and play are not specifically impaired.

References: DSM-5-TR — Reactive Attachment Disorder: Criteria A-G; differential diagnosis with autism spectrum disorder

☐ Approve ☐ Revise ☐ Retire Notes:

A 51-year-old reports intrusive images, nightmares and avoidance since learning by phone that her brother died. Which detail would determine whether the death can meet PTSD Criterion A through learning about it?

A. Whether she had a close, loving relationship with her brother for years

A close relationship is needed, but closeness alone does not settle whether the event qualifies.

B. Whether she learned of the death within a day after it happened

DSM-5-TR sets no time window for learning about the event in Criterion A.

C. Whether her symptoms began within the first month after the death

Symptom timing bears on the delayed-expression specifier, not on whether the event qualifies under Criterion A.

D. Whether the death was violent or accidental rather than from illness ✓ KEY

When learning of a close other's actual or threatened death, the event must have been violent or accidental.

Rationale: PTSD Criterion A can be met by learning that a traumatic event happened to a close family member or close friend. When the event involves actual or threatened death, it must have been violent or accidental; a death from natural causes such as illness does not qualify. Intense distress after an expected or natural death may instead be assessed as grief, prolonged grief disorder, adjustment disorder or a depressive disorder.

References: DSM-5-TR — Posttraumatic Stress Disorder: Criterion A3 (learning of violent or accidental death)

☐ Approve ☐ Revise ☐ Retire Notes:

A 41-year-old attorney insists that every memo follow her exact formatting rules, redoes her team's work, and works so many hours she has no hobbies. She sees her standards as correct and is puzzled that others complain. She has no intrusive, unwanted thoughts or rituals. Which feature MOST points to obsessive-compulsive personality disorder rather than OCD?

A. Her rigid perfectionism feels right to her and has no true obsessions ✓ **KEY**

OCPD is an ego-syntonic, pervasive perfectionism without the intrusive obsessions and compulsions of OCD.

B. Her drive for order is so strong that it interferes with her work life

Impairment occurs in both OCD and OCPD, so it does not distinguish the two disorders.

C. She spends more than one hour a day on her work and her formatting

The one-hour marker is an OCD time-consuming example, and long work hours alone do not separate the disorders.

D. She is perfectionistic only at work and is relaxed with her friends

A personality disorder is pervasive across contexts; a work-limited pattern would argue against OCPD.

Rationale: Obsessive-compulsive personality disorder is a pervasive pattern of preoccupation with orderliness, perfectionism and control that the person usually experiences as reasonable (ego-syntonic). OCD is defined by intrusive, unwanted obsessions and compulsions performed to reduce distress. When both sets of criteria are met, both can be diagnosed, but the absence of true obsessions and compulsions points to OCPD.

References: DSM-5-TR — Obsessive-Compulsive Personality Disorder: differential diagnosis with obsessive-compulsive disorder

☐ Approve ☐ Revise ☐ Retire Notes:

A 19-year-old pulls out her eyebrow hairs while studying, leaving visible bare patches. She feels relief as she pulls, has tried many times to stop, and is embarrassed by how her face looks. She has no concerns about a flaw in her appearance before pulling. Which diagnosis BEST fits?

A. Body dysmorphic disorder, since she is upset about how her face looks

In BDD, hair removal aims to fix a perceived defect; her distress follows the pulling rather than driving it.

B. Excoriation disorder, since she has repeatedly tried to stop pulling

Excoriation disorder involves recurrent skin picking that causes lesions, not hair pulling.

C. Trichotillomania, given hair loss from pulling and attempts to stop ✓ **KEY**

Recurrent hair pulling causing hair loss, with repeated efforts to stop and distress, meets trichotillomania criteria.

D. Obsessive-compulsive disorder, since pulling eases her inner tension

Her pulling is not driven by obsessions or rules, so a body-focused repetitive behavior disorder fits better.

Rationale: Trichotillomania (hair-pulling disorder) requires recurrent pulling out of one's hair resulting in hair loss, repeated attempts to decrease or stop, and clinically significant distress or impairment. Pulling is often preceded by tension or boredom and followed by relief or gratification. It is distinguished from BDD, where hair is removed to correct a perceived appearance flaw, and from OCD, where behavior responds to obsessions.

References: DSM-5-TR — Trichotillomania (Hair-Pulling Disorder): Criteria A-E and differential diagnosis

☐ Approve ☐ Revise ☐ Retire Notes:

A 27-year-old who smoked cannabis several times a day for two years stopped 4 days ago. She reports irritability, anxiety, trouble sleeping with vivid dreams, poor appetite and restlessness. She says, "Weed isn't addictive, so this must be something else." How should the counselor understand her symptoms?

A. As a new anxiety disorder, because cannabis has no withdrawal state

DSM-5-TR includes cannabis withdrawal; attributing her symptoms to a new disorder ignores the clear timing.

B. As likely cannabis withdrawal, which DSM-5-TR recognizes in heavy use ✓ **KEY**

Three or more of these symptoms within about a week of stopping heavy, prolonged use meets cannabis withdrawal criteria.

C. As rebound mood symptoms, which only appear in people with depression

Withdrawal symptoms occur in people without depression; they reflect physiological adaptation to heavy use.

D. As symptoms that cannot be judged until she has two weeks of abstinence

Withdrawal is identified by symptoms that start within about one week; there is no two-week waiting rule.

Rationale: DSM-5-TR includes cannabis withdrawal, which follows cessation of heavy and prolonged use (daily or almost daily over at least a few months). Three or more symptoms develop within about one week, such as irritability or anger, anxiety, sleep difficulty or disturbing dreams, decreased appetite, restlessness, depressed mood or physical discomfort. Recognizing withdrawal helps counselors normalize symptoms and plan relapse prevention.

References: DSM-5-TR — Cannabis Withdrawal: Criteria A-B

☐ Approve ☐ Revise ☐ Retire Notes:

A counseling intern drafts a report stating that a client who drinks six energy drinks daily has "caffeine use disorder, moderate." Her supervisor asks her to check DSM-5-TR. Which correction is accurate?

A. Caffeine use disorder exists but is only coded as mild in severity

DSM-5-TR has no official caffeine use disorder diagnosis, so no severity levels apply.

B. Caffeine use disorder is coded only if withdrawal has been observed

There is no official caffeine use disorder to code; observed withdrawal is a separate diagnosis.

C. Caffeine problems are coded as an other specified stimulant use disorder

Caffeine is not classed with stimulants such as amphetamine and cocaine for use disorder coding.

D. Caffeine use disorder is listed only as a condition for further study ✓ **KEY**

DSM-5-TR places caffeine use disorder in Section III, though caffeine intoxication and withdrawal are diagnosable.

Rationale: In DSM-5-TR, caffeine intoxication and caffeine withdrawal are official diagnoses in the substance-related chapter, but caffeine use disorder appears only among the Conditions for Further Study in Section III. Counselors should not assign a diagnosis that is proposed for research rather than clinical use. Caffeine withdrawal, for example, involves headache plus symptoms such as fatigue, dysphoric mood, poor concentration or flu-like symptoms after abrupt cessation.

References: DSM-5-TR — Section III Conditions for Further Study: Caffeine Use Disorder; Caffeine-Related Disorders

☐ Approve ☐ Revise ☐ Retire Notes:

For the past 5 months, a 7-year-old has had rapid eye blinking, facial grimacing and occasional grunting sounds that come and go. A medical cause and substance effects have been ruled out. Which diagnosis is MOST appropriate now?

A. Tourette's disorder, as he has both motor tics and a vocal tic

Tourette's disorder requires tics persisting more than 1 year since first onset; his tics began 5 months ago.

B. Persistent motor or vocal tic disorder, since tics recur daily

Persistent tic disorder needs more than 1 year of tics and either motor or vocal tics, not both.

C. Provisional tic disorder, as tics have lasted under one year ✓ KEY

Motor and/or vocal tics present for less than 1 year since first onset, beginning before age 18, fit provisional tic disorder.

D. Stereotypic movement disorder, as the movements are repetitive

Stereotypies are rhythmic, often soothing movements starting earlier in childhood; sudden blinks and grunts are tics.

Rationale: Tic disorders are distinguished by tic type and duration. Provisional tic disorder covers single or multiple motor and/or vocal tics present for less than 1 year since first onset, beginning before age 18. Tourette's disorder requires both multiple motor tics and at least one vocal tic for more than 1 year, and persistent (chronic) tic disorder requires only motor or only vocal tics for more than 1 year.

References: DSM-5-TR — Tic Disorders: Provisional Tic Disorder Criteria A-D; comparison with Tourette's and persistent tic disorder

☐ Approve ☐ Revise ☐ Retire Notes:

A 7-year-old with average intelligence and normal vision is far behind peers in catching a ball, using scissors and writing legibly. His teachers say the clumsiness has been present since he was a toddler, and no neurological condition has been found. Which diagnosis BEST fits?

A. Developmental coordination disorder affecting his motor skills ✓ KEY

Motor skills well below age expectations, starting early and not due to intellectual, visual or neurological causes, fit DCD.

B. Specific learning disorder with impairment in written expression

His handwriting problem is part of a broader motor deficit, not a deficit in spelling, grammar or written organization.

C. Mild intellectual developmental disorder with motor slowness

He has average intelligence, so intellectual developmental disorder does not explain the motor deficits.

D. Attention-deficit/hyperactivity disorder, combined presentation

Clumsiness can co-occur with ADHD, but no inattention or hyperactivity symptoms are described.

Rationale: Developmental coordination disorder involves acquisition and execution of coordinated motor skills substantially below age expectations, interfering with daily life and school, with onset in the early developmental period. The deficits are not better explained by intellectual disability, visual impairment or a neurological condition. Because early motor development varies widely, DSM-5-TR notes that DCD is typically not diagnosed before age 5.

References: DSM-5-TR — Developmental Coordination Disorder: Criteria A-D and development and course

☐ Approve ☐ Revise ☐ Retire Notes:

For 8 months, a 25-year-old has shown marked avolition, flat affect and grossly disorganized behavior. A careful evaluation finds no delusions, hallucinations or disorganized speech at any point. Why does this presentation fail schizophrenia Criterion A?

A. Negative symptoms count toward Criterion A only after the first year

Negative symptoms can count toward Criterion A at any time; there is no one-year waiting rule.

B. One of the two must be delusions, hallucinations or disorganized speech ✓ **KEY**

Criterion A requires two of five symptoms, and at least one must be delusions, hallucinations or disorganized speech.

C. Criterion A requires three or more of the five characteristic symptom groups

Only two of the five symptoms are needed; the problem is which symptoms are present, not how many.

D. Disorganized behavior is counted as a negative symptom, not a positive one

Grossly disorganized or catatonic behavior is its own Criterion A item and is not a negative symptom.

Rationale: Schizophrenia Criterion A requires two or more of five symptoms (delusions, hallucinations, disorganized speech, grossly disorganized or catatonic behavior, negative symptoms), each present for a significant portion of a 1-month period. At least one must be delusions, hallucinations or disorganized speech. A presentation limited to negative symptoms and disorganized behavior calls for further assessment of other causes, such as depression, catatonia or a medical condition.

References: DSM-5-TR — Schizophrenia: Criterion A (at least one of items 1-3 required)

☐ Approve ☐ Revise ☐ Retire Notes:

A community educator preparing a talk for parents asks a counselor when schizophrenia most often first appears. Which statement BEST matches DSM-5-TR?

A. Most cases first begin in middle childhood, before puberty starts

Onset before adolescence is rare; childhood-onset schizophrenia is the exception, not the rule.

B. Onset peaks in the mid-40s, with similar ages for men and for women

Peak onset is in the 20s, and the timing differs by gender, with men tending to be earlier.

C. Onset typically occurs after age 60, mainly in women who live alone

Late-life onset is uncommon; new psychosis in older adults calls for a search for medical causes.

D. It usually starts in late teens to mid-30s, often earlier in men ✓ **KEY**

Psychotic features typically emerge between the late teens and mid-30s, peaking in early-to-mid 20s for men, late 20s for women.

Rationale: DSM-5-TR describes typical onset of schizophrenia between the late teens and the mid-30s, with onset before adolescence rare. Peak age at onset is in the early-to-mid 20s for males and the late 20s for females. Many people show a prodromal phase of social withdrawal and declining functioning before psychotic symptoms appear, which is why early detection in adolescence and young adulthood matters.

References: DSM-5-TR — Schizophrenia: Development and Course (age at onset and gender differences)

☐ Approve ☐ Revise ☐ Retire Notes:

A 38-year-old lab technician has always lived alone and spends free time on solitary hobbies. He says he has no wish for close relationships, gets little pleasure from social activity, and is unbothered by praise or criticism. He came to counseling only at his sister's urging. Which diagnosis is MOST consistent?

A. Avoidant personality disorder, given his long pattern of isolation

People with avoidant PD want relationships but avoid them from fear of rejection; he has no such desire.

B. Schizotypal personality disorder, as he is distant from other people

Schizotypal PD adds odd beliefs, perceptual distortions and eccentric behavior, none of which are described.

C. Social anxiety disorder, since he limits his social contact so much

Social anxiety centers on fear of scrutiny; he reports indifference to others, not fear of judgment.

D. Schizoid personality disorder, given his indifference to closeness ✓ KEY

Neither desiring nor enjoying close relationships, choosing solitary activities and indifference to praise fit schizoid PD.

Rationale: Schizoid personality disorder is a pervasive pattern of detachment from social relationships and restricted emotional expression, with little desire for closeness and indifference to praise or criticism. The key differential is motivation: people with avoidant personality disorder long for connection but withdraw out of fear of rejection and inadequacy. Schizotypal personality disorder adds cognitive and perceptual distortions and eccentricity.

References: DSM-5-TR — Schizoid Personality Disorder: Criterion A and differential diagnosis with avoidant and schizotypal PD

☐ Approve ☐ Revise ☐ Retire Notes:

A 16-year-old has shown chronic emptiness, unstable relationships, frantic efforts to avoid abandonment and impulsive self-harm across home, school and peer settings for 18 months. A colleague says personality disorders "cannot be diagnosed before 18." What does DSM-5-TR state?

A. No personality disorder at all may be diagnosed until the person turns 18

This overstates the rule; only antisocial personality disorder carries an age-18 minimum.

B. Most may apply under 18 if traits are pervasive and last at least 1 year ✓ KEY

Personality disorders other than antisocial PD may be diagnosed in minors when features have lasted at least 1 year.

C. Borderline personality disorder alone may be diagnosed before age 18

The exception is not limited to borderline PD; other personality disorders may also be diagnosed in minors.

D. Personality disorders may apply under 18 after traits last 6 months

DSM-5-TR requires features to be present for at least 1 year, not 6 months, in individuals under 18.

Rationale: DSM-5-TR allows personality disorder diagnoses in children and adolescents when the maladaptive traits are pervasive, persistent and unlikely to be limited to a developmental stage or another mental disorder. Features must have been present for at least 1 year in someone under 18. Antisocial personality disorder is the exception: it requires age 18 or older, with conduct disorder evident before age 15.

References: DSM-5-TR — Personality Disorders: General Personality Disorder, development and course (diagnosis under age 18)

☐ Approve ☐ Revise ☐ Retire Notes:

An adult client meets criteria for anorexia nervosa, and her physician reports a current body mass index (BMI) of 16.4. Using DSM-5-TR severity ranges for adults, which specifier applies?

A. Moderate, since her BMI falls within 16 to 16.99 ✓ KEY

DSM-5-TR rates a BMI of 16 to 16.99 as moderate severity in adults.

B. Mild, since her BMI is still above the level of 16

Mild severity is a BMI of 17 or higher; 16.4 falls in the moderate range.

C. Severe, since her BMI is under the level of 17.5

Severe severity is a BMI of 15 to 15.99; 17.5 is not a DSM-5-TR cutoff.

D. Extreme, as any BMI under 18.5 is called extreme

Extreme severity is a BMI under 15; 18.5 marks the general underweight threshold.

Rationale: DSM-5-TR rates current anorexia nervosa severity in adults by BMI: mild is 17 or higher, moderate 16 to 16.99, severe 15 to 15.99, and extreme under 15. For children and adolescents, BMI percentiles are used instead. Severity may be raised to reflect clinical symptoms, functional disability and the need for supervision.

References: DSM-5-TR — Anorexia Nervosa: severity specifiers based on BMI

☐ Approve ☐ Revise ☐ Retire Notes:

Parents worry that their 18-month-old puts sand and paper in her mouth at the park. Their neighbor's 6-year-old has eaten paint chips and chalk daily for 2 months, which is not part of any family or cultural practice. Which conclusion about pica is MOST accurate?

A. Both children meet criteria, as each eats things that are not food

Mouthing objects is developmentally normal in toddlers; pica requires eating that is inappropriate for development.

B. Neither child can be diagnosed, as pica is a diagnosis for adults

Pica is diagnosed at any age, and it is often seen in childhood.

C. The 6-year-old may meet criteria; the toddler's mouthing is typical ✓ KEY

Pica requires developmentally inappropriate eating of nonfood items for at least 1 month; a minimum age of 2 is suggested.

D. Only the toddler meets criteria, as such symptoms in older children fade

The toddler's behavior is developmentally expected, while the older child's persistent eating is not.

Rationale: Pica is persistent eating of nonnutritive, nonfood substances for at least 1 month that is inappropriate to the person's developmental level and not part of a culturally supported or socially normative practice. DSM-5-TR suggests a minimum age of 2 years to exclude normal mouthing of objects by infants and toddlers. Paint chips also raise concern for lead exposure, so medical evaluation is warranted.

References: DSM-5-TR — Pica: Criteria A-D and diagnostic features (minimum age of 2 years)

☐ Approve ☐ Revise ☐ Retire Notes:

A 74-year-old was diagnosed with Parkinson's disease 6 years ago after developing a tremor and slowed movement. Over the past year, he has developed gradual problems with attention, planning and memory that now interfere with paying bills. Which diagnosis BEST fits?

A. Neurocognitive disorder with Lewy bodies, since he has parkinsonism

In Lewy body NCD, cognitive decline comes before or within about a year of motor signs, not years later.

B. Major frontotemporal neurocognitive disorder, given his poor planning

Frontotemporal NCD features early behavioral or language change, not decline years after established Parkinson's.

C. Major neurocognitive disorder due to Parkinson's disease ✓ **KEY**

Decline that develops gradually in the setting of long-established Parkinson's disease fits NCD due to Parkinson's.

D. Delirium caused by his Parkinson's medications and motor symptoms

Delirium has an acute onset with fluctuating attention; his decline has been gradual over a year.

Rationale: Neurocognitive disorder due to Parkinson's disease is diagnosed when cognitive decline arises insidiously in the setting of established Parkinson's disease. When cognitive decline begins before, or within about one year of, the onset of parkinsonian motor signs, NCD with Lewy bodies is more likely. The timing of motor versus cognitive symptoms is therefore the key differential clue.

References: DSM-5-TR — Major or Mild Neurocognitive Disorder Due to Parkinson's Disease: criteria and differential diagnosis with Lewy bodies

☐ Approve ☐ Revise ☐ Retire Notes:

A 34-year-old construction worker lost consciousness briefly after a fall from scaffolding 5 months ago. Since regaining consciousness, he has had persistent problems with attention, slowed thinking and memory, and he struggles to follow work instructions. Which diagnosis should the counselor consider FIRST?

A. Neurocognitive disorder due to traumatic brain injury ✓ **KEY**

Deficits beginning right after a head injury with loss of consciousness and persisting past the acute period fit NCD due to TBI.

B. Adjustment disorder with anxiety following his work fall

Adjustment disorder involves emotional or behavioral distress, not objective cognitive deficits after head injury.

C. Major depressive disorder causing his memory complaints

Depression can impair concentration, but no mood symptoms are described and a head injury explains the onset.

D. Mild neurocognitive disorder due to Alzheimer's disease

Alzheimer's disease shows insidious, gradual onset, usually in late life, not sudden deficits after an injury.

Rationale: Neurocognitive disorder due to traumatic brain injury requires evidence of a TBI with loss of consciousness, posttraumatic amnesia, disorientation or confusion, or neurological signs. The cognitive disorder must present immediately after the injury or after recovery of consciousness and persist past the acute post-injury period. Counselors should coordinate with medical providers for neuropsychological evaluation.

References: DSM-5-TR — Major or Mild Neurocognitive Disorder Due to Traumatic Brain Injury: Criteria A-C

☐ Approve ☐ Revise ☐ Retire Notes:

A 21-year-old falls asleep in class and at meals several times a day despite sleeping 8 hours a night, a pattern lasting 5 months. When she laughs hard, her knees suddenly buckle and her jaw goes slack for a few seconds, though she stays fully aware. Which condition do these features MOST suggest?

A. Hypersomnolence disorder with long sleep and daytime naps

Hypersomnolence disorder involves excessive sleepiness but not cataplexy triggered by laughter.

B. Narcolepsy, with episodes of cataplexy set off by laughing ✓ KEY

Irresistible sleep episodes plus brief loss of muscle tone with laughter, consciousness intact, point to narcolepsy.

C. Panic disorder, with sudden weak spells during strong emotion

Panic attacks involve surges of fear with autonomic symptoms, not laughter-triggered loss of muscle tone.

D. A seizure disorder with brief absences when she gets excited

Absence seizures impair awareness; in cataplexy the person remains conscious throughout.

Rationale: Narcolepsy involves recurrent periods of irrepressible need to sleep, lapsing into sleep or napping within the same day, at least three times per week for 3 months. It also requires cataplexy, hypocretin deficiency, or characteristic REM findings on sleep studies. Cataplexy is brief, sudden bilateral loss of muscle tone with preserved consciousness, often set off by laughter or joking, and should prompt referral to a sleep specialist.

References: DSM-5-TR — Narcolepsy: Criteria A-B (cataplexy)

☐ Approve ☐ Revise ☐ Retire Notes:

Parents say their 6-year-old sits up screaming about an hour after falling asleep several nights a month. He is sweaty, wide-eyed and hard to comfort, then settles back to sleep. In the morning he remembers nothing. Which explanation BEST fits?

A. Nightmare disorder, since he wakes up afraid during the night

Nightmares occur later in the night in REM sleep, and the child wakes fully and recalls the dream.

B. Panic attacks at night, since he shows sweating and great fear

Nocturnal panic involves waking fully alert from sleep, not being unresponsive with amnesia.

C. Separation anxiety disorder, as he becomes upset at bedtime

He is not upset at bedtime about separation; the episodes arise from deep sleep.

D. Sleep terrors, a non-REM arousal pattern common in children ✓ KEY

Screaming arousals early in the night with poor response to comfort and no recall fit sleep terrors.

Rationale: Sleep terrors are a type of non-rapid eye movement sleep arousal disorder: abrupt terrified arousals, usually in the first third of the night, often beginning with a scream, with autonomic arousal, poor response to comfort, and little or no recall. They are most common in childhood and usually decrease with age. Nightmares, by contrast, occur in REM sleep later in the night, and the person wakes fully and remembers the dream.

References: DSM-5-TR — Non-Rapid Eye Movement Sleep Arousal Disorders: sleep terror type; differential with nightmare disorder

☐ Approve ☐ Revise ☐ Retire Notes:

A 48-year-old in counseling for insomnia says that most evenings when she sits or lies down she feels a crawling, uncomfortable urge to move her legs. Walking around eases it, and it is far worse at night than in the morning. This has happened 4 nights a week for 6 months. Which condition should the counselor suspect?

A. Restless legs syndrome, given the evening urge eased by moving ✓ KEY

An urge to move the legs that starts with rest, is relieved by movement and is worse at night defines restless legs syndrome.

B. Generalized anxiety disorder with restlessness during the day

GAD restlessness is tied to worry, not leg sensations that follow rest and are worst at night.

C. Akathisia from a medication, since she feels inner restlessness

Akathisia follows a medication start or change and lacks the clear evening pattern; none is described.

D. Insomnia disorder alone, since her leg discomfort is from fatigue

Insomnia is not diagnosed alone when another sleep-wake disorder, here RLS, better explains the sleep problem.

Rationale: Restless legs syndrome is an urge to move the legs, usually with uncomfortable sensations, that begins or worsens during rest, is partly or fully relieved by movement, and is worse in the evening or night. Symptoms occur at least three times per week for at least 3 months and cause distress or impairment. Because RLS often presents as trouble falling asleep, counselors should ask about leg sensations and refer for medical evaluation.

References: DSM-5-TR — Restless Legs Syndrome: Criteria A-C

☐ Approve ☐ Revise ☐ Retire Notes:

A 22-year-old who burns herself when overwhelmed also reports passive thoughts of death. Her previous counselor had her sign a "no-harm contract" promising not to hurt herself. She asks whether they should sign one again. Which response BEST reflects current practice?

A. Sign a new contract, since a written promise reliably lowers her risk

No-harm contracts lack evidence of reducing self-harm or suicide and can discourage honest disclosure.

B. Sign one only if she agrees to call the counselor before each urge

Adding conditions does not fix the contract's weak evidence, and it shifts focus away from building her own coping.

C. Build a written safety plan with her triggers, coping and contacts ✓ KEY

Collaborative safety planning names warning signs, coping steps, supports and crisis resources and has research support.

D. Skip planning for now, as her self-harm is not suicidal in its intent

Self-injury plus thoughts of death calls for active risk management, not delay.

Rationale: No-harm or no-suicide contracts are not supported by evidence and may give a false sense of security. A collaborative safety plan, such as the Stanley-Brown Safety Planning Intervention, lists warning signs, internal coping strategies, social contacts and settings for distraction, people to ask for help, professional and crisis contacts, and steps to make the environment safer. Because NSSI raises later suicide risk, ongoing risk assessment remains part of care.

References: Stanley-Brown SPI — Safety Planning Intervention components; contrast with no-suicide contracts

☐ Approve ☐ Revise ☐ Retire Notes:

A counselor reviewing an intake form sees that a 20-year-old has several piercings, a large tattoo and ceremonial scarring from a coming-of-age ritual in her community. She denies any other injury to her body. How should the counselor classify these behaviors?

A. As nonsuicidal self-injury, since each one caused deliberate tissue harm

The proposed NSSI criteria exclude socially sanctioned behaviors such as piercing, tattooing and ritual scarring.

B. As socially sanctioned practices that the NSSI criteria exclude ✓ **KEY**

Body modification and culturally sanctioned rituals are excluded from nonsuicidal self-injury.

C. As NSSI only if she reports feeling relief after each of the procedures

Relief does not convert a socially sanctioned practice into NSSI; the exclusion applies regardless.

D. As a warning sign that requires a full suicide risk assessment today

Cultural and cosmetic body modification is not itself a suicide warning sign; routine screening suffices.

Rationale: The DSM-5-TR proposed criteria for nonsuicidal self-injury disorder state that the behavior is not socially sanctioned, such as body piercing, tattooing or part of a religious or cultural ritual, and is not restricted to picking a scab or nail biting. Counselors assess the function and context of body changes with cultural humility rather than labeling all tissue damage as self-injury.

References: DSM-5-TR — Conditions for Further Study: Nonsuicidal Self-Injury Disorder, Criterion E (socially sanctioned behavior excluded)

☐ Approve ☐ Revise ☐ Retire Notes:

A school district asks its counselors when to begin prevention programming about nonsuicidal self-injury. According to DSM-5-TR, at what point in development does NSSI MOST often begin?

A. In early childhood, before children start attending school

Self-injury in very young children is uncommon and often linked to developmental conditions, not typical NSSI.

B. In middle adulthood, often after job or relationship losses

Onset in midlife is uncommon; NSSI most often starts much earlier.

C. In later adulthood, often alongside chronic medical illness

Older adults who self-harm are a high-risk group, but late life is not the typical onset period.

D. In the early teen years, and it can go on for many years ✓ **KEY**

DSM-5-TR notes NSSI most often begins in the early teen years and may continue for many years.

Rationale: DSM-5-TR text on nonsuicidal self-injury notes that it most often starts in the early teen years and can continue for many years. Hospital admissions for NSSI peak around ages 20 to 29 before declining. School-based prevention and staff training in the middle-school years therefore target the developmental window when NSSI typically begins.

References: DSM-5-TR — Conditions for Further Study: Nonsuicidal Self-Injury Disorder, Development and Course

☐ Approve ☐ Revise ☐ Retire Notes:

A 17-year-old began an antidepressant prescribed by his pediatrician 2 weeks ago. His mother asks the counselor what the FDA boxed warning on the medication means for her son. Which response is MOST accurate?

A. Watch for new or worse suicidal thinking, mainly early in treatment ✓ KEY

The boxed warning notes increased suicidal thoughts and behaviors in youth, calling for close monitoring early on.

B. It means antidepressants cannot be given to anyone under age eighteen

The warning does not ban use in youth; some antidepressants are approved for adolescents with monitoring.

C. It is about weight gain, so his diet should be checked each week

The antidepressant boxed warning concerns suicidality in young people, not weight change.

D. It is about risk only for adults over 65, so it does not apply to him

The warning applies to children, adolescents and young adults under 25, not older adults.

Rationale: The FDA boxed warning on antidepressants states that, in short-term studies, they increased the risk of suicidal thinking and behavior in children, adolescents and young adults under age 25. It calls for close monitoring for worsening mood, agitation or suicidality, especially in the first months of treatment and after dose changes. Counselors educate families, encourage communication with the prescriber and monitor risk.

References: APA CPG — Depression across the lifespan: antidepressants in adolescents and suicidality monitoring · FDA Antidepressant Boxed Warning — 2007 labeling revision: suicidality warning extended through age 24; close monitoring early in treatment

☐ Approve ☐ Revise ☐ Retire Notes:

A client who started a new antipsychotic last week arrives with a high fever, heavy sweating, very stiff muscles, a racing heart and confusion. Which action should the counselor take FIRST?

A. Help the client cut the dose in half until a prescriber can be reached

Counselors do not adjust medication, and these signs need emergency care, not a dose change.

B. Arrange emergency medical care for possible neuroleptic malignant syndrome ✓ KEY

Fever, rigidity, autonomic instability and confusion after antipsychotic start suggest NMS, a medical emergency.

C. Treat the confusion as psychosis and schedule a session for later in the week

Fever, rigidity and confusion point to a medical crisis; delay could be fatal.

D. Suggest rest and fluids for a likely viral illness and check in tomorrow

Muscle rigidity with confusion after a new antipsychotic is not a routine illness and needs urgent evaluation.

Rationale: Neuroleptic malignant syndrome is a rare, potentially fatal reaction to dopamine-blocking agents, most often antipsychotics. Hallmarks are high fever, severe muscle rigidity, altered mental status and autonomic instability (sweating, fast heart rate, unstable blood pressure), usually within days of starting or raising the dose. Counselors recognize it as a medical emergency and arrange immediate care.

References: DSM-5-TR — Medication-Induced Movement Disorders and Other Adverse Effects of Medication: Neuroleptic Malignant Syndrome

☐ Approve ☐ Revise ☐ Retire Notes:

A 41-year-old whose brother died by suicide five months ago says neighbors avoid the subject, she keeps asking "why" and replaying their last call, and she blames herself for missing signs. Using Worden's mediators of mourning, which mediator MOST shapes the features of her grief?

A. Historical antecedents, since her earlier losses predict how she copes with this one

Prior losses matter, but nothing in the case points to earlier losses as the source of her guilt and searching.

B. Personality variables, since her age and coping style decide the course of grief

Personality shapes all grief, but it does not explain the stigma, "why" questions and self-blame tied to this death.

C. How the person died, since suicide loss often brings stigma, guilt and searching ✓ **KEY**

Mode of death is a core mediator; suicide loss is marked by "why" questions, guilt, blame and social stigma.

D. Concurrent stresses, since other demands in her life slow her work on the tasks

No competing stressors are described; her symptoms track the nature of the death itself.

Rationale: Worden lists mediators that explain why grief differs between people, including who the person was, the nature of the attachment, how the person died, historical antecedents, personality, social mediators and concurrent stresses. Deaths by suicide commonly leave survivors with intense searching for reasons, guilt, blame and stigma that limits social support. Counselors address these features directly and often link survivors with suicide-loss support groups.

References: Worden (Grief Counseling) — Mediators of mourning: how the person died; grief after suicide

☐ Approve ☐ Revise ☐ Retire Notes:

A father whose wife died suddenly asks whether his 8-year-old son should attend her funeral. The boy has asked to go, but the father worries it will be "too much" for him. What should the counselor recommend?

A. Prepare the boy for what he will see and hear, then let him choose to attend ✓ **KEY**

Children who are prepared and given a choice usually benefit from taking part in family mourning rituals.

B. Keep him home with a sitter so that his memories of his mother stay happy ones

Excluding a child who wants to attend can leave him feeling shut out of the family's grief.

C. Bring him to the service without discussing it so he does not worry ahead

Attending without preparation can be confusing and frightening; explanation beforehand is key.

D. Wait to decide on funerals until he reaches age ten, when grief is clearer

There is no age cutoff; an 8-year-old can understand death as final and can take part when prepared.

Rationale: Research on bereaved children, including the Harvard Child Bereavement Study that Worden directed, supports including children in funerals and other rituals when they are prepared and allowed to choose. Preparation means explaining in concrete terms what will happen, who will be there and how people may act. Taking part helps children accept the reality of the loss and feel included in the family's mourning.

References: Worden (Grief Counseling) — Grieving children: Harvard Child Bereavement Study; children and funerals

☐ Approve ☐ Revise ☐ Retire Notes:

A 35-year-old comes in the day after being robbed at gunpoint. She cannot sleep and is afraid to leave home. The counselor notices her parents' divorce comes up and is tempted to explore how it shaped her. What is the PRIMARY goal of crisis intervention at this point?

A. Uncover how her childhood losses set the stage for her current reaction

Exploring early history is a longer-term therapy goal and can destabilize a client in acute crisis.

B. Reach a full diagnosis of her symptoms before offering any form of help

Crisis work assesses and helps at once; a full diagnostic workup is not a precondition for support.

C. Change the personality traits that left her vulnerable to the robbery

Personality change is not a crisis goal, and the framing blames the client for being a victim.

D. Restore her to at least her pre-crisis level of functioning and safety ✓ KEY

Crisis intervention is brief and focused on safety, coping and a return to prior functioning.

Rationale: Crisis intervention is short-term, present-focused and directive as needed. Its aim is to ensure safety, reduce acute distress and help the client regain at least the level of functioning she had before the crisis. Deeper exploration of developmental history belongs to later counseling, once the client has stabilized.

References: James & Gilliland (Crisis Intervention Strategies) — Goals of crisis intervention: restoring equilibrium and pre-crisis functioning

☐ Approve ☐ Revise ☐ Retire Notes:

A hurricane floods an entire coastal county. Schools, clinics and businesses close for weeks, and thousands of residents lose homes and jobs at once. In James and Gilliland's classification of crises, which type does this event BEST represent?

A. A developmental crisis, arising from a normal change in the course of life

Developmental crises arise from expected life changes such as a birth or retirement, not natural disasters.

B. An ecosystemic crisis, a disaster that strikes a whole community at once ✓ KEY

Ecosystemic crises are natural or human-caused disasters that overwhelm people across a community or region.

C. An existential crisis, rooted in inner conflict over purpose and meaning

Existential crises center on inner questions of purpose and freedom, not a shared external catastrophe.

D. A situational crisis, marked by a sudden event within one person's life

Situational crises are sudden events for an individual or family; a regional disaster goes beyond that scale.

Rationale: James and Gilliland describe four domains of crisis: developmental, situational, existential and ecosystemic. Ecosystemic crises are natural or human-made disasters, such as hurricanes, pandemics or economic collapse, that strike many people and community systems at once. Response therefore involves community-level coordination, not only individual counseling.

References: James & Gilliland (Crisis Intervention Strategies) — Types of crises: developmental, situational, existential and ecosystemic

☐ Approve ☐ Revise ☐ Retire Notes:

A 33-year-old client whose husband has hit her several times says she will pack the car and leave with her two children tomorrow while he is at work. He does not know about her plan. Which counselor response BEST addresses her safety?

A. Suggest she tell him calmly first so the leaving does not come as a shock

Announcing the plan to an abusive partner can trigger escalation before she is safely away.

B. Build a detailed safety plan, since the time of leaving carries added risk ✓ **KEY**

Separation is a period of heightened danger, so planning routes, documents, shelter and contacts is essential.

C. Urge her to delay until a few couples sessions can try to calm the conflict

Couples work is contraindicated with ongoing battering and could expose her plan to her partner.

D. Affirm her courage and reassure her that once she leaves, the danger is over

Risk often rises during and after separation; false reassurance leaves her unprepared.

Rationale: The period around separation is one of the most dangerous times for a person leaving an abusive partner, because the abuser may escalate to regain control. Safety planning covers timing, a safe destination, key documents, money, children, a code word with trusted people and how to reach help. The counselor supports the client's decision while helping her prepare for the added risk.

References: James & Gilliland (Crisis Intervention Strategies) — Intimate partner violence: danger at separation and safety planning

☐ Approve ☐ Revise ☐ Retire Notes:

A 45-year-old client tells her counselor that her partner pushed her into a wall last week. She has no children or dependents in the home, is not in immediate danger, and asks the counselor not to tell anyone. In most U.S. jurisdictions, how should the counselor proceed?

A. Keep the disclosure private, assess risk, and work with her on safety planning ✓ **KEY**

Abuse of a competent adult is generally not reportable by counselors, so they respect autonomy and plan for safety.

B. Report the assault to the police, since counselors must report all partner abuse

Mandated reporting typically covers children and vulnerable adults, not competent adults abused by a partner.

C. Contact adult protective services, since she is a victim of physical abuse

APS serves elders and dependent or vulnerable adults; nothing suggests she falls in either group.

D. Tell her you can no longer see her unless she agrees to leave the partner

Making care conditional on leaving is coercive, ignores her autonomy and abandons her when risk is high.

Rationale: Mandated reporting laws generally cover child abuse and the abuse of elders or dependent adults; a competent adult's disclosure of partner violence usually is not reportable by a counselor. The counselor keeps the information confidential within the usual limits, assesses danger and lethality, and helps the client build a safety plan. If the client faces serious and foreseeable harm, the counselor reassesses the confidentiality limits.

References: State law — Mandated reporting statutes: child, elder and dependent-adult abuse vs adult partner violence · ACA Code of Ethics — B.1.c Respect for Confidentiality; B.2.a Serious and Foreseeable Harm and Legal Requirements

☐ Approve ☐ Revise ☐ Retire Notes:

A counselor is seeing several 5-year-olds referred by a preschool. Which observation should raise the GREATEST concern about possible sexual abuse?

A. A boy who touches his own genitals at nap time and stops when he is redirected

Self-touching that responds to redirection is common, developmentally expected behavior at this age.

B. A girl who asks where babies come from after her aunt has a new baby at home

Curiosity about birth after a new sibling or cousin is normal for young children.

C. A girl who acts out specific adult sex acts with dolls and describes them ✓ **KEY**

Detailed knowledge of adult sexual acts is beyond what a 5-year-old would normally know and warrants concern.

D. A boy who plays doctor with a same-age friend and looks at body differences

Mutual, playful looking between same-age peers is common sexual curiosity in early childhood.

Rationale: Many sexual behaviors in young children, such as self-touching, questions about babies and playful looking with peers, are normal. Behaviors that show knowledge of adult sexual acts beyond a child's developmental level, or that involve force, coercion or large age gaps, are stronger indicators of possible sexual abuse. Such observations meet the reasonable-suspicion standard and should be reported.

References: Gladding (Counseling: A Comprehensive Profession) — Child abuse: indicators of sexual abuse in young children · State law — Child abuse reporting: reasonable suspicion standard

☐ Approve ☐ Revise ☐ Retire Notes:

A school counselor tells the principal that a 12-year-old described being burned with a cigarette by his father. The principal says, "Let me handle it. I'll call the parents in and decide whether CPS needs to know." What should the counselor do?

A. Let the principal decide, since school policy assigns him the reporting role

A school policy cannot shift a mandated reporter's personal legal duty to an administrator.

B. Wait until the principal meets with the parents, then report if he agrees

Contacting the alleged abuser first can endanger the child and delay a required report.

C. Ask the student to write a statement so the principal has proof to act on

Gathering evidence is the role of investigators; the counselor needs only reasonable suspicion to report.

D. Make the report to child protective services herself, as the law requires ✓ **KEY**

Mandated reporters must report personally and promptly; telling a supervisor does not fulfill the duty.

Rationale: In nearly every state, the duty to report suspected child abuse rests with each mandated reporter individually. Informing a principal or supervisor does not discharge it, and supervisors may not block a report. Counselors also avoid alerting a suspected abuser before child protective services can assess the child's safety.

References: State law — Child abuse reporting statutes: individual duty of mandated reporters

☐ Approve ☐ Revise ☐ Retire Notes:

A 30-year-old with a clear Artistic-Social Holland code works as a payroll clerk, a highly Conventional job. She does the work well but feels restless and drained and talks about quitting. Which Holland concept BEST explains her dissatisfaction?

A. Low congruence, since her personality type is a poor fit for her work setting ✓ KEY

Congruence is the match between personality and environment; an Artistic type in a Conventional job is a poor fit.

B. Low consistency, since her two code letters sit far apart on the hexagon

Artistic and Social are adjacent on the hexagon, so her code is consistent.

C. Low differentiation, since her interest scores are flat across all of the six types

Her code is described as clear, which signals a differentiated profile, not a flat one.

D. Low identity, since she lacks any stable picture of her goals and talents

Nothing suggests she is unclear about herself; the problem lies in the fit with her job.

Rationale: Holland proposed that people search for environments that let them use their skills and express their values, and that satisfaction and stability depend on congruence between personality type and work environment. Artistic types value self-expression and unstructured work, which sits opposite Conventional on the hexagon. Consistency, differentiation and identity describe the person's profile rather than the person-environment fit.

References: Sharf (Career Development Theory) — Holland's theory: congruence, consistency, differentiation and identity

☐ Approve ☐ Revise ☐ Retire Notes:

A counselor wants an up-to-date, free U.S. Department of Labor database that describes occupations by tasks, skills, education needed, and wage and outlook data. Which resource should the counselor use?

A. The Dictionary of Occupational Titles

The DOT was the older Labor Department reference; it has been replaced and is no longer updated.

B. The Strong Interest Inventory profile

The Strong is a commercial interest inventory, not an occupational information database.

C. The O*NET occupational database ✓ KEY

*O*NET is the Labor Department's free online database that replaced the DOT.*

D. The Self-Directed Search code booklet

The SDS is a self-scored Holland interest measure, not a federal occupational database.

Rationale: O*NET, the Occupational Information Network, is sponsored by the U.S. Department of Labor and replaced the Dictionary of Occupational Titles. It describes hundreds of occupations by tasks, knowledge, skills, abilities, work context and education, and links to wage and outlook data. Interest inventories such as the Strong and SDS can be paired with O*NET to explore matching occupations.

References: Sharf (Career Development Theory) — Occupational information: O*NET and the Dictionary of Occupational Titles

☐ Approve ☐ Revise ☐ Retire Notes:

An elementary school counselor plans an eight-week small group for students whose parents are divorcing. Several teachers have referred students. What should the counselor do BEFORE the group begins?

A. Place every referred student in the group so that no child is left out

Accepting all referrals skips screening, which may place children whose needs the group cannot meet.

B. Screen each student to judge fit, and get parent consent as policy requires ✓ **KEY**

ASGW guidelines call for screening members for fit, and schools usually require parent consent for groups.

C. Ask each student to share their family story in front of the whole group first

Pressing children to disclose family details at the outset is premature and can feel unsafe.

D. Mix in children from intact homes so members hear about stable families

A topic-focused group is built around shared experience; adding unaffected peers weakens that purpose.

Rationale: The ASGW Best Practice Guidelines call for group workers to screen prospective members so that each person's needs and goals fit the group's purpose. In schools, a brief individual meeting also lets the counselor explain the group and spot children who need individual help instead. Parent or guardian consent is commonly required for small-group counseling under district policy.

References: ASGW Best Practice Guidelines — A.7 Screening and assessment of prospective members · ASCA National Model — Direct student services: small-group counseling

☐ Approve ☐ Revise ☐ Retire Notes:

A high school principal asks the counselor to focus college and career planning on the top 20 percent of the class, since "the others will figure it out." Which response BEST fits the ASCA National Model?

A. Agree, since top students gain the most from the limited counseling time

Rationing career services by achievement conflicts with the model's commitment to every student.

B. Offer career planning only to students who ask for an appointment first

Waiting for requests favors students with more resources and leaves many unserved.

C. Refer the lower-achieving students to outside agencies for their career needs

Career development is a core part of the school counseling program, not something to send elsewhere.

D. Explain that college and career readiness is a program goal for all students ✓ **KEY**

The ASCA National Model frames school counseling, including career development, as serving every student.

Rationale: The ASCA National Model describes a comprehensive school counseling program that promotes academic, career and social/emotional development for all students. Limiting career planning to high achievers would widen equity gaps, especially for first-generation and low-income students. Counselors can use data to advocate for program delivery that reaches every student.

References: ASCA National Model — Comprehensive program for all students: academic, career and social/emotional domains

☐ Approve ☐ Revise ☐ Retire Notes:

In couples counseling, one partner presses for talk and closeness and grows critical when ignored, while the other goes quiet and leaves the room, which makes the first press harder. A counselor using emotionally focused therapy (EFT) would MOST likely do which of the following?

A. Frame the pursue-withdraw cycle as the enemy and reach for the fears beneath it ✓ KEY

EFT de-escalates by naming the negative cycle and accessing underlying attachment emotions.

B. Give the quieter partner a paradoxical task to withdraw on purpose at set times each day

Paradoxical directives belong to strategic therapy, not to EFT's attachment-focused approach.

C. Teach both partners a structured speaker-listener rule to use when they argue

Communication skills drills are part of behavioral couples work, not EFT's emotional focus.

D. Decide which partner is to blame for the conflict and coach that person to change

EFT avoids blame; it treats the cycle, not either partner, as the problem.

Rationale: Emotionally focused therapy, developed by Sue Johnson and Les Greenberg, views couple distress through attachment theory. In early stages the counselor identifies the negative interaction cycle, such as pursue-withdraw, and helps the couple see the cycle as their common enemy. The counselor then accesses the attachment fears and longings beneath each partner's moves and restructures interactions toward safe connection.

References: Goldenberg (Family Therapy) — Emotionally focused couple therapy: negative cycles and attachment

☐ Approve ☐ Revise ☐ Retire Notes:

A 14-year-old who was adopted as an infant from Guatemala by White parents has started asking about her birth mother and her heritage. Her parents feel hurt and worry they have failed her. What should the counselor convey to the family?

A. Her questions suggest a weak attachment that should be treated before more talk

Curiosity about origins does not signal attachment problems; it is common in securely attached adoptees.

B. Parents should steer her away from these topics until she turns eighteen

Shutting down the topic can leave her feeling her questions are shameful or disloyal.

C. Such questions are a normal part of identity work and open talk will help ✓ KEY

Adolescent adoptees often explore origins as part of identity, and open family communication supports them.

D. Her interest shows she wishes to leave the family and live with her birth kin

Interest in birth family and heritage is not a rejection of the adoptive family.

Rationale: Adolescence brings intensified identity exploration, and adopted teens often add questions about birth parents and, in transracial or international adoption, about ethnic and cultural heritage. Adoptees fare best when parents are open, accept these questions and support connection to the child's heritage. The counselor normalizes the questions and helps parents respond without taking them as rejection.

References: Berk (Development Through the Lifespan) — Adoptive families and adolescent identity development; transracial adoption

☐ Approve ☐ Revise ☐ Retire Notes:

A 44-year-old says that nothing bad has happened, yet she feels a deep loss. She had always expected to marry and have children, and now believes that will not happen. In Schlossberg's transition model, what kind of transition is she facing?

A. An anticipated transition, since she had long planned for these milestones

Anticipated transitions are expected events that do occur, such as a planned move or graduation.

B. An unanticipated transition, since a sudden event changed her life course

Unanticipated transitions are unexpected events that happen, like a layoff; nothing happened here.

C. A developmental crisis, since her distress reflects a stage she has failed

Schlossberg does not frame transitions as failed stages; her model is based on the event or non-event.

D. A non-event transition, since an expected life event did not take place ✓ KEY

Non-events are expected events that fail to occur, which can change roles, routines and assumptions.

Rationale: Schlossberg defines a transition as any event or non-event that changes relationships, routines, assumptions or roles. She names three types: anticipated transitions, unanticipated transitions and non-events, which are expected events that do not occur, such as a promotion or child that never comes. Non-events are often unrecognized by others, so counselors help clients name and grieve them.

References: Sharf (Career Development Theory) — Schlossberg's transition model: anticipated, unanticipated and non-event transitions

☐ Approve ☐ Revise ☐ Retire Notes:

A school counselor is planning support around puberty for middle school students. Based on research on pubertal timing, which group of students is at GREATEST risk for depression, early sexual activity and substance use?

A. Girls who reach puberty well before most of their same-age classmates ✓ KEY

Early-maturing girls face mismatched social pressure and are at raised risk for these outcomes.

B. Boys who reach puberty well before most of their same-age school classmates

Early-maturing boys are often viewed positively, though some show added risk-taking.

C. Girls who reach puberty at about the same time as their classmates do

On-time maturers generally show the fewest adjustment problems.

D. Boys who reach puberty at about the same time as their classmates do

On-time maturation is not linked to elevated risk in either sex.

Rationale: Pubertal timing relative to peers shapes adjustment. Early-maturing girls tend to have more depression, poorer body image, earlier dating and sexual activity, and more substance use, partly because they attract older peers before they are emotionally ready. Early maturation in boys is more often linked with popularity, though both early-maturing boys and late-maturing boys can face difficulties.

References: Berk (Development Through the Lifespan) — Adolescence: pubertal timing and adjustment

☐ Approve ☐ Revise ☐ Retire Notes:

A 46-year-old client says, "Everyone says I'm due for a midlife crisis. Should I expect to blow up my life soon?" Which statement BEST reflects life-span research on midlife?

A. Most adults go through a crisis between ages forty and forty-five

Large studies find dramatic midlife crises affect only a minority of adults.

B. Few people have a true crisis, though many reflect on goals and time ✓ **KEY**

Midlife reappraisal is common, but sudden turmoil and upheaval are not the typical course.

C. A midlife crisis is needed to move on to a healthy later adulthood

No theory or research shows a crisis is required for healthy development.

D. Midlife crises affect men almost only, as their careers start to peak

Both men and women reassess at midlife, and crises are uncommon in both.

Rationale: Popular culture portrays midlife as a time of crisis, but research finds that only a minority of adults experience intense turmoil. Many do engage in midlife reappraisal, weighing time left, goals and relationships, and most report turning points rather than upheaval. Counselors can normalize reflection without predicting crisis.

References: Berk (Development Through the Lifespan) — Middle adulthood: is there a midlife crisis?

☐ Approve ☐ Revise ☐ Retire Notes:

A retired 74-year-old librarian still reads daily, leads a weekly book club and keeps in touch with former coworkers. She says, "I'm still the same person, just with more time." Which theory of aging BEST describes her adjustment?

A. Disengagement theory, as she pulls back from her social roles

Disengagement theory predicts mutual withdrawal from society, which she is not doing.

B. Activity theory, as she replaces each role she has lost

Activity theory stresses replacing lost roles with new ones; she is extending her existing patterns.

C. Continuity theory, as she keeps long-held interests going ✓ **KEY**

Continuity theory holds that older adults adapt by preserving familiar activities, ties and identity.

D. Socioemotional selectivity, as she sheds most social ties

This theory involves narrowing ties to close ones; she has kept her wider network.

Rationale: Continuity theory proposes that older adults strive to maintain consistency between past and present, using familiar skills, activities and relationships to adapt to aging. Disengagement theory, now largely discredited, described mutual withdrawal, while activity theory emphasized replacing lost roles. Socioemotional selectivity theory describes a focus on emotionally close relationships.

References: Berk (Development Through the Lifespan) — Late adulthood: disengagement, activity, continuity and socioemotional selectivity theories

☐ Approve ☐ Revise ☐ Retire Notes:

An 81-year-old man whose wife died four months ago has given away his tools and fishing gear, says he is "just tired of it all," and lives alone with his hunting rifles. He denies being depressed. What should the counselor do FIRST?

A. Accept his denial, since older men often show grief as plain fatigue

Denial does not rule out risk; his warning signs call for direct assessment.

B. Plan grief counseling to help him work through the recent loss of his wife

Grief work may help later, but warning signs of suicide must be assessed first.

C. Refer him to his doctor to check for a physical cause of his tiredness

A medical check may be useful, but it does not address the immediate suicide risk.

D. Ask him directly about suicidal thoughts, plans, intent and gun access ✓ KEY

Recent widowhood, giving away belongings, hopelessness and firearms call for a direct risk assessment.

Rationale: Older men, especially those who are widowed and live alone, have among the highest suicide rates, and they often deny depression or present with fatigue and withdrawal. Warning signs here include giving away possessions, hopeless statements and access to firearms. The counselor asks directly about suicidal ideation, plans, intent and means, using a structured tool such as the C-SSRS, and then addresses lethal means and safety.

References: C-SSRS — Screening for suicidal ideation, intent, plan and behavior · Berk (Development Through the Lifespan) — Late adulthood: suicide risk among older adults

☐ Approve ☐ Revise ☐ Retire Notes:

A 38-year-old accountant lost most of her vision to diabetic retinopathy over the past year. Her mood has improved in counseling, and she now wants to return to work but is unsure how she would manage the tasks. Which next step BEST supports her goal?

A. Advise her to apply for long-term disability, since her field needs sight

Assuming she cannot work in her field limits her options and ignores available adaptations.

B. Refer her to the state vocational rehabilitation agency for job services ✓ KEY

State VR agencies provide assessment, assistive technology, training and job placement for people with disabilities.

C. Suggest she pick a new career with no reading so her vision is not a factor

Steering her away from her career ignores screen readers and other tools that make the work possible.

D. Wait to talk about work until she has fully accepted her loss of eyesight

Adjustment is ongoing; delaying her stated goal is not needed and may slow recovery.

Rationale: Public vocational rehabilitation programs, funded under the Rehabilitation Act, help eligible people with disabilities prepare for, find and keep jobs. Services include vocational assessment, assistive technology such as screen readers, training and placement, often delivered by rehabilitation counselors. Supporting a return to valued work also aids psychological adjustment to an acquired disability.

References: Gladding (Counseling: A Comprehensive Profession) — Rehabilitation counseling: vocational services for people with disabilities

☐ Approve ☐ Revise ☐ Retire Notes:

A counselor writes intake notes on a new adult client as "a person with autism." In session, the client says, "I'm autistic. It is part of who I am, not something I carry around." What should the counselor do?

A. Use the identity-first language the client prefers in talk and notes ✓ KEY

Following the client's own language for their identity shows respect and cultural humility.

B. Keep using person-first language, since it is the professional standard

Person-first language is not required when a client prefers identity-first terms.

C. Avoid naming autism at all so the client does not feel defined by it

Avoiding the term ignores the client's stated view that autism is part of their identity.

D. Explain why person-first terms are more respectful and kinder to use

Correcting the client imposes the counselor's values over the client's self-definition.

Rationale: Person-first language ("person with a disability") was promoted to avoid defining people by a condition, but many autistic and Deaf people prefer identity-first language that treats disability as part of identity and culture. Culturally responsive counselors ask about and use the client's preferred terms. Disability is a dimension of diversity, and language choices convey respect.

References: Sue & Sue (Counseling the Culturally Diverse) — Counseling persons with disabilities: language, identity and disability culture · MSJCC (Ratts et al., 2016) — Client worldview: understanding clients' identities as they define them

☐ Approve ☐ Revise ☐ Retire Notes:

A 34-year-old gay man works at a small firm where coworkers often tell anti-gay jokes. He is out to family and friends but not at work, and asks whether he "should" come out there. Which counselor response is MOST appropriate?

A. Encourage him to come out soon, since hiding will harm his mental health

Pushing disclosure ignores the real risks in his workplace and his right to decide.

B. Advise him to keep it private, since disclosure at work is seldom worth the risk

Steering him toward concealment is also directive and overlooks possible benefits of disclosure.

C. Tell him the jokes are likely harmless and not worth worrying much about

Dismissing the jokes invalidates a real minority stressor and his concern for safety.

D. Help him weigh safety, supports, rights and values to reach his own choice ✓ KEY

Disclosure is the client's decision; the counselor helps him weigh risks, resources and goals.

Rationale: Coming out is a series of decisions made in each setting, and the risks and benefits differ by context. Concealment can add stress, but disclosure in a hostile workplace can bring harassment or job loss. An affirming counselor helps the client weigh safety, legal protections, support and personal values, and respects his choice.

References: Sue & Sue (Counseling the Culturally Diverse) — Counseling LGBTQ individuals: disclosure decisions and workplace climate · MSJCC (Ratts et al., 2016) — Counseling and advocacy interventions: client-led decisions in oppressive settings

☐ Approve ☐ Revise ☐ Retire Notes:

A transgender man, assigned female at birth, tells his counselor that he is attracted only to men. A student in practicum assumes this means the client is "really straight." Which statement corrects the student's error?

A. Sexual orientation follows gender assigned at birth, so he is straight

This repeats the error; orientation is described relative to a person's gender identity.

B. Transgender clients cannot be described by any sexual orientation term

Transgender people hold the full range of sexual orientations, as cisgender people do.

C. Orientation and gender identity differ; a man drawn to men may be gay ✓ **KEY**

As a man attracted to men, he may identify as gay; identity and attraction are separate dimensions.

D. His orientation can be known only after any medical transition is done

Orientation does not depend on medical transition and is defined by the client.

Rationale: Gender identity is a person's internal sense of their gender, while sexual orientation describes whom a person is attracted to. The two are independent, and orientation labels are understood in relation to the person's gender identity, not the sex assigned at birth. Counselors use the terms clients choose for themselves.

References: Sue & Sue (Counseling the Culturally Diverse) — Counseling LGBTQ individuals: sexual orientation vs gender identity

☐ Approve ☐ Revise ☐ Retire Notes:

A bilingual 29-year-old who immigrated from Brazil at 15 speaks fluent English in sessions. When describing her grandmother's death, she pauses, says a phrase in Portuguese, then apologizes. What should the English-speaking counselor do?

A. Ask her to keep to English so the counselor does not miss anything said

Limiting her to English can block access to emotional material stored in her first language.

B. Invite her to use Portuguese when it fits, then explore what it meant ✓ **KEY**

Feelings tied to early memories are often more available in the first language; inviting it deepens the work.

C. Refer her to a Portuguese-speaking counselor for the rest of treatment

Referral is unnecessary for a fluent client; the counselor can welcome both languages.

D. Treat the language switch as a sign of avoidance to gently confront

Code-switching here reflects emotional access, not resistance, and confronting it would shame her.

Rationale: For bilingual clients, emotions and memories formed in the first language often remain more vivid in that language. Switching languages can signal emotional depth rather than avoidance. Counselors who welcome the first language and then explore its meaning with the client honor culture and can reach material that a second language keeps at a distance.

References: Sue & Sue (Counseling the Culturally Diverse) — Language barriers and bilingual clients in counseling

☐ Approve ☐ Revise ☐ Retire Notes:

A 50-year-old who was a surgeon in Syria now works nights stocking shelves while his license is reviewed. He is irritable, sleeps poorly, and says, "Here I am nobody." Which factor MOST likely drives his distress?

A. Loss of status and role, a common source of stress after immigration ✓ KEY

Downward occupational mobility strips identity and respect and is a well-known acculturative stressor.

B. Integration, since he has adopted both his old and new cultures fully

Integration is the strategy linked to the best adjustment and is not shown in his account.

C. A personality disorder, as shown by his long-standing pride and anger

His irritability began after migration; nothing shows a long-standing, pervasive pattern.

D. Culture shock that will fade on its own once his first year has passed

His distress stems from an ongoing loss of role that time alone may not resolve.

Rationale: Many immigrants and refugees face downward occupational mobility because credentials are not recognized, which brings loss of income, status and professional identity. This role loss is a major source of acculturative stress and can show up as irritability, low mood and sleep problems. Counselors validate the loss and may help with credentialing resources and advocacy.

References: Sue & Sue (Counseling the Culturally Diverse) — Immigrants and refugees: loss of status and acculturative stress

☐ Approve ☐ Revise ☐ Retire Notes:

A 57-year-old client says her imam has been a steady source of comfort since her son's arrest, and she wishes the imam and counselor could "work together" to support her. How should the counselor respond?

A. Decline, since religious leaders should stay apart from clinical care

The Code encourages considering religious leaders as supports when the client wants this.

B. Call the imam that day to share an update on her progress in sessions

Contacting the imam without the client's consent would breach confidentiality.

C. Advise her to rely on the counselor alone so her care stays consistent

Discouraging a valued support ignores her worldview and narrows her resources.

D. Discuss what she wants and, with written consent, coordinate with him ✓ KEY

With consent, counselors may enlist religious and community leaders as positive supports.

Rationale: ACA Code standard A.1.d directs counselors to recognize that support networks hold various meanings in clients' lives and to consider enlisting religious or spiritual leaders, family and friends as positive resources, with client consent. The counselor clarifies what the client hopes for and what information may be shared before any contact. This respects both her faith and her confidentiality.

References: ACA Code of Ethics — A.1.d Support Network Involvement

☐ Approve ☐ Revise ☐ Retire Notes:

A Muslim client who usually meets at 5 p.m. mentions that Ramadan begins next week and that she fasts from dawn until sunset. What is the BEST response?

A. Suggest she eat a light snack before sessions so that she can stay focused

Eating would break her fast; the suggestion puts the counselor's convenience over her religious practice.

B. Ask how fasting affects her, and plan times together that suit her needs ✓ **KEY**

Asking about her experience and planning together respects her practice and her role in decisions.

C. Suggest pausing counseling for the month so her fasting is not disrupted

Suspending care assumes her needs and may interrupt progress without her input.

D. Keep the usual time, since changing it might imply that she is different

Ignoring a stated practice is not neutral; it overlooks a real effect on her day.

Rationale: Religious practices such as fasting during Ramadan affect energy, schedules and evening routines, since many people break the fast at sunset. Culturally responsive counselors ask how a practice affects the client and adapt logistics collaboratively. This conveys respect for the client's faith without assuming what she needs.

References: Sue & Sue (Counseling the Culturally Diverse) — Religious diversity: counseling Muslim clients

☐ Approve ☐ Revise ☐ Retire Notes:

A 36-year-old Army veteran with PTSD also has a history of mild traumatic brain injury from a blast eight years ago. He has some trouble concentrating. His previous counselor said trauma-focused therapy was "off the table" because of the brain injury. Which statement BEST reflects VA/DoD guidance?

A. Trauma therapy should wait until neuropsychological testing rules out deficits

Testing may guide adaptations, but it is not a required step before trauma-focused care.

B. Medication should be the main treatment, as therapy is too hard after a TBI

Guidance favors trauma-focused psychotherapy, and mild TBI does not make it too demanding.

C. A history of mild TBI does not rule out evidence-based trauma-focused therapy ✓ **KEY**

VA/DoD guidance treats PTSD with the same first-line therapies when mild TBI is present.

D. PTSD symptoms should be treated as brain injury effects with cognitive rehab

Assigning all symptoms to TBI overlooks treatable PTSD and the first-line therapies for it.

Rationale: Mild TBI and PTSD often co-occur in veterans and share symptoms such as poor concentration, irritability and sleep problems. VA/DoD guidance recommends treating PTSD with the same evidence-based trauma-focused psychotherapies, such as prolonged exposure or cognitive processing therapy, whether or not a history of mild TBI is present. Counselors may adapt delivery, for example with written aids, but should not withhold first-line care.

References: VA/DoD CPG — Management of PTSD and Acute Stress Disorder (2023): co-occurring mild TBI

☐ Approve ☐ Revise ☐ Retire Notes:

A Marine veteran back from two deployments drives down the center of the road, swerves around trash bags, and sits with his back to the wall in restaurants. His wife calls him "paranoid." Which counselor framing is MOST helpful?

A. These actions were trained survival skills in combat that now need to be reset ✓ **KEY**

Framing them as once-adaptive responses reduces shame and sets up recalibration for civilian life.

B. These actions are early signs of a psychotic disorder needing urgent referral

Learned, situation-linked vigilance with intact reality testing is not psychosis.

C. These actions show a personality flaw that he must take responsibility to correct

Blaming his character ignores the trauma and training behind the behavior and adds shame.

D. These actions are minor quirks that will fade without help as time goes by

Hypervigilance can persist and strain family life; dismissing it delays useful help.

Rationale: Trauma-informed care views many post-trauma reactions as adaptations that once protected the person. For combat veterans, scanning, avoiding roadside objects and guarding exits were trained, life-saving skills. Naming them as adaptive in theater but costly at home reduces shame and opens the door to recalibrating them and to assessing for PTSD.

References: SAMHSA TIP 57 — Trauma-informed care: trauma reactions as adaptive responses; veterans and service members

☐ Approve ☐ Revise ☐ Retire Notes:

Treatment Planning

85 items · NCE weight 9% (14 of 160 scored items)

Q536 of 1000 · nce-s-trt-001 · Counseling and Helping Relationships · SMART goals and measurable objectives · easy · In review

A counselor is writing a treatment plan for a 34-year-old client with social anxiety. Which objective is BEST written according to SMART criteria?

A. Client will feel much more confident in social settings by the end of treatment.

Feeling "more confident" is not observable or measurable, and "end of treatment" is not a specific time frame.

B. Client will start one conversation with a coworker at least 3 days a week within 6 weeks. ✓ **KEY**

This is specific, measurable (frequency count), achievable, relevant to social anxiety, and time-bound (6 weeks).

C. Client will understand the cognitive roots of social anxiety and work through them fully.

"Understand" and "work through" are vague internal processes with no measurable criterion or deadline.

D. Client will attend weekly counseling sessions and complete homework as the counselor assigns.

Attendance and compliance describe the treatment process, not a measurable change in the presenting problem.

Rationale: SMART objectives are Specific, Measurable, Achievable, Relevant and Time-bound. A good objective names an observable behavior, a criterion for success, and a deadline, so progress can be tracked and the plan revised. Vague feeling states and process statements (attendance) do not meet the standard.

References: Wiger (Documentation) — Treatment plans: behaviorally measurable goals and objectives

► **Check:** Wiger reference detail describes the concept, not an exact chapter title.

☐ Approve ☐ Revise ☐ Retire Notes:

Q537 of 1000 · nce-s-trt-002 · Counseling and Helping Relationships · matching level of care to need · medium · In review

A 41-year-old client with an alcohol use disorder has relapsed twice while attending weekly outpatient sessions. There is no withdrawal risk, no acute medical or psychiatric concern, and the client has stable housing and a supportive partner. Which level of care is MOST appropriate?

A. Medically managed intensive inpatient treatment in a hospital setting

Hospital-level care is for severe, unstable withdrawal or acute medical/psychiatric needs, none of which are present.

B. Long-term residential treatment with 24-hour structure and support

Residential care is more restrictive than needed for a client with stable housing and a supportive recovery environment.

C. An intensive outpatient program with several sessions per week ✓ **KEY**

Repeated relapse despite weekly outpatient care, with no withdrawal, medical or housing barriers, points to a step up to the next more intensive outpatient level.

D. Continue weekly outpatient sessions with no change to the treatment plan

Repeated relapse at the current intensity signals that the present level is not meeting the client's needs.

Rationale: Placement criteria such as the ASAM Criteria match clients to the least restrictive level of care that safely meets their needs across multiple dimensions (withdrawal, medical, emotional, readiness, relapse potential, recovery environment). When a client is not progressing at one level and has no acute risks, the usual step is to the next more intensive level, not a jump to residential or hospital care.

References: ASAM Criteria — Multidimensional assessment and least restrictive level of care; outpatient vs intensive outpatient

► **Check:** ASAM Criteria 4th ed. (2023) renamed some levels; confirm the level-of-care wording is current.

☐ Approve ☐ Revise ☐ Retire Notes:

A 29-year-old veteran meets criteria for posttraumatic stress disorder after a combat deployment. According to major clinical practice guidelines, which psychotherapy approach has the strongest research support as a first-line treatment?

A. Trauma-focused therapy such as cognitive processing therapy or prolonged exposure ✓ KEY

APA and VA/DoD guidelines give their strongest recommendations to trauma-focused therapies such as CPT and PE.

B. A single session of psychological debriefing held soon after returning home

Single-session debriefing is not recommended; it has not been shown to prevent or treat PTSD and may be harmful.

C. Open-ended nondirective supportive counseling focused on current life stressors

Nondirective supportive counseling is often used as a control condition and is not a first-line PTSD treatment.

D. Progressive muscle relaxation training used alone as the primary treatment

Relaxation may help with arousal, but used alone it lacks the support of trauma-focused treatments for PTSD.

Rationale: Evidence-based treatment selection integrates the best research evidence with clinical expertise and client preferences. For PTSD, the APA (2017) and VA/DoD guidelines recommend trauma-focused psychotherapies such as cognitive processing therapy and prolonged exposure as first-line treatments.

References: APA CPG — Clinical Practice Guideline for the Treatment of PTSD (2017): CPT, PE, CBT and CT strongly recommended · VA/DoD CPG — Management of PTSD and Acute Stress Disorder: trauma-focused psychotherapy as first-line treatment

► **Check:** VA/DoD 2023 reference detail is paraphrased; confirm wording.

☐ Approve ☐ Revise ☐ Retire Notes:

A counselor has completed an intake with a 52-year-old Korean American client who reports grief and insomnia after her husband's death. What is the BEST way for the counselor to develop the treatment plan?

A. Write the plan alone based on the diagnosis, then review it with the client next week

Presenting a finished plan treats the client as a recipient rather than a partner and misses her priorities.

B. Use a standard grief protocol so the client receives the same plan as other clients

A one-size-fits-all plan ignores individual goals, cultural values and preferences.

C. Ask the client's adult children what goals they would like their mother to work on

Family input may be welcome with consent, but the client herself is the primary partner in setting her goals.

D. Work with the client to identify her goals and the methods she finds acceptable ✓ KEY

Collaborative planning engages the client in choosing goals and methods, respecting her values and culture.

Rationale: Treatment plans should be developed jointly with the client. Collaboration respects autonomy, incorporates cultural values and preferences, and increases engagement and follow-through. The ACA Code directs counselors and clients to work jointly in devising counseling plans that offer reasonable promise of success.

References: ACA Code of Ethics — A.1.c Counseling Plans

☐ Approve ☐ Revise ☐ Retire Notes:

A counselor gives a 23-year-old client the PHQ-9 at every session. After eight sessions of treatment for depression, the client's scores have not meaningfully changed from intake. What should the counselor do NEXT?

A. Stop giving the PHQ-9, since repeated testing seems to be discouraging the client

Dropping the measure removes the very data that revealed the lack of progress.

B. Review the scores with the client and revise the treatment plan together ✓ **KEY**

Measurement-based care uses nonresponse as a signal to discuss progress with the client and adjust the plan.

C. Continue the same approach unchanged, since depression often improves slowly

Ignoring eight sessions of flat scores delays a needed change in treatment.

D. Refer the client out at once, since the lack of change shows a poor counselor fit

Flat scores call for review and adjustment first; immediate referral is premature.

Rationale: Routine outcome monitoring with a standardized measure lets the counselor detect clients who are not responding. Feedback on scores should be shared with the client and used to revisit goals, interventions or level of care. Measurement-based care has been shown to improve outcomes, especially for clients at risk of treatment failure.

References: Hays (Assessment) — Outcome assessment and progress monitoring with standardized measures

☐ Approve ☐ Revise ☐ Retire Notes:

During a first session, a 47-year-old client mentions that she also sees a psychiatrist who prescribes her antidepressant. What should the counselor do to coordinate care?

A. Call the psychiatrist right away to share the intake findings and diagnosis

Sharing information without the client's authorization breaches confidentiality.

B. Ask the client to stop seeing the psychiatrist to avoid conflicting treatment

Medication management and counseling are complementary; there is no basis for asking her to stop.

C. Request the client's written consent to inform and consult with the psychiatrist ✓ **KEY**

The ACA Code directs counselors to request a release so they can inform the other professional and collaborate.

D. Note the psychiatrist in the file and leave all contact entirely up to the client

This is passive; counselors are expected to seek permission to coordinate with other providers.

Rationale: When counselors learn that a client is in a professional relationship with another mental health professional, they request a release to inform that professional and work toward a collaborative relationship. Coordination improves safety and consistency of care, while written consent protects confidentiality.

References: ACA Code of Ethics — A.3 Clients Served by Others

☐ Approve ☐ Revise ☐ Retire Notes:

A program director is training new counselors at a community mental health agency. When should discharge and aftercare planning ideally begin for a new client?

A. At intake, revisiting the plan throughout the course of treatment ✓ KEY

Planning for discharge from the start keeps treatment goal-directed and prepares the client for transition.

B. In the final session, once the client has met every treatment goal

Waiting until the last session leaves too little time to arrange supports and aftercare.

C. When the insurer signals that authorized sessions are running out

Letting payer limits drive discharge timing is reactive and can leave gaps in care.

D. Only after the client asks to end treatment or stops attending

This leaves aftercare to chance and misses the chance to plan termination together.

Rationale: Discharge and aftercare planning should begin at admission and be updated as treatment progresses. Early planning defines what successful completion looks like, identifies community supports and step-down services, and reduces the risk of relapse or abrupt termination.

References: Wiger (Documentation) — Discharge planning and the discharge summary

► **Check:** Wiger reference detail describes the concept, not an exact chapter title.

☐ Approve ☐ Revise ☐ Retire Notes:

A counselor is completing a Stanley-Brown Safety Planning Intervention with a 19-year-old college student who has recent suicidal ideation. Which step does the safety plan address FIRST?

A. Listing professionals and crisis lines to contact in an emergency

Professional and crisis contacts come later in the plan, after self-directed and social steps.

B. Making the environment safe by limiting access to lethal means

Means safety is a key part of the plan but is addressed as the final step.

C. Naming people the student can ask directly for help in a crisis

Reaching out to trusted people for help follows warning signs and internal coping.

D. Recognizing personal warning signs that a crisis may be developing ✓ KEY

The plan begins with identifying warning signs so the client knows when to use the plan.

Rationale: The Stanley-Brown Safety Plan proceeds in order: (1) warning signs, (2) internal coping strategies, (3) people and social settings that provide distraction, (4) people to ask for help, (5) professionals and agencies to contact, and (6) making the environment safe. Recognizing warning signs comes first because it tells the client when to begin using the plan.

References: Stanley-Brown SPI — Six-step safety plan: Step 1, recognizing warning signs

☐ Approve ☐ Revise ☐ Retire Notes:

A 38-year-old client says, "I know my drinking is hurting my marriage, and part of me wants to cut back, but I'm just not sure I'm ready to give it up." Which stage of change does this statement BEST reflect?

A. Precontemplation

In precontemplation the person does not see a problem or intend to change; this client recognizes the problem.

B. Contemplation ✓ KEY

Acknowledging the problem while feeling ambivalent about changing is the hallmark of contemplation.

C. Preparation

Preparation involves a commitment to act soon and small steps toward change, which are absent here.

D. Action

Action involves actively modifying the behavior, which this client has not begun.

Rationale: In the Transtheoretical Model, contemplation is marked by awareness that a problem exists and serious thought about change, combined with ambivalence and no firm commitment. Counselors often use motivational interviewing to explore and resolve that ambivalence.

References: Transtheoretical Model — Stages of change: contemplation and ambivalence

☐ Approve ☐ Revise ☐ Retire Notes:

A 60-year-old client stopped smoking four months ago. He uses nicotine gum, avoids places where he used to smoke, and calls a friend when cravings hit. According to Prochaska and DiClemente, which stage is he in?

A. Preparation

Preparation precedes the behavior change; this client has already quit.

B. Maintenance

Maintenance generally begins after about six months of sustained change; he is at four months.

C. Action ✓ KEY

He has changed the behavior within the past six months and is actively using change strategies.

D. Contemplation

Contemplation involves thinking about change without acting; he has already acted.

Rationale: The action stage covers roughly the first six months after a person has made an overt change in behavior, when effort and relapse risk are highest. Maintenance begins once the change has been sustained for more than about six months and focuses on preventing relapse.

References: Transtheoretical Model — Action vs maintenance: the six-month criterion

► **Check:** Action vs. maintenance rests on the conventional ~6-month line in the Transtheoretical Model.

☐ Approve ☐ Revise ☐ Retire Notes:

Three months into a new school-based anger management group, a counselor gathers feedback from students and teachers so she can refine the sessions while the program is still running. This is an example of which type of evaluation?

A. Formative evaluation ✓ KEY

Formative evaluation occurs during implementation to improve the program as it runs.

B. Summative evaluation

Summative evaluation judges overall effectiveness or outcomes after the program is completed.

C. Needs assessment

A needs assessment happens before a program is designed, to identify gaps and priorities.

D. Cost-benefit analysis

Cost-benefit analysis compares program costs with monetary value of outcomes, not process feedback.

Rationale: Formative evaluation is conducted while a program is under way to give feedback for improvement. Summative evaluation is conducted at the end to judge whether the program achieved its goals and should be continued, expanded or ended.

References: Erford (Research and Evaluation in Counseling) — Program evaluation: formative vs summative evaluation

☐ Approve ☐ Revise ☐ Retire Notes:

Before creating any new services, a rural counseling center surveys residents, local physicians and school staff to find out which mental health concerns are most common and which services are missing. What is the center conducting?

A. An outcome evaluation

Outcome evaluation measures the effects of an existing program; no program exists yet.

B. A process evaluation

Process evaluation examines how an existing program is being delivered.

C. A fidelity review

A fidelity review checks whether a program is delivered as designed, which requires a program in place.

D. A needs assessment ✓ KEY

Gathering data on community problems and service gaps before planning services is a needs assessment.

Rationale: A needs assessment systematically identifies the gap between current conditions and desired conditions in a population, using surveys, key informants, focus groups or existing data. It is typically the first step in program planning and guides which services to develop.

References: Erford (Research and Evaluation in Counseling) — Needs assessment as the first phase of program evaluation

☐ Approve ☐ Revise ☐ Retire Notes:

A school counselor records a student's disruptive outbursts during baseline, introduces a token system, removes it, and then reintroduces it (an ABAB design). What is the main purpose of the withdrawal phase?

A. To let the student rest so the token system does not lose its appeal

Withdrawal is not about preventing satiation; it is a test of experimental control.

B. To show that behavior changes when and only when the treatment changes ✓ **KEY**

A return toward baseline when treatment is removed, and improvement when it returns, supports a causal link.

C. To compare the student's behavior with a randomly assigned control group

Single-subject designs use the person as his or her own control; there is no separate group.

D. To produce enough data points to compute a reliable correlation value

The goal of the reversal is to demonstrate experimental control, not to compute a correlation.

Rationale: In an ABAB (reversal or withdrawal) single-subject design, behavior is measured at baseline (A), during treatment (B), after treatment is withdrawn (A), and after it is reinstated (B). If behavior worsens on withdrawal and improves again on reinstatement, the counselor has stronger evidence that the intervention, not some other factor, caused the change.

References: Erford (Research and Evaluation in Counseling) — Single-subject designs: AB and ABAB (withdrawal) designs

☐ Approve ☐ Revise ☐ Retire Notes:

A researcher delivers a new career-readiness curriculum to one intact ninth-grade class and uses a second existing class at the same school as a comparison. Both classes complete pretests and posttests. Which design is this?

A. A true experimental pretest-posttest control group design

A true experiment requires random assignment of participants to conditions, which did not occur.

B. A single-subject multiple-baseline design across settings

Single-subject designs track individuals over repeated phases, not two classes compared once.

C. A quasi-experimental nonequivalent control group design ✓ **KEY**

Using intact groups without random assignment, with pre- and posttests, is a nonequivalent group quasi-experiment.

D. A correlational design examining two naturally linked variables

The researcher manipulated an independent variable (the curriculum), so this is not correlational.

Rationale: The defining feature of a true experiment is random assignment to conditions. When a researcher manipulates an independent variable but uses pre-existing groups, the design is quasi-experimental. Pretests help assess initial group differences, but selection remains a threat to internal validity.

References: Erford (Research and Evaluation in Counseling) — Experimental vs quasi-experimental designs; random assignment

☐ Approve ☐ Revise ☐ Retire Notes:

A counselor selects the 15 students with the highest scores on a test anxiety scale for a six-week group. There is no comparison group, all students complete the group, and nothing notable happens at school during that time. Posttest scores are lower. Which threat to internal validity MOST likely accounts for part of this drop?

A. Experimental mortality, or attrition of group members

All students completed the group, so loss of participants cannot explain the change.

B. History, or outside events occurring during the study period

The stem rules out notable outside events during the six weeks.

C. Diffusion of treatment between groups being compared

Diffusion requires a comparison group that is exposed to the treatment; there is none.

D. Statistical regression of extreme scores toward the mean ✓ KEY

Participants selected for extreme scores tend to score closer to the mean on retest even without treatment.

Rationale: Statistical regression is a threat to internal validity whenever participants are chosen because of extreme scores. Because no measure is perfectly reliable, extreme scorers tend to move toward the mean on retesting, which can be mistaken for a treatment effect. A comparison group selected the same way helps rule this out.

References: Erford (Research and Evaluation in Counseling) — Threats to internal validity: statistical regression

☐ Approve ☐ Revise ☐ Retire Notes:

A researcher sets alpha at .05 and finds $p = .03$ when comparing a new intervention with treatment as usual, so she rejects the null hypothesis. If the intervention in fact has no real effect, what has occurred?

A. A Type I error, a false positive finding ✓ KEY

Rejecting a null hypothesis that is actually true is a Type I error; its probability is set by alpha.

B. A Type II error, a false negative finding

A Type II error is failing to reject a false null, missing a real effect; here the null was rejected.

C. A correct decision, since p fell below alpha

A p -value below alpha does not guarantee a true effect; if there is none, rejecting the null was an error.

D. A power failure caused by too small a sample

Low power raises the risk of Type II errors, not of falsely detecting an effect that does not exist.

Rationale: A Type I error (alpha) is rejecting a true null hypothesis, concluding an effect exists when it does not. A Type II error (beta) is failing to reject a false null. The p -value is the probability of results at least this extreme if the null is true; it is not the probability that the null is true, so a significant p can still be a false positive.

References: Erford (Research and Evaluation in Counseling) — Hypothesis testing: Type I and Type II errors, alpha and p -values

☐ Approve ☐ Revise ☐ Retire Notes:

A study of 4,000 clients finds that a brief online module reduces anxiety scores compared with a waitlist, $p < .001$, with Cohen's $d = 0.08$. How should a counselor BEST interpret these results?

A. The module has a large effect, as shown by the very small p-value

The p-value reflects sample size as well as effect; it does not indicate magnitude.

B. The difference is statistically significant but practically trivial ✓ **KEY**

With a very large sample, tiny effects reach significance; $d = 0.08$ is well below even a small effect (0.2).

C. The result is likely a Type II error because the sample was so large

A Type II error is missing a real effect; here an effect was detected, and large samples reduce Type II risk.

D. The module causes anxiety to drop by about 8 percent for most clients

Cohen's d is a difference in standard deviation units, not a percentage reduction.

Rationale: Statistical significance tells whether an effect is unlikely under the null hypothesis; effect size tells how large it is. Cohen's guidelines for d are about 0.2 small, 0.5 medium and 0.8 large. Large samples can make trivial differences significant, so counselors should look at effect size to judge clinical or practical importance.

References: Erford (Research and Evaluation in Counseling) — Effect size (Cohen's d) vs statistical significance

☐ Approve ☐ Revise ☐ Retire Notes:

A survey of 600 adolescents finds that hours spent on social media correlate $r = .45$ with scores on a depressive symptom scale. Which conclusion is BEST supported by this finding?

A. Heavy social media use causes depressive symptoms in teenagers

A correlation cannot establish causation; depression could drive use, or a third variable could affect both.

B. Teens who use social media more tend to report more depressive symptoms ✓ **KEY**

A positive correlation means higher values on one variable tend to go with higher values on the other, without implying cause.

C. About 45 percent of depressive symptoms are explained by social media use

Shared variance is r squared ($.45 \times .45$, about 20 percent), and even that describes association, not explanation by cause.

D. Reducing social media time will lower depression scores for most teens

Predicting the effect of changing one variable is a causal claim that requires an experimental design.

Rationale: Correlational designs show whether and how strongly two variables move together, but not why. The direction of influence is unknown, and a third variable (such as loneliness or poor sleep) may account for both. The coefficient of determination (r squared) gives the proportion of shared variance; here about 20 percent.

References: Erford (Research and Evaluation in Counseling) — Correlational research: correlation vs causation and the coefficient of determination

☐ Approve ☐ Revise ☐ Retire Notes:

A statistics instructor asks counseling students to classify several variables by scale of measurement. Which variable is measured on an interval scale?

A. Clients' ranked order of improvement

Ranks show order but not equal distances between positions, which makes them ordinal.

B. Number of sessions each client attended

A count has equal intervals and a true zero (no sessions), which makes it a ratio scale.

C. Temperature in degrees Fahrenheit ✓ **KEY**

Fahrenheit has equal intervals but an arbitrary zero point, the defining feature of an interval scale.

D. Clients' primary diagnostic category

Diagnostic categories are named groups with no inherent order, which makes them nominal.

Rationale: The four scales build on each other: nominal (categories), ordinal (rank order), interval (equal units but no true zero), and ratio (equal units plus a true zero). Because interval data lack a true zero, ratios are not meaningful; 80 degrees Fahrenheit is not twice as hot as 40 degrees.

References: Erford (Research and Evaluation in Counseling) — Scales of measurement: nominal, ordinal, interval and ratio

☐ Approve ☐ Revise ☐ Retire Notes:

Annual household incomes of clients at a community counseling agency are mostly low, but a small number of clients report very high incomes. Which statement about this distribution is accurate?

A. It is positively skewed, and the mean is higher than the median ✓ **KEY**

A tail of a few very high values is a positive skew, and the mean is pulled toward that tail, above the median.

B. It is negatively skewed, and the mean is lower than the median

A negative skew has its tail toward low values; here the tail is toward high incomes.

C. It is positively skewed, and the mode is higher than the mean

In a positive skew the mode sits at the peak on the low end, below both the median and the mean.

D. It is symmetrical, so the mean, median and mode are about equal

A few extreme high values make the distribution asymmetrical, so the three measures separate.

Rationale: Skew is named for the direction of the tail. In a positively skewed distribution, the typical order is mode < median < mean, because the mean is sensitive to extreme scores. For skewed data such as income, the median is usually the best single measure of central tendency.

References: Erford (Research and Evaluation in Counseling) — Descriptive statistics: measures of central tendency and skewed distributions

☐ Approve ☐ Revise ☐ Retire Notes:

Scores on a depression inventory are normally distributed with a mean of 50 and a standard deviation of 10. A client scores 60. Approximately what percentage of the norm group scored at or below this client?

A. 34 percent

About 34 percent of scores fall between the mean and +1 SD; this omits the 50 percent below the mean.

B. 50 percent

Fifty percent falls at or below the mean of 50, not at a score one SD above it.

C. 68 percent

About 68 percent fall within plus or minus 1 SD of the mean, which is a band, not a cumulative percentage.

D. 84 percent ✓ KEY

A score of 60 is $z = +1$; 50 percent fall below the mean plus about 34 percent between the mean and +1 SD.

Rationale: The standard deviation describes how spread out scores are around the mean. In a normal distribution, about 68 percent of scores fall within 1 SD of the mean, 95 percent within 2 SD, and 99.7 percent within 3 SD. A score 1 SD above the mean ($z = +1$, $T = 60$) sits at about the 84th percentile.

References: Erford (Research and Evaluation in Counseling) — Standard deviation, the normal curve and percentile ranks

☐ Approve ☐ Revise ☐ Retire Notes:

A researcher randomly assigns 90 clients to one of three conditions (individual CBT, a mindfulness group, or a waitlist) and compares the groups on posttest anxiety scores measured on an interval scale. Which statistical test is MOST appropriate?

A. An independent-samples t-test

A t-test compares only two group means; running several t-tests inflates the Type I error rate.

B. A one-way analysis of variance ✓ KEY

One categorical independent variable with three levels and one interval dependent variable calls for a one-way ANOVA.

C. A chi-square test of independence

Chi-square tests relationships between categorical variables (frequencies), not differences in mean scores.

D. A Pearson product-moment correlation

Pearson r measures the linear relationship between two continuous variables, not group differences.

Rationale: Here the independent variable is treatment condition (three levels) and the dependent variable is the anxiety score. A one-way ANOVA compares three or more group means in a single test while holding the Type I error rate at alpha; post hoc tests then show which groups differ. A t-test is used for two groups, chi-square for categorical outcomes, and Pearson r for relationships between two continuous variables.

References: Erford (Research and Evaluation in Counseling) — Selecting inferential statistics: t-test, ANOVA, chi-square and correlation

☐ Approve ☐ Revise ☐ Retire Notes:

A counselor-researcher is planning a study comparing two interventions and wants to reduce the risk of missing a true but modest difference between them. Which change would MOST directly increase statistical power?

A. Lowering alpha from .05 to .01 to guard against false positives

A stricter alpha makes it harder to reject the null, which lowers power and raises Type II risk.

B. Using a smaller sample so each participant gets more attention

Smaller samples increase sampling error and reduce power to detect a real effect.

C. Increasing the number of participants assigned to each condition ✓ **KEY**

Larger samples reduce standard error, making a true effect of a given size more likely to be detected.

D. Switching from a one-tailed test to a two-tailed test of the effect

Splitting alpha across two tails lowers power for an effect in the predicted direction.

Rationale: Power (1 - beta) is the probability of correctly rejecting a false null hypothesis. It rises with larger sample size, larger effect size, a more lenient alpha, a one-tailed test (when the direction is correct), and more reliable measures. Researchers run a power analysis before a study, often targeting .80, to choose a sample size that can detect the expected effect.

References: Erford (Research and Evaluation in Counseling) — Statistical power, sample size and Type II error

☐ Approve ☐ Revise ☐ Retire Notes:

A trial finds that a new anxiety program works well. Participants knew they were in a closely watched study, received extra attention from research staff, and liked being part of something new. Critics doubt that the results will hold in routine agency practice. Which threat are the critics MOST likely describing?

A. Reactive effects of the experimental arrangements ✓ **KEY**

Changes caused by knowing one is studied (a Hawthorne-type effect) limit generalization to ordinary settings, a threat to external validity.

B. Statistical regression of extreme scores toward the mean

Regression is an internal validity threat tied to selecting extreme scorers, which the stem does not describe.

C. Instrumentation changes in how outcomes were measured

Instrumentation is an internal validity threat involving shifts in measurement, not generalizability.

D. Selection differences between the groups being compared

Selection is an internal validity threat about nonequivalent groups, not about generalizing to practice.

Rationale: External validity concerns whether results generalize to other people, settings and times. Reactive arrangements, such as the Hawthorne effect, occur when the research situation itself (attention, novelty, awareness of being studied) contributes to the outcome, so effects may shrink in routine care. Internal validity threats such as history, regression, selection and instrumentation instead question whether the treatment caused the change at all.

References: Erford (Research and Evaluation in Counseling) — Threats to external validity: reactive effects of experimental arrangements

☐ Approve ☐ Revise ☐ Retire Notes:

A school counselor wants to show that a social skills intervention caused improvement for three elementary students on the autism spectrum. Once learned, the skills are not expected to fade, and withdrawing the intervention would be ethically questionable. Which design BEST fits these conditions?

A. An AB design with a single baseline and treatment phase

An AB design lacks replication, so it cannot rule out other explanations such as maturation or history.

B. An ABAB reversal design that withdraws the intervention

Reversal requires withdrawing treatment and expects behavior to return to baseline, neither of which fits here.

C. A one-group pretest-posttest design with all three students

This pre-experimental design has no control for threats such as maturation or history.

D. A multiple-baseline design staggered across the students ✓ KEY

Starting the intervention at different times for each student shows control without withdrawing treatment.

Rationale: In a multiple-baseline design, baselines begin together but the intervention is introduced at staggered points across participants, behaviors or settings. If each behavior changes only when the intervention starts for it, the counselor gains evidence of causation without a reversal. It is the design of choice when behavior is not reversible or withdrawal would be unethical; a simple AB design shows change but not experimental control.

References: Erford (Research and Evaluation in Counseling) — Single-subject designs: AB vs multiple-baseline designs

☐ Approve ☐ Revise ☐ Retire Notes:

A researcher conducts in-depth interviews with 12 parents who have lost a child to overdose. Her goal is to describe the essence of how parents live through this kind of grief, setting aside her own assumptions as she analyzes the data. Which qualitative tradition does this study reflect?

A. Grounded theory

Grounded theory aims to generate a theory of a process through constant comparison and theoretical sampling.

B. Phenomenology ✓ KEY

Describing the essence of a lived experience and bracketing researcher assumptions are hallmarks of phenomenology.

C. Ethnography

Ethnography studies the shared patterns of a culture-sharing group through prolonged fieldwork and observation.

D. Case study

A case study examines one or a few bounded systems in depth using multiple sources of data.

Rationale: Phenomenology seeks the essential structure of a lived experience shared by several people, typically through in-depth interviews, with the researcher bracketing (epoche) personal assumptions. Grounded theory builds theory from data, ethnography describes a culture, and a case study explores a bounded case in its context.

References: Erford (Research and Evaluation in Counseling) — Qualitative traditions: phenomenology, grounded theory, ethnography and case study

☐ Approve ☐ Revise ☐ Retire Notes:

After coding interviews with first-generation college students, a counselor-researcher shares her preliminary themes with the participants and asks whether the themes accurately capture their experiences. Which trustworthiness strategy is she using?

A. Triangulation

Triangulation compares multiple data sources, methods, investigators or theories, not participant review of findings.

B. Peer debriefing

Peer debriefing involves a colleague, not the participants, reviewing and questioning the analysis.

C. Member checking ✓ KEY

Returning interpretations to participants to confirm their accuracy is member checking.

D. Audit trail

An audit trail documents decisions throughout the study so an outside reviewer can follow them.

Rationale: Trustworthiness is the qualitative counterpart of validity and reliability, with criteria such as credibility, transferability, dependability and confirmability (Lincoln and Guba). Member checking is a key credibility strategy; triangulation, peer debriefing, prolonged engagement, thick description and audit trails support other criteria.

References: Erford (Research and Evaluation in Counseling) — Trustworthiness in qualitative research: member checking and triangulation

☐ Approve ☐ Revise ☐ Retire Notes:

A state counseling association plans to survey its members and wants school, clinical mental health and rehabilitation counselors represented in the sample in the same proportions as in the full membership, with members chosen at random. Which sampling method BEST meets this goal?

A. Stratified random sampling ✓ KEY

Dividing the population into subgroups and randomly sampling from each guarantees proportional representation.

B. Simple random sampling

Simple random sampling gives everyone an equal chance but does not guarantee each subgroup's proportion.

C. Convenience sampling

Convenience sampling uses whoever is easily available and is neither random nor proportional.

D. Snowball sampling

Snowball sampling recruits through referrals from participants and is used for hard-to-reach groups.

Rationale: Stratified random sampling divides a population into strata on a key characteristic and then randomly selects from each stratum, often in proportion to its size. It improves representativeness on that characteristic compared with simple random sampling, while convenience and snowball samples are nonprobability methods that limit generalization.

References: Erford (Research and Evaluation in Counseling) — Probability vs nonprobability sampling: stratified random sampling

☐ Approve ☐ Revise ☐ Retire Notes:

A researcher locates 48 published and unpublished studies of group counseling for adolescent depression and statistically combines their effect sizes to estimate an overall effect. What type of study is this?

A. A narrative literature review

A narrative review summarizes studies in words without statistically pooling their results.

B. A longitudinal cohort study

A cohort study follows one group of participants over time and collects new data.

C. A randomized controlled trial

An RCT randomly assigns its own participants to conditions rather than combining prior studies.

D. A meta-analysis ✓ **KEY**

Quantitatively pooling effect sizes across many studies to estimate an overall effect is meta-analysis.

Rationale: Meta-analysis converts the results of many studies to a common effect size metric (such as Cohen's d) and combines them, weighting larger studies more heavily. Including unpublished studies helps reduce publication bias. Meta-analyses of randomized trials are often placed at the top of evidence hierarchies used in evidence-based practice.

References: Erford (Research and Evaluation in Counseling) — Meta-analysis and effect sizes

☐ Approve ☐ Revise ☐ Retire Notes:

A college counseling center builds a logic model for a new peer-support program. Which entry belongs in the outcomes column of the model rather than in the outputs column?

A. Number of peer-support sessions held each semester

Sessions delivered are an output, a count of activity, not a change in participants.

B. Number of students who attended at least one session

Attendance counts are outputs that show reach, not program effects.

C. Lower loneliness scores among participating students ✓ **KEY**

A change in participants' loneliness is an outcome, the intended effect of the program.

D. Number of peer supporters who finished the training course

Trained supporters are a product of program activities, which makes this an output.

Rationale: A logic model links inputs (resources) to activities, outputs (direct products such as sessions held and people served), and outcomes (changes in knowledge, behavior or well-being). Process evaluation examines whether activities and outputs are delivered as planned; outcome evaluation examines whether the intended changes occurred.

References: Erford (Research and Evaluation in Counseling) — Program evaluation: logic models, process vs outcome evaluation

☐ Approve ☐ Revise ☐ Retire Notes:

The parents and teacher of an 8-year-old boy report frequent tantrums when classwork becomes frustrating. Which objective for his treatment plan is MOST measurable?

A. He will use his calm-down card instead of a tantrum in 4 of 5 hard tasks by March ✓ KEY

This names an observable replacement behavior, a success criterion (4 of 5 tasks) and a deadline.

B. He will gain better control of his emotions and feel happier at school this year

Emotional control and happiness are not defined in observable terms and lack a success criterion.

C. He will learn why tantrums upset classmates and grow more empathy toward them

Understanding and empathy are internal states with no observable measure or time frame.

D. He will meet with the school counselor every week to talk through angry feelings

Attending sessions describes the intervention, not a measurable change in the problem behavior.

Rationale: Objectives for children should describe a specific, observable behavior that teachers or parents can count, a criterion for success, and a target date. Framing the objective as a replacement behavior (using a coping tool) rather than only the absence of tantrums also gives the child something concrete to practice and adults something to reinforce.

References: Wiger (Documentation) — Treatment plans: observable, measurable objectives

► **Check:** Wiger reference detail describes the concept, not an exact chapter title.

☐ Approve ☐ Revise ☐ Retire Notes:

A 26-year-old man with schizophrenia was discharged from a psychiatric unit two weeks ago. He is taking his medication and has no suicidal or homicidal ideation, but he has missed several outpatient appointments and struggles to manage daily tasks on his own. Consistent with the least restrictive environment, which plan is MOST appropriate?

A. Readmit him to the inpatient unit until his attendance becomes consistent

He has no acute risk; hospitalizing him for missed appointments is far more restrictive than needed.

B. Add assertive community treatment or case management to support him at home ✓ KEY

Bringing intensive supports to him in the community addresses engagement and daily functioning in the least restrictive setting.

C. Arrange long-term placement in a locked residential facility for his safety

A locked setting is unjustified for a client who is stable and not dangerous to himself or others.

D. Continue monthly outpatient visits and close the case if he misses another

This ignores clear signs that his current level of support is not enough and risks disengagement.

Rationale: The least restrictive environment principle calls for providing care in the most integrated, least confining setting that can safely meet the client's needs. When a client is stable but struggling to engage and function, the first step is to intensify community-based supports, such as assertive community treatment or case management, rather than move him to a more restrictive setting.

References: Gladding (Counseling: A Comprehensive Profession) — Clinical mental health counseling: community-based care and the least restrictive setting

► **Check:** Gladding reference detail is a concept paraphrase; confirm a better-matched source if available.

☐ Approve ☐ Revise ☐ Retire Notes:

A 33-year-old client finishing treatment for alcohol use disorder is building a relapse prevention plan with her counselor. Based on the cognitive-behavioral relapse prevention model, which element is MOST central to the plan?

A. Agreeing that any single lapse means treatment failed and must start over

This all-or-nothing view feeds the abstinence violation effect, which turns lapses into full relapses.

B. Avoiding all talk of cravings so that the urges lose their power over time

Avoiding discussion of cravings leaves her unprepared; the model teaches recognizing and coping with urges.

C. Focusing the plan mainly on the past events that first led her to drink

Relapse prevention is present- and future-focused, centered on upcoming risks and skills.

D. Identifying her high-risk situations and rehearsing coping responses ✓ **KEY**

Spotting personal high-risk situations and practicing coping skills for them is the core of relapse prevention.

Rationale: Marlatt and Gordon's relapse prevention model holds that relapse risk rises when a person meets a high-risk situation without an effective coping response. Plans therefore identify triggers, build and rehearse coping skills, increase self-efficacy, and treat a lapse as a learning opportunity rather than a failure, countering the abstinence violation effect.

References: Corey (Theory & Practice) — Cognitive behavioral approaches: relapse prevention, high-risk situations and coping skills

► **Check:** Confirm the Corey edition in use covers relapse prevention; Marlatt & Gordon is the primary source but is not on the approved list.

☐ Approve ☐ Revise ☐ Retire Notes:

An interdisciplinary team at an adolescent residential program decides that clients who miss group will lose family phone calls for a week. The counselor on the team believes this decision raises ethical concerns. What should the counselor do FIRST?

A. Report the team to the state licensing board for unethical practice

Outside avenues are pursued only after attempts to resolve the concern within the team have failed.

B. Tell the clients' families privately that she disagrees with the team

Going around the team to families undermines the team and does not address the decision itself.

C. Raise the concern with the team and try to resolve it within the team ✓ **KEY**

The ACA Code directs counselors to first attempt to resolve ethical concerns about team decisions within the team.

D. Follow the decision without comment, since team consensus outranks her

Counselors keep their own ethical obligations on a team and may not simply defer to consensus.

Rationale: Counselors on interdisciplinary teams keep the focus on how best to serve clients and clarify the professional and ethical obligations of the team and its members. When a team decision raises ethical concerns, they first try to resolve the concern within the team; if that fails, they pursue other avenues consistent with client well-being.

References: ACA Code of Ethics — D.1.d Establishing Professional and Ethical Obligations

► **Check:** Confirm the section number D.1.d for resolving team ethical concerns within the team (vs D.1.c Interdisciplinary Teamwork).

☐ Approve ☐ Revise ☐ Retire Notes:

A 31-year-old graduate student has had recurrent unexpected panic attacks for six months. She now avoids crowded buses and worries constantly that her racing heart signals a heart attack. Medical causes have been ruled out. Which psychotherapy approach is the BEST first-line choice?

A. Supportive listening that steers away from discussing body sensations

Avoiding feared sensations maintains the fear of them; nondirective support is not a first-line treatment for panic disorder.

B. Cognitive behavioral therapy that includes interoceptive exposure ✓ **KEY**

CBT with psychoeducation, cognitive restructuring of catastrophic misinterpretations, and interoceptive and situational exposure is the best-supported psychotherapy for panic disorder.

C. Helping her plan routes that avoid the situations where attacks occur

Planned avoidance reinforces agoraphobic avoidance and the belief that the situations are dangerous.

D. Dream analysis aimed at uncovering the unconscious source of her fear

Dream analysis is not an evidence-based first-line treatment for panic disorder and does not target the panic cycle.

Rationale: Panic disorder is maintained by catastrophic misinterpretation of bodily sensations and by avoidance. Cognitive behavioral treatment, such as Panic Control Treatment, combines psychoeducation, cognitive restructuring, and exposure to feared body sensations (interoceptive exposure) and situations. Guidelines name CBT as the first-line psychological treatment.

References: Barlow PCT — Panic Control Treatment: interoceptive exposure and cognitive restructuring · NICE guidelines — CG113 Generalised anxiety disorder and panic disorder in adults: CBT for panic disorder

☐ Approve ☐ Revise ☐ Retire Notes:

A 24-year-old client meets criteria for borderline personality disorder. She cuts herself several times a month during intense emotional storms, and reducing self-harm is her top priority. Which treatment has the strongest research support for this presentation?

A. Brief solution-focused therapy limited to three sessions

A three-session brief model does not provide the skills training and structure needed for recurrent self-harm in BPD.

B. Prolonged exposure therapy targeting traumatic memories

Prolonged exposure targets PTSD; starting it with uncontrolled self-harm is not the first-line approach for BPD.

C. A psychoeducational group on personality disorder myths

Psychoeducation alone does not teach the emotion regulation and distress tolerance skills that reduce self-harm.

D. Comprehensive dialectical behavior therapy (DBT) program ✓ **KEY**

Comprehensive DBT (individual therapy, skills group, phone coaching, consultation team) has the strongest evidence for reducing self-harm in BPD.

Rationale: Dialectical behavior therapy was developed for chronically suicidal and self-harming clients with borderline personality disorder. Its comprehensive form combines individual therapy, skills training in mindfulness, distress tolerance, emotion regulation and interpersonal effectiveness, between-session coaching, and a therapist consultation team. Guidelines recommend it when reducing recurrent self-harm is a priority.

References: NICE guidelines — CG78 Borderline personality disorder: consider a comprehensive DBT programme when reducing recurrent self-harm is a priority

☐ Approve ☐ Revise ☐ Retire Notes:

A clinical supervisor is reviewing the common factors versus specific factors debate with a counselor-in-training. Across comparisons of well-established (bona fide) therapies, which finding is MOST consistent with the common factors research?

A. Alliance quality predicts outcome more than which model is used ✓ KEY

Meta-analyses find the alliance reliably predicts outcome, while differences among bona fide treatments are small.

B. Specific techniques explain most of the variance in outcomes

Specific ingredients account for a small share of outcome variance compared with relational and client factors.

C. Counselor years of experience is the strongest single predictor

Years of experience is a weak predictor of outcome; it does not outweigh the alliance.

D. Matching the model to the diagnosis removes therapist effects

Therapist effects persist across models; some counselors get better results regardless of the approach used.

Rationale: The common factors view (Rosenzweig, Frank, Lambert, Wampold) holds that elements shared by effective therapies, such as the alliance, empathy, expectancy and a credible rationale, account for much of the change. Comparisons of bona fide treatments show small differences, while the alliance consistently predicts outcome. This does not make specific methods irrelevant; many conditions, such as PTSD and panic, have treatments with specific evidence.

References: Corey (Theory & Practice) — Integrative approaches: common factors and the therapeutic relationship as a predictor of outcome

► **Check:** Common factors findings (Wampold & Imel; Lambert) are well established, but confirm the Corey edition discusses common factors research explicitly.

☐ Approve ☐ Revise ☐ Retire Notes:

A 45-year-old man with obsessive-compulsive disorder is nervous about exposure and response prevention and asks whether there are other options. Which response BEST reflects shared decision making?

A. Tell him ERP is the evidence-based choice, so the plan cannot change

Insisting on one option without exploring his concerns and preferences removes him from the decision.

B. Let him choose any approach he likes without discussing the evidence

Shared decision making requires the counselor to bring evidence; deferring entirely abandons that role.

C. Review the options, evidence and his concerns, then decide jointly ✓ KEY

Shared decision making combines the counselor's knowledge of the evidence with the client's values and preferences in a joint choice.

D. Postpone treatment planning until his anxiety about ERP has resolved

Delaying planning leaves him untreated; fear of ERP is something to discuss and plan for, not wait out.

Rationale: Evidence-based practice integrates the best research evidence, clinical expertise and client characteristics, values and preferences. In shared decision making the counselor explains the options with their benefits and burdens, explores the client's concerns, and arrives at a plan together. Here that might include explaining how ERP is graded and client-paced.

References: APA CPG — Evidence-based practice: integrating patient values and preferences into treatment decisions

☐ Approve ☐ Revise ☐ Retire Notes:

A 19-year-old Mexican American client with depression tells her counselor that decisions in her family are made together and that she wants her mother involved in her care. Which approach to the treatment plan is MOST culturally responsive?

A. Include her mother in planning as she wishes, keeping core methods ✓ **KEY**

Cultural adaptation honors her value of family involvement while keeping the active ingredients of effective treatment.

B. Keep the plan individual, since adult clients decide treatment alone

This imposes an individualistic worldview and ignores the client's own stated wish for family involvement.

C. Assume every Latina client prefers family therapy over individual work

This stereotypes the client; adaptation should follow her stated preferences, not group generalizations.

D. Ask her mother to set the goals because she is the family decision maker

The client remains the decision maker about her own care; involving family does not mean handing over her goals.

Rationale: Culturally adapted treatment adjusts how a plan is delivered (who is involved, language, metaphors, goals framed in the client's values) while preserving the core components of effective treatment. The adaptation should be driven by the individual client's stated values, here familismo and collective decision making, rather than by assumptions about a group.

References: Sue & Sue (Counseling the Culturally Diverse) — Culturally appropriate intervention strategies: collectivism and family involvement · MSJCC (Ratts et al., 2016) — Counseling and advocacy interventions: integrating client worldview into counseling

☐ Approve ☐ Revise ☐ Retire Notes:

A group practice announces that it is adopting measurement-based care. Which practice BEST describes this approach?

A. Giving a symptom inventory at intake and again at discharge only

Pre-post measurement evaluates results after the fact but cannot guide treatment while it is happening.

B. Routinely scoring brief measures and using them to guide treatment ✓ **KEY**

Measurement-based care means regular standardized measurement whose results are reviewed with the client and used to adjust treatment.

C. Basing progress notes on the counselor's sense of client improvement

Clinical judgment alone tends to miss clients who are not improving or are getting worse.

D. Sending yearly satisfaction surveys to all of the practice's clients

Satisfaction surveys are program evaluation; they do not track an individual client's symptoms over time.

Rationale: Measurement-based care is the systematic administration of brief, validated measures (for example, the PHQ-9 or GAD-7) at regular intervals, with results reviewed with the client and used to inform clinical decisions. It improves detection of clients who are not responding.

References: Hays (Assessment) — Use of assessment in counseling: monitoring client progress with standardized measures

☐ Approve ☐ Revise ☐ Retire Notes:

A counseling center begins giving the Outcome Questionnaire-45 (OQ-45) before every session and sends counselors color-coded alerts based on expected recovery curves. According to research on this kind of routine outcome monitoring with feedback, which clients benefit MOST?

A. Clients who are improving faster than their expected recovery curve

On-track clients do about as well with or without feedback; they are not where the largest gains appear.

B. Clients whose intake scores fell within the normal, nonclinical range

Clients starting in the functional range have little room to improve, so feedback adds little for them.

C. Clients in brief treatment who are seen for only one or two sessions

Feedback needs repeated measurement over time to identify a trajectory and prompt a change in course.

D. Clients flagged as not on track, who are at risk of deteriorating ✓ **KEY**

Feedback alerts help counselors notice and respond to not-on-track clients, improving their outcomes and reducing deterioration.

Rationale: The OQ-45 measures symptom distress, interpersonal relations and social role functioning. Lambert and colleagues found that giving counselors feedback on clients predicted to be not on track, especially paired with clinical support tools that review alliance, motivation and supports, improved outcomes and reduced deterioration. Counselors on their own tend to underestimate which clients are getting worse.

References: Lambert (Outcome Monitoring) — OQ-45 feedback and alerts for clients not on track · Hays (Assessment) — Outcome assessment and routine outcome monitoring

☐ Approve ☐ Revise ☐ Retire Notes:

A 56-year-old client's scores on a weekly anxiety measure have risen by more than the measure's reliable change index since intake, and she arrives at session five looking tense. How should the counselor respond to this deterioration?

A. Keep the current plan, since symptoms often worsen before improving

Assuming early worsening is expected ignores the warning sign; reliable deterioration predicts poor outcome if unaddressed.

B. Discharge her and refer elsewhere because treatment is not working

Abrupt discharge without exploring the cause is premature and risks abandonment.

C. Review scores with her, explore causes and revise the plan together ✓ **KEY**

Sharing the data, checking the alliance, stressors and fit of the plan, and adjusting collaboratively is the recommended response.

D. Stop giving the measure, because her rising scores may upset her more

Dropping measurement removes the information needed to respond; the scores should be discussed openly.

Rationale: Reliable deterioration on an outcome measure is a signal to act, not to wait. The counselor discusses the change with the client, assesses risk, explores the alliance, life events and treatment fit, and adjusts the plan, consulting or referring for added services if needed. Documenting the change and the response is part of good practice.

References: Hays (Assessment) — Monitoring progress: reliable change and responding to nonresponse · Wiger (Documentation) — Progress notes and treatment plan updates when progress stalls or reverses

☐ Approve ☐ Revise ☐ Retire Notes:

A 28-year-old client had a suicidal crisis over the weekend. He did not use his safety plan, saying it "didn't feel real" and that he had forgotten where he put it. He is now safe and engaged. What should the counselor do with the plan?

A. Revise it with him, find the barriers and make each step more usable ✓ KEY

Safety plans are living documents; reviewing what got in the way and rewriting steps in his words makes the plan more usable.

B. Replace it with a written no-suicide contract that he agrees to sign

No-suicide contracts lack evidence of preventing suicide and do not give the client coping steps.

C. Set the plan aside, since he has shown that safety planning fails him

One unused plan points to barriers to fix, not a reason to abandon an evidence-supported intervention.

D. Keep the original plan but tell him to read it once a day as homework

Repeating an unchanged plan ignores why it failed; it needs collaborative revision first.

Rationale: The Stanley-Brown Safety Planning Intervention is developed collaboratively, written in the client's own words, kept somewhere easy to reach (such as a phone), and reviewed and revised after crises. Asking what got in the way of using each step lets the counselor and client make the plan realistic.

References: Stanley-Brown SPI — Reviewing and revising the safety plan; identifying barriers to using each step

☐ Approve ☐ Revise ☐ Retire Notes:

While adding crisis resources to a safety plan, a 17-year-old client asks what happens if she contacts 988. Which description is accurate?

A. It is a line that sends police to the caller's location on every contact

Most 988 contacts are resolved by phone, text or chat; emergency dispatch is reserved for imminent danger.

B. It connects her by call, text or chat to trained crisis counselors 24/7 ✓ KEY

988 Suicide and Crisis Lifeline offers call, text and chat access to trained crisis counselors at any hour.

C. It is reserved for people who are in the middle of a suicide attempt

988 serves anyone in suicidal, mental health or substance use crisis, or worried about someone else.

D. It is a text-only service that answers messages during business hours

988 is available by phone, text and chat around the clock.

Rationale: The 988 Suicide and Crisis Lifeline became the national three-digit number in the United States in July 2022. It routes calls, texts and chats to trained crisis counselors 24 hours a day. In a Stanley-Brown safety plan it belongs in the step listing professionals and agencies to contact in a crisis.

References: SAMHSA 988 Lifeline — 988 by call, text or chat, 24/7 · Stanley-Brown SPI — Step 5: professionals and agencies to contact in a crisis

☐ Approve ☐ Revise ☐ Retire Notes:

A counselor writes this objective for a client with panic disorder: "Client will reduce panic attacks from four per week to one or fewer per week, as recorded in a daily panic log." Which SMART element is missing?

A. A specific behavior that is targeted for change

The objective names a specific behavior: the frequency of panic attacks.

B. A way to measure progress toward the objective

The daily panic log and the counts per week make the objective measurable.

C. A target that is realistic for this client

Reducing attacks to one or fewer per week is a reasonable, attainable target for treated panic disorder.

D. A time frame for reaching the stated target ✓ **KEY**

The objective gives no date or number of weeks, so it is not time-bound.

Rationale: SMART objectives are specific, measurable, achievable (attainable), relevant and time-bound. Adding a time frame, such as "within 10 weeks," tells the client and counselor when to judge whether the objective was met and supports documentation of medical necessity and progress.

References: Wiger (Documentation) — Treatment plans: writing behaviorally measurable, time-bound objectives

☐ Approve ☐ Revise ☐ Retire Notes:

A 34-year-old client with severe depression has worsened despite three weeks in an intensive outpatient program (three hours, three days a week). He is not suicidal, lives with a supportive spouse, and can stay safe at night, but he needs structured treatment most of each weekday. Which level of care BEST fits?

A. Weekly individual outpatient counseling plus a medication review

This is less intensive than the IOP he is already failing in, so it would be a step down when he needs a step up.

B. Continued IOP at the same schedule for another three weeks

He is worsening at this intensity; continuing unchanged does not match his increased need.

C. A partial hospitalization program attended most of each weekday ✓ **KEY**

PHP offers several hours of daily structured treatment while he returns home at night, matching his need without 24-hour care.

D. Admission to a locked inpatient psychiatric unit for stabilization

He has no imminent risk and can stay safe at home, so 24-hour inpatient care would be more restrictive than needed.

Rationale: Levels of care run from outpatient to intensive outpatient (about 9 or more hours a week for adults), partial hospitalization (about 20 or more hours a week, usually most of each weekday), and 24-hour residential or inpatient care. The best match is the least restrictive level that meets current need; worsening in IOP without imminent danger points to PHP.

References: LOCUS (AACP) — Level-of-care intensity: intensive outpatient versus partial hospitalization

☐ Approve ☐ Revise ☐ Retire Notes:

Which client presentation MOST clearly meets criteria for inpatient psychiatric hospitalization rather than a less restrictive level of care?

A. A man with chronic passive death wishes, no plan, and strong supports

Passive ideation without plan or intent, with supports, can usually be managed in outpatient care with safety planning.

B. A woman with a plan, intent and access to means who cannot stay safe ✓ **KEY**

Imminent risk with a plan, intent, access to means and inability to maintain safety requires 24-hour protective care.

C. A teen with worsening grades and conflict who refuses to go to school

Functional decline without imminent danger calls for outpatient or intensive outpatient services, not hospitalization.

D. A man with panic attacks who missed work twice this month from fear

Panic with some missed work is treated effectively on an outpatient basis.

Rationale: Inpatient hospitalization is reserved for clients who are an imminent danger to themselves or others, or who are so impaired they cannot care for themselves, and who cannot be kept safe at a less restrictive level. Suicidal ideation with a plan, intent, access to means and an inability to engage in safety planning is the classic indication.

References: Gladding (Counseling: A Comprehensive Profession) — Crisis counseling: suicide risk and the decision to hospitalize

☐ Approve ☐ Revise ☐ Retire Notes:

A 20-year-old client in counseling for perfectionism mentions that she has been vomiting after most meals, has lost 15 pounds in two months, and feels dizzy when she stands up. The counselor has no eating disorder training. What is the BEST course of action?

A. Continue the current work and add eating concerns to future goals

Purging with rapid weight loss and dizziness signals possible medical danger that cannot wait for future sessions.

B. End counseling now and give her a list of eating disorder programs

Abruptly ending services without a coordinated transition risks abandonment.

C. Read about eating disorders and begin treating them in next session

Reading alone does not provide the training and supervision needed to practice within competence.

D. Arrange a prompt medical exam and specialist referral with a release ✓ **KEY**

Her symptoms need urgent medical evaluation, and care should be coordinated with a qualified specialist with her consent.

Rationale: Purging, rapid weight loss and dizziness can indicate electrolyte imbalance or other medical instability, so a prompt medical evaluation comes first. Because eating disorder treatment falls outside this counselor's competence, the counselor coordinates a referral to a qualified provider with a signed release, supporting the transition rather than abandoning the client.

References: ACA Code of Ethics — C.2.a Boundaries of Competence; A.11.a Competence Within Termination and Referral

☐ Approve ☐ Revise ☐ Retire Notes:

A 42-year-old client is completing an intensive outpatient program for depression and will step down to weekly counseling. Which aftercare step does the MOST to reduce the risk that he drops out after discharge?

A. Book his first outpatient visit with a named counselor before he leaves ✓ KEY

A confirmed appointment with a named provider, made before discharge, is a warm handoff that improves follow-through.

B. Give him a list of local counselors to call once he feels he needs one

Leaving the client to make contact on his own increases the chance that no follow-up happens.

C. Tell him to return to the program if symptoms come back after discharge

This plans only for relapse and leaves a gap in continuing care.

D. Mail a copy of his discharge summary to his home address after he leaves

A summary sent to the client does not connect him to the next provider.

Rationale: The period right after leaving a more intensive level of care carries elevated risk of disengagement and relapse. Effective aftercare planning schedules the next appointment before discharge, shares records with the receiving provider with consent, and updates the safety and relapse prevention plans.

References: Wiger (Documentation) — Discharge summaries and aftercare recommendations

► **Check:** Warm-handoff and pre-discharge scheduling are standard continuity-of-care practice; confirm Wiger is the best fit or cite a SAMHSA continuity-of-care source.

☐ Approve ☐ Revise ☐ Retire Notes:

A 50-year-old client was referred by his physician for heavy cannabis use. He says, "It helps me relax. I don't see what the problem is." Which intervention BEST matches his stage of change?

A. Help him set a quit date and list strategies for handling cravings

Quit dates and coping plans are action-stage tasks; pushing them now tends to raise resistance.

B. Teach relapse prevention skills for high-risk situations he may face

Relapse prevention fits the action and maintenance stages, after change has begun.

C. Build rapport and offer nonjudgmental feedback to raise awareness ✓ KEY

In precontemplation, consciousness raising and nonconfrontational feedback help the client begin to consider change.

D. Confront his denial firmly so that he accepts that he has a problem

Confrontation increases defensiveness and is not supported for clients in precontemplation.

Rationale: A client who sees no problem is in precontemplation. Stage-matched work at this point builds the relationship, raises awareness through information and personalized feedback, and explores the client's own view of the behavior. Action-oriented tasks fit later stages.

References: Transtheoretical Model — Precontemplation: consciousness raising and stage-matched processes of change · Miller & Rollnick (MI) — Avoiding the righting reflex and reducing discord

☐ Approve ☐ Revise ☐ Retire Notes:

A counselor uses a decisional balance exercise with a client who is weighing whether to stop gambling. According to research on the Transtheoretical Model, how does decisional balance typically shift as people move from precontemplation toward action?

A. The cons of changing rise steadily until they overtake the pros

This reverses the pattern; perceived cons of change tend to decrease as people progress.

B. The pros of changing increase until they come to outweigh the cons ✓ **KEY**

Pros of change rise across stages and typically cross over the cons before the action stage.

C. Pros and cons both stay stable, and only self-efficacy shifts

Self-efficacy does rise, but decisional balance also shifts in a predictable way across stages.

D. Pros and cons drop together as the client stops thinking about it

Movement toward action involves more, not less, weighing of the pros of change.

Rationale: Decisional balance is the weighing of the pros and cons of changing. In precontemplation the cons outweigh the pros; the two are roughly balanced in contemplation, which feeds ambivalence; and the pros come to outweigh the cons before action. Prochaska described the pros as rising more steeply than the cons fall (the strong and weak principles).

References: Transtheoretical Model — Decisional balance across the stages of change; strong and weak principles of progress

☐ Approve ☐ Revise ☐ Retire Notes:

A counselor-in-training writes "Client will have healthier relationships" as the only entry under objectives in a treatment plan. Her supervisor explains how goals and objectives differ. Which statement BEST describes that difference?

A. A goal states a broad aim; objectives are measurable steps toward it ✓ **KEY**

Goals describe the desired end state in broad terms; objectives break the goal into specific, observable, time-limited steps that show progress.

B. A goal is set by the counselor; objectives are chosen by the client

Both goals and objectives are developed collaboratively; authorship is not what separates them.

C. A goal lists the interventions; objectives list the diagnoses treated

Interventions and diagnoses are separate parts of the plan; neither defines the goal or the objective.

D. A goal is reached in one session; objectives take the full treatment

This reverses the relationship: objectives are the shorter-term steps, and the goal is the longer-term outcome.

Rationale: In treatment planning, a goal is a broad, long-term statement of what the client hopes to achieve, such as healthier relationships. Objectives translate that goal into specific, measurable, time-bound steps (for example, using an "I" statement in two disagreements per week for four weeks, recorded in a log). The supervisee's entry is a goal, so it still needs measurable objectives beneath it.

References: Wiger (Documentation) — Treatment plans: distinguishing long-term goals from measurable short-term objectives

☐ Approve ☐ Revise ☐ Retire Notes:

A 27-year-old client with severe social anxiety rarely speaks in groups larger than three people. At intake, her counselor writes the objective: "Client will give a 10-minute presentation to her 200-person department within two weeks." Which SMART criterion does this objective MOST clearly fail?

A. Specific, because it does not say which behavior will change

The behavior (a 10-minute presentation to a named audience) is clearly specified.

B. Measurable, because a presentation cannot be counted or observed

Giving a presentation is directly observable and easy to verify, so the objective is measurable.

C. Achievable, because the step is too large for her starting point ✓ KEY

Jumping from avoiding groups of three to presenting to 200 people in two weeks is not realistic; graded steps are needed.

D. Time-bound, because it does not include any deadline for change

The objective includes a clear deadline of two weeks, so it is time-bound.

Rationale: SMART objectives are Specific, Measurable, Achievable (attainable), Relevant and Time-bound. An objective can be precise and measurable yet still fail if it is not realistic given the client's current functioning. Setting an unattainable first step invites failure and discouragement; a graded series of objectives, such as speaking once in a small meeting, then a larger one, fits the client's starting point and the logic of gradual exposure.

References: Wiger (Documentation) — Treatment plans: objectives that are realistic and attainable for the client's current level of functioning

☐ Approve ☐ Revise ☐ Retire Notes:

A 44-year-old man seeks counseling for stress after losing his job. During the intake, he mentions drinking six to eight beers most nights but says, "That is not why I am here." The counselor believes the drinking is a major concern. How should the counselor approach the treatment plan?

A. List alcohol abstinence as the primary goal since it is most urgent

Imposing a goal the client has rejected undermines the alliance and is likely to increase resistance and dropout.

B. Decline to treat the job stress until he agrees to address drinking

Making care conditional on accepting the counselor's agenda is coercive and ignores the client's stated needs.

C. Leave drinking out of the plan and the sessions to keep the alliance

Ignoring a clinically significant concern avoids conflict but fails the client; it should be explored respectfully.

D. Start with his job stress goal and openly explore links to drinking ✓ KEY

Beginning with the client's priority builds engagement while the counselor transparently shares the concern and explores how drinking affects his stress.

Rationale: Collaborative treatment planning starts from the goals the client brings while allowing the counselor to share clinical concerns honestly. Here the counselor can begin with job-loss stress, voice the concern about drinking without imposing it, and use a motivational approach to explore how alcohol affects sleep, mood and coping. Plans built jointly and consistent with the client's circumstances are more likely to succeed.

References: ACA Code of Ethics — A.1.c Counseling Plans: counselors and clients work jointly to devise counseling plans · Miller & Rollnick (MI) — Focusing process: negotiating a shared agenda when client and counselor priorities differ

☐ Approve ☐ Revise ☐ Retire Notes:

A 63-year-old Vietnamese immigrant client nods and agrees to every goal the counselor proposes during treatment planning, then misses the next two sessions. She has said she was raised to defer to doctors and teachers. Which response is MOST likely to produce a truly collaborative plan?

A. Ask her to sign the plan as written since she already agreed to it

Her agreement may reflect deference rather than true endorsement; a signature does not make the plan collaborative.

B. Invite her priorities indirectly, such as what she hopes will change ✓ **KEY**

Open, respectful questions about her own hopes let her shape the plan without having to openly contradict an authority figure.

C. Refer her to a counselor who is also from a Vietnamese background

Shared ethnicity does not guarantee a good match, and referral skips the counselor's own task of adapting the process.

D. Let her family set the treatment goals because she defers to others

Family input may be welcome if she wants it, but handing goal-setting to others removes her voice entirely.

Rationale: Agreement is not the same as collaboration. When a client's cultural norms discourage disagreeing with authority, quick assent to every proposal and later missed sessions suggest the plan does not reflect her priorities. Culturally responsive planning reduces the power gap by asking about her hopes, concerns and preferred pace in indirect, respectful ways, checking understanding, and revising the plan with her.

References: Sue & Sue (Counseling the Culturally Diverse) — Culturally appropriate intervention: deference to authority and indirect communication styles · ACA Code of Ethics — A.1.c Counseling Plans: plans consistent with the abilities, temperament and circumstances of clients

☐ Approve ☐ Revise ☐ Retire Notes:

A counselor works as a behavioral health care manager in a primary care clinic that uses the collaborative care model. She tracks every patient's PHQ-9 scores in a registry and reviews the caseload weekly with a consulting psychiatrist. What is the main purpose of the registry review?

A. To spot patients who are not improving so treatment can be adjusted ✓ **KEY**

Systematic caseload review flags patients not responding so the team can change treatment, the treat-to-target core of collaborative care.

B. To transfer every depressed patient to the psychiatrist's own care

In collaborative care the psychiatrist consults on the caseload; patients stay with their primary care provider and care manager.

C. To produce billing records that replace the clinical progress notes

A registry supports clinical decision-making; it does not replace progress notes or exist mainly for billing.

D. To find patients who no longer need any primary care involvement

The model keeps behavioral health care inside primary care; it is not designed to remove patients from it.

Rationale: Integrated behavioral health in primary care often follows the collaborative care model: a primary care provider, a behavioral health care manager and a consulting psychiatrist share a defined caseload. Measurement-based tracking in a registry and regular systematic caseload review identify patients who are not reaching targets, prompting changes in medication or psychotherapy. This treat-to-target approach is a key reason the model improves depression outcomes.

References: Gladding (Counseling: A Comprehensive Profession) — Clinical mental health counseling settings: integrated behavioral health in primary care · Lambert (Outcome Monitoring) — Measuring and monitoring progress to identify clients not on track

► **Check:** Collaborative care (registry, care manager, psychiatric consultant, treat-to-target) is well established in the AIMS/Unutzer literature; confirm Gladding edition discusses integrated care in this detail or swap to a more specific source if added to the library.

☐ Approve ☐ Revise ☐ Retire Notes:

During a routine visit, a family physician learns that a 35-year-old patient has been having panic attacks. She walks the patient down the hall, introduces him by name to the clinic's counselor, and explains why she thinks they should meet that same day. What is this practice called?

A. A cold referral

A cold referral gives the patient a name or number to call later, with no direct introduction.

B. A case conference

A case conference is a meeting among professionals about a client, not a live introduction of the client.

C. A warm handoff ✓ KEY

A warm handoff is a direct, in-person introduction from one provider to another, ideally during the same visit.

D. A consultation

Consultation is professional-to-professional advice; the patient is not transferred to the consultant's care.

Rationale: A warm handoff occurs when one provider personally introduces a patient to another, explaining the reason for the connection, often in the same visit. Compared with handing the patient a phone number, it transfers the trust built with the first provider, reduces stigma and logistical barriers, and increases the chance that the patient actually begins behavioral health care.

References: Gladding (Counseling: A Comprehensive Profession) — Clinical mental health counseling settings: referral and integrated care with medical providers

► **Check:** Confirm the Gladding edition names the warm handoff explicitly; the concept is standard in integrated-care literature (e.g., AHRQ).

☐ Approve ☐ Revise ☐ Retire Notes:

A 39-year-old client has met her treatment goals for generalized anxiety and will be discharged from an outpatient agency. Her counselor is preparing the discharge summary. Which entry is MOST essential to include?

A. A full transcript of the client's comments during the final session

Verbatim session content is not required and adds detail that is not needed for continuity of care.

B. Progress on each treatment goal and the aftercare recommendations ✓ KEY

A discharge summary documents why treatment ended, progress on each goal, status at discharge and the aftercare plan.

C. The counselor's personal reactions to the client across treatment

Counselor reactions belong in supervision, not in the client record that may follow her to future providers.

D. A copy of every weekly anxiety score the client has ever obtained

Scores are already in the record; the summary reports overall change rather than reproducing every data point.

Rationale: A discharge (closing) summary gives future providers and reviewers a concise picture of the episode of care: reason for referral and for discharge, diagnosis, services provided, progress toward each treatment-plan goal, condition at discharge, and specific aftercare recommendations and referrals. Linking discharge to the plan's goals shows that treatment ended for a clinical reason and supports continuity of care.

References: Wiger (Documentation) — Discharge summary: reason for discharge, progress toward goals, and aftercare recommendations

☐ Approve ☐ Revise ☐ Retire Notes:

A 46-year-old client with recurring suicidal thoughts keeps a handgun in his bedside drawer. He has no current plan or intent and is working on a safety plan with his counselor. He says the gun is for protection and he does not want to sell it. Which approach is MOST consistent with best practice?

A. Ask him to sign a contract promising not to harm himself with the gun

No-suicide contracts lack evidence of preventing suicide and do nothing to reduce access to lethal means.

B. Accept his choice and leave lethal means out of the safety plan to respect autonomy

Respecting autonomy does not mean skipping lethal means counseling; access to a firearm is a major risk factor.

C. Tell him counseling cannot continue until the firearm leaves the home

An ultimatum damages the alliance and may push him out of care, which raises rather than lowers risk.

D. Explore temporary off-site storage with someone he trusts during a crisis ✓ **KEY**

Collaborative lethal means counseling seeks temporary, acceptable ways to put time and distance between the client and the firearm.

Rationale: Making the environment safe is a core step of the Stanley-Brown Safety Planning Intervention. Lethal means counseling works collaboratively, respecting the client's values while finding acceptable options, such as temporary storage with a trusted friend, a locked safe with the key held by someone else, or ammunition stored separately. Because suicidal crises are often brief, added time and distance from a firearm can be lifesaving.

References: Stanley-Brown SPI — Step 6: Making the environment safe (lethal means counseling)

☐ Approve ☐ Revise ☐ Retire Notes:

An agency compares two depression programs by dividing each program's total cost by the number of clients who achieved remission, finding \$2,400 per remission for one and \$3,100 for the other. Outcomes are not converted to dollar values. Which type of analysis is this?

A. Cost-benefit analysis

Cost-benefit analysis expresses outcomes in dollars (such as savings from fewer hospital days) and compares them with costs.

B. Summative needs assessment

A needs assessment identifies unmet needs before or during planning; it does not compare program costs per outcome.

C. Cost-effectiveness analysis ✓ **KEY**

Cost-effectiveness analysis compares programs on cost per unit of a nonmonetary outcome, such as cost per remission.

D. Formative process evaluation

Formative or process evaluation examines how a program is running so it can be improved, not its cost per outcome.

Rationale: Both analyses weigh costs against results, but they express results differently. Cost-benefit analysis converts outcomes into monetary terms so that benefits and costs can be compared directly, often as a benefit-cost ratio. Cost-effectiveness analysis keeps outcomes in natural units, such as remissions or symptom-free days, and reports the cost per unit of outcome, which suits counseling programs whose results are hard to price.

References: Erford (Research and Evaluation in Counseling) — Program evaluation: cost-benefit versus cost-effectiveness analysis

☐ Approve ☐ Revise ☐ Retire Notes:

A school district adopts an evidence-based bullying prevention curriculum, but after one year bullying reports have not declined. Teachers report that they often skipped lessons to make time for test preparation. What should the program evaluator examine FIRST?

A. Whether the curriculum's original research trials were published

The curriculum is already evidence-based; reviewing its publication history does not explain the local result.

B. Whether the curriculum was delivered with fidelity as designed ✓ **KEY**

Skipped lessons suggest poor implementation fidelity; a program not delivered as designed cannot be judged ineffective.

C. Whether a different curriculum would cost the district less

Cost comparisons may come later, but they do not address why this program did not produce the expected outcome.

D. Whether students liked the curriculum more than last year's

Satisfaction is a weak proxy for outcome and does not address the reported gaps in delivery.

Rationale: Fidelity (treatment integrity) monitoring checks whether a program is delivered as intended in dose, content and quality. When outcomes disappoint, evaluators first ask whether the program was actually implemented; otherwise a failure of delivery is mistaken for a failure of the program. Involving stakeholders such as teachers, students and administrators in the evaluation helps uncover implementation barriers like competing demands on class time.

References: Erford (Research and Evaluation in Counseling) — Program evaluation: implementation (process) evaluation and treatment fidelity; stakeholder involvement

☐ Approve ☐ Revise ☐ Retire Notes:

A researcher randomly assigns college students to receive either four or eight sessions of mindfulness training and then compares their scores on a perceived stress scale. In this study, what is the dependent variable?

A. Scores on the perceived stress scale ✓ **KEY**

The dependent variable is the outcome measured to see whether it changes in response to the manipulated variable.

B. The number of mindfulness sessions

The number of sessions is manipulated by the researcher, which makes it the independent variable.

C. The random assignment of the students

Random assignment is a design feature that controls for group differences; it is not a variable being measured.

D. The college the students are enrolled in

The college is a constant or possible extraneous variable, not the outcome under study.

Rationale: The independent variable is what the researcher manipulates or uses to form groups (here, four versus eight sessions). The dependent variable is the outcome measured to see whether it depends on the independent variable (here, perceived stress scores). A quick test: the dependent variable is the effect, measured after the cause is varied.

References: Erford (Research and Evaluation in Counseling) — Research design: independent and dependent variables

☐ Approve ☐ Revise ☐ Retire Notes:

A college counseling center compares students who signed up for a new resilience workshop with students who chose not to attend. At pretest, workshop students already reported more social support and higher motivation. Workshop students show greater gains at posttest. Which threat to internal validity is MOST important here?

A. Maturation of both groups over the full semester

Maturation would affect both groups similarly over the same period, so it does not explain the difference between groups.

B. Testing effects from taking the same pretest

Both groups took the pretest, so practice effects would not explain why one group gained more.

C. Instrumentation changes in the outcome measure

Nothing suggests the measure or its scoring changed between pretest and posttest.

D. Selection bias from groups that chose themselves ✓ KEY

Students chose their own group and differed at pretest, so preexisting differences may explain the gains.

Rationale: Selection is the main threat in quasi-experimental designs that compare nonequivalent groups without random assignment. When participants choose whether to receive the intervention, the groups can differ in motivation, support or other traits that also affect the outcome. Pretest differences are a warning sign; random assignment, matching or statistical controls such as ANCOVA reduce, but in quasi-experiments do not eliminate, this threat.

References: Erford (Research and Evaluation in Counseling) — Quasi-experimental designs: selection as a threat to internal validity

☐ Approve ☐ Revise ☐ Retire Notes:

A researcher first analyzes outcome scores from 200 clients in a grief support program. She then interviews 15 clients whose scores did not improve to understand why the program did not help them. Which research design is she using?

A. An exploratory sequential mixed methods design

An exploratory sequential design starts with qualitative data and uses it to build or guide a later quantitative phase.

B. An explanatory sequential mixed methods design ✓ KEY

An explanatory sequential design collects quantitative data first, then qualitative data to explain the quantitative results.

C. A convergent parallel mixed methods study design

A convergent design collects both types of data at about the same time and compares them, rather than in sequence.

D. A practitioner-led action research design

Action research is cyclical inquiry by practitioners to improve their own practice; the defining feature here is the quant-then-qual sequence.

Rationale: Mixed methods research combines quantitative and qualitative data in one study. In an explanatory sequential design, the quantitative phase comes first and the qualitative phase follows to explain unexpected or important quantitative findings, such as why some clients did not improve. In an exploratory sequential design the order is reversed, and in a convergent design both strands are collected together and merged.

References: Erford (Research and Evaluation in Counseling) — Mixed methods designs: explanatory sequential, exploratory sequential and convergent

► **Check:** Design names follow Creswell & Plano Clark; confirm Erford uses the same labels (older texts say "sequential explanatory" and "concurrent triangulation").

☐ Approve ☐ Revise ☐ Retire Notes:

A research team plans a randomized trial of a new intervention for adults with severe depression, some of whom report suicidal ideation. One team member proposes a no-treatment control group to get the cleanest comparison. Which comparison condition is MOST ethically defensible?

A. Treatment as usual, so control clients still receive standard care ✓ KEY

An active treatment-as-usual comparison answers the research question without withholding needed care from high-risk clients.

B. A no-treatment group, provided participants give informed consent

Consent does not make it acceptable to withhold effective care from severely depressed, at-risk participants.

C. A placebo pill group, so neither group knows which they received

An inert placebo still withholds active treatment and is a poor match for a psychotherapy intervention.

D. A six-month waitlist group with no contact until the waiting ends

A long waitlist without monitoring delays care and leaves suicidal risk unassessed, which is not defensible here.

Rationale: Researchers must minimize harm to participants. When an effective standard treatment exists and participants are at serious risk, withholding care for the sake of a cleaner comparison is hard to justify. Comparing the new intervention with treatment as usual (an active control) keeps everyone in care. Waitlist or no-treatment controls are more defensible for low-risk problems, short delays, and with monitoring and a plan to offer treatment afterward.

References: ACA Code of Ethics — G.1.e Precautions to Avoid Injury: researchers take reasonable precautions to avoid causing physical, emotional or social injury to participants · Erford (Research and Evaluation in Counseling) — Ethics in research: control groups and withholding treatment

☐ Approve ☐ Revise ☐ Retire Notes:

A study comparing a new group intervention with a waitlist reports Cohen's $d = 0.50$ on a measure of anxiety. Using Cohen's conventional benchmarks, how large is this effect?

A. Negligible

Effects below about 0.20 are usually described as negligible; 0.50 is well above that.

B. Small

A small effect is about $d = 0.20$.

C. Medium ✓ KEY

Cohen described $d = 0.50$ as a medium effect.

D. Large

A large effect is about $d = 0.80$ or above.

Rationale: Cohen's d expresses the difference between two group means in standard deviation units. Cohen's conventional benchmarks are 0.20 for a small effect, 0.50 for a medium effect and 0.80 for a large effect. Unlike a p value, an effect size describes how much difference an intervention makes and does not depend on sample size.

References: Erford (Research and Evaluation in Counseling) — Effect size: Cohen's d benchmarks of .20, .50 and .80

☐ Approve ☐ Revise ☐ Retire Notes:

A trial reports that clients receiving a sleep intervention improved 5 points more on an insomnia scale than controls, with a 95% confidence interval for the difference of 2 to 8 points. Which interpretation is MOST accurate?

A. Ninety-five percent of treated clients improved between 2 and 8 points

The interval describes uncertainty about the average group difference, not the range of individual client changes.

B. The difference is significant at .05 because the interval excludes 0 ✓ **KEY**

A 95% interval that does not contain zero corresponds to rejecting the null hypothesis of no difference at $\alpha = .05$.

C. The study has a 5% chance of being repeated with the same 5-point result

Confidence level is not a probability of replicating the exact point estimate.

D. The intervention has a large effect because the whole interval is positive

An interval above zero shows a reliable difference, not a large one; effect size needs a standardized measure.

Rationale: A confidence interval gives a range of plausible values for the population parameter, here the true mean difference. If the procedure were repeated many times, 95% of intervals built this way would contain the true value. When a 95% interval for a difference excludes zero, the result is statistically significant at the .05 level. The interval's width also shows precision: narrower intervals reflect more precise estimates.

References: Erford (Research and Evaluation in Counseling) — Inferential statistics: confidence intervals and their relation to hypothesis testing

☐ Approve ☐ Revise ☐ Retire Notes:

A client's score on a depression measure drops from 38 to 26. The reliable change index for this measure is 8 points, and the cutoff separating clinical from nonclinical populations is 20. Using the Jacobson and Truax approach, how should this change be classified?

A. Recovered, since the change is larger than the reliable change index

Recovery requires both reliable change and a final score past the cutoff; 26 is still above 20.

B. Unchanged, since the final score is still in the clinical range

A 12-point drop exceeds the 8-point reliable change index, so the change is larger than measurement error.

C. Deteriorated, since the score has not yet fallen below the cutoff

Deterioration means reliable change in the worse direction; this client's score improved.

D. Improved, since the change is reliable but the cutoff is not crossed ✓ **KEY**

The 12-point drop exceeds the reliable change index, but 26 remains in the clinical range, so the client is improved, not recovered.

Rationale: Jacobson and Truax defined clinically significant change with two criteria: the change must be reliable (larger than expected from measurement error, judged by the reliable change index) and the client must move from the dysfunctional range into the functional range past a cutoff. Clients meeting both are recovered; reliable change without crossing the cutoff is improved; no reliable change is unchanged; reliable worsening is deteriorated. This distinguishes meaningful change for one client from mere statistical significance in a group.

References: Lambert (Outcome Monitoring) — Reliable change index and clinical cutoff (Jacobson & Truax) for classifying client change · Erford (Research and Evaluation in Counseling) — Clinical versus statistical significance

☐ Approve ☐ Revise ☐ Retire Notes:

A 52-year-old client drinks about a fifth of vodka daily and wants to stop. The last time he quit on his own, he had a withdrawal seizure and was taken to an emergency department. He has stable housing and a supportive sister. Which recommendation is MOST appropriate as the first step?

A. Begin weekly outpatient counseling with a goal of stopping alcohol use within two weeks

Weekly outpatient care offers no medical monitoring; stopping abruptly with his seizure history puts him at serious risk.

B. Enroll him in an intensive outpatient program and have him taper his drinking on his own

A self-directed taper is unreliable and unsupervised; IOP does not provide the medical management his withdrawal risk requires.

C. Refer him for medically monitored withdrawal management before planning ongoing treatment ✓ KEY

A history of withdrawal seizures signals high withdrawal risk, so medically supervised withdrawal management is the safe first step.

D. Arrange a stay in a sober living home so he can stop drinking in an alcohol-free setting

Sober living provides a substance-free environment but no medical care; it does not address dangerous withdrawal.

Rationale: Level-of-care decisions in addiction treatment consider several dimensions, and acute intoxication and withdrawal potential is assessed first because it can be life-threatening. A prior withdrawal seizure with heavy daily drinking signals high risk for complicated alcohol withdrawal. He should first receive medically monitored withdrawal management; his supportive environment can then help him step into an appropriate ongoing level of care.

References: ASAM Criteria — Withdrawal potential dimension: history of withdrawal seizures and medically monitored withdrawal management

☐ Approve ☐ Revise ☐ Retire Notes:

A 36-year-old teacher meets criteria for mild alcohol use disorder. She has no withdrawal symptoms or medical problems, is motivated to cut back, and lives with a supportive partner. Her counselor recommends a 28-day residential program "just to be safe." What is the MAIN concern with this recommendation?

A. It places her at a more intensive level than needed and may disrupt her work and supports ✓ KEY

Care should be the least intensive level that safely meets her needs; residential care would pull her from her job and partner without clinical need.

B. Residential programs are designed only for clients who have already failed outpatient care

Placement is based on current multidimensional need, not on a required sequence of failed lower levels.

C. A 28-day stay is too short to change a drinking pattern that has lasted for several years

The problem is not the length of the stay but that residential care is not indicated for her at all.

D. Her motivation shows she does not need any formal treatment for her alcohol use at this time

Motivation is a strength, but she still meets criteria for a disorder and can benefit from outpatient treatment.

Rationale: Placement criteria aim to match clients to the least intensive level of care that is safe and effective, based on a multidimensional assessment. Over-placement wastes resources and can harm clients by disrupting protective factors such as employment, family and community ties. With no withdrawal risk, no medical or psychiatric complications and a supportive home, outpatient treatment fits this client.

References: ASAM Criteria — Multidimensional assessment and placement in the least intensive level of care that is safe and effective

☐ Approve ☐ Revise ☐ Retire Notes:

A 58-year-old retired nurse has had trouble falling and staying asleep at least four nights a week for over a year. Medical and substance-related causes have been ruled out, and she would prefer not to take sleep medication. Which psychological treatment has the strongest evidence as a first-line approach?

A. Sleep hygiene education given on its own, such as limiting caffeine and keeping a dark room

Sleep hygiene is a useful component, but guidelines advise against using it alone because it is not effective as a single therapy.

B. Psychodynamic therapy aimed at uncovering the early conflicts that are keeping her awake

Psychodynamic therapy is not an established first-line treatment for chronic insomnia.

C. Progressive muscle relaxation practiced in session, with no change to her sleep schedule

Relaxation can help, but alone it lacks the support of the multicomponent package that restructures sleep behavior.

D. Cognitive behavioral therapy for insomnia, including stimulus control and sleep restriction ✓ **KEY**

CBT-I is the recommended first-line treatment for chronic insomnia in adults, with strong research support.

Rationale: Cognitive behavioral therapy for insomnia (CBT-I) is a multicomponent treatment that combines stimulus control, sleep restriction, cognitive work on sleep-related beliefs and sleep hygiene education. Clinical practice guidelines strongly recommend it as the first-line treatment for chronic insomnia in adults. Sleep hygiene education is helpful as part of the package but is not recommended as a stand-alone therapy.

References: AASM Clinical Practice Guideline (Insomnia) — Behavioral and psychological treatments for chronic insomnia: strong recommendation for multicomponent CBT-I; sleep hygiene not recommended as a single therapy

☐ Approve ☐ Revise ☐ Retire Notes:

A 33-year-old client with bipolar I disorder takes lithium as prescribed. Her mood episodes tend to follow disruptions such as overnight shifts, travel across time zones and conflicts with her partner. Her psychiatrist asks the counselor to provide an adjunctive psychotherapy suited to this pattern. Which approach BEST fits?

A. Exposure and response prevention focused on reducing her worry about future mood swings

ERP is a treatment for obsessive-compulsive disorder and does not target the rhythm disruptions linked to her episodes.

B. Interpersonal and social rhythm therapy to stabilize daily routines and address conflicts ✓ **KEY**

IPSRT was developed for bipolar disorder; it regularizes sleep and daily routines and addresses interpersonal problems that trigger episodes.

C. Person-centered therapy alone, letting her choose the focus of each session as it unfolds

A supportive nondirective approach does not provide the structured rhythm and interpersonal work her pattern calls for.

D. Psychoanalytic therapy several times weekly to explore unconscious roots of her moods

Intensive psychoanalytic therapy is not an evidence-based adjunct for bipolar disorder and does not target rhythm stability.

Rationale: Interpersonal and social rhythm therapy (IPSRT), developed by Ellen Frank, combines interpersonal psychotherapy with behavioral work on stabilizing daily routines and sleep-wake cycles. It rests on the idea that disrupted social rhythms destabilize circadian functioning and can trigger mood episodes in bipolar disorder. It is used as an adjunct to medication, which fits a client whose episodes follow schedule disruptions and relationship conflicts.

References: IPSRT — Social rhythm stabilization and interpersonal problem areas as an adjunct to medication in bipolar disorder

☐ Approve ☐ Revise ☐ Retire Notes:

An experienced counselor declines to use session-by-session outcome measures, saying, "After 20 years, I can tell when a client is getting worse." Which research finding MOST directly challenges this view?

A. Clinicians rarely predict which clients will deteriorate; measure-based alerts flag most ✓ **KEY**

Studies comparing clinician predictions with actuarial alerts found that therapists identified very few deteriorating clients, while measures flagged most of them.

B. Experienced clinicians tend to underestimate how much progress their clients are making

Research suggests the opposite bias: clinicians tend to overestimate client improvement.

C. Most clients who get worse in counseling say so openly to their counselor within a session

Clients often do not voice deterioration directly, which is one reason structured measures are useful.

D. Standardized measures are accurate only when they are given at intake and at the final session

Pre-post measurement cannot detect deterioration while there is still time to respond; frequent monitoring is the point.

Rationale: Research on routine outcome monitoring found that clinicians, including experienced ones, are poor at predicting which clients will deteriorate, while statistical early-warning systems based on expected recovery curves identify most of them. Clinicians also tend to overrate their own effectiveness. Feedback from frequent standardized measures helps counselors catch clients who are not on track and change course before dropout or treatment failure.

References: Lambert (Outcome Monitoring) — Clinician judgment versus actuarial prediction in detecting client deterioration

☐ Approve ☐ Revise ☐ Retire Notes:

A counselor plans to track a client's anxiety with a brief standardized self-report measure over the course of treatment. When should the counselor give the measure for the FIRST time?

A. Once the client has learned the coping skills, so the scores show what they have gained

Waiting until skills are learned removes the starting point needed to show how much the client has changed.

B. At the midpoint of treatment, when the counselor is deciding whether to change approaches

A midpoint score has nothing to be compared against unless an earlier baseline was taken.

C. At intake or before treatment starts, to set a baseline for comparing later scores ✓ **KEY**

A pretreatment score is the baseline against which every later score is compared.

D. At the final session, so the counselor can document the client's status at discharge

A discharge score alone cannot show change; it needs a baseline taken before treatment.

Rationale: Progress monitoring depends on a baseline: a measurement taken at intake or before treatment begins. Later scores are interpreted relative to that starting point, which allows the counselor to judge whether change is meaningful. Without a baseline, improvement or deterioration cannot be shown.

References: Hays (Assessment) — Outcome assessment: establishing a baseline before treatment for progress monitoring

☐ Approve ☐ Revise ☐ Retire Notes:

A 41-year-old client has met all the goals in her treatment plan for depression, and her scores have stayed in the nonclinical range for two months. She says she feels ready to stop. Her counselor would like to keep meeting weekly to "work on deeper issues." What should the counselor do?

A. Continue weekly sessions, since further personal growth is still possible for the client

Continuing when goals are met and the client wants to stop serves the counselor's agenda, not a clinical need.

B. Review her gains, plan for maintaining them, and discharge her with an option to return ✓ **KEY**

When the client no longer needs counseling, the counselor ends it with a planned discharge and aftercare guidance.

C. End services at this session without further discussion, since the treatment goals are met

Ending abruptly skips the review, relapse-prevention planning and aftercare that a planned discharge includes.

D. Write new long-term goals in the plan so that continued weekly sessions are justified

Adding goals the client did not ask for to justify services is not collaborative and is ethically questionable.

Rationale: Counselors end counseling when it is reasonably apparent that the client no longer needs assistance, is unlikely to benefit or is being harmed. A planned discharge reviews progress toward goals, consolidates gains, identifies early warning signs and coping strategies, and tells the client how to return if needed. The client's readiness and goal attainment, not the counselor's wish to continue, guide the decision.

References: ACA Code of Ethics — A.11.c Appropriate Termination · Wiger (Documentation) — Discharge summary: progress toward goals and aftercare recommendations

☐ Approve ☐ Revise ☐ Retire Notes: _____

A new counselor asks why the agency uses a written safety plan with clients at risk of suicide instead of asking them to sign a "no-suicide contract." Which explanation is MOST accurate?

A. A signed contract legally shields the counselor, but it is too formal for most clients

No-suicide contracts offer no reliable legal protection, and formality is not the main problem with them.

B. A contract is the better tool, but a safety plan takes less session time to complete

Contracts are not the better tool, and a good safety plan takes time to build collaboratively.

C. A safety plan replaces the need for any further suicide risk assessment of the client

A safety plan supplements ongoing risk assessment; it does not replace it.

D. A safety plan builds specific coping steps, while a contract lacks evidence of benefit ✓ **KEY**

Safety plans give clients concrete steps and resources for a crisis; no-suicide contracts lack research support and can give false reassurance.

Rationale: No-suicide contracts ask a client to promise not to harm themselves but give them nothing to do in a crisis, lack evidence of effectiveness and may give counselors false reassurance. A safety plan, such as the Stanley-Brown Safety Planning Intervention, is developed collaboratively and lists warning signs, internal coping strategies, social contacts, people to ask for help, professional and crisis resources, and steps to make the environment safer.

References: Stanley-Brown SPI — Safety planning as an alternative to no-suicide contracts

☐ Approve ☐ Revise ☐ Retire Notes: _____

A 47-year-old client who has smoked for 30 years tells his counselor that he has picked a quit date two weeks from now, bought nicotine patches and told his family. Which counselor response BEST matches his current stage of change?

A. Help him build a specific quit plan that anticipates triggers and names his coping steps ✓ KEY

He is in preparation, so the task is to firm up a concrete, workable action plan.

B. Give him information about the health risks of smoking to raise his awareness of them

Consciousness raising fits precontemplation; he is already convinced and taking steps.

C. Explore his mixed feelings by listing the pros and cons of smoking and of quitting

A decisional balance exercise suits contemplation, when the client is still ambivalent about changing.

D. Focus on preventing relapse in the months after he has stayed smoke-free for a while

Relapse prevention for sustained change belongs to the maintenance stage, which he has not reached.

Rationale: In the Transtheoretical Model, the preparation stage describes people who intend to act within about the next month and have already taken small steps, such as setting a date or buying supplies. Stage-matched interventions in preparation help the client turn intention into a concrete plan, anticipate obstacles and strengthen commitment. Interventions suited to earlier stages can feel redundant to a client who has already decided.

References: Transtheoretical Model — Preparation stage: intention to act within 30 days and stage-matched action planning

☐ Approve ☐ Revise ☐ Retire Notes:

A 29-year-old client stopped using methamphetamine eight months ago but used again for three days after a breakup. She tells her counselor, "I'm back at square one." According to Prochaska and DiClemente's model, which statement BEST reflects how relapse is understood?

A. Relapse sends most people back to precontemplation, so change must restart from the start

Only a small share of people who relapse regress to precontemplation; most do not start over.

B. Relapse after six months shows the client had not truly reached the maintenance stage

Relapse can occur from maintenance; it does not mean the earlier progress was not real.

C. Relapse is common, and most people recycle to contemplation or preparation and try again ✓ KEY

The spiral model shows that most people who relapse recycle to contemplation or preparation and draw on what they learned.

D. Relapse shows that the stages of change model does not apply to stimulant use disorders

The model has been applied across many addictive behaviors, including stimulant use, and expects relapse.

Rationale: Prochaska, DiClemente and Norcross described change as a spiral rather than a straight line. Relapse is common, and most people who relapse do not regress all the way to precontemplation; they recycle to contemplation or preparation and try again, often with lessons from earlier attempts. Framing relapse this way helps counselors counter the client's "square one" thinking and reduce shame.

References: Transtheoretical Model — Spiral model of change: relapse and recycling through the stages

☐ Approve ☐ Revise ☐ Retire Notes:

A client's primary care physician calls the counselor to coordinate care and asks for "everything you have," including the private process notes the counselor keeps separate from the client's clinical record. Under HIPAA, which statement is accurate?

- A. HIPAA bars the counselor from sharing any client information with a physician without consent
HIPAA allows disclosure of protected health information to another provider for treatment; it does not bar all sharing.
- B. The physician may obtain the process notes because they are part of the client's medical record
Psychotherapy notes kept separate from the record are not part of the general medical record and receive extra protection.
- C. All records, process notes included, may be sent because disclosure for treatment is permitted
The treatment exception does not cover psychotherapy notes, which need specific authorization.
- D. The process notes need the client's specific authorization; record information may be shared ✓ KEY
Psychotherapy notes generally need a separate authorization, while information in the clinical record may be shared for treatment.

Rationale: HIPAA gives psychotherapy notes, the counselor's separate notes analyzing session content, special protection: most disclosures, including to other providers, require the client's specific authorization. Information in the clinical record, such as diagnosis, treatment plan, symptoms and progress summaries, may be disclosed to another provider for treatment purposes. Many counselors still obtain a release before coordinating care, and state law or ethics codes may require stricter consent.

References: HIPAA — Privacy Rule 45 CFR 164.508(a)(2): authorization required for psychotherapy notes; 164.506 disclosures for treatment

☐ Approve ☐ Revise ☐ Retire Notes:

A 30-year-old client seeks counseling because panic attacks keep her from driving on the highway to work. Her counselor adds this objective: "Client will walk 30 minutes three times a week for the next eight weeks, as recorded in an exercise log." Which SMART criterion is this objective MOST likely to fail?

- A. Measurable, because walking cannot be counted or recorded in any reliable way
Walking time and frequency are easy to record in a log, so the objective is measurable.
- B. Relevant, because it is not clearly tied to the problem the client came in for ✓ KEY
Nothing links walking to her highway-driving panic, so the objective does not fit her presenting concern.
- C. Time-bound, because the objective does not give any time period for the change
The objective states an eight-week period, so it is time-bound.
- D. Specific, because the objective does not state how often or how long she will walk
The objective names the activity, duration and frequency, so it is specific.

Rationale: SMART objectives are Specific, Measurable, Achievable, Relevant and Time-bound. This objective is precise, countable and time-limited, but it does not connect to the client's stated problem of panic while driving on highways. Relevant objectives flow from the presenting concern and the goal, such as graded driving exposure tracked in a log.

References: Wiger (Documentation) — Treatment plans: objectives linked to the presenting problem and long-term goals

☐ Approve ☐ Revise ☐ Retire Notes:

A 30-year-old man with a mild intellectual disability lives in a group home and is starting counseling for anger after conflicts with a housemate. The group home manager offers to write his treatment goals for him to save time. What is the BEST approach?

A. Build goals with him in plain words with visual aids, inviting staff input if he agrees ✓ KEY

Adapting the process to his abilities keeps him at the center of planning, with staff involved only with his consent.

B. Accept the manager's goals, since staff observe his daily behavior more than he can report

Staff observations are useful input, but letting staff set his goals removes him from his own treatment planning.

C. Write the goals in standard clinical language and read them aloud to him for his signature

Having him sign a plan he did not shape, in language he may not understand, is not meaningful collaboration.

D. Delay planning until a court-appointed guardian can be found to decide on his goals

Nothing indicates he lacks capacity or has a guardian; a disability alone does not remove his right to participate.

Rationale: Counselors and clients work jointly to devise counseling plans that fit the client's abilities, developmental level and circumstances. For a client with an intellectual disability, collaboration means adapting the process, using plain language, concrete examples and visual supports, rather than handing planning to others. Support staff can contribute valuable information when the client agrees.

References: ACA Code of Ethics — A.1.c Counseling Plans: plans consistent with the abilities, developmental level and circumstances of clients

☐ Approve ☐ Revise ☐ Retire Notes:

At the end of its first full year, a middle school's peer mediation program is evaluated so the school board can decide whether to continue funding it. Which type of evaluation is this?

A. Formative evaluation

Formative evaluation gathers feedback during a program to improve it, not to judge it for a funding decision.

B. Needs assessment

A needs assessment identifies gaps before a program is designed; it does not judge an existing program.

C. Single-subject design

A single-subject design tracks one person's behavior across phases, not a whole program's worth.

D. Summative evaluation ✓ KEY

Summative evaluation judges a program's overall effectiveness at the end of a cycle to inform decisions such as continuing funding.

Rationale: Summative evaluation takes place at the end of a program or program cycle and judges its overall merit or effectiveness, often to inform decisions about continuing, expanding or ending it. Formative evaluation, by contrast, happens while a program is running and is used to improve it. A board deciding on future funding needs a summative judgment.

References: Erford (Research and Evaluation in Counseling) — Program evaluation: formative vs summative evaluation

☐ Approve ☐ Revise ☐ Retire Notes:

To estimate the need for youth mental health services in a county, an agency analyzes existing census data, school dropout rates, juvenile arrest records and emergency department visits for self-harm. No residents or providers are contacted directly. Which needs assessment method is the agency using?

A. Key informant approach

This method gathers views from knowledgeable people such as clinicians or community leaders, who were not contacted here.

B. Community forum approach

A community forum invites residents to public meetings to voice needs; no residents were involved.

C. Social indicators approach ✓ KEY

This method infers need from existing public statistics such as census, school and health records.

D. Field survey of residents

A field survey collects new data directly from a sample of residents, which the agency did not do.

Rationale: Needs assessments can draw on several methods. The social indicators approach infers community need from existing public data, such as census figures, health statistics and school or court records. Key informant interviews, community forums and field surveys all collect new information directly from people. Many needs assessments combine methods because existing data alone may miss what residents and providers see.

References: Warheit et al. (Needs Assessment) — Social indicators approach: inferring need from existing public statistics · Erford (Research and Evaluation in Counseling) — Needs assessment methods in program evaluation

☐ Approve ☐ Revise ☐ Retire Notes:

An evaluator of a new parenting program at a community center records how many sessions are offered, who attends, whether the target families of toddlers are being reached and whether facilitators cover each planned module. She does not yet measure changes in parenting. Which type of evaluation is she conducting?

A. Outcome evaluation

An outcome evaluation measures changes in participants, such as parenting skills, which she is not yet doing.

B. Process evaluation ✓ KEY

Process evaluation examines how a program is delivered: reach, attendance and whether it runs as planned.

C. Needs assessment

A needs assessment identifies gaps before a program is built; this program is already running.

D. Cost-benefit study

A cost-benefit study compares program costs with outcomes converted to dollars; no costs are examined here.

Rationale: Process (implementation) evaluation asks whether a program is being delivered as intended and to the intended population, tracking things like attendance, reach and coverage of planned content. Outcome evaluation asks whether participants changed as a result. Process data also help interpret outcomes, because a program that is not delivered as planned cannot fairly be judged ineffective.

References: Erford (Research and Evaluation in Counseling) — Program evaluation: process vs outcome evaluation

☐ Approve ☐ Revise ☐ Retire Notes:

A counselor in a rural clinic refers a 66-year-old client with worsening depression to a telepsychiatry service for a medication evaluation. The client signs a release so the two providers can share information. What should the counselor do to BEST complete the referral?

A. Follow up to confirm the appointment took place and share relevant information as agreed ✓ **KEY**

Closing the loop confirms the client connected with care and lets both providers coordinate treatment.

B. Give the client the service's phone number and leave the scheduling and follow-up to him

Passing on a number without follow-up is a passive referral and raises the risk that the client never connects.

C. End counseling once the referral is made, since the psychiatrist now manages his depression

A medication evaluation adds to counseling; ending services would leave the client without psychotherapy.

D. Wait to discuss the referral until his next monthly session in case he brings it up himself

Waiting passively for the client to raise it can leave a worsening depression without timely follow-up.

Rationale: A referral is complete only when the client actually connects with the new service. Good care coordination includes helping the client schedule, confirming the appointment occurred, and, with consent, sharing information so both providers work from the same plan. Active follow-up matters especially for older or rural clients, who may face transportation, technology or access barriers.

References: Gladding (Counseling: A Comprehensive Profession) — Clinical mental health counseling settings: referral, follow-up and coordination with medical providers

☐ Approve ☐ Revise ☐ Retire Notes:

Counseling Skills and Interventions

300 items · NCE weight 30% (48 of 160 scored items)

Q621 of 1000 · nce-s-cou-001 · Counseling and Helping Relationships · psychodynamic and psychoanalytic theory · easy · In review

After a tense meeting in which she felt belittled by her supervisor, a client goes home and yells at her teenage son over a minor chore. Which defense mechanism does this BEST illustrate?

A. Projection

Projection attributes one's own unacceptable impulses to someone else; she is not accusing her son of her feelings.

B. Displacement ✓ **KEY**

Displacement redirects an impulse from a threatening target (the supervisor) to a safer one (the son).

C. Reaction formation

Reaction formation expresses the opposite of the true impulse, such as excessive friendliness toward the supervisor.

D. Sublimation

Sublimation channels an impulse into a socially valued outlet, such as vigorous exercise; yelling at her son is not that.

Rationale: In Freudian theory the ego uses defense mechanisms to reduce anxiety. Displacement shifts feelings away from the person who aroused them toward a less threatening target, as when anger at a boss is discharged at a family member.

References: Corey (Theory & Practice) — Psychoanalytic Therapy: ego-defense mechanisms (displacement)

☐ Approve ☐ Revise ☐ Retire Notes:

Q622 of 1000 · nce-s-cou-002 · Counseling and Helping Relationships · psychodynamic and psychoanalytic theory · medium · In review

Several months into treatment, a client begins reacting to the counselor's neutral comments with hurt and anger, saying, "You sound just like my father, always judging me." From a psychoanalytic perspective, the counselor should view this reaction as:

A. Countertransference that the counselor ought to disclose to the client

Countertransference is the counselor's own emotional reaction to the client; here the reaction is the client's.

B. Resistance that the counselor should confront firmly and directly

Analysts explore resistance rather than confront it firmly, and this reaction is better understood as transference.

C. A realistic response to the counselor's own critical style

The comments were neutral, so the client is reacting to a figure from the past rather than to the counselor's actual behavior.

D. Transference that offers material for analysis and interpretation ✓ **KEY**

The client is redirecting feelings about a significant past figure onto the counselor, which analysts treat as central therapeutic material.

Rationale: Transference occurs when a client unconsciously shifts feelings and attitudes from significant early figures onto the therapist. In psychoanalytic work, analyzing and interpreting transference helps clients gain insight into unresolved conflicts and how they repeat in current relationships.

References: Corey (Theory & Practice) — Psychoanalytic Therapy: transference and its interpretation

☐ Approve ☐ Revise ☐ Retire Notes:

An Adlerian counselor tells a client that his frequent helplessness seems to get his siblings to handle his responsibilities for him. Once the hidden payoff is named, the behavior becomes less appealing to the client. This technique is known as:

A. Spitting in the client's soup ✓ KEY

Exposing the purpose or payoff of a behavior "spoils" it, so the client can no longer use it as innocently as before.

B. Acting as if

Acting as if asks the client to behave as though already the person he wishes to be; it does not expose a payoff.

C. Catching oneself

Catching oneself has the client notice self-defeating behavior in the moment; here the counselor names its purpose.

D. The push-button technique

The push-button technique has clients alternate pleasant and unpleasant images to see that they create their feelings.

Rationale: Adlerian therapy assumes behavior is purposeful. "Spitting in the client's soup" means pointing out the hidden goal or payoff of a self-defeating behavior; the client may continue it, but it no longer tastes as good.

References: Corey (Theory & Practice) — Adlerian Therapy: techniques of the reorientation phase

☐ Approve ☐ Revise ☐ Retire Notes:

In Adler's view of psychological birth order, which child is MOST associated with the experience of being "dethroned" when a new sibling arrives?

A. The second-born child

Adler described the second child as racing to catch up with the older sibling, not as dethroned.

B. The youngest child

The youngest is described as the "baby" who may be pampered; no later sibling displaces them.

C. The oldest child ✓ KEY

The firstborn enjoys undivided attention until a sibling is born and must then adjust to losing that favored position.

D. The only child

An only child is never displaced by a sibling, although Adler noted they may struggle with sharing.

Rationale: Adler emphasized psychological (not merely ordinal) birth position within the family constellation. The oldest child is initially the center of attention and is "dethroned" by a sibling's birth, which can shape a lifestyle focused on responsibility, achievement, or regaining status.

References: Corey (Theory & Practice) — Adlerian Therapy: birth order and the family constellation

☐ Approve ☐ Revise ☐ Retire Notes:

Carl Rogers proposed that clients move toward growth when the counselor offers congruence, accurate empathic understanding, and which other core condition?

A. Interpretation of defenses

Interpreting defenses is a psychoanalytic technique, not a Rogerian core condition.

B. Structured homework assignments

Homework is typical of cognitive and behavioral approaches, not person-centered therapy.

C. Unconditional positive regard ✓ KEY

Rogers named unconditional positive regard, nonjudgmental acceptance of the client, as a core condition.

D. Confrontation of irrational beliefs

Disputing irrational beliefs is central to REBT, not to the person-centered approach.

Rationale: Rogers held that congruence (genuineness), unconditional positive regard, and accurate empathic understanding are necessary and sufficient conditions for therapeutic change. The counselor's attitudes, rather than techniques, release the client's actualizing tendency.

References: Corey (Theory & Practice) — Person-Centered Therapy: core therapeutic conditions

☐ Approve ☐ Revise ☐ Retire Notes:

A college student tells her counselor she feels worthwhile only when she earns top grades, the one achievement her parents consistently praised while growing up. From a person-centered perspective, her distress MOST likely stems from:

A. Internalized conditions of worth that distort her self-concept ✓ KEY

Rogers held that conditional acceptance from significant others leads people to value themselves only when meeting those conditions, creating incongruence.

B. An unresolved conflict between her id impulses and her superego

Id-superego conflict is a psychoanalytic formulation, not a person-centered one.

C. Irrational demands that she must perform well to be acceptable

Absolutistic "musts" are the REBT explanation of disturbance, not Rogers's.

D. A mistaken private logic formed in her early family constellation

Private logic and the family constellation are Adlerian concepts.

Rationale: Rogers described how children who receive only conditional positive regard introject conditions of worth. They then deny or distort experiences that do not fit, producing incongruence between self-concept and experience, which shows up as anxiety and vulnerability.

References: Corey (Theory & Practice) — Person-Centered Therapy: conditions of worth and incongruence

☐ Approve ☐ Revise ☐ Retire Notes:

Which theorist founded logotherapy, drawing on his experiences in Nazi concentration camps to argue that people can find meaning even in unavoidable suffering?

A. Rollo May

May was an American existential psychologist known for his work on anxiety, freedom, and love, not logotherapy.

B. Irvin Yalom

Yalom described the existential givens and group therapeutic factors but did not found logotherapy.

C. Fritz Perls

Perls founded gestalt therapy.

D. Viktor Frankl ✓ KEY

Frankl developed logotherapy, centered on the will to meaning, and wrote *Man's Search for Meaning*.

Rationale: Viktor Frankl's logotherapy holds that the primary human motivation is the will to meaning. Meaning can be found through creative work, experiences and relationships, and the attitude one takes toward unavoidable suffering.

References: Corey (Theory & Practice) — Existential Therapy: Frankl and the search for meaning

☐ Approve ☐ Revise ☐ Retire Notes:

Irvin Yalom described four "ultimate concerns," or givens of existence, that are central to existential psychotherapy. Which set lists them?

A. Anxiety, guilt, shame, and existential despair

These are emotional states that may arise from facing the givens, not the givens themselves.

B. Death, freedom, isolation, and meaninglessness ✓ KEY

Yalom identified death, freedom (and responsibility), existential isolation, and meaninglessness as the ultimate concerns.

C. Survival, belonging, power, freedom, and fun

These are Glasser's five basic needs from choice theory.

D. Safety, love, esteem, and self-actualization

These resemble levels of Maslow's hierarchy of needs.

Rationale: Yalom organized existential therapy around four givens: death, freedom, existential isolation, and meaninglessness. Anxiety arises when people confront these givens, and therapy helps clients face them authentically rather than through defensive avoidance.

References: Corey (Theory & Practice) — Existential Therapy: Yalom's ultimate concerns

☐ Approve ☐ Revise ☐ Retire Notes:

A client grieving her late father says she never told him how angry she was about his drinking. The counselor invites her to imagine her father sitting in an empty chair and to speak to him directly, in the present tense. This intervention is MOST characteristic of:

A. Gestalt therapy ✓ KEY

The empty chair technique is a hallmark gestalt experiment for working through unfinished business in the here and now.

B. Rational emotive behavior therapy

REBT disputes irrational beliefs; it does not rely on enactment with an empty chair.

C. Reality therapy

Reality therapy focuses on current behavior and the WDEP system, not on enacted dialogues.

D. Solution-focused brief therapy

SFBT emphasizes exceptions and preferred futures rather than expressing unfinished feelings.

Rationale: Gestalt therapy, associated with Fritz and Laura Perls, uses experiments such as the empty chair to bring unfinished business into present awareness. Speaking directly to the imagined person helps the client experience and complete blocked feelings.

References: Corey (Theory & Practice) — Gestalt Therapy: unfinished business and the empty-chair technique

☐ Approve ☐ Revise ☐ Retire Notes:

When a coworker criticizes him, a client reports that he wants to shout back but instead clenches his jaw and digs his fingernails into his own palms. In gestalt terms, this pattern of turning back onto oneself what one would like to do to others is called:

A. Introjection

Introjection is uncritically swallowing others' values or beliefs without assimilating them.

B. Confluence

Confluence is blurring the boundary between self and environment so that differences are not experienced.

C. Retroflexion ✓ KEY

Retroflexion is doing to oneself what one wants to do to someone else, such as turning anger inward.

D. Deflection

Deflection is avoiding contact through distraction, humor, or vagueness.

Rationale: Gestalt therapy describes several contact-boundary disturbances: introjection, projection, retroflexion, deflection, and confluence. Retroflexion turns an impulse meant for the environment back against the self, often seen in self-harm, muscle tension, or self-blame.

References: Corey (Theory & Practice) — Gestalt Therapy: contact and resistances to contact

☐ Approve ☐ Revise ☐ Retire Notes:

Joseph Wolpe's systematic desensitization pairs deep relaxation with gradual imagined exposure to a fear hierarchy. The technique is based on which principle?

A. Operant extinction

Operant extinction removes reinforcement for a behavior; desensitization targets a conditioned anxiety response.

B. Reciprocal inhibition ✓ KEY

Wolpe reasoned that a response incompatible with anxiety, such as relaxation, inhibits anxiety when paired with the feared stimulus.

C. Negative reinforcement

Negative reinforcement increases behavior by removing an aversive stimulus; it is not the basis of the technique.

D. Vicarious learning

Vicarious learning through modeling comes from Bandura's social learning theory.

Rationale: Systematic desensitization is a classical-conditioning procedure. The client learns relaxation, builds an anxiety hierarchy, and then imagines each step while relaxed. Because relaxation and anxiety are incompatible, the relaxed state reciprocally inhibits the fear response.

References: Corey (Theory & Practice) — Behavior Therapy: systematic desensitization and reciprocal inhibition

☐ Approve ☐ Revise ☐ Retire Notes:

To reduce a 7-year-old's hitting, his parents remove his tablet for the evening each time he hits his sister, and the hitting decreases. Which operant procedure are they using?

A. Positive reinforcement

Positive reinforcement adds a desirable stimulus to increase a behavior.

B. Negative reinforcement

Negative reinforcement removes an aversive stimulus to increase a behavior.

C. Positive punishment

Positive punishment adds an aversive stimulus, such as extra chores, to decrease a behavior.

D. Negative punishment ✓ KEY

Removing a desired item (the tablet) to decrease a behavior is negative punishment.

Rationale: In operant conditioning, "positive" means adding and "negative" means removing a stimulus; reinforcement increases behavior and punishment decreases it. Taking away a privilege to reduce hitting is therefore negative punishment (sometimes called response cost).

References: Corey (Theory & Practice) — Behavior Therapy: operant conditioning techniques

☐ Approve ☐ Revise ☐ Retire Notes:

Aaron Beck's cognitive triad of depression describes a negative view of which three areas?

A. The self, others, and the past

The triad concerns the future, not the past, and the world rather than others alone.

B. Thoughts, feelings, and behaviors

This is the general CBT model of interacting domains, not the cognitive triad.

C. The past, present, and future

The triad is not a time sequence; it includes the self and the world.

D. The self, the world, and the future ✓ **KEY**

Beck described depressed clients as viewing themselves, their world, and their future negatively.

Rationale: Beck proposed that depression is maintained by a negative cognitive triad: "I am inadequate" (self), "Everything goes wrong" (world), and "It will never get better" (future). Cognitive therapy helps clients test these appraisals against evidence.

References: Corey (Theory & Practice) — Cognitive Behavior Therapy: Beck's cognitive triad

☐ Approve ☐ Revise ☐ Retire Notes:

A mother tells her counselor that her son's soccer team lost the championship because she had to miss the game for work. In Beck's cognitive therapy, this is an example of which cognitive distortion?

A. Magnification

Magnification exaggerates the importance of an event rather than wrongly taking responsibility for it.

B. Personalization ✓ **KEY**

She attributes an external event to herself without a basis for making the connection.

C. Dichotomous thinking

Dichotomous thinking sorts experiences into all-or-nothing categories.

D. Selective abstraction

Selective abstraction draws conclusions from one detail while ignoring the larger context.

Rationale: Personalization is the tendency to relate external events to oneself when there is no basis for doing so. Beck catalogued distortions such as arbitrary inference, selective abstraction, overgeneralization, magnification and minimization, personalization, and dichotomous thinking.

References: Corey (Theory & Practice) — Cognitive Behavior Therapy: Beck's systematic errors in reasoning

☐ Approve ☐ Revise ☐ Retire Notes:

In Albert Ellis's ABCDE model of rational emotive behavior therapy, what does the "D" represent?

A. Describing the activating event

The activating event is the "A" in the model.

B. Developing new behaviors

New behaviors may follow, but "D" is a cognitive step, not behavior development.

C. Disputing irrational beliefs ✓ KEY

D is disputing, the active challenge of the irrational beliefs ("B") that cause disturbed consequences ("C").

D. Defining emotional consequences

Emotional and behavioral consequences are the "C" in the model.

Rationale: Ellis's model holds that an Activating event does not directly cause emotional Consequences; Beliefs about the event do. The counselor Disputes irrational beliefs, producing an Effective new philosophy (E) and healthier feelings.

References: Corey (Theory & Practice) — Rational Emotive Behavior Therapy: the A-B-C framework

☐ Approve ☐ Revise ☐ Retire Notes:

A client says, "I must get this promotion. If I don't, it will be awful, I won't be able to stand it, and it will prove I'm worthless." According to Ellis, which element is the CORE of this irrational belief, from which the others derive?

A. Demandingness, or absolutistic "musts" ✓ KEY

Ellis held that rigid demands ("I must") are primary, and awfulizing, low frustration tolerance, and self-downing derive from them.

B. Catastrophic misinterpretation

Catastrophic misinterpretation of bodily sensations is a cognitive model of panic, not Ellis's core belief.

C. Internalized conditions of worth

Conditions of worth are a person-centered concept.

D. Introjected parental messages

Introjection is a gestalt contact disturbance, not the REBT core of irrationality.

Rationale: REBT identifies demandingness ("must," "should," "ought" held as absolutes) as the core of irrational thinking. Awfulizing, "I-can't-stand-it-itis" (low frustration tolerance), and global self-rating are derivatives that follow from rigid demands.

References: Corey (Theory & Practice) — Rational Emotive Behavior Therapy: musturbation and derivative beliefs

☐ Approve ☐ Revise ☐ Retire Notes:

Reality therapy is often taught through the WDEP system (Wants, Doing and direction, Evaluation, Planning). Who developed and expanded this system?

A. Robert Wubbolding ✓ **KEY**

Wubbolding developed the WDEP system as a practical framework for applying Glasser's reality therapy.

B. Albert Ellis

Ellis founded REBT and the ABCDE model.

C. Steve de Shazer

De Shazer co-developed solution-focused brief therapy.

D. Michael White

White co-developed narrative therapy.

Rationale: William Glasser founded reality therapy and choice theory; Robert Wubbolding extended it with the WDEP system. Counselors explore what clients want, what they are doing, whether it is working, and a simple, attainable plan.

References: Corey (Theory & Practice) — Reality Therapy: the WDEP system

☐ Approve ☐ Revise ☐ Retire Notes:

A high school student says he skips class because "nobody there likes me anyway." After exploring what he wants and what he is currently doing, the reality therapist asks, "Is skipping class helping you get the friendships you want?" This question reflects which part of WDEP?

A. Wants

Wants were explored earlier; this question judges whether behavior serves those wants.

B. Doing and direction

Doing describes current behavior; it does not ask the client to judge its effectiveness.

C. Evaluation ✓ **KEY**

Asking whether current behavior is getting the client what he wants is self-evaluation, the core of reality therapy.

D. Planning

Planning comes after evaluation and creates a specific, attainable course of action.

Rationale: In Wubbolding's WDEP system, Evaluation asks clients to judge whether their total behavior is meeting their needs and wants. Glasser's five basic needs are survival, love and belonging, power, freedom, and fun; here the student is pursuing belonging ineffectively.

References: Corey (Theory & Practice) — Reality Therapy: self-evaluation in the WDEP system

☐ Approve ☐ Revise ☐ Retire Notes:

A counselor asks a client, "Suppose that tonight, while you sleep, the problem that brought you here is solved. When you wake up, what will be the first thing you notice that tells you things are different?" This is an example of:

A. An exception question

Exception questions ask about past times the problem was absent or less severe.

B. The miracle question ✓ KEY

Imagining the problem gone overnight and describing the difference is the classic miracle question.

C. A scaling question

Scaling questions ask clients to rate progress or confidence on a numerical scale.

D. A coping question

Coping questions ask how the client has managed to keep going despite difficulties.

Rationale: The miracle question, developed within solution-focused brief therapy by Steve de Shazer and Insoo Kim Berg, invites clients to describe a preferred future in concrete, observable terms. Those details then become the basis for goals and small next steps.

References: Corey (Theory & Practice) — Postmodern Approaches: solution-focused brief therapy, the miracle question

☐ Approve ☐ Revise ☐ Retire Notes:

A counselor says, "On a scale from 0 to 10, where 10 means the problem is fully resolved, you said you are at a 4. What would tell you that you had moved to a 5?" This technique and the approach it comes from are MOST closely associated with:

A. Michael White and David Epston

White and Epston developed narrative therapy, known for externalizing conversations.

B. Jay Haley and Cloé Madanes

Haley and Madanes are associated with strategic family therapy and directives.

C. William Miller and Stephen Rollnick

Miller and Rollnick developed motivational interviewing; MI uses importance rulers, but this framing is SFBT scaling.

D. Steve de Shazer and Insoo Kim Berg ✓ KEY

Scaling questions focused on small next steps are a signature solution-focused technique from de Shazer and Berg.

Rationale: Solution-focused brief therapy, developed by Steve de Shazer and Insoo Kim Berg, uses scaling questions to make progress concrete and to identify the next small, observable step. The focus is on what is already working rather than on the problem's history.

References: Corey (Theory & Practice) — Postmodern Approaches: solution-focused brief therapy, scaling questions

☐ Approve ☐ Revise ☐ Retire Notes:

A counselor working with a 10-year-old who refuses to go to school asks, "When does the Worry show up, and how does it try to convince you to stay home?" The counselor is using which technique?

A. Reframing

Reframing offers a new meaning for a behavior; it does not separate the problem from the person.

B. Paradoxical intention

Paradoxical intention asks the client to deliberately do or wish for the feared behavior.

C. Externalizing the problem ✓ KEY

Naming the Worry as a separate entity that acts on the child externalizes the problem.

D. Cognitive restructuring

Cognitive restructuring identifies and replaces distorted thoughts rather than personifying the problem.

Rationale: Narrative therapy, developed by Michael White and David Epston, holds that "the person is not the problem; the problem is the problem." Externalizing conversations separate clients from problem-saturated identities so they can examine and resist the problem's influence.

References: Corey (Theory & Practice) — Postmodern Approaches: narrative therapy, externalizing conversations

☐ Approve ☐ Revise ☐ Retire Notes:

A narrative counselor learns that a client who describes himself as "a hopeless addict" turned down a drink at a family wedding last month. The counselor explores this event in detail to begin building an alternative story. In narrative therapy, such events are called:

A. Unique outcomes ✓ KEY

White used "unique outcomes" for moments that contradict the dominant problem story and seed re-authoring.

B. Exceptions

"Exceptions" is the solution-focused term for times the problem is absent; the question asks for narrative terminology.

C. Peak experiences

Peak experiences are Maslow's term for moments of intense fulfillment.

D. Corrective emotional experiences

This psychodynamic term refers to re-experiencing old conflicts with a new, healthier outcome in therapy.

Rationale: Narrative therapists listen for unique outcomes: times when the client acted outside the problem-saturated story. Thickening these accounts helps re-author an alternative, preferred identity story.

References: Corey (Theory & Practice) — Postmodern Approaches: narrative therapy, unique outcomes and re-authoring

► **Check:** 'Exceptions' (SFBT) sits beside the key 'unique outcomes' (narrative); confirm one best answer reads clearly.

☐ Approve ☐ Revise ☐ Retire Notes:

Which statement BEST reflects a central principle of feminist therapy?

A. Distress mainly reflects unresolved intrapsychic conflict

Feminist therapy looks beyond intrapsychic causes to social, political, and cultural context.

B. The personal is political, so context shapes distress ✓ KEY

"The personal is political" holds that individual problems are linked to social structures and oppression.

C. The counselor holds expert authority to direct the client

Feminist therapy emphasizes an egalitarian relationship that minimizes power differences.

D. A diagnosis is required before exploring social context

Feminist therapists are cautious about diagnostic labels and prioritize the client's context.

Rationale: Feminist therapy rests on principles such as "the personal is political," an egalitarian counseling relationship, valuing women's experiences, and a commitment to social change. Symptoms are often reframed as understandable responses to oppressive conditions.

References: Corey (Theory & Practice) — Feminist Therapy: key principles

☐ Approve ☐ Revise ☐ Retire Notes:

A counselor helps a client identify the messages she received growing up about what women "should" be, where those messages came from, and how they influence her guilt about setting limits with family today. This feminist intervention is called:

A. Lifestyle assessment

Lifestyle assessment is an Adlerian technique exploring family constellation and early recollections.

B. Genogram construction

A genogram maps multigenerational family patterns, a Bowenian tool.

C. Cognitive restructuring

Cognitive restructuring targets distorted thoughts without centering gender socialization.

D. Gender-role analysis ✓ KEY

Gender-role analysis examines the impact of gender-role expectations on a client's well-being.

Rationale: Gender-role analysis helps clients recognize gender-role messages, their sources, and their consequences, and then decide which to keep or change. Along with power analysis, it is a signature feminist therapy intervention.

References: Corey (Theory & Practice) — Feminist Therapy: gender-role analysis and intervention

☐ Approve ☐ Revise ☐ Retire Notes:

In motivational interviewing, the core skills are summarized by the acronym OARS: open questions, affirmations, reflections, and what?

A. Solutions

MI avoids prematurely supplying solutions; the client voices reasons for change.

B. Summaries ✓ KEY

Summaries collect and link what the client has said, often highlighting change talk.

C. Silence

Silence can be useful, but it is not part of the OARS acronym.

D. Self-disclosure

Self-disclosure is not one of the OARS core skills.

Rationale: Miller and Rollnick describe OARS (open questions, affirming, reflecting, summarizing) as the foundational communication skills of MI. These skills support engaging, focusing, evoking, and planning.

References: Miller & Rollnick (MI) — Core interviewing skills: OARS

☐ Approve ☐ Revise ☐ Retire Notes:

A client says, "I know I should cut back on drinking, but it's the only thing that helps me relax after work." Which counselor response is MOST consistent with motivational interviewing?

A. "You need to understand that drinking is harming your health."

Unsolicited warnings reflect the righting reflex and tend to elicit sustain talk.

B. "Why haven't you been able to cut back on your drinking yet?"

Asking why the client has not changed invites defense of the status quo.

C. "It sounds like you are in denial about how serious this is."

Labeling and confronting denial increases discord, which MI avoids.

D. "It helps you unwind, and you also see reasons to cut back." ✓ KEY

A double-sided reflection acknowledges both sides of ambivalence and ends on the change side.

Rationale: MI treats ambivalence as normal. A double-sided reflection captures both sustain talk and change talk, typically ending with the change side, so the client hears his own reasons for change without feeling pushed. Confrontation and advice-giving tend to strengthen sustain talk.

References: Miller & Rollnick (MI) — Responding to sustain talk and discord: double-sided reflections

☐ Approve ☐ Revise ☐ Retire Notes:

A DBT skills group is teaching TIPP skills for crisis survival and practicing radical acceptance of painful realities that cannot be changed right now. These skills belong to which DBT module?

A. Distress tolerance ✓ **KEY**

Crisis survival skills such as TIPP and reality-acceptance skills such as radical acceptance are taught in distress tolerance.

B. Emotion regulation

Emotion regulation focuses on understanding emotions, reducing vulnerability, and changing unwanted emotions.

C. Interpersonal effectiveness

Interpersonal effectiveness teaches asking for what one needs and saying no (for example, DEAR MAN).

D. Core mindfulness

Mindfulness teaches observing, describing, and participating nonjudgmentally (wise mind).

Rationale: Marsha Linehan's DBT skills training has four modules: mindfulness, distress tolerance, emotion regulation, and interpersonal effectiveness. Distress tolerance helps clients survive crises without making things worse and accept reality as it is.

References: Corey (Theory & Practice) — Behavior Therapy: dialectical behavior therapy skills modules

☐ Approve ☐ Revise ☐ Retire Notes:

Marsha Linehan developed dialectical behavior therapy initially for chronically suicidal clients. The central dialectic of the approach is the balance between:

A. Insight and catharsis

Insight and catharsis are associated with psychodynamic and experiential approaches.

B. Structure and spontaneity

This is not the organizing dialectic of DBT.

C. Acceptance and change ✓ **KEY**

DBT balances validation and acceptance of the client with behavioral change strategies.

D. Thinking and feeling

DBT integrates reason and emotion in "wise mind," but its central dialectic is acceptance and change.

Rationale: DBT integrates cognitive-behavioral change strategies with acceptance practices drawn from Zen and mindfulness. Linehan developed it for chronically suicidal clients, many meeting criteria for borderline personality disorder, who experienced pure change-focused therapy as invalidating.

References: Corey (Theory & Practice) — Behavior Therapy: dialectical behavior therapy, acceptance and change

☐ Approve ☐ Revise ☐ Retire Notes:

Two parents in a tense marriage each confide in their 15-year-old daughter about their frustrations with the other, and the daughter now feels responsible for keeping the peace. A Bowenian family counselor would MOST likely describe this pattern as:

A. Triangulation ✓ KEY

Bowen described how anxious two-person systems draw in a third person to stabilize tension, forming a triangle.

B. Emotional cutoff

Emotional cutoff is managing unresolved fusion by distancing from family, the opposite of being drawn in.

C. Sibling position

Sibling position concerns how birth order shapes functioning, not parental conflict.

D. Societal regression

Societal regression applies Bowen's concepts to society as a whole.

Rationale: In Bowen family systems theory, the triangle is the smallest stable relationship unit; dyads under stress pull in a third person. Bowenian counselors help members increase differentiation of self, often using genograms to trace multigenerational patterns, so they can stay connected without being triangled.

References: Corey (Theory & Practice) — Family Systems Therapy: Bowen's triangulation and differentiation of self

☐ Approve ☐ Revise ☐ Retire Notes:

Parents tell a family counselor they always agree on rules. The counselor asks them to decide, right now in session, whether their son may stay out late Saturday, then watches as the son interrupts and his mother sides with him. In Minuchin's structural family therapy, this technique is called:

A. Prescribing the symptom

Prescribing the symptom is a paradoxical directive from strategic therapy (Haley, Madanes).

B. Enactment ✓ KEY

Enactment has the family act out a transaction in session so the counselor can observe and modify its structure.

C. Detriangulation

Detriangulation is a Bowenian goal of removing a third person from a two-person conflict.

D. Unbalancing

Unbalancing is a structural technique in which the counselor temporarily sides with one member to shift power; that is not what is described.

Rationale: Minuchin's structural family therapy focuses on subsystems, boundaries, and hierarchy. In an enactment, the counselor has family members interact in session rather than report on their interactions, revealing patterns such as a cross-generational coalition that the counselor can then help restructure.

References: Corey (Theory & Practice) — Family Systems Therapy: structural family therapy, enactment

► **Check:** 'Unbalancing' is also a Minuchin technique; confirm the scenario reads clearly as an enactment.

☐ Approve ☐ Revise ☐ Retire Notes:

In the third session of a community anger-management group, members begin arguing about the attendance rule, and two members openly question whether the leader is qualified to run the group. According to Tuckman's model, the group is MOST likely in which stage?

A. Forming

Forming is the orientation stage of politeness, dependence on the leader and tentative participation, before open conflict emerges.

B. Storming ✓ KEY

Storming is marked by conflict, challenges to the leader and struggles over control and rules as members test the group.

C. Norming

Norming follows storming; members have settled conflicts, agreed on norms and developed cohesion.

D. Performing

Performing is the productive stage in which the group works on its goals with little energy spent on conflict.

Rationale: Tuckman (1965; adjourning added with Jensen in 1977) described forming, storming, norming, performing and adjourning. Conflict over rules and direct challenges to the leader's authority are the hallmark of storming, which leaders should treat as a normal developmental step rather than a sign the group is failing.

References: Gladding (Counseling: A Comprehensive Profession) — Group work: Tuckman stages of group development

☐ Approve ☐ Revise ☐ Retire Notes:

During the transition stage of a counseling group, a member crosses his arms and says to the leader, 'Honestly, this group is a waste of my time.' Other members fall silent. Which leader response BEST fits Corey's guidance for this stage?

A. Remind him that the group rules ask members to stay positive and to avoid discouraging the others in the room.

Invoking a rule to suppress the comment shuts down a challenge the group needs to work through and models avoidance of conflict.

B. Move to a structured exercise so the silence ends quickly and the group can return to the planned topic of the day.

Changing the subject bypasses the conflict; unaddressed resistance in the transition stage tends to go underground and erode trust.

C. Ask the other members whether they agree with him so that the group can vote on whether to keep meeting each week.

Polling members turns a personal reaction into a referendum, can isolate the member and puts the group itself on trial.

D. Respond without defensiveness and invite him to say more about what isn't working for him in the group so far. ✓ KEY

Corey advises leaders to meet challenges openly and non-defensively, treating resistance as useful information and modeling direct, respectful handling of conflict.

Rationale: In Corey's model the transition stage is characterized by anxiety, defensiveness, resistance and struggles for control, including challenges to the leader. The leader's task is to recognize and explore these reactions directly and non-defensively, which builds the trust needed to move into the working stage.

References: Corey (Theory & Practice) — Group stages: transition stage, dealing with resistance and challenges to the leader

☐ Approve ☐ Revise ☐ Retire Notes:

A high school counselor plans a six-week group in which students learn and practice specific study and test-anxiety management skills, with brief lessons and structured activities each week. Under the ASGW classification of group work, this is BEST described as which type of group?

A. Psychoeducation group ✓ KEY

Psychoeducation groups emphasize teaching information and skills, often to prevent problems, using structured content and activities.

B. Task group

Task and work groups focus on accomplishing an organizational goal, such as a committee or planning team, rather than member learning.

C. Counseling group

Counseling groups address interpersonal and developmental concerns through member interaction, with less didactic structure.

D. Psychotherapy group

Psychotherapy groups address more severe or chronic problems and personality reconstruction, which this skills group does not target.

Rationale: The ASGW identifies four group types: task/work, psychoeducation, counseling and psychotherapy. A time-limited, structured group whose main aim is teaching skills to a nonclinical population is a psychoeducation group.

References: Gladding (Counseling: A Comprehensive Profession) — Group work: ASGW types of groups

☐ Approve ☐ Revise ☐ Retire Notes:

After a member of a grief group shares that she still sets a place at the table for her late husband, three other members say they have done something similar. She replies, 'I thought I was the only one.' Which of Yalom's therapeutic factors is MOST evident?

A. Instillation of hope

Instillation of hope comes from seeing others improve or cope well, not from discovering that others share the same experience.

B. Altruism

Altruism is the benefit of helping others; the member here is the one receiving relief rather than giving help.

C. Universality ✓ KEY

Universality is the relief of learning that one is not alone or unique in one's problems, thoughts or feelings.

D. Catharsis

Catharsis is the release of strong emotion; the key experience described is recognizing shared experience.

Rationale: Yalom identified 11 therapeutic factors. Universality, the discovery that others share one's 'secret' struggles, often appears early in groups and reduces shame and isolation, which is why the member's statement that she thought she was the only one points to it.

References: Yalom & Leszcz (Group Psychotherapy) — Ch. 1: The therapeutic factors, universality

☐ Approve ☐ Revise ☐ Retire Notes:

In a young adult counseling group, two members repeatedly press a quieter member to 'finally tell us what really happened' with her family, even after she says she is not ready. Which leadership skill is MOST needed at this moment?

A. Linking, by connecting her reluctance to similar feelings that others in the group have described

Linking builds connection among members but does not stop the pressure being placed on her right now.

B. Drawing out, by inviting the quiet member to share a little more so the others feel heard

Drawing her out would join the pressure; she has already stated a boundary about disclosure.

C. Blocking, by stopping the pressure and affirming her right to decide what and when to share ✓ KEY

Blocking protects members from counterproductive behavior such as coercive questioning while respecting their autonomy over disclosure.

D. Summarizing, by restating the main points of the discussion before moving to another member

Summarizing may move the group along but does not address the coercion or protect the member.

Rationale: Group leaders have a duty to protect members from undue pressure and from behaviors that threaten safety. Blocking intervenes in harmful patterns, such as pressuring, gossiping or breaking confidences, without attacking the members doing it.

References: Corey (Theory & Practice) — Group leadership skills: blocking and protecting members · ACA Code of Ethics — A.9.b Protecting Clients (group work)

☐ Approve ☐ Revise ☐ Retire Notes:

In a long-term interpersonal group, a man reacts to the male co-leader with the same deference and hidden resentment he felt toward his father, and competes with an older member as he did with his brother. Over months he recognizes the pattern and responds to them differently. Which of Yalom's therapeutic factors does this process BEST illustrate?

A. Corrective recapitulation of the primary family group ✓ KEY

The group re-creates early family dynamics, and the member relives and then reworks those patterns in a new, corrective way.

B. Imitative behavior modeled on the co-leader and members

Imitative behavior means adopting behaviors seen in others; the core change here is reworking old family-based roles.

C. Socializing techniques learned through group role-plays

Socializing techniques are basic social skills learned in the group, not the reworking of family-of-origin patterns.

D. Existential factors, such as accepting final responsibility

Existential factors involve accepting responsibility, mortality and life's unfairness, not reliving family roles.

Rationale: Yalom describes groups as resembling a family in many ways, so members often relate to leaders and peers as they did to parents and siblings. When those patterns are not merely relived but explored and corrected, the factor is corrective recapitulation of the primary family group.

References: Yalom & Leszcz (Group Psychotherapy) — Ch. 1: Corrective recapitulation of the primary family group

☐ Approve ☐ Revise ☐ Retire Notes:

In Egan's SOLER guideline for nonverbal attending, what does the letter 'L' stand for?

A. Listen without interrupting

Uninterrupted listening matters, but SOLER describes physical posture, and L is not listening.

B. Look at the client's face

Eye contact is the E in SOLER, not the L.

C. Limit note-taking in session

Note-taking is not one of the SOLER behaviors.

D. Lean slightly toward the client ✓ KEY

L stands for leaning toward the client, a posture that signals involvement and interest.

Rationale: SOLER stands for Squarely face the client, Open posture, Lean toward the client, Eye contact and Relaxed manner. Egan presents these as flexible guidelines; the counselor adapts them to the client's culture and comfort, since direct eye contact or close posture is not welcome in every context.

References: Gladding (Counseling: A Comprehensive Profession) — Building the counseling relationship: attending behavior (SOLER)

☐ Approve ☐ Revise ☐ Retire Notes:

A practicum student is eager to start using confrontation and interpretation with clients. According to Ivey's microskills hierarchy, which skills should the student master FIRST?

A. Reflection of meaning and the interpretation of core themes

Reflection of meaning and interpretation sit higher in the hierarchy and rely on the listening foundation.

B. Attending behavior and the basic listening sequence ✓ KEY

Attending and the basic listening sequence are the foundation on which higher-level influencing skills are built.

C. Influencing skills such as directives and feedback

Influencing skills are later in the hierarchy and are effective only after the client feels heard.

D. Skill integration and a personal style of counseling

Skill integration and personal style form the top of the hierarchy, reached after the other skills are mastered.

Rationale: Ivey's hierarchy rests on ethics, multicultural competence and wellness, then attending behavior, then the basic listening sequence (questions, observation, encouraging, paraphrasing, summarizing, reflecting feeling). Focusing, empathic confrontation, reflection of meaning, influencing skills and skill integration build on that base.

References: Gladding (Counseling: A Comprehensive Profession) — Counseling skills: Ivey microskills hierarchy

► **Check:** Ivey's editions differ slightly on the hierarchy's base; the key holds across editions.

☐ Approve ☐ Revise ☐ Retire Notes:

A client says, 'My supervisor criticized my report in front of the whole team again, and I just sat there and said nothing.' Which counselor response is the BEST example of a reflection of feeling?

A. You felt embarrassed, and maybe angry with yourself for not speaking up. ✓ **KEY**

This names the likely emotions behind the statement, which is the defining feature of reflecting feeling.

B. Your supervisor criticized your report in front of the team once again.

This restates the content of the message, which makes it a paraphrase rather than a reflection of feeling.

C. What do you think stopped you from saying something to your supervisor?

This is an open question that explores reasons; it does not reflect the emotion being expressed.

D. Next time, it might help to ask your supervisor to talk with you privately.

This is advice-giving, an influencing response that skips past the client's feelings.

Rationale: A reflection of feeling identifies and names the emotion in the client's message, often tentatively, so the client feels understood and can explore it further. A paraphrase captures content; questions and advice are different skills.

References: Gladding (Counseling: A Comprehensive Profession) — Counseling skills: reflection of feeling versus paraphrasing

☐ Approve ☐ Revise ☐ Retire Notes:

Midway through a session, a client has moved quickly among worries about money, conflict with her sister and trouble sleeping. The counselor wants to pull these threads together, check accuracy and help the client choose a focus. Which skill is MOST suited to this purpose?

A. Paraphrasing

Paraphrasing restates the content of a single statement rather than tying together several themes across the session.

B. Reflection of meaning

Reflection of meaning explores values and deeper significance, not the organization of several topics into a focus.

C. Summarizing ✓ **KEY**

Summarizing condenses several statements or themes, checks understanding and helps set direction or transition.

D. Encouraging

Minimal encouragers keep the client talking; they do not integrate content across topics.

Rationale: Summarizing draws together the main content and feelings from a longer stretch of talk. It is used to begin or end sessions, to make transitions and to help a client who has raised many concerns decide what to work on.

References: Gladding (Counseling: A Comprehensive Profession) — Counseling skills: summarizing

☐ Approve ☐ Revise ☐ Retire Notes:

A client who is tearful and twisting a tissue says, 'I'm completely fine with the divorce. It's no big deal.' Which response BEST illustrates empathic confrontation?

A. You aren't being honest with me or yourself, because you are clearly upset right now.

This accuses and judges the client, which is likely to raise defensiveness rather than invite exploration.

B. Divorce is one of the most stressful life events, so being upset would be quite normal.

This normalizes and reassures, but it does not name the discrepancy between the words and the behavior.

C. Why do you think you are telling me it is no big deal when it seems to matter a lot?

"Why" questions tend to put clients on the defensive and ask for justification rather than reflection.

D. You're saying the divorce is no big deal, and you're also in tears as we talk about it. ✓ **KEY**

This nonjudgmentally places the verbal message beside the nonverbal one and lets the client explore the mixed message.

Rationale: Empathic confrontation identifies discrepancies, mixed messages or conflicts (often with an "on the one hand ... and on the other" format) in a supportive, nonjudgmental way. Its aim is to help the client notice and resolve the incongruity, not to win an argument.

References: Gladding (Counseling: A Comprehensive Profession) — Counseling skills: confrontation of discrepancies

☐ Approve ☐ Revise ☐ Retire Notes:

A counselor says, 'I notice that whenever we get close to talking about your father, you change the subject and glance at me as if checking my reaction. I'm wondering what is happening between us right now.' This statement is BEST described as:

A. Self-disclosure

Self-disclosure shares information about the counselor's own life; here the focus is the current relationship.

B. Immediacy ✓ **KEY**

Immediacy addresses what is occurring in the here-and-now between counselor and client.

C. Interpretation

Interpretation offers a new frame for the client's behavior; this response invites discussion of the present interaction instead.

D. Reframing

Reframing recasts a situation in a different light; it does not focus on the counselor-client relationship.

Rationale: Immediacy is the counselor's direct, present-tense comment on the counseling relationship or on a pattern unfolding in the session. It is useful when trust, tension, dependence or avoidance in the relationship is blocking progress.

References: Gladding (Counseling: A Comprehensive Profession) — Counseling skills: immediacy

☐ Approve ☐ Revise ☐ Retire Notes:

In a child-centered play therapy session, a 6-year-old grows frustrated and raises a wooden block as if to throw it at the counselor. Using Landreth's ACT model of limit setting, which response is BEST?

A. If you throw that block, our playtime is over for today and we will go back out.

Leading with a consequence skips acknowledging the feeling and offering an alternative; consequences come only if limits are repeatedly broken.

B. We don't throw blocks in here. Can you please put it back on the shelf for me?

This states a limit but ignores the child's feeling and gives no acceptable outlet for it.

C. You're really mad. I'm not for throwing. You can throw the soft ball at the wall. ✓ **KEY**

This acknowledges the feeling, communicates the limit and targets an acceptable alternative, the three steps of ACT.

D. It sounds like you are feeling frustrated with how the game is going for you today.

Reflecting the feeling alone leaves the counselor and child unprotected; a limit is needed when safety is at stake.

Rationale: Landreth's ACT model: Acknowledge the child's feeling, Communicate the limit and Target an acceptable alternative. Limits in child-centered play therapy are few, are stated calmly and briefly, and protect physical safety and the relationship while leaving the feeling itself accepted.

References: Gladding (Counseling: A Comprehensive Profession) — Counseling children: child-centered play therapy and limit setting

► **Check:** The key's wording is in Landreth's style, not a direct quote.

☐ Approve ☐ Revise ☐ Retire Notes:

A play therapist spends early sessions building an egalitarian relationship with a 9-year-old, then gathers information about the child's lifestyle and mistaken beliefs, helps the child gain insight, and finally uses reorientation to practice new ways of belonging. This sequence is characteristic of which approach?

A. Adlerian play therapy ✓ **KEY**

Kottman's Adlerian play therapy proceeds through relationship building, exploring lifestyle, gaining insight and reorientation/reeducation.

B. Child-centered play therapy

Child-centered play therapy is nondirective and does not assess lifestyle or work toward planned reeducation phases.

C. Theraplay

Theraplay is a directive, attachment-based approach involving caregivers in structured engagement and nurture activities.

D. Filial therapy

Filial therapy trains parents to conduct child-centered play sessions; it is not organized around lifestyle analysis.

Rationale: Terry Kottman's Adlerian play therapy integrates Adlerian concepts (lifestyle, mistaken beliefs, the Crucial Cs, encouragement) with play. Its four phases are building an egalitarian relationship, exploring the child's lifestyle, helping the child gain insight and reorientation/reeducation.

References: Gladding (Counseling: A Comprehensive Profession) — Counseling children: approaches to play therapy (Adlerian, child-centered, filial)

► **Check:** Kottman's Adlerian play therapy phases are cited via Gladding, which may not cover Kottman in depth.

☐ Approve ☐ Revise ☐ Retire Notes:

A college student arrives at the counseling center hours after learning his scholarship was revoked, saying he 'can't see a way forward.' Following Roberts' seven-stage crisis intervention model, what should the counselor do FIRST?

A. Help him generate options for funding and plan his next steps with financial aid

Generating and exploring alternatives is stage 5, after safety, rapport, problems and feelings have been addressed.

B. Conduct a thorough psychosocial and lethality assessment, including suicide risk ✓ **KEY**

Stage 1 of Roberts' model is a biopsychosocial and lethality assessment; his hopeless statement makes this urgent.

C. Explore and validate his feelings of shock, anger and shame about the revocation

Dealing with feelings and emotions is stage 4 of the model, not the first step.

D. Identify the main problems and the precipitating event that led him to come in today

Identifying major problems and precipitants is stage 3, after assessment and rapport building.

Rationale: Roberts' seven stages are: (1) plan and conduct a thorough biopsychosocial and lethality assessment, (2) establish rapport, (3) identify major problems and precipitants, (4) deal with feelings and emotions, (5) generate and explore alternatives, (6) implement an action plan and (7) follow up. Stages can overlap, but safety assessment comes first.

References: Gladding (Counseling: A Comprehensive Profession) — Crisis counseling: Roberts' seven-stage crisis intervention model

☐ Approve ☐ Revise ☐ Retire Notes:

A family is seen by a crisis counselor two days after their apartment was destroyed in a fire. Which goal BEST reflects the primary aim of crisis intervention?

A. Uncover early losses that make the family vulnerable to grief and resolve them

Exploring early history is a goal of longer-term therapy and can overwhelm people in acute crisis.

B. Restructure family roles and boundaries that may have contributed to stress

Structural change in family organization is a family therapy goal, not the immediate aim of crisis work.

C. Complete a diagnostic evaluation of each member to begin long-term treatment

Crisis intervention is brief and focused on coping; a crisis reaction is not by itself a disorder requiring diagnosis.

D. Restore the family to at least its pre-crisis level of coping and functioning ✓ **KEY**

Crisis intervention seeks to stabilize people and restore equilibrium, at least to the pre-crisis level of functioning.

Rationale: A crisis is a time-limited state in which usual coping is overwhelmed. Crisis intervention is brief, active and present-focused, aiming to ensure safety, mobilize resources and coping and return people to at least their pre-crisis level of functioning; deeper personality or system change is left for later work if needed.

References: Gladding (Counseling: A Comprehensive Profession) — Crisis counseling: goals of crisis intervention

☐ Approve ☐ Revise ☐ Retire Notes:

On Holland's RIASEC hexagon, which pair of personality types is MOST consistent, meaning the two types share the most characteristics?

A. Realistic and Investigative ✓ KEY

Realistic and Investigative are adjacent on the hexagon, so they share the most traits and form a consistent pair.

B. Realistic and Social

Realistic and Social sit opposite each other on the hexagon and are the least consistent pairing.

C. Artistic and Conventional

Artistic and Conventional are opposite types, one valuing self-expression and the other order and structure.

D. Investigative and Enterprising

Investigative and Enterprising are opposite each other on the hexagon, so they are inconsistent.

Rationale: Holland arranged the six types in the order R-I-A-S-E-C around a hexagon. Adjacent types are most consistent, alternate types are intermediate and opposite types (R-S, I-E, A-C) are least consistent. Holland's related ideas include congruence (fit between person and environment) and differentiation.

References: Sharf (Career Development Theory) — Holland's theory of types: the hexagon and consistency

☐ Approve ☐ Revise ☐ Retire Notes:

A 52-year-old nurse manager tells a counselor that her main concerns are holding on to the position she has built, keeping her clinical skills current and innovating within her role. In Super's life-span, life-space theory, these concerns BEST fit which career stage?

A. Establishment

Establishment (about ages 25 to 44) centers on stabilizing in and advancing within a chosen field.

B. Exploration

Exploration (about ages 15 to 24) involves crystallizing, specifying and implementing an occupational choice.

C. Maintenance ✓ KEY

Maintenance (about ages 45 to 64) involves holding, updating and innovating to preserve one's position.

D. Disengagement

Disengagement (about 65 and older) involves decelerating, retirement planning and retirement living.

Rationale: Super's stages are growth, exploration, establishment, maintenance and disengagement, each with developmental tasks; the maintenance tasks are holding, updating and innovating. Super treated the stage ages as approximate, and he stressed that people recycle through stages and that work is one of several life roles shown in the life-career rainbow.

References: Sharf (Career Development Theory) — Super's life-span, life-space theory: maintenance stage tasks

☐ Approve ☐ Revise ☐ Retire Notes:

A client is anxious because she has no clear five-year career plan. The counselor helps her see that past opportunities arose from unexpected events, and encourages curiosity, persistence, flexibility, optimism and risk taking to create and use future chance events. This approach reflects:

A. Theory of work adjustment

The theory of work adjustment (Davis and Lofquist) addresses correspondence between worker needs and job reinforcers.

B. Planned happenstance theory ✓ KEY

Planned happenstance (Mitchell, Levin and Krumboltz) teaches clients to create and capitalize on unplanned events.

C. Circumscription and compromise

Gottfredson's theory explains how children narrow options by sex type and prestige, not how to use chance events.

D. Cognitive information processing

CIP (Sampson, Peterson and colleagues) focuses on the pyramid of information processing and the CASVE decision cycle.

Rationale: Planned happenstance grew out of Krumboltz's social learning theory of career decision making. It reframes indecision as open-mindedness and builds five skills (curiosity, persistence, flexibility, optimism and risk taking) so that clients can turn unplanned events into career opportunities.

References: Sharf (Career Development Theory) — Krumboltz's social learning theory and planned happenstance

☐ Approve ☐ Revise ☐ Retire Notes:

A 4-year-old eagerly plans games and starts projects, but his parents often scold him for 'making messes' when he tries new activities. In Erikson's theory, this child is working on which psychosocial crisis?

A. Autonomy vs. shame and doubt

Autonomy vs. shame and doubt is the toddler stage (about 1 to 3 years), centered on self-control and independence.

B. Industry vs. inferiority

Industry vs. inferiority is the school-age stage (about 6 to 12 years), centered on competence and productivity.

C. Trust vs. mistrust

Trust vs. mistrust is the infancy stage (birth to about 1 year), centered on reliable caregiving.

D. Initiative vs. guilt ✓ KEY

Initiative vs. guilt (about 3 to 6 years) involves planning and starting activities; harsh criticism can foster guilt.

Rationale: Erikson's eight stages are trust vs. mistrust (infancy), autonomy vs. shame and doubt (toddlerhood), initiative vs. guilt (preschool), industry vs. inferiority (school age), identity vs. role confusion (adolescence), intimacy vs. isolation (young adulthood), generativity vs. stagnation (middle adulthood) and integrity vs. despair (late adulthood).

References: Berk (Development Through the Lifespan) — Erikson's psychosocial theory: initiative vs. guilt in early childhood

☐ Approve ☐ Revise ☐ Retire Notes:

An 8-year-old correctly explains that a ball of clay has the same amount of clay after it is rolled into a long snake, but she struggles with a purely hypothetical question about what would happen if people had no need for sleep. In Piaget's theory, she is MOST likely in which stage?

A. Concrete operational ✓ KEY

The concrete operational child (about 7 to 11) masters conservation and logical operations on concrete objects but not abstract hypotheticals.

B. Preoperational

Preoperational children (about 2 to 7) typically fail conservation tasks because of centration.

C. Formal operational

Formal operational thinkers (about 11 and older) can reason about abstract and hypothetical propositions, which she cannot yet do.

D. Sensorimotor

The sensorimotor stage (birth to about 2) involves learning through senses and actions, before symbolic thought.

Rationale: Piaget's four stages are sensorimotor (birth to 2), preoperational (2 to 7), concrete operational (7 to 11) and formal operational (11 and up). Conservation marks concrete operations; abstract, hypothetico-deductive reasoning marks formal operations.

References: Berk (Development Through the Lifespan) — Piaget's cognitive-developmental theory: concrete operational stage

☐ Approve ☐ Revise ☐ Retire Notes:

Asked whether a man should steal an unaffordable medicine to save his wife, a 16-year-old answers, 'No, because if everyone broke the law when they felt like it, society would fall apart.' This reasoning BEST fits which Kohlberg stage?

A. Stage 2: Instrumental exchange

Stage 2 (preconventional) reasoning focuses on self-interest and fair deals, not on maintaining social order.

B. Stage 3: Interpersonal conformity

Stage 3 reasoning centers on being seen as good and winning the approval of others.

C. Stage 4: Law and order orientation ✓ KEY

Stage 4 (conventional) reasoning upholds laws and duties because they maintain the social system.

D. Stage 5: Social contract

Stage 5 (postconventional) sees laws as changeable agreements that can be set aside to protect basic rights such as life.

Rationale: Kohlberg described three levels with two stages each: preconventional (1 punishment and obedience, 2 instrumental purpose), conventional (3 interpersonal conformity, 4 social-order maintenance or law and order) and postconventional (5 social contract, 6 universal ethical principles). Stage is judged by the reasoning given, not by the yes-or-no answer.

References: Berk (Development Through the Lifespan) — Kohlberg's theory of moral development: conventional level

☐ Approve ☐ Revise ☐ Retire Notes:

In the Strange Situation, a 14-month-old stays close to her mother and explores little. She becomes very distressed when her mother leaves, and on reunion she clings to her mother while also pushing away and crying angrily. Which attachment classification BEST fits?

A. Secure

Securely attached infants explore using the parent as a base and are readily comforted on reunion.

B. Anxious-ambivalent (resistant) ✓ KEY

Resistant infants explore little, are highly distressed at separation and both seek and resist contact on reunion.

C. Avoidant

Avoidant infants show little distress at separation and ignore or avoid the parent on reunion.

D. Disorganized/disoriented

Disorganized infants show contradictory, confused behaviors such as freezing or approaching with the head averted.

Rationale: Ainsworth's Strange Situation identified secure, avoidant and anxious-ambivalent (resistant) patterns; Main and Solomon later added disorganized/disoriented. Mixing angry resistance with contact seeking on reunion is the signature of the resistant pattern, often linked to inconsistent caregiving.

References: Berk (Development Through the Lifespan) — Attachment: Ainsworth Strange Situation classifications

☐ Approve ☐ Revise ☐ Retire Notes:

A counselor reviewing an infant assessment reads that, on reunion with a parent, the 12-month-old approached with his face turned away, then froze with a dazed expression. Which statement about this pattern is MOST accurate?

A. It is Ainsworth's original avoidant category, linked mainly to warm but busy caregivers

Avoidant infants show organized avoidance, and the pattern is linked to rejecting, not warm-but-busy, caregiving.

B. It reflects a secure pattern in a temperamentally shy infant who needs time to warm up

Freezing and approach-avoidance are not features of secure attachment, regardless of temperament.

C. It is the resistant pattern, defined by clinging mixed with anger toward the caregiver

Resistant infants show organized ambivalence; the freezing and averted approach described here is different.

D. It fits the disorganized category added by Main and Solomon, tied to frightening care ✓ KEY

Disorganized attachment involves contradictory or dazed behavior and is associated with frightened or frightening caregiving.

Rationale: Main and Solomon identified disorganized/disoriented attachment in infants whose behavior did not fit Ainsworth's three organized patterns. The infant lacks a coherent strategy, often because the caregiver is a source of both comfort and fear, as in maltreatment or unresolved parental trauma.

References: Berk (Development Through the Lifespan) — Attachment: disorganized/disoriented attachment (Main & Solomon)

☐ Approve ☐ Revise ☐ Retire Notes:

The original Sue, Arredondo and McDavis multicultural competencies were organized around attitudes and beliefs, knowledge and skills. Which additional aspirational competency did the Multicultural and Social Justice Counseling Competencies (MSJCC) add?

A. Action ✓ KEY

The MSJCC framework adds action (AKSA), calling counselors to engage in advocacy and intervention at multiple levels.

B. Awareness

Awareness of one's own cultural values and biases was already central to the original competencies.

C. Assessment

Culturally appropriate assessment is part of skills, not a separate aspirational competency in the MSJCC.

D. Accreditation

Accreditation is a program-level standard, not one of the aspirational competencies.

Rationale: The MSJCC (Ratts and colleagues, endorsed by ACA in 2015) uses quadrants of privileged and marginalized counselor and client identities and four developmental domains: counselor self-awareness, client worldview, the counseling relationship, and counseling and advocacy interventions. Each domain includes attitudes and beliefs, knowledge, skills and action.

References: Sue & Sue (Counseling the Culturally Diverse) — Multicultural and Social Justice Counseling Competencies (MSJCC)

▶ **Check:** MSJCC cited via Sue & Sue; consider the primary source (Ratts et al., 2016) when one is added.

☐ Approve ☐ Revise ☐ Retire Notes:

A counselor notices she feels impatient with clients who defer major decisions to their extended families, and she begins examining how her own upbringing shapes that reaction. In the Sue, Arredondo and McDavis framework, this work MOST directly reflects which competency area?

A. Understanding the worldview of the culturally different client

Understanding the client's worldview is a separate area; her focus here is on her own conditioning.

B. Developing culturally appropriate intervention strategies

Intervention strategies concern what the counselor does in session, not examining her own reactions.

C. Awareness of her own cultural values, assumptions and biases ✓ KEY

Examining how her own background shapes reactions to clients is counselor self-awareness, the first competency area.

D. Knowledge of family structures common among collectivist cultures

Knowledge of client cultures matters, but the activity described is reflection on her own biases.

Rationale: Sue, Arredondo and McDavis (1992) described three characteristics of the culturally skilled counselor: awareness of one's own assumptions, values and biases; understanding the worldview of culturally different clients; and developing appropriate intervention strategies. Each is assessed across beliefs and attitudes, knowledge and skills.

References: Sue & Sue (Counseling the Culturally Diverse) — Multicultural counseling competencies: counselor awareness of own assumptions, values and biases

☐ Approve ☐ Revise ☐ Retire Notes:

Months after a racist incident on campus shook her belief that race did not matter, a Black college student now surrounds herself with Black-centered organizations, studies Black history intensely and expresses anger toward White people. In Cross's Nigrescence model, she is MOST likely in which stage?

A. Pre-encounter

Pre-encounter involves low race salience or pro-White attitudes, which she has moved beyond.

B. Immersion-emersion ✓ KEY

Immersion-emersion involves intense involvement in Black culture and often anger toward White people.

C. Encounter

The encounter is the event, here the incident, that challenged her prior worldview; she has moved past it.

D. Internalization

Internalization brings a secure, less reactive Black identity and openness to other groups.

Rationale: Cross's model describes pre-encounter, encounter, immersion-emersion and internalization (with internalization-commitment for sustained action). After an encounter disrupts the old identity, the person often immerses in Black culture and withdraws from or rejects White culture before reaching a more secure, internalized identity.

References: Sue & Sue (Counseling the Culturally Diverse) — Racial/cultural identity development: Cross Nigrescence model

☐ Approve ☐ Revise ☐ Retire Notes:

After a multicultural course, a White counseling student feels guilt and confusion. She now sees racism around her, yet feels torn between loyalty to family members who make racist jokes and her new beliefs. In Helms' White racial identity model, this experience BEST fits which status?

A. Contact

Contact involves obliviousness to racism and a naive, color-blind view, which she has moved past.

B. Reintegration

Reintegration resolves the conflict by retreating to beliefs in White superiority and blaming people of color.

C. Pseudo-independence

Pseudo-independence is an intellectual commitment to change that comes after the conflict of disintegration.

D. Disintegration ✓ KEY

Disintegration involves guilt, confusion and conflict between new awareness of racism and loyalty to one's own group.

Rationale: Helms' White racial identity statuses are contact, disintegration, reintegration, pseudo-independence, immersion/emersion and autonomy. Disintegration is the uncomfortable phase in which new awareness of racism collides with prior beliefs and group loyalties, producing guilt, shame and moral dilemmas.

References: Sue & Sue (Counseling the Culturally Diverse) — White racial identity development: Helms' model

☐ Approve ☐ Revise ☐ Retire Notes:

At an intake, a client with a history of assault becomes visibly tense when the counselor closes the office door and moves her chair to sit between the client and the exit. Which response BEST reflects trauma-informed practice?

A. Name what she noticed, explain the reason for privacy and ask how the client wants the room set up ✓ **KEY**

This offers transparency, voice and choice, restoring a sense of safety and control, which are core trauma-informed principles.

B. Continue the intake as planned so the client can see the office is safe and her anxiety will fade

Pressing on ignores a cue of distress and puts the counselor's agenda ahead of the client's sense of safety.

C. Begin a detailed account of the assault now so that the tension can be processed early in treatment

Pushing for trauma details at intake, before safety and stabilization, risks retraumatization.

D. Reassure her that the building is secure and that no one can come in while the door is fully closed

Reassurance alone does not give the client any control and may increase her sense of being trapped.

Rationale: SAMHSA's trauma-informed approach rests on safety; trustworthiness and transparency; peer support; collaboration and mutuality; empowerment, voice and choice; and attention to cultural, historical and gender issues. Small environmental choices, explained openly, help clients regain control and prevent retraumatization.

References: Gladding (Counseling: A Comprehensive Profession) — Trauma and crisis counseling: principles of trauma-informed care

► **Check:** SAMHSA's six trauma-informed principles are cited via Gladding (SAMHSA TIP 57 is not on the approved list).

☐ Approve ☐ Revise ☐ Retire Notes:

A client has met her goals for managing panic after 14 sessions, and she and the counselor agree to end counseling. Which approach to the final sessions is BEST?

A. End at the next session with a brief goodbye to avoid fostering dependence on counseling

An abrupt ending skips consolidation and relapse planning and gives no room to process feelings about ending.

B. Review her gains, anticipate setbacks, plan to keep progress and discuss feelings about ending ✓ **KEY**

Effective termination consolidates learning, prepares for relapse, addresses feelings and clarifies how to return if needed.

C. Begin work on new goals so that she stays engaged and does not lose momentum after her progress

Adding goals once the agreed goals are met and the client no longer needs services is not in her interest.

D. Reduce session length gradually while avoiding talk about the ending so she is not made anxious

Avoiding the topic of ending denies the client a chance to process it and to plan for maintaining gains.

Rationale: Termination is a planned phase, not an event. Counselors review progress, help clients attribute change to their own efforts, anticipate challenges and relapse, explore feelings about ending and leave the door open, consistent with ACA guidance to terminate when services are no longer needed and to provide pretermination counseling.

References: ACA Code of Ethics — A.11.c Appropriate Termination · Gladding (Counseling: A Comprehensive Profession) — Termination of counseling relationships

☐ Approve ☐ Revise ☐ Retire Notes:

A counselor is interviewing applicants for a 10-week interpersonal process group. Under the ACA Code of Ethics, the main purpose of this pre-group screening is to:

A. confirm that each applicant has a diagnosis that the group can be billed for

Billing eligibility is an administrative matter; the Code frames screening around fit, group process and member welfare.

B. collect a detailed history so the leader can interpret members in session

Some history may be gathered, but screening exists to select suitable members, not to prepare interpretations.

C. select members whose goals fit the group and who will not be harmed by it ✓ **KEY**

A.9.a directs counselors to select members whose needs and goals are compatible with the group, who will not impede it and whose well-being will not be jeopardized.

D. make sure that every member shares the same cultural background and age range

Homogeneity can be useful for some groups, but it is not the ethical purpose of screening, and diversity often enriches a group.

Rationale: ACA Code A.9.a requires counselors to screen prospective group members and, to the extent possible, select those whose needs and goals are compatible with the group's goals, who will not impede the group process and whose well-being will not be jeopardized by the experience. The ASGW Best Practice Guidelines likewise treat screening and orientation as part of group planning.

References: ACA Code of Ethics — A.9.a Screening · Corey (Theory & Practice) — Group work: pre-group screening and selection of members

☐ Approve ☐ Revise ☐ Retire Notes:

A counselor on an acute inpatient psychiatric unit, where the average stay is five days, is designing a daily coping-skills group. Which group format is MOST appropriate?

A. An open group that admits new members as patients arrive ✓ **KEY**

With rapid turnover, an open group lets patients join and leave as they are admitted and discharged, and each session can stand alone.

B. A closed group that keeps the same members for eight weeks

A closed eight-week group is impossible when patients stay about five days; most would be discharged after a few sessions.

C. A closed group with a waiting list for the next full cycle

A waiting list would keep most short-stay patients from attending at all before discharge.

D. A marathon group that meets once for a full weekend session

A single marathon session does not fit a daily skills program or the unpredictable timing of admissions.

Rationale: Open groups replace members as they leave, which suits settings with high turnover such as inpatient units, though new arrivals can slow the growth of cohesion. Closed groups keep the same members from start to finish, which supports stability and cohesion but requires members who can commit for the full duration.

References: Corey (Theory & Practice) — Group work: open versus closed groups

☐ Approve ☐ Revise ☐ Retire Notes:

An elementary school counselor is forming a counseling group for 7-year-olds whose parents recently divorced. Which group size is generally recommended for a group of children this age?

A. 8 to 10 members

Eight to ten is closer to the size often suggested for adult counseling groups; young children need more individual attention.

B. 3 to 5 members ✓ KEY

Young children have short attention spans and need frequent leader attention, so very small groups are recommended.

C. 12 to 15 members

A group this large would leave young children little time to talk and make the group hard to manage.

D. 20 to 25 members

This is a classroom guidance size, suited to psychoeducation lessons rather than a counseling group.

Rationale: Group size depends on members' age, the leader's experience and the type of group. Groups for young children are kept small (often about 3 to 5) and sessions short; adolescent groups are somewhat larger, and an ongoing adult counseling group of about 8 members with one leader is often cited as workable.

References: Corey (Theory & Practice) — Group work: practical considerations, group size and composition

► **Check:** Verify the recommended size range for young children's groups in the current Corey or Gladding edition (commonly cited as about 3 to 5).

☐ Approve ☐ Revise ☐ Retire Notes:

Q684 of 1000 · nce-s-cou-064 · Group Counseling and Group Work · group leadership skills and therapeutic factors · medium · In review

Two co-leaders of an adult support group notice that members have begun bringing complaints about one leader to the other, and the leaders have started to feel competitive with each other. Which step BEST addresses this co-leadership problem?

A. Divide the members so that each leader is responsible for half of the group

Splitting the group between leaders formalizes the division and leaves the competition unaddressed.

B. Let the more experienced co-leader run the sessions while the other observes

Sidelining one leader avoids the conflict instead of resolving it and wastes the benefits of co-leadership.

C. Agree to hide any differences of opinion from the members during sessions

Respectfully voiced differences can model healthy conflict; concealing them tends to feed the splitting.

D. Meet regularly between sessions to talk openly about their working relationship ✓ KEY

Regular meetings to process their relationship let co-leaders address rivalry and present a united, coordinated approach.

Rationale: Co-leadership offers shared perspective, modeling of a healthy relationship, coverage for absences and mutual support. Its main risks are poor communication, competition and members splitting the leaders. Corey stresses that co-leaders should meet before and after sessions to plan and to process their own relationship.

References: Corey (Theory & Practice) — Group leadership: advantages and disadvantages of co-leadership

☐ Approve ☐ Revise ☐ Retire Notes:

In the fourth session of a men's support group, one member has again taken most of the time telling long stories about his week, while others glance at the clock. Which leader response is MOST appropriate?

A. "You've given us a lot to think about. I'd like to hear how others are reacting to what you said." ✓ KEY

This acknowledges the member without attacking him, blocks the monopolizing and opens the floor to the rest of the group.

B. "Let's keep listening, since he clearly needs the group's time today more than anyone else does."

Letting the pattern continue reinforces monopolizing and ignores the frustration building among other members.

C. "You talk too much in here, and it is unfair to the others. Please stay quiet for the rest of today."

A harsh, shaming confrontation can humiliate the member and makes others fear the same treatment.

D. "I'll speak with you privately after group about whether this group is a good fit for your needs."

Raising removal ignores a workable in-group intervention and treats a common group behavior as grounds for exclusion.

Rationale: Monopolizing is one of the most common problem behaviors in groups. Leaders intervene early and respectfully, blocking the behavior while protecting the member's dignity, and they often invite the group to share reactions so members learn to address the pattern directly.

References: Corey (Theory & Practice) — Group work: problem behaviors, the monopolizing member

☐ Approve ☐ Revise ☐ Retire Notes:

In a teen group, several members begin blaming one boy for the group's lack of progress, citing his lateness and jokes, and they bring him up whenever the discussion becomes tense. Which leader response is BEST?

A. Ask the boy to explain to the group why he keeps arriving late and making jokes

Putting him on the spot joins the group's blaming and deepens his role as the scapegoat.

B. Allow the criticism to continue, since honest feedback helps him change his conduct

Collective blaming is not constructive feedback, and letting it continue fails the leader's duty to protect members.

C. Block the blaming and ask the group to look at what is happening among them now ✓ KEY

The leader protects the targeted member and turns attention to the group process, including the feelings members may be displacing onto him.

D. Move the boy to a different group so that the others can resume productive work

Removing the scapegoat leaves the underlying dynamic in place, and the group often finds a new target.

Rationale: Scapegoating occurs when a group displaces its tension onto one member. The leader blocks the attack to protect that member and then explores what the group is avoiding, which usually involves members' own anxieties or conflicts with the leader or one another.

References: Corey (Theory & Practice) — Group work: scapegoating and protecting members · ACA Code of Ethics — A.9.b Protecting Clients (group work)

☐ Approve ☐ Revise ☐ Retire Notes:

A group leader says, "I notice that each time someone starts to talk about anger, the group quickly shifts to joking. What do you make of that?" This statement is an example of which leader focus?

A. A content focus on the topic under discussion

A content focus addresses what is being discussed, such as the anger itself, rather than how the group handles it.

B. A process focus on how the members interact ✓ **KEY**

The leader is commenting on the pattern of interaction in the room, which is a process observation.

C. An educational focus on the facts about anger

An educational focus would teach information about anger rather than reflect a pattern back to the group.

D. An individual focus on one member's history

The observation is about the group as a whole, not about any one member's background.

Rationale: Content is what the group talks about; process is how members relate to one another while they talk, including patterns of avoidance, alliances and shifts in mood. Process comments help members see their here-and-now behavior and are central to interpersonal group work.

References: Yalom & Leszcz (Group Psychotherapy) — Ch. 6: The here-and-now and process illumination

☐ Approve ☐ Revise ☐ Retire Notes:

A leader says, "Amara, what you just described about feeling invisible at work sounds a lot like what Dev shared last week about his family. Dev, how does Amara's story land for you?" Which group leadership skill is the leader using?

A. Cutting off

Cutting off stops a member who is rambling or going off track; nothing is being stopped here.

B. Drawing out

Drawing out invites a quiet member to participate; the main move here is connecting two members' concerns.

C. Summarizing

Summarizing pulls together the main points of a discussion rather than connecting two members.

D. Linking ✓ **KEY**

Linking connects what one member says or feels to another member's concerns, promoting member-to-member interaction.

Rationale: Linking builds cohesion and universality by pointing out common themes between members and encouraging them to speak to each other rather than only to the leader. Related skills include drawing out quieter members and cutting off, or redirecting, members who stray from the group's focus.

References: Corey (Theory & Practice) — Group leadership skills: linking

☐ Approve ☐ Revise ☐ Retire Notes:

Which statement about group cohesiveness is MOST consistent with Yalom's account of it?

A. Highly cohesive groups can express and work through more conflict and hostility. ✓ KEY

Yalom notes that members of cohesive groups feel secure enough to voice anger and disagreement, which can then be worked through.

B. Cohesiveness is mostly fixed by the end of the first session and changes little later.

Cohesion develops and fluctuates over the life of a group; it is not set in the first meeting.

C. Cohesive groups tend to have poorer attendance because members feel less accountable.

Cohesion is linked to better attendance, greater participation and lower dropout, not poorer attendance.

D. Cohesion is a distinct outcome rather than a condition for other therapeutic factors.

Yalom treats cohesiveness as a precondition for other factors, comparable to the relationship in individual therapy.

Rationale: Yalom describes cohesiveness as the group analogue of the therapeutic relationship: the members' sense of belonging and acceptance. It is associated with better attendance, more self-disclosure, greater risk-taking and better outcomes, and cohesive groups can tolerate open conflict because members trust the group to survive it.

References: Yalom & Leszcz (Group Psychotherapy) — Ch. 3: Group cohesiveness

☐ Approve ☐ Revise ☐ Retire Notes:

Q690 of 1000 · nce-s-cou-070 · Group Counseling and Group Work · group stages and dynamics · medium · In review

Midway through a closed 12-week counseling group, a member emails the leader that she has decided to quit the group and will not return. What is the BEST response from the leader?

A. Tell her she agreed to finish all 12 weeks and must attend until the group ends

Members have the right to leave a group; the leader can encourage but not compel continued attendance.

B. Accept her decision and avoid mentioning her absence so the others stay focused

Unexplained departures affect remaining members, who may wonder whether they caused it; silence leaves this unprocessed.

C. Respect her right to leave while encouraging her to talk it over with the group ✓ KEY

Corey advises leaders to respect members' freedom to leave while encouraging them to discuss the decision with the group rather than simply disappear.

D. Replace her at once with a new member from the waiting list to keep the size stable

Adding a member midway changes the closed format and does nothing to address the departure itself.

Rationale: Members have the right to leave a group, but an abrupt exit can leave both the departing member and the remaining members with unfinished business. Leaders explain this during orientation and, when a member wants to leave, encourage discussion of the decision in the group and process the loss with those who remain.

References: Corey (Theory & Practice) — Group work ethics: freedom to leave a group

☐ Approve ☐ Revise ☐ Retire Notes:

In a training group, members persistently treat the leader as all-knowing, wait passively for direction and become resentful when the leader does not supply answers. In Bion's theory of group functioning, this pattern BEST reflects which basic assumption?

A. Fight-flight

Fight-flight is shown when the group attacks something or escapes the task, for example through hostility or changing the subject.

B. Dependency ✓ KEY

In basic assumption dependency the group acts as if a strong leader will provide for and protect it, and members become passive.

C. Pairing

Pairing is a shared hope that two members will produce a future savior or solution, not reliance on the leader.

D. Work group

The work group is Bion's term for the rational, task-focused mode of functioning, not a basic assumption.

Rationale: Wilfred Bion, working at the Tavistock Clinic, proposed that groups alternate between a work group focused on the task and three unconscious basic assumptions: dependency, fight-flight and pairing. Recognizing a basic assumption lets the leader interpret the group-as-a-whole behavior instead of simply meeting the demand.

References: Yalom & Leszcz (Group Psychotherapy) — Group-as-a-whole approaches: Bion's basic assumptions (Tavistock)

► **Check:** Verify that the cited Yalom & Leszcz chapter (group-as-a-whole/Tavistock discussion) covers Bion's basic assumptions; Corey or Gladding may be a better reference.

☐ Approve ☐ Revise ☐ Retire Notes:

Frank Parsons described wise vocational choice as resting on a clear understanding of oneself, knowledge of the requirements and conditions of different jobs, and which third element?

A. Exploration of chance events that may reshape one's path

Using chance events is central to Krumboltz's planned happenstance, not to Parsons' model.

B. Implementation of the self-concept through a series of roles

Implementing the self-concept in work roles is Super's idea, developed decades after Parsons.

C. Resolution of early parent-child needs through one's work

Linking occupational choice to early parent-child relations is Anne Roe's theory.

D. True reasoning on the relations between these two groups ✓ KEY

Parsons' third step was "true reasoning" about how the person's traits match the requirements of occupations.

Rationale: In *Choosing a Vocation* (1909), Parsons set out three steps: understand yourself, know the world of work and use true reasoning to match the two. This matching approach became trait-factor theory and underlies later person-environment fit models such as Holland's.

References: Sharf (Career Development Theory) — Trait and factor theory: Parsons' three-part model

☐ Approve ☐ Revise ☐ Retire Notes:

A client's interest inventory shows a very high Investigative score, while her other five RIASEC scores are all low and close together. In Holland's theory, this profile is BEST described as showing a high degree of:

A. Differentiation ✓ KEY

Differentiation is the degree to which a profile has clear peaks and valleys; one high score above flat low scores is well differentiated.

B. Consistency

Consistency concerns how close a person's top types sit on the hexagon, which needs at least two high types to judge.

C. Congruence

Congruence is the fit between a person's type and the work environment, which a profile alone does not show.

D. Calculus

Calculus is Holland's assumption that hexagon distances reflect type relationships, not a property of one profile.

Rationale: Holland's secondary constructs include differentiation (how distinct the profile is), consistency (how related the top types are on the hexagon), congruence (person-environment fit), identity (clarity and stability of goals) and calculus. A flat profile is undifferentiated and is often associated with less clear career direction.

References: Sharf (Career Development Theory) — Holland's theory: differentiation, consistency and congruence

☐ Approve ☐ Revise ☐ Retire Notes:

Super used career maturity to describe an adolescent's readiness to make age-appropriate career decisions. Because adults recycle through stages rather than progress in a fixed order, which term did Super propose for readiness to cope with changing work and working conditions in adulthood?

A. Career salience

Salience refers to the importance a person gives to the worker role relative to other life roles.

B. Vocational identity

Vocational identity, the clarity and stability of one's goals and interests, is associated with Holland.

C. Career adaptability ✓ KEY

Super introduced career adaptability as the adult counterpart of career maturity; Savickas later made it central to career construction theory.

D. Career crystallizing

Crystallizing is a developmental task of the exploration stage, not an index of adult readiness.

Rationale: Career maturity, measured by tools such as the Career Development Inventory, compares a young person's coping with developmental tasks to that of peers. Because adult careers involve recycling and transitions, Super suggested career adaptability for adults, a concept Savickas expanded into the adapt-abilities of concern, control, curiosity and confidence.

References: Sharf (Career Development Theory) — Super's theory: career maturity and career adaptability

► **Check:** Confirm attribution: Super proposed career adaptability for adults (early 1980s), later developed by Savickas.

☐ Approve ☐ Revise ☐ Retire Notes:

According to Anne Roe's theory, an adult who grew up in a cold, rejecting or neglectful family climate would be MOST likely to gravitate toward which kind of occupation?

A. Person-oriented fields such as service and business contact

Roe linked warm, accepting or overprotective climates, not cold ones, with orientation toward people.

B. Non-person-oriented fields such as technology and science ✓ KEY

Roe proposed that avoidant (rejecting or neglecting) climates lead toward work that does not center on people.

C. Highly creative fields chosen to win approval from parents

Roe did not tie avoidant climates to approval-seeking creative work; arts and entertainment is a field, not this mechanism.

D. Any field at all, since family climate predicts only level

In Roe's model, family climate shapes orientation toward or away from people; level depends on ability and status factors.

Rationale: Roe classified occupations into eight fields and six levels and proposed that early parent-child relations (emotional concentration, avoidance or acceptance) create needs that steer people toward or away from person-oriented work. Research gave her theory limited support, but her field-and-level classification influenced later occupational systems.

References: Sharf (Career Development Theory) — Roe's personality theory of career choice: parent-child relations

☐ Approve ☐ Revise ☐ Retire Notes:

A 14-year-old who once wanted to be an astronaut now talks about whether she has the grades and abilities for engineering and whether the work fits what she values. In Ginzberg's theory, she is in which period?

A. Fantasy period

The fantasy period (to about age 11) involves play-based choices made without regard to ability or reality.

B. Realistic period

The realistic period (about 17 into early adulthood) involves exploration, crystallization and specification of a choice.

C. Crystallization

Crystallization is a substage of the later realistic period, when a clear occupational pattern emerges.

D. Tentative period ✓ KEY

The tentative period (about 11 to 17) moves through interest, capacity, values and transition substages.

Rationale: Ginzberg, Ginsburg, Axelrad and Herma described three periods: fantasy, tentative (interest, capacity, values, transition) and realistic (exploration, crystallization, specification). Ginzberg later revised the theory to view career choice as a lifelong, open-ended process rather than one fixed by early adulthood.

References: Sharf (Career Development Theory) — Ginzberg's developmental theory: fantasy, tentative and realistic periods

☐ Approve ☐ Revise ☐ Retire Notes:

A client must make a major compromise because her preferred occupations are not available locally. According to Gottfredson's theory of circumscription and compromise, which aspect of her self-concept is she likely to protect MOST when compromising?

A. Sextype, how suitable the job seems for her gender ✓ KEY

Gottfredson proposed that sextype, the earliest and most core element to be circumscribed, is the last to be given up in major compromises.

B. Interest field, how well the work matches her likes

Interests develop later and are more peripheral, so they tend to be sacrificed before prestige or sextype in major compromises.

C. Prestige, the social standing of the occupation

Prestige is protected over interests but is given up before sextype when the compromise is severe.

D. Pay level, the salary that the job typically offers

Salary is not one of the self-concept dimensions in Gottfredson's model of circumscription and compromise.

Rationale: Gottfredson described circumscription in four stages: orientation to size and power, to sex roles, to social valuation and to the internal, unique self. Because sextype is incorporated earliest, it is the most protected aspect; in major compromises people tend to give up interest fit first, then prestige, and sextype last.

References: Sharf (Career Development Theory) — Gottfredson's theory of circumscription and compromise

► **Check:** Gottfredson's 1996/2002 revision makes the order depend on how severe the compromise is; verify the key against the current Sharf wording.

☐ Approve ☐ Revise ☐ Retire Notes:

A counselor helps a client set a career goal, clarify her values, generate several alternatives and gather occupational information. In Krumboltz's social learning theory of career decision making, the counselor is building which factor?

A. Genetic endowment and special abilities

Genetic endowment covers inherited qualities such as physical traits and talents, which counseling does not build.

B. Environmental conditions and events

Environmental conditions, such as job markets and policies, are largely outside the individual's control.

C. Task-approach skills ✓ KEY

Task-approach skills are the learned skills, such as goal setting, values clarification and information seeking, used to approach career tasks.

D. Associative learning experiences

Associative learning links neutral stimuli with positive or negative ones; it is not the set of decision skills described.

Rationale: Krumboltz identified four influences on career decisions: genetic endowment and special abilities, environmental conditions and events, learning experiences (instrumental and associative) and task-approach skills. Their interaction produces self-observation generalizations, worldview generalizations and the actions people take.

References: Sharf (Career Development Theory) — Krumboltz's social learning theory: task-approach skills

☐ Approve ☐ Revise ☐ Retire Notes:

A college senior knows her strengths and has researched several occupations, but each time she nears a choice she tells herself, "I always make the wrong decision," and stalls. In the cognitive information processing (CIP) approach, the counselor should focus FIRST on which domain of the pyramid?

A. Self-knowledge domain

She already knows her strengths; her difficulty is not a lack of self-knowledge.

B. Executive processing domain ✓ KEY

Executive processing covers metacognitions such as self-talk and self-monitoring; her negative self-talk is blocking the process.

C. Occupational knowledge domain

She has already researched occupations, so information about options is not the main barrier.

D. Decision-making skills domain

The CASVE cycle may need work later, but the dysfunctional self-talk at the top of the pyramid is what stalls her.

Rationale: Peterson, Sampson, Reardon and Lenz describe a pyramid with knowledge domains (self and occupations) at the base, decision-making skills (the CASVE cycle: communication, analysis, synthesis, valuing, execution) in the middle and executive processing at the top. Negative career thoughts, measured by the Career Thoughts Inventory, operate at the executive level.

References: Sharf (Career Development Theory) — Cognitive information processing: pyramid and executive processing

☐ Approve ☐ Revise ☐ Retire Notes:

A first-generation college student is confident she can master the coursework for an engineering degree, but she doubts that the degree will lead to a good job for someone without family connections in the field. In social cognitive career theory, her doubt BEST reflects low:

A. Self-efficacy beliefs

Self-efficacy is belief in one's ability to perform; she is confident she can do the work.

B. Personal goals

Goals are intentions to pursue an activity or outcome; her doubt concerns expected results, not intentions.

C. Interest development

In SCCT interests grow from efficacy and outcome expectations; the stem describes a belief about results.

D. Outcome expectations ✓ KEY

Outcome expectations are beliefs about the consequences of an action, such as whether a degree will pay off.

Rationale: Lent, Brown and Hackett's social cognitive career theory builds on Bandura and centers on self-efficacy, outcome expectations and personal goals. Contextual supports and barriers, such as lack of professional networks, shape these beliefs and help explain why capable people may avoid a field.

References: Sharf (Career Development Theory) — Social cognitive career theory: self-efficacy, outcome expectations and goals

☐ Approve ☐ Revise ☐ Retire Notes:

An accountant consistently receives excellent performance reviews, but he tells his counselor that the job offers none of the autonomy or variety he values and he dreads going to work. In the theory of work adjustment, his situation is BEST described as:

A. high satisfactoriness and low satisfaction ✓ KEY

His employer is pleased with his performance (satisfactoriness), but his own needs and values are not being met (satisfaction).

B. high satisfaction and low satisfactoriness

This reverses the terms; satisfaction is the worker's view of the job and satisfactoriness is the employer's view of the worker.

C. low satisfaction and low satisfactoriness

His excellent reviews show that the employer finds him satisfactory.

D. high satisfaction and high satisfactoriness

His dread and unmet values show that he is not satisfied with the job.

Rationale: Dawis and Lofquist's theory of work adjustment holds that tenure depends on correspondence between person and environment. Satisfaction reflects whether the job's reinforcers meet the worker's needs and values; satisfactoriness reflects whether the worker's abilities meet the job's requirements. Low satisfaction predicts that the worker may leave even when performance is strong.

References: Sharf (Career Development Theory) — Theory of work adjustment: satisfaction and satisfactoriness

☐ Approve ☐ Revise ☐ Retire Notes:

A 24-year-old has interests and options but says his parents should decide his career, and he puts off every decision he could make himself. In Savickas's career construction theory, which career adapt-ability is MOST in need of development?

A. Concern

Concern is orientation toward and planning for the future; the main problem described is lack of ownership of decisions.

B. Curiosity

Curiosity is exploring self and options; he already has interests and options to choose from.

C. Control ✓ KEY

Control is taking personal responsibility for one's career; deferring decisions and indecision signal a control deficit.

D. Confidence

Confidence is belief in one's ability to solve problems; the stem points to handing decisions to others rather than self-doubt.

Rationale: Savickas identifies four career adapt-abilities: concern (future orientation), control (decisiveness and ownership), curiosity (exploration) and confidence (self-efficacy for problem solving). Career construction counseling also uses the Career Construction Interview to identify life themes that give meaning to career choices.

References: Sharf (Career Development Theory) — Savickas's career construction theory: career adapt-abilities

☐ Approve ☐ Revise ☐ Retire Notes:

A counselor wants a free online database, sponsored by the U.S. Department of Labor, that replaced the Dictionary of Occupational Titles and describes occupations by tasks, skills, knowledge, work context and interest codes. Which resource fits this description?

A. Occupational Outlook Handbook

The Bureau of Labor Statistics handbook focuses on job outlook, pay and training, and it did not replace the DOT.

B. Occupational Information Network ✓ KEY

*O*NET replaced the DOT and describes occupations with detailed descriptors, including RIASEC interest profiles.*

C. Standard Occupational Classification

The SOC is a federal coding system for classifying jobs in statistical data, not a descriptive career database.

D. Guide for Occupational Exploration

The GOE grouped occupations by interest area and was a DOT companion, not its replacement.

Rationale: O*NET, the Occupational Information Network, is the Department of Labor's primary source of occupational information and replaced the DOT. It links occupations to Holland codes and pairs with labor market information, such as the Occupational Outlook Handbook's projections and wage data.

References: Gladding (Counseling: A Comprehensive Profession) — Career counseling: occupational information sources (O*NET, OOH)

☐ Approve ☐ Revise ☐ Retire Notes:

Q704 of 1000 · nce-s-cou-084 · Human Growth and Development · human development theories (Erikson, Piaget, Kohlberg) · easy · In review

A 9-year-old spends his energy on school, sports and friendships with same-sex peers, and sexual interests seem quiet. In Freud's psychosexual theory, he is in which stage?

A. Phallic stage

The phallic stage (about 3 to 6) centers on the Oedipal conflict and identification with the same-sex parent.

B. Genital stage

The genital stage begins at puberty, when sexual impulses reawaken and turn toward mature relationships.

C. Anal stage

The anal stage (about 1 to 3) centers on toilet training and control.

D. Latency stage ✓ KEY

In latency (about 6 to puberty) sexual drives are dormant and energy goes into skills, school and same-sex friendships.

Rationale: Freud's psychosexual stages are oral (birth to 1), anal (1 to 3), phallic (3 to 6), latency (6 to 11) and genital (adolescence). Erikson's industry vs. inferiority covers the same school-age years, with emphasis on competence rather than sexual drives.

References: Berk (Development Through the Lifespan) — Freud's psychosexual theory: latency stage

☐ Approve ☐ Revise ☐ Retire Notes:

A 19-year-old says he will take over his family's restaurant, as his parents always planned, and that he has never seriously considered any other path. In Marcia's framework, his identity status is:

A. Identity foreclosure ✓ KEY

Foreclosure is commitment without exploration, often accepting goals and values chosen by parents.

B. Identity moratorium

Moratorium is active exploration without commitment yet; he has committed without exploring.

C. Identity achievement

Achievement is commitment that follows a period of exploration, which he has not had.

D. Identity diffusion

Diffusion is neither exploring nor committed; he holds a firm commitment.

Rationale: Marcia extended Erikson's identity stage by crossing two dimensions, exploration and commitment, to form four statuses: achievement (explored, committed), moratorium (exploring, not committed), foreclosure (committed, not explored) and diffusion (neither). Statuses can change over time, and foreclosure is not unhealthy in every cultural context.

References: Berk (Development Through the Lifespan) — Marcia's identity statuses

☐ Approve ☐ Revise ☐ Retire Notes:

A 42-year-old architect is questioning whether the youthful vision he built his career around is worth pursuing, reappraising his marriage and wondering what legacy he will leave. In Levinson's seasons of life, he is MOST likely in the:

A. early adult transition

The early adult transition (about 17 to 22) involves leaving adolescence and forming a first sense of the Dream.

B. midlife transition ✓ KEY

The midlife transition (about 40 to 45) involves reappraising the Dream and the life structure built in early adulthood.

C. age 30 transition

The age 30 transition (about 28 to 33) involves adjusting the first adult life structure, not midlife reappraisal.

D. late adult transition

The late adult transition (about 60 to 65) involves facing aging and moving into late adulthood.

Rationale: Levinson described adulthood as alternating periods of building a life structure and transitions that question it. Key concepts include the Dream, a vision of one's adult future, and the mentor; at the midlife transition, people reappraise the Dream and rework the polarities of young-old, destruction-creation, masculine-feminine and attachment-separateness.

References: Berk (Development Through the Lifespan) — Levinson's seasons of life: the midlife transition

► **Check:** Levinson's period ages are approximate; confirm the midlife transition is given as about 40 to 45.

☐ Approve ☐ Revise ☐ Retire Notes:

An 8-year-old's mother is moved to night shifts by her employer, and the child now spends evenings with less supervision and less help with homework. The child has never been to the workplace. In Bronfenbrenner's ecological systems theory, the employer's decision operates at which level?

A. Microsystem

The microsystem includes settings the child directly takes part in, such as home and school.

B. Mesosystem

The mesosystem is the connections between the child's microsystems, such as home-school links.

C. Exosystem ✓ KEY

The exosystem includes settings the child does not take part in, such as a parent's workplace, that still affect the child.

D. Macrosystem

The macrosystem is the broad cultural values, laws and customs that shape all the other levels.

Rationale: Bronfenbrenner nested the child within the microsystem, mesosystem, exosystem and macrosystem, with the chronosystem adding change over time. A parent's workplace policy is a classic exosystem influence: it shapes the child's daily life even though the child never enters that setting.

References: Berk (Development Through the Lifespan) — Bronfenbrenner's ecological systems theory: the exosystem

☐ Approve ☐ Revise ☐ Retire Notes:

A tutor gives a first-grader step-by-step hints on a puzzle he cannot yet solve alone, then gradually withdraws the hints as he becomes able to solve it himself. This adjustment of support is BEST described as:

A. Accommodation

Accommodation is Piaget's term for changing a mental scheme to fit new information.

B. Private speech

Private speech is a child's self-directed talk used to guide thinking, not the adult's support.

C. Centration

Centration is the preoperational tendency to focus on one aspect of a situation.

D. Scaffolding ✓ KEY

Scaffolding adjusts the level of support to the child's current performance and fades it as competence grows.

Rationale: Vygotsky's zone of proximal development is the range of tasks a child cannot yet do alone but can do with help from a more skilled partner. Scaffolding, a term introduced by Wood, Bruner and Ross and applied to Vygotsky's theory, describes how that help is fine-tuned and withdrawn as the child masters the task.

References: Berk (Development Through the Lifespan) — Vygotsky's sociocultural theory: zone of proximal development and scaffolding

☐ Approve ☐ Revise ☐ Retire Notes:

A woman who has long put everyone else's needs ahead of her own comes to believe that caring responsibly must include caring for herself, and that avoiding harm applies to her as well as to others. In Gilligan's ethic of care, this reasoning BEST reflects which level?

A. Morality of nonviolence ✓ KEY

The third level integrates care for self and others, with a principle of not hurting anyone, including oneself.

B. Goodness as self-sacrifice

This second level equates goodness with meeting others' needs at one's own expense, which she is moving beyond.

C. Orientation to survival

The first level focuses on the self and its survival, without concern for responsibility to others.

D. Social contract reasoning

Social contract reasoning is Kohlberg's stage 5, based on justice and rights rather than care.

Rationale: Gilligan argued that Kohlberg's justice-based stages underrepresented an ethic of care. Her levels move from orientation to individual survival, through goodness as self-sacrifice, to the morality of nonviolence, with transitions from selfishness to responsibility and from goodness to truth about one's own needs.

References: Berk (Development Through the Lifespan) — Gilligan's ethic of care and critique of Kohlberg

► **Check:** Confirm Gilligan's level names (orientation to individual survival, goodness as self-sacrifice, morality of nonviolence) as given in Berk.

☐ Approve ☐ Revise ☐ Retire Notes:

A school counselor is planning supports for children growing up with poverty and parental substance use. Longitudinal research on resilience, such as Werner and Smith's Kauai study, MOST consistently identifies which protective factor?

A. Avoiding all stressful experiences throughout early childhood

Resilience develops through coping with manageable stress; complete avoidance of stress is neither possible nor protective.

B. A warm, stable relationship with at least one caring adult ✓ KEY

A close bond with a competent, caring adult, who need not be a parent, is among the most consistent protective factors.

C. An above-average IQ score as measured in the preschool years

Good cognitive skills can help, but a high IQ alone is not the most consistent protective factor.

D. Living in a family that has no history of mental illness

This describes the absence of a risk factor rather than a protective factor that buffers existing risk.

Rationale: Resilience research identifies protective factors at several levels: personal traits such as an easygoing temperament, a warm relationship with a caring adult, and community supports such as schools and mentors. Werner and Smith found that high-risk children who thrived usually had at least one stable, supportive adult in their lives.

References: Berk (Development Through the Lifespan) — Resilience: protective factors in high-risk children

☐ Approve ☐ Revise ☐ Retire Notes:

A strategic therapist tells a client with chronic insomnia that on any night he is still awake at midnight, he must get up and wax the kitchen floor until morning. The task is deliberately designed to be more unpleasant than the symptom itself. Which intervention and originator does this illustrate?

A. Ordeal therapy, associated with Jay Haley ✓ KEY

Haley's ordeal therapy assigns a task more burdensome than the symptom, so giving up the symptom becomes the easier choice.

B. Unbalancing, associated with Salvador Minuchin

Unbalancing is a structural technique in which the therapist temporarily sides with one family member to shift the hierarchy.

C. Paradoxical intention, from Viktor Frankl

Frankl's paradoxical intention would ask the client to try to stay awake; it does not attach an aversive, effortful task to the symptom.

D. Detriangulation, associated with Murray Bowen

Detriangulation is a Bowenian process of removing oneself from a three-person emotional triangle, unrelated to symptom tasks.

Rationale: Jay Haley described ordeal therapy as a strategic directive in which the client must perform an ordeal, ideally one that is good for the person (exercise, chores), every time the symptom occurs. Because the ordeal costs more than the symptom, the client gives up the symptom. It reflects strategic therapy's focus on directives and on changing behavior rather than producing insight.

References: Gladding (Counseling: A Comprehensive Profession) — Strategic family therapy: Haley, directives and ordeals

☐ Approve ☐ Revise ☐ Retire Notes:

A brief therapy team trained at the Mental Research Institute (MRI) in Palo Alto notices that a mother's repeated lectures to her withdrawn son lead him to withdraw further, which prompts more lectures. In the MRI model, what is the PRIMARY target of intervention?

A. The son's unresolved early childhood conflicts

MRI brief therapy deliberately avoids historical, insight-oriented exploration of early conflicts.

B. The family's level of differentiation of self

Differentiation of self is the central construct of Bowen family systems theory, not the MRI model.

C. The attempted solution maintaining the problem ✓ KEY

The MRI team holds that well-meant attempted solutions (here, lecturing) keep the problem going, so interrupting them is the target.

D. The hierarchy and boundaries between subsystems

Restructuring hierarchy and subsystem boundaries is the focus of Minuchin's structural family therapy.

Rationale: In MRI brief therapy (Watzlawick, Weakland, Fisch), problems persist because of the very solutions people repeatedly apply to them. "More of the same" produces only first-order change within the system's existing rules; the therapist aims for second-order change by blocking or reversing the attempted solution, often through reframing or paradoxical directives.

References: Gladding (Counseling: A Comprehensive Profession) — Strategic family therapy: MRI brief therapy, attempted solutions, first- and second-order change

☐ Approve ☐ Revise ☐ Retire Notes:

During a family session about a teenager's failing grades, her father answers every emotional comment with charts, rules and calm, logical analysis, showing little feeling for himself or for others. Which Satir communication stance is he displaying?

A. Blaming

The blamer finds fault and attacks others to protect the self, disregarding others' feelings rather than all feelings.

B. Placating

The placater pleases and appeases others at the expense of the self; the father is not deferring to anyone.

C. Irrelevant

The irrelevant (distracting) stance changes the subject or clowns around, ignoring self, others and context.

D. Super-reasonable ✓ KEY

The super-reasonable (computer) stance relies on cool logic and data while discounting the feelings of self and others.

Rationale: Virginia Satir described four incongruent survival stances people adopt under stress: placating, blaming, super-reasonable and irrelevant. Her goal was congruent communication, in which words, feelings and body language match and self, other and context are all honored.

References: Gladding (Counseling: A Comprehensive Profession) — Experiential family therapy: Satir communication stances

☐ Approve ☐ Revise ☐ Retire Notes:

A family therapist insists early on that he decides who attends and how sessions are run, but then leaves it to the family to take responsibility for what will change. He relies on spontaneity, playfulness and co-therapy, and calls these struggles the "battle for structure" and the "battle for initiative." Which approach is this?

A. Symbolic-experiential (Whitaker) ✓ KEY

Carl Whitaker described the battle for structure and the battle for initiative and emphasized the therapist's spontaneous, often absurd use of self.

B. Structural therapy (Minuchin)

Minuchin focused on joining, mapping and restructuring subsystems and boundaries rather than these two "battles."

C. Bowen systems therapy (Bowen)

Bowen's approach is calm, coaching and theory-driven, aimed at differentiation, not playful or provocative.

D. Validation model (Satir)

Satir was experiential too, but her model centers on self-esteem, communication stances and congruence.

Rationale: Carl Whitaker's symbolic-experiential family therapy values growth and creativity over technique. The therapist wins the battle for structure (control over the conditions of therapy) but must lose the battle for initiative, so that the family owns its decisions about change. Whitaker often worked with a co-therapist and used humor and "craziness" to unsettle rigid patterns.

References: Gladding (Counseling: A Comprehensive Profession) — Experiential family therapy: Whitaker, symbolic-experiential approach

☐ Approve ☐ Revise ☐ Retire Notes:

A man who prides himself on generosity becomes intensely angry at a coworker he calls "greedy and selfish," although his friends say he can be quite stingy himself. A Jungian counselor would MOST likely view his reaction as a projection of which archetype?

A. The persona

The persona is the public mask a person presents; his generous image is persona, but what he projects is the disowned side.

B. The shadow ✓ KEY

The shadow holds the traits a person rejects in himself; they are often projected onto others and met with strong emotion.

C. The anima

The anima is the feminine side of a man's psyche, not the store of disowned negative traits.

D. The Self

The Self is the archetype of wholeness and integration toward which individuation moves.

Rationale: Carl Jung proposed a collective unconscious containing archetypes shared across humanity, including the persona, shadow, anima/animus and Self. Disowned qualities in the shadow are commonly projected onto others, and a strong emotional charge toward someone is a clue. Jungian work aims at individuation, partly by recognizing and integrating the shadow.

References: Corey (Theory & Practice) — Psychoanalytic therapy: Jung's analytical psychology, archetypes and the shadow

☐ Approve ☐ Revise ☐ Retire Notes:

A client raised by emotionally unavailable parents eagerly reports each accomplishment to the counselor, seems to need admiring responses, and feels deflated and empty when the counselor responds neutrally. In Kohut's self psychology, the client is MOST likely seeking which kind of response?

A. An idealizing selfobject response

Idealizing needs involve merging with a calm, admired figure; the client wants to be admired, not to admire.

B. A twinship selfobject response

Twinship (alter ego) needs involve feeling essentially alike with another person, not being admired.

C. A mirroring selfobject response ✓ KEY

Mirroring needs are met by others who reflect back and admire the person's worth and achievements.

D. Splitting of good and bad objects

Splitting is an object relations concept (Klein, Kernberg) about keeping all-good and all-bad images apart, not a need for admiration.

Rationale: Heinz Kohut argued that a cohesive self develops when caregivers serve as selfobjects meeting three needs: mirroring (being admired), idealizing (merging with a strong, calm figure) and twinship (feeling alike). Unmet mirroring needs can leave adults hungry for admiration and fragile in self-esteem. Kohut's self psychology grew out of, and is often grouped with, object relations theory.

References: Corey (Theory & Practice) — Psychoanalytic therapy: contemporary trends, self psychology and object relations

☐ Approve ☐ Revise ☐ Retire Notes:

A counseling student is comparing Freud's psychosexual theory with Erikson's psychosocial theory. Which statement MOST accurately describes a key difference between them?

A. Erikson's stages extend across the entire life span ✓ KEY

Erikson's eight stages run from infancy through late adulthood, whereas Freud's stages end with the genital stage at puberty.

B. Erikson made sexual drives the core of every stage

Erikson shifted the emphasis from sexual drives to social and cultural tasks and the growth of identity.

C. Freud proposed eight stages ending in late adulthood

Freud described five psychosexual stages; the eight life-span stages belong to Erikson.

D. Freud stressed social crises more than instinctual drives

This reverses the theories; Freud stressed instinctual drives, and Erikson stressed psychosocial crises.

Rationale: Freud's five psychosexual stages (oral, anal, phallic, latency, genital) center on instinctual drives and end at adolescence. Erikson kept a stage framework but emphasized ego development, social relationships and culture, and he extended development across the whole life span with eight psychosocial crises from trust vs. mistrust to integrity vs. despair.

References: Berk (Development Through the Lifespan) — Psychoanalytic perspective: Freud's psychosexual and Erikson's psychosocial theories

☐ Approve ☐ Revise ☐ Retire Notes:

Q718 of 1000 · nce-s-cou-098 · Counseling and Helping Relationships · psychodynamic and psychoanalytic theory · easy · In review

Describing a mistake she made at work, a client says aloud to herself, in a scolding tone that echoes her mother, "You should have known better. How careless can you be?" In Eric Berne's transactional analysis, which ego state is she speaking from?

A. Adult

The Adult ego state processes information objectively in the here and now; it does not scold.

B. Critical Parent ✓ KEY

The Critical (Controlling) Parent holds the rules, judgments and criticisms absorbed from caregivers.

C. Adapted Child

The Adapted Child complies with or rebels against parental messages; it receives the scolding rather than delivering it.

D. Free Child

The Free (Natural) Child is spontaneous, playful and expressive, not judgmental.

Rationale: Berne's transactional analysis describes three ego states: Parent (Critical and Nurturing), Adult and Child (Free and Adapted). Self-critical messages copied from caregivers come from the Critical Parent. TA also analyzes transactions between ego states, psychological games with hidden payoffs, and life scripts formed in childhood.

References: Corey (Theory & Practice) — Transactional analysis: ego states, transactions, games and life scripts

☐ Approve ☐ Revise ☐ Retire Notes:

Arnold Lazarus's multimodal therapy assesses clients across seven modalities summarized by the acronym BASIC I.D. What do the final letters "I.D." represent?

A. Insight and defense mechanisms

Insight and defenses are psychodynamic concepts and are not BASIC I.D. modalities.

B. Identity and dream content

Neither identity nor dreams is one of Lazarus's seven modalities.

C. Interpersonal and drugs/biology ✓ KEY

The last two modalities are Interpersonal relationships and Drugs/biological functions.

D. Imagery and developmental history

Imagery is the earlier "I" in BASIC; developmental history is not a modality.

Rationale: Lazarus's technically eclectic multimodal therapy assesses Behavior, Affect, Sensation, Imagery, Cognition, Interpersonal relationships and Drugs/biology. Counselors then choose techniques from any orientation to address problems in each modality, guided by research on what works rather than by a single theory.

References: Corey (Theory & Practice) — Behavior therapy: Lazarus's multimodal therapy and the BASIC I.D.

☐ Approve ☐ Revise ☐ Retire Notes:

A counselor prepares a newly hired paramedic for high-stress calls in three phases: first building an understanding of how stress works, then learning and rehearsing coping skills such as coping self-statements and relaxation, and finally applying them under graded, increasingly realistic stressors. Which approach is the counselor using?

A. Systematic desensitization

Wolpe's systematic desensitization pairs relaxation with an imagined fear hierarchy; it lacks the conceptualization and coping-skills phases.

B. Rational emotive imagery

Rational emotive imagery is an REBT technique for changing emotions by imagining a feared event while feeling a healthier emotion.

C. Cognitive processing therapy

Cognitive processing therapy treats PTSD by challenging stuck points about a past trauma, not by inoculating against future stress.

D. Stress inoculation training ✓ KEY

Meichenbaum's stress inoculation training moves through conceptualization, skills acquisition and rehearsal, and application and follow-through.

Rationale: Donald Meichenbaum's stress inoculation training "inoculates" clients against stress by exposing them to manageable doses after they have learned coping skills. Its three phases are conceptualization, skills acquisition and rehearsal, and application and follow-through. It builds on his self-instructional training, which teaches clients to replace self-defeating inner speech with coping self-statements.

References: Corey (Theory & Practice) — Cognitive behavior therapy: Meichenbaum's cognitive behavior modification and stress inoculation training

☐ Approve ☐ Revise ☐ Retire Notes:

A 46-year-old client returning to the workforce doubts she can learn the spreadsheet software her new job requires. According to Bandura, which source of self-efficacy is generally the MOST powerful way to strengthen her belief that she can succeed?

A. Watching a peer of similar age master the software

Vicarious experience (modeling) raises self-efficacy, especially with similar models, but less strongly than one's own success.

B. Encouragement from her counselor that she can do it

Verbal persuasion helps but is weaker and easily undone by later failure.

C. Succeeding herself at small, graded spreadsheet tasks ✓ **KEY**

Mastery (performance) experiences are the most powerful source of self-efficacy because they provide direct evidence of capability.

D. Feeling physically calmer when she sits down to work on it

Physiological and emotional states shape self-efficacy judgments, but they are the least direct of the four sources.

Rationale: Albert Bandura identified four sources of self-efficacy: mastery experiences, vicarious experiences (modeling), verbal persuasion, and physiological and emotional states. Mastery experiences are the strongest. Counselors therefore break goals into graded steps the client can succeed at, often after modeling and with encouragement.

References: Corey (Theory & Practice) — Behavior therapy: Bandura's social learning theory, modeling and self-efficacy

☐ Approve ☐ Revise ☐ Retire Notes:

A slot machine pays out after an unpredictable number of plays, averaging about one win every 20 pulls. Which reinforcement schedule is this, and how does it typically affect extinction?

A. Fixed interval; quick to extinguish once payouts stop

A fixed-interval schedule reinforces the first response after a set time; payouts here depend on the number of plays.

B. Variable ratio; highly resistant to extinction ✓ **KEY**

Reinforcement after an unpredictable number of responses is a variable-ratio schedule, which produces high, steady responding that is very hard to extinguish.

C. Fixed ratio; very resistant because it is predictable

A fixed ratio pays after a set number of responses; its predictability makes it less resistant to extinction than a variable ratio.

D. Variable interval; extinguishes faster than continuous

Payouts here depend on responses, not time, and intermittent schedules resist extinction more than continuous reinforcement.

Rationale: Skinner's intermittent schedules are fixed ratio, variable ratio, fixed interval and variable interval. Variable-ratio reinforcement produces the highest, steadiest response rates and the greatest resistance to extinction, which is why gambling is so persistent. Continuous reinforcement speeds learning but extinguishes fastest.

References: Corey (Theory & Practice) — Behavior therapy: operant conditioning and schedules of reinforcement

☐ Approve ☐ Revise ☐ Retire Notes:

After being bitten by a neighbor's German shepherd, a 7-year-old now becomes frightened around other large dogs, yet he stays relaxed around small dogs and cats. In classical conditioning terms, his fear of other large dogs and his calm around small dogs reflect, respectively, which two processes?

A. Spontaneous recovery; stimulus generalization

Spontaneous recovery is the return of an extinguished response after a rest period; nothing here was extinguished.

B. Stimulus discrimination; second-order conditioning

This reverses the first process, and second-order conditioning pairs a new neutral stimulus with an existing conditioned stimulus.

C. Stimulus generalization; stimulus discrimination ✓ KEY

Fear spreading to similar stimuli is generalization; responding differently to dissimilar stimuli is discrimination.

D. Higher-order conditioning; response extinction

His calm around small dogs is not extinction, since that fear was never conditioned and then weakened.

Rationale: In Pavlov's model the bite is the unconditioned stimulus, fear the unconditioned response, and the German shepherd becomes a conditioned stimulus. Stimulus generalization occurs when stimuli similar to the conditioned stimulus also elicit the response; stimulus discrimination occurs when the person responds only to stimuli close enough to the original.

References: Corey (Theory & Practice) — Behavior therapy: classical conditioning, generalization and discrimination

☐ Approve ☐ Revise ☐ Retire Notes:

To teach a 4-year-old with a language delay to say "water," a therapist first reinforces any vocal sound, then only "wa," then "wa-ter," and finally only the complete word. Which behavioral technique is the therapist using?

A. Chaining

Chaining links a series of already-learned behaviors into a sequence, such as the steps of handwashing.

B. Token economy

A token economy delivers conditioned reinforcers (tokens) that are later exchanged for backup rewards; it is not defined by successive approximations.

C. Premack principle

The Premack principle uses a high-frequency activity to reinforce a lower-frequency one.

D. Shaping ✓ KEY

Shaping reinforces successive approximations of a target behavior, raising the criterion step by step.

Rationale: Shaping builds a new behavior by reinforcing closer and closer approximations to the goal while no longer reinforcing earlier, cruder forms. It is often combined with other operant tools, such as token economies, in which tokens act as generalized conditioned reinforcers exchanged later for backup reinforcers.

References: Corey (Theory & Practice) — Behavior therapy: operant techniques, shaping and token economies

☐ Approve ☐ Revise ☐ Retire Notes:

A client keeps telling herself, "I'm a failure." Rather than disputing the thought, her counselor asks her to restate it as "I'm having the thought that I'm a failure" and to notice the words without arguing with them. Which acceptance and commitment therapy (ACT) process is the counselor targeting?

A. Cognitive restructuring

Cognitive restructuring is a traditional CBT strategy that tests and replaces the thought; ACT changes the client's relationship to the thought instead.

B. Cognitive defusion ✓ KEY

Defusion helps clients see thoughts as passing words and images rather than literal truths, loosening their grip on behavior.

C. Committed action

Committed action is taking concrete steps in line with chosen values, not changing how one relates to a thought.

D. Values clarification

Values work identifies what matters most to the client; this exercise targets her relationship to a thought.

Rationale: Steven Hayes's ACT targets psychological flexibility through six hexaflex processes: acceptance, cognitive defusion, contact with the present moment, self-as-context, values and committed action. Defusion techniques, such as adding "I'm having the thought that," create distance from thoughts without trying to change their content.

References: Corey (Theory & Practice) — Third-wave behavior therapies: acceptance and commitment therapy, cognitive defusion

☐ Approve ☐ Revise ☐ Retire Notes:

Mindfulness-based cognitive therapy (MBCT), developed by Segal, Williams and Teasdale, was designed primarily for which purpose?

A. Preventing relapse in people with recurrent depression ✓ KEY

MBCT was built to reduce relapse in people who have had repeated major depressive episodes and are currently in remission.

B. Treating acute mania during inpatient hospitalization

MBCT is a group program for people who are currently well, not an acute treatment for mania.

C. Reducing self-harm in clients with borderline traits

This describes dialectical behavior therapy, which also uses mindfulness but was developed by Linehan.

D. Replacing exposure as the core treatment for specific phobias

Exposure remains the core treatment for specific phobias; MBCT was not developed for them.

Rationale: MBCT combines mindfulness practice, adapted from Kabat-Zinn's MBSR, with cognitive therapy in an 8-week group format. It teaches people with recurrent depression to notice early negative thinking and relate to it from a decentered stance, interrupting the rumination that can trigger relapse. Its strongest evidence is for people with three or more prior episodes.

References: NICE guidelines — Depression in adults (NG222): relapse prevention with mindfulness-based cognitive therapy

☐ Approve ☐ Revise ☐ Retire Notes:

In William Glasser's choice theory, all behavior is an attempt to satisfy five basic needs. Which list names them?

A. Safety, esteem, belonging, purpose, and self-actualization

This resembles Maslow's hierarchy of needs, not Glasser's five basic needs.

B. Survival, attachment, mastery, autonomy, and meaning

This mixes concepts from attachment and self-determination theories; Glasser's list includes power, freedom and fun.

C. Love, work, play, spirituality, and self-direction

These echo Adlerian life tasks and wellness models rather than choice theory.

D. Survival, love and belonging, power, freedom, and fun ✓ **KEY**

Glasser named survival, love and belonging, power (achievement), freedom and fun as the five basic needs.

Rationale: Choice theory holds that people choose behaviors to meet five genetically based needs: survival, love and belonging, power or achievement, freedom, and fun. Glasser saw love and belonging as central. Reality therapy helps clients evaluate whether what they are doing is meeting these needs and plan more effective choices.

References: Corey (Theory & Practice) — Reality therapy: choice theory and the five basic needs

☐ Approve ☐ Revise ☐ Retire Notes:

A counselor uses a time-limited psychodynamic approach (such as those of Mann or Strupp and Binder) with a client who has recurring conflicts with authority figures. How does this approach MOST differ from traditional long-term psychoanalysis?

A. Transference is avoided so the work stays short-term

Brief dynamic therapists actively use transference, especially as it relates to the focal pattern.

B. Free association becomes the counselor's main tool

Unstructured free association is typical of classical analysis; brief therapy is more focused and active.

C. Work centers on one focal conflict or relational pattern ✓ **KEY**

Time-limited dynamic therapies select a single central issue or cyclical maladaptive pattern and keep the work on it.

D. The ending date is left open until the client feels ready

A set time limit, often agreed at the start (Mann used 12 sessions), is a defining feature, and the ending itself is used therapeutically.

Rationale: Brief psychodynamic therapies keep psychoanalytic ideas such as transference and unconscious conflict but limit the number of sessions, have the counselor take a more active role, and select a circumscribed focus, such as Mann's central issue or Strupp and Binder's cyclical maladaptive pattern. The fixed ending is used to work through issues of separation and loss.

References: Corey (Theory & Practice) — Psychoanalytic therapy: brief psychodynamic therapy and time-limited dynamic psychotherapy

☐ Approve ☐ Revise ☐ Retire Notes:

A reluctant 16-year-old says only that last night "was bad" because of an argument with his mother. The counselor wants to invite him to tell the story in his own words. Which counselor question BEST serves that goal?

A. Were you upset by the argument?

A closed question invites a yes or no and limits how much he says.

B. Why did you yell at her?

"Why" questions can sound accusatory and prompt defensiveness; it also assumes he yelled.

C. Was it about curfew or grades?

An either/or question is closed and imposes the counselor's guesses about the topic.

D. What happened during the argument? ✓ **KEY**

An open "what" question invites him to describe events in his own words and at his own depth.

Rationale: Open questions, often starting with "what" or "how," encourage clients to elaborate and lead the content. Closed questions are useful for specific facts but narrow the conversation, and "why" questions often provoke defensiveness. Once a client is talking, minimal encouragers such as "mm-hm" help keep the story going.

References: Gladding (Counseling: A Comprehensive Profession) — Counseling skills: open and closed questions

☐ Approve ☐ Revise ☐ Retire Notes:

A grieving client finishes describing her mother's final days in hospice, then stops speaking and looks down with tears in her eyes. What is the MOST helpful counselor response in that moment?

A. Allow a silence so she can stay with what she feels ✓ **KEY**

A respectful, attentive silence gives her room to feel and reflect and signals that the counselor can stay present with her pain.

B. Ask a new question so the pause does not grow awkward

Filling the pause usually serves the counselor's discomfort and pulls her away from her feelings.

C. Offer reassurance that her mother is no longer suffering

Premature reassurance can minimize her grief and close down the emotion she is experiencing.

D. Summarize the session so far and move to coping planning

Shifting to planning at a moment of deep feeling is mistimed and can feel dismissive.

Rationale: Purposeful silence is an attending skill. It allows clients to experience emotion, gather thoughts and decide what to say next, and it communicates acceptance. Beginning counselors often rush to fill silences; the skill lies in reading whether a silence is productive (reflective) or a sign the client is stuck or confused.

References: Gladding (Counseling: A Comprehensive Profession) — Counseling skills: use of silence and attending

☐ Approve ☐ Revise ☐ Retire Notes:

A client in early recovery asks his counselor, "Have you ever struggled with addiction yourself?" The counselor has been in recovery for many years. Which guideline BEST governs whether and how she self-discloses?

A. Share her own recovery story in full detail, since shared experience builds credibility

Lengthy disclosure shifts the focus to the counselor and can burden the client, whatever its intent.

B. Disclose only if it serves the client, keep it brief, and return the focus to him ✓ **KEY**

Helpful self-disclosure is purposeful, brief, relevant to the client's goals, and followed by a quick return of the focus to the client.

C. Decline every personal question, because any disclosure is a boundary violation

Self-disclosure is a recognized skill; used sparingly and for the client's benefit, it is not a boundary violation.

D. Disclose only after he asks a second time, to confirm that his interest is sincere

Making him ask twice is not a recognized guideline; the test is whether disclosure serves his welfare.

Rationale: Counselor self-disclosure should be infrequent, brief, relevant and offered for the client's benefit rather than the counselor's needs. After disclosing, the counselor returns the focus to the client, for example by asking how the disclosure lands. Counselors should consider the client's culture, timing and whether the disclosure could blur roles.

References: Gladding (Counseling: A Comprehensive Profession) — Counseling skills: counselor self-disclosure

☐ Approve ☐ Revise ☐ Retire Notes:

Q732 of 1000 · nce-s-cou-112 · Counseling and Helping Relationships · family systems theories (Bowen, structural, strategic) · hard · In review

A father calls his 15-year-old "stubborn," and the counselor replies, "He sounds determined to stand by what he believes." Later, the counselor suggests the son's school refusal may be his way of keeping an eye on a parent he worries about. Which statement BEST distinguishes the two techniques?

A. The first is reframing, which recasts a whole situation; the second is relabeling a single trait

This reverses the terms; swapping one adjective is relabeling, and recasting the meaning of a situation is reframing.

B. The first is interpretation of a defense; the second is a paradoxical prescription

Neither statement interprets a defense or tells anyone to continue the symptom.

C. Both are confrontations of discrepancies between the family's words and actions

Confrontation points out a discrepancy; these statements offer new, more positive meanings instead.

D. The first relabels a negative trait; the second reframes what the behavior means ✓ **KEY**

Relabeling replaces a negative descriptor with a positive one, while reframing places a behavior in a new context that changes its meaning.

Rationale: Both techniques change perception without changing facts. Relabeling attaches a more positive label to a person or trait ("stubborn" becomes "determined"). Reframing offers a different frame for a whole pattern, often giving a symptom a protective or caring function, which can open new options for the family. Reframing is especially associated with strategic and MRI approaches.

References: Gladding (Counseling: A Comprehensive Profession) — Family counseling techniques: reframing and relabeling

► **Check:** Verify the reframing vs. relabeling distinction against the Gladding glossary, since some texts treat relabeling as a subtype of reframing.

☐ Approve ☐ Revise ☐ Retire Notes:

Pamela Hays's ADDRESSING framework helps counselors consider the many cultural influences shaping a client's identity. In this acronym, what does the first "D" stand for?

A. Developmental disabilities ✓ **KEY**

The first D is developmental disabilities; the second D is disabilities acquired later in life.

B. Dominant-group status

Privilege runs through the whole framework, but dominant-group status is not a letter in the acronym.

C. Diagnostic history

Diagnostic history is a clinical, not cultural, factor and is not part of ADDRESSING.

D. Dialect and language

Language is considered under national origin and ethnicity; it is not one of the letters.

Rationale: ADDRESSING stands for Age and generational influences, Developmental disabilities, Disabilities acquired later in life, Religion and spiritual orientation, Ethnic and racial identity, Socioeconomic status, Sexual orientation, Indigenous heritage, National origin and Gender. Hays designed it to help counselors see clients' intersecting identities and to examine where the counselor holds privilege.

References: Sue & Sue (Counseling the Culturally Diverse) — Multidimensional identity: Hays's ADDRESSING framework

► **Check:** Confirm that the cited Sue & Sue edition discusses Hays's ADDRESSING framework; if not, cite a text that does.

☐ Approve ☐ Revise ☐ Retire Notes:

After years of being passed over at work despite strong reviews, a client says he has little control over his career because of systemic discrimination, and he places responsibility for his situation on that system rather than on himself. In Derald Wing Sue's worldview model, which quadrant BEST fits?

A. Internal control, internal responsibility

IC-IR reflects the mainstream U.S. belief that people control their outcomes and are to blame for failures.

B. External control, external responsibility ✓ **KEY**

EC-ER describes someone who sees outcomes as controlled by others and attributes problems to systemic forces such as discrimination.

C. External control, internal responsibility

EC-IR describes someone who has little control yet blames himself, often linked to internalized oppression.

D. Internal control, external responsibility

IC-ER describes someone who feels able to shape his life while recognizing systemic barriers, not one who feels little control.

Rationale: Sue crossed locus of control (internal or external) with locus of responsibility (internal or person-centered vs. external or system-centered) to form four worldviews. Counselors who assume an IC-IR view may misread an EC-ER client as resistant or blaming. Understanding the client's worldview, and the real barriers behind it, guides culturally responsive goals such as advocacy.

References: Sue & Sue (Counseling the Culturally Diverse) — Worldview: locus of control and locus of responsibility quadrants

☐ Approve ☐ Revise ☐ Retire Notes:

At intake, a counselor briefly asks a Latina client whether she minds working with a counselor of a different ethnicity. He never returns to culture, even when she later describes ethnic bias from her supervisor. In Day-Vines and colleagues' broaching continuum, which style does this MOST reflect?

A. Avoidant

An avoidant counselor does not raise race or culture at all; this counselor did raise it once.

B. Infusing

An infusing counselor builds attention to culture and social justice into all of their work, well beyond the session.

C. Isolating ✓ KEY

An isolating counselor broaches culture in a superficial, one-time way to satisfy an obligation, then drops it.

D. Integrated/congruent

An integrated/congruent counselor broaches routinely and links the client's cultural experiences to the presenting concerns.

Rationale: Broaching is the counselor's consistent, open invitation to explore how race, ethnicity and culture bear on the client's concerns. Day-Vines and colleagues described a continuum of broaching styles: avoidant, isolating, continuing/incongruent, integrated/congruent and infusing. Raising culture once and never connecting it to the client's experience is the isolating style.

References: Sue & Sue (Counseling the Culturally Diverse) — Broaching race and culture in counseling: Day-Vines broaching continuum

► **Check:** Verify the broaching style names and order against Day-Vines et al. (2007) and confirm Sue & Sue covers them.

☐ Approve ☐ Revise ☐ Retire Notes:

A counselor deployed to a family assistance center after a mass shooting will use Psychological First Aid (PFA) as developed by the National Child Traumatic Stress Network and National Center for PTSD. Which practice is NOT one of the PFA core actions?

A. Linking survivors with needed collaborative services

Linkage with collaborative services is one of the eight PFA core actions.

B. Gathering information on current needs and worries

Information gathering about current needs and concerns is a PFA core action.

C. Offering information on coping with stress reactions

Providing information on coping is a PFA core action.

D. Asking survivors to recount the event in detail ✓ KEY

PFA does not require survivors to describe the event in detail; pressing for a detailed account can be distressing and resembles debriefing, which PFA avoids.

Rationale: PFA's eight core actions are contact and engagement, safety and comfort, stabilization, information gathering on current needs and concerns, practical assistance, connection with social supports, information on coping, and linkage with collaborative services. It is evidence-informed, flexible and non-intrusive, and it does not push survivors to retell or process their trauma.

References: Psychological First Aid Field Operations Guide (NCTSN/NCPTSD) — The eight PFA core actions

☐ Approve ☐ Revise ☐ Retire Notes:

A counselor using trauma-focused cognitive behavioral therapy (TF-CBT) with a 9-year-old has completed psychoeducation and parenting skills, relaxation, affective modulation and cognitive coping. Following the PRACTICE sequence, which component comes NEXT?

A. In vivo mastery of trauma reminders

In vivo mastery follows the trauma narrative and is used only when the child avoids safe reminders.

B. Conjoint child-parent sessions

Conjoint sessions, often used to share the narrative with the caregiver, come after the narrative is developed.

C. Enhancing future safety

Enhancing safety is the final component of the PRACTICE sequence.

D. Trauma narrative and processing ✓ KEY

After the stabilization skills, the child gradually develops and processes a narrative of the trauma.

Rationale: TF-CBT, developed by Cohen, Mannarino and Deblinger, follows the PRACTICE components: Psychoeducation and Parenting skills, Relaxation, Affective modulation, Cognitive coping, Trauma narrative and processing, In vivo mastery, Conjoint child-parent sessions and Enhancing safety. Coping skills are built first so the child can tolerate the gradual exposure of the trauma narrative.

References: NICE guidelines — PTSD (NG116): trauma-focused CBT for children and young people

► **Check:** The PRACTICE order is from Cohen, Mannarino and Deblinger; confirm NICE NG116 is an acceptable citation or swap to a source that describes the model.

☐ Approve ☐ Revise ☐ Retire Notes:

After a year at a child advocacy center, a counselor notices lasting changes in her own beliefs: she now sees the world as unsafe, distrusts adults around her own children, and has intrusive images drawn from clients' accounts. Which term BEST describes her experience?

A. Burnout

Burnout is exhaustion, cynicism and reduced effectiveness from workload and work conditions; it need not involve trauma-related changes in beliefs.

B. Vicarious trauma ✓ KEY

Vicarious trauma is a cumulative change in the helper's inner world, such as beliefs about safety and trust, from empathic engagement with clients' trauma.

C. Compassion satisfaction

Compassion satisfaction is the positive fulfillment helpers draw from their work, the opposite of her experience.

D. Countertransference

Countertransference is the counselor's reaction to a particular client rooted in her own history, not a cumulative shift in worldview.

Rationale: Vicarious trauma (McCann and Pearlman) is a lasting shift in a helper's beliefs about safety, trust and control that results from repeated exposure to clients' trauma. Compassion fatigue (Figley) emphasizes the emotional cost of caring, including secondary traumatic stress, while burnout stems from work conditions. Remedies include supervision, caseload balance, peer support and personal self-care.

References: Gladding (Counseling: A Comprehensive Profession) — Counselor wellness: burnout, compassion fatigue and vicarious trauma · ACA Code of Ethics — C.2.g Impairment

☐ Approve ☐ Revise ☐ Retire Notes:

While talking about an assault, a client begins to dissociate: she stares blankly, speaks in a monotone, and says the room feels "far away." What should the counselor do FIRST?

A. Guide her to name things she can see and feel her feet on the floor ✓ KEY

Grounding techniques that engage the senses and body bring her back into the present moment and her window of tolerance.

B. Encourage her to keep going so the memory can be fully processed today

Continuing while she is dissociated is unlikely to be therapeutic and can overwhelm her further.

C. Ask her to close her eyes and picture the memory in greater detail

Closing her eyes and deepening the memory increases the risk of further dissociation.

D. Interpret her dissociation as resistance to the work of the session

Dissociation is a protective response to overwhelming arousal, not resistance to be interpreted.

Rationale: When a client dissociates or becomes flooded, trauma-informed counselors first re-establish safety and present-moment awareness through grounding, such as the 5-4-3-2-1 senses exercise, feeling the feet on the floor, holding an object or naming the date and place. Trauma processing resumes only when the client is back within her window of tolerance.

References: Gladding (Counseling: A Comprehensive Profession) — Crisis and trauma counseling: grounding and stabilization

☐ Approve ☐ Revise ☐ Retire Notes:

In eye movement desensitization and reprocessing (EMDR), the client holds the disturbing image, negative belief and body sensations in mind while the counselor leads sets of eye movements, and distress ratings are taken until they fall toward zero. In Shapiro's eight-phase protocol, which phase is this?

A. Preparation

Preparation builds the relationship, explains EMDR and teaches calming skills before memories are processed.

B. Assessment

Assessment identifies the target image, negative and positive cognitions and baseline ratings without bilateral stimulation.

C. Desensitization ✓ KEY

Desensitization uses sets of bilateral stimulation while the client attends to the target, until the SUD rating approaches 0.

D. Reevaluation

Reevaluation happens at the start of the next session to check whether earlier gains have held.

Rationale: Francine Shapiro's EMDR protocol has eight phases: history taking, preparation, assessment, desensitization, installation, body scan, closure and reevaluation. Bilateral stimulation (eye movements, taps or tones) is used during desensitization, installation of the positive cognition and the body scan. EMDR is recommended by several PTSD practice guidelines.

References: APA CPG — PTSD guideline (2017): EMDR as a conditionally recommended psychotherapy

☐ Approve ☐ Revise ☐ Retire Notes:

During a lifestyle assessment, an Adlerian counselor asks a 38-year-old client to describe a single, specific incident from early childhood that she can picture clearly, including what she felt at the time. The counselor is LEAST concerned with whether the memory is factually accurate. What is the main purpose of gathering these early recollections?

A. To uncover repressed trauma so it can be worked through in transference

Recovering repressed material and working it through in the transference is a psychoanalytic goal, not the Adlerian use of early memories.

B. To reveal her private logic and core convictions about self, others and life ✓ KEY

Adlerians treat early recollections as projective: the memories a person keeps reflect present convictions, goals and basic mistakes in the lifestyle.

C. To identify the conditioned stimuli that first triggered her current anxiety

Locating conditioned stimuli is a behavioral assessment task; Adlerians are not seeking a learning history of specific triggers.

D. To confirm the accuracy of her family history before choosing a diagnosis

Adlerians value early recollections for what the client selects and how she tells it, not for historical accuracy or diagnosis.

Rationale: In Adlerian lifestyle assessment, early recollections are single, specific memories, usually from before about age 10. Because people remember events consistent with their current view of life, the memories serve as a projective window into private logic, goals and basic mistakes. Factual accuracy matters little; the themes and the client's stance within the memory matter most.

References: Corey (Theory & Practice) — Adlerian Therapy: lifestyle assessment and early recollections

☐ Approve ☐ Revise ☐ Retire Notes: _____

An Adlerian counselor asks a client who feels stuck in a low mood to close his eyes and recall a pleasant memory, notice the feelings that come with it, then recall a painful memory and notice how his feelings shift, and then return to the pleasant one. The aim is to show him that he can influence his feelings by what he chooses to think about. This is called:

A. Rational emotive imagery

Rational emotive imagery (REBT) has the client imagine a feared event and change the emotion by changing the belief, not alternate pleasant and painful memories.

B. Acting as if

Acting as if invites the client to behave as though he were already the person he wishes to be, not to switch between memories.

C. The push-button technique ✓ KEY

The push-button technique alternates pleasant and unpleasant images so the client sees he can "push the button" for the feeling he wants.

D. Systematic desensitization

Systematic desensitization pairs relaxation with a graded fear hierarchy; it is a behavioral fear-reduction method.

Rationale: The Adlerian push-button technique teaches that feelings are not simply things that happen to us. By deliberately calling up pleasant and then unpleasant memories and noticing the emotional shift, the client learns he can create or change feelings by what he chooses to focus on, which supports responsibility and encouragement.

References: Corey (Theory & Practice) — Adlerian Therapy: reorientation techniques (push-button technique)

☐ Approve ☐ Revise ☐ Retire Notes: _____

A 15-year-old who failed her first algebra test tells her school counselor she is "just bad at math" and wants to drop the class. She studied every evening for the next test and raised her score to a C. Which counselor response BEST reflects the Adlerian use of encouragement?

A. "You're so smart. I was sure you would get an A one of these days."

This is praise that rates the person and sets an outcome standard; Adlerians distinguish such praise from encouragement.

B. "A C is still a C. Let's set a firm goal of a B on the next test."

This focuses on the shortfall and raises the bar, which can deepen discouragement rather than build courage.

C. "Maybe math just isn't your thing. Many people do fine without it."

This colludes with her discouraged belief that she lacks ability, which is the mistaken conviction to be challenged.

D. "You studied each night, and your score rose. Your effort paid off." ✓ KEY

Encouragement recognizes effort and improvement, building the courage and faith in herself that she can grow.

Rationale: Adlerians view many problems as rooted in discouragement, and they see encouragement as the most powerful method for change. Encouragement focuses on effort, improvement and the person's strengths, and it is not contingent on success. Praise, by contrast, rewards outcomes and can teach that worth depends on performing well.

References: Corey (Theory & Practice) — Adlerian Therapy: encouragement as a core process

☐ Approve ☐ Revise ☐ Retire Notes:

Carl Rogers (1957) proposed six conditions that are necessary and sufficient for therapeutic personality change. Besides the counselor's congruence, unconditional positive regard and empathy, which condition describes the state of the client entering the relationship?

A. The client is incongruent, being vulnerable or anxious ✓ KEY

Rogers specified that the client is in a state of incongruence, vulnerable or anxious; this is one of the six conditions.

B. The client has insight into the childhood origins of distress

Insight into childhood origins is a psychodynamic aim and was not among Rogers's conditions.

C. The client agrees to specific, measurable treatment goals

Measurable goal setting is more typical of behavioral and brief approaches; Rogers did not require it.

D. The client has received an accurate diagnosis from the counselor

Rogers considered diagnosis unnecessary for change and potentially harmful to the relationship.

Rationale: Rogers's six conditions are: two persons are in psychological contact; the client is incongruent, vulnerable or anxious; the counselor is congruent in the relationship; the counselor experiences unconditional positive regard; the counselor experiences empathic understanding of the client's internal frame of reference; and the client at least minimally perceives the counselor's regard and empathy. He held that no particular technique, insight or diagnosis was required.

References: Corey (Theory & Practice) — Person-Centered Therapy: necessary and sufficient conditions for change

☐ Approve ☐ Revise ☐ Retire Notes:

A 60-year-old client caring for his wife with dementia asks his counselor, "Just tell me straight: should I move her into a memory care facility or not?" Which counselor response is MOST consistent with a person-centered, nondirective stance?

A. "Most families in your situation find a facility is the better choice."

This gives advice from the counselor's frame of reference and shifts the decision away from the client.

B. "Let's list the pros and cons this week and score each one at home."

A structured decision task is a directive, problem-solving move rather than following the client's experience.

C. "You feel torn, and you wish someone would tell you what is right." ✓ **KEY**

This empathic reflection stays in his frame of reference and trusts his capacity to reach his own decision.

D. "Perhaps your guilt about her illness is what makes deciding so hard."

This is an interpretation that imposes the counselor's hypothesis about the cause of his difficulty.

Rationale: Person-centered counselors trust the client's actualizing tendency and capacity for self-direction. Rather than advising, assigning tasks or interpreting, they offer accurate empathy, genuineness and unconditional positive regard, so the client can explore his own experience and arrive at his own choice.

References: Corey (Theory & Practice) — Person-Centered Therapy: nondirective stance and the client as expert

☐ Approve ☐ Revise ☐ Retire Notes:

A counseling student is summarizing common criticisms of the person-centered approach for a theories exam. Which criticism is MOST often raised?

A. It casts the counselor as an expert who interprets the client's hidden motives

This describes classical psychoanalysis; person-centered counselors avoid the interpretive expert role.

B. Its low structure may not suit clients who want or expect active direction ✓ **KEY**

Critics note that a nondirective stance can leave some clients, including many from cultures that expect an active helper, without enough guidance.

C. It depends heavily on standardized testing and diagnosis to guide treatment

Rogers de-emphasized testing and diagnosis, so this is the opposite of the approach.

D. It treats the therapeutic relationship as secondary to specific techniques

The relationship is the core of person-centered therapy, which places little weight on techniques.

Rationale: Person-centered therapy is praised for its emphasis on the relationship and the client's resources, and its core conditions are well supported by alliance research. Common criticisms are that it can be too passive or unstructured for clients in crisis or those who expect direction, and that some practitioners reduce it to simple reflection without real presence.

References: Corey (Theory & Practice) — Person-Centered Therapy: limitations and criticisms; multicultural perspective

☐ Approve ☐ Revise ☐ Retire Notes:

A 29-year-old client has been offered a job abroad and says he feels anxious about the choice because no one can make it for him and he will have to live with the results. How would an existential counselor MOST likely view this anxiety?

A. As normal anxiety that comes with freedom and can spur growth and choice ✓ **KEY**

Existentialists see normal anxiety as an unavoidable part of facing freedom and responsibility, and as a potential source of growth.

B. As a symptom to be reduced quickly with relaxation and breathing skills

Symptom reduction is a behavioral aim; existential counselors do not try to eliminate normal anxiety.

C. As the result of repressed impulses breaking through his ego defenses

This is a psychoanalytic account of anxiety, not an existential one.

D. As the product of an irrational demand that he dispute in homework

Disputing irrational demands is REBT; existential therapy treats this anxiety as a meaningful response to real givens.

Rationale: Existential therapy distinguishes normal anxiety, a fitting response to freedom, responsibility, death and isolation, from neurotic anxiety, which is out of proportion and blocks living. Rollo May described normal anxiety as something to face, not remove, because it can motivate authentic choice. The counselor helps the client own his freedom rather than escape the discomfort.

References: Corey (Theory & Practice) — Existential Therapy: normal and neurotic anxiety; freedom and responsibility

☐ Approve ☐ Revise ☐ Retire Notes:

Which pairing of existential theorist and contribution is accurate?

A. Viktor Frankl: the four ultimate concerns of death, freedom, isolation and meaninglessness

The four ultimate concerns were described by Irvin Yalom, not Frankl.

B. Irvin Yalom: logotherapy and the will to meaning as the primary human motivation

Logotherapy and the will to meaning are Frankl's contributions.

C. Rollo May: the I-Thou relationship as the basis of genuine human encounter

The I-Thou relationship comes from the philosopher Martin Buber, though existential counselors draw on it.

D. Rollo May: bringing existential ideas to American therapy and writing on anxiety ✓ **KEY**

May was a key figure in introducing existential thought to U.S. psychotherapy and wrote extensively on anxiety, courage and freedom.

Rationale: Viktor Frankl founded logotherapy, emphasizing the will to meaning. Rollo May helped bring European existential thought to American psychotherapy, writing on normal anxiety and the courage to face freedom. Irvin Yalom identified four ultimate concerns: death, freedom, isolation and meaninglessness. Martin Buber, a philosopher, described the I-Thou relationship.

References: Corey (Theory & Practice) — Existential Therapy: key figures (Frankl, May, Yalom) and philosophical roots

☐ Approve ☐ Revise ☐ Retire Notes:

A client dreads sweating heavily when giving presentations, and the more he fears it, the more he sweats. His counselor asks him, before his next talk, to try to sweat as much as he possibly can and to show the audience just how much he can sweat. Which Frankl technique is this?

A. Dereflexion

Dereflexion redirects attention away from oneself and the symptom toward other meanings; it does not ask the client to wish for the symptom.

B. Flooding

Flooding is a behavioral exposure method of prolonged, intense contact with the feared stimulus, not a humorous wish for the symptom.

C. Paradoxical intention ✓ KEY

Frankl's paradoxical intention asks clients to intend the very thing they fear, breaking the cycle of anticipatory anxiety.

D. Ordeal therapy

Ordeal therapy (Haley) attaches a burdensome task to the symptom; it is strategic, not logotherapeutic.

Rationale: In paradoxical intention, Frankl asked clients to wish for or deliberately produce what they fear, often with humor. Doing so interrupts anticipatory anxiety, the fear of the symptom that keeps the symptom going. Frankl paired it with dereflexion, which moves attention away from hyper-reflection on oneself.

References: Corey (Theory & Practice) — Existential Therapy: Frankl's logotherapy, paradoxical intention and dereflexion

☐ Approve ☐ Revise ☐ Retire Notes:

A client speaks of her partner and herself as "we" when describing every feeling and opinion. She says they "never disagree," and she becomes anxious and vague when the counselor asks what she alone wants. In gestalt terms, this blurring of the boundary between self and other is called:

A. Introjection

Introjection is swallowing others' beliefs and standards whole without examining them.

B. Confluence ✓ KEY

Confluence blurs the self-other boundary so that differences are not felt, keeping conflict and separate wants out of awareness.

C. Projection

Projection attributes to others the traits or feelings one disowns in oneself.

D. Deflection

Deflection avoids contact through humor, vagueness or changing the subject.

Rationale: Gestalt therapy names several disturbances at the contact boundary: introjection, projection, retroflexion, deflection and confluence. In confluence the person merges with others, seeks constant agreement and loses touch with personal wants. Counselors help such clients notice differences and ask, "What do you feel? What do you want?"

References: Corey (Theory & Practice) — Gestalt Therapy: contact and resistances to contact (confluence)

☐ Approve ☐ Revise ☐ Retire Notes:

A client gives a long, detached account of an argument he had with his brother two years ago. As he talks, his voice tightens and he taps his foot quickly. Which response is MOST consistent with gestalt therapy?

A. "What are you aware of right now as you tell me this? Notice your foot." ✓ KEY

This brings him into the here and now and directs attention to present, nonverbal experience, the heart of gestalt awareness work.

B. "Why do you think you and your brother end up fighting like that so often?"

Gestalt counselors avoid "why" questions, which invite intellectualizing rather than present awareness.

C. "What were you telling yourself during the argument that upset you?"

Identifying self-talk is a cognitive-behavioral move focused on the past event rather than present experience.

D. "It sounds as if your brother reminds you of a parent from childhood."

This is a psychodynamic interpretation; gestalt counselors favor client discovery over counselor interpretation.

Rationale: Gestalt therapy holds that awareness in the present moment is itself curative. Counselors shift clients from talking about past events to experiencing what is happening now, often by noticing body language, voice and breath. "What" and "how" questions are preferred to "why" questions, which tend to produce explanations instead of contact.

References: Corey (Theory & Practice) — Gestalt Therapy: the now, awareness, and attention to nonverbal behavior

☐ Approve ☐ Revise ☐ Retire Notes:

Arnold Beisser's paradoxical theory of change is a foundation of gestalt practice. Which statement BEST captures it?

A. Change happens when the counselor prescribes the symptom the client most wants to stop

Prescribing the symptom is a strategic family therapy directive, not Beisser's theory.

B. Change happens when clients replace irrational demands with flexible preferences

Replacing demands with preferences is the REBT model of change.

C. Change happens when the counselor interprets resistance until the client gains insight

Interpreting resistance to produce insight is a psychoanalytic method.

D. Change happens when one becomes what one is, not when one tries to be what one is not ✓ KEY

Beisser held that change comes through full awareness and acceptance of who one is now, not through striving to be different.

Rationale: Beisser's paradoxical theory of change states that the more a person tries to be who he or she is not, the more the person stays the same. Change occurs when the person fully experiences and owns current reality. Gestalt counselors therefore do not push for change; they heighten awareness of what is, trusting that integration follows.

References: Corey (Theory & Practice) — Gestalt Therapy: the paradoxical theory of change

☐ Approve ☐ Revise ☐ Retire Notes:

A client who believes it would be unbearable to look foolish is assigned homework: wear a brightly mismatched outfit to the grocery store and loudly ask a clerk where the milk is while standing next to it. She is to notice that she can tolerate others' reactions and still accept herself. This REBT technique is called:

A. Paradoxical intention

Paradoxical intention (Frankl) asks clients to wish for a feared symptom, such as trying to blush or sweat.

B. A shame-attacking exercise ✓ KEY

Shame-attacking exercises have clients do harmless "foolish" acts in public to learn that disapproval is tolerable and not a verdict on their worth.

C. Rational emotive imagery

Rational emotive imagery is done in imagination, not by acting in public.

D. Systematic desensitization

Systematic desensitization pairs relaxation with a graded hierarchy, usually in imagination, to reduce fear.

Rationale: Albert Ellis developed shame-attacking exercises as in vivo homework. Clients deliberately do something they consider shameful but that harms no one, then work on not putting themselves down. REBT relies heavily on such homework to practice new rational beliefs between sessions.

References: Corey (Theory & Practice) — REBT: behavioral techniques, homework and shame-attacking exercises

☐ Approve ☐ Revise ☐ Retire Notes:

After failing the bar exam, a 27-year-old law graduate says, "I failed, so I'm a failure." Her REBT counselor works toward unconditional self-acceptance (USA). Which goal BEST reflects this concept?

A. Restore her self-esteem by listing past successes that show her real worth

Ellis criticized self-esteem as conditional on achievement; rebuilding it through successes keeps worth tied to performance.

B. Help her see the exam was unimportant so the result need not upset her

REBT does not deny that the outcome is bad; appropriate disappointment is a healthy negative emotion.

C. Rate the failed exam as a poor result while accepting herself as fallible ✓ KEY

USA means rating behaviors and outcomes, not the whole self, and accepting oneself as a fallible human regardless of performance.

D. Trace how her parents' high standards first taught her to fear failure

REBT focuses on present beliefs; tracing childhood origins is not needed for unconditional self-acceptance.

Rationale: Ellis argued that global self-rating is irrational because a person is too complex to sum up with one score. Unconditional self-acceptance asks clients to evaluate their actions and outcomes, sometimes harshly, while accepting themselves without conditions. He contrasted this with self-esteem, which rises and falls with success and approval.

References: Corey (Theory & Practice) — REBT: unconditional self-acceptance versus self-esteem

☐ Approve ☐ Revise ☐ Retire Notes:

A counselor asks a client to imagine vividly that he is turned down for a date, let himself feel the depressed panic that follows, and then, while keeping the image, change the feeling to disappointment. Afterward they discuss what he told himself to make the change. This REBT technique is:

A. Rational emotive imagery ✓ KEY

Rational emotive imagery has clients hold a vivid image of a feared event and shift an unhealthy emotion to a healthy one by changing the belief.

B. The push-button technique

The Adlerian push-button technique alternates pleasant and unpleasant memories; it does not keep one feared image constant.

C. Covert sensitization

Covert sensitization pairs an unwanted behavior with imagined aversive consequences to reduce that behavior.

D. Systematic desensitization

Systematic desensitization pairs relaxation with graded fear images; it does not ask the client to feel and then transform an emotion.

Rationale: In rational emotive imagery, the client imagines one of the worst things that could happen, allows the unhealthy emotion, and then changes it to a healthy negative emotion such as disappointment or concern. The change happens through a shift in belief, which the client then identifies and practices, often daily as homework.

References: Corey (Theory & Practice) — REBT: emotive techniques (rational emotive imagery)

☐ Approve ☐ Revise ☐ Retire Notes:

A client says she has felt sad "every single day" since moving to a new city. Her counselor asks, "Think about this past week. When was the sadness even a little lighter, and what was different then?" This is an example of:

A. A scaling question

Scaling questions ask the client to rate the problem or progress on a numerical scale, such as 0 to 10.

B. The miracle question

The miracle question asks the client to imagine the problem solved overnight and describe what would be different.

C. A coping question

Coping questions ask how the client has kept going despite difficulty, not when the problem was less present.

D. An exception question ✓ KEY

Exception questions search for times the problem was absent or less severe, so the client can repeat what worked.

Rationale: Solution-focused counselors assume that problems are never constant. Exception questions direct attention to times the problem was absent or milder and to what the client was doing then. These exceptions become the raw material for solutions: do more of what works.

References: Corey (Theory & Practice) — Solution-Focused Brief Therapy: exception questions

☐ Approve ☐ Revise ☐ Retire Notes:

A father raising three children alone after his wife's death says nothing is going well, and he cannot recall a single time things were better. Which response is MOST consistent with a solution-focused coping question?

A. "Let's find out what in your past is keeping you so stuck right now."

Searching the past for the cause of the problem is problem-focused, which SFBT deliberately avoids.

B. "With so much on you, how have you managed to keep the kids fed and here?" ✓ KEY

A coping question highlights the strength and effort already present when the client sees no exceptions.

C. "On a scale of 0 to 10, how bad would you say things are for you today?"

This is a scaling question, and framing it around how bad things are keeps the focus on the problem.

D. "It sounds like the grief is still very raw. Tell me more about the loss."

This empathic invitation is appropriate in many models but is not an SFBT coping question.

Rationale: When clients cannot identify exceptions, solution-focused counselors use coping questions, such as "How have you managed to keep going?" These questions bring out the client's resources and small successes already present in a hard situation. They respect the difficulty while shifting attention toward competence.

References: Corey (Theory & Practice) — Solution-Focused Brief Therapy: coping questions and client competence

☐ Approve ☐ Revise ☐ Retire Notes:

A 16-year-old is sent to counseling by his school after a fight. He says he has no problem and is only there because the principal required it. In de Shazer's terms, what is the counselor-client relationship, and what is the BEST response?

A. Complainant; ask him to watch for times when things go a bit better

A complainant sees a problem but not himself as part of the solution; this teen does not yet see a problem at all.

B. Customer; give him an action task to practice before the next session

A customer sees a problem and is ready to act; assigning tasks to an uncommitted client invites failure.

C. Visitor; compliment him for coming and avoid assigning him any tasks ✓ KEY

A visitor sees no problem to work on; the counselor offers sincere compliments, explores what he might want, and gives no tasks.

D. Customer; confront his denial and set firm goals for his anger at once

Confronting denial is not a solution-focused stance, and the teen is not in a customer relationship.

Rationale: De Shazer described three kinds of relationships: visitor (no complaint or goal yet), complainant (sees a problem but expects others to change) and customer (sees a problem and is willing to act). These describe the relationship at a moment, not fixed client traits. With visitors, counselors build rapport through compliments and curiosity about what the client might want, saving tasks for later.

References: Corey (Theory & Practice) — Solution-Focused Brief Therapy: customer, complainant and visitor relationships; compliments

☐ Approve ☐ Revise ☐ Retire Notes:

A mother of twins says, "A good mother never needs help, so I'm failing." Her narrative counselor asks, "Where did you learn that idea about good mothers? Who benefits when mothers believe it? Are there times it doesn't fit your life?" The counselor is MAINLY engaged in:

A. Deconstructing a dominant discourse ✓ KEY

Deconstruction questions unpack the taken-for-granted discourses behind a problem story so the client can choose whether to accept them.

B. Disputing an irrational belief

Disputing tests a belief's logic and evidence (REBT); narrative questions trace a belief's cultural origins and effects instead.

C. Interpreting an unconscious conflict

Narrative counselors take a not-knowing stance and do not interpret hidden motives.

D. Naming an Adlerian basic mistake

Basic mistakes in the lifestyle are an Adlerian concept tied to private logic, not cultural discourse.

Rationale: Michael White and David Epston drew on the idea that problem stories are shaped by dominant cultural discourses. Deconstruction exposes where those discourses came from, whose interests they serve and how they have shaped the client's thin, problem-saturated description of herself. This opens space for a thicker, preferred story.

References: Corey (Theory & Practice) — Narrative Therapy: deconstruction and dominant discourses

☐ Approve ☐ Revise ☐ Retire Notes:

A young adult reclaiming her life from an eating disorder invites three friends to a session. While she talks with the counselor, the friends listen; then they describe what stood out to them, what it brought up in their own lives and how it moved them, and she responds. This narrative practice is known as:

A. Externalizing conversation

Externalizing separates the person from the problem in language; it does not depend on an audience of witnesses.

B. Family sculpting

Family sculpting (Satir) has members arrange one another physically to show relationships.

C. Reflecting team critique

Outsider witnesses do not critique or evaluate the client; reflecting teams also come from Andersen's work with clinicians.

D. Definitional ceremony ✓ KEY

In a definitional ceremony, outsider witnesses hear the preferred story and retell it, which authenticates and thickens it.

Rationale: White adapted the definitional ceremony from anthropologist Barbara Myerhoff. The client tells her preferred story, outsider witnesses retell what resonated with them, and the client then responds to the retellings. The audience helps acknowledge and thicken the new identity claims so they are not held by the client alone.

References: Goldenberg (Family Therapy) — Narrative therapy: definitional ceremonies and outsider witnesses

► **Check:** Confirm that Goldenberg (9th ed.) covers definitional ceremonies/outsider witnesses; Corey covers them only briefly. Reflecting-team distractor wording ("critique") chosen so it is clearly not the practice described.

☐ Approve ☐ Revise ☐ Retire Notes:

After a session, a narrative counselor mails a client a letter that quotes the client's own words about how he stood up to "the Anger," notes questions the counselor is still curious about and names the steps he took. What is the MAIN purpose of such therapeutic letters?

A. To give him the counselor's formal diagnosis and expert view of his case

Narrative counselors avoid the expert, diagnostic stance; letters center the client's words, not a diagnosis.

B. To document and thicken his emerging story so it lasts between sessions ✓ **KEY**

Letters record the alternative story in lasting form, extending and thickening it beyond the session.

C. To assign behavioral homework with rewards for each completed step

Contingent rewards for homework belong to behavior therapy, not narrative letter writing.

D. To keep a legal record of the session in place of case notes

Therapeutic letters are an intervention addressed to the client; they do not replace clinical records.

Rationale: David Epston in particular developed letter writing as a narrative practice. Letters capture unique outcomes and the client's preferred story in a durable, rereadable form, turning a thin problem story into a thicker, richly described one. White and Epston also used certificates and documents to celebrate new identities.

References: Corey (Theory & Practice) — Narrative Therapy: letters and documents that support the alternative story

☐ Approve ☐ Revise ☐ Retire Notes:

Which counselor practice BEST reflects the egalitarian relationship emphasized in feminist therapy?

A. Keeping a neutral blank screen so the client can project onto the counselor

The blank-screen stance is psychoanalytic and widens the power gap feminist counselors try to reduce.

B. Making the counselor's diagnosis the main basis for all treatment decisions

Feminist counselors are cautious about diagnostic labels and do not make the counselor's view the sole authority.

C. Demystifying counseling and treating the client as expert on her life ✓ **KEY**

Demystifying the process and honoring the client's expertise reduce the power difference and support collaboration.

D. Avoiding any talk of values or social issues so the counselor stays neutral

Feminist counselors see neutrality as impossible and openly address values and social context.

Rationale: Feminist therapy views the counseling relationship as egalitarian. Counselors demystify the process through informed consent, share their orientation and values, use appropriate self-disclosure and treat clients as experts on their own lives. This models healthier uses of power and supports empowerment.

References: Corey (Theory & Practice) — Feminist Therapy: the egalitarian therapeutic relationship

☐ Approve ☐ Revise ☐ Retire Notes:

A home health aide who immigrated five years ago blames herself for "being weak" because she has not confronted an agency that underpays her. Her counselor helps her map the differences in power between her and the agency, including economic, legal and gender factors, and explore ways she might access power. This feminist intervention is:

A. Power analysis ✓ KEY

Power analysis examines how unequal access to power shapes the client's situation and identifies ways to use or gain power.

B. Gender-role analysis

Gender-role analysis explores messages about how women and men should be; this work centers on broader power differences.

C. Cognitive reframing

Reframing offers a new meaning for the same facts but does not map structural power or plan to access it.

D. Assertiveness drills

Assertiveness training may follow, but the intervention described is an analysis of power, not skill rehearsal.

Rationale: In power analysis, feminist counselors help clients see the differences in power between groups in society and in their own relationships, and how these shape their options and self-blame. Clients then identify kinds of power they can use. Shifting the view from personal weakness to social context is a core feminist move.

References: Corey (Theory & Practice) — Feminist Therapy: power analysis and intervention

☐ Approve ☐ Revise ☐ Retire Notes:

Relational-cultural theory (RCT), developed by Jean Baker Miller, Judith Jordan and colleagues at the Stone Center, challenges traditional models of development. According to RCT, psychological growth occurs MAINLY through:

A. Separating from others to achieve independence and a firm, autonomous self

RCT critiques the separation-individuation model that treats autonomy as the goal of maturity.

B. Resolving unconscious drives and conflicts formed in early psychosexual stages

This describes Freudian theory, not RCT's focus on connection.

C. Learning new behaviors through reinforcement in structured, graded tasks

This is a behavioral account of change and omits the relational focus of RCT.

D. Growth-fostering relationships marked by mutual empathy and empowerment ✓ KEY

RCT holds that people grow through and toward connection, in relationships of mutual empathy and mutual empowerment.

Rationale: Relational-cultural theory argues that people grow through connection rather than separation. Growth-fostering relationships involve mutual empathy and mutual empowerment, and chronic disconnection, often linked to marginalization and power, leads to isolation and suffering. RCT is closely tied to feminist therapy and attends to culture and social justice.

References: Corey (Theory & Practice) — Feminist Therapy: relational-cultural theory (Miller, Jordan)

☐ Approve ☐ Revise ☐ Retire Notes:

Near the end of a session about her drinking, a client says, "I'm going to call the outpatient program on Monday and set up an intake." Using the DARN-CAT framework, which kind of change talk is this?

A. Desire

Desire statements express wanting or wishing ("I want to cut down"), not a pledge to act.

B. Commitment ✓ KEY

"I'm going to" signals an intention to act, which is commitment, a form of mobilizing change talk.

C. Reasons

Reasons statements give specific benefits of change ("I'd sleep better").

D. Taking steps

Taking steps describes an action already done ("I called them yesterday"); this call has not happened yet.

Rationale: Miller and Rollnick group change talk into preparatory talk (Desire, Ability, Reasons, Need) and mobilizing talk (Commitment, Activation, Taking steps). Commitment language such as "I will" or "I'm going to" predicts change more strongly than preparatory talk. Counselors listen for, reflect and strengthen change talk.

References: Miller & Rollnick (MI) — Evoking: preparatory (DARN) and mobilizing (CAT) change talk

☐ Approve ☐ Revise ☐ Retire Notes:

A father who smokes in the car with his young children says being a present, healthy dad is "the most important thing in my life." Which response BEST develops discrepancy in a way consistent with the spirit of motivational interviewing?

A. "Your kids matter most to you. How does smoking fit with that?" ✓ KEY

This reflects his stated value and invites him to voice the gap between it and his behavior, evoking his own reasons.

B. "You need to know that secondhand smoke in a car harms children's lungs."

Lecturing reflects the righting reflex and tends to provoke sustain talk rather than change talk.

C. "If your kids matter most, then you would have quit smoking long ago."

This is a confrontational argument that challenges his sincerity and is likely to increase discord.

D. "Let's set a quit date now, and I'll give you a list of nicotine aids."

Jumping to planning before he voices his own motivation skips evoking and risks resistance.

Rationale: Developing discrepancy means helping clients notice the gap between their values or goals and their current behavior, so the client, not the counselor, voices the argument for change. The MI spirit of partnership, acceptance, compassion and evocation rules out lecturing or confrontation, which tend to trigger sustain talk and discord.

References: Miller & Rollnick (MI) — MI spirit; developing discrepancy between values and behavior

☐ Approve ☐ Revise ☐ Retire Notes:

After building rapport with a client who has diabetes, depression and money worries, the counselor and client agree that they will concentrate on his eating and medication habits for now. In the four processes of motivational interviewing, this step is called:

A. Engaging

Engaging is establishing the working relationship, which has already occurred here.

B. Evoking

Evoking draws out the client's own motivation for change once a direction has been chosen.

C. Focusing ✓ KEY

Focusing is developing and keeping a specific direction or change goal for the conversation.

D. Planning

Planning comes last, when the client is ready to commit to a specific plan of action.

Rationale: Miller and Rollnick describe four overlapping processes: engaging (building the relationship), focusing (agreeing on a direction), evoking (drawing out the client's own motivation) and planning (developing commitment and a concrete plan). Each builds on the one before, and counselors may return to earlier processes as needed.

References: Miller & Rollnick (MI) — The four processes: engaging, focusing, evoking, planning

☐ Approve ☐ Revise ☐ Retire Notes:

After a client self-harms, her DBT therapist works with her to trace, step by step, her vulnerability factors that day, the prompting event, each thought, feeling and action leading to the behavior, and the consequences that followed. This procedure is called:

A. Radical acceptance

Radical acceptance is a distress tolerance skill of fully accepting reality as it is; it does not map links to a behavior.

B. A thought record

A thought record tracks situations, automatic thoughts and alternative responses (CBT), not a full behavioral sequence.

C. Wise mind practice

Wise mind is a mindfulness skill integrating emotion mind and reasonable mind, not an analysis of a behavior.

D. Behavioral chain analysis ✓ KEY

Chain analysis maps every link from vulnerability and prompting event to the target behavior and its consequences.

Rationale: In DBT, a behavioral chain analysis is done after a target behavior such as self-harm. Therapist and client identify vulnerabilities, the prompting event, the chain of thoughts, emotions, sensations and actions, the behavior and its consequences. Each link becomes a point where a skill could be used instead, and a solution analysis follows.

References: Corey (Theory & Practice) — Dialectical behavior therapy: behavioral chain analysis

☐ Approve ☐ Revise ☐ Retire Notes:

A client in a DBT skills group practices asking her roommate to pay his share of the rent. At one point she says, "If you get your half to me by Friday, I'll stop bringing it up and things can get back to normal between us." In the DEAR MAN skill, this statement illustrates:

A. Assert

Assert is clearly asking for what one wants or saying no ("I need your half of the rent").

B. Reinforce ✓ KEY

Reinforce means rewarding the person ahead of time by explaining the positive effects of getting what one asks for.

C. Negotiate

Negotiate is being willing to give to get, such as offering alternatives or a compromise.

D. Express

Express is sharing one's feelings and opinions about the situation using "I" statements.

Rationale: DEAR MAN is a DBT interpersonal effectiveness skill: Describe the situation, Express feelings, Assert the request, Reinforce the other person in advance, stay Mindful, Appear confident and Negotiate. Reinforcing explains what the other person gains by agreeing, which makes a yes more likely.

References: Linehan (DBT Skills Training Manual) — Interpersonal effectiveness: DEAR MAN skills

☐ Approve ☐ Revise ☐ Retire Notes:

A client says she freezes and goes silent whenever her manager raises his voice. Her DBT therapist responds, "Growing up, speaking up when your father yelled got you punished, so freezing makes sense given what you learned." In Linehan's levels of validation, this response BEST illustrates:

A. Validating in terms of past learning or biology ✓ KEY

This level explains the response as understandable given the client's history, even if it does not fit the present situation.

B. Normalizing in terms of present circumstances

This level says the response would make sense for anyone in the current situation, without reference to the client's history.

C. Articulating the client's un verbalized feelings

This level puts into words what the client has not said, such as a feeling she has not named.

D. Radical genuineness with the client as an equal

Radical genuineness treats the client as a capable equal rather than fragile; it does not explain a response by history.

Rationale: Linehan described six levels of validation: (1) staying awake and attentive, (2) accurate reflection, (3) articulating the un verbalized, (4) validating in terms of past learning or biology, (5) normalizing in terms of present context, and (6) radical genuineness. Validation balances DBT's change strategies, communicating that the client's responses make sense, which supports the acceptance side of the acceptance-change dialectic.

References: Linehan (DBT Skills Training Manual) — Validation and its levels · Corey (Theory & Practice) — Dialectical behavior therapy: validation strategies and the acceptance-change dialectic

► **Check:** The six validation levels are set out in Linehan (1997, "Validation and psychotherapy"); confirm the skills manual edition cited presents all six.

☐ Approve ☐ Revise ☐ Retire Notes:

After four sessions on grief, a 58-year-old client leaves a voicemail cancelling her next appointment and saying she 'just doesn't need counseling anymore.' The counselor believes important work remains. What is the BEST response?

A. Close the file without further contact, since the client has made her choice

Respecting autonomy does not require silence; a brief outreach to offer closure and options is good practice.

B. Reach out, invite a closing session to review progress and offer referral options ✓ **KEY**

This respects her decision while offering pretermination counseling, a chance to explore her reasons and a path back or elsewhere.

C. Call several times each week until she agrees to resume her regular appointments

Repeated pressure disregards client autonomy and could feel coercive.

D. Mail a letter stating that ending now will most likely cause her grief to worsen

Predicting harm to persuade her to return is a pressure tactic and is not supported by the facts at hand.

Rationale: Clients may end counseling early for many reasons, including satisfaction, cost, discomfort or a rupture in the relationship. The counselor respects the choice but reaches out once to explore the decision, offer a closing session to consolidate gains and provide referrals, leaving the door open to return.

References: ACA Code of Ethics — A.11.c Appropriate Termination · Gladding (Counseling: A Comprehensive Profession) — Termination: premature termination initiated by clients

☐ Approve ☐ Revise ☐ Retire Notes:

After 12 sessions and several changes in approach, a client with social anxiety shows no measurable improvement, and the client and counselor agree that counseling is not helping. According to the ACA Code of Ethics, what should the counselor do?

A. Continue the same sessions, since ending now would amount to abandonment

Ending a relationship that is not helping, with proper preparation and referral, is not abandonment; continuing without benefit is the concern.

B. End services at the next visit without discussion to avoid more expense

An abrupt ending without discussion denies the client pretermination counseling.

C. Keep meeting monthly until the client's insurance benefits are used up

Continuing only to use available benefits does not serve the client and raises ethical concerns.

D. Provide pretermination counseling and recommend other service providers ✓ **KEY**

The Code directs counselors to end services that are not benefiting the client, with pretermination counseling and referral as needed.

Rationale: ACA Code A.11.c directs counselors to terminate when it is reasonably apparent that the client no longer needs help, is not likely to benefit or is being harmed. Counselors provide pretermination counseling and recommend other service providers when necessary, so the client leaves with a path to help that may fit better.

References: ACA Code of Ethics — A.11.c Appropriate Termination

☐ Approve ☐ Revise ☐ Retire Notes:

A school counselor's individual sessions with a 7-year-old will end in three weeks because he has met his goals. Which approach to ending is MOST developmentally appropriate?

A. Mark the remaining sessions on a calendar together and plan a goodbye ritual ✓ **KEY**

Concrete countdowns and a closing ritual give a young child advance notice, predictability and a sense of closure.

B. Tell him at the start of the final session so he has less time to be upset

Last-minute notice gives the child no time to prepare and can feel like abandonment.

C. Avoid mentioning the ending and simply stop scheduling him for sessions

An unexplained stop leaves the child confused and may feel like rejection.

D. Ask his teacher to explain the ending to him after the last session is over

The ending belongs in the relationship; handing it off to someone else after the fact avoids closure.

Rationale: Children understand time concretely, so endings are announced several sessions ahead and made visible, for example by counting down sessions on a calendar. A goodbye ritual, such as a drawing, a memory book or a review of what the child learned, honors the relationship and consolidates gains.

References: Gladding (Counseling: A Comprehensive Profession) — Counseling children: preparing for and ending the relationship

☐ Approve ☐ Revise ☐ Retire Notes:

A client says, 'I keep picking up extra shifts even though I'm exhausted. My parents gave up everything to bring us to this country.' Which counselor response is the BEST example of a reflection of meaning?

A. You're feeling exhausted and a bit trapped by all the extra shifts you work.

This names emotions, which makes it a reflection of feeling rather than of meaning.

B. You've been working many extra shifts lately, even though you're worn out.

This restates content, which makes it a paraphrase.

C. Working this hard seems to be a way of honoring what your parents sacrificed. ✓ **KEY**

This captures the value and purpose that give the behavior its significance for the client, the core of reflecting meaning.

D. Perhaps you overwork because part of you fears you'll disappoint your parents.

This offers a new, counselor-generated explanation (a hidden fear), which is an interpretation.

Rationale: Reflection of meaning, in Ivey's microskills, goes beneath content and feeling to the values, purposes and significance a client attaches to experience. It stays close to what the client has implied, whereas interpretation offers a new frame drawn from the counselor's perspective.

References: Gladding (Counseling: A Comprehensive Profession) — Counseling skills: reflection of meaning versus interpretation

☐ Approve ☐ Revise ☐ Retire Notes:

A client says, 'Between my night classes and my daughter's soccer schedule, I haven't had a free evening in a month.' Which counselor response is the BEST paraphrase?

A. Your evenings have been taken up by school and family commitments for weeks. ✓ KEY

This restates the essential content in fresh, concise words, which is what a paraphrase does.

B. Between night classes and soccer, you haven't had a free evening in a month.

Repeating the client's words almost verbatim is parroting, which can sound mechanical.

C. You're feeling worn out and maybe resentful that there's no time for you.

This focuses on emotion, making it a reflection of feeling.

D. It sounds like you may have trouble saying no when others need something.

This adds a counselor hypothesis about the client, which is interpretation.

Rationale: A paraphrase restates the content of a client's message in the counselor's own, usually briefer, words. It shows accurate listening and invites the client to confirm or correct. Verbatim repetition (parroting) adds little and can feel mechanical.

References: Gladding (Counseling: A Comprehensive Profession) — Counseling skills: paraphrasing and the pitfall of parroting

☐ Approve ☐ Revise ☐ Retire Notes:

A client who was open in earlier sessions has arrived late three weeks in a row, answers in single words and avoids looking at the counselor. The counselor senses growing distance between them. Which response BEST demonstrates immediacy?

A. You've been late three times now, and our policy is that sessions can't run over.

Stating policy addresses logistics, not what is happening in the relationship.

B. Coming late may be your way of resisting the hard work we have started to do here.

This is an interpretation of motive and may increase defensiveness.

C. Let's go back to the goals we set in our first session and review your progress.

Shifting to goals sidesteps the relational shift the counselor is noticing.

D. I'm sensing some distance between us lately, and I wonder what's happening for you. ✓ KEY

The counselor tentatively names what is occurring between them right now and invites the client to explore it.

Rationale: Immediacy is the counselor's direct, tentative comment on what is happening in the counseling relationship in the present moment. Used when tension or distance develops, it brings the relationship itself into the conversation so ruptures can be explored and repaired.

References: Gladding (Counseling: A Comprehensive Profession) — Counseling skills: relationship immediacy

☐ Approve ☐ Revise ☐ Retire Notes:

During intake, a client looks down or past the counselor most of the time and moves her chair back slightly when the counselor leans in. She explains that in her family, looking an elder or professional directly in the eye is considered disrespectful. What should the counselor do?

A. Gently encourage more eye contact to build trust and openness over time

This imposes the counselor's cultural norm on the client.

B. Adjust own eye contact and distance, following the client's comfort cues ✓ **KEY**

Attending behaviors are adapted to the client's cultural norms for gaze and personal space.

C. Note the gaze avoidance as a possible sign of depression or social anxiety

The client has given a cultural explanation; pathologizing it risks misdiagnosis.

D. Lean in closer to show warmth so that she feels more engaged in session

She has signaled discomfort with closeness; moving in would ignore her cue.

Rationale: Guidelines such as SOLER reflect a particular cultural norm. Comfort with eye contact (kinesics) and conversational distance (proxemics) varies across cultures and individuals, so culturally responsive attending means reading and following the client's cues rather than treating one style as the standard.

References: Sue & Sue (Counseling the Culturally Diverse) — Nonverbal communication: proxemics and kinesics

☐ Approve ☐ Revise ☐ Retire Notes:

During child-centered play therapy, a 5-year-old holds up a paintbrush and asks the counselor, 'What color should I make the sky?' Which response BEST fits this approach?

A. Blue, just like the real sky outside.

Answering for the child directs the play and takes away a chance for self-direction.

B. Why do you want me to pick for you?

"Why" questions ask young children to explain motives and can feel evaluative.

C. In here, you get to decide that. ✓ **KEY**

Returning responsibility communicates trust in the child and builds decision-making and self-control.

D. Let's paint it together and find out.

Joining to share the choice still shifts direction away from the child.

Rationale: In child-centered play therapy (Axline, Landreth), the counselor follows the child's lead. Key responses include tracking (describing what the child is doing, such as "You're putting that one on top") and returning responsibility, which hands decisions back to the child to foster autonomy and self-responsibility.

References: Gladding (Counseling: A Comprehensive Profession) — Counseling children: child-centered play therapy responses

☐ Approve ☐ Revise ☐ Retire Notes:

A counselor trains parents to hold weekly 30-minute nondirective play sessions with their own child at home, using reflective listening, tracking and limit setting, while the counselor supervises and gives feedback on their skills. Which approach is this?

A. Filial therapy ✓ KEY

Filial therapy, developed by Bernard and Louise Guerney, trains parents as the agents of child-centered play sessions.

B. Theraplay

Theraplay (Jernberg) is an adult-led, attachment-based approach using structured, playful engagement, not nondirective play.

C. Parent-child interaction therapy

PCIT (Eyberg) coaches parents live, usually by earpiece, and includes a parent-directed discipline phase.

D. Sandplay therapy

Sandplay (Kalff, building on Lowenfeld's World Technique) is a Jungian method in which the child builds scenes in a sand tray with the therapist.

Rationale: Filial therapy treats parents as partners: the counselor teaches child-centered play skills, supervises practice sessions and then helps parents generalize the sessions to home. It aims to strengthen the parent-child relationship as the vehicle of change.

References: Gladding (Counseling: A Comprehensive Profession) — Counseling children: filial therapy and sand tray approaches

☐ Approve ☐ Revise ☐ Retire Notes:

A 34-year-old client says he badly wants a close relationship, yet he pulls away whenever someone gets close because he expects to be hurt and doubts that he deserves love. In Bartholomew and Horowitz's four-category model of adult attachment, which style BEST fits?

A. Preoccupied

Preoccupied adults see themselves negatively but others positively, so they pursue closeness anxiously rather than withdraw.

B. Dismissing-avoidant

Dismissing adults view themselves positively and downplay any wish for closeness.

C. Secure

Secure adults hold positive views of self and others and are comfortable with intimacy.

D. Fearful-avoidant ✓ KEY

He wants closeness but avoids it, with negative views of both himself and others.

Rationale: Bartholomew and Horowitz crossed a model of self (worthy or unworthy) with a model of others (trustworthy or not). Secure is positive-positive, preoccupied is negative self and positive other, dismissing is positive self and negative other, and fearful is negative on both, producing a desire for closeness alongside avoidance.

References: Berk (Development Through the Lifespan) — Early adulthood: adult attachment styles and intimate relationships

► **Check:** Confirm Berk 7th ed covers Bartholomew and Horowitz's four-category adult model; the model itself is standard.

☐ Approve ☐ Revise ☐ Retire Notes:

Bowlby proposed that through repeated experiences with caregivers, a child builds mental representations of whether others are dependable and whether the self is worthy of care, which then guide expectations in later relationships. What did Bowlby call these representations?

A. Secure base

A secure base (emphasized by Ainsworth) is the caregiver from whom a child explores, not the mental representation itself.

B. Internal working models ✓ KEY

Bowlby's internal working models are expectations about self and others built from early caregiving.

C. Object constancy

Object constancy is Mahler's term for holding a stable inner image of the caregiver.

D. Cognitive schemas

Schemas are Piaget's and Beck's terms for organized knowledge or core beliefs, not Bowlby's attachment concept.

Rationale: Bowlby argued that attachment experiences are internalized as working models of self and others. These models operate largely outside awareness and shape how a person seeks support, interprets others' behavior and forms close relationships later in life.

References: Berk (Development Through the Lifespan) — Bowlby's ethological theory of attachment: internal working model

☐ Approve ☐ Revise ☐ Retire Notes:

For two years a Korean American graduate student rejected anything she saw as White culture and worked only within Asian American groups. Lately she feels uneasy with that rigid stance, wonders whether it limits her personal autonomy and is sorting out which values are truly her own. In the Racial/Cultural Identity Development (R/CID) model, she is MOST likely in which stage?

A. Dissonance

Dissonance is the earlier stage in which experiences first challenge a conforming, majority-oriented view.

B. Resistance and immersion

This describes her previous two years of rejecting the dominant culture and endorsing her own group.

C. Introspection ✓ KEY

Discomfort with rigid group views and a search for personal autonomy mark the introspection stage.

D. Integrative awareness

Integrative awareness reflects a secure, flexible identity that she is still working toward.

Rationale: The R/CID model (Atkinson, Morten and Sue, expanded by Sue and Sue) describes five stages: conformity, dissonance, resistance and immersion, introspection and integrative awareness. Introspection follows immersion, as the person questions rigid loyalty to the group and works out an individual identity.

References: Sue & Sue (Counseling the Culturally Diverse) — Racial/Cultural Identity Development (R/CID) model

☐ Approve ☐ Revise ☐ Retire Notes:

After a classmate's remark about her family's background, a 15-year-old Mexican American student begins asking her grandparents about their lives in Mexico, joins a Latinx student club and reads Chicano history, though she has not settled what her heritage means to her. In Phinney's model of ethnic identity, she is in which stage?

A. Ethnic identity search (moratorium) ✓ KEY

Active exploration of heritage, often prompted by an encounter, without firm commitment marks the search stage.

B. Unexamined ethnic identity

This first stage involves little interest in or exploration of ethnicity; she is now actively exploring.

C. Achieved ethnic identity

Achievement requires a secure, committed sense of what ethnicity means, which she has not yet reached.

D. Integrative awareness

Integrative awareness is the final stage of the R/CID model, not part of Phinney's model.

Rationale: Phinney applied Marcia's identity statuses to ethnicity: an unexamined identity (diffuse or foreclosed), an ethnic identity search or moratorium often sparked by a significant experience, and an achieved identity marked by clarity and commitment. Exploration supported by family and community fosters well-being.

References: Berk (Development Through the Lifespan) — Adolescence: ethnic identity (Phinney)

☐ Approve ☐ Revise ☐ Retire Notes:

A counselor who holds several privileged identities is working with a client from a marginalized racial and religious group. She sets aside time to explore with the client how their differences in power and privilege affect trust, communication and the work they are doing together. In the MSJCC, this falls MOST directly within which developmental domain?

A. Counselor self-awareness

Self-awareness concerns the counselor's own identities and biases; here the focus is how the two identities interact.

B. Counseling relationship ✓ KEY

This domain addresses how privileged and marginalized statuses of counselor and client shape the relationship.

C. Client worldview

Client worldview concerns understanding the client's identities, values and experiences of oppression.

D. Counseling and advocacy interventions

This domain covers intervening at individual through international levels, not exploring the dyad itself.

Rationale: The MSJCC places the counselor-client pair in one of four quadrants defined by privileged and marginalized statuses. It then describes four developmental domains: counselor self-awareness, client worldview, the counseling relationship, and counseling and advocacy interventions, with attitudes and beliefs, knowledge, skills and action (AKSA) within each.

References: MSJCC (Ratts et al., 2016) — Quadrants and developmental domains: the counseling relationship

☐ Approve ☐ Revise ☐ Retire Notes:

An older counseling text lists ataque de nervios as a 'culture-bound syndrome.' DSM-5-TR uses a broader term that includes cultural syndromes, cultural idioms of distress and cultural explanations or perceived causes. What is that term?

A. Culture-specific disorders

DSM-5-TR moved away from framing these patterns as disorders tied to one culture.

B. Atypical presentations

This implies a deviation from a supposedly typical norm, the view the newer framework rejects.

C. Ethnic variant syndromes

This is not a DSM-5-TR term.

D. Cultural concepts of distress ✓ KEY

DSM-5-TR groups cultural syndromes, idioms of distress and explanations under cultural concepts of distress.

Rationale: DSM-5-TR replaced "culture-bound syndromes" with cultural concepts of distress, recognizing that all distress is culturally shaped and that these concepts may not map onto a single DSM disorder. The Cultural Formulation Interview helps counselors explore the client's own understanding of the problem.

References: DSM-5-TR — Section III: Culture and Psychiatric Diagnosis, cultural concepts of distress

☐ Approve ☐ Revise ☐ Retire Notes:

Using James and Gilliland's six-step model of crisis intervention, a counselor has defined the problem from the client's point of view, ensured the client's safety and provided support. What is the counselor's NEXT step?

A. Obtain commitment to a plan

Obtaining commitment is the sixth and final step.

B. Make a concrete plan

Making plans is the fifth step, after alternatives are explored.

C. Examine alternatives ✓ KEY

The fourth step is exploring options, supports and coping mechanisms with the client.

D. Complete a full diagnosis

Formal diagnosis is not a step in the model; assessment runs throughout.

Rationale: James and Gilliland's model has three listening steps (define the problem, ensure client safety, provide support) followed by three acting steps (examine alternatives, make plans, obtain commitment). Assessment of the client's state and safety continues throughout all six steps.

References: James & Gilliland (Crisis Intervention Strategies) — Six-step model: define the problem, ensure safety, provide support, examine alternatives, make plans, obtain commitment

☐ Approve ☐ Revise ☐ Retire Notes:

In Kanel's ABC model of crisis intervention, a counselor first builds rapport through attending and reflection. The counselor then identifies the precipitating event, the client's perception of it and the resulting distress and impaired functioning. What is this second phase called?

A. Boiling down the problem ✓ KEY

The B phase narrows the focus to the trigger, the client's perception and the subjective distress.

B. Coping

Coping is the C phase, which follows and focuses on current and new coping strategies.

C. Achieving contact

Achieving contact is the A phase, the rapport building described first in the stem.

D. Brief safety planning

Safety is assessed throughout but is not the name of a phase in the ABC model.

Rationale: Kanel's ABC model has three phases: A, achieving (developing and maintaining) contact; B, boiling down the problem by identifying the precipitating event, perceptions and distress; and C, coping, in which the counselor explores past and new coping and builds a plan. Its focus on perception reflects the view that crises arise from how events are interpreted.

References: Kanel (A Guide to Crisis Intervention) — ABC model: A, achieving contact; B, boiling down the problem; C, coping

☐ Approve ☐ Revise ☐ Retire Notes:

Before launching a support group at a refugee resettlement agency, a counselor studies the community's needs, the agency's resources and support, and cultural attitudes toward discussing personal problems in a group. In the ASGW Best Practice Guidelines, this planning task is called:

A. Pre-group screening

Screening assesses individual prospective members, not the community and organizational context.

B. Ecological assessment ✓ KEY

ASGW asks group workers to assess the community, agency and cultural context before forming a group.

C. Professional disclosure

Professional disclosure informs members about the leader and the group; it is not a needs assessment.

D. Group processing

Processing is the later, ongoing reflection on what happens in sessions.

Rationale: The ASGW Best Practice Guidelines organize group work into planning, performing and group processing. In planning, an ecological assessment of community needs, organizational support and cultural factors guides whether and how a group is offered. Processing, by contrast, means regular reflection on sessions with co-leaders, members or supervisors.

References: ASGW Best Practice Guidelines — Best Practice in Planning: ecological assessment

☐ Approve ☐ Revise ☐ Retire Notes:

The T-group (training group), first developed at the National Training Laboratories in Bethel, Maine, used here-and-now feedback to teach human relations skills. Whose field theory and group dynamics research led to this movement?

A. Carl Rogers

Rogers developed the basic encounter group, which grew alongside but separately from T-groups.

B. Jacob Moreno

Moreno developed psychodrama and is credited with coining the term group therapy.

C. Joseph Pratt

Pratt led early educational "classes" for patients with tuberculosis in the early 1900s.

D. Kurt Lewin ✓ KEY

Lewin's field theory and group dynamics work led his colleagues to found NTL and the T-group.

Rationale: Kurt Lewin studied how group forces shape individual behavior. His work led to the National Training Laboratories in 1947, where T-groups used here-and-now feedback to build interpersonal awareness. Rogers later extended the experiential group into the encounter group movement.

References: Gladding (Counseling: A Comprehensive Profession) — History of group work: Lewin, T-groups and encounter groups

☐ Approve ☐ Revise ☐ Retire Notes:

A counselor suggests that a recently widowed client try a community bereavement group run by and for people who have lost a spouse, with no professional leader. Compared with a counseling or therapy group, what BEST characterizes this kind of group?

A. It uses a professional leader trained to treat diagnosable mental disorders

This describes a psychotherapy group, not a peer-run group.

B. It follows a strict time-limited format with formal screening of all members

Mutual-aid groups are usually open and ongoing, without formal clinical screening.

C. It is peer-led, with members who share a concern offering one another support ✓ KEY

Self-help and mutual-aid groups are led by members who share the problem and help one another.

D. It focuses mainly on reshaping personality through deep insight and analysis

Reconstructive personality change is a goal of some long-term therapy groups, not of mutual-aid groups.

Rationale: Self-help and mutual-aid groups, such as Alcoholics Anonymous or widow-to-widow programs, are organized around a shared concern and led by members rather than professionals. They emphasize support, hope, universality and practical help, and they can complement rather than replace professional counseling.

References: Gladding (Counseling: A Comprehensive Profession) — Group work: self-help and mutual-aid groups

☐ Approve ☐ Revise ☐ Retire Notes:

A 15-year-old in a court-mandated anger management group sits with his hood up and says, 'I'm only here because the judge made me.' What is the leader's BEST initial response?

- A. Acknowledge he did not choose to come and invite him to say how that feels** ✓ KEY
Openly naming involuntary status respects his autonomy and turns resistance into material for the group.
- B. Explain that members who do not take part may be reported back to the court**
Leading with threats reinforces the power struggle and deepens resistance.
- C. Ask the other members to persuade him that the group will really help him**
Recruiting peers to convince him puts him on the spot and can feel like ganging up.
- D. Ignore the remark and move on so he gets no attention for his resistance**
Ignoring a direct statement misses a chance to engage him and may model avoidance.

Rationale: Adolescents, and especially involuntary members, often test leaders and guard their autonomy. Acknowledging their reluctance openly, respecting choices about what to disclose and inviting them to talk about being sent helps build trust and often reduces resistance more than pressure or avoidance.

References: Gladding (Counseling: A Comprehensive Profession) — Group work with adolescents and involuntary members

☐ Approve ☐ Revise ☐ Retire Notes:

In the ninth session of a cohesive interpersonal group, Malik tells Rosa that her advice-giving feels 'preachy,' and Rosa responds sharply. The room goes quiet. What should the leader do?

- A. Move on to a new topic to protect the cohesion the group has built so far**
Avoiding conflict teaches members that strong feelings are unsafe and wastes useful material.
- B. Help both speak directly to each other and explore what the exchange stirs up** ✓ KEY
Keeping the conflict in the here-and-now with direct communication makes it a source of learning.
- C. Ask Malik and Rosa to work out their disagreement privately after the session**
Moving conflict outside the group fosters subgrouping and removes it from shared work.
- D. Point out that Malik's feedback was too harsh and ask him to rephrase it now**
Taking sides shuts down exploration and positions the leader as judge.

Rationale: Yalom notes that conflict is inevitable and, in a cohesive group, can be highly productive. The leader keeps it in the here-and-now, encourages members to speak directly and own their reactions, and helps the group examine what the exchange reveals about each person's interpersonal patterns.

References: Yalom & Leszcz (Group Psychotherapy) — Conflict in the therapy group (the advanced group)

☐ Approve ☐ Revise ☐ Retire Notes:

A member describes feeling ignored at family dinners. The leader asks her, 'Is there anyone in this room who you feel might be ignoring you right now?' In Yalom's two-tiered approach to the here-and-now, this question is an example of:

A. Process illumination

Illumination is the second tier, in which the group reflects on and makes sense of what has happened.

B. Universality

Universality is a therapeutic factor, the relief of learning others share one's problems.

C. Catharsis

Catharsis is the expression of strong emotion, not a leader technique for shifting focus.

D. Here-and-now activation ✓ KEY

Steering an outside concern into members' relationships in the room is the first, activating tier.

Rationale: Yalom describes two tiers of here-and-now work. The leader first activates the here-and-now by moving the focus from outside, past events to members' reactions to one another in the room. The leader then illuminates the process, helping members reflect on and understand what just occurred; activation without illumination yields emotion without learning.

References: Yalom & Leszcz (Group Psychotherapy) — Ch. 6: The here-and-now, activation and illumination of process

☐ Approve ☐ Revise ☐ Retire Notes:

A 9-year-old's frequent stomachaches keep his worried mother home from work. A therapist directs the boy to pretend to have a stomachache at set times and the mother to pretend to comfort him, turning the symptom into play while the mother takes back her caretaking role. This technique is MOST closely associated with:

A. Salvador Minuchin

Minuchin's structural therapy uses enactments, joining and boundary making rather than pretend directives.

B. Murray Bowen

Bowen emphasized differentiation, triangles and genograms, with the therapist as a calm coach.

C. Cloé Madanes ✓ KEY

Madanes developed pretend techniques within strategic family therapy.

D. Virginia Satir

Satir's experiential approach focuses on communication stances, self-esteem and sculpting.

Rationale: Madanes viewed some children's symptoms as attempts to help a parent that also upset the family hierarchy. Pretend techniques ask the child to pretend to have the symptom and the parent to pretend to help, making the symptom voluntary and playful so it loses its function and the parent resumes an appropriate caregiving role.

References: Goldenberg (Family Therapy) — Strategic family therapy: Madanes and pretend techniques

☐ Approve ☐ Revise ☐ Retire Notes:

In a group setting, a guide uses a client's family chronology to have members role-play scenes from the lives of the client's parents and grandparents, helping the client see them as people and let go of old learnings. This experiential technique, developed by Virginia Satir, is called:

A. Family reconstruction ✓ KEY

Satir's family reconstruction re-enacts multigenerational scenes to free the "star" from outdated patterns.

B. Psychodrama

Psychodrama, created by Jacob Moreno, uses role-play but is not Satir's multigenerational technique.

C. Family sculpting

Sculpting arranges members in a physical tableau of present relationships rather than re-enacting family history.

D. Genogram construction

A genogram maps multigenerational patterns on paper (associated with Bowen) and does not involve role-play.

Rationale: Satir's family reconstruction guides a "star" through key events in three generations, using a family chronology and role players. Family sculpting, also used by Satir and by Duhl and Kantor, positions members physically to show closeness, distance and power. Both are experiential techniques that make family patterns visible and felt.

References: Goldenberg (Family Therapy) — Experiential family therapy: Satir, family reconstruction and sculpting

☐ Approve ☐ Revise ☐ Retire Notes:

Parents of a 14-year-old have grounded him for longer and longer periods for breaking curfew, with no effect. In MRI brief therapy terms, which outcome would represent second-order change?

A. The parents extend the grounding from one week to a full month

More of the same solution is first-order change in degree, not in kind.

B. The family changes the rules governing how curfew is set and handled ✓ KEY

Altering the rules of the system itself is second-order change.

C. The parents add the loss of his phone to the grounding as a penalty

Adding another penalty stays within the same punitive pattern, so it is first-order.

D. The parents keep the same rules but enforce them more strictly

Stricter enforcement of unchanged rules is a first-order adjustment.

Rationale: Watzlawick, Weakland and Fisch distinguished first-order change, a change within a system whose rules stay the same, from second-order change, a change in the system's rules or structure. MRI therapists look for interventions such as reframing that interrupt "more of the same" attempted solutions.

References: Goldenberg (Family Therapy) — MRI interactional view: first-order and second-order change

☐ Approve ☐ Revise ☐ Retire Notes:

Parents decide to stop going into their 3-year-old's room when she cries at bedtime. On the second and third nights, her crying becomes louder and lasts longer than before. How should the counselor explain this?

A. Spontaneous recovery; the plan has failed and should be stopped now

Spontaneous recovery is the return of a behavior after extinction appeared complete; the plan has not failed.

B. Negative reinforcement; her crying is being made stronger by them

Nothing aversive is being removed after crying, so this is not negative reinforcement.

C. Punishment; ignoring the crying is causing her new emotional harm

Withholding attention is extinction, not punishment, and the increase is predictable.

D. Extinction burst; it is expected and fades if parents stay consistent ✓ **KEY**

A temporary surge in a behavior when reinforcement stops is an extinction burst.

Rationale: When a previously reinforced behavior no longer produces its reinforcer, it often increases in frequency or intensity before declining, a pattern called an extinction burst. If caregivers give in during the burst, they reinforce a more intense version of the behavior on an intermittent schedule, so consistency is essential.

References: Corey (Theory & Practice) — Behavior therapy: operant extinction and the extinction burst

☐ Approve ☐ Revise ☐ Retire Notes:

To reduce a child's hand-biting, a behavior analyst delivers a token at the end of every 10-minute interval in which no hand-biting occurs, regardless of what else the child is doing. Which procedure is this?

A. DRA (alternative behavior)

DRA reinforces a specific replacement behavior, such as asking for help, rather than mere absence of the target.

B. DRL (low rates of behavior)

DRL reinforces the target behavior when it occurs at or below a set rate, for behaviors to reduce but not eliminate.

C. DRO (other behavior) ✓ **KEY**

DRO reinforces the absence of the target behavior over an interval.

D. Premack principle

The Premack principle uses a high-probability activity to reinforce a low-probability one.

Rationale: Differential reinforcement of other behavior (DRO) delivers reinforcement when the target behavior has not occurred for a set interval, sometimes called differential reinforcement of zero rates. DRA reinforces a specified alternative behavior, DRI an incompatible one and DRL a low rate of the target itself.

References: Corey (Theory & Practice) — Behavior therapy: differential reinforcement procedures

☐ Approve ☐ Revise ☐ Retire Notes:

An 18-month-old who recently toddled off happily to explore now repeatedly returns to his mother, shadows her, wants her to share each discovery and becomes upset when she leaves, yet sometimes pushes her away. In Mahler's separation-individuation theory, he is in which subphase?

A. Rapprochement ✓ KEY

Around 16 to 24 months, awareness of separateness brings renewed clinging alongside a push for autonomy.

B. Practicing

Practicing (about 10 to 16 months) is the earlier, exhilarated period of exploring with little concern for the mother.

C. Differentiation

Differentiation (about 5 to 10 months) is when the infant first begins to distinguish self from mother.

D. Object constancy

Object constancy (about 24 to 36 months) brings a stable inner image of the mother that eases separations.

Rationale: Mahler described separation-individuation subphases following the early autistic and symbiotic phases: differentiation, practicing, rapprochement and the move toward object constancy. The rapprochement crisis, with its mix of clinging and pushing away, is often linked to later difficulties with closeness and separateness.

References: Corey (Theory & Practice) — Psychoanalytic therapy: object relations theory and Mahler's separation-individuation

☐ Approve ☐ Revise ☐ Retire Notes:

A 2-year-old insists on bringing a worn stuffed rabbit to child care and clutches it when her father leaves. In Winnicott's object relations theory, what function does this transitional object serve?

A. It signals an insecure attachment that will need professional treatment

Transitional objects are a normal part of development, not a sign of pathology.

B. It soothes her and bridges her move from union with a parent to separateness ✓ KEY

Winnicott saw the transitional object as an intermediate space that helps the child tolerate separation.

C. It reflects fixation at the oral stage caused by weaning that came too early

This applies Freud's psychosexual fixation, not Winnicott's concept.

D. It is a part-object she splits into all-good and all-bad parts to manage fear

Splitting and part-objects come from Melanie Klein, not the transitional object concept.

Rationale: Winnicott described the transitional object as the child's first "not-me" possession, standing between inner and outer reality and helping the child manage the gradual move to independence. It develops within a holding environment provided by a good-enough mother, who meets needs well at first and then fails in small, tolerable ways.

References: Corey (Theory & Practice) — Psychoanalytic therapy: object relations (Winnicott, Klein)

► **Check:** Confirm Corey's object-relations section names Winnicott's transitional object and good-enough mother.

☐ Approve ☐ Revise ☐ Retire Notes:

Glasser pictured total behavior as a car, with basic needs as the engine and wants as the steering wheel. Which two components are the front wheels, the parts of behavior a person controls most directly?

A. Feeling and physiology

Feeling and physiology are the back wheels; they follow along and change indirectly when acting and thinking change.

B. Wants and needs

In the metaphor, needs are the engine and wants steer the car; they are not components of total behavior.

C. Acting and thinking ✓ KEY

Acting and thinking are the front wheels: the components over which people have the most direct control.

D. Perceiving and remembering

Perception plays a role in choice theory (the perceived world), but it is not one of the four wheels of total behavior.

Rationale: In choice theory, total behavior has four inseparable components: acting, thinking, feeling and physiology. Glasser argued that we have almost total control over acting and thinking and only indirect control over feeling and physiology. Reality therapists therefore focus on what the client is doing and thinking, expecting feelings and body states to follow.

References: Corey (Theory & Practice) — Reality Therapy: choice theory and total behavior

☐ Approve ☐ Revise ☐ Retire Notes:

A 17-year-old on probation says nothing matters to him, yet he lights up when he talks about his grandmother and about someday becoming a firefighter. A reality therapist notes that these belong to his quality world. In choice theory, the quality world is BEST described as:

A. A personal album of the people, things and ideas that best satisfy his needs ✓ KEY

Glasser described the quality world as a set of specific pictures of the people, activities, possessions and beliefs that we believe will best satisfy our needs.

B. The stored record of past failures that has built up into a failure identity

Failure identity was an early Glasser concept about how people see themselves, not the store of need-satisfying pictures.

C. An unconscious storehouse of repressed wishes that drives his current choices

Repressed unconscious wishes are a psychoanalytic idea; choice theory emphasizes present, conscious choices.

D. The shared set of perceptions about reality that he holds with his peer group

The quality world is highly individual; it is not a set of perceptions shared with a group.

Rationale: The quality world is the core of choice theory's motivation: everyone carries a personal "album" of pictures of what would satisfy the needs for survival, love and belonging, power, freedom and fun. A reality therapist tries to learn what is in the client's quality world and to become part of it, then helps the client evaluate whether current behavior is getting him what he wants.

References: Corey (Theory & Practice) — Reality Therapy: the quality world

☐ Approve ☐ Revise ☐ Retire Notes:

A client agreed to walk for 20 minutes three mornings this week. She returns saying she "just couldn't find the time." Which response BEST reflects the reality therapy principles of accepting no excuses and using no punishment?

A. "That is disappointing. We agreed on this, so I expect you to make up the missed walks."

Expressing disappointment and imposing a make-up penalty is criticism and punishment, which reality therapists avoid because it damages the relationship.

B. "Tell me more about what made this week so hard. Your schedule may explain a great deal."

Inviting a long account of why the plan failed accepts the excuse and keeps the focus off what she will do next.

C. "Maybe this plan asked too much of you. Let's set exercise aside and work on something else."

Dropping the goal is giving up on the client, which Glasser explicitly cautioned against.

D. "Do you still want this? If so, let's make a smaller plan you are sure you can carry out." ✓ KEY

The counselor neither accepts the excuse nor punishes; she checks the want and helps make a simpler, more attainable plan.

Rationale: When a plan is not carried out, reality therapists do not accept excuses, do not criticize or punish, and do not give up. Instead they return to the client's wants, have the client evaluate what she is doing, and make a new plan that is simple, attainable and within her control (the SAMIC qualities of a good plan).

References: Corey (Theory & Practice) — Reality Therapy: planning and commitment; no excuses, no punishment, do not give up

☐ Approve ☐ Revise ☐ Retire Notes:

A 56-year-old client says, "My son moved across the country for work last month. The house is so quiet now, and I still catch myself setting four places at dinner." Which response is the BEST reflection of feeling?

A. "Your son moved away for work last month, and the house is quieter now."

This restates the content of what she said (a paraphrase) but does not name the emotion beneath it.

B. "You are feeling a real sense of loss and loneliness with him gone." ✓ KEY

It identifies the underlying emotions (loss, loneliness) that her words and the image of the extra place setting convey.

C. "How often do the two of you talk now that he lives so far away?"

A question gathers information and shifts focus to logistics rather than reflecting her feeling.

D. "Many parents feel this way at first, and it gets easier with time."

Reassurance and normalizing can feel dismissive and do not reflect her specific feeling back to her.

Rationale: A reflection of feeling names the emotion the client is expressing, often one implied rather than stated, so the client feels understood and can explore it further. Paraphrasing captures content, questions gather information, and reassurance can cut exploration short.

References: Gladding (Counseling: A Comprehensive Profession) — Counseling skills: reflection of feeling versus paraphrasing

☐ Approve ☐ Revise ☐ Retire Notes:

A counselor opens the fourth session by saying, "Last week you talked about the tension with your sister, the late nights at work and how little sleep you are getting, and you decided to try leaving the office by six. Where would you like to begin today?" Which skill is the counselor using, and for what purpose?

A. Paraphrasing, to restate a single message in fresher and shorter words

A paraphrase restates one recent client message; this response pulls together material from an entire prior session.

B. Interpretation, to offer a new frame of reference for the client's issues

The counselor adds no new meaning or explanation; she simply organizes what the client already said.

C. Summarizing, to tie earlier material together and give the session focus ✓ **KEY**

Summaries condense several themes over a longer span and are often used to open a session, provide continuity and set direction.

D. Immediacy, to address what is happening in the relationship right now

Immediacy concerns the here-and-now counselor-client relationship, which this response does not address.

Rationale: Summarizing condenses several client statements, themes or feelings over a longer stretch of time. Counselors use it to begin a session (linking to the last one), to shift topics, to close a session, and to check shared understanding. It differs from paraphrasing in scope, and from interpretation in that it adds no new frame.

References: Gladding (Counseling: A Comprehensive Profession) — Counseling skills: summarization and its uses

☐ Approve ☐ Revise ☐ Retire Notes:

A client says, "I'm completely over the breakup," as her eyes fill with tears and she twists a ring on her finger. Which counselor response is the BEST example of a skilled confrontation?

A. "You say you're over it, and yet your eyes are filling with tears." ✓ **KEY**

It describes the discrepancy between her words and her nonverbal behavior, tentatively and without judgment, and leaves her room to explore it.

B. "You clearly are not over it, and denying it will only make things harder."

This tells the client what she feels and labels her as denying; it is judgmental and likely to provoke defensiveness.

C. "Why are you crying right now if you really are over the breakup?"

"Why" questions about a discrepancy tend to feel accusatory and invite justification rather than exploration.

D. "It sounds like you have worked very hard to move on from this breakup."

This supportive reflection ignores the discrepancy entirely, so it is not a confrontation.

Rationale: In counseling, confrontation means pointing out a discrepancy (between words and nonverbal behavior, between statements, or between words and actions) in a caring, descriptive way. The "you say ... and yet ..." format names both sides without judgment and invites the client to examine the mixed message. Accusing, interpreting or asking "why" tends to raise defenses.

References: Gladding (Counseling: A Comprehensive Profession) — Counseling skills: confrontation of discrepancies

☐ Approve ☐ Revise ☐ Retire Notes:

Over six sessions, a client has said repairing his marriage is his top goal, yet he has cancelled each couples appointment he scheduled. The alliance is strong. The counselor gently notes this discrepancy, and the client is quiet, then says, "I guess I've been avoiding it because I'm afraid she'll tell me she wants out." What should the counselor do NEXT?

A. Restate the discrepancy more firmly so that its importance is not lost on him

He has already acknowledged the discrepancy; pressing it harder adds pressure without adding understanding.

B. Shift to a lighter topic to ease the tension that the confrontation created

Retreating just as he opens up signals that his fear is too much to talk about and loses the opening the confrontation made.

C. Ask him to commit now to attending the next couples appointment he schedules

Moving straight to a commitment skips exploration of the fear that drives the avoidance and risks hollow compliance.

D. Reflect his fear and explore what avoiding the sessions has protected him from ✓ KEY

Following a confrontation, the counselor attends to the client's response; he has accepted the discrepancy, so exploring the underlying fear deepens the work.

Rationale: A confrontation is only as useful as what follows it. The counselor watches how the client responds (denial, partial acceptance or full recognition) and builds on it. Here the client has recognized the discrepancy and named a fear, so empathic reflection and exploration of that fear come before planning new behavior.

References: Gladding (Counseling: A Comprehensive Profession) — Counseling skills: confrontation and evaluating the client's response

► **Check:** Applied-skills judgment item; confirm the Gladding locator on confrontation and that "explore before planning" is the clearly best sequencing.

☐ Approve ☐ Revise ☐ Retire Notes:

A client whose panic attacks resolved months ago has a history of losing caregivers in childhood. Three sessions before a jointly planned ending, she reports two new panic attacks and asks whether she can keep coming "for as long as it takes." What is the BEST response?

A. Agree to continue open-ended weekly counseling, since her symptoms have now returned

Reflexively extending treatment can confirm her fear that she cannot cope without the counselor and skips the meaning of the ending.

B. Explore her feelings about ending, normalize a brief return of symptoms and review gains ✓ KEY

A temporary return of symptoms near termination is a familiar reaction, especially with a history of loss; processing the ending and her skills is the core task.

C. Keep to the plan and avoid discussing the attacks, so as not to reinforce her symptoms

Ignoring the attacks and her feelings about ending leaves important material unprocessed and may feel like another abandonment.

D. Refer her to a new counselor now so that she has ongoing support once the sessions end

An abrupt transfer just before ending sidesteps the termination work and offers no reason why a new counselor is needed.

Rationale: Termination can stir up feelings of loss, particularly for clients with histories of abandonment, and a temporary regression is common. The counselor normalizes this, explores what ending means to the client, reviews her progress and coping skills, and may adjust the pace (for example, spacing sessions) rather than dropping or reflexively extending the plan.

References: Gladding (Counseling: A Comprehensive Profession) — Termination: client reactions and resistance to ending

☐ Approve ☐ Revise ☐ Retire Notes:

A client has met his counseling goals. Rather than stopping abruptly, the counselor proposes moving from weekly sessions to sessions every three weeks, followed by a booster session two months later. What is the MAIN purpose of this plan?

A. To let him practice new skills on his own while support is still available ✓ KEY

Spacing sessions and adding a booster lets the client rely on himself while the counselor can still help consolidate gains and plan for setbacks.

B. To keep him from forming an unhealthy dependence in the early sessions

The plan addresses the end of counseling, not the formation of dependence early in treatment.

C. To meet an ACA requirement that each ending include follow-up sessions

The ACA Code of Ethics does not require follow-up sessions for every termination.

D. To delay the ending until he no longer reports any symptoms whatsoever

The goals are already met; waiting for a symptom-free state is not the aim of tapering.

Rationale: Gradually spacing sessions (fading) and scheduling a booster or follow-up session are common termination practices. They allow the client to test and consolidate new skills independently, give room to address relapse prevention, and make the ending less abrupt.

References: Gladding (Counseling: A Comprehensive Profession) — Termination: fading sessions and follow-up

☐ Approve ☐ Revise ☐ Retire Notes:

A client reports frequent headaches that keep her home from social events. Her Adlerian counselor asks, "If these headaches went away tomorrow, what would you be doing differently in your life?" What is the PRIMARY purpose of this question?

A. To help her picture a future with no problem, as in a miracle question

The miracle question is a solution-focused technique for building goals; the Adlerian question has a diagnostic aim.

B. To uncover a repressed conflict that is being converted into pain

Conversion of repressed conflict is a psychoanalytic formulation, not the purpose Adlerians give this question.

C. To learn whether the symptom serves a purpose, such as avoiding a task ✓ KEY

"The Question" helps the counselor see whether the symptom has a social purpose, such as excusing the client from a life task she fears.

D. To rate how much the headaches interfere with her daily functioning

Rating functional impairment is an assessment task; the question is meant to reveal the goal of the symptom.

Rationale: Adlerians view symptoms as purposeful. "The Question" (asking what the client would do if the symptom were gone) helps distinguish problems that may be largely physical from those that serve a goal, such as avoiding friendship, work or love tasks. If the answer names something the client is avoiding, the symptom may be safeguarding her from it.

References: Corey (Theory & Practice) — Adlerian Therapy: "The Question" and the purposeful nature of symptoms

☐ Approve ☐ Revise ☐ Retire Notes:

A teacher consults a school counselor about a 9-year-old who tears up classmates' artwork and makes cruel remarks to the teacher. The teacher says she feels deeply hurt and wonders how a child could treat her this way. Using Dreikurs's goals of misbehavior, which goal is the child MOST likely pursuing?

A. Attention

An attention goal typically leaves the adult feeling annoyed or irritated, not hurt.

B. Power

A power goal typically leaves the adult feeling angry, provoked or challenged.

C. Display of inadequacy

Display of inadequacy leaves the adult feeling hopeless or discouraged as the child gives up.

D. Revenge ✓ KEY

When adults feel hurt, the child is often seeking revenge, trying to hurt others as he feels hurt.

Rationale: Rudolf Dreikurs, who extended Adler's ideas to parenting and schools, described four mistaken goals of misbehavior: attention, power, revenge and display of inadequacy. A practical clue is the adult's own reaction: annoyance (attention), anger (power), hurt (revenge) and hopelessness (inadequacy). Identifying the goal helps adults stop reinforcing it and encourage the child.

References: Corey (Theory & Practice) — Adlerian Therapy: Dreikurs and the goals of misbehavior

► **Check:** Dreikurs's four goals are standard; confirm that the cited Corey edition covers them (Gladding's child/school chapters are an alternative locator).

☐ Approve ☐ Revise ☐ Retire Notes:

A client sees himself as "a calm person who doesn't get angry." When his partner says he shouted during an argument, he insists he was "just being emphatic." In Rogers's theory of personality, this BEST illustrates:

A. Reaction formation against an unacceptable id impulse of anger

Reaction formation (expressing the opposite of an impulse) is a Freudian defense rooted in id-superego conflict, not Rogers's framework.

B. Distortion of an experience that threatens the self-concept ✓ KEY

Rogers held that experiences inconsistent with the self-concept are distorted or denied to awareness, maintaining incongruence.

C. Introjection of a parent's rules, swallowed whole without question

Introjection is a gestalt contact boundary disturbance about uncritically accepting others' values.

D. Minimization, a cognitive distortion that should be disputed

Minimization is a CBT term, and person-centered counselors do not dispute thoughts.

Rationale: In Rogers's theory, when an experience (here, anger) does not fit the self-concept, the person may perceive it selectively (distortion) or keep it out of awareness entirely (denial). The gap between self-concept and experience is incongruence, which produces vulnerability and anxiety. The core conditions make it safe enough to let such experiences into awareness.

References: Corey (Theory & Practice) — Person-Centered Therapy: incongruence, distortion and denial

☐ Approve ☐ Revise ☐ Retire Notes:

A client says, "I suppose I'm kind of annoyed with my sister, but it's not a big deal," while clenching her fist. Which option BEST matches how a person-centered counselor and a gestalt counselor would each MOST likely respond?

A. Person-centered: reflect the annoyance she is starting to own; gestalt: ask her to give her fist a voice ✓ KEY

Person-centered counselors follow and empathically reflect the client's emerging feeling; gestalt counselors direct attention to present bodily awareness and often invite the client to speak as the body part.

B. Person-centered: ask her to exaggerate the clenched fist; gestalt: reflect the annoyance she now owns

The pairing is reversed: exaggeration is a gestalt experiment, and nondirective reflection is person-centered.

C. Person-centered: interpret her anger as displaced; gestalt: dispute her belief that anger is no big deal

Interpreting displacement is psychoanalytic and disputing beliefs is REBT; neither fits these two approaches.

D. Person-centered: advise her on how to talk with her sister; gestalt: explore early memories of the sister

Person-centered counselors avoid advice, and gestalt work stays with present experience rather than early memories.

Rationale: Both approaches are humanistic and value awareness, but they differ in counselor activity. A person-centered counselor stays nondirective, offering empathy so the client can more fully own her feeling. A gestalt counselor is more active, noticing nonverbal behavior and creating experiments, such as giving the fist a voice, to heighten here-and-now awareness.

References: Corey (Theory & Practice) — Person-Centered Therapy: empathic reflection and the nondirective stance · Corey (Theory & Practice) — Gestalt Therapy: attending to nonverbal behavior and experiments

☐ Approve ☐ Revise ☐ Retire Notes:

A 44-year-old client says, "I have no choice but to stay in a job I hate. My family expects it." Which counselor response is MOST consistent with existential therapy?

A. Dispute the irrational belief that he must meet his family's expectations

Disputing "must" beliefs is the core REBT technique, not the existential focus.

B. Explore how his early bond with his father shaped this need to comply

Tracing present behavior to early relationships is a psychodynamic focus.

C. Help him see that staying is a choice and explore what it means to him ✓ KEY

Existential counselors highlight freedom and responsibility, helping clients recognize that they are choosing even when they claim they have no choice.

D. Rehearse assertive statements he can use when his family raises it

Assertiveness rehearsal is a behavioral skills intervention.

Rationale: Existential therapy holds that people are free and therefore responsible for their choices. Statements such as "I have no choice" are seen as ways of avoiding that responsibility. The counselor helps the client own his choices, face the anxiety that comes with freedom, and decide how he wants to live.

References: Corey (Theory & Practice) — Existential Therapy: freedom and responsibility

☐ Approve ☐ Revise ☐ Retire Notes:

A 67-year-old client recently told his cancer is terminal says, "What's the point of anything now?" He is not suicidal. Which counselor response is MOST characteristic of an existential approach rather than a cognitive behavioral one?

A. "What evidence tells you that nothing has a point? Let's examine it together."

Testing the evidence for a thought is cognitive restructuring, a CBT method.

B. "Facing death has you asking what matters. What gives your days meaning now?" ✓ **KEY**

The existential counselor treats the question as a genuine confrontation with death and meaning and invites him to explore it.

C. "Let's schedule a few small, pleasant activities each day to lift your mood."

Scheduling pleasant activities is behavioral activation, a CBT strategy for low mood.

D. "Tell me about earlier losses in your life that this news is stirring up."

Linking present reactions to past losses reflects a psychodynamic focus.

Rationale: A CBT counselor might treat "What's the point?" as a hopeless automatic thought to test or as low mood to activate. An existential counselor sees it as an authentic encounter with the givens of death and meaninglessness and helps the client search for meaning, values and connection in the time that remains. Yalom argued that facing death honestly can deepen how one lives, and Frankl that meaning can be found even in unavoidable suffering.

References: Corey (Theory & Practice) — Existential Therapy: death, meaning and the search for meaning

☐ Approve ☐ Revise ☐ Retire Notes:

Two counselors treat clients who fear elevators. One teaches relaxation and works up a ranked list of feared scenes. The other has the client ride an elevator repeatedly for a long session, without leaving, until the anxiety subsides. Which statement accurately contrasts these approaches?

A. Flooding pairs relaxation with each step; desensitization keeps the client at peak fear

This reverses the methods: relaxation pairing belongs to systematic desensitization.

B. Both rely on reciprocal inhibition, but flooding is limited to scenes that are imagined

Flooding does not rely on a competing relaxation response, and it can be done in vivo or in imagination.

C. Desensitization blocks escape; flooding lets the client stop at any step of a hierarchy

This reverses them: flooding prevents escape, while desensitization moves gradually and lets the client pause.

D. Flooding is intense and blocks escape; desensitization is graded and paired with relaxation ✓ **KEY**

Flooding uses prolonged, high-intensity exposure with response prevention; systematic desensitization pairs relaxation with a graded hierarchy.

Rationale: Wolpe's systematic desensitization uses relaxation training, an anxiety hierarchy and gradual exposure so that relaxation inhibits anxiety. Flooding exposes the client to a highly feared stimulus for a prolonged period, in vivo or imaginal, while preventing escape, so anxiety extinguishes. Flooding often works faster but is more distressing and needs careful informed consent.

References: Corey (Theory & Practice) — Behavior Therapy: systematic desensitization, in vivo exposure and flooding

☐ Approve ☐ Revise ☐ Retire Notes:

A counselor, a 13-year-old and his parents write and sign an agreement: on each school night he finishes his homework by 8 p.m., he earns 30 minutes of video games, and his father will check the work and deliver the reward. Which behavioral technique is this?

A. Contingency contract ✓ KEY

A contingency contract is a written agreement that specifies the target behavior, the consequence and who delivers it.

B. Token economy

A token economy uses tokens or points later exchanged for backup reinforcers; no tokens are involved here.

C. Response cost

Response cost removes a reinforcer after an unwanted behavior; this plan adds a reward for a wanted one.

D. Covert sensitization

Covert sensitization pairs an unwanted behavior with an imagined aversive event.

Rationale: Contingency contracts make the "if-then" relationship between behavior and consequence explicit. Effective contracts define the behavior in observable terms, specify the reinforcer, name who monitors and delivers it, and are agreed to by everyone involved, which increases the client's sense of ownership.

References: Corey (Theory & Practice) — Behavior Therapy: operant techniques and contingency contracts

☐ Approve ☐ Revise ☐ Retire Notes:

A client says, "My boss walked right past me this morning without saying hello. I'm going to get fired." Which counselor response BEST illustrates Socratic questioning?

A. "Your boss was probably just busy, so I really doubt you will be fired."

Reassurance supplies the counselor's conclusion instead of helping the client examine the thought himself.

B. "That is catastrophizing, so try replacing it with a positive thought."

Labeling the distortion and prescribing a replacement is didactic, not guided discovery.

C. "What are some other reasons your boss might have walked past you?" ✓ KEY

The open question guides the client to generate alternative explanations and weigh the evidence for himself.

D. "How did you feel in that moment when your boss walked past you?"

Exploring the feeling is useful but does not question the accuracy of the automatic thought.

Rationale: Socratic questioning, central to Beck's guided discovery, uses open questions to help clients examine evidence, consider alternative explanations and reach their own conclusions about a thought. Counselor-supplied reassurance or labeling may be accurate but tends to be less persuasive than conclusions the client reaches.

References: Corey (Theory & Practice) — Cognitive Behavior Therapy: Beck's Socratic dialogue and guided discovery

☐ Approve ☐ Revise ☐ Retire Notes:

A client with depression says she used to love her pottery class and will start going again "once I feel more motivated." Which response BEST reflects behavioral activation?

A. Agree to wait until her mood lifts, since pushing activity now could set her up to fail

Behavioral activation holds that waiting for motivation keeps clients stuck; action is expected to come first.

B. Schedule a pottery session this week and rate her mood and mastery after it ✓ **KEY**

BA works "outside-in": the client schedules valued activity according to a plan, not a mood, and tracks its effects.

C. First identify and dispute the core belief that is behind her lack of motivation

Disputing core beliefs is cognitive restructuring; BA targets behavior and avoidance directly.

D. Explore the earlier losses that may account for her loss of interest in pottery

Exploring past losses reflects a psychodynamic focus rather than activation.

Rationale: Behavioral activation treats depression by reducing avoidance and increasing contact with rewarding, valued activities. Clients monitor activities and mood, then schedule specific activities according to a plan rather than waiting to feel like it, often rating pleasure and mastery. Motivation tends to follow action rather than precede it.

References: APA CPG — Depression guideline (2019): behavioral therapy, including behavioral activation

☐ Approve ☐ Revise ☐ Retire Notes:

On a thought record, a client has written the situation, her automatic thought ("Everyone at the party thought I was boring") and her emotion (embarrassment, 85%). What does a cognitive therapy thought record typically ask her to do NEXT?

A. Trace the thought back to the childhood memory that first produced it

Finding the childhood origin of a thought is not the next step of a thought record.

B. Repeat a positive affirmation until the thought no longer feels true

Affirmations replace the thought without examining it, which is not cognitive restructuring.

C. Describe the thought in more detail without judging if it is accurate

Noticing thoughts without evaluating them is closer to mindfulness or defusion than to a thought record.

D. Weigh the evidence and write a more balanced, alternative response ✓ **KEY**

Thought records move from identifying the thought to evaluating it and writing an adaptive response, then re-rating the emotion.

Rationale: A thought record structures cognitive restructuring: situation, automatic thought, emotion and its intensity, then evaluation of the thought (evidence for and against, alternative explanations) leading to a balanced or adaptive response, and finally a re-rating of the emotion. It teaches clients to treat thoughts as hypotheses rather than facts.

References: Corey (Theory & Practice) — Cognitive Behavior Therapy: Beck's cognitive therapy and recording automatic thoughts

☐ Approve ☐ Revise ☐ Retire Notes:

Freud called it the "fundamental rule" of psychoanalysis: the client reports whatever comes to mind, however trivial, illogical or embarrassing it seems, without censoring it. This technique is called:

A. Free association ✓ KEY

Free association asks the client to say everything that comes to mind, opening a path to unconscious material.

B. Dream analysis

Dream analysis interprets the manifest and latent content of dreams; it is a separate technique.

C. Interpretation

Interpretation is the analyst's explanation of the meaning of material, not the client's uncensored reporting.

D. Catharsis

Catharsis is the release of pent-up emotion, which may occur but is not the reporting rule itself.

Rationale: Free association is the basic method of psychoanalysis. By suspending conscious censorship, the client lets associations lead toward repressed wishes and conflicts. The analyst attends especially to blocks, pauses and changes of topic, which may signal resistance.

References: Corey (Theory & Practice) — Psychoanalytic Therapy: free association

☐ Approve ☐ Revise ☐ Retire Notes: _____

After gaining the insight that he ends relationships once they become close, a client begins arriving late and "forgets" a session. Weeks later he describes pulling away from a new partner in the same way. From a psychoanalytic perspective, which statement BEST describes what is happening and how to proceed?

A. The lateness reflects low motivation, so firm attendance rules with consequences should be set

Treating the lateness as a compliance problem misses its unconscious meaning, which is the analyst's main interest.

B. The repetition shows the insight was wrong, so the original interpretation should be dropped

Repetition after insight is expected; it does not by itself show the interpretation was wrong.

C. The lateness is resistance to be explored, and the repeated pattern calls for working through ✓ KEY

Lateness and forgetting after a painful insight are classic resistance, and insight is consolidated by working through the conflict repeatedly.

D. The relapse shows a need for catharsis, so his feelings should be discharged more fully

Emotional discharge alone was abandoned by Freud as a mechanism of lasting change.

Rationale: In psychoanalytic therapy, resistance is anything that keeps unconscious material from awareness, such as lateness, missed sessions or changing the subject, and it is interpreted rather than punished. A single insight rarely changes a pattern; working through is the repeated exploration of the same conflict as it appears in many situations and in the transference until change takes hold.

References: Corey (Theory & Practice) — Psychoanalytic Therapy: interpreting resistance and working through

☐ Approve ☐ Revise ☐ Retire Notes: _____

A 40-year-old moved across the country at 20 and has not spoken to his parents in ten years. He says, "I've put all that behind me." His father also stopped speaking to his own family as a young man, and the client's marriage now swings between intense closeness and angry distance. How would a Bowenian therapist MOST likely understand and approach this?

A. His distance shows a high level of differentiation, so therapy should focus only on the marriage

For Bowen, physical or emotional distance is not differentiation; cutoff reflects unresolved fusion with the family.

B. The family's rigid boundaries should be restructured by having it enact the conflict in session

Boundaries and enactments are structural (Minuchin) concepts, not Bowen's.

C. The marriage problems call for a paradoxical directive to have the couple fight on purpose

Paradoxical directives are a strategic technique; Bowen coaching emphasizes calm, reflective self-work.

D. Cutoff manages unresolved fusion passed down the generations; he would be coached to reconnect ✓ KEY

Emotional cutoff handles fusion by distance, and its repetition across generations reflects multigenerational transmission; Bowen coaching helps clients re-engage their families while staying differentiated.

Rationale: Bowen saw emotional cutoff as a way of managing unresolved attachment by distance, not as a sign of maturity; the fusion tends to reappear in other close relationships, such as a marriage. Multigenerational transmission explains how such patterns and levels of differentiation pass down family lines. Therapy often involves a genogram and coaching the client to make calm, person-to-person contact with family of origin.

References: Goldenberg (Family Therapy) — Bowen family systems theory: emotional cutoff and multigenerational transmission process

☐ Approve ☐ Revise ☐ Retire Notes:

In a first session with a family, a counselor lets the father speak first as the family expects, picks up the family's own phrases and pace, and follows their story. Afterward she sketches a diagram showing a diffuse boundary between the mother and son and a rigid one around the father. In structural family therapy, these two activities are BEST described as:

A. Triangulating and then detriangulating the parental subsystem

Triangulation and detriangulation are Bowen concepts about drawing in a third person, not this sequence.

B. Joining the family, then mapping its boundaries and subsystems ✓ KEY

Accommodating to the family's style and hierarchy is joining; the boundary diagram is a structural map.

C. Prescribing the symptom, then reframing the family's problem

Symptom prescription and reframing are strategic techniques, and neither occurs here.

D. Building a genogram to trace patterns across three generations

A genogram traces multigenerational patterns (Bowen); a structural map shows current boundaries and alignments.

Rationale: Minuchin's structural therapist first joins the family, accommodating to its style through maintenance, tracking and mimesis, to become accepted as a leader of the therapeutic system. Family mapping then charts current structure: subsystems, hierarchy and boundaries that are clear, diffuse (enmeshed) or rigid (disengaged), guiding later restructuring interventions.

References: Goldenberg (Family Therapy) — Structural family therapy: joining, accommodating and family mapping

☐ Approve ☐ Revise ☐ Retire Notes:

At intake, a counselor explains to a client with a history of sexual assault what each part of the interview involves and why questions are asked. She lets the client choose where to sit and whether the door stays open, and says she may skip any question. These practices BEST reflect:

A. Trauma-informed principles of safety, transparency and choice that limit retraumatization ✓ KEY

Explaining procedures, offering choices and control, and attending to physical safety are core trauma-informed practices.

B. Early exposure to trauma cues so that habituation can begin right from the first session

Nothing here exposes the client to trauma cues; the aim is to reduce, not evoke, distress at intake.

C. Analytic neutrality and abstinence so that transference can develop without interference

Neutrality and abstinence are psychoanalytic stances and do not describe sharing control with the client.

D. Standardized intake procedures that apply the very same steps to every client at the agency

Offering individual choices is the opposite of a rigid, uniform procedure.

Rationale: Trauma-informed care assumes that many clients have trauma histories and organizes services to avoid retraumatization. Practices include physical and emotional safety, transparency about what will happen, and collaboration, choice and empowerment, so that clients regain a sense of control that trauma took away.

References: SAMHSA TIP 57 — Trauma-informed care principles: safety, trustworthiness, choice and avoiding retraumatization

☐ Approve ☐ Revise ☐ Retire Notes:

A veteran with PTSD writes an impact statement about why he thinks a friend died in an ambush and how it changed his beliefs about safety, trust, power, esteem and intimacy. His therapist then helps him identify "stuck points," such as "It was my fault," and examine them with structured worksheets. Which treatment is this?

A. Prolonged exposure therapy

Prolonged exposure (Foa) centers on repeated imaginal and in vivo exposure, not written impact statements and stuck-point worksheets.

B. EMDR

EMDR (Shapiro) uses bilateral stimulation while the client holds the memory in mind, within an eight-phase protocol.

C. Cognitive processing therapy ✓ KEY

CPT (Resick) uses the impact statement, stuck points and worksheets across five themes: safety, trust, power/control, esteem and intimacy.

D. Stress inoculation training

Stress inoculation training (Meichenbaum) builds coping skills in three phases; it does not target stuck points.

Rationale: Cognitive processing therapy, developed by Patricia Resick, is a trauma-focused cognitive therapy recommended in major PTSD practice guidelines. Clients write an impact statement, identify stuck points (often self-blame or overgeneralized beliefs), and use worksheets to challenge them, with later sessions on safety, trust, power and control, esteem and intimacy.

References: APA CPG — PTSD guideline (2017): cognitive processing therapy · VA/DoD CPG — PTSD guideline: trauma-focused psychotherapies (CPT, PE, EMDR)

☐ Approve ☐ Revise ☐ Retire Notes:

According to classic crisis theory, which statement BEST describes an acute crisis?

A. A sign of underlying mental illness that calls for long-term treatment

Crisis is viewed as a normal response to an overwhelming event, not evidence of psychopathology.

B. A property of the event, so anyone facing it will be in crisis

Crisis depends on the person's perception of the event and coping resources; the same event may not cause a crisis for everyone.

C. A chronic state that lasts until the person receives counseling

Crisis states are time-limited; a person reaches some new equilibrium with or without help.

D. A time-limited state that resolves within weeks, for better or worse ✓ **KEY**

Caplan described crisis as a temporary disequilibrium that usually resolves within about four to six weeks, in an adaptive or maladaptive direction.

Rationale: Crisis theory, building on Lindemann and Caplan, holds that a crisis arises when a person perceives an event as overwhelming their usual coping. The state is time-limited, commonly cited as about four to six weeks, and resolves at a higher, equal or lower level of functioning. Because it contains both danger and opportunity, brief, prompt intervention can tip the outcome toward growth.

References: James & Gilliland (Crisis Intervention Strategies) — Approaching crisis intervention: definitions of crisis and basic crisis theory

► **Check:** The "four to six weeks" duration is the commonly taught figure from Caplan; confirm wording against the cited James & Gilliland edition.

☐ Approve ☐ Revise ☐ Retire Notes:

Hours after a house fire, a client sits staring at the floor, cannot say where she will sleep tonight, and does not respond to questions about whom she might call. Her triage assessment shows severe impairment. Using James and Gilliland's continuum of crisis worker involvement, which stance is MOST appropriate?

A. Nondirective, letting her generate her own plan so that her sense of control is not taken away

A nondirective stance fits clients who are mobile and able to act; she cannot currently generate options.

B. Directive, taking charge to assess safety, give clear guidance and arrange immediate shelter ✓ **KEY**

When a client is immobile and severely impaired, the worker becomes directive, taking temporary control to meet urgent needs.

C. Collaborative, working as an equal partner to weigh options that she suggests for the night

Collaboration suits clients with partial mobility; she is not yet able to contribute options.

D. Deferred, holding off on any action until a full psychosocial history has been completed

Crisis work addresses immediate safety and needs first; a full history is not a prerequisite for action.

Rationale: James and Gilliland describe a continuum of worker involvement matched to the client's level of mobility, as gauged by triage assessment of affective, behavioral and cognitive functioning. Immobile clients need a directive worker; partially mobile clients, a collaborative one; and mobile clients, a nondirective one. The stance shifts toward less direction as the client regains functioning.

References: James & Gilliland (Crisis Intervention Strategies) — Six-step model: directive, collaborative and nondirective involvement; Triage Assessment Form

☐ Approve ☐ Revise ☐ Retire Notes:

Partway through a session, a court-referred client crosses his arms and says, "You're just like my probation officer, always telling me what to do." Which counselor response BEST reflects how motivational interviewing handles discord?

A. "I'm not telling you what to do. I'm only giving you the facts here."

Defending oneself argues with the client and tends to increase discord.

B. "If you don't engage here, I will have to report that to your officer."

A warning or threat is confrontational and escalates the conflict in the relationship.

C. "You're tired of being told what to do. Here, the choices are yours." ✓ KEY

Reflecting his frustration and emphasizing autonomy de-escalates discord without arguing.

D. "Let's get back to your plan. What will you change before next week?"

Pushing ahead ignores the strain in the relationship and is likely to deepen it.

Rationale: In current MI, discord refers to tension in the working relationship, distinct from sustain talk about the behavior. Counselors respond without arguing: reflecting, apologizing, affirming or shifting focus, and emphasizing the client's autonomy. This replaces the older idea of "rolling with resistance" and keeps the counselor from triggering further defensiveness.

References: Miller & Rollnick (MI) — Responding to sustain talk and discord; emphasizing autonomy

☐ Approve ☐ Revise ☐ Retire Notes:

Asked to rate on a 0 to 10 scale how important it is to her to stop vaping, a 19-year-old says, "Maybe a 3." Which follow-up question is MOST likely to elicit change talk?

A. "Why a 3 and not a 1 or a 2?" ✓ KEY

Asking why the number is not lower invites her to state her own reasons for change, which is change talk.

B. "Why only a 3 and not higher?"

Asking why the number is not higher invites her to defend the status quo, eliciting sustain talk.

C. "What would get you to a 10?"

Jumping to the top of the scale is a large leap that tends to bring out reasons it is not possible.

D. "Isn't a 3 a bit low for you?"

This implies judgment and reflects the righting reflex, which tends to provoke defensiveness.

Rationale: Importance and confidence rulers are a common MI strategy for evoking change talk. The key follow-up is to ask why the client chose that number rather than a lower one, which prompts her to voice her own reasons for change. Asking why the number is not higher tends to elicit sustain talk instead.

References: Miller & Rollnick (MI) — Evoking change talk: importance and confidence rulers

☐ Approve ☐ Revise ☐ Retire Notes:

A counselor holds individual pre-group interviews for a 12-week interpersonal process group. She spends each interview asking about symptoms, history and current stressors, then tells applicants whether they have been accepted. Her supervisor says an essential element of screening is missing. Which element is it?

A. Requiring a formal DSM-5-TR diagnosis before any applicant joins

A diagnosis is not required for most counseling groups, and it says little about fit with a particular group.

B. Letting applicants ask questions and judge whether the group fits ✓ **KEY**

Screening is a two-way process: applicants learn about the group and leader and decide whether it meets their needs.

C. Ranking applicants by a personality inventory to pick the strongest group

Selection rests on fit with group goals and readiness, not on ranking applicants by a test score.

D. Asking applicants to agree to meet socially with members between sessions

Outside socializing is not encouraged in therapy groups and is not a screening goal.

Rationale: Pre-group screening selects members whose needs and goals fit the group, who will not impede the process and whose well-being will not be jeopardized by it. It is also a chance for prospective members to interview the leader, learn about the group's purpose and procedures and decide for themselves whether to join. A one-sided interview that only gathers clinical data misses that mutual decision.

References: ASGW Best Practice Guidelines — Planning: group and member preparation (screening and orientation) · ACA Code of Ethics — A.9.a Screening

☐ Approve ☐ Revise ☐ Retire Notes:

A counselor is preparing the informed consent discussion for a new adult therapy group. Which statement BEST belongs in that discussion?

A. The leader guarantees that every member will keep what is said confidential.

Leaders cannot guarantee that other members will keep confidentiality, and consent must state this limit.

B. Once you join, you are expected to attend until the final group session ends.

Members keep the right to leave; consent explains how to leave, not that leaving is barred.

C. You may feel pressure to share, but you decide what and how much to disclose. ✓ **KEY**

Naming the risk of group pressure and affirming the member's control over disclosure is core group consent content.

D. The leader will decide how much personal material each member shares weekly.

Members, not the leader, decide what they disclose; leaders protect members from coercion.

Rationale: Informed consent for a group covers its purpose, procedures and leader qualifications, the limits of confidentiality among members, and the psychological risks of participation, such as pressure to disclose, confrontation and changes that affect outside relationships. Members are told that they control their own disclosures and how to leave the group if they choose.

References: ASGW Best Practice Guidelines — Planning: group and member preparation (informed consent) · ACA Code of Ethics — A.2.b Types of Information Needed; B.4.a Group Work

☐ Approve ☐ Revise ☐ Retire Notes:

A counselor in a community clinic is planning an open-ended outpatient interpersonal therapy group for adults. Which plan for size and session length is MOST consistent with Yalom's recommendations?

A. About 3 members, meeting weekly for 30 minutes

Three members is too few for rich interaction, and 30 minutes leaves no time to warm up and work.

B. About 15 members, meeting weekly for 45 minutes

Fifteen members is too many for interaction-focused work, and 45 minutes is too short.

C. About 12 members, meeting weekly for 3 hours

Twelve is above the usual range, and sessions beyond about 2 hours tend to tire members.

D. About 7 members, weekly for 80-90 minutes ✓ **KEY**

Yalom places the ideal interactional group near 7 or 8 members, with sessions of about 80 to 90 minutes.

Rationale: Yalom suggests that an interactional therapy group works best with about 7 or 8 members, within an acceptable range of roughly 5 to 10. Sessions need time for a warm-up period and for working through themes, so most groups meet for 80 to 90 minutes; beyond about 2 hours members tend to become weary. Groups for children or for highly impaired inpatients are usually smaller and shorter.

References: Yalom & Leszcz (Group Psychotherapy) — Creation of the group: group size and duration of meetings

☐ Approve ☐ Revise ☐ Retire Notes:

Q834 of 1000 · nce-s-cou-214 · Group Counseling and Group Work · group leadership skills and therapeutic factors · medium · In review

In the sixth session of a process group, members keep switching topics whenever anyone mentions loneliness. The leader is considering a self-disclosure. Which disclosure is MOST consistent with Yalom's guidance on leader transparency?

A. "I went through a lonely divorce years ago, so let me tell you how I got through it."

A detailed personal history shifts the focus to the leader and rarely helps members with their own here-and-now work.

B. "I feel a bit shut out as we keep changing topics. I wonder what others feel now." ✓ **KEY**

A brief here-and-now disclosure of the leader's reaction models openness and invites members to examine the process.

C. "I have struggles of my own lately, so I am one more member of the group tonight."

Presenting oneself as a member blurs the leader role and burdens the group with the leader's problems.

D. "I keep my reactions private so that you can project onto me more freely here."

A blank-screen stance is not what Yalom advises; judicious transparency aids the interactional group.

Rationale: Yalom favors leader self-disclosure that is focused on the here-and-now and offered in the service of members' work, such as sharing a reaction to what is happening in the room. Lengthy disclosure of the leader's past or current personal problems tends to redirect attention, burden members and blur the leader role, while total opacity deprives the group of useful modeling.

References: Yalom & Leszcz (Group Psychotherapy) — The therapist: transparency and the use of the self

☐ Approve ☐ Revise ☐ Retire Notes:

Near the end of a co-led depression group, a member quietly says she has stockpiled pills and "probably won't be around next week." Other members look stunned. What should the leaders do?

A. Note her comment and plan to raise it with her at the start of next week's session

Waiting a week ignores an imminent risk statement; risk must be assessed immediately.

B. Ask members to share reasons to live so the group can talk her out of the plan now

Peer persuasion is not a risk assessment and puts members in a role they cannot safely fill.

C. Remove her from the group and give her a referral for individual counseling instead

Removal without assessing and managing her immediate safety abandons her at a moment of risk.

D. Acknowledge it, have one leader assess her risk privately now, and help the group process ✓ **KEY**

Her safety comes first through an immediate individual assessment, while the other leader helps members with their reactions.

Rationale: A disclosure of suicidal intent during group calls for immediate action. The leaders acknowledge what was said, one leader (or the leader right after the group) conducts a private risk assessment and safety planning, and the group is helped to process its reactions now and in later sessions. Co-leadership makes it possible to attend to the individual without abandoning the group.

References: Gladding (Counseling: A Comprehensive Profession) — Group work: handling a member in crisis; co-leadership · C-SSRS — Suicidal ideation with intent and plan: immediate risk assessment

► **Check:** Best-practice consensus rather than a single named standard; SME to confirm the Gladding locator.

☐ Approve ☐ Revise ☐ Retire Notes:

Three members of an interpersonal group have begun meeting for coffee before each session. In group they sit together, trade knowing glances and fall silent when one of them is challenged. Following Yalom, how should the leader BEST respond?

A. Ban all contact between members outside of group sessions for the rest of the group

Outside contact is not itself the problem, and a ban tends to drive it underground.

B. Invite the members to talk openly in group about their outside meetings and their effect ✓ **KEY**

Bringing the subgroup into the open turns a threat to cohesion into here-and-now material.

C. Meet privately with the three members and ask them to sit apart from each other in group

A private meeting adds secrecy and leaves the rest of the group out of the work.

D. Let the subgroup run its course, since cohesive pairs tend to strengthen the whole group

Secret subgroups whose loyalty outweighs the group's goals tend to undermine cohesion.

Rationale: Subgrouping becomes destructive when members' loyalty to the subgroup outweighs their commitment to the group's goals and when what happens outside stays secret. Yalom advises that extragroup contact be brought back into the group and discussed openly, so that it can be understood rather than forbidden.

References: Yalom & Leszcz (Group Psychotherapy) — Problem group members and subgrouping; extragroup socializing

☐ Approve ☐ Revise ☐ Retire Notes:

In a long-term group, Theo bristles at every comment from Ines, an assertive older member, and accuses her of "always looking down on me," though others see her remarks as mild. He later realizes she reminds him of his critical older sister. Drawing on Sullivan, Yalom calls Theo's distorted perception of Ines:

A. Parataxic distortion ✓ KEY

Parataxic distortion is perceiving another person through the lens of past relationships rather than as they are.

B. Consensual validation

Consensual validation is the corrective process of checking one's view against others' views, not the distortion.

C. Projective identification

This object relations term involves inducing one's projected feelings in another; Yalom uses Sullivan's term here.

D. Family recapitulation

Corrective recapitulation of the family group is a therapeutic factor, not the term for one member's distortion of another.

Rationale: Yalom borrows Sullivan's term parataxic distortion for the tendency to perceive others in the group through the template of earlier relationships, a form of transference that can occur between members as well as toward the leader. Feedback from other members, a process Sullivan called consensual validation, helps the member see the distortion and test it against reality.

References: Yalom & Leszcz (Group Psychotherapy) — Interpersonal learning: parataxic distortion and consensual validation

☐ Approve ☐ Revise ☐ Retire Notes:

A counselor wants to know whether her 10-week anxiety management group actually reduced members' symptoms. Which evaluation approach BEST answers that question?

A. Hand out a satisfaction survey at the last session of the group

Satisfaction tells whether members liked the group, not whether symptoms changed.

B. Record her own impressions of each member's progress each week

Leader impressions are subjective and open to bias; they do not measure symptom change.

C. Count how many members attended at least eight of ten sessions

Attendance shows engagement, not whether the group reduced symptoms.

D. Give a standardized anxiety measure before and after the group ✓ KEY

A valid symptom measure given at pre- and post-test directly assesses change in the target outcome.

Rationale: Outcome evaluation asks whether the group achieved its goals. Administering a reliable, valid measure of the target problem before and after the group (and, ideally, at points during it) provides evidence of change; satisfaction, attendance and leader impressions are process indicators, not outcome data.

References: ASGW Best Practice Guidelines — Performing and processing: evaluation of group outcomes · Erford (Research and Evaluation in Counseling) — Program evaluation: outcome evaluation with pretest-posttest measures

☐ Approve ☐ Revise ☐ Retire Notes:

A counselor is launching a video-based support group for caregivers who live across her state. Which step is MOST specific to the safe running of this telehealth group?

A. Ask members to keep their cameras turned off to protect each other's identities

Cameras off removes nonverbal cues and makes it harder to verify who is present and to monitor distress.

B. Skip pre-group screening, since online members can simply log off if it is a poor fit

Screening is as necessary for online groups as for in-person groups.

C. Get each member's location and emergency contact and set privacy rules for joining ✓ KEY

Knowing where members are, having emergency contacts and agreeing on privacy and no-recording rules address telehealth risks.

D. Use any free video app members already know, since familiarity improves attendance

Platforms must meet security and privacy requirements; convenience alone is not sufficient.

Rationale: Distance group work adds risks: members may join from places where others can overhear, could record sessions, and may be far from the counselor in a crisis. Counselors address these in informed consent and planning by verifying identity and location, collecting emergency contacts, using secure platforms and setting norms such as joining from a private space and not recording.

References: ACA Code of Ethics — H.2.a Informed Consent and Disclosure; H.3 Client Verification · ASGW Best Practice Guidelines — Planning: trends and technological changes

☐ Approve ☐ Revise ☐ Retire Notes:

For her first months as a hospital social worker, Amina followed every unit norm and sought her colleagues' approval. Now, secure in the role, she is pushing the team to adopt her ideas for discharge planning and asserting her own views. In Tiedeman and O'Hara's decision-making model, Amina is in which stage?

A. Crystallization

Crystallization is part of anticipation, when options are clarified before any choice is made.

B. Induction

Induction describes her earlier months, when she sought acceptance and conformed to the group.

C. Reformation ✓ KEY

In reformation the person, now accepted, acts assertively to change the group toward her own views.

D. Integration

Integration follows reformation, when individual and group goals are balanced into a stable sense of self.

Rationale: Tiedeman and O'Hara divided career decision making into anticipation (exploration, crystallization, choice and clarification) and implementation or adjustment (induction, reformation and integration). In induction the new member seeks acceptance; in reformation she becomes assertive and tries to change the group; in integration she achieves a balance between her own goals and the group's.

References: Sharf (Career Development Theory) — Tiedeman and O'Hara: anticipation and implementation stages of decision making

☐ Approve ☐ Revise ☐ Retire Notes:

A counselor uses Krumboltz's happenstance learning theory with a 30-year-old who wants to "figure out the one right career." According to this theory, how should the success of their counseling be judged?

A. By whether the client names one final occupation by the last session

The theory explicitly rejects a single career decision as the goal of counseling.

B. By the match between the client's interest code and a chosen field

Matching traits to occupations is a trait-factor goal; the theory uses assessments to spark learning.

C. By what the client accomplishes in the real world outside sessions ✓ **KEY**

Krumboltz held that success is assessed by the actions and learning the client achieves outside counseling.

D. By how certain the client reports feeling about future job choices

Certainty is not the aim; Krumboltz reframed open-mindedness and indecision as useful.

Rationale: Krumboltz's happenstance learning theory (2009) holds that the goal of career counseling is to help clients learn to take actions toward more satisfying lives, not to make a single decision. Assessments stimulate learning rather than match traits to jobs, clients learn exploratory actions that generate beneficial unplanned events, and success is judged by what the client accomplishes in the real world.

References: Sharf (Career Development Theory) — Krumboltz: happenstance learning theory and its four propositions

☐ Approve ☐ Revise ☐ Retire Notes:

A 47-year-old engineer has cut back his hours to care for his aging father, coaches his daughter's soccer team and is taking an evening course. He says work "matters less right now than it did five years ago." Which concept from Super's life-career rainbow BEST describes this?

A. Role salience, the shifting weight of concurrent life roles ✓ **KEY**

The rainbow shows several roles held at once across theaters, with their importance changing over the life span.

B. Career maturity, readiness for age-appropriate career tasks

Career maturity concerns readiness for developmental tasks, mainly in adolescence, not the balance of life roles.

C. Circumscription, the narrowing of acceptable occupations

Circumscription is Gottfredson's concept of eliminating occupations by sex type and prestige.

D. Congruence, the fit between personality and work setting

Congruence is Holland's concept of person-environment fit, not the weight of life roles.

Rationale: Super's life-career rainbow pictures the life span as arcs of roles such as child, student, leisurite, citizen, worker and homemaker, played in theaters such as home, community, school and workplace. Several roles are active at once, and their salience rises and falls over time, so a worker role can recede while caregiving, parenting and student roles grow.

References: Sharf (Career Development Theory) — Super: life-career rainbow, life roles and role salience

☐ Approve ☐ Revise ☐ Retire Notes:

A college sophomore scores very low on the Vocational Identity scale of Holland's My Vocational Situation. What does this score MOST likely indicate?

A. Her interests clash with the setting of her current campus job

This describes incongruence between person and environment, not vocational identity.

B. Her picture of her goals, interests and talents is unclear ✓ **KEY**

Holland defined vocational identity as having a clear and stable picture of one's goals, interests and talents.

C. She has a highly consistent code of adjacent types on the hexagon

Consistency describes how closely related a person's top types are, a different Holland concept.

D. She doubts her ability to finish the math courses for her major

Doubts about specific tasks reflect self-efficacy, a social cognitive career theory construct.

Rationale: Holland defined vocational identity as the possession of a clear and stable picture of one's goals, interests and talents. The My Vocational Situation measures vocational identity along with needs for occupational information and perceived barriers; low identity scores suggest a client who would benefit from clarifying self-knowledge before choosing among options.

References: Sharf (Career Development Theory) — Holland: vocational identity and the My Vocational Situation

☐ Approve ☐ Revise ☐ Retire Notes:

A counselor follows Fouad and Bingham's culturally appropriate career counseling model with a Somali American client. They have built a culturally appropriate relationship and identified her career issues. What is the NEXT step in the model?

A. Choose and apply culturally appropriate interventions

Interventions come after the impact of culture has been assessed and goals are set.

B. Set career counseling goals with the client together

Goal setting follows the assessment of cultural variables so that goals reflect them.

C. Support decision making, then plan implementation

Decision making and implementation are the final steps of the model.

D. Assess the impact of cultural variables on the issues ✓ **KEY**

After the relationship and the career issues, the model calls for assessing how cultural variables shape those issues.

Rationale: Fouad and Bingham's culturally appropriate career counseling model has seven steps: establish a culturally appropriate relationship, identify career issues, assess the impact of cultural variables on those issues, set counseling goals, make culturally appropriate interventions, support decision making, and implement and follow up. Placing cultural assessment before goals keeps goals and interventions grounded in the client's worldview.

References: Sharf (Career Development Theory) — Career counseling with culturally diverse clients: Fouad and Bingham's culturally appropriate model

► **Check:** Seven-step sequence from Fouad & Bingham (1995); SME to confirm that Sharf (6th ed.) presents the model and its step order.

☐ Approve ☐ Revise ☐ Retire Notes:

For more than a year a client has swung back and forth between staying in nursing and returning to art school, each time convincing herself the other path is right. In Pryor and Bright's chaos theory of careers, this pattern reflects which attractor?

A. Point attractor

A point attractor is movement toward a single fixed goal, not oscillation between two.

B. Torus attractor

A torus attractor is a complex but repeating routine that screens out change and chance.

C. Pendulum attractor ✓ KEY

A pendulum attractor describes swinging between two options or roles without resolution.

D. Strange attractor

The strange attractor is an open system that embraces complexity and chance, the pattern the theory favors.

Rationale: Pryor and Bright view careers as complex, dynamic systems shaped by nonlinear change and chance. They describe closed-system attractors (point, pendulum and torus) that represent attempts to control and simplify, and the strange attractor, an open-system pattern that combines stability with openness to change. A pendulum attractor is oscillation between two poles, such as two occupations.

References: Sharf (Career Development Theory) — Chaos theory of careers (Pryor and Bright): attractors

► **Check:** SME to confirm the chaos theory of careers is covered in the cited Sharf edition; attractor definitions follow Pryor & Bright (2011).

☐ Approve ☐ Revise ☐ Retire Notes:

Duc, 18, tells his counselor he will study pharmacy because his parents and grandparents decided it is best for the family, and he seems at ease with this. His family holds strongly collective social values. From the standpoint of Brown's values-based theory, how should the counselor view this choice?

A. As identity foreclosure that should be challenged so he chooses alone

Labeling a values-consistent family decision as pathology imposes an individualist norm.

B. As a sign of low career maturity needing decision-skills training

Deferring to family is consistent with collective values, not a deficit in maturity.

C. As secondary, since interests drive choice more than values do

Brown placed values, not interests, at the center of occupational choice.

D. As consistent with collective values, where family shares the choice ✓ KEY

Brown proposed that people with collective social values often make career decisions with, or in deference to, the family.

Rationale: Brown's values-based theory treats work values as central to occupational choice and recognizes that cultural values shape the decision process itself. People holding individualistic values tend to decide independently, while those with collective values tend to involve or defer to family or group. Counselors are urged to understand and work within the client's value system rather than impose an individualistic model.

References: Sharf (Career Development Theory) — Brown's values-based theory of occupational choice: cultural and work values

☐ Approve ☐ Revise ☐ Retire Notes:

A counselor is redesigning a career-choice workshop and wants to include the components that Brown and Ryan Krane's meta-analysis linked to the largest effects. Which set should she build in?

A. Written exercises, individual feedback, work information, modeling and support building ✓ **KEY**

These are the five critical ingredients Brown and Ryan Krane identified in career choice interventions.

B. Aptitude testing, computer guidance alone, job fairs, résumé drills and interview practice

Computer guidance used alone showed smaller effects, and the rest are not among the identified ingredients.

C. Group discussion, relaxation training, dream analysis, personality typing and goal rewards

These components were not among the ingredients linked to stronger career choice outcomes.

D. Interest inventories, self-scored results, labor statistics, cold calls and job shadowing

Self-interpreted inventories lack the individualized interpretation and feedback found to matter.

Rationale: Brown and Ryan Krane (2000) found five critical ingredients in effective career choice interventions: written exercises, individualized interpretation and feedback, information about the world of work, modeling opportunities, and attention to building support for choices in the client's social network. Interventions including more of these ingredients produced larger effects.

References: Brown & Ryan Krane (Career Interventions) — Five critical ingredients of effective career interventions · Sharf (Career Development Theory) — Effectiveness of career counseling: critical ingredients (Brown & Ryan Krane, 2000)

☐ Approve ☐ Revise ☐ Retire Notes:

Q848 of 1000 · nce-s-cou-228 · Human Growth and Development · human development theories (Erikson, Piaget, Kohlberg) · easy · In review

Parents of a 16-year-old worry because over one school year he has tried out different friend groups, clothing styles, music and political views. He is otherwise doing well. From Erikson's perspective, how should the counselor understand this?

A. As normal exploration within the identity vs. role confusion stage ✓ **KEY**

Erikson saw adolescent experimentation with roles and values as a healthy part of forming an identity.

B. As a delayed crisis of the industry vs. inferiority stage of school age

Industry vs. inferiority concerns mastery and competence in middle childhood, not trying out identities.

C. As an early sign of the intimacy vs. isolation crisis of young adults

Intimacy vs. isolation follows identity and centers on close committed relationships.

D. As a warning sign that he lacks a self and needs intensive treatment

Role experimentation in a well-functioning teen is expected, not a sign of disorder.

Rationale: In Erikson's fifth stage, identity vs. role confusion, adolescents try out roles, values and commitments as they work toward a coherent sense of who they are. Erikson described a psychosocial moratorium, a period of permitted experimentation, as healthy; role confusion becomes a concern only when the young person cannot form any stable sense of self.

References: Berk (Development Through the Lifespan) — Adolescence: Erikson's identity vs. role confusion

☐ Approve ☐ Revise ☐ Retire Notes:

While talking with her grandmother on the phone, a 4-year-old nods in answer to questions and holds up a new toy, saying, "Look at this one!" Which feature of Piaget's preoperational stage does this show?

A. Animistic thinking

Animism is giving lifelike qualities to objects, which is not shown here.

B. Egocentrism ✓ KEY

She assumes her grandmother sees what she sees, failing to take another's viewpoint.

C. Lack of object permanence

Object permanence develops in the sensorimotor stage and is not what this shows.

D. Failure to conserve

Conservation concerns amounts that stay the same despite changes in appearance.

Rationale: Piaget described preoperational children as egocentric: they have difficulty distinguishing others' viewpoints from their own, as in the three-mountains task. A child who nods on the phone or holds up a toy for a listener who cannot see it assumes the other person shares her perspective.

References: Berk (Development Through the Lifespan) — Piaget's preoperational stage: egocentrism and the three-mountains problem

☐ Approve ☐ Revise ☐ Retire Notes:

A woman resolves a moral dilemma by focusing on preserving relationships and avoiding harm to everyone involved. Under Kohlberg's scoring she would likely be placed at Stage 3, below the postconventional level. Which critique, associated with Carol Gilligan, addresses this?

A. Kohlberg ignored the role of logical thought in moral judgment

Kohlberg built his stages on Piaget's account of cognitive development; logic was central.

B. Kohlberg's stages describe behavior rather than moral reasoning

Kohlberg studied reasoning about dilemmas, not behavior, and Gilligan did not argue otherwise.

C. Kohlberg built a justice bias from male samples, undervaluing care ✓ KEY

Gilligan argued that a theory based on males privileged justice and rated an ethic of care as less mature.

D. Kohlberg's stages must be completed fully before anyone advances

This restates Kohlberg's invariant sequence; it is not Gilligan's critique.

Rationale: Kohlberg's stages were derived from longitudinal interviews with boys and place reasoning based on abstract justice and rights at the postconventional level. Gilligan argued that this framework undervalues an ethic of care, which emphasizes relationships and responsibility, so care-oriented reasoning is often scored at Stage 3. Later research found smaller gender differences than Gilligan proposed, but the critique broadened the study of moral reasoning.

References: Berk (Development Through the Lifespan) — Moral development: Kohlberg's theory and Gilligan's ethic of care

☐ Approve ☐ Revise ☐ Retire Notes:

Parents of a 10-year-old set many strict rules, rarely explain them beyond "because I said so," and show little warmth or responsiveness to his feelings. Which parenting style in Baumrind's framework is this, and which outcome does research MOST often link to it?

A. Permissive; impulsive behavior and poor self-control in children

Permissive parents are warm but make few demands, the opposite of this pattern.

B. Uninvolved; the poorest outcomes across nearly all measured areas

Uninvolved parents are low in both demands and warmth, but these parents are highly demanding.

C. Authoritative; high self-esteem and strong social competence

Authoritative parents pair high demands with warmth and explanation, which is missing here.

D. Authoritarian; anxiety, withdrawal and lower self-esteem ✓ KEY

High demands with low warmth and no explanation is authoritarian parenting, linked to anxious, withdrawn children.

Rationale: Baumrind classified parenting by demandingness and responsiveness. Authoritative parents are high in both; authoritarian parents are high in demands but low in warmth and explanation; permissive parents are warm but undemanding. Maccoby and Martin later added the uninvolved style, low in both. Authoritarian parenting is associated with anxiety, withdrawal, lower self-esteem and, in some children, defiance.

References: Berk (Development Through the Lifespan) — Early childhood: child-rearing styles (Baumrind)

☐ Approve ☐ Revise ☐ Retire Notes:

After a breakup, a 15-year-old tells her school counselor, "Nobody has ever hurt like this. My mom could never understand, because what I feel is completely different from anything she went through." In Elkind's account of adolescent egocentrism, this reflects:

A. The imaginary audience

The imaginary audience is the belief that everyone is watching and judging, not that one's feelings are unique.

B. The personal fable ✓ KEY

The personal fable is the belief that one's experiences and feelings are unique and that one is special.

C. Apparent hypocrisy

Apparent hypocrisy is voicing ideals without seeing that one's own behavior contradicts them.

D. Identity foreclosure

Foreclosure is Marcia's status of commitment without exploration, not an egocentric belief.

Rationale: Elkind described two forms of adolescent egocentrism. The imaginary audience is the sense of being constantly observed and evaluated by others; the personal fable is the belief that one is unique, so that no one else could understand one's feelings, and sometimes that one is invulnerable to harm. Both reflect the new capacity for formal operational thought turned back on the self.

References: Berk (Development Through the Lifespan) — Adolescence: imaginary audience and personal fable (Elkind)

☐ Approve ☐ Revise ☐ Retire Notes:

After a case presentation, a White supervisor tells a Black counseling intern, "You are so articulate. I wasn't expecting that." The supervisor means it as praise. In Sue's taxonomy of racial microaggressions, this remark is BEST classified as a:

A. Microassault

Microassaults are explicit, often deliberate attacks such as slurs, not well-meant comments.

B. Microinvalidation

Microinvalidations negate a person's experiences, as with color-blind statements.

C. Microinsult ✓ KEY

An ascription of intelligence that conveys surprise at competence is a classic microinsult.

D. Stereotype threat

Stereotype threat is a target's performance worry, not a type of microaggression.

Rationale: Sue described three forms of microaggression. Microassaults are explicit, usually conscious attacks. Microinsults are often unconscious communications that demean a person's heritage, such as expressing surprise that a person of color is articulate. Microinvalidations exclude or negate the thoughts, feelings or reality of people of color, such as claiming not to see race.

References: Sue & Sue (Counseling the Culturally Diverse) — Racial microaggressions: microassaults, microinsults and microinvalidations

☐ Approve ☐ Revise ☐ Retire Notes:

Working with a Cambodian refugee, a counselor learns the client's own terms for her distress, asks how her community explains such symptoms and frames goals within those meanings. This approach reflects which perspective?

A. An etic perspective

An etic view applies concepts assumed to be universal across cultures.

B. An emic perspective ✓ KEY

An emic view seeks to understand the client from within her own culture's concepts and meanings.

C. An assimilation stance

Assimilation is an acculturation strategy of adopting the dominant culture.

D. A universalist stance

Universalism, like the etic view, emphasizes what is common across cultures.

Rationale: An etic (universal) perspective assumes that concepts and methods apply across cultures, while an emic (culture-specific) perspective seeks to understand behavior and distress through the meanings of the client's own culture. Culturally responsive counselors balance the two, using emic understanding to avoid imposing outside frameworks.

References: Sue & Sue (Counseling the Culturally Diverse) — Etic (universal) and emic (culture-specific) perspectives

☐ Approve ☐ Revise ☐ Retire Notes:

Linh, 17, was born in the United States to Vietnamese parents. At home and at her temple she has been taught to speak Vietnamese, honor her elders and keep Tet traditions, and she embraces these norms. The process by which she has been socialized into and kept her heritage culture is called:

A. Acculturation

Acculturation refers to changes from contact with a different, often dominant, culture.

B. Assimilation

Assimilation is an acculturation strategy of adopting the dominant culture over one's heritage.

C. Marginalization

Marginalization is low connection to both the heritage and the dominant culture.

D. Enculturation ✓ KEY

Enculturation is socialization into, and retention of, the norms of one's own heritage culture.

Rationale: Enculturation is the process of being socialized into and maintaining the norms, values and practices of one's heritage culture. Acculturation is the process of change that results from contact with another culture, usually the dominant one. For second-generation clients the two processes run side by side, and each can be assessed separately.

References: Sue & Sue (Counseling the Culturally Diverse) — Acculturation and enculturation in culturally diverse clients

► **Check:** Enculturation definition follows Kim & Abreu (2001); SME to confirm the Sue & Sue locator.

☐ Approve ☐ Revise ☐ Retire Notes:

A Black woman tells her counselor that she is repeatedly passed over at her firm. Her complaint was dismissed because the firm promotes many White women and several Black men. Which framework BEST helps the counselor understand her experience?

A. An additive model, summing her separate race and gender barriers

Adding single-axis barriers misses the distinct form of exclusion created where they meet.

B. Minority stress theory, applied to her single most salient identity

Minority stress concerns chronic stress from stigma; focusing on one identity misses the overlap.

C. Crenshaw's intersectionality, in which overlapping identities interact ✓ KEY

Intersectionality explains how race and gender combine into discrimination invisible to single-axis analysis.

D. Internalized oppression, seeing her concerns as self-blame to correct

Nothing suggests self-blame; recasting systemic exclusion as internal invalidates her experience.

Rationale: Kimberlé Crenshaw coined intersectionality to explain how Black women's discrimination claims fell through the cracks when courts examined race and sex separately. Overlapping identities interact to produce distinct experiences of privilege and oppression that single-axis or additive views miss, so counselors attend to the whole person and the systems acting on her.

References: Sue & Sue (Counseling the Culturally Diverse) — Multiple identities and intersectionality · MSJCC (Ratts et al., 2016) — Intersection of identities; privileged and marginalized statuses

☐ Approve ☐ Revise ☐ Retire Notes:

In a multicultural course, a White student says, "I worked hard for everything I have, so I don't have privilege." The instructor introduces Peggy McIntosh's concept of White privilege. Which statement BEST captures that concept?

A. Unearned advantages granted by systems, often invisible to those holding them ✓ KEY

McIntosh described privilege as unearned, systemic and largely invisible to those who have it.

B. Advantages held only by White people with wealth or high-status occupations

Racial privilege operates across class lines, although class shapes it.

C. Benefits that exist only when a White person holds conscious racial prejudice

Privilege operates through systems regardless of an individual's intent or beliefs.

D. Proof that White people's achievements reflect no individual effort or merit

Privilege does not deny effort; it describes advantages that exist apart from effort.

Rationale: McIntosh likened White privilege to an invisible knapsack of unearned assets, such as being able to assume one will not be followed in a store. Privilege is conferred by social systems regardless of intent or effort and is often invisible to those who hold it. Recognizing it is part of the counselor self-awareness called for in the multicultural and social justice competencies.

References: Sue & Sue (Counseling the Culturally Diverse) — White privilege and the invisible knapsack (McIntosh) · MSJCC (Ratts et al., 2016) — Counselor self-awareness: privileged and marginalized statuses

☐ Approve ☐ Revise ☐ Retire Notes:

A counselor offers a 10-year-old a tray of sand and shelves of miniature figures, invites her to "make a world," and quietly witnesses the scene she builds. Dora Kalff developed this sandplay approach from which theoretical tradition?

A. Adlerian theory

Adlerian play therapy is Kottman's approach, built on lifestyle and mistaken beliefs.

B. Behavioral theory

Behavioral approaches target observable behavior through reinforcement, not symbolic worlds.

C. Person-centered theory

Child-centered play therapy (Axline, Landreth) is person-centered, not sandplay.

D. Jungian analytical theory ✓ KEY

Kalff built sandplay on Jung's ideas of symbols, the unconscious and the self.

Rationale: Margaret Lowenfeld created the World Technique, in which children build scenes in a sand tray with miniatures. Dora Kalff, a Swiss Jungian, adapted it into sandplay, viewing the tray as a "free and protected space" in which symbols from the unconscious emerge and the self can be expressed and healed.

References: Gladding (Counseling: A Comprehensive Profession) — Counseling children: play therapy and sandplay

► **Check:** SME to confirm Gladding locator for sandplay; facts (Lowenfeld World Technique; Kalff Jungian sandplay) are standard.

☐ Approve ☐ Revise ☐ Retire Notes:

In the Adult Attachment Interview, a mother describes a childhood with a harsh, rejecting father. She recounts painful memories in a coherent, balanced way, reflects on how they affected her and shows some understanding of her father's struggles. Her interview is MOST likely classified as:

A. Dismissing

Dismissing interviews minimize attachment and offer idealized but unsupported or sparse memories.

B. Preoccupied

Preoccupied interviews are entangled and angry or vague, with long, incoherent passages.

C. Unresolved

Unresolved interviews show lapses in reasoning when discussing loss or trauma.

D. Autonomous ✓ KEY

A coherent, reflective account of adverse experiences is classified autonomous (secure), often called earned security.

Rationale: The Adult Attachment Interview, developed by George, Kaplan and Main, classifies adults by the coherence of their narrative about childhood rather than by how positive the experiences were. A coherent, reflective account of a difficult childhood is classified autonomous (secure), sometimes called earned security. Parents' AAI classifications predict their infants' Strange Situation classifications.

References: Berk (Development Through the Lifespan) — Attachment: continuity across generations and the Adult Attachment Interview

☐ Approve ☐ Revise ☐ Retire Notes:

Harlow raised infant rhesus monkeys with two surrogate mothers: one made of bare wire that held a feeding bottle and one covered in soft cloth without food. What did the monkeys' behavior show?

A. Attachment rests mainly on contact comfort, not feeding ✓ KEY

The monkeys clung to the cloth mother and ran to her when frightened, even though the wire mother fed them.

B. Attachment forms to whichever surrogate supplies food

This drive-reduction view is what Harlow's results contradicted.

C. Infants form no attachment to either surrogate mother

The monkeys formed a clear attachment to the cloth surrogate.

D. Imprinting occurs only in a narrow window after birth

Imprinting in a critical period is Lorenz's finding with geese, not Harlow's.

Rationale: Harlow's surrogate-mother studies showed that infant monkeys spent most of their time clinging to the soft cloth mother and sought her out when frightened, visiting the wire mother only to feed. The results challenged drive-reduction views that attachment arises from feeding and supported the importance of contact comfort.

References: Berk (Development Through the Lifespan) — Ethological theory of attachment; Harlow's contact comfort studies

☐ Approve ☐ Revise ☐ Retire Notes:

A 34-year-old nurse works through a high fever because "real professionals do not take sick days," a rule her first charge nurse drilled into her. When her gestalt counselor asks whether she herself agrees with the rule, she looks puzzled and cannot say. Which contact boundary disturbance does this BEST illustrate?

A. Projection

Projection disowns parts of oneself by attributing them to others; she is not assigning her own traits to anyone.

B. Introjection ✓ KEY

She has swallowed another person's standard whole, without chewing it over to decide whether it fits her.

C. Deflection

Deflection avoids contact through vagueness, humor or changing the subject; the issue here is an unexamined rule.

D. Retroflection

Retroflection turns onto oneself what one would like to do to others, such as self-blame in place of anger.

Rationale: Gestalt therapy describes disturbances at the contact boundary between person and environment. In introjection a person uncritically accepts the beliefs and standards of others without assimilating them, so the rules feel like "shoulds" that are not truly the person's own. Gestalt work helps the client become aware of introjects and decide which to keep and which to reject.

References: Corey (Theory & Practice) — Gestalt Therapy: contact and resistances to contact (introjection)

☐ Approve ☐ Revise ☐ Retire Notes:

While describing how her brother dismissed her at a family dinner, a client keeps making a small brushing-away motion with one hand. Her gestalt counselor asks her to make the motion larger and larger and then to give the hand a voice. Which technique is the counselor using?

A. Empty-chair dialogue

The empty chair has the client speak to an imagined person or part of the self, not amplify a gesture.

B. Making the rounds

Making the rounds is a group exercise in which a member speaks to or does something with each other member.

C. Reversal technique

Reversal asks the client to play the opposite of how they usually present, such as a shy client acting bold.

D. Exaggeration exercise ✓ KEY

Exaggeration amplifies a small movement or gesture so the client becomes more aware of the feeling it carries.

Rationale: Gestalt counselors attend closely to nonverbal behavior as a clue to feelings outside awareness. In the exaggeration exercise the client repeats and enlarges a movement, posture or phrase, which intensifies the feeling attached to it and helps the client own what the body is expressing.

References: Corey (Theory & Practice) — Gestalt Therapy: techniques (the exaggeration exercise)

☐ Approve ☐ Revise ☐ Retire Notes:

Midway through a session, a client says, "I'm stuck. I don't know what I feel, and I need you to tell me what to do," and then falls silent. Her gestalt counselor sees this as the impasse layer that Perls described. What would the counselor MOST likely do next?

A. Invite the client to stay with and fully experience the stuck feeling ✓ KEY

Gestalt counselors help clients remain with the impasse so they can work through it, rather than rescuing them from it.

B. Offer concrete advice so the client can move past the stuck point

Advice would accept the client's bid for outside support and block her from finding her own way through.

C. Teach a relaxation exercise to reduce the client's discomfort quickly

Lowering the discomfort right away would help her escape the impasse, not experience and move through it.

D. Interpret the silence as resistance rooted in an early childhood conflict

Interpreting the past is a psychoanalytic move; gestalt work stays with present awareness.

Rationale: Perls described layers of neurosis (phony, phobic, impasse, implosive and explosive). At the impasse, clients feel stuck and try to get the environment, including the counselor, to provide direction and support. The gestalt counselor resists rescuing and encourages the client to stay with the stuckness, because fully experiencing it allows the client to find inner support and move through it.

References: Corey (Theory & Practice) — Gestalt Therapy: layers of neurosis (the impasse)

☐ Approve ☐ Revise ☐ Retire Notes:

A retired bus driver tells his counselor, "I can't stand waiting at the pharmacy. Ten minutes in that line is unbearable, so I just leave without my medication." In Ellis's REBT, which irrational derivative BEST describes his belief?

A. Personalization

Personalization is a Beck distortion in which a person takes undue responsibility for outside events.

B. Global self-rating

Global rating condemns a whole person, as in "I am worthless"; he is not rating himself.

C. Low frustration tolerance ✓ KEY

Insisting that discomfort is unbearable ("I can't stand it") is Ellis's low frustration tolerance.

D. Overgeneralization

Overgeneralization draws a sweeping rule from one event; his belief concerns tolerating discomfort.

Rationale: Ellis held that rigid demands produce derivative irrational beliefs: awfulizing, low frustration tolerance ("I can't stand it") and global rating of self or others. Low frustration tolerance leads people to avoid short-term discomfort at the cost of long-term goals, as when this client leaves without his medication. REBT disputes the belief that discomfort cannot be borne.

References: Corey (Theory & Practice) — Rational Emotive Behavior Therapy: irrational beliefs and low frustration tolerance

☐ Approve ☐ Revise ☐ Retire Notes:

A client insists that his partner must agree with him on every major decision. His REBT counselor asks, "How does it follow that because you would strongly prefer her agreement, she therefore must agree?" Which type of disputing is this?

A. Empirical disputing

Empirical disputing asks for evidence, such as "Where is the proof that she must agree?"

B. Functional disputing

Functional disputing asks whether the belief helps, such as "Where does this demand get you?"

C. Logical disputing ✓ KEY

Logical disputing shows that a demand does not follow from a preference, however strong the preference is.

D. Philosophical disputing

Philosophical disputing asks whether life can still be satisfying even if the wish is not met.

Rationale: In the D step of the ABCDE model, REBT counselors challenge irrational beliefs in several ways. Logical disputes expose the leap from "I want" to "it must be"; empirical disputes test the belief against evidence; functional disputes question whether the belief serves the client's goals; philosophical disputes broaden the client's view of a satisfying life.

References: Corey (Theory & Practice) — Rational Emotive Behavior Therapy: disputing irrational beliefs (logical, empirical, functional, philosophical)

☐ Approve ☐ Revise ☐ Retire Notes:

A 26-year-old graduate student with panic attacks tells her REBT counselor, "Now I hate myself for being so weak that I panic at all." Following Ellis, what would the counselor typically address FIRST?

A. Her self-damning beliefs about having panic, before the panic itself ✓ KEY

Ellis often targets the secondary disturbance first, because self-condemnation about a symptom blocks work on it.

B. Graded exposure to the bodily cues of panic, before any self-anger

Exposure may come later; Ellis viewed the disturbance about the disturbance as the first obstacle.

C. The childhood experiences that may first have produced the panic

REBT focuses on present beliefs rather than on tracing the historical origins of symptoms.

D. Reassurance that panic is not dangerous and will fade with time

Reassurance does not dispute the demand that she must not be weak, so the self-damning belief remains.

Rationale: REBT distinguishes primary disturbance (here, the panic) from secondary disturbance, the "disturbance about the disturbance" such as hating oneself for panicking. Ellis advised that counselors often tackle the secondary problem first, because self-condemnation adds suffering and interferes with work on the primary problem. Unconditional self-acceptance helps the client view the symptom as a problem to solve rather than proof of worthlessness.

References: Ellis & Dryden (REBT) — Secondary disturbance (disturbance about disturbance) and addressing it first · Corey (Theory & Practice) — Rational Emotive Behavior Therapy: secondary disturbance (disturbance about disturbance)

☐ Approve ☐ Revise ☐ Retire Notes:

At the end of a first session, a counselor tells a couple who fight about chores, "Between now and the next time we meet, notice what happens in your relationship that you want to keep happening." In de Shazer's approach, this assignment is known as:

A. A paradoxical directive

A paradoxical directive prescribes the symptom; this task asks the couple to notice what already works.

B. An externalizing question

Externalizing separates the person from the problem in narrative therapy; this is an observation task.

C. A skills-rehearsal assignment

Nothing new is being practiced; the couple is asked only to notice existing positive events.

D. The formula first session task ✓ KEY

De Shazer's formula first session task asks clients to observe what they want to continue.

Rationale: De Shazer and colleagues developed the formula first session task as a standard assignment that shifts attention from problems to what is already going well. Because clients notice and report on positive events, the next session can build on existing strengths and exceptions rather than on a problem history.

References: Corey (Theory & Practice) — Solution-Focused Brief Therapy: formula first session task

☐ Approve ☐ Revise ☐ Retire Notes:

A counselor opens a second session by asking, "What's been better since we met?" A woman caring for her father with Parkinson's disease says there was one calm evening at home. Which response is MOST consistent with solution-focused practice?

A. "That's good. Now, what has been hardest for you this past week?"

Moving straight to what was hardest drops the exception before it can be explored and built on.

B. "What did you do that helped make that evening calmer than usual?" ✓ KEY

Asking how she made it happen amplifies the exception and credits her with the change.

C. "One calm evening is a start. What usually ruins the other evenings?"

Asking what ruins evenings returns the focus to problem talk rather than to what worked.

D. "Let's look at what stops calm evenings from happening more often."

Analyzing barriers is problem-focused; SFBT studies the exception to find what to do more of.

Rationale: Solution-focused counselors open later sessions by asking what is better and then elicit, amplify and reinforce any improvement (sometimes summarized as EARS). Asking in detail how the client made the better time happen highlights her own resources and points to what she can do more of. Problem analysis is set aside in favor of building on exceptions.

References: Corey (Theory & Practice) — Solution-Focused Brief Therapy: exceptions and building on what works in later sessions

☐ Approve ☐ Revise ☐ Retire Notes:

Solution-focused brief therapy was developed mainly by which pair of clinicians at the Brief Family Therapy Center in Milwaukee?

A. Steve de Shazer and Insoo Kim Berg ✓ KEY

De Shazer and Berg developed SFBT at the Brief Family Therapy Center in Milwaukee.

B. Michael White and David Epston

White and Epston developed narrative therapy in Australia and New Zealand.

C. Jay Haley and Cloé Madanes

Haley and Madanes are associated with strategic family therapy.

D. Paul Watzlawick and John Weakland

Watzlawick and Weakland worked at the Mental Research Institute in Palo Alto.

Rationale: Steve de Shazer and Insoo Kim Berg developed solution-focused brief therapy at the Brief Family Therapy Center in Milwaukee, drawing on the MRI brief therapy tradition but shifting attention from problems to solutions. Its hallmark techniques include the miracle question, exception-finding and scaling questions.

References: Corey (Theory & Practice) — Solution-Focused Brief Therapy: key figures (de Shazer and Berg)

☐ Approve ☐ Revise ☐ Retire Notes:

A narrative counselor first asks a 15-year-old, "How much of your school week has the Panic taken over?" Later the counselor asks, "When have you had some influence over the Panic, even a little?" This paired line of questioning is known as:

A. Scaling questioning

Scaling questions ask for a 0 to 10 rating and come from solution-focused therapy.

B. Circular questioning

Circular questions, from the Milan team, ask family members about relationships and differences among them.

C. The miracle question

The miracle question asks clients to imagine life after the problem is gone overnight.

D. Relative influence questioning ✓ KEY

Mapping the problem's influence on the person, then the person's influence on the problem, is relative influence questioning.

Rationale: White and Epston described relative influence questioning in two parts. The counselor first maps the influence of the externalized problem on the person's life and relationships, then maps the person's influence over the problem. The second set of questions brings forward unique outcomes that can anchor a preferred story.

References: Corey (Theory & Practice) — Narrative Therapy: externalizing conversations and relative influence questioning

☐ Approve ☐ Revise ☐ Retire Notes:

A 70-year-old widower says his life feels empty since his wife died. His narrative counselor asks who belongs in his "club of life," what his late wife would say she valued in him, and whether some members whose voices put him down should have less say. This practice is called:

A. Outsider-witness retelling

Outsider witnesses are invited audiences who listen and retell; no audience is present here.

B. Re-membering conversation ✓ KEY

Re-membering reviews and revises the membership of the client's club of life, upgrading some voices and downgrading others.

C. Deconstructive listening

Deconstructive listening attends to gaps and ambiguities in a story; it does not review life's membership.

D. Scaffolding conversation

Scaffolding moves clients in small steps from the familiar toward new ideas, not toward reviewing relationships.

Rationale: Drawing on Barbara Myerhoff, Michael White described identity as shaped by an "association" or club of life whose members include living and dead, present and absent figures. In re-membering conversations, the counselor helps the client honor members who support a preferred identity, such as a deceased spouse, and revoke or reduce the membership of those whose voices are unhelpful.

References: White (Maps of Narrative Practice) — Ch. 3, re-membering conversations and the club of life · Corey (Theory & Practice) — Narrative Therapy: re-authoring and re-membering conversations

☐ Approve ☐ Revise ☐ Retire Notes:

A counseling student is asked to describe the counselor's position in narrative therapy as Michael White described it. Which description is MOST accurate?

A. An expert interpreter who uncovers the hidden meaning in the client's story

Narrative counselors avoid the expert-interpreter role; clients are the experts on their own lives.

B. A neutral observer who avoids any influence over the story being told

Narrative counselors do shape the conversation through their questions; they are not neutral observers.

C. Decentered but influential, treating the client as the expert on their life ✓ KEY

White described the counselor as decentered, with the client's knowledge central, yet influential through questioning.

D. A directive strategist who assigns tasks designed to disrupt the problem

Assigning disruptive tasks is characteristic of strategic therapy rather than narrative practice.

Rationale: Michael White described the ideal narrative stance as "decentered and influential." The client's own knowledge and skills are at the center, and the counselor does not impose expert interpretations; at the same time, the counselor shapes the conversation through careful questions that help the client externalize problems and build a richer preferred story.

References: Corey (Theory & Practice) — Narrative Therapy: the therapist's role and the client as expert

☐ Approve ☐ Revise ☐ Retire Notes:

A 41-year-old Latina office manager was told by human resources that she is "too emotional" about repeated sexual comments from a supervisor. Her feminist counselor describes her anger and poor sleep as understandable responses to an unjust situation rather than signs of personal pathology. Which intervention is the counselor using?

A. Gender-role analysis

Gender-role analysis explores the gender messages a client absorbed and their effects; this changes how symptoms are framed.

B. Reframing and relabeling ✓ KEY

Feminist reframing moves the problem from the client's defects to social and environmental conditions.

C. Bibliotherapy

Bibliotherapy uses readings to educate the client; no reading is assigned in this scenario.

D. Assertiveness training

Assertiveness training builds skills for stating needs; the counselor is reinterpreting her reactions.

Rationale: In feminist therapy, reframing and relabeling shift the focus from "blaming the victim" toward the social and political factors that contribute to a client's distress. Symptoms such as anger or poor sleep may be understood as coping or survival responses to oppression or harassment, which reduces self-blame and can open the way to change and social action.

References: Corey (Theory & Practice) — Feminist Therapy: techniques (reframing and relabeling)

☐ Approve ☐ Revise ☐ Retire Notes:

A client's insurer requires a diagnosis before it will pay for counseling. Her counselor practices from a feminist perspective. Which approach to DSM-5-TR diagnosis is MOST consistent with feminist therapy?

A. Use diagnosis cautiously and collaboratively, weighing social context and bias ✓ KEY

Feminist counselors share the process with clients and consider how oppression and bias shape symptoms.

B. Refuse to diagnose, since any label is a tool of patriarchal social control

Some feminist counselors decline to diagnose, but most use it with caution and collaboration.

C. Diagnose as any counselor would, since a diagnosis is a neutral clinical fact

Feminist therapy questions the neutrality of diagnosis and its potential for gender and cultural bias.

D. Diagnose only after the client has finished a complete gender-role analysis

No such rule exists; the issue is how diagnosis is used, not which technique must come first.

Rationale: Feminist therapists are wary of diagnostic labels that locate problems inside the individual and that may reflect gender, cultural or other bias. When a diagnosis is needed, many feminist counselors discuss it openly with the client, decide on it together and consider how social context, oppression and trauma contribute to symptoms. This fits the approach's egalitarian relationship and "personal is political" principle.

References: Corey (Theory & Practice) — Feminist Therapy: the role of assessment and diagnosis

☐ Approve ☐ Revise ☐ Retire Notes:

A DBT skills trainer describes a state of mind that integrates "emotion mind" and "reasonable mind," allowing a person to sense what is true and act from inner balance. What is this state called?

A. Beginner's mind

Beginner's mind refers to approaching each moment freshly; it is not the synthesis of the two states.

B. Radical acceptance

Radical acceptance is a distress tolerance skill for accepting reality fully, not a state of mind.

C. Self-soothing

Self-soothing is a distress tolerance skill using the five senses to comfort oneself.

D. Wise mind ✓ KEY

Wise mind is the integration of emotion mind and reasonable mind taught in the mindfulness module.

Rationale: Linehan's mindfulness module describes three states of mind. Emotion mind is driven by feelings, reasonable mind by logic and facts, and wise mind integrates both with intuitive knowing. The "what" and "how" mindfulness skills help clients reach wise mind before acting.

References: Linehan (DBT Skills Training Manual) — Mindfulness skills: wise mind (emotion mind and reasonable mind)

☐ Approve ☐ Revise ☐ Retire Notes:

After a minor slip of the tongue at a party, a client feels intense shame and wants to hide in her room all weekend. She and her DBT therapist agree that the shame does not fit the facts. Which emotion regulation skill would the therapist MOST likely coach?

A. Self-soothing with the five senses in her room

Self-soothing is a distress tolerance skill and would not counter the urge to hide.

B. Radical acceptance of the mistake she made

Radical acceptance is a distress tolerance skill; it does not change an unjustified emotion.

C. Opposite action, going out to see her friends ✓ KEY

When an emotion does not fit the facts, acting opposite to its urge (hiding) helps reduce it.

D. DEAR MAN, asking her friends for reassurance

DEAR MAN is an interpersonal effectiveness skill for making requests, not for changing shame.

Rationale: Opposite action is a core emotion regulation skill in DBT. When an emotion and its intensity do not fit the facts, the client acts fully opposite to the action urge; for unjustified shame, that means approaching and engaging rather than hiding. Repeated opposite action reduces the emotion over time.

References: Linehan (DBT Skills Training Manual) — Emotion regulation skills: checking the facts and opposite action

☐ Approve ☐ Revise ☐ Retire Notes:

A client's DBT diary card shows that this week she cut herself once, skipped her skills group twice and had a painful argument with her sister. According to the DBT target hierarchy for individual sessions, which issue should the therapist address FIRST?

A. The self-harm, as a life-threatening behavior ✓ KEY

Life-threatening behaviors, including self-harm, are the top priority in individual DBT sessions.

B. The missed group, as therapy-interfering behavior

Therapy-interfering behavior is the second target, addressed after life-threatening behavior.

C. The argument, as a quality-of-life interfering issue

Quality-of-life issues come third in the hierarchy, after the two higher targets.

D. Whichever topic the client most wishes to discuss

DBT uses the diary card to set the agenda by target priority, not by client preference alone.

Rationale: In stage 1 of DBT, individual therapy follows a target hierarchy: first life-threatening behaviors such as suicidal acts and self-harm, then therapy-interfering behaviors such as missed sessions, then quality-of-life interfering behaviors, and then increasing behavioral skills. The diary card shows which targets occurred during the week and guides the session agenda.

References: Linehan (DBT Skills Training Manual) — Overview of DBT: stage 1 target hierarchy and diary cards · Corey (Theory & Practice) — Dialectical behavior therapy: treatment targets

☐ Approve ☐ Revise ☐ Retire Notes:

In a child-centered play therapy session, a 4-year-old silently drives a toy truck into a tower of blocks and watches it fall. The counselor says, "You're driving that right into there, and down it goes." This response is BEST described as:

A. Interpretation

Interpretation assigns meaning to the play, such as anger at a sibling; the counselor only describes it.

B. Tracking ✓ KEY

Tracking describes what the child is doing, conveying the counselor's full attention without judgment.

C. Limit setting

Knocking down blocks harms no one, and the counselor sets no limit.

D. Esteem building

Esteem-building responses credit the child's effort or ability, such as "You figured that out."

Rationale: Child-centered play therapists use tracking to describe the child's play behavior aloud, often without naming toys the child has not named. Tracking shows the child that the counselor is present and interested, and it lets the child lead. Interpretations and evaluations are avoided because they shift direction away from the child.

References: Gladding (Counseling: A Comprehensive Profession) — Counseling children: child-centered play therapy responses (tracking)

☐ Approve ☐ Revise ☐ Retire Notes:

A counselor about to begin child-centered play therapy with a 7-year-old wonders whether to read a list of playroom rules in the first few minutes. Following Landreth's guidance, what is the BEST approach to limits?

- A. Review all of the playroom rules at the start so the child knows what to expect
A list of rules at the outset can inhibit play and suggests that the counselor expects misbehavior.
- B. Ask the parent to explain the rules before the child enters the playroom
Limits belong in the child-counselor relationship and are set by the counselor when needed.
- C. Post the written rules on the wall and point to them when problems arise
Written rules do not fit a young child's needs and replace the relationship with an impersonal list.
- D. Set each limit only when it is needed, as situations arise during the play ✓ KEY
Landreth advises that limits are not needed until they are needed, so they are set when they arise.

Rationale: Landreth recommends against opening play therapy with a list of rules, which can make a child cautious and signal distrust. The counselor offers a general, permissive introduction and sets specific limits, such as not hurting oneself or the counselor, only when a situation calls for them, using the ACT steps.

References: Gladding (Counseling: A Comprehensive Profession) — Counseling children: child-centered play therapy and limit setting

☐ Approve ☐ Revise ☐ Retire Notes:

A parent asks why the counselor plans to use play therapy with her 5-year-old instead of talking through the child's fears about a new baby in the family. Which explanation BEST reflects developmental reasoning?

- A. Play keeps the child busy so the counselor can watch for any hidden symptoms
Play is the medium of therapy itself, not a distraction that allows observation.
- B. Children this age find talk boring, so playing is a reward for coming each week
Play is not used as a reward; it matches how young children process and express experience.
- C. Young children lack abstract verbal reasoning, so play is their natural language ✓ KEY
Preoperational children express feelings and work through experience in play more readily than in words.
- D. Play lets the counselor interpret unconscious drives without the child's input
Modern play therapy follows the child's lead; it is not a way to bypass the child to interpret drives.

Rationale: In Piaget's preoperational stage (about ages 2 to 7), children think symbolically but lack the abstract reasoning and vocabulary needed to describe complex feelings. Play is their natural means of expression; as Landreth put it, toys are children's words and play is their language. Play therapy therefore meets young children at their developmental level.

References: Berk (Development Through the Lifespan) — Early childhood: Piaget's preoperational stage and make-believe play · Gladding (Counseling: A Comprehensive Profession) — Counseling children: rationale for play therapy

☐ Approve ☐ Revise ☐ Retire Notes:

For several weeks a psychodynamic counselor has noticed that a client's rage at his wife's mild comments seems to echo his anger at his critical mother. When is an interpretation of this link MOST likely to be useful?

A. When the material is close to awareness and the client can tolerate hearing it ✓ KEY

Interpretations work best when the client is nearly ready to see the link and can absorb it.

B. As early as possible, so the client gains insight before his defenses harden

Premature interpretations tend to raise anxiety and resistance, and the client may reject them.

C. Only after the client asks the counselor directly to explain what is happening

Timing depends on the client's readiness, not on whether the client explicitly requests an interpretation.

D. Once the counselor is certain of the link, whatever the client's current readiness

Counselor certainty does not make an interpretation usable; the client's readiness is what matters.

Rationale: In psychoanalytic practice, interpretation explains the meaning of behavior, dreams, resistance and transference. Corey notes that timing matters: interpretations should be offered when the phenomenon is close to conscious awareness, should start from the surface and move only as deep as the client can go emotionally, and should follow a sound working relationship.

References: Corey (Theory & Practice) — Psychoanalytic Therapy: interpretation and its timing

☐ Approve ☐ Revise ☐ Retire Notes:

A doctoral student loves her evening run but keeps putting off writing her dissertation. Her counselor suggests that she go running only after she has written for one full hour. Which behavioral principle is the counselor applying?

A. Shaping

Shaping reinforces successive approximations of a target behavior, not one activity used as a reward for another.

B. The Premack principle ✓ KEY

A high-frequency activity (running) is made contingent on a low-frequency one (writing) to reinforce it.

C. Negative reinforcement

Negative reinforcement removes an aversive stimulus; here a preferred activity is added after writing.

D. Stimulus generalization

Generalization is responding to stimuli similar to the original; it is not a reinforcement plan.

Rationale: The Premack principle, sometimes called "Grandma's rule," holds that a more probable behavior can reinforce a less probable one when access to it depends on first doing the less probable behavior. Counselors use it in self-management plans by making a favorite activity contingent on completing an avoided task.

References: Corey (Theory & Practice) — Behavior Therapy: operant techniques and self-management (Premack principle)

☐ Approve ☐ Revise ☐ Retire Notes:

In a family session, a quiet 16-year-old's complaints are repeatedly overridden by her father and older brother, who form a strong coalition. A structural family therapist deliberately sides with the teenager for a period, backing her view until the family's hierarchy begins to shift. This technique is:

A. Detriangulation

Detriangulation is a Bowenian goal of removing oneself from a triangle, not siding with a member.

B. Reframing

Reframing gives a new meaning to a behavior; the therapist here is changing alliances, not meanings.

C. Enactment

An enactment has the family act out a transaction in session; it does not involve taking a side.

D. Unbalancing ✓ KEY

Minuchin's unbalancing has the therapist temporarily join one member to change the family's power structure.

Rationale: In structural family therapy, Minuchin used unbalancing to challenge a rigid hierarchy or coalition. By temporarily affiliating with one family member, often the less powerful one, the therapist disrupts the usual distribution of power and forces the family to find new ways of relating. The therapist must later restore balance and avoid becoming fixed in a coalition.

References: Goldenberg (Family Therapy) — Structural family therapy: restructuring techniques (unbalancing)

☐ Approve ☐ Revise ☐ Retire Notes:

A shy 23-year-old says he wishes he were the kind of person who could start conversations with coworkers. His Adlerian counselor asks him, for one week, to behave as though he already were that person. Which technique is this?

A. Spitting in the soup

Spitting in the soup names the hidden payoff of a behavior to make it less appealing.

B. Catching oneself

Catching oneself means noticing an old pattern as it starts; it does not ask for new role behavior.

C. Acting as if ✓ KEY

Acting as if invites the client to try on the role of the person he wishes to be.

D. Push-button technique

The push-button technique has clients alternate pleasant and unpleasant memories to see that they create feelings.

Rationale: Adlerian counselors use "acting as if" to help clients test new behavior that they believe is beyond them. Like an actor trying on a role, the client behaves for a set time as though he already had the desired quality, which can challenge mistaken beliefs about his limits and build courage.

References: Corey (Theory & Practice) — Adlerian Therapy: reorientation techniques (acting as if)

☐ Approve ☐ Revise ☐ Retire Notes:

Rogers described accurate empathic understanding as sensing the client's private world "as if" it were the counselor's own. What does the "as if" quality emphasize?

A. The counselor appears to share the client's feelings in order to build rapport faster

Rogers did not mean pretending; genuine empathy requires congruence, not an act.

B. The counselor enters the client's world while keeping a separate sense of self ✓ **KEY**

The counselor feels the client's world deeply without losing the "as if" and merging with it.

C. The counselor imagines how they themselves would feel in the client's situation

Imagining one's own reaction projects the counselor's frame onto the client instead of grasping the client's.

D. The counselor treats the client's account as a hypothesis to be checked against facts

Testing the client's account against facts is a cognitive stance, not person-centered empathy.

Rationale: For Rogers, accurate empathy means sensing the client's feelings and meanings as if they were one's own, but without ever losing the "as if" quality. The counselor stays a separate person, which keeps the counselor from being overwhelmed or confusing personal reactions with the client's experience, while still communicating deep understanding.

References: Corey (Theory & Practice) — Person-Centered Therapy: accurate empathic understanding

☐ Approve ☐ Revise ☐ Retire Notes:

An existential counselor notices that a 52-year-old engineer talks easily about his job and his duties to relatives, but goes blank when asked about his own values, feelings and sense of who he is. Which mode of being-in-the-world appears MOST neglected?

A. Eigenwelt (own world) ✓ **KEY**

The Eigenwelt is one's relationship with oneself, including values, feelings and self-awareness.

B. Umwelt (natural world)

The Umwelt is the physical and biological world; nothing suggests he neglects his body or surroundings.

C. Mitwelt (social world)

The Mitwelt is the world of relationships with others, which he engages in readily.

D. Überwelt (spiritual world)

The Überwelt concerns ideals and beliefs about the world; the clearest gap is his relation to himself.

Rationale: Existential therapists, building on Binswanger and May, describe being-in-the-world in several modes: the Umwelt (natural or physical world), the Mitwelt (world of relationships), the Eigenwelt (relationship with oneself) and, in van Deurzen's addition, the Überwelt (spiritual or ideal world). A person can be engaged in some modes while neglecting others, and therapy explores the neglected dimension.

References: Corey (Theory & Practice) — Existential Therapy: modes of being in the world (Umwelt, Mitwelt, Eigenwelt, Überwelt)

☐ Approve ☐ Revise ☐ Retire Notes:

A client says, "If I make a mistake in the meeting, people will notice." The counselor asks, "And if they notice, what would that mean about you?" The counselor repeats this kind of question after each answer until the client says, "That I'm incompetent." Which technique is this?

A. Decatastrophizing

Decatastrophizing examines how likely and how bad a feared outcome is, rather than its meaning about the self.

B. Thought stopping

Thought stopping interrupts unwanted thoughts; it does not explore what lies beneath them.

C. Cognitive rehearsal

Cognitive rehearsal mentally practices coping with a future situation step by step.

D. Downward arrow technique ✓ KEY

Repeatedly asking what a thought would mean moves from automatic thoughts down to a core belief.

Rationale: In cognitive therapy, the downward arrow technique uncovers intermediate and core beliefs that lie beneath automatic thoughts. The counselor repeatedly asks what the thought would mean if it were true, until the client reaches a basic belief about the self, such as "I'm incompetent," which can then become a target for change.

References: J. Beck (CBT: Basics and Beyond) — Identifying intermediate and core beliefs: the downward arrow technique · Corey (Theory & Practice) — Cognitive Behavior Therapy: Beck's cognitive therapy, identifying core beliefs

☐ Approve ☐ Revise ☐ Retire Notes:

A father admits that he nags, criticizes and threatens his 14-year-old about grades, and their relationship has grown cold. In Glasser's choice theory, what would a reality therapist encourage him to use in place of these "deadly habits"?

A. Consistent consequences set out in a written behavior contract

Consequences used to force compliance still rely on external control, which Glasser rejected.

B. Praise and tangible rewards delivered right after good grades

Rewarding to control, or bribing, is itself one of Glasser's seven deadly habits.

C. Caring habits such as listening, supporting and encouraging ✓ KEY

Glasser's caring habits build the relationship that external control damages.

D. Firmer limits paired with calm, logical explanation of rules

Firm limits imposed to control the teen do not replace control with connection.

Rationale: Glasser contrasted external control psychology with choice theory. His seven deadly habits (criticizing, blaming, complaining, nagging, threatening, punishing and bribing or rewarding to control) damage relationships, while the seven caring habits (supporting, encouraging, listening, accepting, trusting, respecting and negotiating differences) strengthen the connection people need to meet their basic needs.

References: Corey (Theory & Practice) — Reality Therapy: choice theory, external control and caring versus deadly habits

☐ Approve ☐ Revise ☐ Retire Notes:

In a primary care clinic, a 58-year-old client asks a counselor, "Is it really that bad to have four beers every night?" Which response BEST follows the elicit-provide-elicit approach used in motivational interviewing?

A. "What have you heard about it? I can share what we know, then hear your view." ✓ KEY

This asks what the client knows, offers information with consent, and plans to ask for the client's reaction.

B. "Yes, that is well above safe limits, so you should cut back to two at most."

Giving unsolicited advice and a directive reflects the righting reflex and invites sustain talk.

C. "Why do you think you need four beers a night just to unwind after work?"

A "why" question about his need can sound judgmental and draws reasons for drinking.

D. "Let's hold that question and first talk about how your week has gone."

Deflecting the question misses a natural chance to exchange information.

Rationale: Motivational interviewing uses elicit-provide-elicit to share information in a way that respects autonomy. The counselor first asks what the client already knows or wants to know, then offers information with permission and in a neutral way, and then asks what the client makes of it. This avoids the righting reflex and keeps the client active in the conversation.

References: Miller & Rollnick (MI) — Exchanging information: elicit-provide-elicit

☐ Approve ☐ Revise ☐ Retire Notes:

A 30-year-old with PTSD and alcohol use disorder is in early recovery and becomes overwhelmed when trauma memories come up. Her counselor begins Seeking Safety, the model developed by Lisa Najavits. Which feature BEST describes this model?

A. It requires a detailed trauma narrative before coping skills are taught

Seeking Safety does not require clients to tell their trauma story in detail.

B. It is present-focused, teaching safe coping for trauma and substance use ✓ KEY

Seeking Safety is an integrated, present-focused model that builds safe coping skills for both conditions.

C. It treats the drinking first and delays trauma work until sobriety is stable

Seeking Safety addresses trauma and substance use at the same time rather than in sequence.

D. It uses repeated imaginal exposure to the worst memory in every session

Repeated imaginal exposure belongs to prolonged exposure, not to Seeking Safety.

Rationale: Seeking Safety is an integrated, present-focused treatment for co-occurring trauma and substance use. Its goal is safety: it teaches cognitive, behavioral and interpersonal coping skills that apply to both problems, without requiring clients to explore trauma memories in detail. This makes it suited to early recovery, when detailed trauma processing may destabilize the client.

References: SAMHSA TIP 57 — Trauma-specific services: integrated models for trauma and substance use (Seeking Safety)

☐ Approve ☐ Revise ☐ Retire Notes:

In the first two sessions of a closed group for adults coping with job loss, members talk politely about safe topics, look to the leader for direction and quietly test whether it is safe to share. According to Corey's model of group development, which stage is this group MOST likely in?

A. Transition stage

The transition stage is marked by open anxiety, defensiveness, conflict and challenges to the leader, not polite caution.

B. Working stage

The working stage features trust, cohesion and deep self-disclosure, which this group has not yet reached.

C. Initial stage ✓ KEY

Orientation, tentative and superficial sharing, reliance on the leader and testing of trust define the initial stage.

D. Final stage

The final stage involves consolidating learning and preparing to end, which does not fit a group in its first sessions.

Rationale: Corey describes groups moving from a pregroup stage through initial, transition, working and final stages. In the initial stage members orient themselves, share cautiously, look to the leader for structure and test whether the group is a safe place. Leaders help by clarifying norms and goals and modeling openness so that trust can develop.

References: Gladding (Counseling: A Comprehensive Profession) — Groups in counseling: stages of group development (initial stage)

☐ Approve ☐ Revise ☐ Retire Notes:

After several tense sessions in which members argued and challenged the leader, a workplace communication group begins agreeing on how to give feedback. Members start supporting each other and speak of the group as "we." According to Tuckman, which stage has the group entered?

A. Norming ✓ KEY

Norming follows storming: the group settles on shared rules, cohesion grows and members begin to support one another.

B. Performing

Performing comes after norming, when roles are flexible and the group's energy goes mainly into productive work.

C. Forming

Forming is the opening stage of orientation and dependence on the leader, which this group has passed.

D. Adjourning

Adjourning, added later by Tuckman and Jensen, concerns ending the group and is not described here.

Rationale: Tuckman's sequence is forming, storming, norming and performing, with adjourning added later with Jensen. After the conflict of storming, a group that works through its disagreements develops shared norms, cohesion and a sense of "we" in the norming stage. Performing follows, when the group can focus its energy on its task.

References: Gladding (Counseling: A Comprehensive Profession) — Groups in counseling: Tuckman's stages of group development

☐ Approve ☐ Revise ☐ Retire Notes:

A closed 10-week counseling group for college students has two sessions left. Which leader focus BEST fits Corey's description of the tasks of the final stage?

- A. Introducing deeper confrontation so that members can open up new core issues to work on
Opening major new issues near the end leaves members with work they cannot finish in the time left.
- B. Reviewing ground rules and helping each member define personal goals for the coming weeks
Setting ground rules and goals are initial-stage tasks, not tasks for the last two sessions.
- C. Encouraging members to challenge the leader's authority so that hidden conflict can surface
Challenges to the leader are typical of the transition stage and are not a final-stage aim.
- D. Helping members consolidate learning, voice feelings about ending and plan to apply gains ✓ KEY
Consolidating learning, dealing with feelings about separation and planning transfer to daily life are core final-stage tasks.

Rationale: In the final stage, the leader helps members review what they have learned, express feelings about the group ending, finish unfinished business and plan how to carry changes into their lives. Leaders avoid opening major new issues that cannot be worked through. Follow-up after the group can support transfer of learning.

References: Gladding (Counseling: A Comprehensive Profession) — Groups in counseling: termination of a group and the final stage

☐ Approve ☐ Revise ☐ Retire Notes:

A hospital counselor chairs a committee of nurses, chaplains and social workers that meets monthly to develop a new discharge-planning protocol. Under the Association for Specialists in Group Work (ASGW) classification of groups, this is BEST described as which type?

- A. Psychoeducation group
Psychoeducation groups teach information and skills to members to prevent problems or build coping.
- B. Task and work group ✓ KEY
Task and work groups, such as committees and planning teams, exist to accomplish a defined work goal.
- C. Counseling group
Counseling groups address members' personal and interpersonal concerns, not an organizational product.
- D. Psychotherapy group
Psychotherapy groups treat members with more serious or long-standing psychological problems.

Rationale: ASGW identifies four group work specializations: task and work groups, psychoeducation groups, counseling groups and psychotherapy groups. A committee meeting to produce a protocol is a task group, because its purpose is to complete work rather than to change members personally. Leaders of task groups attend to both the task and the group's process.

References: Gladding (Counseling: A Comprehensive Profession) — Groups in counseling: types of groups (task/work, psychoeducational, counseling, psychotherapy)

☐ Approve ☐ Revise ☐ Retire Notes:

A counselor runs an open-ended outpatient therapy group. A new member will join next week, and several veteran members worry that "starting over" will slow the group down. What is the BEST way to bring the newcomer in?

A. Prepare the newcomer beforehand and let veteran members convey the group's norms and work ✓ **KEY**

Pre-group preparation plus the veterans' modeling lets the newcomer learn the culture while the group keeps working.

B. Pause the group's ongoing themes for several weeks so the newcomer can catch up on its history

Halting the group's work to review history frustrates veterans and is not needed for the newcomer to join.

C. Ask the newcomer to stay silent and only observe for the first few sessions before taking part

Enforced silence isolates the newcomer and delays the engagement that helps a new member settle in.

D. Tell the veterans their worry is unfounded and move straight back to the planned session topic

Dismissing the veterans' reactions misses useful here-and-now material about change in the group.

Rationale: In open groups, new members are prepared individually before joining, and veteran members usually transmit the group's norms quickly by example. The group does not need to stop its work, and members' reactions to a newcomer, such as worry or rivalry, can be explored as here-and-now material.

References: Yalom & Leszcz (Group Psychotherapy) — Problems of membership: adding new members to an ongoing group

☐ Approve ☐ Revise ☐ Retire Notes:

In a substance use recovery group, a new member says, "Hearing Rosa talk about two years sober makes me think I might actually get there too." Which of Yalom's therapeutic factors is MOST evident?

A. Universality

Universality is relief at learning others share one's problems, not optimism drawn from another's progress.

B. Imitative behavior

Imitative behavior means copying another's behaviors; the member describes hope, not modeling a skill.

C. Instillation of hope ✓ **KEY**

Seeing a member who has improved with the same problem builds belief that one's own recovery is possible.

D. Development of socializing techniques

This factor concerns learning basic social skills, which is not what the member describes.

Rationale: Instillation of hope is the belief that the group can help, and it grows when members see others further along in recovery. Yalom notes that self-help and recovery groups lean heavily on this factor by featuring members who have succeeded. Hope keeps members in treatment long enough for other factors to work.

References: Yalom & Leszcz (Group Psychotherapy) — The therapeutic factors: instillation of hope

☐ Approve ☐ Revise ☐ Retire Notes:

A retired teacher in a depression group says the best part of the group was learning that her encouragement helped another member apply for a job: "For the first time in months, I feel useful to someone." Which of Yalom's therapeutic factors does this BEST illustrate?

A. Universality

Universality is discovering that others share one's problems; she is describing the value of helping.

B. Altruism ✓ KEY

Being able to help another member raises self-worth and pulls the helper out of self-absorption.

C. Catharsis

Catharsis is the release of strong feeling, which is not the source of her sense of usefulness.

D. Imparting information

Imparting information is didactic teaching or advice received, not the benefit of giving help to others.

Rationale: Yalom's altruism factor holds that members benefit from giving as well as receiving: offering support, insight and encouragement shows them they have something of value to others. This is especially powerful for people with depression or low self-esteem who see themselves as a burden.

References: Yalom & Leszcz (Group Psychotherapy) — The therapeutic factors: altruism

☐ Approve ☐ Revise ☐ Retire Notes:

Late in a cancer survivors' group, a member says the group taught him that however much support he receives, he must still face his own mortality and take ultimate responsibility for how he lives. Which of Yalom's therapeutic factors does this BEST reflect?

A. Universality

Universality is the relief that others share one's problems; his insight is about facing life alone and choosing.

B. Catharsis

Catharsis is emotional release; his statement describes an insight about mortality and responsibility.

C. Interpersonal learning

Interpersonal learning is about how one affects and is seen by others, not about existential givens.

D. Existential factors ✓ KEY

Recognizing mortality, aloneness and ultimate responsibility for one's life defines the existential factors.

Rationale: Yalom's existential factors include recognizing that life can be unfair, that pain and death cannot be escaped, that one faces life alone however close one is to others, and that one is ultimately responsible for how one lives. Members of groups facing serious illness often rank these factors highly. The distractors name other factors with a different focus.

References: Yalom & Leszcz (Group Psychotherapy) — The therapeutic factors: existential factors

☐ Approve ☐ Revise ☐ Retire Notes:

In the fifth session of a support group for new parents, one member has said little since the first meeting, though she listens closely and nods often as others talk. Which leader response is BEST?

A. Note her nonverbal reactions and invite her, without pressure, to share what is resonating ✓ KEY

Drawing out a quiet member gently, using what the leader observes, invites participation while respecting her pace.

B. Ask the other members to wait in silence until she takes a turn and speaks to the group today

Putting her on the spot in front of the group creates pressure and can increase her withdrawal.

C. Tell her privately after the session that she must speak up more if she wants to stay in the group

A coercive ultimatum violates members' right to decide what they share and damages trust.

D. Leave her alone, since a quiet member is unaffected by the group and unlikely to benefit from it

Silent members can be deeply engaged; ignoring her misses a chance to include her and learn what silence means.

Rationale: Silence can reflect anxiety, cultural norms, deep engagement or feeling unwelcome, so leaders explore it rather than assume. Drawing out is a leadership skill that invites a quiet member to participate, often by noting observable reactions, while respecting the member's right to choose what to share. Pressuring or ignoring the member are both unhelpful.

References: Gladding (Counseling: A Comprehensive Profession) — Groups in counseling: leadership skills and problem behaviors (silent members, drawing out)

☐ Approve ☐ Revise ☐ Retire Notes:

In a high school counseling group, a member starts describing in detail what an absent member's boyfriend supposedly did at a party, and other members lean in to hear more. Which leader skill is MOST needed right now?

A. Linking

Linking connects members' shared concerns; it would not stop harmful talk about an absent member.

B. Drawing out

Drawing out invites quieter members to speak, which would add to the gossip rather than stop it.

C. Summarizing

Summarizing pulls themes together; it does not intervene in a counterproductive behavior.

D. Blocking ✓ KEY

Blocking stops counterproductive behavior such as gossip or invading an absent member's privacy.

Rationale: Blocking is the leader's intervention to stop behavior that harms members or the group, such as gossiping, breaking confidentiality, scapegoating or pressuring disclosure. The leader blocks the behavior, not the person, and can redirect the speaker toward her own feelings in the here and now.

References: Gladding (Counseling: A Comprehensive Profession) — Groups in counseling: group leadership skills (blocking)

☐ Approve ☐ Revise ☐ Retire Notes:

An 11-year-old tells his counselor he has crossed hairstylist and bus driver off his list because "people in my family go to college and get important jobs." In Gottfredson's theory of circumscription and compromise, this reasoning reflects which stage?

A. Orientation to size and power

This first stage (about ages 3 to 5) involves seeing jobs as adult roles, not ranking them by status.

B. Orientation to sex roles

At about ages 6 to 8 children rule out jobs as wrong for their gender, which is not his reason here.

C. Orientation to social valuation ✓ KEY

At about ages 9 to 13 children rule out jobs seen as too low in prestige for their social class or ability.

D. Orientation to internal unique self

From about age 14, youth weigh interests, values and personality within the options left, a later stage.

Rationale: Gottfredson proposed that children progressively eliminate (circumscribe) occupations: first by size and power, then by sex type, then by prestige and social valuation, and finally by fit with the internal, unique self. Around ages 9 to 13, children become aware of social class and set a floor of acceptable prestige, ruling out jobs seen as beneath their family's expectations.

References: Sharf (Career Development Theory) — Gottfredson's theory of circumscription and compromise: stages of circumscription

☐ Approve ☐ Revise ☐ Retire Notes:

Using the CASVE cycle from the cognitive information processing approach, a counselor has helped a client understand her values, skills and the options open to her. They now expand her list of possible occupations and then narrow it to three or four realistic choices. Which phase are they in?

A. Analysis

Analysis, the step already completed, builds understanding of self and options.

B. Synthesis ✓ KEY

Synthesis elaborates options and then crystallizes them into a short list of realistic choices.

C. Valuing

Valuing comes next, weighing each short-listed option's costs and benefits for self and others.

D. Communication

Communication is the first phase, when a person recognizes a gap between where they are and want to be.

Rationale: The CASVE cycle of Sampson, Peterson, Lenz and Reardon moves through Communication, Analysis, Synthesis, Valuing and Execution. In Synthesis, the person first elaborates possible options and then crystallizes them into a manageable set of about three to five. Valuing then ranks those options, and Execution turns the first choice into an action plan.

References: Sharf (Career Development Theory) — Cognitive information processing approach: the CASVE cycle

☐ Approve ☐ Revise ☐ Retire Notes:

In Savickas's career construction interview, a counselor asks a client whom she admired when she was growing up, other than her parents. According to career construction theory, what do her answers MOST directly reveal?

A. The occupations her parents expected her to enter and how she has met those expectations

The question deliberately excludes parents; it targets figures she chose for herself.

B. Her current level of career adaptability across concern, control, curiosity and confidence

Adaptability resources are assessed separately; the role model question is about self-concept.

C. Her self-concept: the traits she has chosen as a blueprint for the self she is constructing ✓ KEY

Role models reveal the qualities a person has chosen to incorporate, a blueprint for self-construction.

D. Her Holland code, since admired figures predict which work setting will be most congruent

Magazines and favorite settings point to preferred work environments; role models speak to the self.

Rationale: In Savickas's career construction interview, early role models are treated as a blueprint for the self: the traits a client admired are ones she has chosen to build into her own identity. Other questions, such as favorite magazines or television shows, point to preferred work settings, and early recollections reveal life themes or preoccupations. Together they help the client author a coherent career story.

References: Sharf (Career Development Theory) — Savickas's career construction theory: the career construction interview and role models

☐ Approve ☐ Revise ☐ Retire Notes:

A 21-year-old who earlier explored several fields has committed to nursing and is now deciding to specialize in pediatric intensive care. In Ginzberg, Ginsburg, Axelrad and Herma's theory, she is in which substage of the realistic period?

A. Specification ✓ KEY

Specification is the final realistic substage, when a person selects a specific job or specialty.

B. Crystallization

Crystallization is committing to a general field, which she has already done by choosing nursing.

C. Exploration

Exploration is the first realistic substage, when options are still being sampled broadly.

D. Transition

Transition is the last substage of the tentative period in adolescence, not of the realistic period.

Rationale: Ginzberg and colleagues described fantasy, tentative and realistic periods of career choice. The tentative period includes interest, capacity, value and transition substages. The realistic period, from about age 17 into the early twenties, moves through exploration, crystallization around a field and specification of a particular job or specialty.

References: Sharf (Career Development Theory) — Developmental theories: Ginzberg and colleagues' periods of career choice

☐ Approve ☐ Revise ☐ Retire Notes:

A 20-year-old says he has not thought much about what he wants in a career, religion or politics, has no plans and does not see why it matters. He is not distressed about this. Which of Marcia's identity statuses BEST fits?

A. Identity moratorium

Moratorium involves active exploration without commitment, often with some anxiety.

B. Identity foreclosure

Foreclosure is commitment without exploration, usually to values taken from parents.

C. Identity achievement

Achievement follows exploration with firm commitments, neither of which he shows.

D. Identity diffusion ✓ KEY

Diffusion is neither exploring nor committing, often with apathy about identity questions.

Rationale: Marcia classified identity status by two dimensions: exploration and commitment. Achievement has both, moratorium has exploration without commitment, foreclosure has commitment without exploration, and diffusion has neither. Diffused young people often seem aimless or unconcerned about who they are becoming.

References: Berk (Development Through the Lifespan) — Adolescence: Marcia's identity statuses

☐ Approve

☐ Revise

☐ Retire

Notes:

A school counselor sets up regular meetings between a 9-year-old's parents and his teacher so that home and school follow the same homework and behavior plan. In Bronfenbrenner's ecological systems theory, the counselor is strengthening which system?

A. Microsystem

Each setting alone, home or classroom, is a microsystem; the links between them are something more.

B. Mesosystem ✓ KEY

The mesosystem is the set of connections between microsystems, such as home and school.

C. Exosystem

The exosystem holds settings the child is not in that affect him, such as a parent's workplace.

D. Macrosystem

The macrosystem is the broad culture, values and laws, not specific home-school links.

Rationale: Bronfenbrenner described nested systems: the microsystem of immediate settings, the mesosystem of connections among those settings, the exosystem of settings that affect the child indirectly, the macrosystem of cultural values and laws, and the chronosystem of change over time. Strong home-school partnerships are a classic mesosystem support.

References: Berk (Development Through the Lifespan) — History, theory and research strategies: Bronfenbrenner's ecological systems theory

☐ Approve

☐ Revise

☐ Retire

Notes:

An 8-year-old explains that he shares his snack with a classmate "because then she'll share her cookies with me tomorrow." For him, right action is whatever serves his own interests through fair trades. In Kohlberg's theory, this reasoning BEST fits which stage orientation?

A. Punishment and obedience

Stage 1 reasoning centers on avoiding punishment from authority, not on making even trades.

B. Interpersonal conformity

Stage 3 reasoning seeks approval as a good person and to maintain relationships.

C. Instrumental exchange ✓ KEY

Stage 2 sees right action as meeting one's own needs through fair, tit-for-tat exchanges.

D. Social contract orientation

Stage 5 views laws as flexible agreements serving the greater good, far beyond this reasoning.

Rationale: Kohlberg's preconventional level has two stages: Stage 1, punishment and obedience, and Stage 2, the instrumental purpose orientation, in which right action is what satisfies one's own needs, often through equal exchange ("you scratch my back, I'll scratch yours"). Conventional reasoning (Stages 3 and 4) shifts toward approval and social order.

References: Berk (Development Through the Lifespan) — Moral development: Kohlberg's preconventional level (Stage 2, instrumental purpose orientation)

☐ Approve ☐ Revise ☐ Retire Notes:

A 5-year-old talks aloud to herself while working on a difficult puzzle ("No, the corner piece goes here first...") and talks more as the task gets harder. How would Vygotsky MOST likely interpret this behavior?

A. As self-guiding speech that helps her plan and that will later become silent inner speech ✓ KEY

Vygotsky saw private speech as a tool for self-regulation that rises with task difficulty and is later internalized.

B. As egocentric speech that shows she cannot yet take the perspective of a nearby listener

This is Piaget's view of such speech, which Vygotsky disputed.

C. As a sign of possible language delay that calls for a speech-language evaluation soon

Private speech is typical in the preschool years and is not a sign of delay.

D. As attention-seeking behavior that adults should ignore so that it decreases over time

The child is directing her own actions, not seeking attention, so ignoring it misreads its purpose.

Rationale: Piaget called children's self-talk egocentric speech, reflecting an inability to take others' perspectives. Vygotsky argued instead that this private speech is a tool for guiding thought and behavior, increases when tasks are challenging, and gradually becomes silent inner speech. Research has generally supported Vygotsky's view.

References: Berk (Development Through the Lifespan) — Early childhood cognition: Vygotsky's private speech versus Piaget's egocentric speech

☐ Approve ☐ Revise ☐ Retire Notes:

In the Strange Situation, a 12-month-old explores the toys with little attention to his mother, shows little distress when she leaves and, when she returns, looks away and does not seek contact. Which attachment classification BEST fits?

A. Secure

Secure infants use the parent as a base, may be upset at separation and seek comfort on reunion.

B. Resistant (ambivalent)

Resistant infants are highly distressed at separation and both seek and resist contact on reunion.

C. Disorganized

Disorganized infants show confused or contradictory behavior such as freezing or dazed looks.

D. Avoidant ✓ KEY

Avoidant infants show little separation distress and avoid or ignore the parent on reunion.

Rationale: Ainsworth's Strange Situation classifies infants mainly by reunion behavior. Avoidant infants seem unresponsive to the parent, are not distressed by separation and turn away on return; this pattern is linked to caregiving that rebuffs bids for comfort. Main and Solomon later added the disorganized category.

References: Berk (Development Through the Lifespan) — Emotional and social development in infancy: measuring attachment security (the Strange Situation)

☐ Approve ☐ Revise ☐ Retire Notes:

Ainsworth's home observations linked infants' later attachment classifications to how their caregivers had responded during the first year. Which caregiving pattern is MOST strongly associated with secure attachment?

A. Responding to each whimper at once so that the infant rarely experiences any distress at all

Sensitivity is about accurate, well-matched responses, not removing all distress.

B. Holding the infant as many hours as possible, whatever signals the infant may be giving

Contact that ignores the infant's cues is intrusive rather than sensitive.

C. Noticing the infant's signals, reading them accurately and responding promptly and fittingly ✓ KEY

Sensitive responsiveness, as Ainsworth defined it, is the caregiving feature most tied to secure attachment.

D. Giving consistent physical care on a fixed schedule while limiting emotional responsiveness

Routine care with little emotional responsiveness is more consistent with avoidant patterns.

Rationale: Ainsworth found that sensitive caregiving, meaning noticing the infant's signals, interpreting them accurately and responding promptly and appropriately, predicted secure attachment. Inconsistent care was linked to resistant attachment, and rejecting or emotionally unavailable care to avoidant attachment. Research since then shows a moderate link between sensitivity and security.

References: Berk (Development Through the Lifespan) — Emotional and social development in infancy: factors that affect attachment security (quality of caregiving)

☐ Approve ☐ Revise ☐ Retire Notes:

A client mentions that she skipped her sister's wedding. The counselor asks, "Why did you do that?" and the client becomes guarded. What is the MOST likely problem with the counselor's question?

A. "Why" questions can sound judgmental and lead the client to defend or justify herself ✓ KEY

"Why" questions often imply criticism and invite explanations and defensiveness rather than exploration.

B. It is a closed question, so it limits the client to a brief yes-or-no type of response

A "why" question is open, not closed; the problem is its judgmental tone.

C. It shifts the focus from the client's own experience to her sister's needs and feelings

The question stays on the client's actions; it does not shift focus to the sister.

D. It asks about the past, and counselors should limit questions to the present moment

Exploring past events is often useful; the timing of the event is not the issue.

Rationale: Questions beginning with "why" tend to put clients on the defensive because they can sound like a demand for justification. "What" and "how" questions, such as "What led to that decision?", invite description and exploration. Counselors also avoid strings of questions that make a session feel like an interrogation.

References: Gladding (Counseling: A Comprehensive Profession) — Counseling skills: questions and the problems with "why" questions

☐ Approve ☐ Revise ☐ Retire Notes:

Reviewing a recording, a supervisor notes that a trainee often changes the subject, jumping to topics that interest the trainee instead of staying with what the client just said. Which component of Ivey's attending behavior should the trainee practice?

A. Eye contact

Eye contact is a visual attending behavior; the problem described is in what the trainee says.

B. Vocal qualities

Vocal qualities concern tone, pace and volume rather than the topics the trainee follows.

C. Verbal tracking ✓ KEY

Verbal tracking means staying with the client's topic and not jumping to new subjects.

D. Body language

Posture and gestures matter, but they do not address topic jumping in the trainee's speech.

Rationale: Ivey describes attending behavior through visuals (eye contact), vocal qualities, verbal tracking and attentive body language. Verbal tracking means following the client's story without topic jumps, which shows the client that the counselor is listening. Counselors also notice which topics they selectively attend to.

References: Gladding (Counseling: A Comprehensive Profession) — Counseling skills: attending behavior and Ivey's microskills (verbal tracking)

☐ Approve ☐ Revise ☐ Retire Notes:

A 70-year-old client says, "Moving in with my daughter means I'll see my grandkids every day. But I'm giving up the house I lived in for forty years." Which response BEST reflects the client's feelings?

A. You will get to see your grandchildren every day once you move in with your daughter.

This paraphrases part of the content and leaves out both the feelings and the loss.

B. Have you thought about whether you could keep the house and visit the family more often?

This question moves to problem solving before the client's feelings have been heard.

C. Many people your age move in with family, and most adjust well within the first year.

Reassurance with a generalization minimizes the client's feelings and may sound ageist.

D. You're looking forward to being near the grandkids, and also grieving leaving your home. ✓ **KEY**

This captures both sides of the client's mixed feelings without choosing between them.

Rationale: Clients often express mixed or contradictory feelings about one event. An accurate reflection of feeling names both emotions and links them to their sources, often with "and" rather than "but" so that neither feeling cancels the other. Paraphrasing content, advising or reassuring misses the emotional message.

References: Gladding (Counseling: A Comprehensive Profession) — Counseling skills: reflection of feeling and mixed emotions

☐ Approve ☐ Revise ☐ Retire Notes:

A counselor paraphrases, "So you're angry that your boss ignored your idea in the meeting." The client pauses and says, "Not angry, really. More like invisible." What is the BEST next response from the counselor?

A. Explain that anger is a common reaction to being ignored and is often hidden under hurt

Insisting on the counselor's label overrides the client's correction and weakens the alliance.

B. Accept the correction: "Invisible, as if your ideas don't register. Did I get that right?" ✓ **KEY**

Taking in the correction and checking accuracy keeps the reflection client-centered and builds trust.

C. Move on to problem solving so that the client does not feel stuck on the word choice itself

Skipping past the correction misses the client's more precise and meaningful feeling.

D. Ask the client to describe a time she felt angry at work so the anger can be explored more

This pursues the counselor's hypothesis rather than the feeling the client has just named.

Rationale: Paraphrases and reflections are tentative hypotheses that the client can confirm or correct. When a client corrects one, the counselor accepts the new wording, reflects it and checks for accuracy (a "check-out"). Clients often refine their own understanding through this exchange, so a corrected reflection is still useful.

References: Gladding (Counseling: A Comprehensive Profession) — Counseling skills: paraphrasing, accuracy and checking out

☐ Approve ☐ Revise ☐ Retire Notes:

After a counselor gently points out a discrepancy, a client says, "Okay, I see that I keep missing my kids' games, even though I say family comes first. I'm still not sure what to do about it." On Ivey's Client Change Scale, this response is BEST classified as:

A. Denial

Denial would mean not seeing or refusing to admit the discrepancy, which the client openly accepts.

B. Partial examination

Partial examination involves working on only part of the discrepancy; he acknowledges all of it.

C. Acceptance and recognition ✓ KEY

He fully accepts the discrepancy but has not yet generated a new way of acting.

D. Transcendence and new constructs

The top level involves new, larger ways of thinking and acting, which he has not reached.

Rationale: Ivey's Client Change Scale rates client responses to confrontation from denial (level 1), through partial examination (2), acceptance and recognition (3) and creation of a new solution (4), to transcendence or development of new, more inclusive constructs (5). A client who fully owns the discrepancy but has no plan yet is at level 3, and the counselor can now help generate new options.

References: Ivey et al. (Intentional Interviewing) — Confrontation: the Client Change Scale (level 3, acceptance and recognition) · Gladding (Counseling: A Comprehensive Profession) — Counseling skills: confrontation and evaluating the client's response

☐ Approve ☐ Revise ☐ Retire Notes:

Each session, a client asks her counselor what she should do and then thanks him warmly for "knowing best." The counselor notices he has begun giving advice he would not usually give. Which response BEST uses immediacy?

A. "I notice we've settled into a pattern where you ask and I answer. What's that like for you?" ✓ KEY

Naming the pattern in their relationship as it happens and inviting her view is relationship immediacy.

B. "You seem to rely on others for answers, and that likely began in your family growing up."

This is an interpretation about her past, not a here-and-now comment on their relationship.

C. "Let's set a rule that I will not answer any more direct questions about what you should do."

Imposing a rule changes behavior without exploring what is happening between them.

D. "You are capable of making good decisions, and I trust you to figure out what is best for you."

Reassurance may be kind, but it does not address the relational pattern the counselor noticed.

Rationale: Immediacy is the counselor's direct, present-tense comment on what is happening between counselor and client, often a pattern that mirrors the client's other relationships. Naming the pattern tentatively and inviting the client's experience opens it for exploration. It also helps the counselor step out of a role the client may be pulling him into.

References: Gladding (Counseling: A Comprehensive Profession) — Counseling skills: immediacy

☐ Approve ☐ Revise ☐ Retire Notes:

A client has met all of her counseling goals, but her counselor keeps suggesting "a few more sessions" because he enjoys their work and will miss her. What should the counselor do FIRST?

A. Keep meeting, since extra sessions are unlikely to harm a client who is doing well

Continuing counseling that no longer serves the client puts the counselor's needs ahead of hers.

B. Recognize his own reactions, consult a colleague and start planning a timely ending ✓ **KEY**

Naming his countertransference, seeking consultation and moving toward termination protects the client.

C. Ask the client if she would like to keep coming monthly for some ongoing social support

Offering ongoing sessions for social contact blurs the professional role and serves the counselor.

D. End counseling at the next session without discussion so his feelings do not interfere

An abrupt ending without preparation denies the client a planned, collaborative termination.

Rationale: The ACA Code of Ethics directs counselors to end a counseling relationship when the client no longer needs or benefits from it. A counselor's own sadness or reluctance about ending is a form of countertransference that can prolong counseling for the counselor's benefit. Recognizing it, consulting and planning a well-prepared termination keeps the work centered on the client.

References: ACA Code of Ethics — A.11.c Appropriate Termination · Gladding (Counseling: A Comprehensive Profession) — Termination: counselor resistance to ending

☐ Approve ☐ Revise ☐ Retire Notes:

A university counselor learns that several transgender students have been denied campus housing that matches their gender identity. She works with residence life administrators to revise the university's housing policy. In the MSJCC, this intervention targets which socioecological level?

A. Intrapersonal

Intrapersonal work addresses an individual's own thoughts, feelings and identity.

B. Interpersonal

Interpersonal work addresses relationships with family, friends and peers.

C. Public policy

Public policy refers to local, state and federal laws, not one university's internal rules.

D. Institutional ✓ **KEY**

Changing the policies of a social institution such as a university is institutional-level advocacy.

Rationale: The Multicultural and Social Justice Counseling Competencies place counseling and advocacy interventions at socioecological levels: intrapersonal, interpersonal, institutional, community, public policy and international or global. Institutions include schools, universities, churches and agencies, so changing a university housing policy is institutional work. Public policy work targets laws and government policies.

References: MSJCC (Ratts et al., 2016) — Counseling and advocacy interventions: socioecological levels (institutional)

☐ Approve ☐ Revise ☐ Retire Notes:

A 13-year-old with a Black father and a White mother tells her school counselor that classmates keep asking "what she really is," and she feels she has to pick one racial group to belong to. In Poston's biracial identity development model, which stage BEST fits?

A. Personal identity

Personal identity is the early childhood stage, when self-concept rests on personal traits rather than group.

B. Choice of group categorization ✓ KEY

In this stage, social pressure pushes a biracial youth to choose one racial or ethnic identity.

C. Enmeshment and denial

Enmeshment/denial follows the choice and brings guilt or confusion about rejecting one heritage.

D. Integration of both racial heritages

Integration is the final stage of valuing all of one's heritages, which she has not reached.

Rationale: Poston's five-stage model of biracial identity development runs from personal identity, through choice of group categorization, enmeshment/denial and appreciation, to integration. In the choice stage, peers, family and society press the young person to identify with one group. Counselors can validate the pressure and support exploration of all heritages.

References: Poston (Biracial Identity) — Stage 2, choice of group categorization · Sue & Sue (Counseling the Culturally Diverse) — Racial/cultural identity development: biracial and multiracial identity (Poston's model)

☐ Approve ☐ Revise ☐ Retire Notes:

A White counseling student now rejects racism intellectually and seeks out classmates of color, often asking them to explain racism to her, while still judging their experiences by White norms. In Helms's White racial identity model, which status BEST fits?

A. Reintegration

Reintegration is a retreat to idealizing Whites and blaming people of color, which she has moved past.

B. Pseudo-independence ✓ KEY

An intellectual commitment to racial justice while leaning on people of color and White standards marks this status.

C. Immersion and emersion

In immersion/emersion, she would turn to other Whites to explore what it means to be White.

D. Disintegration

Disintegration involves guilt, confusion and conflicting loyalties, not a settled intellectual stance.

Rationale: Helms's model moves from contact, disintegration and reintegration (abandoning racism) to pseudo-independence, immersion/emersion and autonomy (building a nonracist White identity). In pseudo-independence, a person accepts racial equality intellectually but still relies on people of color to explain racism and views them through White frames. Immersion/emersion shifts the focus to understanding one's own Whiteness.

References: Sue & Sue (Counseling the Culturally Diverse) — White racial identity development: Helms's White racial identity statuses

☐ Approve ☐ Revise ☐ Retire Notes:

Core Counseling Attributes

80 items · NCE weight 8% (13 of 160 scored items)

Q921 of 1000 · nce-s-cor-001 · Counseling and Helping Relationships · empathy and its levels · medium · In review

A 34-year-old client says, "My boss praised my project in front of everyone, but then gave the promotion to someone else. I don't know what to think." Which counselor response is an interchangeable (Level 3) response on Carkhuff's empathy scale?

A. You should ask your boss directly why you were passed over this time.

Advice-giving ignores the client's feelings entirely; it is subtractive (Level 1 or 2), not interchangeable.

B. You feel confused because the praise and the promotion decision don't match. ✓ **KEY**

This captures the feeling (confusion) and its stated cause at the same depth the client expressed, which is the definition of a Level 3 interchangeable response.

C. Underneath the confusion, you may fear that your hard work goes unrecognized.

This goes beyond what the client expressed to an underlying feeling and meaning, making it additive (Level 4), not interchangeable.

D. Lots of people get passed over at work, and it usually works out fine in the end.

Reassurance minimizes the experience and shifts away from the client's feelings, a subtractive response.

Rationale: Carkhuff's scale rates counselor responses from 1 to 5. Level 3 is the minimally facilitative, interchangeable level: the counselor reflects essentially the same feeling and meaning the client expressed, adding nothing and taking nothing away. Levels 1 and 2 subtract from the client's message, and Levels 4 and 5 add depth beyond it.

References: Gladding (Counseling: A Comprehensive Profession) — Carkhuff's scale of empathic understanding, Levels 1-5

☐ Approve ☐ Revise ☐ Retire Notes:

Q922 of 1000 · nce-s-cor-002 · Counseling and Helping Relationships · empathy and its levels · hard · In review

In a sixth session, a graduate student says, "I keep telling everyone I'm fine with moving home after graduation. My parents are happy, so I guess I should be too." Which counselor response BEST illustrates additive (Level 4) empathy?

A. So you're moving home after graduation, and your parents are pleased with it.

This paraphrases the content but omits feeling; it is at best Level 2, slightly subtractive.

B. You feel fine about moving home, since your parents are happy with the plan.

This repeats the client's surface statement and misses the doubt in "I guess I should be"; it does not add depth.

C. Why do you think your parents' happiness matters so much in this decision?

A why-question invites intellectualizing and does not communicate understanding of the feeling, so it is not an empathic response.

D. You sound unsure, as if your own wishes got set aside to keep your parents happy. ✓ **KEY**

This names the implicit ambivalence and the meaning beneath it (setting aside one's own wishes), adding accurately to what the client said: Level 4.

Rationale: Additive empathy (Carkhuff Levels 4-5) goes beyond the client's explicit words to reflect feelings and meanings the client implied but did not fully state, which helps the client explore at a deeper level. To be additive rather than inaccurate, the reflection must stay grounded in cues the client gave, such as the hedged "I guess I should be."

References: Gladding (Counseling: A Comprehensive Profession) — Advanced (additive) empathy, Carkhuff Levels 4-5

► **Check:** Option A is rated Level 2 in its rationale; raters may disagree on the exact level (it is clearly not additive).

☐ Approve ☐ Revise ☐ Retire Notes:

On Carkhuff's five-level scale of empathic understanding, a counselor response rated at Level 1 or Level 2 is described as which of the following?

A. Subtractive, detracting from what the client expressed ✓ KEY

Levels 1 and 2 are subtractive: they ignore or only partly capture the client's feelings and meaning.

B. Interchangeable, matching the client's expressed feeling

Interchangeable, minimally facilitative responses are Level 3.

C. Additive, reflecting feelings below the client's words

Additive responses that reach underlying feelings are Levels 4 and 5.

D. Facilitative, the baseline goal for beginning counselors

Level 3 is the minimally facilitative baseline counselors aim to reach consistently; Levels 1-2 fall below it.

Rationale: Carkhuff's scale runs from Level 1 (the response ignores or contradicts the client's feelings) to Level 5 (the response adds significantly to the client's awareness). Levels 1-2 are subtractive, Level 3 is interchangeable and minimally facilitative, and Levels 4-5 are additive.

References: Gladding (Counseling: A Comprehensive Profession) — Carkhuff's scale of empathic understanding

☐ Approve ☐ Revise ☐ Retire Notes:

A client tearfully describes the recent death of her dog. Which counselor response reflects empathy rather than sympathy?

A. I'm so sorry; I lost my own dog last year and I still cry about it sometimes too.

This shifts the focus to the counselor's own loss and expresses pity or shared distress, which is sympathy and self-disclosure, not empathy.

B. That is such a sad thing to go through, and I feel terrible for you today.

Feeling sorry for the client is sympathy; it conveys the counselor's feeling rather than understanding of the client's.

C. Losing her has left a real emptiness in your days, and the grief feels heavy. ✓ KEY

This communicates accurate understanding of the client's own experience from her frame of reference, which is empathy.

D. At least she had a long life with you, and you gave her a very good home.

"At least" reassurance minimizes the loss and moves away from the client's feelings.

Rationale: Empathy is understanding the client's world as if it were one's own, without losing the "as if" quality, and communicating that understanding back. Sympathy centers on the helper's own feelings of sorrow or pity for the client, which can shift the focus away from the client.

References: Corey (Theory & Practice) — Person-centered therapy: accurate empathic understanding

☐ Approve ☐ Revise ☐ Retire Notes:

A 52-year-old client says, "I yelled at my teenage son last night and called him useless. I'm ashamed to even tell you." Which counselor response BEST demonstrates unconditional positive regard?

A. That was harsh, but I'm sure you are a good parent most of the time anyway.

Reassurance about being a "good parent" is conditional praise; it evaluates the client rather than accepting the client.

B. Calling him useless can do real damage, so let's plan what to say instead.

This jumps to evaluation and correction before the client feels accepted; it is not an expression of UPR.

C. It took courage to share that, and I'm glad you trusted me with it here. ✓ KEY

This accepts the client as a person, without approving or condemning the behavior, and values the client's openness, which is UPR.

D. Don't be ashamed, since all parents lose their temper and say things they regret.

Telling the client not to feel shame dismisses the feeling and normalizes too quickly; it is reassurance, not acceptance.

Rationale: Rogers described unconditional positive regard as a warm acceptance of the client as a person, without conditions of worth. UPR does not mean approving of every behavior; it means the client's worth is not contingent on what is disclosed, which lowers defensiveness and supports self-exploration.

References: Corey (Theory & Practice) — Person-centered therapy: unconditional positive regard and acceptance

☐ Approve ☐ Revise ☐ Retire Notes:

Midway through a session, a client asks, "You look distracted. Are you even listening to me?" The counselor realizes her attention had drifted. Which response BEST demonstrates congruence?

A. You're right, my mind wandered for a moment. I'm with you now; please go on. ✓ KEY

The counselor's words match her inner experience, she owns it briefly, and she returns the focus to the client, which is congruence.

B. Of course I'm listening, and I have been following everything you've said.

Denying a lapse the counselor knows occurred is incongruent; outward words do not match inner experience.

C. It sounds like you worry people don't listen to you. Tell me more about that.

Reflecting the client's concern here deflects from the counselor's actual lapse and avoids genuineness.

D. I've had a long, stressful day with other clients, so I'm tired and distracted.

This is honest but over-discloses and shifts the focus to the counselor's needs rather than the relationship.

Rationale: Congruence (genuineness) means the counselor's outward behavior is consistent with inner experience, without a professional facade. Rogers viewed it as the most basic of the core conditions. Being genuine does not require disclosing every feeling; the counselor shares what is relevant to the relationship and keeps the focus on the client.

References: Corey (Theory & Practice) — Person-centered therapy: congruence or genuineness

☐ Approve ☐ Revise ☐ Retire Notes:

Bordin's pantheoretical model of the working alliance consists of which three components?

A. Empathy, congruence, and positive regard

These are Rogers' core conditions, not Bordin's alliance components.

B. Rapport, insight, and corrective feedback

These are general therapy processes, not the components Bordin named.

C. Trust, transference, and client motivation

Transference is a psychodynamic concept; Bordin's model was explicitly pantheoretical.

D. Agreement on goals, tasks, and the bond ✓ **KEY**

Bordin defined the working alliance as agreement on goals, agreement on tasks, and the emotional bond between counselor and client.

Rationale: Bordin (1979) proposed that the working alliance has three parts: agreement on the goals of counseling, agreement on the tasks used to reach them, and the affective bond between counselor and client. Because it is pantheoretical, the model applies across approaches, and alliance strength is one of the most consistent predictors of outcome.

References: Gladding (Counseling: A Comprehensive Profession) — The working alliance: goals, tasks, and bond

☐ Approve ☐ Revise ☐ Retire Notes:

In the fourth session, a client who has been completing CBT thought records says flatly, "I did the homework again, I guess. Whatever helps you." The client has arrived late twice. What should the counselor do FIRST?

A. Review the thought record in detail and praise the client for completing it

Proceeding with the task ignores the withdrawal cues and leaves a possible rupture unaddressed.

B. Invite the client to talk about how the work together is feeling so far ✓ **KEY**

Openly exploring the relationship addresses a likely withdrawal rupture and checks agreement on tasks and goals before continuing.

C. Set a lateness policy so that attendance does not continue to slip further

Focusing on rules treats the lateness as a compliance issue and can deepen the rupture.

D. Switch to a different intervention, since thought records are not working

Changing tasks unilaterally skips collaboration; the counselor does not yet know what the client wants or is reacting to.

Rationale: Flat compliance ("whatever helps you") and lateness suggest a withdrawal rupture in the alliance, often reflecting disagreement about tasks or goals or strain in the bond. Research on rupture and repair supports raising the relationship directly and nondefensively, which lets the counselor renegotiate tasks and goals collaboratively. Successfully repaired ruptures are associated with better outcomes.

References: Gladding (Counseling: A Comprehensive Profession) — Working alliance: goals, tasks, bond; addressing alliance ruptures

► **Check:** Rupture-repair evidence (Safran et al.) is cited to a general Gladding alliance reference.

☐ Approve ☐ Revise ☐ Retire Notes:

A counselor who has completed many multicultural trainings begins work with a Hmong American client. According to Hook and colleagues' model of cultural humility, which stance BEST reflects cultural humility?

A. Ask the client what their culture means to them and stay open to being corrected ✓ KEY

Cultural humility is an other-oriented, curious stance that treats the client as the expert on their own cultural identity.

B. Draw on prior training about Hmong customs to anticipate the client's values

Applying general group knowledge as a template risks stereotyping; it reflects a knowledge-based, not humble, stance.

C. Set cultural issues aside unless the client raises them first in the session

Waiting for the client to raise culture places the burden on the client and can signal that culture is unwelcome.

D. Treat the training already completed as sufficient competence for this client

Viewing competence as a finished achievement contradicts the ongoing, lifelong nature of humility.

Rationale: Hook et al. (2013) described cultural humility as an interpersonal stance that is other-oriented rather than self-focused, marked by respect, openness, and lack of superiority regarding the client's cultural background. It is an ongoing process of self-reflection rather than a fixed body of knowledge, and clients' perceptions of it are associated with a stronger alliance.

References: Sue & Sue (Counseling the Culturally Diverse) — Cultural humility as an other-oriented stance

☐ Approve ☐ Revise ☐ Retire Notes:

A counselor notices that she schedules a client who lives in public housing for shorter sessions and assigns fewer homework tasks, assuming the client "won't follow through." What is the MOST appropriate next step?

A. Refer the client to a counselor who has more experience with low-income clients

Referral based on the counselor's bias, rather than competence limits, abandons the client and avoids the counselor's own work.

B. Continue as planned, since shorter sessions may reduce the client's burden

This rationalizes the bias instead of examining it and keeps treating the client differently.

C. Examine the assumption in supervision and restore an equitable treatment plan ✓ KEY

Recognizing an implicit bias, exploring it in supervision, and correcting its effect on care reflects counselor self-awareness.

D. Tell the client about the bias so the client can decide whether to continue

Disclosing the bias burdens the client with the counselor's issue; the counselor should first address it through supervision.

Rationale: Counselor self-awareness of attitudes, beliefs, and biases is the first dimension of multicultural competence. Biases about social class can shape clinical decisions without conscious intent. When a counselor notices differential treatment, the ethical response is to examine it, often through supervision or consultation, and correct its effect on services.

References: Sue & Sue (Counseling the Culturally Diverse) — Counselor awareness of own assumptions, values, and biases · ACA Code of Ethics — C.5 Nondiscrimination

☐ Approve ☐ Revise ☐ Retire Notes:

A counselor notices he feels unusually irritated and impatient with an older male client who reminds him of his own critical father. This reaction is BEST described as

A. transference by the client

Transference is the client's projection of past relationships onto the counselor; here the reaction is the counselor's.

B. countertransference ✓ KEY

The counselor's emotional reaction to the client, shaped by his own unresolved history, is countertransference.

C. resistance in the client

Resistance refers to client behavior that impedes change, not the counselor's feelings.

D. compassion fatigue

Compassion fatigue is cumulative exhaustion from trauma exposure, not a reaction tied to a specific personal history.

Rationale: Countertransference refers to the counselor's emotional reactions to a client, particularly those rooted in the counselor's own unresolved conflicts or past relationships. It is not inherently harmful, but unexamined countertransference can distort the counselor's perceptions and responses, so recognizing it is the first step to managing it.

References: Corey (Theory & Practice) — Psychoanalytic therapy: transference and countertransference

☐ Approve ☐ Revise ☐ Retire Notes:

A counselor whose sister died by overdose finds herself giving extra session time and her personal cell number to a young adult client in early recovery. What is the BEST course of action?

A. Keep the extra contact, since it clearly helps the client stay engaged

This keeps a boundary crossing driven by the counselor's needs rather than the client's welfare.

B. Transfer the client right away to avoid any further personal reactions

Abrupt transfer is premature; countertransference can often be managed and abrupt referral may harm the client.

C. Disclose the sister's death so the client understands her strong concern

This self-disclosure serves the counselor's needs and burdens the client.

D. Seek supervision on the reaction and restore usual session boundaries ✓ KEY

Recognizing the countertransference, processing it in supervision, and restoring boundaries protects the client and the work.

Rationale: Countertransference can show up as overinvolvement, rescuing, or boundary crossings that meet the counselor's needs. Effective management involves self-awareness, supervision or personal counseling, and returning to boundaries that serve the client. Referral is reserved for cases where the counselor cannot remain objective after these steps.

References: Corey (Theory & Practice) — Countertransference and the counselor as a person · ACA Code of Ethics — A.6.b Extending Conventional Boundaries

☐ Approve ☐ Revise ☐ Retire Notes:

A supervisor tells a trainee, "Every feeling you have toward a client, not just the conflicts from your own past, can be useful information about the client and the relationship." This statement reflects which view of countertransference?

A. Classical view, treating it as an obstacle to be eliminated

The classical (Freudian) view limits countertransference to the counselor's unresolved conflicts and treats it as interference.

B. Behavioral view, treating it as a learned response to extinguish

Countertransference is not framed as a conditioned response in behavioral theory; this distractor misattributes the concept.

C. Complementary view, limited to the client's projected roles

The complementary conception frames countertransference as the counselor's reaction to the interpersonal pull of the client's style, a narrower idea than all feelings toward the client.

D. Totalistic view, including all counselor reactions to clients ✓ **KEY**

The totalistic view defines countertransference as all of the counselor's emotional reactions and sees them as potentially useful data.

Rationale: The classical view, rooted in Freud, defines countertransference narrowly as the counselor's unconscious, conflict-based reactions to the client's transference and treats it as an impediment. The totalistic view broadens it to all of the counselor's emotional responses and treats them as a source of understanding. Contemporary integrative views keep the focus on conflict-based reactions while stressing that, once understood, they can inform the work.

References: Gelso & Hayes (Countertransference) — Ch. 1: classical, totalistic, complementary and integrative conceptions · Corey (Theory & Practice) — Psychoanalytic therapy: countertransference and its therapeutic use

☐ Approve ☐ Revise ☐ Retire Notes:

A college student says, "I've been hooking up with different people most weekends, and my roommate thinks I'm a mess." Which counselor response BEST reflects a nonjudgmental stance?

A. Your roommate may have a point, so let's look at the risks you're taking.

Siding with the roommate conveys the counselor's judgment before understanding the client's view.

B. Your roommate sees it one way. I'm curious how you feel about these weekends. ✓ **KEY**

This neither condemns nor endorses the behavior and invites the client's own perspective, which is nonjudgmental.

C. There's nothing wrong with that at all, and your roommate should not judge you.

Approval is also an evaluation; it imposes the counselor's values in the opposite direction.

D. What do you think is missing in your life that leads you to keep doing this?

This presumes the behavior signals a deficit, a judgment embedded in the question.

Rationale: A nonjudgmental stance means the counselor suspends both disapproval and approval and seeks to understand the client's experience from the client's frame of reference. Counselors avoid imposing their own values on clients, which allows clients to explore their choices without becoming defensive.

References: ACA Code of Ethics — A.4.b Personal Values

☐ Approve ☐ Revise ☐ Retire Notes:

A 70-year-old widower arrives early for his first session and seems nervous. Which counselor behavior BEST conveys warmth and respect?

A. Greet him by name, ask how he prefers to be addressed, and offer him a seat ✓ KEY

This shows genuine interest and respects his preferences and dignity, communicating warmth and respect.

B. Call him "sweetie" and speak slowly and loudly so he can follow along easily

Endearments and exaggerated speech are elderspeak; they are patronizing and disrespectful.

C. Hand him the intake forms and ask him to wait until his appointment time

This is procedural and distant, and it ignores his visible nervousness.

D. Tell him there is no need to be nervous because most clients feel fine later

This dismisses his feelings and offers reassurance in place of attunement.

Rationale: Warmth is conveyed through friendly, caring verbal and nonverbal behavior, and respect is shown by treating the client as a capable person with preferences and dignity. Asking how a client wishes to be addressed is a simple, culturally responsive way to communicate both, and avoiding ageist communication such as elderspeak is part of respect.

References: Gladding (Counseling: A Comprehensive Profession) — Building a counseling relationship: warmth and respect

☐ Approve ☐ Revise ☐ Retire Notes:

Q936 of 1000 · nce-s-cor-016 · Social and Cultural Diversity · awareness of power and privilege in the counseling relationship · medium · In review

A White counselor working with a Black client hears the client say, "I wasn't sure I could talk about racism at work with you." Which counselor response BEST shows awareness of power and privilege in the relationship?

A. I don't really see color, so you can talk to me about anything you need to here.

Color-blind statements deny the relevance of race and the client's concern, which is a microinvalidation.

B. I've had many Black clients, so I understand a lot about what you face at work.

Claiming understanding based on experience with other clients overstates the counselor's knowledge and dismisses the concern.

C. As a White counselor I may miss things, and I'd like to hear it if you're willing. ✓ KEY

This names the racial difference and the limits of the counselor's perspective, and invites the client's experience without pressure.

D. Let's focus on what you can control at work, since we can't change the people there.

Redirecting away from racism minimizes a systemic experience and avoids the topic the client raised.

Rationale: Counselors from privileged groups hold both professional and social power. Broaching racial and cultural differences directly and nondefensively, and acknowledging the limits of one's own perspective, signals that the client's experiences of oppression are welcome. Color-blind or minimizing responses are common microaggressions that weaken trust.

References: Sue & Sue (Counseling the Culturally Diverse) — Power, privilege, and microaggressions in counseling; color-blindness

☐ Approve ☐ Revise ☐ Retire Notes:

A 41-year-old man ordered to counseling after a road-rage incident sits down and says, "This is a joke. The other guy cut me off and I'm the one stuck here wasting my Thursday." Which counselor response BEST conveys empathy?

A. The court decided you need to be here, so let's make good use of the hour today.

This sides with the court and moves to the agenda without acknowledging the client's feelings, a subtractive response.

B. Getting angry on the road is risky, and that is what we need to look at together.

This confronts the behavior before the client feels heard, which tends to increase defensiveness in a mandated client.

C. You're frustrated, because it feels unfair that you're the one paying the price. ✓ **KEY**

This accurately reflects the feeling (frustration) and its meaning (unfairness) from the client's frame of reference without agreeing or arguing.

D. I hear that, but many people find these sessions useful once they give them a try.

"I hear that, but" discounts the feeling and shifts to persuasion, which is reassurance rather than empathy.

Rationale: Empathy with an angry or mandated client means communicating understanding of the client's feelings and perspective, not defending the referral source or confronting the behavior first. An accurate reflection lowers defensiveness and builds the alliance needed for later work on the behavior that led to the referral.

References: Gladding (Counseling: A Comprehensive Profession) — Empathy and working with reluctant or resistant clients

☐ Approve ☐ Revise ☐ Retire Notes:

A client says, "My sister borrowed money again and didn't pay it back. She's completely selfish, and anyone in my place would cut her off for good." Which counselor response communicates empathy WITHOUT signaling agreement with the client's judgment?

A. You feel hurt and fed up, and cutting her off seems like the only way to stop it. ✓ **KEY**

This reflects the client's feelings and how the situation looks to the client, conveying understanding without endorsing the conclusion that the sister is selfish.

B. She does sound selfish, and you'd be right to protect yourself from her by now.

This agrees with the client's evaluation of the sister, confusing empathy with agreement and taking sides.

C. Maybe your sister is struggling in ways that you might not know about yet, though.

Offering the sister's side challenges the client prematurely and communicates that the counselor does not understand the client.

D. Cutting off family is a big step, so let's think carefully before you decide.

This shifts to cautionary advice and skips reflecting the client's feelings, so it is not an empathic response.

Rationale: Empathy is understanding the client's world "as if" it were one's own while keeping the "as if" quality. It reflects the client's feelings and perceptions as the client experiences them; it neither agrees with nor disputes the client's judgments. Distinguishing empathy from agreement lets the counselor stay with the client without becoming an ally against third parties.

References: Corey (Theory & Practice) — Person-centered therapy: accurate empathic understanding and the "as if" quality

☐ Approve ☐ Revise ☐ Retire Notes:

A 29-year-old client who left a stable nursing job to work as a touring musician says she is happy with the choice, though money is tight. The counselor privately thinks the decision is unwise. Which approach BEST reflects unconditional positive regard and respect for client autonomy?

A. Point out the financial risks so the client can reconsider before it is too late

Steering the client toward the counselor's preferred choice imposes the counselor's values on a decision the client has not asked to revisit.

B. Accept her choice as hers to make and explore the goals she brings to counseling ✓ KEY

Valuing the client without conditions and following her own goals honors UPR and self-determination, even when the counselor would choose differently.

C. Praise the decision warmly so the client feels supported in following her dream

Praise is an evaluation too; UPR is acceptance of the person, not approval of choices, and the praise would be incongruent with the counselor's view.

D. Share the honest concern about the career change since genuineness matters most

Genuineness does not require voicing every personal opinion; volunteering disapproval of a lawful life choice imposes values.

Rationale: Unconditional positive regard means prizing the client as a person regardless of whether the counselor agrees with the client's choices. Combined with respect for self-determination, it means counselors do not impose personal values on clients' lawful life decisions and instead work on the goals clients bring.

References: Corey (Theory & Practice) — Person-centered therapy: unconditional positive regard and acceptance · ACA Code of Ethics — A.4.b Personal Values

☐ Approve ☐ Revise ☐ Retire Notes: _____

Rogers held that when a counselor provides unconditional positive regard, empathy, and congruence, clients move toward growth because of an innate drive to develop their capacities. He called this drive the

A. striving for superiority

Striving for superiority (or significance) is Adler's concept of the movement toward overcoming felt inferiority.

B. will to meaning

The will to meaning is Frankl's existential concept, central to logotherapy.

C. hierarchy of needs

The hierarchy of needs is Maslow's model of motivation, not Rogers' term for the growth drive.

D. actualizing tendency ✓ KEY

Rogers described the actualizing tendency as the organism's inherent tendency to develop its capacities in ways that maintain and enhance it.

Rationale: Person-centered theory rests on trust in the actualizing tendency, an inborn movement toward growth and fulfillment. The counselor does not direct change; instead, the core conditions create a climate in which the client's own actualizing tendency can operate and conditions of worth lose their hold.

References: Corey (Theory & Practice) — Person-centered therapy: the actualizing tendency

☐ Approve ☐ Revise ☐ Retire Notes: _____

A client describes years of failed fertility treatments. The counselor went through a similar loss and feels a wave of sadness. Which response BEST reflects genuineness and intentional use of self without inappropriate self-disclosure?

A. I'm moved hearing how much hope and heartbreak these years have held for you. ✓ **KEY**

The counselor shares an authentic, here-and-now reaction that is relevant to the client and keeps the focus on the client's experience.

B. I went through fertility treatments too, so I know exactly how painful this is.

This disclosure shifts focus to the counselor and claims identical experience, which can silence the client's own version of the loss.

C. Let's look at the coping strategies that have helped you through these years.

Moving straight to coping sidesteps the emotion in the room; the counselor masks her reaction rather than using it.

D. Many couples face this, and there are other paths to parenthood worth exploring.

Normalizing and problem-solving prematurely minimizes the grief and does not reflect a genuine, attuned response.

Rationale: Congruence means the counselor's outward expression matches inner experience, without a professional facade. Genuineness does not require telling one's personal history; intentional use of self means sharing reactions only when they serve the client. Disclosures of the counselor's own life should be brief, purposeful, and returned quickly to the client.

References: Corey (Theory & Practice) — Person-centered therapy: congruence; therapist self-disclosure

☐ Approve ☐ Revise ☐ Retire Notes:

During a video session, a client says, "On this screen it's hard to tell whether what I say really lands with you." Which counselor adjustment BEST supports genuineness in telehealth?

A. Ask more questions so the client can see the counselor is paying close attention

More questions do not convey the counselor's reactions and may feel like an interview rather than a genuine relationship.

B. Reassure the client that the counselor is listening carefully even if it is hard to see

Reassurance asks the client to trust without changing anything about how the counselor's presence comes across.

C. Put more reactions into words, since the camera filters out many nonverbal cues ✓ **KEY**

Verbalizing reactions compensates for the reduced nonverbal channel so the client can experience the counselor as present and authentic.

D. Suggest switching to phone sessions so that video issues stop being a distraction

Phone sessions remove even more nonverbal information and avoid the client's concern rather than addressing it.

Rationale: Video sessions reduce the nonverbal cues, such as posture, small facial changes, and eye contact, that usually convey a counselor's presence. To remain genuine and transparent, counselors can name their reactions more explicitly, look toward the camera, and check in about how the connection feels to the client.

References: Gladding (Counseling: A Comprehensive Profession) — Genuineness; technology and distance counseling

► **Check:** Telehealth genuineness guidance is general practice wisdom; confirm the cited Gladding chapter addresses technology-assisted counseling or swap the source.

☐ Approve ☐ Revise ☐ Retire Notes:

A 15-year-old brought to counseling by his mother crosses his arms and says, "She's the one who wanted this. I don't have anything to talk about." Which counselor response is MOST likely to begin building a working alliance?

A. Your mom is really worried about you, so it would help to tell me what has been going on.

This aligns the counselor with the parent's agenda and pressures the teen, weakening the bond.

B. Coming here wasn't your idea. I'm curious if anything would make this worth it to you. ✓ KEY

This acknowledges the teen's lack of choice and invites his own goals, building agreement on goals and the bond.

C. That's fine. We can just sit quietly until you decide you're ready to start talking.

Passive waiting leaves the teen without a reason to engage and does nothing to establish shared goals.

D. Most teens feel that way at first, but they usually end up liking counseling a lot.

Generalized reassurance dismisses his stated position and can sound like the counselor is selling something.

Rationale: Adolescents brought by parents often arrive reluctant. The alliance grows when the counselor acknowledges the teen's autonomy, separates from the parent's agenda, and invites the teen to name goals of his own. In Bordin's terms, this begins building agreement on goals and the bond.

References: Gladding (Counseling: A Comprehensive Profession) — Working alliance with reluctant clients and adolescents

☐ Approve ☐ Revise ☐ Retire Notes:

Q944 of 1000 · nce-s-cor-024 · Social and Cultural Diversity · awareness of power and privilege in the counseling relationship · hard · In review

A court-mandated client on probation asks at intake, "So whatever I say in here goes straight to my probation officer, right?" The client has signed a release limited to attendance and progress reports. What is the BEST response?

A. Reassure the client that sessions are confidential so that he will speak more freely

Promising full confidentiality is inaccurate given the signed release and would undermine trust when reports are sent.

B. Tell the client that the probation officer will decide what is shared from sessions

The scope of disclosure is set by the release and law, not by the officer; this also cedes the client's informed role.

C. Suggest that the client avoid sharing anything that could get him into legal trouble

This advises concealment and undermines the purpose of counseling instead of clarifying the actual limits.

D. Explain exactly what the release permits to be shared and invite questions about it ✓ KEY

Transparent, specific information about the limits of confidentiality acknowledges the power imbalance and supports informed participation.

Rationale: Mandated clients face a real power imbalance: the counselor reports to a system that holds consequences over them. Counselors address this by explaining clearly, at the start and as needed, what will and will not be disclosed and to whom. Honest transparency about the counselor's dual accountability is the foundation for trust in mandated work.

References: ACA Code of Ethics — B.1.d Explanation of Limitations

☐ Approve ☐ Revise ☐ Retire Notes:

In a first session, a counselor told a U.S.-born Korean American client, "Your English is excellent." The client became quiet for the rest of the hour. At the next session, what should the counselor do FIRST?

A. Wait to see whether the client brings up the comment before addressing it in session

Waiting puts the burden of raising the harm on the client and lets the rupture persist.

B. Explain that the remark was meant as a compliment and was not intended to offend

Defending intent centers the counselor and minimizes the impact on the client.

C. Name the comment, take responsibility for its impact, and invite the client's view ✓ **KEY**

Nondefensively acknowledging a microaggression and inviting the client's experience is the core of rupture repair with cultural humility.

D. Focus on the presenting problem so the session is not sidetracked by the earlier slip

Moving on avoids the rupture; unaddressed microaggressions are linked to weaker alliances and dropout.

Rationale: Remarks that assume an Asian American is a foreigner ("alien in one's own land") are a recognized microaggression. When a counselor commits one, cultural humility calls for owning the impact rather than defending intent, and openly inviting the client's reaction. Repairing such ruptures can strengthen the relationship.

References: Sue & Sue (Counseling the Culturally Diverse) — Racial microaggressions: alien in one's own land

☐ Approve ☐ Revise ☐ Retire Notes:

A second-generation Nigerian American counselor begins working with a 24-year-old woman who immigrated from Nigeria two years ago. The counselor thinks, "I grew up with this, so I already know what her parents expect of her." Which stance BEST reflects cultural humility?

A. Draw on shared heritage to explain the family expectations the client is facing

Using one's own background as a template assumes sameness and replaces the client's account with the counselor's.

B. Ask how she sees her family's expectations and stay open to differences from her own ✓ **KEY**

This treats the client as the expert on her experience and stays alert to differences in generation, region, ethnicity, and immigration history.

C. Refer her to a counselor of another culture to avoid any bias from the shared heritage

A shared background is not grounds for referral; the task is to examine assumptions, not avoid the client.

D. Disclose the shared background early so the client knows she will be understood

Promising understanding on the basis of shared heritage reinforces the assumption of sameness.

Rationale: Cultural humility applies even when counselor and client share a cultural background. Within-group differences in generation, acculturation, region, language, religion, and personal values can be large, and assuming sameness can lead to blind spots or overidentification. A humble, curious stance invites the client to define what her culture means to her.

References: Sue & Sue (Counseling the Culturally Diverse) — Within-group differences and the dangers of assumed similarity

☐ Approve ☐ Revise ☐ Retire Notes:

A counselor with strong religious convictions works with a client who wants help deciding whether to leave her faith community. The counselor notices a pull to encourage the client to stay. What is the MOST appropriate action?

A. Refer the client elsewhere, because the counselor's beliefs conflict with the goal

The ACA Code bars referral based solely on the counselor's personal values; the counselor must first work to bracket them.

B. Share the counselor's own beliefs openly so the client can weigh them in deciding

Offering personal religious views on the client's decision imposes the counselor's values.

C. Gently point out what the client may lose by leaving her supportive community

Emphasizing losses of only one option steers the client toward the counselor's preference.

D. Seek consultation, set aside personal views, and follow the client's own process ✓ **KEY**

Recognizing the pull, getting consultation, and keeping the focus on the client's goals reflect self-awareness and A.4.b.

Rationale: Self-awareness includes noticing when personal religious or political values press on clinical work. The ACA Code directs counselors to be aware of their own values and avoid imposing them, seeking training where they are at risk of doing so, and it prohibits referral based solely on the counselor's personal values.

References: ACA Code of Ethics — A.4.b Personal Values · ACA Code of Ethics — A.11.b Values Within Termination and Referral

☐ Approve ☐ Revise ☐ Retire Notes:

A counselor whose parents had a bitter divorce works with a 16-year-old caught in a custody dispute. She finds herself angry at the client's father and urging the teen to "stand up to him." This pattern is BEST described as

A. overidentification ✓ **KEY**

The counselor is merging her own history with the client's, losing objectivity and taking sides, a form of countertransference.

B. parallel process

Parallel process refers to dynamics in counseling being replicated in supervision, or the reverse.

C. advocacy

Advocacy addresses systemic barriers on behalf of a client; it is not driven by the counselor's unresolved personal history.

D. client transference

Transference is the client's projection onto the counselor; here the reaction originates in the counselor.

Rationale: Overidentification occurs when a counselor sees the client's situation through the lens of his or her own experience, losing the "as if" boundary of empathy. It is a common form of countertransference that can lead the counselor to take sides or push the client toward choices the counselor wished for, so it calls for self-reflection and supervision.

References: Corey (Theory & Practice) — The counselor as a person: countertransference and overidentification

► **Check:** Confirm Corey (Theory & Practice) discusses overidentification by name; Corey's Issues & Ethics may be the better home for it.

☐ Approve ☐ Revise ☐ Retire Notes:

A client says, "I still smoke weed most nights to fall asleep. Go ahead and give me the lecture." Which counselor response BEST reflects a nonjudgmental stance?

A. No lecture. I'd like to understand what it does for you and how it's working. ✓ KEY

This sets aside evaluation and invites the client's own view of benefits and costs, a nonjudgmental and collaborative stance.

B. I won't lecture, but you know nightly use can make your sleep worse over time.

The "but" delivers the lecture anyway, conveying judgment and likely increasing defensiveness.

C. It sounds like you already know it's a problem, or you wouldn't expect a lecture.

This interprets the comment as an admission of a problem, imposing the counselor's conclusion.

D. Lots of people use cannabis for sleep now, so it isn't a big concern for me.

Dismissing the topic is approval, another kind of judgment, and closes off exploration.

Rationale: A nonjudgmental stance suspends both disapproval and approval in favor of curiosity about the client's experience. With substance use, exploring what the substance does for the client and how it is working invites honest self-evaluation and is consistent with motivational interviewing's spirit of acceptance.

References: Miller & Rollnick (MI) — Spirit of MI: acceptance (absolute worth, autonomy support)

☐ Approve ☐ Revise ☐ Retire Notes:

A 33-year-old single mother mentions in passing that she does sex work to pay her bills, then says she came to counseling because of anxiety about her son's problems at school. What should the counselor do?

A. Shift the focus to her work, since it is likely the underlying source of her anxiety

Assuming the work is the real problem overrides the client's stated goal and reflects the counselor's judgment.

B. Follow her stated concern while leaving room to discuss work or safety if she wishes ✓ KEY

Respecting the client's agenda without ignoring possible safety needs reflects a nonjudgmental, client-centered stance.

C. Give her referrals for job training so she has other options to support her family

Unrequested job referrals imply her work must change, imposing values she did not ask the counselor to apply.

D. Avoid any mention of her work so that she does not feel judged in this session

Treating the topic as unspeakable can also signal judgment and may miss safety concerns she wants to raise.

Rationale: A nonjudgmental stance means the counselor does not treat a stigmatized behavior as the presumed problem. The client sets the agenda, and the counselor stays open to discussing work, health, or safety when the client chooses, without pathologizing or avoiding the topic.

References: ACA Code of Ethics — A.4.b Personal Values

☐ Approve ☐ Revise ☐ Retire Notes:

A client arrives sobbing and breathing rapidly after learning she is being evicted at the end of the week. Which counselor response BEST conveys warmth in this moment?

A. Hand her a list of housing resources so she can start making calls right away
Resources may help later, but moving to tasks while she is flooded ignores her emotional state.

B. Ask her to describe the eviction notice in detail to see what her options are
Detailed fact-gathering before she is calmer is task-focused and can add to her distress.

C. Tell her firmly that panicking will not help and that she needs to calm down
A directive to stop panicking is cold and dismissive, and it rarely reduces distress.

D. Sit with her, speak in a calm, steady voice, and acknowledge how scared she is ✓ **KEY**
A calm, caring presence and acknowledgment of her fear communicate warmth and help her regulate before problem-solving.

Rationale: Warmth is communicated largely through nonverbal behavior, including tone of voice, pacing, and attentive presence, together with acknowledgment of the client's feelings. In acute distress, a warm and steady presence helps the client regulate enough to engage in the practical problem-solving that follows.

References: Gladding (Counseling: A Comprehensive Profession) — Warmth and nonverbal communication in the helping relationship

☐ Approve ☐ Revise ☐ Retire Notes:

Q952 of 1000 · nce-s-cor-032 · Social and Cultural Diversity · awareness of power and privilege in the counseling relationship · medium · In review

A counselor is meeting with a Spanish-speaking client through a trained professional interpreter. Which practice BEST preserves the client's voice and respects the counseling relationship?

A. Ask the interpreter to summarize what the client says to keep sessions brief
Summaries filter the client's words through the interpreter and lose emotional content.

B. Ask the client's bilingual teenage son to interpret, since the family trusts him
Family members, especially children, compromise accuracy and confidentiality and shift family power roles.

C. Speak directly to the client in the first person rather than to the interpreter ✓ **KEY**
Addressing the client directly keeps her at the center of the relationship and the interpreter in a conduit role.

D. Speak with the interpreter after sessions to learn the cultural meaning of her words
Relying on the interpreter as cultural expert behind the client's back can sideline her own account of her experience.

Rationale: Interpreted sessions add a third person and can shift power away from the client. Best practice is to use a trained interpreter rather than family, brief the interpreter beforehand, and speak directly to the client in the first person so that the relationship remains between counselor and client.

References: ACA Code of Ethics — A.2.c Developmental and Cultural Sensitivity · Sue & Sue (Counseling the Culturally Diverse) — Language barriers and use of interpreters

☐ Approve ☐ Revise ☐ Retire Notes:

A counselor who sincerely holds egalitarian values reviews her recorded sessions for supervision. She notices that with her Latino clients she offers fewer reflections, interrupts more often, and moves to advice sooner than with her White clients. She had not been aware of the difference. This pattern BEST illustrates

A. implicit bias operating outside her conscious awareness ✓ KEY

Automatic, unintended differences in behavior toward a group, despite consciously held egalitarian beliefs, are the hallmark of implicit (unconscious) bias.

B. explicit prejudice that she consciously holds but conceals

Explicit prejudice is consciously endorsed. The counselor sincerely holds egalitarian values and was unaware of the pattern.

C. cultural encapsulation produced by a narrow training program

Wrenn's cultural encapsulation describes defining reality through one's own cultural lens. It does not specifically explain unaware, group-specific differences in her microskills.

D. countertransference rooted in conflicts from her own family

Countertransference stems from the counselor's personal history with significant others. Nothing suggests a family conflict; the pattern tracks client ethnicity.

Rationale: Implicit bias refers to attitudes and associations that operate automatically and outside awareness, so well-intentioned counselors can treat groups differently without meaning to. Sue and Sue stress that good intentions do not protect against unintentional bias; recorded sessions, supervision and ongoing self-examination are how counselors detect it.

References: Sue & Sue (Counseling the Culturally Diverse) — Implicit bias and unintentional racism in well-intentioned helping professionals

☐ Approve ☐ Revise ☐ Retire Notes:

A 47-year-old client in a larger body seeks counseling for grief after his mother's death. He has not raised any concern about his weight. Over three sessions, the counselor finds herself suggesting exercise plans and asking about his eating. Which action BEST reflects counselor self-awareness?

A. Add a weight-loss goal to the plan, since weight affects mood and grief

This imposes the counselor's agenda on a client who came for grief and acts on the bias rather than examining it.

B. Notice her assumptions about his weight and refocus on the grief he brought ✓ KEY

Recognizing the counselor-driven assumption and returning to the client's own goal is self-aware, client-centered practice consistent with ACA A.4.b.

C. Refer him to a dietitian so his eating habits can be addressed by an expert

A referral the client did not request still acts on the counselor's assumption about his body rather than on his presenting concern.

D. Ask him directly whether his weight played a part in his mother's death

This question extends the counselor's weight focus into his grief and may shame him; it reflects the bias rather than awareness of it.

Rationale: Self-awareness means noticing when personal values or assumptions, including those about body size, are steering the work. ACA Standard A.4.b directs counselors to avoid imposing their own values and to respect the client's goals. The counselor should examine where the weight focus came from and return to the grief the client brought.

References: ACA Code of Ethics — A.4.b Personal Values · Corey (Theory & Practice) — The counselor as a person: self-awareness and personal values

☐ Approve ☐ Revise ☐ Retire Notes:

The week after a national election, a client spends several minutes praising a candidate the counselor strongly opposes. The counselor notices her jaw tightening. The client then asks, "You agree with me, right?" Which counselor response BEST reflects self-aware, ethical practice?

A. Honestly, I see it differently, but I respect your right to your own view.

Sharing political disagreement shifts focus to the counselor and risks imposing values; congruence does not require disclosing every opinion.

B. I do. A lot of people feel that way, so I can see why you are hopeful now.

Feigning agreement is incongruent and also brings the counselor's supposed politics into the session.

C. I keep my politics out of here, but I want to hear what this means for you. ✓ KEY

This gently declines to disclose, keeps the focus on the client, and invites exploration, which manages the counselor's reaction without acting on it.

D. Since we differ on this, another counselor might be a better fit for you.

ACA A.11.b cautions against referral based solely on the counselor's personal values; a political difference is not grounds for referral.

Rationale: Counselors are expected to notice their own value reactions, including political ones, and keep them from steering the session (ACA A.4.b). Self-disclosure of opinions should serve the client, and a politically charged question is usually best met by declining warmly and exploring its meaning for the client. Referral solely because of a values difference is discouraged under A.11.b.

References: ACA Code of Ethics — A.4.b Personal Values · ACA Code of Ethics — A.11.b Values Within Termination and Referral

☐ Approve ☐ Revise ☐ Retire Notes:

At a community mental health center, a 36-year-old client referred by his caseworker glares at the counselor and says, "You're just another paid stranger. You don't actually care what happens to me." Which counselor response BEST conveys warmth and respect?

A. I have years of training, so you can trust that I know how to help you.

Defending credentials centers the counselor and argues with the client's experience rather than respecting it.

B. I can't work with you if you speak to me in that tone during sessions.

The client is distrustful, not abusive. An early limit on tone reads as punitive and confirms his expectation that no one cares.

C. Let's set that aside and get through the intake questions for today.

Ignoring the statement dismisses the client's concern and treats him as paperwork, the opposite of respect.

D. Sounds like people haven't earned your trust. I'd like to, at your pace. ✓ KEY

This reflects the meaning behind the hostility without defensiveness and communicates a respectful wish to earn trust over time.

Rationale: Warmth and respect are tested most when a client is hostile. A nondefensive response that reflects the client's distrust, treats it as understandable, and leaves the client in control of the pace communicates care. Defending oneself, setting premature limits, or ignoring the comment tend to confirm the client's expectations.

References: Gladding (Counseling: A Comprehensive Profession) — Warmth, respect and working with reluctant or resistant clients

☐ Approve ☐ Revise ☐ Retire Notes:

A 60-year-old Vietnamese immigrant father comes to counseling about conflict with his teenage son. In the first session he says, "You are the expert. Please tell me what I should do." Which counselor approach BEST combines respect with responsiveness to his request?

A. Offer some concrete suggestions and structure, checking how each idea fits his family ✓ **KEY**

Honoring his expectation of an active, expert helper while asking how ideas fit his family respects both his cultural frame and his autonomy.

B. Explain that counselors do not give advice and that he must find his own answers

A strictly nondirective stance may be experienced as unhelpful or as a lack of competence, and it dismisses his stated preference.

C. Tell him exactly what rules to set for his son, since he asked for clear direction

Directing without his input ignores his knowledge of his own family and values; structure should still be collaborative.

D. Ask him to first explore his deeper feelings about his son before any guidance

Insisting on emotional exploration before addressing his practical request disregards what he came for and may lead to early dropout.

Rationale: Sue and Sue note that many clients, especially those from cultures that value hierarchy and practical help, expect the counselor to take an active, directive role, and that a purely nondirective style can lead to early termination. Respect means meeting the client's expectations with structure and concrete input while still checking that suggestions fit the client's values.

References: Sue & Sue (Counseling the Culturally Diverse) — Culturally appropriate helping styles: directive versus nondirective approaches

☐ Approve ☐ Revise ☐ Retire Notes:

A 38-year-old Army veteran referred for military sexual trauma tells the counselor in her second session, "I'm not ready to talk about what happened. I don't know if I ever will be." Which counselor response BEST respects the client's pace?

A. Avoiding it keeps symptoms going, so let's try to cover the details today.

Pressing for details before the client consents overrides her choice and can feel retraumatizing.

B. That's okay. You decide what we talk about and when, and we can go slowly. ✓ **KEY**

This affirms her control over disclosure, a core trauma-informed principle, and conveys respect for her readiness.

C. Could you give me just a short outline so I know what we're working with?

Asking for an outline right after she declines still pressures disclosure and disregards her stated limit.

D. Unless you talk about it, I'm not sure that counseling can help you much.

Making help conditional on disclosure is coercive and undermines safety and trust.

Rationale: Respecting a client's pace is especially important with trauma survivors. SAMHSA's trauma-informed principles emphasize safety, trustworthiness, and client voice and choice; counselors let clients control when and how much they disclose, building stabilization and trust first.

References: SAMHSA TIP 57 — Trauma-informed principles: safety, choice and client control over disclosure

☐ Approve ☐ Revise ☐ Retire Notes:

A 26-year-old client in outpatient treatment for opioid use returns after relapsing following 90 days of abstinence. He looks down and says, "You must be so disappointed in me." Which counselor response BEST demonstrates unconditional positive regard?

A. I'm a bit disappointed, but I know you can do better than this next time.

This makes the counselor's regard contingent on his behavior, which is the opposite of unconditional positive regard.

B. Relapse is just part of recovery, so it's really not a big deal at all.

This minimizes a serious event and offers reassurance rather than acceptance of the person and his feelings.

C. My regard for you hasn't changed. I'm glad you came back to talk about it. ✓ **KEY**

This separates his worth from the relapse and welcomes him back, communicating acceptance that does not depend on his behavior.

D. Let's go over what you did wrong so this does not happen to you again.

Opening with what he did wrong frames the session around judgment; relapse review can come later from a stance of acceptance.

Rationale: Unconditional positive regard is acceptance of the client as a person, not approval of every behavior, and it does not rise and fall with the client's performance. After a relapse, communicating steady regard reduces shame and makes honest exploration of what happened more likely.

References: Corey (Theory & Practice) — Person-centered therapy: unconditional positive regard and acceptance

☐ Approve ☐ Revise ☐ Retire Notes:

According to Carl Rogers, incongruence between a person's self-concept and actual experience develops primarily when the person

A. fails to resolve unconscious conflicts between the id and superego

This is a psychoanalytic explanation of conflict, not Rogers's account of incongruence.

B. learns maladaptive behavior patterns through repeated reinforcement

This is a behavioral explanation; Rogers focused on the self-concept and conditions of worth.

C. holds irrational beliefs that they must win approval from others

Irrational beliefs about needing approval belong to Ellis's REBT, not person-centered theory.

D. internalizes conditions of worth set by significant others ✓ **KEY**

Rogers held that conditional regard from significant others leads people to internalize conditions of worth, which produces incongruence.

Rationale: Rogers proposed that when love and approval are conditional, people internalize conditions of worth and deny or distort experiences that do not fit them, creating incongruence. Unconditional positive regard in counseling counters this by allowing clients to accept all of their experience without losing the counselor's regard.

References: Corey (Theory & Practice) — Person-centered therapy: conditions of worth and incongruence

☐ Approve ☐ Revise ☐ Retire Notes:

In person-centered theory, a counselor who is congruent is one who

A. shares every thought and feeling with the client as soon as it arises

Congruence does not mean impulsively disclosing all reactions; expression should be appropriate and in the client's service.

B. is aware of inner experience and expresses it authentically when fitting ✓ **KEY**

Congruence is the match between the counselor's inner experience, awareness of it, and outward expression, without a professional facade.

C. hides personal reactions to maintain a neutral and professional stance

Hiding behind a professional facade is the opposite of congruence in Rogers's view.

D. mirrors the client's emotional tone and posture to build rapport fast

Mirroring is a rapport technique; it can be done without any genuineness at all.

Rationale: Rogers described congruence, or genuineness, as the counselor being real: inner experience, awareness and communication are consistent, with no facade. Corey notes that this does not mean impulsively sharing every feeling; genuine expression is appropriate and serves the relationship.

References: Corey (Theory & Practice) — Person-centered therapy: congruence or genuineness

☐ Approve ☐ Revise ☐ Retire Notes:

A 31-year-old client who says coworkers avoid her asks, "Be honest. Do you find me annoying? Everyone else does." In their sessions, the counselor has felt irritated when the client cuts her off mid-sentence. Which response BEST demonstrates congruence in the client's service?

A. Not at all. I truly enjoy every single minute of our time together in here.

This reassurance contradicts the counselor's actual experience, so it is incongruent and withholds useful feedback.

B. Honestly, yes. You can be pretty exhausting for most people to be around.

This is a global, hurtful judgment rather than a specific, caring observation; genuineness is not the same as bluntness.

C. At times I feel cut off when you jump in. I wonder if others feel that too. ✓ **KEY**

This honestly shares a specific, behavior-based reaction in the here and now and links it to the client's concern, which is genuine and therapeutic.

D. What makes you want to ask me that question at this point in our work?

Exploring the question can be useful, but used here it sidesteps a direct, sincere request for honest feedback.

Rationale: Congruence means the counselor's responses match inner experience, but it is expressed with care and for the client's benefit. Sharing a specific, behavioral reaction in the moment (immediacy) gives the client accurate interpersonal feedback she may not get elsewhere. False reassurance is incongruent, and blunt global judgments are not genuineness but a failure of respect.

References: Corey (Theory & Practice) — Person-centered therapy: congruence; appropriate expression of counselor reactions

☐ Approve ☐ Revise ☐ Retire Notes:

A clinic director wants an early indicator of which clients are likely to benefit from counseling, regardless of the counselor's theoretical approach. Based on psychotherapy outcome research, which measure is the MOST useful early predictor?

A. The specific theoretical orientation the counselor practices

Comparative outcome research finds that bona fide approaches produce broadly similar outcomes, so orientation is a weak predictor.

B. A demographic match between the counselor and the client

Demographic matching has shown inconsistent, generally small effects on outcome compared with the alliance.

C. The total length of the initial intake assessment session

Intake length is not an established predictor of counseling outcome.

D. The client's rating of the working alliance in early sessions ✓ KEY

Meta-analyses find that the alliance, especially as rated by the client early in treatment, predicts outcome across approaches.

Rationale: The working alliance is one of the most consistently replicated predictors of counseling outcome. Meta-analyses (e.g., Horvath and colleagues; Flückiger and colleagues) show a moderate, reliable relationship between alliance and outcome across theoretical approaches, and alliance ratings taken early in treatment, often by the third to fifth session, already predict results.

References: Gladding (Counseling: A Comprehensive Profession) — The working alliance and common factors in counseling outcome

► **Check:** Alliance-outcome meta-analytic findings (Horvath et al., 2011; Flückiger et al., 2018) are not themselves in the source library; SME should confirm the Gladding locator covers early alliance as an outcome predictor.

☐ Approve ☐ Revise ☐ Retire Notes:

In the third session of couples counseling, Dana interrupts her wife and says to the counselor, "Every time she talks, you nod along. You're clearly on her side." What should the counselor do FIRST?

A. Acknowledge Dana's experience, invite her to say more, and check in with both partners ✓ KEY

Taking the perceived split seriously and exploring it openly with both partners repairs the alliance with Dana without abandoning her wife.

B. Reassure Dana that counselors stay neutral, then return to where the session left off

Reassurance without exploration dismisses Dana's experience and leaves the split alliance unrepaired.

C. Explain to Dana that her wife's points have simply been clearer and more specific

This confirms Dana's perception that the counselor is siding with her wife and deepens the split.

D. Schedule separate sessions with Dana alone to rebuild her trust before joint work

Moving to private sessions before addressing the issue together can create new imbalances and secrets and delays repair in the room.

Rationale: In couple and family counseling the counselor holds an alliance with each member, and a split alliance, where one member feels less allied with the counselor, is linked to poorer engagement and dropout. The counselor should address the rupture openly and nondefensively in session, maintaining balanced (multipartial) alliances with all members.

References: Friedlander et al. (Alliances in Couple & Family Therapy) — Split alliances and SOFTA safety/emotional connection; repairing in-session ruptures · Goldenberg (Family Therapy) — Therapeutic alliance in couple and family therapy; balanced alliances with each member

☐ Approve ☐ Revise ☐ Retire Notes:

A 50-year-old Diné (Navajo) client being seen for anxiety mentions that her family is arranging a ceremony with a traditional healer next month. Which counselor response BEST reflects cultural humility?

A. Suggest she wait on the ceremony until counseling has had a chance to work

This ranks the counselor's approach above the client's healing tradition and imposes the counselor's worldview.

B. Describe what she has read about Navajo ceremonies to show her knowledge

Taking an expert stance on the client's culture is the opposite of humility and may be inaccurate for this client and family.

C. Ask what the ceremony means to her and how she wants it to fit with counseling ✓ **KEY**

This positions the client as the expert on her culture and invites collaboration between traditional and counseling approaches.

D. Note it as possible magical thinking to explore further in the assessment

Pathologizing a culturally normative healing practice reflects cultural bias, not humility.

Rationale: Cultural humility is an other-oriented, lifelong stance of self-reflection in which the counselor treats the client as the expert on her own culture and attends to power differences. Sue and Sue encourage counselors to respect and, where the client wishes, collaborate with Indigenous healing practices rather than dismiss or pathologize them.

References: Sue & Sue (Counseling the Culturally Diverse) — Counseling Indigenous clients; respecting traditional and Indigenous healing · MSJCC (Ratts et al., 2016) — Counseling relationship: client worldview and cultural healing practices

☐ Approve ☐ Revise ☐ Retire Notes:

A 19-year-old says, "I dropped out of college to stream video games full time. My parents keep calling me lazy." Which counselor response BEST reflects a nonjudgmental stance?

A. Dropping out this young could really limit your options later in life.

This evaluates the decision as a mistake and aligns the counselor with a negative judgment.

B. Your parents may have a point, since streaming is a hard way to earn money.

Siding with the parents judges the client's choice and is likely to harm trust.

C. Good for you. Following your passion is the right call at your age, I think.

Approval is also a judgment; a nonjudgmental stance neither condemns nor endorses the choice.

D. That's a big change. What drew you to streaming, and how is it going so far? ✓ **KEY**

This acknowledges the change and invites the client's own perspective with curiosity, neither praising nor criticizing.

Rationale: A nonjudgmental stance means suspending evaluation of the client's choices, positive or negative, so the client can explore them openly. Curious, open-ended responses keep the focus on the client's experience; both criticism and praise place the counselor's values at the center.

References: ACA Code of Ethics — A.4.b Personal Values

☐ Approve ☐ Revise ☐ Retire Notes:

A client who works two hourly jobs has missed two sessions after short-notice shift changes. She apologizes repeatedly and seems afraid of being dropped. The agency discharges clients after three missed sessions. Which counselor response BEST reflects awareness of power and privilege?

A. Name the shift-work barrier and explore flexible times or telehealth together ✓ KEY

This recognizes a structural barrier rather than blaming the client and uses the counselor's power to adapt services with her.

B. Remind her of the attendance policy and that one more absence ends services

Leading with the policy uses institutional power punitively and ignores the economic constraints driving the absences.

C. Interpret the missed sessions as ambivalence about change to explore in session

Framing a work-schedule barrier as resistance pathologizes the client and overlooks her circumstances.

D. Suggest she ask her manager for a fixed schedule so she can attend reliably

This assumes a level of workplace control that hourly workers often lack, reflecting the counselor's own privileged frame.

Rationale: The MSJCC ask counselors to recognize how privileged and marginalized statuses, including socioeconomic status, shape the counseling relationship and access to services. Counselors hold institutional power and can use it to reduce barriers rather than interpret structural constraints as client resistance.

References: MSJCC (Ratts et al., 2016) — Counseling relationship; privileged and marginalized statuses and institutional barriers

☐ Approve ☐ Revise ☐ Retire Notes:

A counselor working with a 68-year-old recently widowed man, with his permission, has begun calling his bank, drafting his letters, and booking his appointments. She thinks warmly, "He would be lost without me." Which risk does this pattern MOST directly create?

A. Turning the client's gratitude into hostile negative transference

Nothing in the scenario points to hostility; the more direct risk lies in what the client stops doing for himself.

B. Fostering dependence that undermines the client's own competence ✓ KEY

Rescuing driven by the counselor's need to be needed takes over tasks the client could learn to do, encouraging dependence and eroding self-efficacy.

C. Making termination easier because the alliance is so strong

Dependence typically makes termination harder, not easier.

D. Causing the counselor to grow emotionally detached from him

The pattern reflects overinvolvement, not detachment.

Rationale: Countertransference can be positive as well as negative. Corey describes the counselor's need to be needed and rescuing as countertransference that leads counselors to do for clients what clients can do for themselves. The result is dependence and reduced client self-efficacy, which supervision can help the counselor recognize.

References: Corey (Theory & Practice) — Countertransference: the need to be needed and rescuing clients

☐ Approve ☐ Revise ☐ Retire Notes:

A counselor tells a 45-year-old Mexican American client, "It must help to have your big family around you right now." The client replies, "Actually, my family is part of the stress. My friends from work are who I lean on." Which counselor response BEST reflects cultural humility?

A. Families can be complicated, but they usually come through in a real crisis.

This holds on to the counselor's assumption about family support and quietly dismisses the client's correction.

B. I'm sorry to hear that. Maybe we can work on repairing things with them.

This sets a family-repair goal the client did not ask for and still treats family closeness as the expected norm.

C. Thank you for correcting me; I assumed. Tell me about who supports you. ✓ **KEY**

The counselor owns the assumption without defensiveness, thanks the client, and lets the client define her own support system.

D. That surprises me, since family is so central in Mexican American culture.

This applies a group generalization over the individual's own account, which is stereotyping rather than humility.

Rationale: Cultural humility treats the client as the expert on how culture shows up in her life. When a client corrects an assumption, the humble response is to accept the correction openly, acknowledge the error, and get curious about the client's actual experience. Group-level knowledge can inform hypotheses but cannot override what an individual client says about herself.

References: Sue & Sue (Counseling the Culturally Diverse) — Within-group differences; avoiding stereotyping in culturally responsive practice

☐ Approve ☐ Revise ☐ Retire Notes:

A Filipina American counselor begins working with a 58-year-old White, heterosexual, Christian man who owns a hardware store in a small farming town and is struggling after his business partner died. The counselor thinks, "His culture is the mainstream one, so there isn't much cultural material to explore." Which stance BEST reflects cultural humility?

A. Explore how his faith, rural roots, class and ideas of manhood shape his grief ✓ **KEY**

Everyone has a culture. Inviting him to describe how faith, place, class and gender norms shape his loss applies humility to a client from dominant groups.

B. Focus on grief alone, because culture matters mainly for marginalized clients

This treats dominant-group identity as culture-free, the very assumption the counselor should be questioning.

C. Rely on her knowledge of small-town life to anticipate his values and beliefs

Presuming to know his values from group knowledge is the opposite of the other-oriented, open stance humility requires.

D. Begin by discussing her own ethnic identity so he sees what culture involves

Centering the counselor's identity at the outset shifts focus away from the client and does not explore his worldview.

Rationale: Cultural humility is not reserved for clients from marginalized groups. Members of dominant groups also hold cultural identities, such as religion, regional and rural culture, social class and gender-role norms, that shape how they experience distress and help-seeking. Treating the majority culture as a neutral default is a form of cultural encapsulation; a humble counselor stays curious about every client's worldview.

References: Sue & Sue (Counseling the Culturally Diverse) — Everyone has a culture; Whiteness as an invisible default and cultural encapsulation · MSJCC (Ratts et al., 2016) — Client worldview: privileged and marginalized client identities

☐ Approve ☐ Revise ☐ Retire Notes:

Which description BEST captures cultural humility as distinct from cultural competence?

A. A body of knowledge about specific groups that a counselor masters in training

Mastering group-specific knowledge is closer to the knowledge component of cultural competence, not humility.

B. A credential earned after completing a set number of multicultural CE hours

Humility is an ongoing stance, not an endpoint that can be certified by hours of training.

C. A technique of matching clients with counselors who share their background

Matching may help some clients, but it is a service-delivery choice, not the counselor's own stance of humility.

D. A lifelong process of self-reflection, openness, and attention to power ✓ KEY

Cultural humility is defined as an ongoing commitment to self-reflection and self-critique, openness to the client's view, and redressing power imbalances.

Rationale: Cultural humility, a term introduced by Tervalon and Murray-Garcia, emphasizes lifelong self-evaluation and critique, an other-oriented openness to the client's cultural identity, and attention to power imbalances. It complements competence: rather than an endpoint of mastery, it is a continuing way of being with clients that keeps the counselor learning.

References: Sue & Sue (Counseling the Culturally Diverse) — Cultural humility versus cultural competence; lifelong self-reflection

► **Check:** Tervalon & Murray-Garcia (1998) originated the term; confirm the cited Sue & Sue edition discusses cultural humility in these terms.

☐ Approve ☐ Revise ☐ Retire Notes:

A 44-year-old man says, "I've been seeing someone else for six months. My wife has no idea. You probably think I'm a terrible person now." Which counselor response BEST reflects a nonjudgmental stance?

A. I don't think that, but you'll need to tell your wife before we can go on.

Making disclosure a condition of counseling imposes the counselor's values and sets an agenda the client has not chosen.

B. You expected me to judge you. I'd like to understand what's happening for you. ✓ KEY

This reflects his fear of judgment and conveys acceptance and curiosity without endorsing or condemning the affair.

C. Affairs are very common, so you really shouldn't feel bad about this at all.

Normalizing and reassuring takes a stance on the behavior and dismisses whatever guilt or conflict he may feel.

D. How do you think your wife would feel if she found out about all of this?

Though phrased as a question, it implies moral evaluation and shifts focus from his experience to an imagined accusation.

Rationale: A nonjudgmental stance means suspending moral evaluation so the client can explore his experience openly. It neither condemns nor approves the behavior. ACA Standard A.4.b asks counselors to avoid imposing their own values; responding to the client's fear of judgment and inviting exploration keeps the focus on his goals.

References: ACA Code of Ethics — A.4.b Personal Values

☐ Approve ☐ Revise ☐ Retire Notes:

A 62-year-old client laid off from a factory job says, "Immigrants took all the jobs around here. People like that are ruining this country." The counselor, herself the daughter of immigrants, feels a flash of anger. Which response BEST reflects a nonjudgmental stance while staying ethical?

A. Losing that job hit hard. Tell me what these months have been like for you. ✓ **KEY**

This attends to the loss and fear beneath the statement, neither endorsing nor attacking his view, and keeps the focus on his experience.

B. I need to tell you that my own parents were immigrants, and that remark hurt.

Disclosing her hurt at this point centers the counselor's reaction and is likely to rupture the relationship before it forms.

C. The research shows immigrants don't really take jobs away from local workers.

Debating the facts turns the session into an argument and signals judgment rather than understanding.

D. It sounds like you're right that outsiders are causing a lot of your problems.

Agreeing with the generalization abandons neutrality and reinforces a blaming belief rather than exploring it.

Rationale: Nonjudgmental does not mean agreeing. The counselor sets aside her reaction in the moment, monitors it, and responds to the client's underlying experience of loss. Consistent with ACA A.4.b, she avoids imposing her values; she can process her own anger in supervision and later explore how his beliefs affect his goals.

References: ACA Code of Ethics — A.4.b Personal Values · Corey (Theory & Practice) — The counselor as a person: personal values and dealing with difficult client views

☐ Approve ☐ Revise ☐ Retire Notes:

A 67-year-old woman meeting by telehealth after eight months of sobriety says, "I drank at my granddaughter's wedding. I blew everything. I'm back at zero." Which counselor response BEST reflects a nonjudgmental stance?

A. A wedding was a risky place to be. Why didn't you use your coping plan there?

The "why didn't you" question carries blame and is likely to deepen her shame.

B. Well, eight months is good, but you'll need to start counting from day one.

This confirms her all-or-nothing view of the lapse and minimizes the progress she has made.

C. You're being hard on yourself. Let's look at what led up to it, together. ✓ **KEY**

This acknowledges her self-criticism without blame and invites shared, curious review of the lapse as something to learn from.

D. I'm a little disappointed, but I know you can get back on track this week.

Voicing disappointment adds the counselor's evaluation to her shame, even with encouragement attached.

Rationale: A nonjudgmental response to a lapse counters the abstinence violation effect, the guilt and "I've blown it" thinking that can turn a single lapse into a full relapse. Treating the lapse as information about high-risk situations, rather than as moral failure, keeps the client engaged and supports recovery.

References: Marlatt & Donovan (Relapse Prevention) — Abstinence violation effect; lapse versus relapse

☐ Approve ☐ Revise ☐ Retire Notes:

A middle school counselor receives a 13-year-old sent by the assistant principal after a hallway fight. The student slumps in the chair and says, "Just write me up already. You all work for the office." Which counselor response BEST reflects awareness of power in this setting?

A. Remind the student that cooperating now will help keep the consequences lighter

Using discipline as leverage confirms that the counselor is an arm of the office and uses institutional power coercively.

B. Wait silently until the student is ready to explain what happened in the fight

Silence without explanation leaves the student's assumption about the counselor's role unaddressed.

C. Tell the student that everything said here will stay strictly between the two of them

Promising absolute confidentiality is inaccurate in a school setting and could later break trust.

D. Explain her role apart from discipline, what she would share, and offer choices ✓ KEY

Clarifying her role and the limits of confidentiality, and giving the student some control, directly addresses the power imbalance the student names.

Rationale: In schools, courts and other mandated settings, clients often see counselors as agents of the institution. Naming the counselor's role honestly, explaining what will and will not be shared, and offering real choices reduces the power differential and builds trust. Overpromising confidentiality or leveraging consequences each misuse the counselor's position.

References: MSJCC (Ratts et al., 2016) — Counseling relationship: power, privilege and oppression influencing the relationship · ACA Code of Ethics — B.1.d Explanation of Limitations

☐ Approve ☐ Revise ☐ Retire Notes:

A 55-year-old Haitian Creole-speaking woman comes to an intake for depression with her adult son, who offers to interpret. As the session goes on, he answers some questions without asking her and gives brief summaries of her longer replies. What is the BEST course of action?

A. Continue with the son, since he knows her history and she seems comfortable

Comfort does not offset the filtering already happening; her voice and privacy are being shaped by a family member.

B. Arrange a trained interpreter, explaining it protects her privacy and voice ✓ KEY

A trained interpreter renders her words fully and neutrally, restoring her voice and confidentiality that family interpreting compromises.

C. Ask the son to translate word for word and to stop adding his own answers

Instructions do not remove the family role, power dynamics, or limits on what she can disclose in front of her son.

D. Rely on her limited English so that no third party needs to be in the room

Forcing her to use a language she does not command well silences emotional content and deepens the power imbalance.

Rationale: Language carries power: a client who cannot speak for herself in her own language loses control over her story. Family interpreters, however willing, may filter, edit or answer for the client, and their presence limits disclosure of sensitive material. Using a trained professional interpreter is the standard of practice that protects accuracy, confidentiality and the client's voice.

References: Sue & Sue (Counseling the Culturally Diverse) — Language barriers; problems with using family members as interpreters · ACA Code of Ethics — A.2.c Developmental and Cultural Sensitivity

☐ Approve ☐ Revise ☐ Retire Notes:

A 50-year-old Black man who was once hospitalized involuntarily says at intake, "You've got all those letters after your name. The last one decided what was wrong with me and never asked me a thing." Which counselor response BEST addresses the power differential?

A. My training is extensive, so you can trust that I'll get the diagnosis right.

Leaning on credentials reasserts the expert power the client distrusts and dismisses his past experience.

B. That provider was probably following protocol, so try not to take it personally.

Defending the system invalidates his experience and aligns the counselor with the institution he distrusts.

C. Let's not focus on the past; what matters is the work we're going to do now.

Redirecting away from his experience avoids the trust issue he has raised and quietly silences it.

D. That sounds like it left you unheard. You know your life best; let's work together. ✓ **KEY**

This validates his experience, names him as the expert on his life, and invites a collaborative relationship that shares power.

Rationale: Credentials give counselors real institutional power, including the power to diagnose and document. Clients with histories of coercive or dismissive care, often compounded by systemic racism, may reasonably distrust that power. Acknowledging the experience and positioning the client as a collaborator and the expert on his own life helps balance the relationship.

References: MSJCC (Ratts et al., 2016) — Counseling relationship: counselor and client privileged and marginalized statuses · Sue & Sue (Counseling the Culturally Diverse) — Cultural mistrust and the credibility of the helping professional

☐ Approve ☐ Revise ☐ Retire Notes:

Q978 of 1000 · nce-s-cor-058 · Counseling and Helping Relationships · empathy and its levels · medium · In review

A 78-year-old man tells his 27-year-old counselor, "You're young enough to be my granddaughter. What could you know about burying a wife of 55 years?" Which counselor response BEST conveys empathy?

A. Age doesn't matter much here; I've been trained to work with grief at any age.

Citing training defends the counselor and sidesteps the doubt and loss the client is expressing.

B. You're right, I haven't lived that. I want to understand what losing her is like. ✓ **KEY**

This honestly acknowledges the generational gap and communicates a sincere wish to enter his world of loss.

C. I lost my grandmother last year, so I do know a little of what you're feeling.

This self-disclosure equates different losses and shifts the focus to the counselor's experience.

D. Would you feel more comfortable seeing a counselor who's closer to your age?

Offering a transfer at the first sign of doubt avoids the relationship rather than responding to his feelings.

Rationale: Empathy does not require shared experience; it requires trying to understand the client's world from his frame of reference. With a client of a different generation, the counselor can honestly acknowledge the difference and convey genuine interest in learning his experience, which often strengthens credibility more than defending one's qualifications.

References: Corey (Theory & Practice) — Person-centered therapy: accurate empathic understanding of the client's frame of reference

☐ Approve ☐ Revise ☐ Retire Notes:

A counselor at a refugee resettlement agency tells her supervisor that she cries alongside clients as they describe their losses, then feels drained and has trouble thinking clearly in session. The supervisor suggests she shift the balance of her empathy. Which shift is the supervisor MOST likely recommending?

A. More affective empathy, so that clients can see how deeply she shares their pain

She is already highly affectively attuned; adding more emotional sharing would deepen the overwhelm she reports.

B. Expressing sympathy and pity instead, since they are less taxing to the counselor

Sympathy feels for the client from outside their frame of reference and is not a substitute for empathy.

C. More cognitive empathy, grasping the client's view while keeping her own footing ✓ KEY

Cognitive empathy, accurate perspective-taking with emotional separateness, keeps the "as if" quality and protects clear thinking.

D. Emotional detachment, keeping reflections of feeling to an absolute minimum in session

Detachment would abandon empathy altogether and damage the relationship rather than rebalance it.

Rationale: Affective empathy is feeling with the client; cognitive empathy is accurately understanding the client's perspective. Rogers described empathy as sensing the client's world "as if" it were one's own without losing the "as if" quality. When affective resonance turns into personal distress, strengthening the cognitive side helps counselors stay present and effective while reducing burnout and vicarious trauma risk.

References: Corey (Theory & Practice) — Person-centered therapy: accurate empathic understanding and the "as if" quality · Davis (Multidimensional Empathy) — Perspective taking (cognitive) vs empathic concern and personal distress (affective)

☐ Approve ☐ Revise ☐ Retire Notes:

In person-centered counseling, offering unconditional positive regard means the counselor

A. values the client as a person without requiring approval of every behavior ✓ KEY

UPR is acceptance of the person's worth that does not depend on the client's behavior; it is not approval of all actions.

B. agrees with the client's decisions so the client feels supported and safe

UPR does not require agreeing with choices; acceptance of the person is distinct from endorsement of decisions.

C. offers frequent praise to raise the client's self-esteem in each session

Praise is conditional by nature, tied to things the counselor approves of, which is the opposite of unconditional regard.

D. keeps a neutral blank screen so that clients project feelings onto it

The blank screen is a psychoanalytic stance used to foster transference, not person-centered acceptance.

Rationale: Rogers described unconditional positive regard as a nonpossessive caring and acceptance of the client as a person of worth, without conditions. It allows clients to explore feelings and behaviors without fearing the loss of the counselor's regard. Acceptance of the person does not mean approval of every behavior.

References: Corey (Theory & Practice) — Person-centered therapy: unconditional positive regard and acceptance

☐ Approve ☐ Revise ☐ Retire Notes:

A 23-year-old client starting treatment for panic attacks asks, "Just tell me straight. Is this going to fix me?" The counselor believes the approach is likely to help but cannot be certain it will work for him. Which response BEST demonstrates congruence?

- A. Yes, this approach works for nearly everyone, so you'll be panic-free soon.
Overpromising misrepresents what the counselor actually believes and could undermine trust if progress is slow.
- B. What do you think it would mean for you if this didn't end up working out?
Deflecting a direct question with a question avoids being genuine with a client who asked for honesty.
- C. It's too early in our work for me to give you any kind of answer about that.
This withholds the counselor's honest, informed view, which she does in fact have.
- D. I can't promise that, but many people improve, and we'll track how it goes. ✓ KEY
This matches what the counselor genuinely believes, offering honest hope without false certainty.

Rationale: Congruence means the counselor's outward communication matches inner experience. When a client asks a direct question, a genuine counselor answers honestly, which here means conveying realistic hope while admitting uncertainty. False reassurance and evasion are both incongruent and can erode the trust that genuineness is meant to build.

References: Corey (Theory & Practice) — Person-centered therapy: congruence or genuineness

☐ Approve ☐ Revise ☐ Retire Notes:

A 48-year-old client with marked paranoid features stiffens when the counselor mentions his former employer. "How do you know where I worked? Have you been talking to people about me?" The detail came from the intake form he completed. Which response is MOST likely to protect the working alliance?

- A. Reassure him warmly that she is on his side and that he is completely safe here
Effusive warmth and reassurance tend to raise suspicion in clients with paranoid features rather than ease it.
- B. Gently note that his reaction may reflect a pattern of mistrust worth exploring
Interpreting his suspicion in this moment feels accusatory and is likely to deepen the rupture.
- C. Calmly say it came from his intake form and offer to show him the form itself ✓ KEY
A plain, factual and transparent answer respects his concern and demonstrates that nothing is being hidden.
- D. Apologize for bringing it up and move the conversation on to another subject
Dropping the topic without explanation leaves his suspicion intact and can look like evasion.

Rationale: Clients with paranoid features are highly sensitive to hidden motives. A straightforward, consistent and transparent style, explaining exactly where information came from and how it is used, builds trust better than warmth, interpretation or avoidance. Repairing a moment of suspicion this way strengthens the bond component of the alliance.

References: Gladding (Counseling: A Comprehensive Profession) — Working alliance: building trust and repairing ruptures · DSM-5-TR — Paranoid personality disorder: pervasive distrust and suspiciousness of others

► **Check:** The recommendation of a matter-of-fact, transparent (not effusively warm) style with paranoid clients is standard clinical guidance but is not stated in DSM-5-TR itself; DSM is cited for the feature only.

☐ Approve ☐ Revise ☐ Retire Notes:

A counselor in a community reentry program, herself a mother of young children, is assigned a 35-year-old man who served a sentence for physically abusing a child. She notices she is curt with him, ends sessions early, and dreads his appointments. What is the MOST appropriate action?

A. Refer him to a colleague right away, since her values conflict with his past

ACA A.11.b cautions against referral based solely on the counselor's personal values; she should first examine and manage the reaction.

B. Explore her reaction in supervision and watch how it is affecting his care ✓ **KEY**

Supervision helps her understand and manage negative countertransference so it stops shaping her behavior toward him.

C. Tell him her feelings openly so he understands the impact of his offense

Disclosing her reaction this way would be punitive and serve her needs rather than his treatment.

D. Keep the shorter sessions, since his offense limits what counseling can do

Continuing reduced care acts out the countertransference and denies him equitable treatment.

Rationale: Negative countertransference toward clients who have caused harm can show up as coldness, shortened sessions and avoidance. The counselor's task is to recognize the reaction, explore it in supervision, and keep it from compromising care. ACA A.11.b asks counselors to refrain from referring prospective and current clients based solely on personally held values; referral is for genuine lack of competence after efforts to manage the reaction.

References: Corey (Theory & Practice) — Countertransference: recognizing and managing negative reactions to clients · ACA Code of Ethics — A.11.b Values Within Termination and Referral

☐ Approve ☐ Revise ☐ Retire Notes:

At his first session, a 26-year-old graduate student from Ghana whose name is Kwabena says, "Most people here just call me Kevin. It's easier." Which counselor response BEST conveys respect?

A. I'm happy to use whichever you prefer. If Kwabena, will you help me say it? ✓ **KEY**

This leaves the choice with him and shows willingness to learn his name, conveying respect without pressure.

B. Kevin is fine with me, then. It will keep things simple for both of us here.

Accepting the anglicized name for convenience skips asking what he prefers and centers ease over his identity.

C. I'll call you Kwabena, because it's important to honor your true heritage.

Insisting on his birth name overrides his choice; respect means honoring his preference, whatever it is.

D. Do you feel people here haven't respected your culture since you arrived?

Jumping to a probing question about discrimination presumes a problem before learning what he wants to be called.

Rationale: Respect includes honoring how clients wish to be addressed. Rather than defaulting to convenience or imposing a choice, the counselor asks for his preference and offers to learn correct pronunciation. Small acts like this signal that the client's identity matters and that he directs how it is represented.

References: Gladding (Counseling: A Comprehensive Profession) — Building a counseling relationship: warmth and respect

☐ Approve ☐ Revise ☐ Retire Notes:

A counselor in long-term recovery through a 12-step program works at an outpatient clinic. Reviewing her notes, she sees that she steers nearly every client toward meetings and describes clients who prefer SMART Recovery or medication for opioid use disorder as "not ready." Which step BEST reflects counselor self-awareness?

A. Keep recommending 12-step meetings, since her own recovery shows how well that path works

This generalizes from one personal experience to all clients and leaves the bias in place.

B. Tell each client about her own recovery so they understand why she favors 12-step meetings

Self-disclosure here serves to justify the counselor's preference and adds pressure; it does not examine the bias.

C. Recognize that her own path is shaping her advice, and explore each client's preferred route ✓ **KEY**

Naming how her history colors her judgment, then centering each client's choice, is the core of self-awareness and avoids imposing values.

D. Stop mentioning any mutual-help groups so that she cannot influence clients' choices at all

Withholding useful options overcorrects; the aim is to offer choices without bias, not to remove information.

Rationale: Counselors' personal histories, including their own recovery, can quietly shape what they recommend and how they label clients. Self-awareness means noticing that pattern, examining it (often in supervision), and returning the choice to the client. ACA Standard A.4.b asks counselors to be aware of and avoid imposing their own values, attitudes and beliefs.

References: ACA Code of Ethics — A.4.b Personal Values · Corey (Theory & Practice) — The counselor as a person: personal needs, values and their influence on counseling

☐ Approve ☐ Revise ☐ Retire Notes:

A counselor leading a hospital support group for family caregivers reviews a session recording. She notices that she invites the two members who are nurses to speak more often and responds to them at length, while briefly thanking a member with little formal education. What should the leader do?

A. Reflect on the pattern in supervision and deliberately balance her attention among members ✓ **KEY**

Examining the bias and then consciously equalizing attention addresses both the leader's self-knowledge and the group climate.

B. Ask the two nurses to share less often so that the other members have more time to speak

This places the burden on members for a pattern the leader created and leaves her own bias unexamined.

C. Tell the whole group she has been favoring some members, and apologize to all of them at once

A group-wide confession without first understanding the pattern may embarrass members and centers the leader.

D. Continue as before, because members with clinical training contribute more useful information

This rationalizes the bias; every member's experience is valuable, and unequal attention signals unequal worth.

Rationale: Group leaders communicate members' worth through whom they call on and how fully they respond. Unequal attention based on education or status is a form of bias that can silence members and weaken cohesion. Best practice asks group workers to know their own values and blind spots and to attend to diversity within the group.

References: ASGW Best Practice Guidelines — Best practice in performing: self-knowledge and attention to diversity among members

☐ Approve ☐ Revise ☐ Retire Notes:

A counselor in training asks her supervisor how to become more aware of her own cultural biases. Which activity MOST directly builds this kind of self-awareness?

A. Reading research summaries that describe the values of many different cultural groups

This builds knowledge about others, a different competency from awareness of one's own biases.

B. Treating every client in exactly the same way so that cultural differences stop mattering

A "colorblind" stance ignores culture and leaves the counselor's own assumptions unexamined.

C. Limiting her caseload to clients whose backgrounds closely match her own life experience

Avoiding difference prevents growth and does nothing to surface her assumptions.

D. Examining how her upbringing shaped her values, through reflection and supervision ✓ **KEY**

Looking inward at how her own cultural history formed her values and reactions is the essence of counselor self-awareness.

Rationale: Self-awareness is the counselor's understanding of how their own cultural background, values and assumptions shape their perceptions of clients. It grows through deliberate self-reflection, such as journaling or a cultural genogram, and through supervision and feedback. Learning about other groups is valuable but builds knowledge rather than self-awareness.

References: Sue & Sue (Counseling the Culturally Diverse) — Counselor awareness of own assumptions, values and biases · MSJCC (Ratts et al., 2016) — Counselor self-awareness domain

☐ Approve ☐ Revise ☐ Retire Notes:

A perfectionistic 20-year-old engineering student has felt worthless since childhood. The counselor notices that she becomes animated and praises him when he reports top grades, but grows quiet when he describes setbacks. From a person-centered view, what is the MAIN concern?

A. Praising his achievements may slowly weaken his motivation to keep his grades high

Motivation is not the person-centered concern here; the issue is what her shifting warmth teaches him about his worth.

B. Her responses may repeat the conditions of worth that feed his sense of worthlessness ✓ **KEY**

Warmth that rises and falls with performance is conditional regard, echoing the conditions of worth that Rogers saw as the root of incongruence.

C. He may come to depend on the counselor's approval more than on support from his peers

Dependence could follow, but the central problem is that her regard has become conditional on achievement.

D. Talking about setbacks at length may reinforce a negative core belief about himself

This is a cognitive framing; person-centered theory sees exploring setbacks in an accepting climate as helpful.

Rationale: Rogers proposed that people internalize conditions of worth when they are valued only for meeting others' standards, producing incongruence and fragile self-regard. Unconditional positive regard offers acceptance that does not depend on performance. A counselor whose warmth tracks the client's achievements can unintentionally recreate the very conditions that brought him to counseling.

References: Corey (Theory & Practice) — Person-centered therapy: conditions of worth and unconditional positive regard

☐ Approve ☐ Revise ☐ Retire Notes:

Which counselor statement BEST illustrates unconditional positive regard?

A. I'll keep supporting you as long as you keep working on the goals we set together.

Support that depends on meeting goals is conditional regard.

B. I care about you as a person, whether or not we see this choice the same way. ✓ KEY

This separates acceptance of the person from agreement with a choice, which is the heart of unconditional positive regard.

C. You're a model client. You do everything I suggest for you between our sessions.

Praise tied to compliance signals that the client is valued for doing what the counselor wants.

D. I'm proud of you for making the right decision this week instead of the old one.

This evaluates the client's choice as right or wrong, implying regard depends on the "right" behavior.

Rationale: Unconditional positive regard is Rogers' term for accepting and caring for the client as a person without conditions. It does not require approving of every behavior, but the counselor's valuing of the client does not rise or fall with the client's choices. Praise tied to compliance or "right" decisions conveys conditional acceptance.

References: Corey (Theory & Practice) — Person-centered therapy: unconditional positive regard and acceptance

☐ Approve ☐ Revise ☐ Retire Notes:

In the fifth session, a counselor refers to a grieving client's late son as "Marcus." The client stiffens and says quietly, "His name was Martin." Which counselor response BEST demonstrates congruence?

A. Of course, Martin. Let's go back to what you were saying about the funeral service.

Glossing over the error hides the counselor's real experience and leaves the client's hurt unacknowledged.

B. I see many clients, and names sometimes get mixed up. I'm sorry that it happened.

The excuse is defensive and minimizes the mistake, shifting attention to the counselor's workload.

C. I'm fairly sure the name was written that way in my notes, so I'll check them later.

This deflects responsibility to the paperwork rather than owning the error.

D. I got his name wrong, and I'm sorry. Martin matters, and I want to get it right. ✓ KEY

The counselor openly owns the mistake and conveys real care, so words and inner experience match.

Rationale: Congruence means the counselor is genuine: what they express matches what they actually experience, without a professional facade. When a counselor makes a mistake, a congruent response owns it simply and honestly rather than hiding, excusing or deflecting it. Such moments, handled openly, often strengthen trust.

References: Corey (Theory & Practice) — Person-centered therapy: congruence, or genuineness

☐ Approve ☐ Revise ☐ Retire Notes:

A trainee tells her supervisor, "To be congruent, I should tell clients every reaction I have toward them as soon as I notice it." Which reply from the supervisor MOST accurately describes congruence in person-centered theory?

A. Congruence means your inner experience and outward expression match, not voicing every feeling ✓ **KEY**

Rogers described congruence as awareness and genuineness, with no facade; it does not require impulsively sharing every reaction.

B. Congruence is mainly about matching your body language to the client's posture and vocal tone

This describes mirroring, an attending technique, not genuineness.

C. Congruence requires sharing your personal history so the client sees you as an equal partner

Self-disclosure of history is a separate skill and is not what congruence requires.

D. Congruence is reached by keeping a calm, neutral manner no matter what you feel in a session

Hiding behind a neutral front is the professional facade that congruence rejects.

Rationale: In person-centered theory, congruence means the counselor's inner experience, awareness and outward expression are consistent, so there is no false front. It does not mean impulsively sharing every feeling. Counselors use judgment about expressing persistent reactions that are relevant to the relationship, and they do so in a way that serves the client.

References: Corey (Theory & Practice) — Person-centered therapy: congruence does not mean impulsive sharing of all reactions

☐ Approve ☐ Revise ☐ Retire Notes:

A 45-year-old client seeking help with heavy weekend drinking says his goal is to cut back to two drinks a night, not to quit. He has no history of withdrawal. The counselor believes abstinence would be safer. Which approach BEST strengthens the working alliance?

A. Explain that moderation goals rarely succeed, and make abstinence the first treatment goal

Imposing the counselor's goal undermines agreement on goals, a core component of the alliance.

B. Accept his goal without discussing it at all, so that he does not feel pressured or judged

Silently withholding a relevant concern is not genuine collaboration and skips mutual goal-setting.

C. Share her concern with permission, then agree with him on a goal and tasks he will try ✓ **KEY**

Offering her view respectfully while negotiating shared goals and tasks builds the agreement and bond the alliance depends on.

D. Put off any goal setting until he has kept a drinking diary for several weeks on his own

Monitoring can help, but delaying all goal discussion leaves the alliance without shared direction.

Rationale: Bordin described the working alliance as agreement on goals, agreement on tasks, and an emotional bond. When client and counselor see the goal differently, the counselor can share information with permission while respecting the client's autonomy and negotiating a goal both can work toward. Imposing a goal, or hiding a concern, weakens that collaboration.

References: Gladding (Counseling: A Comprehensive Profession) — The working alliance: goals, tasks and bond · Miller & Rollnick (MI) — Exchanging information with permission; supporting autonomy in goal choice

☐ Approve ☐ Revise ☐ Retire Notes:

At the first meeting of a 10-week court-mandated anger management group, several members say they are only there to get their paperwork signed. Which leader action is MOST likely to begin building a working alliance with the members?

A. Review the attendance rules and the consequences of missing sessions for each member

Rules need to be clear, but leading with consequences emphasizes control rather than shared purpose.

B. Remind them that their anger brought them here, so they should take the group seriously

Lecturing increases defensiveness and does nothing to create agreement on goals.

C. Show a video outlining the curriculum so that members know what the group expects of them

This presents tasks without the members' input, missing agreement on goals and the bond.

D. Acknowledge the mandate and invite each member to name a goal worth working on for them ✓ KEY

Validating the reality of the mandate while eliciting personal goals starts agreement on goals and builds the bond.

Rationale: With mandated members, the alliance begins by acknowledging their situation honestly and finding goals that matter to them within the group's requirements. Bordin's components of goals, tasks and bond apply in groups as in individual work. Leading with rules or lectures tends to deepen resistance rather than engagement.

References: Gladding (Counseling: A Comprehensive Profession) — Working alliance with reluctant and involuntary clients; goals, tasks and bond

☐ Approve ☐ Revise ☐ Retire Notes:

A 34-year-old client who uses a power wheelchair arrives for an intake with a personal care attendant. Which counselor behavior BEST conveys respect?

A. Speaking directly to the client and asking how they would like the room arranged ✓ KEY

Addressing the client directly and inviting their preferences treats them as the expert on their own needs.

B. Directing the intake questions to the attendant, who likely knows the client's needs

Talking past the client to a companion is disrespectful and undermines the client's autonomy.

C. Moving the wheelchair closer to the desk so the client does not have to adjust it

A wheelchair is part of the client's personal space; moving it without asking is intrusive.

D. Telling the client how inspiring it is that they still get out and come to counseling

Calling routine activity "inspiring" is patronizing and frames disability as the client's defining trait.

Rationale: Respect means treating clients as capable adults and the experts on their own lives. With clients who have disabilities, that includes speaking to them directly rather than through companions, asking before offering help, and avoiding patronizing praise. These behaviors communicate that the client, not the disability, is the focus.

References: Sue & Sue (Counseling the Culturally Diverse) — Counseling individuals with disabilities: addressing the person directly and avoiding paternalism

☐ Approve ☐ Revise ☐ Retire Notes:

In a school counseling group for 14-year-olds coping with family changes, a usually quiet member describes crying the night her father moved out. Another member snickers. Which leader response BEST conveys warmth and respect while protecting the group?

A. Ignore the snicker so that the attention stays on the member who just took a risk

Ignoring the behavior leaves the sharer exposed and implies that mocking is acceptable in the group.

B. Calmly block the snicker, then thank the member for trusting the group with this ✓ **KEY**

Blocking protects the vulnerable member, and the warm acknowledgment honors her risk and reinforces safety norms.

C. Ask the member who snickered to leave the room until he is able to act respectfully

Removal is a heavy first response that shames the other teen and shifts focus away from the sharer.

D. Ask the group to vote on whether laughing at another member should be against the rules

Putting safety to a vote in the moment leaves the sharer unprotected and makes respect negotiable.

Rationale: Group leaders shape norms of safety and support, especially early and with adolescents. When a member takes a risk and is met with ridicule, the leader blocks the hurtful behavior promptly and then affirms the member who disclosed. Doing both conveys warmth to the individual and respect for the group as a safe place.

References: Yalom & Leszcz (Group Psychotherapy) — The therapist's basic tasks: culture building and shaping norms of safety and support

☐ Approve ☐ Revise ☐ Retire Notes:

A 52-year-old client tearfully says she has decided to move her mother, who has advanced dementia, into a memory care facility. The counselor grew up in a family where caring for aging parents at home was expected. Which response BEST reflects a nonjudgmental stance?

A. Did you look into all of the in-home care options before deciding on a facility for her?

The question implies the decision was not thought through and carries the counselor's own family values.

B. Many families do find a way to keep a parent at home if they get enough outside help.

This subtly argues for the counselor's preferred choice and adds to the client's guilt.

C. This decision is weighing on you. Tell me what went into it and how you're feeling. ✓ **KEY**

The counselor reflects the client's feeling and invites her story without evaluating the choice.

D. You made the right choice. Memory care is much better for her than staying at home.

Approval is still a judgment; it closes off the client's mixed feelings instead of exploring them.

Rationale: A nonjudgmental stance withholds both disapproval and approval so that the client can explore her own decision and feelings. Here the counselor's family norms create a pull to question the choice, which ACA Standard A.4.b cautions against. Reflecting the emotional weight and inviting exploration keeps the focus on the client's values.

References: ACA Code of Ethics — A.4.b Personal Values

☐ Approve ☐ Revise ☐ Retire Notes:

A counselor helping a client with job stress learns the client drinks six to eight beers most nights. Wanting to stay nonjudgmental, the counselor decides not to ask about the drinking because the client has not raised it as a concern. Which statement BEST evaluates this decision?

- A. It is sound, because exploring an issue the client has not raised imposes counselor values
Assessing a clinically relevant behavior is not value imposition; ignoring it may leave a real risk unaddressed.
- B. It is sound, because asking about substance use is likely to damage the working alliance
Respectful, curious inquiry seldom harms the alliance; avoiding the topic sacrifices sound care.
- C. It is unsound, because the counselor should first warn the client about the health risks
Leading with a warning is directive and likely to provoke defensiveness; assessment and permission come first.
- D. It is unsound, because being nonjudgmental withholds moral judgment, not clinical inquiry ✓ KEY
A nonjudgmental stance concerns how the counselor regards the person, not whether clinically important issues are explored.

Rationale: Being nonjudgmental means suspending moral evaluation of the client, not suspending clinical judgment. Heavy nightly drinking may bear directly on job stress, health and safety, so the counselor should raise it, ideally by asking permission and exploring with curiosity. Acceptance and assessment work together.

References: Miller & Rollnick (MI) — Acceptance in the spirit of MI; raising a concern with permission

☐ Approve ☐ Revise ☐ Retire Notes:

An intake form lists a 30-year-old client's background as "Puerto Rican and Irish." The client has come for help with work stress and uses they/them pronouns. Which approach BEST reflects cultural humility?

- A. Ask how the client describes their identity and which parts of it matter for this concern ✓ KEY
Inviting the client to define their identity and its relevance treats them as the expert on their own culture.
- B. Review literature on Puerto Rican and Irish family values before the next session begins
Group knowledge can inform hypotheses, but it risks substituting generalizations for the client's own account.
- C. Avoid raising culture at all, since the client came in about work stress and not identity
Assuming culture is irrelevant forecloses exploration; identity may shape how work stress is experienced.
- D. Assume the client identifies mostly as Puerto Rican, since that heritage is listed first
Inferring identity from the order of words on a form is an assumption, not humility.

Rationale: Cultural humility emphasizes openness and curiosity about how each client understands their own identities, rather than presuming. Multiracial and multiethnic clients in particular may identify in many ways, and those identities may matter more or less for a given concern. Asking, rather than assuming, keeps the client at the center.

References: Sue & Sue (Counseling the Culturally Diverse) — Multiracial individuals: self-identification and avoiding imposed labels · MSJCC (Ratts et al., 2016) — Client worldview: understanding clients' multiple identities

☐ Approve ☐ Revise ☐ Retire Notes:

A White counselor suggests that a 24-year-old Black client ease evening anxiety by taking long walks through his suburban neighborhood after dark. The client pauses and says, "That's not as simple for me as it might be for you." Which response BEST shows awareness of privilege?

A. Walking really helps with anxiety, so let's find some ways to make it work for you anyway.

This pushes past the client's concern and keeps the counselor's experience as the default.

B. You're right, I assumed. Help me understand what walking at night is like for you. ✓ **KEY**

The counselor owns that the suggestion reflected her own experience and invites the client's reality.

C. I didn't mean anything by it. I suggest walking to all of my clients who have anxiety.

Defending intent and claiming equal treatment overlooks that the same advice carries different risks.

D. Let's skip walking, then. Breathing exercises are something you can do safely at home.

Switching techniques avoids the issue the client raised and misses the chance to repair and learn.

Rationale: Privilege often shows up as assumptions about what is safe or easy, drawn from the counselor's own experience. When a client signals that a suggestion does not fit his reality, the counselor should acknowledge the assumption without defensiveness and explore the client's experience. This repairs trust and reduces the power imbalance in the relationship.

References: MSJCC (Ratts et al., 2016) — Counseling relationship: how privilege and oppression influence the counselor-client relationship · Sue & Sue (Counseling the Culturally Diverse) — White privilege and counselor assumptions

☐ Approve ☐ Revise ☐ Retire Notes:

Q1000 of 1000 · nce-s-cor-080 · Counseling and Helping Relationships · empathy and its levels · easy · In review

A client says, "I finished my degree at 52, and none of my kids came to the graduation. I guess it's no big deal." The counselor replies, "Congratulations! What are your plans now that you've graduated?" On Carkhuff's empathy scale, this response is BEST classified as

A. Interchangeable (Level 3)

An interchangeable response would reflect both the pride and the hurt the client expressed.

B. Additive (Level 4)

An additive response would go beyond the client's words to name deeper feelings, such as disappointment.

C. Subtractive (below Level 3) ✓ **KEY**

The reply ignores the hurt about the missing children and moves to a new topic, taking away from what the client expressed.

D. Additive (Level 5)

Level 5 responses add significantly to the client's feelings and meaning; this one omits the main feeling.

Rationale: On Carkhuff's five-level scale, Level 3 is interchangeable with the client's expressed feeling and meaning, Levels 4 and 5 are additive, and Levels 1 and 2 are subtractive. A response that attends to surface content while missing the core feeling, here the hurt behind "no big deal," falls below the minimally facilitative level.

References: Gladding (Counseling: A Comprehensive Profession) — Carkhuff's scale of empathic understanding: subtractive, interchangeable and additive responses

☐ Approve ☐ Revise ☐ Retire Notes: